

Auszug aus der UAW-Datenbank des BfArM

Stand der Datenbank: 18.03.2013

Einzelfallausdrucke

Kriterien: Ibuprofen, Deutschland ab 1993, SMQ Hepatic disorders

Hinweise zur Interpretation der Daten:

Es handelt sich bei den Berichten sowohl um Spontanmeldungen als auch um Berichte aus systematisierten Untersuchungen, aus Projekten oder aus der Literatur. Die Meldungen sind dem Bundesinstitut für Arzneimittel und Medizinprodukte durch pharmazeutische Unternehmer, durch die Arzneimittelkommissionen der Heilberufe, direkt durch niedergelassene oder in Krankenhäusern tätige Ärzte sowie durch sonstige Meldequellen, z. B. von Patienten, berichtet worden. Es muss beachtet werden, dass es sich um Verdachtsfälle unerwünschter Arzneimittelwirkungen handelt, dass also ein Kausalzusammenhang im Einzelfall nicht sicher belegt ist. Ferner lassen sich auf der Grundlage von Spontanberichten keine Aussagen darüber machen, wie häufig eine bestimmte unerwünschte Wirkung unter der Anwendung eines Arzneimittels vorkommt, und auch vergleichende Angaben darüber, wie viel mal häufiger eine bestimmte unerwünschte Wirkung bei einem Arzneimittel im Verhältnis zu einem anderen Medikament auftritt, sind anhand solcher Berichte kaum möglich.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94004210

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19590104 Age: 35 Year weight (kg): 56 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal pain	199402				not recovered/not resolved
2 Hepatic enzymes increased	199402				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibu-vivimed ibuprofen	Oral	002 Df dosage form/Day	- -	Migraine, unspecified

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95003691

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: 19470120	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic cirrhosis					Unknown
2 Coagulation disorder					Unknown
3 Icterus					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal	Oral	/	-	
ibuprofen			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96012168

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: Age: 11 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt	19960423				Unknown
2 Prothrombin time increased	19960423				Unknown
3 Drowsiness	19960423				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin	Oral	/	-	
ibuprofen lysinate			-	
2 zynafluor d 500	Oral	/	-	
colecalfiferol; sodium fluoride			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97005797

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19270227 Age: 70 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pancreatitis	19970610				Unknown
2 Hepatitis toxic	19970610				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	-	
ibuprofen			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98005913

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: 19650518 Age: 33 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	199807				Unknown
2 Icterus	199807				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin	Oral	/	-	
ibuprofen lysinate			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006936

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19260917 Age: 71 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal discomfort	19980826				Unknown
2 Hepatitis	19980826				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	-	
ibuprofen			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000345

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: Age: 40 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	19980612				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibu-ratiopharm ibuprofen	Unknown	/	-	
2 neurocil levomepromazine maleate	Unknown	/	-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001183

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19260319 Age: 69 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	19951102				Unknown
2 Stevens Johnson syndrome	19951026				Unknown
3 Leukocytosis	19951102				Unknown
4 SGPT increased	19951102				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 imbun	Oral	/	-	
ibuprofen lysinate			-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00500245

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19431106	Age: 55 Year	weight (kg): 93	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver enzymes	199909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid		001 G gram(s)/Day	19990915-19990925	Unspecified infectious and parasitic diseases
2 augmentan amoxicillin trihydrate; clavulanic acid	Oral	1.5 G gram(s)/Day	19990922-19990924	Other diseases of lung, not elsewhere classified
3 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/Day	19990922-19990924	Other general symptoms

Primary source

Qualification: observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00004289

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: 19780314 Age: 18 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Extrasystole ventricular	19961121				Unknown
2 Cardiac arrest	19961121				Unknown
3 Sinus tachycardia	19961121				Unknown
4 Hepatic damage	19961121				Unknown
5 Rhabdomyolysis	19961121				Unknown
6 Memory disturbance	19961121				Unknown
7 Fever	19961121				Unknown
8 Coma	19961121				Unknown
9 Movements spastic involuntary	19961121				Unknown
10 Hypoxic brain damage	19961121				Unknown
11 Shock	19961121				Unknown
12 Overdose intentional	19961121				Unknown
13 Renal failure acute	19961121				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 anafranil clomipramine hydrochloride	Oral	/	-	-
2 aurorix moclobemide	Oral	/	-	-
3 betadorm carbromal; diphenhydramine hydrochloride	Oral	/	-	-
4 metoprolol metoprolol	Intravenous (not otherwis	/	-	-
5 urem forte ibuprofen	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02002573

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: 19440325 Age: 57 Year weight (kg): 54 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20011222				recovered/resolved
2 Faeces pale	20011228				recovered/resolved
3 Urine discolouration	20011228				recovered/resolved
4 Weakness generalized	20011222				recovered/resolved
5 Nausea	20011222				recovered/resolved
6 Cholestasis	2001				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/Day	-20011215	Rheumatoid arthritis
2 thiamazol "henning" thiamazole	Oral	040 Mg milligram(s)/Day	20011213-20011228	Thyrotoxicosis without mention of goitre or other cause

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified iridocyclitis			k.A.	concomitant disease
Neoplasm of unspecified nature of endocrine glands and oti			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.linsenimplantation li
Other specified disorders of gallbladder			k.A.	concomitant disease
Chronic renal failure			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02003649

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: 19510814	Age: 48 Year	weight (kg): 82	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestasis	20000706				Unknown
2 Faeces pale	20000706				Unknown
3 Icterus	20000706				Unknown
4 Raised liver enzymes	20000501				Unknown
5 Hepatotoxic effect	200007				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 anco 600mg filmtabletten ibuprofen	Oral	002 Df dosage form/Day	199909-20000714	Arthralgia
2 mevinacor 20 lovastatin	Oral	001 Df dosage form/Day	199909-20000713	Other and unspecified hyperlipidaemia
3 neotigason acitretin	Oral	050 Mg milligram(s)/Day	199909-20000721	Other psoriasis

Drugs (concomitant)

4 zyloric 100 allopurinol	Oral	001 Df dosage form/Day	199909-	Hyperuricaemia
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other psoriasis			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: psoriasis-arthralien

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02003771

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19300111	Age: 70 Year	weight (kg): 63	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Foetor hepaticus	20000825				recovered/resolved
2 Renal failure acute	20000825		4 Day		recovered/resolved
3 Anuria	20000825				recovered/resolved
4 Confusion	20000825				recovered/resolved
5 Somnolence	20000825				recovered/resolved
6 Bradycardia	20000825		3 Day		recovered/resolved
7 Oliguria	20000825				recovered/resolved
8 Gamma-GT increased	20000825				recovered/resolved
9 Urea blood level increased	20000825				recovered/resolved
10 Hyperpotassaemia	20000825				recovered/resolved
11 Creatinine blood increased	20000825				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cipramil 20mg citalopram hydrobromide	Oral	002 Df dosage form/Day	20000816-20000825	Depression
2 digitoxin 0,07mg digitoxin	Oral	/Week	1995-20000825	Heart failure, unspecified
3 godamed 100 acetylsalicylic acid	Oral	001 Df dosage form/Day	20000807-20000825	Coronary atherosclerosis
4 ibuprofen 600 stada ibuprofen	Oral	001 Df dosage form/Day	20000823-20000825	Lumbago
5 obsidan tabletten 25 mg propranolol hydrochloride	Oral	002 Df dosage form/Day	20000823-20000825	Essential and other specified forms of tremor
6 pentalong 50 pentaerithrityl tetranitrate	Oral	002 Df dosage form/Day	1995-20000825	Heart failure, unspecified
7 silymarin silymarin	Oral	140 Mg milligram(s)/Day	20000817-20000825	Cirrhosis of liver without mention of alcohol
8 verospiron spironolactone	Oral	050 Mg milligram(s)/Day	1995-20000825	Heart failure, unspecified

Drugs (concomitant)

9 bikalm zolpidem tartrate	Oral	001 Df dosage form/Day	20000817-20000825	Specific disorders of sleep
10 levodopa levodopa	Oral	1.5 Df dosage form/Day	20000816-20000825	Paralysis agitans
11 maninil 3,5 glibenclamide	Oral	001 Df dosage form/Day	20000817-20000825	Diabetes mellitus without mention of complication
12 panzytrat 25.000 pancreatin	Oral	003 Df dosage form/Day	20000817-20000825	Other specified diseases of pancreas
13 truxal suspension 20mg/ml chlorprothixene hydrochloride	Oral	060 Gtt drop(s)/Day	20000816-20000825	Specific disorders of sleep
14 zopiclon r.p. zopiclone	Oral	7.5 Mg milligram(s)/Day	20000807-20000825	Specific disorders of sleep

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

case: DE-BFARM-02003539

Patient

Reactions

Drugs (suspected or interacting)

Drugs (concomitant)

Primary source

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02004587

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: 19620802	Age: 39 Year	weight (kg): 78	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	20020226				not recovered/not resolved
2 Oedema legs	20020226				not recovered/not resolved
3 Skin vasculitis NOS	20020226				not recovered/not resolved
4 Vascular purpura	20020226				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam 600 lichtenstein ibuprofen	Oral	600 Mg milligram(s)/Day	20020224-20020225	Arthralgia
			-	

Drugs (concomitant)

2 mtx hexal methotrexate sodium	Oral	13 Mg milligram(s)/Week	-	Psoriatic arthropathy
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Atopy			k.A.	concomitant disease
Allergic rhinitis due to pollen			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005572

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19780205	Age: 22 Year	weight (kg): 60	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatocellular damage NOS	20010110		6 Day		recovered/resolved
2 Abdominal pain aggravated	20010110				recovered/resolved
3 Dizziness	20010110				recovered/resolved
4 Vomiting	20010110				recovered/resolved
5 Diarrhea	20010110				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	001 G gram(s)/Day	20000128-20010103	Headache
2 ibutad 400 ibuprofen	Oral	800 Mg milligram(s)/Day	20001228-20010103	Other syndromes affecting cervical region
3 tetrazepam tetrazepam	Oral	075 Mg milligram(s)/Day	20001228-20010103	Other disorders of muscle, ligament, and fascia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Colitis, enteritis, and gastroenteritis of presumed infectious			k.A.	concomitant disease
Nondependent abuse of tobacco			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02013117

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No				Yes		

Patient

Initials:	Sex: Female	Birth: 19390208	Age: 63 Year	weight (kg): 83	BMI_int: 29.41	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20020923		30 Day		recovering/resolving
2 Raised liver enzymes	20020923		30 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	600 Mg milligram(s)/Day	200206-200209	Arthralgia
ibuprofen			3 Month	

Drugs (concomitant)

2 Kalzium	Unknown	/	-
calcium			-
3 oestrogen-pflaster	Transdermal	/	-
			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified arthropathy			k.A.	concomitant disease
Allergy, unspecified, not elsewhere classified			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03000446

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19350718	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic	200212				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Unknown	/	-	Polymyalgia rheumatica
2 ibuhexal ibuprofen	Unknown	/	-	Polymyalgia rheumatica
3 novalgin metamizole	Unknown	/	-	Polymyalgia rheumatica
4 taxilan perazine	Oral	100 Mg milligram(s)/Day	1991-200301	Unspecified psychosis
5 tramal tramadol hydrochloride	Unknown	/	-	Polymyalgia rheumatica

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03004262

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: 19331104	Age: 68 Year	weight (kg): 79	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	200205				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bezafibrate	Oral	400 Mg milligram(s)/Day	1972-20030114	Other and unspecified hyperlipidaemia
bezafibrate			-	
2 ibuprofen	Oral	400 Mg milligram(s)/	-200301	Low back pain
ibuprofen			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Calculus of gallbladder without mention of cholecystitis			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z. n. cholecystektomie 2000
unbekannt			k.A.	concomitant disease: z. n. hepatitis 1940
Other chronic nonalcoholic liver disease			k.A.	concomitant disease
Hepatomegaly			k.A.	concomitant disease
Unspecified disorder of liver			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-03008601

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes						

Patient

Initials:	Sex: Female	Birth: 20030501	Age: 10 Day	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20030516	deathcause: Collapse
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Prothrombin time prolonged	20030511				recovered/resolved
2 Decreased plasma fibrinogen	20030511				recovered/resolved
3 PTT prolonged	20030511				recovered/resolved
4 Haemorrhage NOS neonatal	20030511				recovered/resolved
5 Injection site reaction	20030511				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Intravenous (not otherwis	/Day	20030507-20030507	Patent ductus arteriosus
2 ibuprofen ibuprofen	Intravenous (not otherwis	/Day	20030510-20030511	Patent ductus arteriosus
3 ibuprofen ibuprofen	Intravenous (not otherwis	/Day	20030508-20030509	Patent ductus arteriosus

Drugs (concomitant)

4 claforan cefotaxime sodium	Unknown	/	20030508-20030515	-
5 dolantin pethidine hydrochloride	Unknown	/	20030502-20030516	-
6 dopamin dopamine hydrochloride	Unknown	/	20030504-20030515	-
7 dormicum midazolam maleate	Unknown	/	20030501-20030515	-
8 esidrix hydrochlorothiazide	Unknown	/	20030508-20030515	-
9 fortecortin dexamethasone	Unknown	/	20030510-20030515	-
10 luminal phenobarbital	Unknown	/	20030501-20030515	-
11 paracetamol paracetamol	Unknown	/	20030510-20030515	-
12 vancomycin vancomycin	Unknown	/	20030508-20030515	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Achondroplasia			k.A.	concomitant disease
Congenital anomaly, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Consumer or other non health professional	observestudytype:	Other studies
Literature:			

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-03008204

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: Age: 28 Year weight (kg): 64 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pancreatitis	200212				recovered/resolved
2 Cholelithiasis	200212				recovered/resolved
3 Liver function tests abnormal NOS	200212				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 spalt liqua ibuprofen	Oral	/	-	Headache
2 trigoa ethinylestradiol; levonorgestrel	Oral	1 Df dosage form/	200207- -	Contraception

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont. k.A.	Comment concomitant disease
Nondependent abuse of tobacco				

Primary source

Qualification: observestudytype: Other studies
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03009633

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19360419	Age: 67 Year	weight (kg): 87	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nausea	20030501				recovered/resolved
2 Appetite lost	20030501				recovered/resolved
3 Sweating attack	20030501				recovered/resolved
4 Epigastric ache	20030501				recovered/resolved
5 Urine discolouration	20030501				recovered/resolved
6 Icterus	20030501		22 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	1.8 G gram(s)/Day	20030424-20030430	Backache, unspecified
			-	

Drugs (concomitant)

2 berlthyrox 50 levothyroxine sodium	Oral	050 Mg milligram(s)/Day	2000-	Unspecified hypothyroidism
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n. cce 1985
Other unspecified back disorders			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: choledocholithiasis mit sonstiger cholez
Other specified disorders of biliary tract			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03010583

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: 19670227	Age: 36 Year	weight (kg): 88	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Itching	20030313				Unknown
2 Nausea	20030313				Unknown
3 Asthenia	20030313				Unknown
4 Hepatitis	20030313				Unknown
5 Upper abdominal pain	20030313				Unknown
6 Faeces pale	20030313				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	1.2 G gram(s)/Day	20030311-20030313	Acute bronchitis
2 ketek telithromycin	Oral	800 Mg milligram(s)/Day	20030311-20030313	Acute bronchitis

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03011892

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19540913	Age: 48 Year	weight (kg): 53	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	20030512				Unknown
2 Appetite lost	20030512				Unknown
3 Tiredness	20030512				Unknown
4 Fullness abdominal	20030512				Unknown
5 Hepatitis	20030512				recovering/resolving
6 Faeces pale	20030512				Unknown
7 Urine discolouration	20030512				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibu kd 800 ibuprofen	Oral	1 Df dosage form/Week	2000-20030521	Other general symptoms
2 neotigason 10 acitretin	Oral	003 Df dosage form/Day	20030109-20030511	Other psoriasis
3 omep 20 mg omeprazole	Oral	001 Df dosage form/Day	200105-20030520	Oesophagitis NOS

Drugs (concomitant)

4 betadermic betamethasone dipropionate; salicylic acid	Transdermal	1 Df dosage form/	1998-	Other psoriasis
5 diprosalic acetylsalicylic acid; betamethasone dipropionate	Transdermal	2 Df dosage form/Week	1998-	Other psoriasis
6 elacutan salbe urea	Transdermal	1 Df dosage form/	1998-	Other psoriasis
7 femikliman uno cimicifuga racemosa extract	Oral	001 Df dosage form/Day	200207-20030512	Unspecified menopausal and postmenopausal disorder
8 osteoplus calcium carbonate; colecalciferol	Oral	0.5 Df dosage form/Day	200205-	Osteoporosis
9 riopan-gel magaldrate	Oral	1 Df dosage form/Week	2000-200305	Oesophagitis NOS

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Kyphoscoliosis and scoliosis			k.A.	concomitant disease
Nondependent abuse of tobacco			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03013021

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 20030717	Age: 5 Day	weight (kg): .7	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Prothrombin time increased	20030722				recovered/resolved
2 C-reactive protein positive	20030722				recovered/resolved
3 Intestinal perforation	20030722		5 Hour		recovered/resolved
4 Abdominal discomfort	20030722		5 Hour		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 pedea ibuprofen	Intravenous (not otherwis /		20030718-20030725 8 Day	Patent ductus arteriosus
2 pedea ibuprofen	Intravenous (not otherwis /		20030718- -	Patent ductus arteriosus

Drugs (concomitant)

3 aminopaed amino acids nos	Unknown	/	-	-
4 aquaSol water	Unknown	/	-	-
5 clindamycin clindamycin	Unknown	/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Premature baby			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03015707

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19760908	Age: 26 Year	weight (kg): 82	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Thrombopenia	20030721				Unknown
2 Leucopenia	20030721				Unknown
3 Elevated liver enzyme levels	20030721				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	200 Mg milligram(s)/	-	Headache
ibuprofen			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Nondependent abuse of tobacco			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04001382

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes						

Patient

Initials:	Sex: Female	Birth: 19770920	Age: 26 Year	weight (kg): 90	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20031209	deathcause: Renal failure, unspecified
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20031124		13 Day		Unknown
2 Renal failure NOS	20031124				Unknown
3 Pericardial effusion	20031124				Unknown
4 Pleural effusion	20031124				Unknown
5 Pleuritis	20031124				Unknown
6 Therapeutic response decreased	200311				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Unknown	1.6 G gram(s)/Day	200311-200311	Backache, unspecified
2 mephenesin mephenesin	Oral	002 Df dosage form/Day	200311-200311	Backache, unspecified
3 novalgin metamizole	Oral	120 Gtt drop(s)/Day	200311-200311	Backache, unspecified
4 rofecoxib rofecoxib	Oral	050 Mg milligram(s)/Day	200311-200311	Backache, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Contraception			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04016445

Patient

Initials: Sex: Female Birth: 19230322 Age: 81 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Respiratory failure	200411				Unknown
2 Pulmonary infiltration	200411				Unknown
3 International normalised ratio increased	200411				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cordarex amiodarone hydrochloride	Oral	1800 Mg milligram(s)/Wee	2000-20041124	Atrial fibrillation
2 diclofenac diclofenac	Oral	/	2004-	Thoracic vertebral fracture T12
3 ibuprofen ibuprofen	Oral	/	2004-	Thoracic vertebral fracture T12
4 marcumar phenprocoumon	Oral	3 Mg milligram(s)/	-	Atrial fibrillation
5 valdecoxib valdecoxib	Oral	/	2004-	

Drugs (concomitant)

6	beloc zok metoprolol succinate	Oral	190 Mg milligram(s)/Day	2000-20041124	Cardiac insufficiency
7	dytide h hydrochlorothiazide; triamterene	Oral	075 Mg milligram(s)/Day	-20041124	Cardiac insufficiency

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diverticula of colon			Unknown	
Osteoporosis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05006761

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth:	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal insufficiency			8 Day		recovered/resolved
2 Hepatic damage					Unknown
3 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetaminophen paracetamol	Oral	8 G gram(s)/Day	-	
2 diclofenac diclofenac		1 G gram(s)/Day	-	Medication error
3 ibuprofen ibuprofen	Unknown	3.6 G gram(s)/Day	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Journal: Toxicol Clin Toxicol (Phila); 2005; 43(1) :31-37 Author: von Mach, et al Title: Experiences of a Poison Center Network with renal insufficiency in acetaminophen overdose	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine		ueber 5.0	mg/dl	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05006592

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes

Patient

Initials: Sex: Male Birth: 19230726 Age: 81 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Aspartate aminotransferase increased	20050313				recovered/resolved
2 Alanine aminotransferase increased	20050313				recovered/resolved
3 Liver function tests NOS abnormal	20050313				recovered/resolved
4 Blood bilirubin increased	20050313				recovered/resolved
5 Drug interaction	20050313				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Arthotec diclofenac sodium; misoprostol	Unknown	/	-	-
2 ibuprofen ibuprofen	Unknown	/	-	-
3 sildenafil sildenafil	Unknown	25 Mg milligram(s)/Day	-	-
4 Tracleer 125 mg Filmtabletten bosentan	Oral	1 Df dosage form/Day	20040417-20050407 11 Month	Pulmonary hypertension primary
5 Tracleer 125 mg Filmtabletten bosentan	Oral	2 Df dosage form/Day	20040408-20050412 12 Month	Pulmonary hypertension primary
6 Tracleer 62,5 mg Filmtabletten bosentan	Oral	2 Df dosage form/Day	20050413- -	Pulmonary hypertension primary
7 Tracleer 62,5 mg Filmtabletten bosentan	Oral	1 Df dosage form/Day	20040317- -	Pulmonary hypertension primary

Drugs (concomitant)

8 Folsaeure folic acid	Unknown	/	-	-
9 Marcumar phenprocoumon	Unknown	/	-	-
10 Novodigal beta-acetyldigoxin	Unknown	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGPT	20050413	175	U/L	
SGOT	20050413	164	U/L	
Bilirubin total	20050602	2,5	MG/DL	

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05007274

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 50 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver transplant	20030307			4 Month	recovered/resolved
2 Acute liver failure	20030202			3 Month	recovered/resolved
3 Hepatotoxicity					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 advil		/	-	Drug use for unknown indication
ibuprofen			-	indication
2 CIMICIFUGA	Oral	500 Mg milligram(s)/Day	200211-20030202	Menopausal symptoms
cimicifuga racemosa root			5 Month	
3 ERYTHROMYCIN		/	200005-200211	Drug use for unknown indication
erythromycin			2 Year	
4 valtrex		/	-	Genital herpes
valaciclovir hydrochloride			-	

Drugs (concomitant)

5 sudafed	/	-	Drug use for unknown indication
pseudoephedrine hydrochloride		-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol abuse			Unknown	
Anemia			Unknown	in the 1990s
Drug abuse			Unknown	8 years ago excessive ibuprofen use

Primary source

Qualification:	Physician	observestudytype:
Literature:	Levitsky J,Alli T,Wisecarver J,Sorrell M. Fulminant Liver Failure Associated with the Use of Black Cohosh. Diges Diseas Scienc 2005 Mar;50:538-539.	
Qualification:	Physician	observestudytype:
Literature:	Teschke R,Schwarzenboeck A. Suspected hepatotoxicity by Cimicifugae racemosae rhizoma (black cohosh, root): Critical analysis and structured causality assessment. Phytomedicine 2009;VOL 16:72-84.	
Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Merck KGaA, 06.10.2006 US-Merck KGaA-6015038

Sender diagnosis: not reported

Narrative

Literature report received from Schaper&Brümmer.

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Levitsky J, Alli TA, Wisecarver J, Sorrell MF. Fulminant Liver Failure Associated with the Use of Black Cohosh. Digest Diseas Scienc 2005;50(3):538-9

*

A 50-year old Caucasian woman was admitted to a local hospital on 02 Feb 03, with the acute onset of scleral icterus, dark-colored urine, and light-colored stools. On physical examination, she appeared jaundiced but had no evidence of encephalopathy or stigmata of chronic liver disease. The total bilirubin (TB) was 7.6 mg/dl; direct bilirubin, 5.4 mg/dL; alanine aminotransferase (ALT), 1474 U/L; aspartate aminotransferase (AST), 2114 U/L; alkaline phosphatase (AP), 232 U/L albumin, 3,3 g/dL; and international normalized ratio (INR), 2.5. The liver, bile ducts, and surrounding vasculature

appeared normal on ultrasonography. No other causes for acute liver failure were identified; hepatitis A IgM, negative; hepatitis B core IgM and surface antigen, negative; hepatitis C antibody and RNA, undetectable; antinuclear antibody, negative; smooth muscle antibody, negative; smooth muscle antibody, negative; anti-liver/kidney microsomal antibody type 1, negative; Epstein-Barr virus IgM, negative; cytomegalovirus IgM, negative; ceruloplasmin, 34 mg/dL (normal range, 17-66 mg/dL). The patient was treated with prednisone, 60 mg daily, for a provisional diagnosis of autoimmune hepatitis. After 1 week, the aminotransferase levels improved but the coagulopathy worsened and encephalopathy developed. She was transferred to the University of Nebraska Medical Center (UNMC) on 14 Feb 03.

In the 5 months prior to the onset of jaundice, the patient was taking black cohosh root (*Clinocifuga racemosak*), 500 mg daily, for the relief of menopausal symptoms. She had no significant medical history or family history of liver disease, did not drink alcohol or use illicit drugs, and was not taking any medications, including other herbal medications, acetaminophen, or nonsteroidal anti-inflammatory drugs. Her examination was notable for jaundice, asterixis, and altered mentation consistent with hepatic encephalopathy. On admission to UNMC, her laboratory tests were remarkable for a platelet count of 80,000/L. TB of 12.3 mg/dL, of 12.3 mg/dL, ALT of 487 U/L, AST of 180 U/L, AP of 224 U/L, albumin of 2.7 g/dL, and INR of 4.0. The patient was started on oral lactulose. In the subsequent weeks, her coagulopathy, encephalopathy, and clinical status remained stable. On March 7, 2003, the patient underwent successful orthotopic liver transplantation. The explanted liver showed features of acute hepatitis with cell necrosis, bridging fibrosis, and cholestasis. She was discharged 1 week, after an uneventful postoperative course.

Follow-up information received via Schaper & Bruemmer GmbH (214/05 US-SCHAPER-000014) on 28 September 2006.

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This follow-up information was obtained from the internet about a judgement from The U.S. District Court for Nebraska on 8 September 2006, which dismissed a lawsuit that had been filed against two manufacturers of black cohosh products in the USA. The suit had been filed by a woman (S.G.) who had required a liver transplant five months after starting to use these products. The Court ruled to exclude the testimony of both of the plaintiffs' experts, and stated that the plaintiffs "have no evidence to establish either general or specific causation." The decision is posted at the following website: http://www.ahpa.org/Portals/0/pdfs/06_0908_BlackCohosh_NebraskaDistrictCt.pdf. There is no doubt, that this judgement belongs to the Levitsky's case report.

In the initial evaluation by Schaper & Bruemmer on 18 May 2005, the causal relationship to black cohosh in this case was assessed as "possible" as the reaction stood in a plausible temporal association with the use of the herbal drug and because other recognised causes for hepatitis had been excluded, as stated in the publication.

According to the Court's judgement, Dr. Levitsky's publication contained several serious factual errors, including a statement that the patient "did not drink alcohol or use illicit drugs, and was not taking any medications, including other herbal medications, acetaminophen, or non-steroidal anti-inflammatory drugs". In contrary, the woman has testified under oath that she regularly drank wine and had used drugs that are known to have influence on the Liver:

The patient was using Valtrex® (valacyclovir hydrochloride) for at least two years as a treatment for genital herpes. This drug lists, in its product information insert, "liver enzyme abnormalities; hepatitis" as a reported adverse reaction.

The patient testified further that she had, in the year preceding her liver failure, used Advil® (Ibuprofen) about three times a week. She also testified that about 8 years prior to this she had used ibuprofen so excessively that she developed anemia which her doctors associated with her overuse of the drug. NSAID medications such as ibuprofen may also cause drug-induced hepatitis.

It is also indicated that the female had used erythromycin for 2.5 years just 4 months before her liver failure. No further information is evident with regard to the indication for use. Although the temporal association of the adverse event with the patient's use of erythromycin is not probable, isolated cases of liver injuries with icterus and rare cases of liver function abnormalities are known side effects of the substance (Horn et al, 1999, Karthik et al 2005).

In addition, one of the authors of the article, who served as an expert witness, has testified that he "knew that the female drank occasionally," and also knew that the patient "was taking Valtrex and had been for at least two years". The expert explained the failure to record the use of this particular drug in the published article as an "oversight," and could not explain why the authors overlooked putting the information in the paper.

In their discussion in the published article, which was fundamental for the initial Schaper & Bruemmer assessment, the authors state, that "The timing of black cohosh administration in relation to the development of liver failure and the absence of an alternative etiology or a prior history of liver disease strongly suggests a cause-effect relationship". Effectively, alternative etiologies were present. In addition to the regular use of alcohol and other hepatotoxic drugs, it is noteworthy that the subject here was taking Valtrex for the treatment of genital herpes. Although generally associated with severely immune-suppressed individuals, herpetic hepatitis is a recognized disease that has also been reported in otherwise healthy persons (Farr et al 1997, Rubin et al 1985).

The recent EMEA statement of July 2006 refers to 42 cases, which have been reported worldwide in association with hepatotoxicity after the intake of black cohosh. In only two of the cases (both involved other Black cohosh containing products than Remifemin) the experts regarded the causal relationship between the use of the herbal medicine and the reported side effect as "probable". The Levitsky's case report is one of these two cases. Based upon this follow-up information, it is obviously that one of these as "probable" classified cases has been inaccurately reported in the literature.

Results of tests and procedures:

- 1) 2003-FEB-02 Total bilirubin (TB) 7.6 mg/dL
- 2) 2003-FEB-02 Direct bilirubin 5.4 mg/dL
- 3) 2003-FEB-02 Alanine aminotransferase (ALT) 1474 U/L
- 4) 2003-FEB-02 Aspartate aminotransferase (AST) 2114 U/L
- 5) 2003-FEB-02 Alkaline phosphatase (AP) 232 U/L
- 6) 2003-FEB-02 Albumin 3.3 g/dL
- 7) 2003-FEB-02 International normalized ratio (INR) 2.5
- 8) 2003-FEB-02 Hepatitis A IgM negative
- 9) 2003-FEB-02 Hepatitis B core IgM negative
- 10) 2003-FEB-02 Surface antigen negative

- 11) 2003-FEB-02 Hepatitis C antibody undetectable
- 12) 2003-FEB-02 Antinuclear antibody negative
- 13) 2003-FEB-02 Smooth muscle antibody negative
- 14) 2003-FEB-02 Anti-liver/kidney microsomal antibody type 1 negative
- 15) 2003-FEB-02 Epstein-Barr virus IgM negative
- 16) 2003-FEB-02 Cytomegalovirus IgM negative
- 17) 2003-FEB-02 Ceruloplasmin 34 mg/dL (17 mg/dL - 66 mg/dL)
- 18) 2003-FEB-14 Platelet count 80.000 /L
- 19) 2003-FEB-14 TB 12.3 mg/dL
- 20) 2003-FEB-14 ALT 487 U/L
- 21) 2003-FEB-14 AST 180 U/L
- 22) 2003-FEB-14 AP 224 U/L
- 23) 2003-FEB-14 Albumin 2.7 g/dL
- 24) 2003-FEB-14 INR 4.0

Schaper & Bruemmer GmbH's comments:

The initial report published by Levitsky contained several serious factual errors. The authors reported, that the patient "did not drink alcohol or use illicit drugs, and was not taking any medications". Based on follow-up information obtained now from the internet, it can be noticed, that this is not true. The regular use of alcohol, the multiple hepatotoxic co-medication and other known diseases should be considered as multiple possible causes of the hepatotoxicity. A causal relationship between the ADR and the intake of Black cohosh can now be assessed as "unlikely". In summary, it is estimated that the present case is without relevance for the manufacturer's Cimicifuga containing product (Remifemin). Also, it is estimated that the safety profile of this drug has remained unchanged. No actions for safety reasons are proposed, at present.

Literature:

Horn S, Aglas F, Horina JH.
Cholestasis and liver cell damage due to hypersensitivity to erythromycin stearate-recurrence following therapy with erythromycin succinate.
Wien Klin Wochenschr 1999; 111(2):76-7.

Karthik SV, Casson D.
Erythromycin-associated cholestatic hepatitis and liver dysfunction in children:
The British experience.
J Clin Gastroenterol 2005; 39(8):743-4.

Raja M, Lewis JH.
Drug- and chemical-induced cholestasis.
Clin Liver Dis 2004; 8:95-132.

Farr RW, Short S, Weissman D.
Fulminant hepatitis during herpes simplex virus infection in apparently immunocompetent adults: Report of two cases and review of the literature.
Clin Infect Dis 1997; 24:1191-4.

Rubin MH, Ward DM, Painter J.
Fulminant hepatic failure caused by genital herpes in a healthy person.
JAMA 1985; 253:1299-301.

Merck KGaA, 06.03.2009 DE-Merck KGaA-6015038

Sender diagnosis: not reported

Narrative

Literature report received from Schaper&Brummer.

*

Levitsky J, Alli TA, Wisecarver J, Sorrell MF. Fulminant Liver Failure Associated with the Use of Black Cohosh. Digest Dis Sci 2005, 50(3):538 to 9

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A 50 year old Caucasian woman was admitted to a local hospital on 02 Feb 03, with the acute onset of scleral icterus, dark colored urine, and light colored stools. On physical examination, she appeared jaundiced but had no evidence of encephalopathy or stigmata of chronic liver disease. The total bilirubin (TB) was 7.6 mg per dL, direct bilirubin, 5.4 mg per dL, alanine aminotransferase (ALT), 1474 U per L aspartate aminotransferase (AST), 2114 U per L, alkaline phosphatase (AP), 232 U per L albumin, 3.3 g per dL, and international normalized ratio (INR), 2.5. The liver, bile ducts, and surrounding vasculature appeared normal on ultrasonography. No other causes for acute liver failure were identified hepatitis A IgM, negative, hepatitis B core IgM and surface antigen, negative, hepatitis C antibody and RNA, undetectable, antinuclear antibody, negative smooth muscle antibody, negative, smooth muscle antibody, negative, anti liver kidney microsomal antibody type 1, negative, Epstein Barr virus IgM, negative, cytomegalovirus IgM, negative, ceruloplasmin, 34 mg per dL (normal range, 17 to 66 mg per dL). The patient was treated with prednisone, 60 mg daily, for a provisional diagnosis of autoimmune hepatitis. After 1 week, the aminotransferase levels improved but the coagulopathy worsened and

encephalopathy developed. She was transferred to the University of Nebraska Medical Center (UNMC) on 14 Feb 03.

In the 5 months prior to the onset of jaundice, the patient was taking black cohosh root (Clinicifuga racemosak), 500 mg daily, for the relief of menopausal symptoms. She had no significant medical history or family history of liver disease, did not drink alcohol or use illicit drugs, and was not taking any medications, including other herbal medications, acetaminophen, or nonsteroidal anti inflammatory drugs. Her examination was notable for jaundice, asterixis, and altered mentation consistent with hepatic encephalopathy. On admission to UNMC, her laboratory tests were remarkable for a platelet count of 80,000 per L, TB of 12.3 mg per dL, of 12.3 mg per dL, ALT of 487 U per L, AST of 180 U per L, AP of 224 U per L, albumin of 2.7 g per dL, and INR of 4.0. The patient was started on oral lactulose. In the subsequent weeks, her coagulopathy, encephalopathy, and clinical status remained stable. On March 7, 2003, the patient underwent successful orthotopic liver transplantation. The explanted liver showed features of acute hepatitis with cell necrosis, bridging fibrosis, and cholestasis. She was discharged 1 week, after an uneventful postoperative course.

Follow up information received via Schaper & Bruemmer GmbH (214 05 US SCHAPER 000014) on 28 September 2006.

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This follow up information was obtained from the internet about a judgement from The U.S. District Court for Nebraska on 8 September 2006, which dismissed a lawsuit that had been filed against two manufacturers of black cohosh products in the USA. The suit had been filed by a woman (S.G.) who had required a liver transplant five months after starting to use these products. The Court ruled to exclude the testimony of both of the plaintiffs experts, and stated that the plaintiffs have no evidence to establish either general or specific causation. The decision is posted at the following website: http://www.ahpa.org/Portals/0/pdfs/06_0908_BlackCohosh_NebraskaDistrictCt.pdf.

There is no doubt, that this judgement belongs to the Levitskys case report.

In the initial evaluation by Schaper & Bruemmer on 18 May 2005, the causal relationship to black cohosh in this case was assessed as possible as the reaction stood in a plausible temporal association with the use of the herbal drug and because other recognised causes for hepatitis had been excluded, as stated in the publication.

According to the Courts judgement, Dr. Levitskys publication contained several serious factual errors, including a statement that the patient did not drink alcohol or use illicit drugs, and was not taking any medications, including other herbal medications, acetaminophen, or non steroidal anti inflammatory drugs. In contrary, the woman has testified under oath that she regularly drank wine and had used drugs that are known to have influence on the Liver:

The patient was using Valtrex® (valacyclovir hydrochloride) for at least two years as a treatment for genital herpes. This drug lists, in its product information insert, liver enzyme abnormalities hepatitis as a reported adverse reaction.

The patient testified further that she had, in the year preceding her liver failure, used Advil® (Ibuprofen) about three times a week. She also testified that about 8 years prior to this she had used ibuprofen so excessively that she developed anemia which her doctors associated with her overuse of the drug. NSAID medications such as ibuprofen may also cause drug induced hepatitis.

It is also indicated that the female had used erythromycin for 2.5 years just 4 months before her liver failure. No further information is evident with regard to the indication for use. Although the temporal association of the adverse event with the patients use of erythromycin is not probable, isolated cases of liver injuries with icterus and rare cases of liver function abnormalities are known side effects of the substance (Horn et al, 1999, Karthik et al 2005).

In addition, one of the authors of the article, who served as an expert witness, has testified that he knew that the female drank occasionally, and also knew that the patient was taking Valtrex and had been for at least two years. The expert explained the failure to record the use of this particular drug in the published article as an oversight, and could not explain why the authors overlooked putting the information in the paper.

In their discussion in the published article, which was fundamental for the initial Schaper & Bruemmer assessment, the authors state, that The timing of black cohosh administration in relation to the development of liver failure and the absence of an alternative etiology or a prior history of liver disease strongly suggests a cause effect relationship. Effectively, alternative etiologies were present. In addition to the regular use of alcohol and other hepatotoxic drugs, it is noteworthy that the subject here was taking Valtrex for the treatment of genital herpes. Although generally associated with severely immune suppressed individuals, herpetic hepatitis is a recognized disease that has also been reported in otherwise healthy persons (Farr et al 1997, Rubin et al 1985).

The recent EMEA statement of July 2006 refers to 42 cases, which have been reported worldwide in association with hepatotoxicity after the intake of black cohosh. In only two of the cases (both involved other Black cohosh containing products than Remifemin) the experts regarded the causal relationship between the use of the herbal medicine and the reported side effect as probable. The Levitskys case report is one of these two cases. Based upon this follow up information, it is obviously that one of these as probable classified cases has been inaccurately reported in the literature.

Results of tests and procedures:

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Schaper & Bruemmer GmbHs comments:

The initial report published by Levitsky contained several serious factual errors. The authors reported, that the patient did not drink alcohol or use illicit drugs, and was not taking any medications. Based on follow up information obtained now from the internet, it can be noticed, that this is not true. The regular use of alcohol, the multiple hepatotoxic co medication and other known diseases should be considered as multiple possible causes of the hepatotoxicity. A causal relationship between the ADR and the intake of Black cohosh can now be assessed as unlikely. In summary, it is estimated that the present case is without relevance for the manufacturers Cimicifuga containing product (Remifemin). Also, it is estimated that the safety profile of this drug has remained unchanged. No actions for safety reasons are proposed, at present.

Literature:

Horn S, Aglas F, Horina JH.
Cholestasis and liver cell damage due to hypersensitivity to erythromycin stearate recurrence following therapy with erythromycin succinate.
Wien Klin Wochenschr 1999 111(2):76 7.

Karthik SV, Casson D.
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Fulminant hepatic failure caused by genital herpes in a healthy person.
JAMA 1985 253:1299 301.

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Follow up information was received from license partner Schaper and Bruemmer GmbH (US SCHAPER 000014) on 10 Feb 2009 included the following new information: New source document (publication) and license partners assessment.

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The patient, a 50 year old woman, took black cohosh root, 500 mg daily, for 5 months prior to the presentation with clinical signs of liver disease. In the case report both alcohol consumption and co medication were denied. In particular, it was stated that she used no illicit drugs and no medications including herbal medications, acetaminophen, or non steroidal anti inflammatory drugs. Later, however, alcohol consumption on a regular basis was admitted, and comedication was documented. The comedication taken suggests major co morbidity. In particular, she was using, or had used, medications that are known or potential hepatotoxins. She had taken erythromycin for 2.5 years ending just 4 months before her liver transplant. She also took such large doses of ibuprofen in the 1990s that she developed anemia. The patient was presently taking valaciclovir, pseudo ephedrine, and ibuprofen at the time of her liver transplant, but the duration of this therapy is not documented. On these and other grounds it was stated that there was no evidence to establish either general or specific causation regarding black cohosh as cause for the liver disease. The patient was put on steroid treatment for the initial diagnosis of autoimmune hepatitis and not for the diagnosis of black cohosh induced autoimmune hepatitis, which was obviously considered only later. Consecutively, the coagulopathy worsened and encephalopathy developed. Five weeks after initial presentation the patient underwent successful orthotopic liver transplantation. She was discharged 1 week after an eventful postoperative course of treatment. The patient has taken drugs previously as well as at the time of her liver transplant. Potentially hepatotoxic drugs such as valaciclovir and ibuprofen had obviously not been discontinued up to her liver transplant, raising the question whether one of these drugs may have caused severe hepatotoxicity and subsequent acute liver failure. It is also not documented that black cohosh treatment has been stopped at first presentation or some time before liver transplantation, since black cohosh was not known to the treating physician. Of major concern is the reported treatment of the patient with valaciclovir, a drug exclusively used for infections by either herpes simplex or herpes zoster. It is highly probable that the patient has had an infection by herpes simplex or zoster and experienced a herpetic liver disease. There is thus little, if any evidence for a causal relationship between black cohosh and the observed acute liver disease when all clinical aspects are considered. In the pre test for black cohosh, causality is unrelated since a non drug cause (herpetic hepatitis) is highly probable. Data related to the alanine aminotransferase course cannot be evaluated, since black cohosh treatment has not been discontinued. The pre test for comedicated drugs is not applicable since treatment was not stopped. The main test for black cohosh is rarely applicable since discontinuation of black cohosh treatment is not documented. Similarly, co medication was taken up to the liver transplantation, the main test is therefore difficult to evaluate. It was assumed that co medicated drugs could have been taken for more than 90 days.

With the post test, some other hepatic diseases have been excluded according to the published laboratory data. The European medicines agency classified the case for black cohosh with +6 points, rendering a probable causality. Causality for co medicated drugs was not assessed, since the respective data have only been documented later and were not known at the time of the evaluation by. It thus appears that the patient experienced a herpetic liver disease treated with valaciclovir. A causative role of black cohosh and co medicated drugs for the observed liver disease could not be demonstrated by qualitative and quantitative causality assessment.

Conclusion: A vigorous causality assesment using a diagnostic algorithm is essential to evaluate the relationship between the observed liver disease and the treatment by Diatery supplements (DDS), to include black cohosh. Using this approach there was no evidence in the present analysis of the evaluated 4 patients for a causal association between Black cohosh and Liver injury.

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Schaper and Bruemmers assessment:

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Initially (18 May 2005) Schaper and Bruemmer had assessed the causal relationship to Cimicifuga as possible. According to the information given in the first follow up (Courts judgment), the case report of Levitsky contained, however, several serious factual errors, including a statement that the patient did not drink alcohol or use illicit drugs, and was not taking any medications, including other herbal medications, acetaminophen, or non steroidal anti inflammatory drugs. Later on, the woman has testified under oath that she regularly drank wine and had used drugs that are known to have influence on the Liver. Therefore, the case should be re assessed. The patient was using Valtrex (valacyclovir hydrochloride) for at least two years as a treatment for genital herpes, which obviously, had not been discontinued up to the liver transplant. This drug lists, liver enzyme abnormalities, hepatitis as a reported adverse reaction. The patient testified further that she had, in the year preceding her liver failure, used Advil (Ibuprofen) about three times a week. About 8 years prior to this she had used ibuprofen so excessively that she developed anaemia which her doctors associated with her overuse of the drug. Non steroidal anti inflammatory drugs medications such as ibuprofen may also cause drug induced hepatitis. Further, the female had used erythromycin for about 2.5 years just 4 months before her liver failure. No further information is evident with regard to the indication for use. Although the temporal association of the adverse event with the patients use of erythromycin is not probable, isolated cases of liver injuries with icterus and rare cases of liver function abnormalities are known side effects of the substance. Of major concern is the reported treatment with valaciclovir, a drug exclusively used for infections by either herpes simplex or herpes zoster. According to Teschke et al 2009, it is highly probable, that the patient has had an infection by herpes simplex or zoster and experienced a herpetic liver disease. Although generally associated with severely immune suppressed individuals, herpetic hepatitis is a recognized disease that has also been reported in otherwise healthy persons. Noteworthy is the testimony of one of the authors of the article, who served as an expert witness. He knew that the female drank occasionally, and also knew that the patient was taking Valtrex and had been for at least two years. The expert explained the failure to record the use of this particular drug in the published article as an oversight, and could not explain why the authors overlooked putting the information in the paper. This omission was corrected later on by the authors. Effectively, alternative etiologies are now present. In addition to the regular use of alcohol and other potentially hepatotoxic drugs, it is noteworthy that the subject here was taking Valtrex for the treatment of genital herpes. It should finally be noted that obviously Remifemin had not been used in this case and that the female had used a black cohosh containing preparation with a daily dose, which is significantly higher than the usually recommended one (500 mg instead of 40mg daily). In summary, the initial case report of Levitsky et al 2005 contained several serious factual errors. The regular use of alcohol, the multiple hepatotoxic co medication and other known diseases should be considered as multiple possible causes of the hepatotoxicity. Most probably, as assumed by Teschke et al. 2009, the patient has had an infection by herpes simplex or zoster and developed herpetic liver disease. Therefore, a causal relationship between the ADR and the intake of lack cohosh can finally be assessed as very unlikely.

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See also cases 6006553(2), 6015040(4), and 6049121.

Merck KGaA, 19.03.2010 DE-Merck KGaA-6015038

Sender diagnosis: not reported

Narrative

Literature report received from Schaper&Bruemmer (SBR).

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Conclusion: A vigorous causality assesment using a diagnostic algorithm is essential to evaluate the relationship between the observed liver disease and the treatment by Dietary supplements (DDS), to include BC. Using this approach there was no evidence in the present analysis of the evaluated 4 patients for a causal association between BC and Liver injury.

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now present. In addition to the regular use of alcohol and other potentially hepatotoxic drugs, it is noteworthy that the subject here was taking Valtrex for the treatment of genital herpes. It should finally be noted that obviously Remifemin had not been used in this case and that the female had used a BC containing preparation with a daily dose, which is significantly higher than the usually recommended one (500 mg instead of 40mg daily). In summary, the initial case report of Levitsky et al 2005 contained several serious factual errors. The regular use of alcohol, the multiple hepatotoxic co medication and other known diseases should be considered as multiple possible causes of the hepatotoxicity. Most probably, as assumed by Teschke et al. 2009, the patient has had an infection by herpes simplex or zoster and developed herpetic liver disease. Therefore, a causal relationship between the ADR and the intake of lack cohosh can finally be assessed as very unlikely.

*See also cases 6006553, 6015040, 6049121.

**FU information was received via SBR at Merck Serono on 11Mar2010 and included the following new information: Full text. The data of 69 cases of initially suspected black cohosh (BC) induced liver disease were reviewed and analyzed to clarify whether BC hepatotoxicity really exists as a disease entity in these cases comparable to toxic liver disease being caused by various drugs and dietary supplements. The cases comprised 11 case reports and 58 spontaneous reports to national regulatory agencies. The analysis includes assessment of causality for BC, data quality of the presented cases, their inconsistencies and confounding variables. The assessed data raise serious doubts on the initial claims of causality for BC in these cases and provide clear evidence of their poor quality, especially when spontaneous reports are considered. There are major inconsistencies for the same patient regarding reported data. Moreover, the analysis of all cases disclosed confounding variables. These include case data quality, uncertainty of BC, quality and identification, undisclosed indication, insufficient adverse event definition, lack of temporal association and dechallenge, inadequate evaluation of alcohol use, comedication, comorbidity, reexposure test, and alternative diagnoses. Conclusion: The presented data do not support the concept of hepatotoxicity in a primarily suspected causal relationship to the use of BC and failure to provide signal of safety concern, further efforts to be taken to dismiss or substantiate the existence of BC hepatotoxicity.

*

See also cases 6004071, 6035427, 6038372, 6041116, 6025834, 6006553, 6015040 and 6049121.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05009589

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 35 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Somnolence					recovered/resolved
2 Intentional overdose					Unknown
3 Transaminases increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	/	-	Suicide attempt
ibuprofen			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Intensivmedizin und Notfallmedizin 2005;42(3): 264-269; Kaes J. Klinischer Stellenwert von Monointoxikationen	

case: DE-BFARM-05013923

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Initials: Sex: Female Birth: 19400316 Age: 65 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver disorder	20050906				not recovered/not resolved
2 Hepatitis	20050906				not recovered/not resolved
3 Transaminase NOS increased	20050906				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 emselex darifenacin hydrobromide	Oral	15 Mg milligram(s)/Day	20050425-20050610 47 Day	Hypertonic bladder
2 emselex darifenacin hydrobromide	Oral	7.5 Mg milligram(s)/Day	20050419-20050425 7 Day	
3 ibuprofen ibuprofen		/	- -	Back pain

Drugs (concomitant)

4	doxazosin	/	-
	doxazosin		-
5	enalapril	/	-
	enalapril		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Essential hypertension			Yes	
Eye operation			Yes	
Spinal column stenosis			Yes	
Hepatic steatosis			Yes	
Obesity			Yes	

Primary source

Qualification: Physician
Literature: observestudytype:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05013714

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: 19700829	Age:	weight (kg): 50	BMI_int: 19.53	Age fetus:
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Patientdeath

deathdate: 20050904	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					fatal
2 Cholestatic hepatitis					Unknown
3 Rhabdomyolysis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 doxycyclin doxycycline		/	- 4 Day	Otitis media NOS
2 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/	- -	Otitis media NOS
3 paracetamol paracetamol	Ophthalmic	/	- -	Otitis media NOS

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus NOS			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05015264

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: 19741129	Age: 30 Year	weight (kg): 50	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function test abnormal					recovered/resolved
2 Sleepiness					recovered/resolved
3 Overdose					recovered/resolved
4 Suicidal ideation					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 gelonida na acetylsalicylic acid; codeine phosphate; paracetamol	Oral	40 Df dosage form/Day	-	Suicidal ideation
2 ibuprofen ibuprofen	Oral	20 Df dosage form/Day	-	Suicidal ideation
3 Imodium loperamide hydrochloride	Oral	24 Df dosage form/Day	-	Suicidal ideation
4 lariam mefloquine	Oral	16 Df dosage form/Day	-	Suicidal ideation
5 talvosilen codeine phosphate; paracetamol	Oral	70 Df dosage form/Day	-	Suicidal ideation

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Janssen-Cilag, Germany, 31.10.2006 DE-JNJFOC-20051002297

Sender diagnosis: not reported

Narrative

Spontaneous report received from a Physician. A-30983.

A 30 year old woman, 50 kilograms, with an unknown medical history, ingested loperamide hydrochloride (capsules, oral): 24 doses, in suicidal ideation on an unspecified date.

The patient also ingested panadeine CO (40 tablets), ibuprofen (20 tablets), mefloquine (16 tablets), and panadeine CO (70 tablets).

At the time of this report, the patient had been hospitalised as a precaution. The patient showed no symptoms.

This report is serious (hospitalisation).

Additional information received 19-JAN-2006:

Initially, after the overdose, the patient showed no relevant effects. In due course an elevation of liver values and sleepiness were observed. The patient was admitted to hospital into the intensive care unit. ACC scheme was prolonged.

The patient recovered.

This report is serious (hospitalisation).

Additional information received 19-JAN-2006:

Clarification 'ACC scheme prolonged' indicated acetylcystein was prolonged in this patient.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05015182

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes						

Patient

Initials:	Sex: Female	Birth: 19700829	Age: 34 Year	weight (kg): 50	BMI_int: 19.53	Age fetus:
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Patientdeath

deathdate:	deathcause: Hepatic coma
deathdate:	deathcause: Cerebral infarction

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic failure	20050822	20050904	14 Day		fatal
2 Hepatitis	20050822	20050904	14 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Clinda-saar 600mg clindamycin hydrochloride	Oral	/	20050711-	Otitis
		Film-coated tablet	5 Day	
2 dolormin ibuprofen lysinate	Unknown	2 Df dosage form/Day	-	
			-	
3 doxycycline doxycycline	Unknown	/	-	
			14 Day	
4 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/	-	
			-	
5 paracetamol paracetamol	Oral	/	-	Otitis
		Film-coated tablet	-	

Drugs (concomitant)

6 insuman comb 15 insulin human; isophane insulin	Unknown	/	-	
			-	
7 insuman comb 25 insulin human; isophane insulin	Unknown	/	-	
			-	
8 novaminsulfon metamizole	Oral	/	20050708-	Otitis
			-	
9 otobacid calcium pantothenate; chloramphenicol; dexamethasone; propyphenazone; tetracaine hydrochloride	Unknown	/	20050621-	Otitis
			-	
10 panotile fludrocortisone acetate; lidocaine hydrochloride; neomycin sulfate; polymyxin b sulfate	Unknown	/	20050714-200507	Otitis
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergic exanthema	20050715		Unknown	...auf Clindamycin
Type I diabetes mellitus			Yes	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05015851

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19760814	Age: 29 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver damage	20051027				not recovered/not resolved
2 Jaundice	20051027				not recovered/not resolved
3 Hepatic enzymes increased	20051027				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin ibuprofen lysinate	Oral	/	20051026-20051026	Upper abdominal pain
2 NOVALGIN metamizole	Intravenous (not otherwis	4 G gram(s)/Day Solution for infusion	20051027-20051027	Upper abdominal pain

Drugs (concomitant)

3 BUSCOPAN hyoscine butylbromide	Intravenous (not otherwis	/	20051027-20051027	Upper abdominal pain
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypokalemia			Yes	Concomitant disease
Lactic dehydrogenase increased			Yes	Concomitant disease
Bilirubin increased			Yes	Concomitant disease

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Sanofi-Aventis Deutschland GmbH, 27.03.2006 DE-AVENTIS-200512474DE

Sender diagnosis: not reported

Narrative

01-Nov-2005 Sanofi-Aventis Germany Pharmacovigilance - initial report

Source: spontaneous report by sales representative with limited information only

Patient: female, 27 years

Medical history: not reported

The sales representative reported on his sister-in-law: on 26-Oct-2005 she took metamizole (Novalgin) and ibuprofen (Dolormin) for treatment of headache and was hospitalised at night because of toxic liver failure. On the next day her liver values improved (nos). Novalgin was not suspected by the hospital physicians.

Further information is requested.

03-Nov-2005 Addendum for follow-up received on 03-Nov-2005

Telephone call with the patient (consumer) revealed that she suffered on 26-Oct-2005 from severe upper abdominal pain and took Dolormin (ibuprofen) tablets 15 minutes before the ambulance arrived. Dose not reported. The patient is a dentist assistant and mentioned that she did not take any drugs or pain relief medication before. The rescue service administered Buscopan (butylscopolaminiumbromid). Blood examination at this time showed increase in bilirubin and LDH. Inflammation parameters were within normal range. After admission to the hospital she was treated with Novalgin infusion (metamizol sodium). In the evening a severe increase of liver enzymes was detected. Mild icterus was observed. Cholelithiasis was excluded via sonography. MRT examination showed only "SLUDGE"-formation. A quick reverse of the hepatic enzyme parameters was observed.

Diagnosis from the hospital discharge report:

suspicion of toxic liver damage. Differential diagnosis: transitory occurrence of gallstones. The hospital physician advised her to avoid non-steroid

antirheumatics in the future.
Further information is requested.

23-Nov-2005 Addendum for follow-up received on 22-Nov-2005

Via phone call with the patient we received the information that she went in the meanwhile again to the hospital because increased liver values were detected again. The patient underwent a liver biopsy and the results revealed a finding which speaks more likely for a kind of toxic reaction. Histology showed infiltration of eosinophils. Her general practitioner assumes that the intake of ibuprofen was responsible for the liver reaction. The patient had taken ibuprofen at home in the past because of headache. On 26-Oct-2005 she received metamizole once at the hospital. The last intake of Novalgine (metamizole) before this administration on 26-Oct-2005 was approximately two years ago. Therefore her GP was not able to give a comment on this. He will contact the hospital to obtain further information.

Today (23-Nov-2005) additional information regarding case assessment was received from her treating physician. He definitely suspects ibuprofen to have caused the liver reaction. He pointed out that up to now no intolerance reaction had been observed. We still requested further details.

09-Jan-2006 Addendum for follow-up received on 09-Jan-2006

Hospital discharge report received. Hospitalisation because of abdominal complaints right sided as like acute cholecystitis. Sonography: no certain hint for lithiasis, finding of hydrocholecystitis without signs of cholecystitis with little sludge, clear enlargement of Ductus cysticus with multiple small septa, no intrahepatic cholestasis. No definite evidence of thrombosis of the portal vein (Differential diagnosis: Budd-Chiari-Syndrome) despite D-Dimer increase to 0.3560 mg/l (ref. < 0.3), no intrahepatic congestion.

Analgetic treatment with:

Novalgine (metamizole) 1 g i.v. on 27-Oct-2005 at 01:00 a.m. + 1 Amp. Butylscopolaminiumbromid

Novalgine (metamizole) 1 g i.v. at 05:40 a.m.

Novalgine (metamizole) 1 g i.v. at 07:45 a.m. + 2 amp. Butylscopolaminiumbromid

Novalgine (metamizole) 1 g i.v. at 02:40 p.m. + 2 amp. Butylscopolaminiumbromid

In addition Sterofundin, Pantozol (pantoprazol), Rocephin (ceftriaxon-dinatrium) and Monoembolex (certoprain-natrium) was given.

Laboratory values on admission on 27-Oct-2005 at 00:58 a.m. (on admission)

WBC 11.9 Gpt/l (Ref. value 3.8-10.0) and at 01:32 p.m. WBC 7.6

Quick 101.0 % and at 04:27 p.m. Quick 97.5

PTT 26.4 sec (Ref. value 26-36) and at 04:27 p.m. 33.7

Potassium 2.80 mmol/l (Ref. value 3.50 - 5.30) and at 01:32 p.m. 4.10 (corrected with administration of KCl-infusions)

ASAT (GOT) 1.43 umol/s.l (Ref. value <0.52) and at 01:32 p.m. 16.41

ALAT (GPT) 0.95 umol/s.l (Ref. value < 0.52) and at 01:32 p.m. 15.69

AP 0.85 umol/s.l (Ref. value 0.67 - 2.50) and at 01:32 p.m. 1.55

Gamma-GT 0.74 umol/s.l (Ref. value < 0.53) and at 01:32 p.m. 3.05

Alpha-amylase 0.91 umol/s.l (Ref. value 0.42-2.08) and at 01:32 p.m. 0.58

Lipase 0.59 umol/s.l (Ref. value 0.13-1.30) and at 01:32 p.m. 0.39

CRP 1.2 (Ref. value < 8.2) and at 01:32 p.m. 2.6

Anti HBs 6859 IU/l

Anti-HAV-IgM, HBsAg, Anti-HBc-IgM, Anti-HCV, CMV-Ak-IgM, CMV-Ak-IgG were all negative.

At around 06:00 p.m. the patient was transferred to another hospital (due to the suspected acute toxic liver failure of unknown origin). Further information is still awaited.

Sanofi-Aventis Deutschland GmbH, 23.05.2006 DE-AVENTIS-200512474DE

Sender diagnosis: not reported

Narrative

Narrative had to be modified for technical reasons (lack of space)

01-Nov-2005 Initial report

Spontaneous report by sales representative on a 27-year-old female patient who was hospitalised on 26-Oct-2005 due to toxic liver failure after intake of metamizole (Novalgine) and ibuprofen for treatment of headache. Her liver values improved the next day. Novalgine was not suspected by the hospital physicians.

03-Nov-2005 Follow-up by patient

On 26-Oct-2005 the patient suffered from severe upper abdominal pain and took Dolormin (ibuprofen) tablets 15 minutes before the ambulance arrived. The rescue service administered Buscopan (butylscopolaminiumbromid). Blood examination at this time showed increase in bilirubin and LDH. Inflammation parameters were within normal range. After admission to the hospital she was treated with metamizole sodium (Novalgine) infusion. In the evening a severe increase of liver enzymes was detected with mild icterus. Cholelithiasis was excluded by sonography. MRT examination showed only "SLUDGE"-formation. A quick reverse of the hepatic enzyme parameters was observed. Diagnosis on hospital discharge report: suspicion of toxic liver damage. Differential diagnosis: transitory occurrence of gallstones. The hospital physician advised her to avoid non-steroid antirheumatics in the future.

22-Nov-2005 Follow-up by patient and physician:

The patient was hospitalised for renewed increase in liver values. The patient underwent a liver biopsy and the results revealed a finding which speaks more likely for a kind of toxic reaction. Histology showed infiltration of eosinophils. Her general practitioner assumes that the intake of ibuprofen was responsible for the liver reaction. The patient had taken ibuprofen at home in the past because of headache. On 26-Oct-2005 she received metamizole once at the hospital. The last intake of metamizole before this administration on 26-Oct-2005 had been approximately two years ago.

Therefore her GP was not able to give a comment on this. He will contact the hospital to obtain further information.

Additional information regarding case assessment by treating physician: He definitely suspects ibuprofen to have caused the liver reaction. He points out that up to now no intolerance reaction had been observed.

09-Jan-2006 Follow-up / hospital discharge report

Hospitalisation because of right abdominal complaints like acute cholecystitis. Sonography: no certain signs of lithiasis, findings of hydrocholecystitis without signs of cholecystitis with little sludge, clear enlargement of Ductus cysticus with multiple small septa, no intrahepatic cholestasis. No definite evidence of thrombosis of the portal vein (Differential diagnosis: Budd-Chiari-Syndrome) despite D-Dimer increase to 0.3560 mg/l (ref. < 0.3), no intrahepatic congestion.

Analgetic treatment with:

Metamizole (Novalgin) 1 g i.v. on 27-Oct-2005 at 01:00 a.m. + 1 amp butylscopolaminium bromide

Metamizole 1 g i.v. at 05:40 a.m.

Metamizole 1 g i.v. at 07:45 a.m. + 2 amp. butylscopolaminium bromide

Metamizole 1 g i.v. at 02:40 p.m. + 2 amp. butylscopolaminium bromide

Concom. medication: sterofundin, pantoprazole, ceftriaxone and certoparin

Laboratory values 27-Oct-2005 at 00:58 a.m. (on admission)

WBC 11.9 Gpt/l (Ref. value 3.8-10.0) and at 01:32 p.m. 7.6

Quick 101.0 % and at 04:27 p.m. 97.5

PTT 26.4 sec (Ref. value 26-36) and at 04:27 p.m. 33.7

Potassium 2.80 mmol/l (3.50 - 5.30) and at 01:32 p.m. 4.10 (corrected with administration of KCl-infusions)

ASAT (GOT) 1.43 umol/s.l (< 0.52) and at 01:32 p.m. 16.41

ALAT (GPT) 0.95 umol/s.l (< 0.52) and at 01:32 p.m. 15.69

AP 0.85 umol/s.l (0.67 - 2.50) and at 01:32 p.m. 1.55

Gamma-GT 0.74 umol/s.l (< 0.53) and at 01:32 p.m. 3.05

Alpha-amylase 0.91 umol/s.l (0.42-2.08) and at 01:32 p.m. 0.58

Lipase 0.59 umol/s.l (0.13-1.30) and at 01:32 p.m. 0.39

CRP 1.2 (< 8.2) and at 01:32 p.m. 2.6

Anti HBs 6859 IU/l

Anti-HAV-IgM, HBsAg, Anti-HBc-IgM, Anti-HCV, CMV-Ak-IgM, CMV-Ak-IgG were all negative.

At around 06:00 p.m. the patient was transferred to another hospital (due to the suspected acute toxic liver failure of unknown origin).

15-MAY-2006 Addendum for follow-up received 12-MAY-2006

Histology of liver puncture sample (18-Nov-2005) / Assessment: liver puncture specimen with moderate hepato-canalicular cholestasis, with predominant emphasis perivenular. Additionally single hepatocyte groups are degenerated balloon-like. The changes are overall unspecific, but in connection with clinical results they can be attributed to a drug-toxic damage. A picture of auto-immune cholangitis has not developed. No noteworthy siderosis, no fibrosis, no sign of malignancy.

Laboratory:

28-Nov-2005: creatinine 67 umol/L (normal 58-96), direct bilirubin 69.4 umol/L (< 3), total bilirubin 94 umol/L (< 21), C-reactive protein <2.0 mg/L (< 2.0), ASAT 0.65 umol/L.s (< 0.52); ALAT 1.13 umol/L.s (< 0.51), GLDH < 50 nmol/L.s (< 50), gamma-GT 0.28 umol/L.s (< 0.63), alkaline phosphatase 1.58 umol/L.s (< 2.0), cholinesterase 66 umol/L.s (65-180), iron in plasma 3.0 umol/L (4.1-24.0), Quick 113 % (70-130), INR 0.9 (no reference value), aPTT 30 sec (26-36), BSR 6/16 mm/h (7-12/<18), glucose 6.7 mmol/L (3.9-5.8)

12-Dec-2005: direct bilirubin 34.5, total bilirubin 51, ASAT 0.70, ALAT 1.04, GLDH < 50, gamma-GT 0.25, alkaline phosphatase 1.34, cholinesterase 73, Quick 108, INR 1.0, aPTT 29, glucose 5.3

09-Jan-2006: creatinine 72, direct bilirubin 7.7, total bilirubin 12, C-reactive protein < 2.0, ASAT 0.39, ALAT 0.51, GLDH < 50, gamma-GT 0.17, alkaline phosphatase 0.85, cholinesterase 96, iron in plasma 8.8, Quick 98, INR 1.0, aPTT 35, BSR 6/14, glucose 4.5

28-Feb-2006: direct bilirubin 2.9, total bilirubin 5, ASAT 0.33, ALAT 0.60, GLDH 69, gamma-GT 0.19, alkaline phosphatase 0.89, cholinesterase 92, Quick 107, INR 1.0, aPTT 32

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05018707

Serious Yes	Death Yes	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth:	Age: 11 Year	weight (kg): 26	BMI_int: 13.27	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Constipation					Unknown
2 Death					fatal
3 Cholestasis					Unknown
4 Drug ineffective					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexamethasone dexamethasone	Oral	5 Mg milligram(s)/Day	-	Pain
2 fentanyl fentanyl	Transdermal	/	-	Pain
3 fentanyl fentanyl	Transdermal	/	-	Pain
4 gabapentin gabapentin	Oral	300 Mg milligram(s)/Day	-	Pain
5 ibuprofen ibuprofen	Oral	400 Mg milligram(s)/Day	-	Pain
6 metamizole metamizole sodium	Oral	100 Gtt drop(s)/Day	-	Pain
7 morphine morphine	Intravenous bolus	/	-	
8 morphine morphine	Oral	80 Mg milligram(s)/Day	-	Pain
9 morphine morphine	Intravenous (not otherwis /		-	
10 piritramide piritramide	Intravenous bolus	3 Mg milligram(s)/	-	Pain

Drugs (concomitant)				
11	alfentanil	/	-	Analgesia
	alfentanil		-	
12	carboplatin	/	-	Chemotherapy
	carboplatin		-	
13	dimenhydrinate	Intravenous (not otherwis /	-	
	dimenhydrinate		-	
14	esketamine	Intravenous (not otherwis /	-	Analgesia
	esketamine		8 Day	
15	etoposide	/	-	Chemotherapy
	etoposide		-	
16	ifosfamide	/	-	Chemotherapy
	ifosfamide		-	
17	lactulose	Oral	/	
	lactulose		-	
18	midazolam hydrochloride	/	-	Anaesthesia
	midazolam hydrochloride		-	
19	oxybate sodium	/	-	Anaesthesia
	oxybate sodium		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Radiotherapy			No	
Hyperthermia therapy			Yes	
Neuroblastoma			Yes	Stage IV neuroblastoma with multiple metastases

Primary source

Qualification:	Physician	observestudytype:
Literature:	Laufer M. et al.; Schmerz 2005 Mar; 19: 3: 220-224 "Therapie extremer Tumorschmerzen mit Morphin und (S)-Ketamin: Falldarstellung eines 11-jährigen Mädchens"	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05018675

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: 19531025	Age: 50 Year	weight (kg): 58	BMI_int: 21.83	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	20040922			3 Day	recovering/resolving
2 Nausea	20040922			3 Day	recovering/resolving
3 Hepatitis acute	20040922			3 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	3 Df dosage form/Day	20040920-20040926	Back pain
ibuprofen			7 Day	

Drugs (concomitant)

2 Atenolol AL comp.mite	Oral	0.5 Df dosage form/Day	200106-20040927	Hypertension arterial
atenolol; chlortalidone			-	
3 ciprohexal 500 mg	Oral	2 Df dosage form/Day	20040922-20040924	Urine discolouration
ciprofloxacin hydrochloride			3 Day	
4 eudorlin	Oral	/	199401-20041028	Headache
caffeine; paracetamol; propyphenazone			-	
5 remifemin	Oral	/	200205-20040927	Climacteric disturbance
cimicifuga racemosa root			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06000832

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Male Birth: 19580410 Age: 47 Year weight (kg): 83 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cirrhosis liver	20051110				not recovered/not resolved
2 Ascites	20051110				not recovered/not resolved
3 Hepatitis cholestatic	20051110				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Esprenit 600mg Filmtabletten	Oral	2400 Mg milligram(s)/Day	200510-	Joint pain
ibuprofen		Film-coated tablet	-	

Drugs (concomitant)

2 timolol augentropfen	Ophthalmic	/	-	
timolol maleate			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sinus tachycardia			Yes	
Nicotine dependence		2003	No	
Drug eruption			Yes	
Gammaglobulins decreased	200511		Yes	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
IgG	200511	Abfall/decreased	NA	
IgA	200511	normal/normal	NA	
IgM	200511	normal/normal	NA	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06012910

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19440415	Age: 61 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestasis	20060101				recovered/resolved with sequel
2 Quick's value decreased	20060101				recovered/resolved with sequel
3 Acute hepatic failure	20060101				recovered/resolved with sequel
4 Transaminases increased	20060101				recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cephazolin		/	-	Antibiotic prophylaxis
cephazolin			-	
2 ibuprofen		/	-	Postoperative analgesia
ibuprofen			-	
3 trazodone		/	-	Postoperative analgesia
trazodone			-	

Drugs (concomitant)

4 loratadin	/	-	Preoperative care
loratadine		-	
5 paracetamol	/	-	Preoperative care
paracetamol		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Osteochondrosis			Yes	
Intervertebral disc operation	20051216		No	
Pseudoradicular syndrome			Yes	
Lumbar disc lesion			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06013544

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminase value increased					recovered/resolved
2 Drug intoxication					recovered/resolved
3 Somnolence					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	-	
ibuprofen			-	

Primary source

Qualification:	observestudytype:
Literature:	Kaes J,von Mach M,Weilemann I,Wiechelt J,Lauterbach M,Eich A,et al. Klinischer Stellenwert von Monointoxikationen mit Ibuprofen, Diclofenac und Metamizol. Intensivmed Notfallmed 2005;42:3:264-269.
Qualification:	Consumer or other non health professional observestudytype:
Literature:	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 30.03.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-01525GD

Sender diagnosis: not reported

Narrative

The case of a patient is presented who developed somnolence and increased transaminases after monointoxication with ingested ibuprofen (group > 400 mg/kg). The patient recovered. According to the Poisoning Severity Score, the symptoms were rated moderate. Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant). The present study was undertaken to analyze the frequency and the clinical significance of monointoxications with the most frequently used generics of non-steroidal anti-inflammatory drugs without consideration of acetylsalicylic acid. The particular aim was to determine the critical dose of diclofenac, ibuprofen and metamizol at which intensive care monitoring is required. During the study period from 1. January 1995 until 31. December 2001 the inquiries to the poison center in Mainz for monointoxications with pain killers were documented. The clinical severity was assessed using the Poisoning Severity Score. A total of 1281 monointoxications (68% men) were evaluated.

11 additional individual case reports are presented in this paper:

2006-DE-01521GD
 2006-DE-01523GD
 2006-DE-01524GD
 2006-DE-01526GD
 2006-DE-01527GD
 2006-DE-01530GD
 2006-DE-01531GD
 2006-DE-01532GD
 2006-DE-01535GD
 2006-DE-01537GD
 2006-DE-01539GD

3 cumulative literature reports are also available:

2006-DE-01540GD
 2006-DE-01542GD
 2006-DE-01543GD

The case is from a study report written in German. The English translation is ordered.

Boehringer Ingelheim GmbH, Germany, 18.04.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-01525GD

Sender diagnosis: not reported

Narrative

The case of a patient is presented who developed somnolence and increased transaminases after monointoxication with ingested ibuprofen (group > 400 mg/kg). The patient recovered. According to the Poisoning Severity Score, the symptoms were rated moderate.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The present study was undertaken to analyze the frequency and the clinical significance of monointoxications with the most frequently used generics of non-steroidal anti-inflammatory drugs without consideration of acetylsalicylic acid. The particular aim was to determine the critical dose of diclofenac, ibuprofen and metamizol at which intensive care monitoring is required. During the study period from 1. January 1995 until 31. December 2001 the inquiries to the poison center in Mainz for monointoxications with pain killers were documented. The clinical severity was assessed using the Poisoning Severity Score. A total of 1281 monointoxications (68% men) were evaluated.

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2006-DE-01531GD

2006-DE-01532GD

2006-DE-01535GD

2006-DE-01537GD

2006-DE-01539GD

3 cumulative literature reports are also available:

2006-DE-01540GD

2006-DE-01542GD

2006-DE-01543GD

The case is from a study report written in German. The English translation is ordered.

The English translation provided the following information:

The basis for this study is data from the Poison Information Centre in Mainz which originated as documentation of telephone consultations on poisonings between 1 January 1995 and 31 December 2001. A total of 140,773 cases were discussed during this period. The data was obtained on the basis of a standardized questionnaire and evaluated. After the consultation all the doctors attending the cases were sent a uniform questionnaire to document further developments and the outcome. In this way written follow-ups of 59,702 cases of poisoning were obtained. The severity of the poisoning was determined on the basis of the major symptoms which occurred in the cases followed up.

In the period from 1 January 1995 until 31 December 2001 a total of 4705 cases of poisoning with NSAR (not including acetylsalicylic acid) and steroidal anti-inflammatories were documented in the Poison Information Centre in Mainz. These included 1281 monointoxications (68 % men, 32 % women; 77 % adults, 12 % adolescents; 11 % under 14 years or unknown).

The patient developed an increase in transaminases.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06015647

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19811009	Age: 24 Year	weight (kg): 85	BMI_int: 27.44	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function test abnormal	20060321			81 Day	recovering/resolving
2 Jaundice	20060321			81 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clexane enoxaparin		/	-	Thrombosis prophylaxis
2 CONCEPLAN M ethinylestradiol; norethisterone	Oral	1 Df dosage form/24 Hour Tablet	2002-2006 4 Year	Contraception
3 HEPARIN-FRACTION, SODIUM SALT heparin-fraction, sodium salt	Oral	/	-	Thrombosis prophylaxis
4 IBUPROFEN ibuprofen	Oral	/	-	Pain
5 normensal ethinylestradiol; norethisterone	Oral	/	2002-2006 4 Year	Contraception
6 SEVOFLURANE LIQUID FOR INHALATION sevoflurane	Respiratory (inhalation)	/	20051230-20051230 -	Anesthesia

Drugs (concomitant)

7 PROPOFOL propofol	Intravenous (not otherwis /		20051230-20051230 -	Anesthesia
8 REMIFENTANIL HYDROCHLORIDE remifentanil hydrochloride	Intravenous (not otherwis /		20051230-20051230 -	Anesthesia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Nonsmoker			Unknown	
Abstains from alcohol			Unknown	
Lower limb fracture			Unknown	
Drug allergy			Unknown	allergy to Novalgin
Hepatitis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT	20060321	1450	NA	
GPT	20060328	730	NA	
SGOT	20060321	726	NA	
SGOT	20060328	596	NA	
Bilirubin total	20060321	12.8	NA	
Bilirubin total	20060328	24.1	NA	

DE-BFARM-06015647			
Bilirubin	20060321	12.8	mg/dl
Bilirubin	20060323	16.9	mg/dl
Bilirubin	20060325	20.2	mg/dl
Bilirubin	20060327	24.1	mg/dl
Bilirubin	20060330	23.7	mg/dl
Bilirubin	20060322	12.8	mg/dl
Bilirubin direct	20060327	16.00	mg/dl
Bilirubin direct	20060325	14.00	mg/dl
Bilirubin direct	20060323	12.30	mg/dl
Bilirubin direct	20060321	7.80	mg/dl
Bilirubin direct	20060322	8.40	mg/dl
Bilirubin direct	20060330	17.20	mg/dl
Bilirubin unconjugated	20060325	2.10	mg/dl
Bilirubin unconjugated	20060330	2.70	mg/dl
Bilirubin unconjugated	20060327	2.30	mg/dl
Bilirubin unconjugated	20060323	2.10	mg/dl
Bilirubin unconjugated	20060322	1.90	mg/dl
Bilirubin unconjugated	20060321	1.90	mg/dl
Gamma-glutamyltransferase	20060323	126++	U/l
Gamma-glutamyltransferase	20060324	122++	U/l
Gamma-glutamyltransferase	20060322	123++	U/l
Gamma-glutamyltransferase	20060321	142++	U/l
Gamma-glutamyltransferase	20060325	128++	U/l
Gamma-glutamyltransferase	20060327	131++	U/l
Gamma-glutamyltransferase	20060330	122++	U/l
SGOT	20060327	596++	U/l
SGOT	20060321	726++	U/l
SGOT	20060323	558++	U/l
SGOT	20060324	481++	U/l
SGOT	20060325	454++	U/l
SGOT	20060322	622++	U/l
SGOT	20060330	688++	U/l
SGPT	20060321	1451++	U/l
SGPT	20060330	596++	U/l
SGPT	20060327	736++	U/l
SGPT	20060325	736++	U/l
SGPT	20060324	870++	U/l
SGPT	20060323	1157++	U/l
SGPT	20060322	1218++	U/l
Alkaline phosphatase	20060404	255	U/L
ANA		Negative	NA
Bilirubin direct		11.5	MG/DL
Bilirubin total		19.8	MG/DL
Bilirubin total	20060328	24.1	NA

Bilirubin total	20060321	12.8	NA
Bilirubin total	20060404	19.8	MG/DL
Blood bilirubin indirect		2.5	MG/DL
Cholinesterase	20060404	3100	U/L
GGT	20060404	121	U/L
GPT	20060328	730	NA
GPT	20060321	1450	NA
HIV test		Negative	NA
SGOT	20060328	596	NA
SGOT	20060321	726	NA
SGOT	20060404	516	U/L
SGPT	20060404	391	U/L
Epstein-Barr virus serology		Negative	NA
Mycoplasma antibody		Negative	NA
CMV IgG antibody		Negative	NA
Bilirubin	20060602	1.19	NA
Bilirubin	20060406	20.58+	NA
Bilirubin	20060512	2.39+	NA
Bilirubin	20060421	6.22+	NA
Bilirubin	20060411	10.98+	NA
Calcium	20060411	2.31	NA
Gamma GT	20060406	134+	NA
Gamma GT	20060421	85+	NA
Gamma GT	20060512	44+	NA
Gamma GT	20060602	42+	NA
Gamma GT	20060411	145+	NA
Hematocrit	20060411	0.42	NA
Hematocrit	20060421	0.40	NA
Hemoglobin	20060411	13.5	NA
Hemoglobin	20060421	13.6	NA
Iron	20060411	24.70	NA
Iron	20060406	36.40+	NA
MCH	20060411	30.5	NA
MCH	20060421	31.1	NA
MCHC	20060421	33.9	NA
MCHC	20060411	32.5	NA
MCV	20060421	91.6	NA
MCV	20060411	94.1	NA
Potassium	20060411	4.0	NA
Red blood cell count	20060421	4.38	NA
Red blood cell count	20060411	4.42	NA
SGOT	20060421	67+	NA
SGOT	20060602	18	NA
SGOT	20060406	400+	NA

SGOT	20060411	291+	NA
SGOT	20060512	21	NA
SGPT	20060512	23	NA
SGPT	20060406	347+	NA
SGPT	20060411	272+	NA
SGPT	20060421	111+	NA
SGPT	20060602	15	NA
Sodium	20060411	140	NA
Thrombocyte count	20060411	246	NA
Thrombocyte count	20060421	242	NA
Leukocyte count NOS	20060421	9.31	NA
Leukocyte count NOS	20060411	7.25	NA
Red blood cell sedimentation rate	20060511	22/40	NA
Red blood cell sedimentation rate	20060420	30/50	NA
Red blood cell sedimentation rate	20060411	30/54	NA

Narratives (nur bei elektronischem Eingang)ABBOTT, 10.04.2006 DE-ABBOTT-06P-062-0329479-00

Sender diagnosis: not reported

Narrative

Report from Germany of increased liver function tests and jaundice coincident with SEVOFLURANE (SEVOFLURANE LIQUID FOR INHALATION) therapy. On 30 Dec 2005, the patient began SEVOFLURANE LIQUID FOR INHALATION therapy for anesthesia for fractured lower leg surgery. On 21 Mar 2006, lab test reveal increased liver function tests and the patient was jaundice. On 21 Mar 2006, the patient was hospitalized for increased liver function test and jaundice. On 28 Mar 2006, the liver transaminases were decreasing, whereas the total bilirubin increased. The increased liver function tests and jaundice have not resolved. The reporter believed the increased liver function tests and jaundice were possibly related to SEVOFLURANE LIQUID FOR INHALATION therapy.

Die in diesem Fallbericht geschilderte Symptomatik ist in den Angaben zum Arzneimittel in adäquater Form beschrieben. Einen Kausalzusammenhang mit der Anwendung unseres Arzneimittels halten wir jedoch aufgrund der zeitlichen Relationen fuer wenig wahrscheinlich.

Reporter's comment:ABBOTT, 10.05.2006 DE-ABBOTT-06P-062-0329479-00

Sender diagnosis: not reported

Narrative

Report from Germany of increased liver function tests and jaundice coincident with SEVOFLURANE (SEVOFLURANE LIQUID FOR INHALATION) therapy. On 30 Dec 2005, the patient began SEVOFLURANE LIQUID FOR INHALATION therapy for anesthesia for fractured lower leg surgery. On 21 Mar 2006, lab test reveal increased liver function tests and the patient was jaundice. On 21 Mar 2006, the patient was hospitalized for increased liver function test and jaundice. On 28 Mar 2006, the liver transaminases were decreasing, whereas the total bilirubin increased. The increased liver function tests and jaundice have not resolved. The reporter believed the increased liver function tests and jaundice were possibly related to SEVOFLURANE LIQUID FOR INHALATION therapy.

Follow-up information received on 03 May 2005: Suspect medication was added or revised. In 2002, the patient began NORMENSAL therapy for contraception. In 2006, while the patient was in the hospital, NORMENSAL therapy was discontinued. The increased liver function test started to normalize. On an unreported date, the patient was discharged. The reporter believed an alternative etiology for the increased liver function tests was NORMENSAL therapy. NORMENSAL therapy was also considered suspect.

normalisieren, sieht die Meldequelle nun dieses Medikament als wahrscheinlichste Ursache der Symptomatik.

Reporter's comment:

GRUNENTHAL, 30.06.2006 DE-GRUNENTHAL-2006 11074

Sender diagnosis: not reported

Narrative

SUBJECT/PATIENT ID

Manufacturer Control Number 2006-11074

ADVERSE EVENT(S)

reported Term/MedDRA PT: Hepatitis / Hepatitis

outcome: recovering/resolving

reported serious: yes

seriousness(company): serious

seriousness criterion: hospitalization

expectedness (company): unlisted

causality(reporter): possible

causality(company): unlikely

intensity: severe

reported Term/MedDRA PT: Ikterus / Jaundice

outcome: recovering/resolving

reported serious: yes

seriousness(company): serious

seriousness criterion: hospitalization

expectedness (company): unlisted

causality(reporter): possible

causality(company): unlikely

intensity: severe

reported Term/MedDRA PT: Aspartate aminotransferase increased / Aspartate aminotransferase increased

outcome: recovering/resolving

reported serious: yes

seriousness(company): serious

seriousness criterion: hospitalization

expectedness (company): unlisted

causality(reporter): possible

causality(company): unlikely

intensity: not reported

reported Term/MedDRA PT: alanine aminotransferase increased / Alanine aminotransferase increased

outcome: recovering/resolving

reported serious: yes

seriousness(company): serious

seriousness criterion: hospitalization

expectedness (company): unlisted

causality(reporter): possible

causality(company): unlikely

intensity: not reported

reported Term/MedDRA PT: Gamma GT increased / Gamma-glutamyltransferase increased

outcome: recovering/resolving

reported serious: yes

seriousness(company): serious

seriousness criterion: hospitalization

expectedness (company): unlisted

causality(reporter): possible

causality(company): unlikely

intensity: not reported

REPORTER AND SUBJECT/PATIENT INFORMATION

This spontaneous case from Germany was reported by a physician and concerns a 24 years old female caucasian european origin subject/patient (height 176 cm / weight 85 kg).

MEDICAL HISTORY

The subject's/patient's medical history included:

Lower limb fracture

Surgery

Contraception

Drug hypersensitivity (allergy to Novalgin)

SUSPECTED MEDICINAL PRODUCT(S)

substance/tradename: ETHINYLESTRADIOL NORETHISTERONE / CONCEPLAN M

route of administration: oral

daily dose: 1 DF

therapy stop date: 23-MAR-2006

therapy duration: 2920 d 0 h 0 m

indication for use: Contraception

substance/tradename: IBUPROFEN / IBUPROFEN

substance/tradename: ENOXAPARIN SODIUM / CLEXANE

CONCOMITANT MEDICINAL PRODUCT(S)

No concomitant products reported.

ADVERSE EVENT DETAILS

On 21-MAR-2006, about 8 years after start of administration of CONCEPLAN M the patient experienced icterus with increased laboratory values for liver enzymes and bilirubin (see below). Hepatitis and icterus was reported by the physician. Hospital examination did not reveal the cause for the events. A liver biopsy was not performed.

RELEVANT PHYSICAL FINDINGS

Gamma GT:

21-MAR-2006 142 U/l

22-MAR-2006 123 U/l

23-MAR-2006 126 U/l

24-MAR-2006 122 U/l

25-MAR-2006 128 U/l

27-MAR-2006 131 U/l

30-MAR-2006 122 U/l

Aspartate aminotransferase:

21-MAR-2006 726 U/l

22-MAR-2006 622 U/l

23-MAR-2006 558 U/l

24-MAR-2006 481 U/l

25-MAR-2006 454 U/l

27-MAR-2006 596 U/l

30-MAR-2006 688 U/l

Alanine aminotransferase:

21-MAR-2006 1451 U/l

22-MAR-2006 1218 U/l

23-MAR-2006 1157 U/l

24-MAR-2006 870 U/l

25-MAR-2006 736 U/l

27-MAR-2006 736 U/l

30-MAR-2006 596 U/l

Bilirubin conjugated:

21-MAR-2006 7.80 mg/dl (direct bilirubin - 10.90 mg/dl)

22-MAR-2006 8.40 mg/dl (direct bilirubin - 10.90 mg/dl)

23-MAR-2006 12.30 mg/dl (direct bilirubin - 14.80 mg/dl)

25-MAR-2006 14.00 mg/dl (direct bilirubin - 18.10 mg/dl)

27-MAR-2006 16.00 mg/dl (direct bilirubin - 21.80 mg/dl)

30-MAR-2006 17.20 mg/dl (direct bilirubin - 21.00 mg/dl)

Blood bilirubin unconjugated:

21-MAR-2006 1.90 mg/dl
 22-MAR-2006 1.90 mg/dl
 23-MAR-2006 2.10 mg/dl
 25-MAR-2006 2.10 mg/dl
 27-MAR-2006 2.30 mg/dl
 30-MAR-2006 2.70 mg/dl

Blood bilirubin:

21-MAR-2006 12.8 mg/dl
 22-MAR-2006 12.8 mg/dl
 23-MAR-2006 16.9 mg/dl

Other relevant tests:

Biopsy of liver is pending.

REPORTER'S ASSESSMENT STATEMENT

The reporter did not explicitly state a causality assessment (implied causality).

ABBOTT, 11.07.2006 DE-ABBOTT-06P-062-0329479-00

Sender diagnosis: not reported

Narrative

Report from Germany of jaundice and increased liver function tests coincident with SEVOFLURANE (SEVOFLURANE LIQUID FOR INHALATION) therapy. On 30 Dec 2005, the patient began SEVOFLURANE LIQUID FOR INHALATION therapy for anesthesia for fractured lower leg surgery. On 21 Mar 2006, lab test reveal increased liver function tests and the patient was jaundice. On 21 Mar 2006, the patient was hospitalized for increased liver function test and jaundice. On 28 Mar 2006, the liver transaminases were decreasing, whereas the total bilirubin increased. The increased liver function tests and jaundice have not resolved. The reporter believed the increased liver function tests and jaundice were possibly related to SEVOFLURANE LIQUID FOR INHALATION therapy.

Follow-up information received on 03 May 2005: Suspect medication was added or revised. In 2002, the patient began NORMENSAL therapy for contraception. In 2006, while the patient was in the hospital, NORMENSAL therapy was discontinued. The increased liver function test started to normalize. On an unreported date, the patient was discharged. The reporter believed an alternative etiology for the increased liver function tests was NORMENSAL therapy. NORMENSAL therapy was also considered suspect.

Follow-up information received on 30 Jun 2006: Suspect medication has been added or revised. It was clarified that in 2002, the patient began CONCEPLAN therapy for contraception. On an unreported date, the patient began IBUPROFEN therapy for pain and HEPARIN-FRACTION SODIUM SALT therapy for thrombosis prophylaxis. On 04 Apr 2006, the patient was discharged from the hospital with decreasing lab parameters. Monitoring of liver values was recommended, and a liver biopsy had been recommended, but was refused by the patient. It was also recommended to refrain from the use of oral contraceptives. The hospital report stated that no clear evidence of a certain etiology could be found. CONCEPLAN, IBUPROFEN, and HEPARIN-FRACTION SODIUM SALT were also considered suspect.

LABORATORY/COMMENTS:

Follow-up information received on 30 Jun 2006:

22 Mar 2006 Ultrasound: Steatosis hepatitis, splenomegaly, pre-existing hepatopathy?, no cholestasis.

22 Mar 2006 Physical examination: Without relevant findings

04 Apr 2006 Conjugated bilirubin 11.5 mg/dl
 04 Apr 2006 Unconjugated bilirubin 2.5 mg/dl
 Unknown date Serology HA/B/C: Negative
 Unknown date Leptospirosis: Negative
 Unknown date Serum lipase activity (SLA): Negative
 Unknown date Anti-smooth muscle antibody (ASMA): Negative
 Unknown date Liver-kidney microsome (LKM): Negative
 Unknown date Antineutrophilic cytoplasm (ANCA): Negative
 Unknown date Antibody PR3 (Ab-PR3): Negative
 Unknown date Antimitochondrial antibody (AMA): Negative
 Unknown date Antibody-MPO (Ab-MPO): Negative
 Unknown date Cuprum (Cu) 24-hr urine: 141 mcg (10-60)
 Unknown date Coeruloplasmin i.S.: 0,293 g/l (0.2-0.6)

ABBOTT, 11.08.2006 DE-ABBOTT-06P-062-0329479-00

Sender diagnosis: not reported

Narrative

Report from Germany of jaundice and increased liver function tests coincident with SEVOFLURANE (SEVOFLURANE LIQUID FOR INHALATION) therapy. On 30 Dec 2005, the patient began SEVOFLURANE LIQUID FOR INHALATION therapy for anesthesia for fractured lower leg surgery. On 21 Mar 2006, lab test reveal increased liver function tests and the patient was jaundice. On 21 Mar 2006, the patient was hospitalized for increased liver function test and jaundice. On 28 Mar 2006, the liver transaminases were decreasing, whereas the total bilirubin increased. The increased liver function tests and jaundice have not resolved. The reporter believed the increased liver function tests and jaundice were possibly related to SEVOFLURANE LIQUID FOR INHALATION therapy.

Follow-up information received on 03 May 2005: Suspect medication was added or revised. In 2002, the patient began NORMENSAL therapy for contraception. In 2006, while the patient was in the hospital, NORMENSAL therapy was discontinued. The increased liver function test started to normalize. On an unreported date, the patient was discharged. The reporter believed an alternative etiology for the increased liver function tests was NORMENSAL therapy. NORMENSAL therapy was also considered suspect.

Follow-up information received on 30 Jun 2006: Suspect medication has been added or revised. It was clarified that in 2002, the patient began CONCEPLAN therapy for contraception. On an unreported date, the patient began IBUPROFEN therapy for pain and HEPARIN-FRACTION SODIUM SALT therapy for thrombosis prophylaxis. On 04 Apr 2006, the patient was discharged from the hospital with decreasing lab parameters. Monitoring of liver values was recommended, and a liver biopsy had been recommended, but was refused by the patient. It was also recommended to refrain from the use of oral contraceptives. The hospital report stated that no clear evidence of a certain etiology could be found. CONCEPLAN, IBUPROFEN, and HEPARIN-FRACTION SODIUM SALT were also considered suspect.

Follow-up information received on 03 Aug 2006: Laboratory information was added or revised.

LABORATORY/COMMENTS:

Follow-up information received on 30 Jun 2006:

22 Mar 2006 Ultrasound: Steatosis hepatitis, splenomegaly, pre-existing hepatopathy?, no cholestasis.

22 Mar 2006 Physical examination: Without relevant findings

04 Apr 2006 Conjugated bilirubin 11.5 mg/dl

04 Apr 2006 Unconjugated bilirubin 2.5 mg/dl

Unknown date Serology HA/B/C: Negative

Unknown date Leptospirosis: Negative

Unknown date Serum lipase activity (SLA): Negative

Unknown date Anti-smooth muscle antibody (ASMA): Negative

Unknown date Liver-kidney microsome (LKM): Negative

Unknown date Antineutrophilic cytoplasm (ANCA): Negative

Unknown date Antibody PR3 (Ab-PR3): Negative

Unknown date Antimitochondrial antibody (AMA): Negative

Unknown date Antibody-MPO (Ab-MPO): Negative

Unknown date Cuprum (Cu) 24-hr urine: 141 mcg (10-60)

Unknown date Coeruloplasmin i.S.: 0,293 g/l (0.2-0.6)

Follow-up information received on 03 Aug 2006:

11 Apr 2006 EVB: 17.9+

21 Apr 2006 EVB: 17.6+

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06033097

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 19360306	Age: 70 Year	weight (kg): 102	BMI_int: 27.67	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug interaction					
2 Quick's value decreased					recovered/resolved
3 Restlessness					recovered/resolved
4 Chest pressure sensation					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	/	-	Periodontitis
ibuprofen		Tablet	-	
2 marcumar	Unknown	/	-	Drug use for unknown
phenprocoumon			-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Chest pressure sensation			Unknown	
Artificial cardiac pacemaker user			Unknown	
Bronchial carcinoma			Unknown	
Gout			Unknown	
Periodontitis			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Quick's test		26	TEST REPORTED AS QUICK'S VALUE	
Quick's test		30		
Quick's test		13	TEST REPORTED AS QUICK'S VALUE	
Quick's test		11	TEST REPORTED AS QUICK'S VALUE	

Narratives (nur bei elektronischem Eingang)

ROCHE, 26.07.2006 453275

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE WAS REPORTED BY A PHARMACIST AND CONCERNS AN APPROXIMATELY 70 YEAR OLD MALE PATIENT WHO EXPERIENCED DRUG INTERACTION, DECREASED QUICK VALUES, RESTLESSNESS AND CHEST OPPRESSION WHILE RECEIVING PHENPROCOUMON (MARCUMAR) TAKEN FOR AN UNKNOWN INDICATION AND IBUPROFEN TAKEN FOR DENTAL INFLAMMATION.

THE PATIENT HAS A MEDICAL HISTORY OF PACEMAKER, GOUT AND STATUS POST LEFT LUNG EXCISION DUE TO BRONCHIAL CARCINOMA.

ON AN UNSPECIFIED DATE, THE PATIENT COMMENCED PHENPROCOUMON. HE ALSO RECEIVED IBUPROFEN ONE DOSE FORM AND THE FOLLOWING DAY, HE RECEIVED ANOTHER TWO DOSE FORMS (ONE IN THE MORNING AND ONE IN THE EVENING). THE SAME DAY, IN THE EVENING, HE DEVELOPED RESTLESSNESS AND CHEST OPPRESSION. THE PATIENT WAS HOSPITALISED DUE TO A SUSPICION FOR ANGINA PECTORIS. IT WAS REPORTED THAT THE PATIENT EXPERIENCED A DRUG INTERACTION BETWEEN PHENPROCOUMON

AND IBUPROFEN. A QUICK'S TEST SHOWED A DECREASE IN QUICK VALUES FROM 26 TO 13 AND LATER TO 11. IT WAS REPORTED THAT THE PATIENT WAS DISCHARGED A DAY LATER AS THE SUSPICION FOR ANGINA PECTORIS WAS NOT CONFIRMED. AT THE TIME OF THIS REPORT, ALL THE EVENTS HAD RESOLVED. THE REPORTER ASSESSED ALL EVENTS AS RELATED TO THE USE OF PHENPROCUMON. NO FURTHER INFORMATION WAS AVAILABLE.

UPDATE INFORMATION WAS RECEIVED AND THE FOLLOWING DATA WAS ADDED TO THE CASE: THE PRIORITY OF THE CASE WAS CHANGED FROM LOW TO HIGH, PATIENT'S INITIALS, DATE OF BIRTH, ESTIMATED AGE, HEIGHT, WEIGHT, STRENGTH OF IBUPROFEN WAS CHANGED FROM 500 MG TO 400 MG AND INDICATION WAS CHANGED FROM CHEST OPPRESSION TO DENTAL INFLAMMATION, DOSAGE DETAILS FOR IBUPROFEN WERE CHANGED, THE SERIOUSNESS CRITERIA OF ALL EVENTS WAS CHANGED FROM NON-SERIOUS TO NEW/PROLONGED IN-PATIENT HOSPITALISATION, NEW EVENTS OF RESTLESSNESS AND CHEST OPPRESSION, THE EVENT OUTCOME FOR DECREASED QUICK'S VALUE WAS CHANGED FROM UNKNOWN TO RESOLVED, THE CAUSALITY FOR DECREASED QUICK VALUES WAS CHANGED FROM UNKNOWN TO YES, LAB TEST, MEDICAL HISTORY.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06062742

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19490429	Age: 57 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Alopecia	200606				recovered/resolved with sequel
2 Visual disturbance	200606				recovered/resolved with sequel
3 Dizziness	200606				recovered/resolved with sequel
4 Nausea	200606				recovered/resolved with sequel
5 Anorexia	200606				recovered/resolved with sequel
6 Jaundice	200606				recovered/resolved with sequel
7 Transaminases increased	200606				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibuprofen AbZ 400 mg Filmtabletten ibuprofen	Oral	/	200605- -	Knee pain

Drugs (concomitant)

2 carbimazole carbimazole		/	- -	
3 diclofenac diclofenac	Oral	/	2006-200605 -	Pain
4 iodine iodine		/	- -	
5 levothyroxine sodium levothyroxine sodium	Oral	125 Mg milligram(s)/Day	- -	Goiter exophthalmic

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Reflux gastritis			Unknown	
Cholelithiasis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06042560

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: 19850528	Age:	weight (kg): 80	BMI_int: 24.69	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Collapse circulatory					recovered/resolved
2 Renal failure acute					recovered/resolved
3 Effusion pleural					recovered/resolved
4 Ascites					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	600 Mg milligram(s)/	-	Headache
ibuprofen			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06050168

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19730512	Age: 33 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	20060818			2 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolo-puren ibuprofen	Oral	2 Df dosage form/Day	20060817- -	Toothache

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
ERCP	1998		No	
Choledocholithiasis			No	
Cholecystectomy	1996		No	
Tooth extraction			No	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06050794

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					Yes

Patient

Initials:	Sex: Male	Birth:	Age: 46 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal pain					Unknown
2 Fever					Unknown
3 Abdominal abscess					recovered/resolved
4 Pleural effusion					Unknown
5 Streptococcal sepsis					recovered/resolved
6 Hepatosplenomegaly					Unknown
7 Suture line infection					Unknown
8 Arthralgia					Unknown
9 Dysphagia					recovered/resolved
10 Weight loss					Unknown
11 Perforation stomach					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		/	-	Rheumatoid arthritis
ibuprofen			-	
2 prednisolone		/	-	Rheumatoid arthritis
prednisolone			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Appendectomy			No	

Primary source

Qualification:	observestudytype:
Literature:	Hueber A,Rech J,Kallert S,Requadt C,Cavallaro A,Kalden J,Schulze-Koops H. Paraneoplastic syndrome, infection or arthritis: difficulties in diagnosis. Int J Clin Pract 2006 Oct;60:10:1310-1312.
Qualification:	Physician
Literature:	observestudytype:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 30.10.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-05699GD

Sender diagnosis: not reported

Narrative

A patient with rheumatoid arthritis is presented whose symptoms and findings were highly suspicious of malignant disease but were caused by sepsis related to a complication of the drug treatment (with ibuprofen and prednisolone) of his underlying disease. The case demonstrates the difficulties to differentiate between paraneoplasia, infection and autoimmune disease and the potentially life-threatening consequences of their therapy.

A 46-year-old Caucasian male was admitted to the hospital because of weight loss of 13 kg in the last month, dysphagia and recent onset of remittent arthralgias.

8 weeks before, the patient suffered from pain of small and large joints for the first time. At that time, the diagnosis of an undifferentiated polyarthritis was made, and he was treated by a general practitioner with prednisolone 20 mg once daily and ibuprofen 500 mg twice daily (bid). Few days after initiation of treatment, the patient was admitted to the hospital with acute abdominal pain, which was attributed to a perforation of the stomach. After operation, he recovered, and the post-operative course was without complications.

3 weeks after the operation, the patient was admitted to the authors' department because of dramatic weight loss, dysphagia and arthralgias.

The past medical history was unremarkable except for an appendectomy 29 years ago. The patient smoked 20 cigarettes a day for the last 30 years.

At physical examination the patient presented acutely sick. He was, nevertheless, afebrile and had normal respiration and blood pressure. The pulse rate was at 130/min. Furthermore, he had experienced massive weight loss and presented with a body mass index of 15.7 (height: 1.75 m and weight: 48 kg) and dysphagia. His mouth opening was limited and he had white patches of about 1 - 1.5 cm in diameter on the tongue mucosa. The abdominal post-operative suture showed a 2-cm infected insufficiency at its lower end.

The patient had no joint pain, but movement of the upper arms caused muscle pain.

Representative laboratory values on admission to the hospital were haemoglobin 10.9 g/dl (normal: 13 - 17 g/dl), leucocyte count 25.1×10^3 cells/mcl (normal: $4 - 10 \times 10^3$ cells/mcl), with a left shift and relative lymphopenia, thrombocyte count 510×10^3 cells/mcl (normal: $140 - 400 \times 10^3$ cells/mcl), elevated liver enzymes and hypoalbuminemia. C-reactive protein was 213 mg/l (normal: < 5 mg/l), and immunoglobulin levels were elevated.

Abdominal ultrasound revealed echopoor lesions in the liver segments 2 and 6 - 8 with a size of up to 5 cm in diameter. Computer tomography (CT) revealed hepatosplenomegaly and multiple hypodense lesions, some of which divided by septums, in the liver, spleen and questionable lesions in both kidneys.

Furthermore, swallowing tests showed that the patient had a dysfunction in deglutition, but a neoplastic process in the area of ears, nose and throat could not be detected.

Inflammation was obviously based on the initial laboratory results. Therefore, antibiotic therapy with piperacillin/tazobactam 4/0.5 g iv every 8 h and parenteral nutrition were started. Clinical evaluation with weight loss and dysphagia, however, was strongly indicative of a malignant disease.

With the working hypothesis of an infection or a neoplasia with unknown origin, biopsy of the liver lesions was initiated.

Surprisingly, pus drained from the liver lesions. Microbiological analysis revealed *Streptococcus intermedius*, and consequently, the antibiotic regime was augmented to meropenem [intravenous (iv), 1 g every 8 h], vancomycin (iv, 1 g every 12 h) and linezolid (peroral, 600 mg every 12 h). During the following days, the patient developed pleural effusions, which did not result from malignancy but from subphrenic abscesses. To shrink the bacterial caverns, 3 drainages were placed into the biggest abscesses and rinsed out regularly. After the intervention, the patient had fever of up to 39.5 degC due to systemic sepsis.

With appropriate treatment, the patient recovered rapidly from the sepsis; dysphagia resolved, but subsequently he increasingly suffered from joint pain, clinically concordant with active rheumatoid arthritis.

Additional evidence for rheumatoid arthritis was provided by a positive rheumatoid factor, antibodies to cyclic citrullinated peptides and hyperperfusion of hand joints in skeletal scintigraphy. Joint pain was treated with the Cox-2-inhibitor rofecoxib 25-mg bid and sustained morphine release (buprenorphine 35 mcg/h).

After 36 days, the patient was discharged from the hospital with oral antibiotic amoxicillin and clavulanic acid 875/125-mg bid and pain medication as stated above.

For a follow-up and initiation of a basic therapy for rheumatoid arthritis, the patient was admitted again to the hospital 4 weeks later. In the meantime, he had experienced no fever, night sweats, weight loss or dysphagia. However, his joint pain had worsened and caused limitations of movement. Morning stiffness had prolonged to 30 mins. Physical examination revealed a disease activity score (DAS28) of 6.08, indicating highly active disease.

Abdominal CT demonstrated remaining abscesses; however, the number and sizes were significantly reduced.

Basic therapy for rheumatoid arthritis was started cautiously with sulfasalazine and low-dose corticosteroids (prednisolone 12.5 mg per day).

About 4 months after the first admission, the abscesses were completely resolved. Methotrexate (10 mg per week) was added for arthritis therapy, and corticosteroids were tapered down.

6 weeks later, the patient was in clinical remission, with a DAS28 of 2.56. Current medication is methotrexate as stated above, prednisolone 5 mg/day, sulfasalazine 2 g/day and ibuprofen 600 mg bid.

The authors state: "In conclusion, we demonstrate a patient who presented with weight loss, dysphagia and multiple liver and spleen abscesses, which were the consequence of complications of an early rheumatoid arthritis and its therapy, although initially highly suspicious of a malignant disease. At the first medical contact, the patient was complaining about arthralgias, and at that time, an early undifferentiated inflammatory arthritis was diagnosed. Due to therapy-related side-effects, the patient developed gastric perforation, and therefore, this case emphasises that the initial combination of NSAIDs with steroids dangerously leads to a highly increased risk for gastrointestinal ulceration, as has been reported elsewhere. As a result of perforation, the patient developed abdominal septical abscesses, and at presentation to our hospital, he had experienced weight loss and dysphagia, symptoms unrelated to arthritis but suggestive of neoplasia or systemic infection."

Boehringer Ingelheim GmbH, Germany, 31.10.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-05699GD

Sender diagnosis: not reported

Narrative

A patient with rheumatoid arthritis is presented whose symptoms and findings were highly suspicious of malignant disease but were caused by sepsis related to a complication of the drug treatment (with ibuprofen and prednisolone) of his underlying disease. The case demonstrates the difficulties to differentiate between paraneoplasia, infection and autoimmune disease and the potentially life-threatening consequences of their therapy.

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3 weeks after the operation, the patient was admitted to the authors' department because of dramatic weight loss, dysphagia and arthralgias.

The past medical history was unremarkable except for an appendectomy 29 years ago. The patient smoked 20 cigarettes a day for the last 30 years.

At physical examination the patient presented acutely sick. He was, nevertheless, afebrile and had normal respiration and blood pressure. The pulse rate was at 130/min. Furthermore, he had experienced massive weight loss and presented with a body mass index of 15.7 (height: 1.75 m and weight: 48 kg) and dysphagia. His mouth opening was limited and he had white patches of about 1 - 1.5 cm in diameter on the tongue mucosa. The abdominal post-operative suture showed a 2-cm infected insufficiency at its lower end.

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With appropriate treatment, the patient recovered rapidly from the sepsis; dysphagia resolved, but subsequently he increasingly suffered from joint pain, clinically concordant with active rheumatoid arthritis.

Additional evidence for rheumatoid arthritis was provided by a positive rheumatoid factor, antibodies to cyclic citrullinated peptides and hyperperfusion of hand joints in skeletal scintigraphy. Joint pain was treated with the Cox-2-inhibitor rofecoxib 25-mg bid and sustained morphine release (buprenorphine 35 mcg/h).

After 36 days, the patient was discharged from the hospital with oral antibiotic amoxicillin and clavunilate 875/125-mg bid and pain medication as stated above.

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6 weeks later, the patient was in clinical remission, with a DAS28 of 2.56. Current medication is methotrexate as stated above, prednisolone 5 mg/day, sulfasalazine 2 g/day and ibuprofen 600 mg bid.

The authors state: "In conclusion, we demonstrate a patient who presented with weight loss, dysphagia and multiple liver and spleen abscesses, which were the consequence of complications of an early rheumatoid arthritis and its therapy, although initially highly suspicious of a malignant disease. At the first medical contact, the patient was complaining about arthralgias, and at that time, an early undifferentiated inflammatory arthritis was diagnosed. Due to therapy-related side-effects, the patient developed gastric perforation, and therefore, this case emphasises that the initial combination of NSAIDs with steroids dangerously leads to a highly increased risk for gastrointestinal ulceration, as has been reported elsewhere. As a result of perforation, the patient developed abdominal septical abscesses, and at presentation to our hospital, he had experienced weight loss and dysphagia, symptoms unrelated to arthritis but suggestive of neoplasia or systemic infection."

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06056633

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abnormal liver function tests	20061112	20061129	18 Day		recovered/resolved
2 Agitation	20061107				Unknown
3 Myalgia	20061112				not recovered/not resolved
4 Ophthalmitis	20061110	200611			recovered/resolved
5 Pain in arm	20061112				Unknown
6 Sleeplessness	20061107	200611			recovered/resolved
7 Confusion	20061107				Unknown
8 Hallucinations	20061107	20061107			recovered/resolved
9 Headache	20061107	20061107			recovered/resolved
10 Hearing impaired	200611				not recovered/not resolved
11 Increased intraocular pressure	20061110	200611			recovered/resolved
12 Aggressive behavior	20061110	200611			recovered/resolved
13 Bacterial infection	20061112	20061129	18 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 HEDERA HELIX hedera helix	Oral	/	-	Product used for unknown indication
2 ibuprofen 200 mg ibuprofen	Oral	200 Mg milligram(s)/	200611-200611	Respiratory infection
3 ibuprofen 200 mg ibuprofen	Oral	200 Mg milligram(s)/	200611-200611	Bronchitis
4 UNACID PD ORAL sultamicillin tosilate	Oral	4 Df dosage form/Day	200611-200611	Respiratory infection
5 UNACID PD ORAL sultamicillin tosilate	Oral	4 Df dosage form/Day	200611-200611	Bronchitis
6 wick medinait dextromethorphan hydrobromide; doxylamine succinate; ephedrine sulfate; paracetamol		/Day	20061110-	Product used for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pollinosis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20061115	300	U/L	
Alkaline phosphatase	20061110	180	U/L	
Body temperature	200611	36.9	Centigrade	
GGT	20061115	271	U/L	

GGT	20061110	211	U/L
GPT	20061115	82	U/L
GPT	20061110	88	U/L
Lactate dehydrogenase	20061115	226	U/L

Narratives (nur bei elektronischem Eingang)

PFIZER INC, 22.11.2006 DE-PFIZER INC-2006138302

Sender diagnosis: not reported

Narrative

This physician reports of a 44-year-old male patient who was treated with Unacid PD oral (dosage was not provided) due to bronchitis and respiratory infection (exact therapy onset was not provided). Additionally, he was treated with Ibuprofen 200 mg for the same indication. On 07Nov2006 he developed an intense state of agitation/restlessness, sleeplessness, "confused talking" and aggressiveness/irritability. Unacid PD oral was discontinued due to the events on 08Nov2006. The patient still showed signs of irritability and aggressiveness on 09Nov2006. The outcome of the other events is unknown. The patient is going to consult the reporting physician on 10Nov2006. The reporting physician noticed that the patient's wife showed similar adverse drug reactions under clarithromycin in 2005.

Company Clinical Comment.

The information available in this report is limited, and does not allow a medically meaningful assessment of the case. In particular the following relevant information is not available: patient's history, presence of respiratory failure with hypoxemic and/or hypercapnic disorders, oxygenotherapy needed, start data of Unacid administration, and concomitant medication used, and their indications.

This case will be reassessed when additional information becomes available.

PFIZER INC, 06.02.2007 DE-PFIZER INC-2006138302

Sender diagnosis: not reported

Narrative

This physician reports of a 44-year-old male patient who was treated with Unacid PD oral (dosage was not provided) due to bronchitis and respiratory infection (exact therapy onset was not provided). Additionally, he was treated with Ibuprofen 200 mg for the same indication. On 07Nov2006 he developed an intense state of agitation/restlessness, sleeplessness, "confused talking" and aggressiveness/irritability. Unacid PD oral was discontinued due to the events on 08Nov2006. The patient still showed signs of irritability and aggressiveness on 09Nov2006. The outcome of the other events is unknown. The patient is going to consult the reporting physician on 10Nov2006. The reporting physician noticed that the patient's wife showed similar adverse drug reactions under clarithromycin in 2005.

Follow-up (23Jan2007).

Follow up was received from the same physician. This 44-year old male patient without a relevant medical history beside pollinosis presented first to his office on 06Nov2006 with a severe common cold and a reduced general condition. Due to suspicion of a beginning pleurisy, the patient received a prescription for Unacid PD oral 2 x 2 tablets. The patient concomitantly took Ibu Ct 200 mg (ibuprofen) and Hedelix solution by his own. On 07Nov2006 the patient presented again and complained to have experienced a terrible headache, sleep disorder and hallucinations during the past night. The patient related these symptoms to the additional intake of ibuprofen. As a measure of precaution, the physician also discontinued Unacid PD oral. On 10Nov2006 the patient had red eyes and had an aggressive behavior, whereas the common cold and the pulmonary findings had improved. On 12Nov2006 the patient complained about pain which made him unable to move his arm. Both, an internal medical and orthopedic surgical investigation were uneventful. Attacks of pyrexia, which the patient reported additionally, could not be verified by the reporting physician (body temperature measured in his office was 36.9). Laboratory values showed increased gamma GT, transaminases and alkaline phosphatase, but normal bilirubin; furthermore a mild anemia, increased leukocytes and platelets. The erythrocyte sedimentation rate ESR was increased (90/96). The treating physician attributed these findings to a bacterial infection and prescribed another antibiotic (not specified), which the patient, however, did not take. Instead, he administered large amounts of Wick Medinait, which was recommended by the pharmacist. The laboratory values were found to have returned to normal by 29Nov2006. In the meantime, the patient had been presented to an ophthalmologist because of his red eyes, who diagnosed an ophthalmitis with a transitorily increased intraocular pressure. The ophthalmic symptoms improved on a not further specified treatment with corticoids. The physician further reported that the patient had been suffering from myalgias and a not further specified hearing problem since this episode of illness in Nov2006. Despite several investigations, no cause could be found. The reporting physician and all other physicians involved recommended a rheumatological investigation, but the patient has not been willing to do so yet.

Company Clinical Evaluation:

Based on the positive temporal association between treatment administration and onset of the events; a causality relationship cannot be completely excluded. However, all reported events seem to be consequence of a patient's underlying systemic/rheumatological illness which has not been further investigated. This case will be reassessed when additional information becomes available.

PFIZER INC, 23.03.2007 DE-PFIZER INC-2006138302

Sender diagnosis: not reported

Narrative

This physician reports of a 44-year-old male patient who was treated with Unacid PD oral (dosage was not provided) due to bronchitis and respiratory infection (exact therapy onset was not provided). Additionally, he was treated with Ibuprofen 200 mg for the same indication. On 07Nov2006 he developed an intense state of agitation/restlessness, sleeplessness, "confused talking" and aggressiveness/irritability. Unacid PD oral was discontinued due to the events on 08Nov2006. The patient still showed signs of irritability and aggressiveness on 09Nov2006. The outcome of the other events is unknown. The patient is going to consult the reporting physician on 10Nov2006. The reporting physician noticed that the patient's wife showed similar adverse drug reactions under clarithromycin in 2005.

Follow-up (23Jan2007).

Follow up was received from the same physician. This 44-year old male patient without a relevant medical history beside pollinosis presented first to his office on 06Nov2006 with a severe common cold and a reduced general condition. Due to suspicion of a beginning pleurisy, the patient received a prescription for Unacid PD oral 2 x 2 tablets. The patient concomitantly took Ibu Ct 200 mg (ibuprofen) and Hedelix solution by his own. On 07Nov2006 the patient presented again and complained to have experienced a terrible headache, sleep disorder and hallucinations during the past night. The patient related these symptoms to the additional intake of ibuprofen. As a measure of precaution, the physician also discontinued Unacid PD oral. On 10Nov2006 the patient had red eyes and had an aggressive behavior, whereas the common cold and the pulmonary findings had improved. On 12Nov2006 the patient complained about pain which made him unable to move his arm. Both, an internal medical and orthopedic surgical investigation were uneventful. Attacks of pyrexia, which the patient reported additionally, could not be verified by the reporting physician (body temperature measured in his office was 36.9). Laboratory values showed increased gamma GT, transaminases and alkaline phosphatase, but normal bilirubin; furthermore a mild anemia, increased leukocytes and platelets. The erythrocyte sedimentation rate ESR was increased (90/96). The treating physician attributed these findings to a bacterial infection and prescribed another antibiotic (not specified), which the patient, however, did not take. Instead, he administered large amounts of Wick Medinait, which was recommended by the pharmacist. The laboratory values were found to have returned to normal by 29Nov2006. In the meantime, the patient had been presented to an ophthalmologist because of his red eyes, who diagnosed an ophthalmitis with a transiently increased intraocular pressure. The ophthalmic symptoms improved on a not further specified treatment with corticoids. The physician further reported that the patient had been suffering from myalgias and a not further specified hearing problem since this episode of illness in Nov2006. Despite several investigations, no cause could be found. The reporting physician and all other physicians involved recommended a rheumatological investigation, but the patient has not been willing to do so yet.

Follow-up (12Mar2007):

The same reporting physician added that the patient took 2 coated tablets of Unacid PD oral in the evening of 06Nov2006 and 2 coated tablets of Unacid PD oral in the morning of 07Nov2006. From 10Nov2006 he used up completely 1 bottle of Wick Medinait for the day and 1 bottle Wick Medinait for the night.

Company Clinical Evaluation:

Based on the positive temporal association between treatment administration and onset of the events; a causality relationship cannot be completely excluded. However, all reported events seem to be consequence of a patient's underlying systemic/rheumatological illness which has not been further investigated. This case will be reassessed when additional information becomes available.

McNeil Germany, Local Safety Officers, 10.12.2012 DE-JNJFOC-20070200474

Sender diagnosis: not reported

Narrative

This spontaneous report received by BRM on 01-FEB-2007 via a business partner (Pfizer 2007305423) from a physician concerns a 44-year-old male from Germany.

The patient's medical history and concurrent conditions included: pollinosis. The patient's weight was not reported. The patient was treated with ibuprofen (unspecified, oral) 200 mg (frequency unknown) initiated in NOV-2006 to NOV-2006 for bronchitis, respiratory infection and beginning

pleurisy. Non-company suspect drugs included: Unacid PD Oral (sultamicillin tosilate) 2 doses twice daily initiated on 06-NOV-2006 to 08-NOV-2006 for bronchitis, respiratory infection and beginning pleurisy. Concomitant medications included heder helix. The patient presented at a physician's office on 06-NOV-2006 with a severe common cold and reduced general condition and was treated with ibuprofen, sultamicillin tosilate and heder helix. On 07-NOV-2006 the patient presented again and complained to have experienced a terrible headache, sleeplessness and hallucinations ("confused talking") during the previous night. The patient related his symptoms to the additional intake of ibuprofen. The physician discontinued Unacid PD Oral (sultamicillin tosilate). On 10-NOV-2006 the patient had red eyes and an aggressive behavior and the common cold and pulmonary findings had improved. On 12-NOV-2006 the patient complained about pain, which made him unable to move his arm. Both an internal medical and orthopaedic surgical investigation were uneventful. The patient also reported episodes of pyrexia but could not be verified by the physician (body temperature measured in his office was 36.9 degrees Celsius). On 10-NOV-2006 the patient had the following laboratory tests; alkaline phosphatase (AP)-180 IU/L, gamma glutamyl transferase (GGT)-211 IU/L, glutamate-pyruvate transamin (GPT) - 88 IU/L and a "normal" bilirubin. Other laboratory values included a hemoglobin "decreased down to 11.5", leukocytes "increased up to 11500" and thrombocytes "increased up to 676000", all which were reported to be due to infection. Also reported was mild anemia and increased platelets. The erythrocyte sedimentation rate (ESR) was increased (90/96). The treating physician attributed these findings to a bacterial infection and prescribed another antibiotics (not specified) which the patient did not take. Instead he treated himself with large amounts of Wick Medinait (dextromethorphan/paracetamol/ephedrine/doxylamine), which was recommended by the pharmacist. On 29-NOV-2006 the patient's laboratory results had returned to normal. In the meantime the patient had presented to an ophthalmologist because of red eyes, who diagnosed an ophthalmitis with a transitory increased intraocular pressure. The ophthalmic symptoms improved on a not further specified treatment with corticosteroids. The physician further reported that the patient had been suffering from myalgias and a not further specified hearing problem since this episode of illness in NOV-2006. Despite several investigations, no cause could be found. The reporting physician and all other physicians involved recommended a rheumatological investigation, but the patient had not been willing to do so at the time of the report. Treatment with ibuprofen and sultamicillin tosilate was withdrawn. This report was serious (medically significant).

Additional information was received on 29-NOV-2012.

It was determined that Manufacturer Report Number 2007305423 was a duplicate of this case. All relevant information regarding this case will be submitted under Manufacturer Report Number 20070200474.

Patient was treated with non company suspect heder helix (unspecified formulation, oral) and vicks medinait (formulation and route of administration unspecified) for an unspecified indication. Heder helix was previously reported as concomitant medication. On 07-NOV-2006, the patient experienced confusion, on 10-NOV-2006, the patient experienced increased intraocular pressure and oph<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06058484

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth:	Age: 27 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt					Unknown
2 Drug abuse					Unknown
3 Renal insufficiency			8 Day		recovered/resolved
4 Overdose					Unknown
5 Hepatic damage			8 Day		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC diclofenac sodium		/	-	
2 IBUPROFEN ibuprofen		/	-	
3 PARACETAMOL paracetamol	Oral	/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal insufficiency			Unknown	

Primary source

Qualification: observestudytype:

Literature: von Mach M,Hermanns-Clausen M,Koch I,Hengstler J,Lauterbach M,Kaes J,Weilemann L. Experiences of a poison center network with renal insufficiency in acetaminophen overdose: an analysis of 17 cases. J Toxicol Clin Toxicol 2005;43:1:31-37.

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 12.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06545GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D: creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 27;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 8000;

Possibly influencing factor: renal insufficiency, 1000 mg diclofenac (overdose), 3600 mg ibuprofen (overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 2;

Duration of renal insufficiency: 8 days.

It was outlined in general that in 13 patients acetaminophen was ingested with suicidal intention (including the case above). Renal insufficiency was transient and normalized between 2 days and 4 weeks after the acute acetaminophen overdose.

Due to the medical circumstances the case was rated serious: seriousness criterion "other" (medically significant).

The authors state: "These observations indicated that the risk for renal insufficiency after acetaminophen overdose in the present 17 patients did not correlate with the ingested amount of acetaminophen and that nephrotoxicity occurred even below the level of hepatotoxicity. The most frequently observed condition which therefore might increase the risk for renal insufficiency in acute acetaminophen overdose was concomitant ingestion of nephrotoxic substances, predominantly nonsteroidal anti-inflammatory drugs, which are well known for their acute and chronic nephrotoxicity. ... An additional predisposing condition seemed to be a pre-existing renal insufficiency with acetaminophen causing an acute-on-chronic renal failure that was observed in 3 patients."

16 additional cases are presented in this paper:

2006-DE-06518GD

2006-DE-06526GD

2006-DE-06527GD

2006-DE-06528GD

2006-DE-06530GD

2006-DE-06532GD

2006-DE-06533GD

2006-DE-06535GD

2006-DE-06544GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

Boehringer Ingelheim GmbH, Germany, 13.12.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-06545GD

Sender diagnosis: not reported

Narrative

A patient with renal insufficiency in acute acetaminophen overdose is reported.

This case is from a retrospective analysis of patients with acetaminophen-related nephrotoxicity reported to a poison center network from 1995 to 2003. Renal insufficiency was defined as elevated serum creatinine of more than double of the normal range (> 2.4 mg/dl [212 µmol/l]). Patients were classified into 4 groups (A: creatinine 2.4-5.0 mg/dl, B: creatinine > 5.0 mg/dl requiring no dialysis, C: creatinine > 5.0 mg/dl requiring dialysis, D:

creatinine > 5.0 mg/dl with fatal outcome).

Patient details:

Age (years): 27;

Gender: male;

Group: B;

Ingested amount of acetaminophen (mg): 8000;

Possibly influencing factor: renal insufficiency, 1000 mg diclofenac (overdose), 3600 mg ibuprofen (overdose);

Risk of hepatic toxicity (by nomogram 4 - 24 h after overdose): - ;

Hepatic damage: yes;

Time of presentation (after overdose): day 1;

Time of onset of renal insufficiency (after overdose): day 2;

Duration of renal insufficiency: 8 days.

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2006-DE-06535GD

2006-DE-06544GD

2006-DE-06548GD

2006-DE-06550GD

2006-DE-06551GD

2006-DE-06552GD

2006-DE-06553GD

2006-DE-06556GD

2006-DE-06560GD

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07001151

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials: Sex: Female Birth: 19590301 Age: 46 Year weight (kg): 67 BMI_int: 24.61 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Alkaline phosphatase increased	200607	200702	7 Month		recovered/resolved
2 Bilateral parotitis	200603				Unknown
3 Cerebral infarction	200601				Unknown
4 CRP increased	20061211				recovered/resolved
5 Drug level decreased	20061106				recovered/resolved
6 Vaginitis bacterial	200602				Unknown
7 Exacerbation of psoriasis					Unknown
8 Perianal abscess	200601				Unknown
9 Septicaemia	200602				Unknown
10 Venous occlusion					Unknown
11 Erysipelas	200602				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuflam		/	-	
ibuprofen			-	
2 IMMUNOSPORIN	Oral	100 Mg milligram(s)/Day	20061216-	Psoriatic arthropathy
ciclosporin			-	
3 IMMUNOSPORIN	Oral	200 Mg milligram(s)/Day	200510-	Psoriatic arthropathy
ciclosporin			-	
4 IMMUNOSPORIN		/	200510-	Psoriatic arthropathy
ciclosporin			-	

Drugs (concomitant)					
5	ALLOPURINOL allopurinol	Oral	150 Mg milligram(s)/Day	-	
6	ASS "HEXAL" acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	200605-	Vein disorder NOS
7	BISOPROLOL bisoprolol		10 Mg milligram(s)/Day	200606-	Tachycardia
8	bondronat ibandronic acid		1 Df dosage form/30 Day	-	
9	EINSALPHA alfacalcidol	Intravenous (not otherwis	1 Df dosage form/8 Day	-	
10	ERYPO FS epoetin alfa	Subcutaneous	1 Df dosage form/21 Day	-	
11	ibuprofen ibuprofen		600 Mg milligram(s)/Day	-	
12	insuman rapid insulin human; isophane insulin		/	-	
13	ISOPTIN verapamil hydrochloride		/	-200606	Tachycardia
14	LANTUS insulin glargine		/	-	
15	L-THYROXIN levothyroxine sodium		150 Mg milligram(s)/Day	-	
16	NEFRO ZINC zinc		1 Df dosage form/21 Day	-	
17	NOVAMINSULFON-RATIOPHARM metamizole		/	-	
18	OMEPRAZOL "AZUPHARMA" omeprazole	Oral	20 Mg milligram(s)/Day	-	
19	solu-dacortin prednisolone sodium succinate	Intravenous (not otherwis	1 Df dosage form/21 Day	-	
20	Urbason methylprednisolone		0.5 Df dosage form/Day	-	
21	VITAMIN B KOMPLEX calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Intravenous (not otherwis	1 Df dosage form/8 Day	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Multimorbidity			Unknown	
Anemia			Unknown	
Vitamin D decreased			Unknown	Low vitamine D3 level
Diabetic nephropathy			Unknown	
Autoimmune disorder			Unknown	
Nerve root injury			Unknown	nerve opticus atrophy
Dialysis	199809		Unknown	dialysis requirement due to preexisting diabetes mellitus
Cardiac disorder			Unknown	Open foramen ovale
Erythrocytes decreased			Unknown	
Herniotomy			Unknown	
Tibia fracture	20000704		Unknown	
Renal failure			Unknown	Terminal renal failure
Potassium increased			Unknown	
Phosphate increased			Unknown	
Penicillin allergy	199810		Unknown	Penicillin allergy 10/98 and 07/99
Muscle weakness			Unknown	
Infection NOS			Unknown	life-threatening infections
Hysterectomy	199812		Unknown	
Hypertension arterial	1994		Unknown	Arterial hypertension
Hyperparathyroidism			Unknown	Hyperparathyroidism PTH>1800
Hepatitis B			Unknown	
Haemoglobin decreased			Unknown	
Haematocrit decreased			Unknown	
Gout			Unknown	
Glaucoma	200104		Unknown	
Diabetes mellitus	1992		Unknown	
Creatinine increased			Unknown	
Calcium decreased			Unknown	
Arrhythmia	199911		Unknown	Paroxysmale supraventricular tachycardia
Caesarean section	1992		Unknown	

Primary source

Qualification: Physician observestudytype:
 Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 18.01.2007 DE-002147023-PHNU2007DE00506

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 09 Jan 2007. This patient was treated with Immunosporin (ciclosporin microemulsion) since about Oct 2005, for an unspecified indication. The patient was multimorbid and was on concomitant ibuprofen and other medications (in total 14 different, unspecified drugs), according to reporter. In Dec 2006, a "drastic but slow" increase of alkaline phosphatase was noted. Starting from 187 (pretreatment value), an increase to "above 1000" was noted (unit, reference range and dates not reported). No further data were provided.

The available information was considered inadequate to fully assess the case.

Serious spontaneous report [medically significant hazard], assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [underlying condition, multimorbidity] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 22.01.2007 DE-002147023-PHNU2007DE00506

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 09 Jan 2007. This patient was treated with Immunosporin (ciclosporin microemulsion) since about Oct 2005, for an unspecified indication. The patient was multimorbid and was on concomitant ibuprofen and other medications (in total 14 different, unspecified drugs), according to reporter. In Dec 2006, a "drastic but slow" increase of alkaline phosphatase was noted. Starting from 187 (pretreatment value), an increase to "above 1000" was noted (unit, reference range and dates not reported). No further data were provided.

Follow up received on 11 Jan 2007 (phone call from physician). Further patient details were given. The patient (a polymedicated female from Turkey, with known dialysis requirement due to preexisting diabetes mellitus) was treated with Immunosporin for psoriatic arthritis. According to reporter, the treating nephrologist withdrew Immunosporin after AP values exceeding 1000 U/l were noticed. The alkaline phosphatase improved (Jan 2007: 700 U/l), but, the patient showed a massive psoriatic exacerbation, subsequently. The reporting physician suspected Immunosporin as causally related, and also regarded the (long term) comedication with Ibufam (ibuprofen) as co-suspected.

Exacerbation of psoriasis after discontinuation of ciclosporin (disease progression) [medically significant hazard, already listed in the Basic Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 20.02.2007 DE-002147023-PHNU2007DE00506

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 09 Jan 2007. This patient was treated with Immunosporin (ciclosporin microemulsion) since about Oct 2005, for an unspecified indication. The patient was multimorbid and was on concomitant ibuprofen and other medications (in total 14 different, unspecified drugs), according to reporter. In Dec 2006, a "drastic but slow" increase of alkaline phosphatase was noted. Starting from 187 (pretreatment value), an increase to "above 1000" was noted (unit, reference range and dates not reported). No further data were provided.

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Follow-up report received on 09 Feb 2007 and 16 Feb 2007 (combined). Cerebral infarction (Jan 2006), perianal abscess (Jan 2006), sepsis by erysipiel on left lower leg (Feb 2006), purulent vaginitis (Feb 2006), central venous occlusion, parotitis both sides (Mar 2006) were mentioned as additional diagnosis. Immunosporin treatment started Oct 2005 at a dose of 150 mg. Alkaline phosphatase (AP) increase started in Dec 2006 until Feb 2007. AP value was 169 U/L in Mar 2006 and continuously increase to 573 U/L in Jul 2006 and to 1027 U/L in Dec 2006. After ciclosporin dose reduction (2x50 mg/d) the AP values improved to 350 U/L (Feb 2007) and the patient recovered. Ciclosporin plasma levels were low (range 29-68; target range 75-300 ug/L) and CRP was increased (range 30-153; normal range 0-5 mg/L). The provided lab data showed other abnormal values, however these were confirmed as pre-existing and independent from actual Immunosporin treatment. The relationship to Immunosporin was assessed as probable. Alternative causative factor was hyperparathyroidism.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07003238

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth:	Age: 84 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibu 400 - 1 A Pharma ibuprofen	Oral	1200 Mg milligram(s)/Day	- 3 Week	

Drugs (concomitant)

2 amiodarone amiodarone	Oral	200 Mg milligram(s)/	-	
3 berodual fenoterol hydrobromide; ipratropium bromide	Oral	6 Df dosage form/Day	-	
4 enalapril enalapril	Oral	5 Mg milligram(s)/	-	
5 novalgine metamizole	Oral	90 Gtt drop(s)/	-	
6 osyrol spironolactone	Oral	25 Mg milligram(s)/	-	
7 pantozol pantoprazole sodium	Oral	20 Mg milligram(s)/	-	
8 torasemide torasemide	Oral	5 Mg milligram(s)/	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-07003242

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes					Yes		

Initials: Sex: Male Birth: Age: 44 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Myalgia	20061112				not recovered/not resolved
2 Aggression	20061110				recovered/resolved
3 Insomnia	20061107				Unknown
4 Headache	20061107				recovered/resolved
5 Bacterial infection	20061112				Unknown
6 Agitation	20061112				recovered/resolved
7 Panophthalmitis	20061112				Unknown
8 Intraocular pressure increased	20061110				Unknown
9 Hearing impaired	20061112				not recovered/not resolved
10 Hallucination	20061107				recovered/resolved
11 Liver function test abnormal	20061112				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	200 Mg milligram(s)/Day	200611-200611	Bronchitis
2 unacid pd oral sultamicillin tosilate	Oral	/	-	-

Drugs (concomitant)

3	hedelix hedera helix	Oral	/	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pollinosis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07007567

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes		Yes		YES	

Patient

Initials:	Sex: Female	Birth: 20051110	Age: 0 Day	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Atrial septal defect	20051110				recovered/resolved
2 Aortic coarctation	20051110				recovered/resolved
3 Pulmonary hypertension	20051110				recovered/resolved
4 Jaundice neonatal	20051110				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aciclovir	Transplacental	/	-	Herpes dermatitis
aciclovir			-	
2 amoxicillin	Transplacental	/	-	Cystitis
amoxicillin			-	
3 Arbutinum	Transplacental	/	-	Cystitis
homeopatics nos			-	
4 citalopram	Transplacental	/	-	Depression
citalopram			-	
5 ibuprofen	Transplacental	/	-	Pain NOS
ibuprofen			-	
6 ofloxacin	Transplacental	/	-	Cystitis
ofloxacin			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07005232

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19580311	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Autoimmune hepatitis	200612				recovering/resolving
2 Jaundice	200612				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 FORMIGRAN	Unknown	2* /Day	200612-200612	Migraine
naratriptan hydrochloride		Tablet	-	
2 ibuprofen	Unknown	/	-	Migraine
ibuprofen			-	
3 maxalt	Unknown	/	-	
rizatriptan benzoate			-	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 13.03.2007 DE-GlaxoSmithKline-B0458882A

Sender diagnosis: not reported

Narrative

This case was reported by a female pharmacy assistant who was also the patient and described the occurrence of autoimmune hepatitis when she received naratriptan hydrochloride (Formigran) tablet for migraine.

Concurrent medications included Ibuprofen and rizatriptan benzoate (Maxalt).

On an unknown date in December 2006 the patient started naratriptan hydrochloride one tablet twice per day. On an unknown date in December 2006, the patient experienced autoimmune hepatitis and jaundice. She was admitted to hospital. It was noted that the patient had no history of alcohol misuse. The events were considered to be jeopardising and requiring intervention. At the time of reporting, the events were improved.

GlaxoSmithKline, 04.05.2007 DE-GlaxoSmithKline-B0458882A

Sender diagnosis: not reported

Narrative

This case was reported by a female pharmacy assistant who was also the patient and described the occurrence of autoimmune hepatitis when she received naratriptan hydrochloride (Formigran) tablet for migraine.

Concurrent medications included Ibuprofen and rizatriptan benzoate (Maxalt).

On an unknown date in December 2006 the patient started naratriptan hydrochloride one tablet twice per day. On an unknown date in December 2006, the patient experienced autoimmune hepatitis and jaundice. She was admitted to hospital. It was noted that the patient had no history of alcohol misuse. The events were considered to be jeopardising and requiring intervention. At the time of reporting, the events were improved.

Follow-up information received on 25 April 2007:

The patient was a 49-year-old female.

Co-suspect medication included Maxalt and Ibuprofen.

The patient stated that the physician currently assumes a viral hepatitis to be involved. Physicians report states that actual diagnosis was unclear chronic hepatitis, prolonged viral adjunctive hepatitis or atypical autoimmune hepatitis.

The patient was going to visit a specialised physician for liver illness.

GlaxoSmithKline, 04.05.2007 DE-GlaxoSmithKline-B0458882A

Sender diagnosis: not reported

Narrative

This case was reported by a female pharmacy assistant who was also the patient and described the occurrence of autoimmune hepatitis when she received naratriptan hydrochloride (Formigran) tablet for migraine.

Concurrent medications included Ibuprofen and rizatriptan benzoate (Maxalt).

On an unknown date in December 2006 the patient started naratriptan hydrochloride one tablet twice per day. On an unknown date in December 2006, the patient experienced autoimmune hepatitis and jaundice. She was admitted to hospital. It was noted that the patient had no history of alcohol misuse. The events were considered to be jeopardising and requiring intervention. At the time of reporting, the events were improved.

Follow-up information received on 25 April 2007:

The patient was a 49-year-old female.

Co-suspect medication included Maxalt and Ibuprofen.

The patient stated that the physician currently assumes a viral hepatitis to be involved. Physicians report states that actual diagnosis was unclear chronic hepatitis, prolonged viral adjunctive hepatitis or atypical autoimmune hepatitis.

The patient was going to visit a specialised physician for liver illness.

Follow up information received 25 April 2007:

Liver parameters are already improved.

FIRST LOOK

Country: Deutschland Type: Spontaneous report case: DE-BFARM-07008989

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes Yes Yes Yes Yes Yes

Patient

Initials: Sex: Female Birth: Age: 18 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Thrombocytopenia					Unknown
2 Transaminases increased					Unknown
3 Leucopenia					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibu-ratiopharm 400 mg (Filmdabletten)	Oral	/	-	Fever
ibuprofen		Film-coated tablet	-	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 29.03.2007 DE-RATIOPH-2007R031273

Sender diagnosis: not reported

Narrative

Reported terms: transaminases increased(Transaminases increased) leucopenia(Leukopenia) thrombocytopenia(Thrombocytopenia)

Case reference number 2007R031273 is a spontaneous case report sent by a physician which refers to a female aged 18 years.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Ibu-ratiopharm 400 mg (Filmdabletten) (IBUPROFEN), indication fever on an unknown date. The patient developed transaminases increased (Transaminases increased)(start date unknown), leucopenia (Leukopenia)(start date unknown) and thrombocytopenia (Thrombocytopenia)(start date unknown).

The following action was taken: for Ibu-ratiopharm 400 mg (Filmdabletten) (IBUPROFEN) - Unknown

The physician wanted to know by phone if the above mentioned reactions can be caused by ibuprofen. She suspected another cause, but she didn't want to speak out on it because clinical findings are still missing.

The outcome for transaminases increased (Transaminases increased) is Unknown. The outcome for leucopenia (Leukopenia) is Unknown. The outcome for thrombocytopenia (Thrombocytopenia) is Unknown.

Rechallenge information for Ibu-ratiopharm 400 mg (Filmdabletten) is not applicable. Dechallenge information for Ibu-ratiopharm 400 mg (Filmdabletten) is not applicable.

The reporter's assessment for drug Ibu-ratiopharm 400 mg (Filmdabletten) related with transaminases increased is as follows: Unknown. The reporter's assessment for drug Ibu-ratiopharm 400 mg (Filmdabletten) related with leucopenia is as follows: Unknown. The reporter's assessment for drug Ibu-ratiopharm 400 mg (Filmdabletten) related with thrombocytopenia is as follows: Unknown. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

ratiopharm GmbH, 11.04.2007 DE-RATIOPH-2007R031273

Sender diagnosis: not reported

Narrative

Reported terms: transaminases increased(Transaminases increased) leucopenia(Leukopenia) thrombocytopenia(Thrombocytopenia)

Case reference number 2007R031273 is a spontaneous case report sent by a physician which refers to a female aged 18 years.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Ibu-ratiopharm 400 mg (Filmdabletten) (IBUPROFEN), indication fever on an unknown date. The patient developed transaminases increased (Transaminases increased)(start date unknown), leucopenia (Leukopenia)(start date unknown) and thrombocytopenia (Thrombocytopenia)(start date unknown).

The following action was taken: for Ibu-ratiopharm 400 mg (Filmdabletten) (IBUPROFEN) - Unknown

The physician wanted to know by phone if the above mentioned reactions can be caused by ibuprofen. She suspected another cause, but she didn't want to speak out on it because clinical findings are still missing.

The outcome for transaminases increased (Transaminases increased) is Unknown. The outcome for leucopenia (Leukopenia) is Unknown. The outcome for thrombocytopenia (Thrombocytopenia) is Unknown.

Rechallenge information for Ibu-ratiopharm 400 mg (Filmdabletten) is not applicable. Dechallenge information for Ibu-ratiopharm 400 mg (Filmdabletten) is not applicable.

The reporter's assessment for drug Ibu-ratiopharm 400 mg (Filmdabletten) related with transaminases increased, with leucopenia and with thrombocytopenia is as follows: Unknown.

Follow-up 27-MAR-2007:

The physician reported that the reactions are not related to ibuprofen. They were caused by underlying disease. CLASSIFICATION: Adverse Event: Yes, Adverse Reaction: No, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07011367

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: 19691229	Age: 37 Year	weight (kg): 110	BMI_int: 30.47	Age fetus:
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Patientdeath

deathdate: 20070409	deathcause: Hypoxic brain damage
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Respiratory insufficiency	20070407			1 Day	fatal
2 Anaphylactic shock					fatal
3 Resuscitation	20070407		60 Minute	1 Day	fatal
4 Cardiac ischaemia					fatal
5 Oedema glottis	20070407			1 Day	fatal
6 Circulatory collapse					fatal
7 Dyspnoea					fatal
8 Epiglottic oedema	20070407			1 Day	fatal
9 Hepatic shock					fatal
10 Brain hypoxia					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Dolo-Dobendan benzocaine; cetylpyridinium chloride	Oral	7 Df dosage form/Day	20070407-20070407	Throat pain 1440 Minute
2 IBUHEXAL (NGX) ibuprofen	Oral	400 Mg milligram(s)/Day Film-coated tablet	20070407-20070407	Throat pain 1440 Minute
3 mallebrin hexetidine	Oral	5 ml millilitre(s)/Day	20070407-20070407	Throat pain 1440 Minute
4 wick medinait dextromethorphan hydrobromide; doxylamine succinate; ephedrine sulfate; paracetamol	Oral	30 ml millilitre(s)/Day	20070407-20070407	Nasopharyngitis 1440 Minute

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Apnoea			Unknown	
Larynx pain			Unknown	
Dyspnoea			Unknown	
Laryngeal oedema			Unknown	
Abdominal operation			No	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 08.05.2007 DE-SZ09-GXKR2007DE03773

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 26 Apr 2007 from the German health authority (BfArM case number: DE-BFARM-07011367) reported by a physician:

This patient with an unknown medical history developed oedema glottis with respiratory insufficiency during treatment with IbuHEXAL 400 (ibuprofen; batch no 7C6787) for sore throat. Treatment was started on 07 Apr 2007 at a dosage of one tablet. On the same day treatment with several other

drugs was started that were: Wick MediNait (dextromethorphan, paracetamol, ephedrine and doxylamine), Mallebrin Konzentrat (aluminium chloride), and Dolo Dobendan (cetylpyridine, benzocain). None of these received drugs were excluded from a causal correlation with the adverse event. Severe oedema glottis with respiratory insufficiency occurred also on 07 Apr 2007, but at first intubation efforts failed. Cardiopulmonary resuscitation was tried for more than one hour but without success so that brain death was diagnosed on 09 Apr 2007. The reporter assessed the causality of the adverse event and administration of ibuprofen as probable.

Reporter's comment:

NOVARTIS, 17.01.2008 DE-SZ09-GXKR2007DE03773

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 26 Apr 2007 from the German health authority (BfArM case number: DE-BFARM-07011367) reported by a physician:

This patient with an unknown medical history developed oedema glottis with respiratory insufficiency during treatment with IbuHEXAL 400 (ibuprofen; batch no 7C6787) for sore throat. Treatment was started on 07 Apr 2007 at a dosage of one tablet. On the same day treatment with several other drugs was started that were: Wick MediNait (dextromethorphan, paracetamol, ephedrine and doxylamine), Mallebrin Konzentrat (aluminium chloride), and Dolo Dobendan (cetylpyridine, benzocain). None of these received drugs were excluded from a causal correlation with the adverse event. Severe oedema glottis with respiratory insufficiency occurred also on 07 Apr 2007, but at first intubation efforts failed. Cardiopulmonary resuscitation was tried for more than one hour but without success so that brain death was diagnosed on 09 Apr 2007. The reporter assessed the causality of the adverse event and administration of ibuprofen as probable.

Significant follow-up information received on 14 Jan 2008 from the German health authority (BfArM case number: DE-BFARM-07011367) forwarded by the drug commission of German physicians (case number: 144627) submitting the hospital discharge report and an extract from the autopsy report leading to a change of coded adverse event terms:

This patient died from an anaphylactic shock with oedema glottis, respiratory insufficiency, resuscitation, brain hypoxia, cardiac ischaemia, and hepatic shock during treatment with IbuHEXAL 400 (ibuprofen). Concomitantly three other drugs were taken which were considered causative. An autopsy was performed on 01 May 2007: The patient died of a hypoxic brain damage followed by brain death. It was assumed that a drug-related anaphylactic shock caused the brain damage.

NOVARTIS, 17.01.2008 DE-NOVNCHHQ-GXKR2007DE03773

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 26 Apr 2007 from the German health authority (BfArM case number: DE-BFARM-07011367) reported by a physician:

This patient with an unknown medical history developed oedema glottis with respiratory insufficiency during treatment with IbuHEXAL 400 (ibuprofen; batch no 7C6787) for sore throat. Treatment was started on 07 Apr 2007 at a dosage of one tablet. On the same day treatment with several other drugs was started that were: Wick MediNait (dextromethorphan, paracetamol, ephedrine and doxylamine), Mallebrin Konzentrat (aluminium chloride), and Dolo Dobendan (cetylpyridine, benzocain). None of these received drugs were excluded from a causal correlation with the adverse event. Severe oedema glottis with respiratory insufficiency occurred also on 07 Apr 2007, but at first intubation efforts failed. Cardiopulmonary resuscitation was tried for more than one hour but without success so that brain death was diagnosed on 09 Apr 2007. The reporter assessed the causality of the adverse event and administration of ibuprofen as probable.

Significant follow-up information received on 14 Jan 2008 from the German health authority (BfArM case number: DE-BFARM-07011367) forwarded by the drug commission of German physicians (case number: 144627) submitting the hospital discharge report and an extract from the autopsy report leading to a change of coded adverse event terms:

This patient died from an anaphylactic shock with oedema glottis, respiratory insufficiency, resuscitation, brain hypoxia, cardiac ischaemia, and hepatic shock during treatment with IbuHEXAL 400 (ibuprofen). Concomitantly three other drugs were taken which were considered causative. An autopsy was performed on 01 May 2007: The patient died of a hypoxic brain damage followed by brain death. It was assumed that a drug-related anaphylactic shock caused the brain damage.

Procter & Gamble, 22.01.2008 DE-Procter&Gamble-UKO07000437

Sender diagnosis: not reported

Narrative

On 02-May-2007 a physician reported according to the patient's partner that a 36 year old male patient used MediNait Erkaltungs-Saft für die Nacht on an unspecified date for an unspecified time and experienced dyspnoea and a collapse shortly after intake. The physician was called and he found the

patient with an epiglottis swelling and being "like dead". A reanimation on-site and later in the hospital was unsuccessful as the patient could not be intubated. The patient died in the hospital probably due to an allergic shock. The patient had bought several cold medicines in the pharmacy in the morning of the same day. It was not specified whether the patient had taken also the other medicines.

The physician stated that the autopsy report was not available to him, but that this was sent to the public attorney office. He also said that an Adverse Event report form was sent by the hospital's internal medicine department to the German Authority (BfArM) and the Arzneimittelkommission der Deutschen Ärzteschaft. This again was not available to him. Medical History: No information provided. Concomitant Medication: No information provided. Outcome of the event: fatal. No further information provided.

On 01-Jun-2007 we received via the Arzneimittelkommission der deutschen Ärzteschaft (# 144627) additional information from the hospital and on 03-Jun-2007 an additional report was received directly from the hospital. It was reported that the 37 year old male patient (DOB 1969, 110 kg, 190 cm) used MediNait on 07-Apr-2007 to treat larynx oedema, apnoea and pharyngolaryngeal pain and experienced acute glottis edema. He was hospitalized for intensive care from 07-Apr-2007 to 10-Apr-2007. Following details were provided:

1. Anamnesis:

The patient had already complained of laryngeal pain and dyspnea in the morning. He therefore took DoloDobendan lozenges. The symptoms worsened in the evening. He therefore took a painkiller (tablet) and Wick MediNait. Within approximately 10 minutes of taking Wick MediNait (according to the patient's partner), this was followed by acutely progressive dyspnea, gasping/stridor, then collapse (as observed by partner). When the emergency doctor arrived on the scene, the patient required cardiopulmonary reanimation (CPR); he was apneic. During CPR, intubation was tried; laryngoscopy revealed that the glottis was swollen closed; intubation was not possible. As CPR was continued, cricothyrotomy was tried unsuccessfully; then ventilation via laryngeal mask airway was tried. As CPR was continued, the patient was rushed to the hospital. On arrival at the hospital, there was pulseless electrical activity / bradycardia; the pupils were dilated and fixed (unresponsive to light).

2. Physical Examination:

38-year old man, weight approximately 110 kg, height approximately 190 cm. Skin and visible mucous membranes pale. Larynx swollen at laryngoscopy. Thyroid not enlarged. Skin emphysema at the neck following attempted cricothyrotomy. Lungs: no breath sounds. Heart: not evaluable during CPR. Abdomen bloated, soft, no palpable masses. Sparse peristalsis. Foot pulses palpable bilat. Comatose patient; dilated, fixed pupils; no corneal reflex elicitable.

3. Hospital Course:

On admission, the patient required CPR for acute glottis edema. Difficult in-hospital endotracheal intubation, followed by emergency tracheotomy. Thereafter, the patient was hemodynamically stable, but there were transient ST segment elevations suggestive of hypoxia-induced myocardial infarction; significantly elevated liver enzymes from shock liver. No brainstem reflexes; brain death diagnosed on 09-Apr-2007, based on a flat EEG. After an in-depth discussion, his partner eventually agreed that the patient should be available for organ donation. On April 10, both kidneys were removed. Thereafter, intensive care unit therapy was stopped.

As this was not a natural death, the patient's body was autopsied by the University of Cologne Department of Forensic Medicine at the request of the DA's office.

The medications taken by the patient were handed over to the police. A suspected serious side effect was reported to the AKdÄ (Drug Commission of the German Medical Association) and BfArM (German licensing agency for medicinal products and medical devices).

4. Diagnoses:

Death. CPR on 07-Apr-2007 for acute glottis edema with hypoxia; Resuscitation efforts over 60 minutes in total, Out-of-hospital intubation was tried unsuccessfully, as was cricothyrotomy; Ventilation via laryngeal mask airway was also tried; Emergency ventilation via Abbocath in cricothyrotomy position; Successful intubation 60 minutes after index event; Emergency tracheotomy (open) on 07-Apr-2007.

Complications: Shock, ARDS (acute respiratory distress syndrome), Myocardial resuscitation injury with ST segment elevation, Suspected disseminated intravascular coagulation, Suspected nonocclusive mesenteric ischemia (lactate 92 mg/dL), Hypoxic brain injury: Computerized cranial tomography (CCT) on 09-Apr-2007 (Generalized hypoxic brain injury; congestion of blood in the subarachnoid space), Brain death diagnosed on 09-Apr-2007 at 8:00 PM.

Treatment: Emergency tracheotomy on 07-Apr-2007, (Mechanical) ventilation on 07 to 10-Apr-2007, PiCCO controlled catecholamine and volume therapy, Cooling over 24 hours

5. Lab data:

The report also included serial laboratory results.

Medical History: Larynx oedema, Larynx pain, Apnoea. Concomitant Medications: Mallebrin (hexetidine), Dolo-Dobendan (cetylpyridinium chloride, benzocaine), Ibuthexal (ibuprofen). Outcome of the event: fatal. No further information provided.

ADDITIONAL INFORMATION:

On 10-Jan-2008 an anonymised excerpt from the preliminary expert report on the autopsy report dated 01-May-2007 was received via BfArM:

The autopsy findings taken as a whole are consistent with the development of hypoxic brain damage in the patient a few days before his death, with subsequent brain death.

According to the documents available it is suspected that hypoxic brain damage in the patient (â€¦ is attributable) to a drug reaction in the form of anaphylactic shock. The autopsy findings taken as a whole are consistent with this medical statement.

The mucosal swelling in the area of the epiglottis observed at the time of autopsy and a constriction of the airway possibly caused by this swelling is basically consistent with an allergic or even toxic drug reaction. In this case, from the forensic viewpoint, there is basically also the possibility of incorrect regulation of the water and electrolyte balance and also a local exertion of pressure by the ventilation tube associated with assisted ventilation and circulatory activity in the period between the onset of brain death and the removal of organs.

Examination of the heart showed evidence from the macroscopic findings of a myocardial infarction occurring at least several hours previously based on a fresh occlusion of the circumflex branch of the left coronary artery. The large sagittal sinus of the cranial cavity is also occluded due to formation of an extensive blood clot. Both the development of the blood clot and also the subsequent occurrence of death of myocardial cells are, according to current knowledge, undoubtedly due to circulatory dysregulation after occurrence of brain death in Mr. R.

Neither during the removal of the kidneys in hospital in the context of an organ donation (â€¦) nor during forensic post mortem examination of the cadaver of Mr R were there any signs of external force applied to the body of Mr R which would have been able to explain the death or to have had any effect on the fatal course of events. The rib fractures observed during post mortem examination, located near the sternum, are basically due to the resuscitation measures, with performance of cardiac massage, carried out as reported by the emergency doctor.

(End of excerpt of autopsy report)

By order of the Public Prosecutor's office, fine tissue investigations were then performed on the organ samples reserved during autopsy. The findings obtained in this examination are consistent with the comments in the autopsy report.

The results of the chemical and toxicological investigations which were also ordered are currently still awaited.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07010076

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19780120	Age: 29 Year	weight (kg): 65	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug overdose deliberate self-inflicted	20070402				recovered/resolved
2 Somnolence	20070402				recovered/resolved
3 Tachycardia	20070402				recovered/resolved
4 Confusion	20070402				recovered/resolved
5 Bowel sounds decreased	20070402				recovered/resolved
6 Abnormal ECG	20070402				recovered/resolved
7 CK increased	20070402				recovered/resolved
8 Polyuria	20070402				recovered/resolved
9 Renal failure	20070402				recovered/resolved
10 Vigilance decreased	20070402				recovered/resolved
11 Hepatic enzyme increased	20070402				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMISULPRIDE	Oral	/	20070402-20070402	
amisulpride			1 Day	
2 ATOSIL	Oral	/	20070402-20070402	
isopromethazine hydrochloride			1 Day	
3 ibuprofen	Oral	/	20070402-20070402	
ibuprofen			1 Day	
4 TREVILOR RETARD	Oral	/	20070402-20070402	
venlafaxine hydrochloride		Prolonged-release capsule	1 Day	
5 ZIPRASIDONE	Oral	/	20070402-20070402	
ziprasidone			1 Day	

Primary source

Qualification: Other Health Professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Wyeth-Pharma GmbH, 11.04.2007 DEWYE320203APR07

Sender diagnosis: not reported

Narrative

Information was received from a clinician via the Poison Information Center in Mainz regarding a 29-year-old female patient who took a drug overdose deliberately self-inflicted and experienced confusion, tachycardia, decreased bowel sounds and somnolence.

MEDICAL HISTORY:

Relevant medical history was not provided.

PRODUCT DETAILS:

The patient was suspected of having taken 120 capsules of Trevilor retard (venlafaxine hydrochloride capsule, extended release) (overdose amount 12750 mg), 50 tablets of Amisulpride (overdose amount 20000 mg), 10 tablets of Atosil (isopromethazine hydrochloride) (overdose amount 250 mg) and 10 tablets of Ibuprofen (overdose amount 4000 mg) (oral) on 02-Apr-2007.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient took a drug overdose deliberately self-inflicted (intentional overdose) on 02-Apr-2007 and experienced somnolence (somnolence), confusion (confusion state), bowel sounds decreased (bowel sounds abnormal) and tachycardia (tachycardia). The overdose was classified as a

suicide attempt. It is unknown if the patient was receiving daily therapy with Trevilor. The Poison Information Center recommended to hospitalize (ICU) the patient, to monitor her and to perform a blood gas analysis. Her laboratory parameters concerning CK-values, kidney and liver parameters should be controlled. It was further advised to administer charcoal and to induce diarrhoea. They should administer sodium bicarbonate as needed and they should place a stomach tube if necessary. In addition a intestine stimulation should be done. Her treatment must include a stomach protection. The patient was advised to consult a psychiatrist. The outcome is unknown.

TEST RESULTS:

Physical examination (results: bowel sounds present slightly) and heart rate (results: Tachycardia 130 -< 140) were done on 02-Apr-2007.

The outcome was unknown.

Sophia Venzke Iris Eierhoff

Reporter's comment:

Wyeth-Pharma GmbH, 23.05.2007 DEWYE320203APR07

Sender diagnosis: not reported

Narrative

Additional information received regarding therapy details and the outcome. Additionally new events were reported.

Information was received from a clinician via the Poison Information Center in Mainz regarding a 29-year-old female patient who took a drug overdose deliberately self-inflicted and experienced bowel sounds decreased, confusion, tachycardia, somnolence, renal failure, hepatic enzyme increased, vigilance decreased, polyuria, abnormal ECG and CK increased.

MEDICAL HISTORY:

Relevant medical history was not provided.

PRODUCT DETAILS:

The patient was suspected of having taken 120 capsules of Trevilor retard (venlafaxine hydrochloride capsule, extended release) (overdose amount 12750 mg), 50 tablets of Amisulpride (overdose amount 20000 mg), 10 tablets of Atosil (isopromethazine hydrochloride) (overdose amount 250 mg), 10 tablets of Ibuprofen (overdose amount 4000 mg) (oral) and an unknown overdose amount of Zeldox (ziprasidone) on 02-Apr-2007.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient took a drug overdose deliberately self-inflicted (intentional overdose) on 02-Apr-2007 and experienced somnolence (somnolence), confusion (confusion state), bowel sounds decreased (bowel sounds abnormal), tachycardia (tachycardia), renal failure (renal failure), raised hepatic enzyme increased (hepatic enzyme increased), vigilance decreased (disturbance in attention), polyuria (polyuria), abnormal ECG (electrocardiogram abnormal) and CK increased (blood creatine phosphokinase increased). The overdose was classified as a suicide attempt. It is unknown if the patient was receiving daily therapy with Trevilor. As the Poison Information Center recommended the patient was hospitalized (ICU), monitored and her laboratory parameters concerning CK-values, kidney and liver parameters were controlled. Charcoal was administered and diarrhoea was induced as advised. Additionally symptomatic therapy was performed and an infusion were administered. The patient consulted a psychiatrist as advised. The patient recovered.

TEST RESULTS:

Heart rate (results: Tachycardia 130 -< 140) and physical examination (results: bowel sounds present slightly) were done on 02-Apr-2007. Additionally, blood creatine phosphokinase (results: CK max 1100 U/l) was done on an unspecified date.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07072644

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19490316	Age: 58 Year	weight (kg): 60	BMI_int: 21.26	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Asthenia	20070618				recovering/resolving
2 Faeces discoloured	20070618				recovered/resolved
3 SGOT increased	20070618				recovered/resolved
4 SGPT increased	20070618				recovered/resolved
5 Gamma GT increased	20070618				recovering/resolving
6 Alkaline phosphatase increased	20070618				recovering/resolving
7 Creatinine increased					recovering/resolving
8 Bilirubinaemia	20070618				recovering/resolving
9 Liver function tests abnormal					Unknown
10 Cholestatic icterus	20070618				recovering/resolving
11 Vomiting					recovering/resolving
12 Pruritus	20070618				recovering/resolving
13 Nausea	20070618				recovering/resolving
14 Hepatitis toxic drug-induced					recovering/resolving
15 Hepatic failure	20070618				recovering/resolving
16 Diarrhoea	20070618				recovering/resolving
17 Impaired renal function	20070618		15 Day		recovering/resolving
18 Hepatic failure	20070618		15 Day		not recovered/not resolved
19 C-reactive protein increased	20070618				recovering/resolving
20 Hypokalaemia	20070618				recovering/resolving
21 Quick's value decreased	20070618				recovering/resolving
22 Leukocytosis	20070618				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-20070617	Ache
diclofenac			-	
2 HERCEPTIN	Intravenous (not otherwis	330 Mg milligram(s)/Day	200402-20070605	Breast cancer metastatic
trastuzumab			-	
3 ibuhexal	Oral	/	-20070617	Pain NOS
ibuprofen			-	
4 ZOMETA	Intravenous (not otherwis	4 Mg milligram(s)/Day	200507-20070605	Bone metastases
zoledronic acid			-	

Drugs (concomitant)

5	aluminium hydroxide gel	Oral	124.9 Mg milligram(s)/	-
	aluminium hydroxide; aluminium hydroxide gel			-
6	magnesium carbonate	Oral	83.8 Mg milligram(s)/	-
	magnesium carbonate			-
7	METAMIZOLE	Oral	/	-
	metamizole sodium			-
8	METOCLOPRAMID	Oral	/	-
	metoclopramide hydrochloride			-
9	OCTACAINE	Oral	5 Mg milligram(s)/	-
	octacaine			-
10	tepilta	Oral	/	-
	aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide; oxetacaine			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lymph node excision	200703		Unknown	partial lymphadenectomy right clavicular and left cervical
Breast cancer	200211		Yes	breast cancer left diagnosed in Nov 2002 with ablatio in Sep 2003; breast cancer right dignosed in M
Blood transfusion	20070612		Unknown	
Pruritus			Unknown	
Drug dependence			Unknown	
Metastatic carcinoma			Unknown	
Radiation therapy	200703		Unknown	Postoperative, with 50.4 Gy
Bone metastases	200507		Yes	Thoracal Spine Th12 and left foot
Lymph node metastases	200703		Unknown	
Radiation therapy	200507		Unknown	
Breast operation			Unknown	
Chemotherapy			Unknown	
Anaemia			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		
Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.08.2007 DE-NOVNCHHQ-GXKR2007DE06957

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 17 Aug 2007 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of metastatic breast cancer with breast ablation, polychemotherapy and radiation developed hepatic failure with renal impairment during treatment with IbuHEXAL (ibuprofen) and diclofenac (MAH unknown) for pain. Both drugs were taken irregularly at an unknown dosage for several months until 17 Jun 2007. Concomitantly the patient received several other drugs, of which Trastuzumab (herceptin) and zoledronic acid were also considered causative. The reporter stated that the patient took ibuprofen and diclofenac pain-dependent on her own initiative. The adverse events icterus with nausea and vomiting, pruritus and diarrhoea occurred on 18 Jun 2007. The patient was hospitalised on 20 Jun 2007 with abnormal laboratory values so as leucocytosis, increased CRP, decreased Quick's value, hypokalaemia and abnormal liver values. A liver biopsy was performed and revealed a drug-induced toxic hepatic failure. The patient was treated with amoxicillin/clavulanic acid, ondansetron, prednisolon, pantoprazol and received substitution of glucose, vitamin K, iron, and sodiumchloride. When the patient was released from hospital on 05 Jul 2007 the condition was improving. The reporter assessed the causality of the adverse events and administration of ibuprofen and diclofenac as possible.

NOVARTIS, 23.08.2007 DE-NOVNCHHQ-GXKR2007DE06957

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 17 Aug 2007 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of metastatic breast cancer with breast ablation, polychemotherapy and radiation developed hepatic failure with renal impairment during treatment with IbuHEXAL (ibuprofen) and diclofenac (MAH unknown) for pain. Both drugs were taken irregularly at an unknown dosage for several months until 17 Jun 2007. Concomitantly the patient received several other drugs, of which Trastuzumab (herceptin) and zoledronic acid were also considered causative. The reporter stated that the patient took ibuprofen and diclofenac pain-dependent on her own initiative. The adverse events icterus with nausea and vomiting, pruritus and diarrhoea occurred on 18 Jun 2007. The patient was hospitalised on 20 Jun 2007 with abnormal laboratory values so as leucocytosis, increased CRP, decreased Quick's value, hypokalaemia and abnormal liver values. A liver biopsy was performed and revealed a drug-induced toxic hepatic failure. The patient was treated with amoxicillin/clavulanic acid, ondansetron, prednisolon, pantoprazol and received substitution of glucose, vitamin K, iron, and sodiumchloride. When the patient was released from hospital on 05 Jul 2007 the condition was improving. The reporter assessed the causality of the adverse events and administration of ibuprofen and diclofenac as possible.

NOVARTIS, 23.08.2007 DE-SZ09-GXKR2007DE06957

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 17 Aug 2007 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of metastatic breast cancer with breast ablation, polychemotherapy and radiation developed hepatic failure with renal impairment during treatment with IbuHEXAL (ibuprofen) and diclofenac (MAH unknown) for pain. Both drugs were taken irregularly at an unknown dosage for several months until 17 Jun 2007. Concomitantly the patient received several other drugs, of which Trastuzumab (herceptin) and zoledronic acid were also considered causative. The reporter stated that the patient took ibuprofen and diclofenac pain-dependent on her own initiative. The adverse events icterus with nausea and vomiting, pruritus and diarrhoea occurred on 18 Jun 2007. The patient was hospitalised on 20 Jun 2007 with abnormal laboratory values so as leucocytosis, increased CRP, decreased Quick's value, hypokalaemia and abnormal liver values. A liver biopsy was performed and revealed a drug-induced toxic hepatic failure. The patient was treated with amoxicillin/clavulanic acid, ondansetron, prednisolon, pantoprazol and received substitution of glucose, vitamin K, iron, and sodiumchloride. When the patient was released from hospital on 05 Jul 2007 the condition was improving. The reporter assessed the causality of the adverse events and administration of ibuprofen and diclofenac as possible.

NOVARTIS, 09.01.2008 DE-NOVNCHHQ-GXKR2008DE00464

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 03 Jan 2008 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of breast cancer on the left side diagnosed in Nov 2002 and on the right side diagnosed in Mar 2004 with bilateral ablation, chemotherapeutic combination and radiotherapy for bone metastases in Jul 2005, lymph node metastases with partial metastectomy and radiation in Mar 2007, and repeated blood transfusion for anaemia developed hepatocellular damage with hepatic failure, icterus, discoloured stools, nausea, vomiting, diarrhoea, pruritus, asthenia, hyperbilirubinaemia, transaminases increased, gamma GT increased, alkaline phosphatase increased, prothrombin level decreased, blood potassium decreased, leucocytosis, CRP increased, creatinine increased and renal function abnormal during treatment with IbuHEXAL (ibuprofen) for pain taken at an unknown dosage occasionally for several months. Concomitantly the patient received several other drugs, of which trastuzumab, zoledronate and diclofenac were considered causative as well. The adverse event occurred on 18 Jun 2007. Two days later the patient was admitted to hospital, 13 days after she had received the last infusion of trastuzumab and zoledronate. The patient was released from hospital 2 ½ weeks later with markedly improved condition. The reporter provided no causality assessment.

NOVARTIS, 09.01.2008 DE-SZ09-GXKR2008DE00464

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 03 Jan 2008 from the German health authority (BfArM case number: DE-BFARM-07072644), reported by a physician:

This patient with a medical history of breast cancer on the left side diagnosed in Nov 2002 and on the right side diagnosed in Mar 2004 with bilateral ablation, chemotherapeutic combination and radiotherapy for bone metastases in Jul 2005, lymph node metastases with partial metastasectomy and radiation in Mar 2007, and repeated blood transfusion for anaemia developed hepatocellular damage with hepatic failure, icterus, discoloured stools, nausea, vomiting, diarrhoea, pruritus, asthenia, hyperbilirubinaemia, transaminases increased, gamma GT increased, alkaline phosphatase increased, prothrombin level decreased, blood potassium decreased, leucocytosis, CRP increased, creatinine increased and renal function abnormal during treatment with IbuHEXAL (ibuprofen) for pain taken at an unknown dosage occasionally for several months. Concomitantly the patient received several other drugs, of which trastuzumab, zoledronate and diclofenac were considered causative as well. The adverse event occurred on 18 Jun 2007. Two days later the patient was admitted to hospital, 13 days after she had received the last infusion of trastuzumab and zoledronate. The patient was released from hospital 2 ½ weeks later with markedly improved condition. The reporter provided no causality assessment.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07086224

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19550801	Age: 50 Year	weight (kg): 125	BMI_int: 47.05	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hypotension	20060309	20060406	29 Day		not recovered/not resolved
2 Hepatitis toxic	20060309	20060406	29 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Cordinate plus hydrochlorothiazide; valsartan	Oral	1 Df dosage form/Day	20060210-20060317 36 Day	Hypertension arterial
2 Delix ramipril	Oral	2.5 Mg milligram(s)/Day	20060115-20060209 26 Day	Hypertension arterial
3 Ibufexal ibuprofen	Oral	1600 Mg milligram(s)/Day	20060106-20060320 74 Day	Pain
4 Katadolon flupirtine maleate	Oral	300 Mg milligram(s)/Day	20060106-20060321 75 Day	Pain
5 NORVASC amlodipine besilate	Oral	5 Mg milligram(s)/Day	20050101-20060319 14 Month	Hypertension arterial
6 Torem 10 torasemide	Oral	10 Mg milligram(s)/Day	20060101- -	Edema peripheral

Drugs (concomitant)

7 l-thyroxin levothyroxine sodium	Oral	25 Mg milligram(s)/Day	19980101- -	Hypothyroidism
8 Pantozol pantoprazole sodium	Oral	80 Mg milligram(s)/Day	20040101- -	Prevention
9 xalatan latanoprost	Ophthalmic	1 Df dosage form/Day	20010101- -	Glaucoma

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Adipositas per magna			Yes	
Hypertension arterial	20000101		Yes	
Glaucoma	20010101		Yes	
Hypothyroidism	19970101		Yes	
Psoriasis	19950101		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20060313	unknown	24.39	
Bilirubin direct	20060313	unknown	49.5	
Blood bilirubin total	20060313	unknown	61.3	
GGT	20060313	unknown	6.31	

Alkaline phosphatase	20060313	unknown	3.27
Alanine aminotransferase	20060313	unknown	30.93

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07072736

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: 19450705	Age: 61 Year	weight (kg): 70	BMI_int: 22.86	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug interaction					Unknown
2 Quick's value decreased	20070615				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	/	-	Drug use for unknown indication
ibuprofen			-	indication
2 marcumar	Unknown	/	-	Drug use for unknown indication
phenprocoumon			-	indication

Drugs (concomitant)

3 ezetrol	/	-	
ezetimibe		-	
4 moxonidine	/	-	
moxonidine		-	
5 novaminsulfon	/	-	
metamizole		-	
6 tozem	/	-	
torasemide		-	
7 voltaren dispers	/	-	Rheumatism
diclofenac sodium	Tablet	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Rheumatism			Unknown	
Paget-von Schroetter syndrome			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Quick's test		RESULT: DECREASED	UNITS	NO UNITS AND NORMAL RANGE PROVIDED
Quick's test		35 UNIT %	NORMAL RANGE	NORMAL RANGE NOT PROVIDED.
Quick's test		7 TEST REPORTED	UNITS	QUICK'S TEST (DECREASED FROM +
INR		2.28	NORMAL RANGE	RANGE AND UNIT NOT PROVIDED
INR		RESULT: > 6	UNITS	DECREASED FROM 2.28 TO > 6

Narratives (nur bei elektronischem Eingang)

ROCHE, 13.08.2007 DE-ROCHE-505673

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE WAS REPORTED BY A PHYSICIAN AND CONCERNS A 61 YEAR OLD MALE PATIENT WHO EXPERIENCED

MASSIVE DECREASED QUICK'S VALUE FROM 35% TO 7% FOLLOWING A DRUG INTERACTION BETWEEN PHENPROCOUMON (MARCUMAR) AND IBUPROFEN (IBUPROFEN) TAKEN FOR UNKNOWN INDICATIONS.

HE HAD A MEDICAL HISTORY OF PAGET VON SCHROETTER SYNDROME AND RHEUMATISM . ALSO, HE WAS RECEIVING VARIOUS MEDICATIONS CONCOMITANTLY.

THE PATIENT COMMENCED PHENPROCOUMON AND IBUPROFEN. ON 15 JUNE 2007, THE PATIENT DEVELOPED DRUG INTERACTION BETWEEN PHENPROCOUMON AND IBUPROFEN WITH SEVERE DECREASE IN QUICK'S VALUE. IT WAS REPORTED THAT TREATMENT WITH PHENPROCOUMON HAD BEEN CHANGED AND NSAR HAD BEEN DISCONTINUED. TESTS CONDUCTED ON UNSPECIFIED DATES REVEALED QUICK'S VALUE WAS AT 35% AND 7%. ASLO, INR WAS REPORTED AT 2.28 AND THEN INCREASED FROM 2.28 TO > 6. IT WAS FURTHER ADDED THAT THE QUICK'S VALUE DECREASED NOTEDLY AFTER THE DOSE REDUCTION OF CORTISONE AND THE EVENT RESOLVED AFTER TEN DAYS. THE PHYSICIAN ASSESSED THE EVENT AS MEDICALLY SIGNIFICANT AND SUSPECTED A CORRELATION WITH DICLOFENAC.

THE PHYSICIAN ASSESSED THE EVENT MASSIVE DECREASED QUICK'S VALUE FROM 35% TO 7% TO BE CERTAINLY RELATED TO PHENPROCOUMON AND TO THE DRUG INTERACTION BETWEEN PHENPROCOUMON AND IBUPROFEN. NO FURTHER INFORMATION WAS AVAILABLE.

UPDATE INFORMATION WAS RECEIVED AND THE FOLLOWING DATA WERE ADDED TO THE CASE: CASE PRIORITY CHANGED FROM LOW TO HIGH, PATIENT INITIALS, GENDER, DATE OF BIRTH, AGE, HEIGHT AND WEIGHT; REPORTER'S FULL NAME, ADDRESS, PHONE AND FAX NUMBER; NON-SUSPECTS; EVENT TERM CHANGED FROM DECREASED QUICK'S VALUE TO MASSIVE DECREASED QUICK'S VALUE FROM 35% TO 7%. MEDICALLY SIGNIFICANT EVENT, OUTCOME CHANGED FROM UNKNOWN TO RESOLVED, RESOLVED DATE; MEDICAL HISTORY, LAB TEST, DETAILED CLINICAL COURSE.

*** DATA ELEMENTS THAT WERE TRUNCATED NOW FOLLOW ***

<ichicsrmessageheader.patient.drug(medicinalproduct=VOLTAREN DISPERS)>

<drugdosagetext>

IT WAS REPORTED THAT ON 10 JUNE 2007, THE PATIENT INCREASED INTAKE OF VOLTAREN DISPERS 50 MG, 3 DOSE FORM DAILY

</drugdosagetext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07103068

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Proteinuria					Unknown
2 Still's disease					recovered/resolved
3 Acute liver failure					recovered/resolved
4 Transaminases increased					Unknown
5 Night sweat					Unknown
6 Myalgia					Unknown
7 Lymphadenopathy					Unknown
8 Lack of drug effect					Unknown
9 Leukocytosis					
10 Anaemia hypochromic					Unknown
11 Arthralgia					recovered/resolved
12 Coagulopathy					recovered/resolved
13 CRP increased					Unknown
14 Exanthem					recovered/resolved
15 Fatigue					recovered/resolved
16 Hyponatraemia					Unknown
17 Hypocalcaemia					Unknown
18 Hypoalbuminaemia					recovered/resolved
19 Hyperbilirubinaemia					recovered/resolved
20 Fever					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	1800 Mg milligram(s)/Day	-	Fever
ibuprofen			-	
2 Paracetamol	Unknown	4000 Mg milligram(s)/Day	-	Fever
paracetamol			-	

Drugs (concomitant)

3 Ceftriaxon	Unknown	/	-	Drug use for unknown indication
ceftriaxone sodium			-	
4 Clarithromycin	Unknown	/	-	Drug use for unknown indication
clarithromycin			-	
5 methylprednisolone	Intravenous (not otherwis	250 Mg milligram(s)/	-	Still's disease
methylprednisolone		Solution for infusion	-	
6 methylprednisolone	Intravenous (not otherwis	250 Mg milligram(s)/	-	Still's disease
methylprednisolone		Solution for infusion	3 Day	
7 Piperacillin/Tazobactam	Unknown	/	-	Drug use for unknown indication
piperacillin; tazobactam			-	
8 prednisolone	Oral	400 Mg milligram(s)/Day	-	Still's disease
prednisolone		Tablet	-	
9 prednisolone	Oral	12 Mg milligram(s)/Day	-	Still's disease
prednisolone		Tablet	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Still's disease			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Linde B, Oelzner P, Katenkamp K, Hein G, Wolf G. [Fulminate liver failure in a 39-year-old female patient with leukocytosis, unclear Fever, and arthralgic pain.] Med Klin (Munich) 2007; 102: 846-51	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
ALT increased		1.03	umol/l.s	
AST increased		1.06	umol/l.s	
Bilirubin		313	umol/L	
C-reactive protein increased		185	mg/l	
ESR		93	mm/h	
Ferritin high		1740	Âµg/l	
Gamma-glutamyltransferase increased		2.73	Âµmol/l.s	
INR		2.05	NA	
LDH increased		12.48	umol/l.s	
Leukocytosis		23.6	Gpt/l	
Quick's test		29	%	
Sonogram		mild hepatosplenomegaly		
Liver biopsy		subacute hepatitis with necrosis + an cholangitis		
Albumin		16	G/L	
Alkaline phosphatase		8.57	MCMOL/L	
Bilirubin direct		228.5	MCMOL/L	
Blood cholinesterase		41	MCMOL/L	
C-reactive protein		28.3	MG/L	
Calcium		1.9	MMOL/L	
Fibrinogen		1.0	G/L	
Lactate dehydrogenase		15.54	MCMOL/L	
Serum ferritin		15405	MCG/L	
SGOT		15.93	MCMOL/L	
SGPT		26.21	MCMOL/L	
Sodium		134	MMOL/L	
Ammonia		133	MCMOL/L	
Protein total		34	G/L	

Narratives (nur bei elektronischem Eingang)

Krewel Meuselbach GmbH, 31.10.2007 DE-KREWEL-000414

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of

corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol.

could have also played a role in the development of the liver failure. A causality with the concomitant drugs cannot be excluded. 4 g paracetamol is a subtoxic daily dose that could be toxic in special patient groups e.g. diseases like Still's disease. Liver failure is listed in cases of high doses for Captin our paracetamol containing medicinal products. The patient recovered after treatment. Further measures are not necessary.

Reporter's comment:

AbZ Pharma GmbH, 02.11.2007 DE-ABZ-2007A106056

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with Paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of Paracetamol. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

to diverse antibiotic treatment. But the opinion of the authors is endorsed that the most probable cause for hepatic failure in this case was the underlying Still disease. Actions regarding the wording of the RSI is not necessary.

Reporter's comment:

ratiopharm GmbH, 02.11.2007 DE-RATIOPH-2007R105989

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with Paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of Paracetamol. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

to diverse antibiotic treatment. But the opinion of the authors is endorsed that the most probable cause for hepatic failure in this case was the underlying Still disease. Actions regarding the wording of the RSI is not necessary.

Reporter's comment:

Dr. R. Pflieger Chemische Fabrik GmbH, 05.11.2007 DE-PFLEGER-000042

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol and ibuprofen was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: Underlying Still's disease, REACTION ABATE AFTER STOPPING DRUG: Not applicable.

CAUSALITY ASSESSMENT for ibuprofen: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: unlikely, NOTE: Other risk factor: Underlying Still's disease, REACTION ABATE AFTER STOPPING DRUG: Not applicable.

Molimin Arzneimittel GmbH, 06.11.2007 DE-MOLIMIN-000011

Sender diagnosis: not reported

Narrative

A 39-year-old female patient developed fulminate liver failure following treatment with paracetamol 1000 mg four times daily. The patient presented with a nonitching macular-papular exanthema on both thighs with spreading to the trunk. She complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. Paracetamol and ibuprofen was started and liver function worsened and an icterus developed. The diagnosis of an underlying Still's disease was made. Liver biopsy revealed subacute hepatitis with necrosis and an accompanying cholangitis. Liver function improved on administration of corrective treatment. The authors state that liver failure was probably induced by a reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol.

CAUSALITY ASSESSMENT for paracetamol: Temporal Relationship: plausible, Pharmacological Relationship: plausible, Observed reaction is a Sign/Symptom of Underlying Disease: probable, History of Observed Event without drug involvement: no, Other Risk Factors: probable, Sufficiency of Documentation: sufficient, OVERALL CAUSALITY ASSESSMENT: possible, NOTE: Other risk factor: Underlying Still's disease, REACTION ABATE AFTER STOPPING DRUG: Not applicable.

Still's disease must be considered as part of the differential diagnosis of acute liver failure.

Steiner & Co. Deutsche Arzneimittelgesellschaft mbH & Co., 08.11.2007 DE-STEINERCO-000097

Sender diagnosis: not reported

Narrative

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 °C, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 µg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 µmol/l.s, aspartate aminotransferase 1.06 µmol/l.s, gamma-glutamyltransferase 2.73 µmol/l.s, and lactate dehydrogenase 12.48 µmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 × 1,000 mg/day) and ibuprofen (3 × 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

MAHs assessment:

classification: serious; causality assessment: possibly related; labeled for hepatitis / liver failure; the cholangitis is regarded as secondary to the significant liver damage.

As Azur and Azur compositum contain 350 mg, 450 mg and 600 mg of paracetamol, respectively, it is not possible to take a single dose of 1000 mg without parting the medicinal product. With regard to the reduced liver function caused by the underlying Still's disease and with a view to concomitant drugs one cannot clearly appoint a causal relationship of paracetamol nor either rule out. However, paracetamol is known to cause liver failure following high dose/long term administration. A corresponding hint is given in the SPCs of Azur and Azur compositum. Furthermore, it is mentioned not to use Azur and Azur compositum in cases of hepatocellular insufficiency. Therefore, there is no change of the safety profile, no measures to be taken.<dbtext>To_Char_NARRATIVE_2</dbtext>

Steiner & Co. Deutsche Arzneimittelgesellschaft mbH & Co. 09.11.2007 DE-STEINERCO-000097

Sender diagnosis: not reported

Narrative

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 °C, and night sweats. An outpatient treatment with nonsteroidal anti-inflammatories, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 µg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 µmol/l.s, aspartate aminotransferase 1.06 µmol/l.s, gamma-glutamyltransferase 2.73 µmol/l.s, and lactate dehydrogenase 12.48 µmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 × 1,000 mg/day) and ibuprofen (3 × 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

MAHs assessment:

classification: serious; causality assessment: possibly related; labeled for hepatitis / liver failure; the cholangitis is regarded as secondary to the significant liver damage.

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Follow-up report created on 09-Nov-2007 following an error message of the EMEA because of a wrong entry in the field "date received by manufacturer". In fact, no new information was received today, this update is due only because of technical reporting errors.<dbtext>

To_Char_NARRATIVE_2</dbtext>

Steiner & Co. Deutsche Arzneimittelgesellschaft mbH & Co. 13.11.2007 DE-STEINERCO-000097

Sender diagnosis: not reported

Narrative

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 °C, and night sweats. An outpatient treatment with nonsteroidal anti-inflammatories, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 µg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 µmol/l.s, aspartate aminotransferase 1.06 µmol/l.s, gamma-glutamyltransferase 2.73 µmol/l.s, and lactate dehydrogenase 12.48 µmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 × 1,000 mg/day) and ibuprofen (3 × 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

MAHs assessment:

classification: serious; causality assessment: possibly related; labeled for hepatitis / liver failure; the cholangitis is regarded as secondary to the significant liver damage.

As Azur and Azur compositum contain 350 mg, 450 mg and 600 mg of paracetamol, respectively, it is not possible to take a single dose of 1000 mg without parting the medicinal product. With regard to the reduced liver function caused by the underlying Still's disease and with a view to concomitant drugs one cannot clearly appoint a causal relationship of paracetamol nor either rule out. However, paracetamol is known to cause liver failure following high dose/long term administration. A corresponding hint is given in the SPCs of Azur and Azur compositum. Furthermore, it is mentioned not to use Azur and Azur compositum in cases of hepatocellular insufficiency. Therefore, there is no change of the safety profile, no measures to be taken.

Follow-up report created on 09-Nov-2007 following an error message of the EMEA because of a wrong entry in the field "date received by manufacturer". In fact, no new information was received today, this update is due only because of technical reporting errors.

Follow-up report created on 12-Nov-2007 following a 2nd error message of the EMEA despite correction of the field "date received by manufacturer". No new information was received until to date. Thus, this update is again due only because of a technical reporting error.<dbtext>

To_Char_NARRATIVE_2</dbtext>

Boehringer Ingelheim GmbH, Germany, 22.11.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-06834GD

Sender diagnosis: not reported

Narrative

A female patient with Still's disease is reported, in whom symptomatic antiphlogistic treatment with ibuprofen and paracetamol was administered in addition to antibiotic therapy. The patient experienced fulminate liver failure.

In the opinion of the authors, the most likely cause of the liver failure was hepatotoxic adverse drug reactions (especially due to paracetamol) in combination with a pre-existing liver damage associated with the concomitant Still's disease (Still's disease was judged to be the main factor). An involvement of the antibiotic therapy in the development of the liver failure was also considered by the authors.

The 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degC, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and oral penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 mcg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 mcmol/l.s, aspartate aminotransferase 1.06 mcmol/l.s, gamma-glutamyltransferase 2.73 mcmol/l.s, and lactate dehydrogenase 12.48 mcmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued (intravenous ceftriaxone was started, later switched to piperacillin/tazobactam and finally to clarithromycin) without any improvement. In addition, acetaminophen (up to 4 x 1,000 mg/day) and ibuprofen (up to 3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital and treated with acetylcysteine for suspected paracetamol toxicity. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. 16 Months after the incident, the patient was without symptoms under prednisone 3 mg/day; the liver function was normal.

The authors state: 'A mild to moderate increase in liver enzymes is often found as part of this disease (Still's disease). Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen.'

This is a summary of an article written in German. An English translation has been ordered.

Boehringer Ingelheim GmbH, Germany, 06.12.2007 DE-Boehringer Ingelheim GmbH, Germany-2007-DE-06834GD

Sender diagnosis: not reported

Narrative

A female patient with Still's disease is reported, in whom symptomatic antiphlogistic treatment with ibuprofen and paracetamol was administered in addition to antibiotic therapy. The patient experienced fulminate liver failure.

In the opinion of the authors, the most likely cause of the liver failure was hepatotoxic adverse drug reactions (especially due to paracetamol) in combination with a pre-existing liver damage associated with the concomitant Still's disease (Still's disease was judged to be the main factor). An

involvement of the antibiotic therapy in the development of the liver failure was also considered by the authors.

The 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degC, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and oral penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 mcg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 mcmol/l.s, aspartate aminotransferase 1.06 mcmol/l.s, gamma-glutamyltransferase 2.73 mcmol/l.s, and lactate dehydrogenase 12.48 mcmol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued (intravenous ceftriaxon was started, later switched to piperacillin/tazobactam and finally to clarithromycin) without any improvement. In addition, acetaminophen (up to 4 x 1,000 mg/day) and ibuprofen (up to 3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital and treated with acetylcysteine for suspected paracetamol toxicity. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. 16 Months after the incident, the patient was without symptoms under prednisone 3 mg/day; the liver function was normal.

The authors state: 'A mild to moderate increase in liver enzymes is often found as part of this disease (Still's disease). Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen.'

This is a summary of an article written in German. An English translation has been ordered.

The English translation of the case report:

A 39-year-old woman initially presented to a hospital elsewhere with an extensive, reddish, non-pruritic maculopapular rash on both thighs which subsequently spread to the trunk and the other extremities. Over the previous week, she had had difficulty swallowing and had experienced a feeling of tightness in the throat. She reported symmetric arthralgias and myalgias and had also had spiking fevers of up to 38 degC and night sweats, as well as symptoms of marked fatigue, loss of vitality and reduced performance. The patient had no known history of chronic illness, was a non-smoker and had not been abroad in the last few months. As the fever was of unknown origin and there was no definitive source of infection, she was treated empirically with non-steroidal anti-inflammatory drugs (NSAIDs), antihistamines and oral penicillin on an outpatient basis.

Laboratory studies carried out at the hospital of presentation revealed a leukocytosis of $23.6 \times 10^9/l$ (normal $3.8 - 9.8 \times 10^9/l$) with predominant neutrophilia, in addition to a C-reactive protein (CRP) of 185 mg/l (normal < 5 mg/l), an erythrocyte sedimentation rate (ESR) of 52/93 mm/h (normal $6 - 10$ mm/h) and a ferritin level of 1740 $\mu g/l$ (normal $10 - 130$ mcg/l). Levels of transaminases were also increased (alanine aminotransferase (ALAT) 1.03 mcmol/l (normal < 0.74 mcmol/l), aspartate aminotransferase (ASAT) 1.06 mcmol/l (normal < 0.58 mcmol/l)), as were levels of gamma-glutamyltransferase (gamma-GT) (2.73 mcmol/l; normal < 0.92 mcmol/l) and lactate dehydrogenase (LDH) (12.48 mcmol/l; normal < 4.13 mcmol/l).

Ultrasound findings were unremarkable except for a mild hepatosplenomegaly. No pathognomonic features were identified on radiological examination of the chest, wrists or ankles or on echocardiography. Extensive serological testing, which comprised assays for hepatitis A, B and C, Epstein-Barr virus (EBV), cytomegalus virus (CMV), syphilis and human immunodeficiency virus (HIV), gave negative results, as did repeated blood cultures. On suspicion that the rash was infection-related, intravenous antibiotic treatment was started, initially with ceftriaxone, then with piperacillin plus tazobactam and finally, as fever spikes of up to 40 degC continued to occur, clarithromycin. Symptomatic treatment with paracetamol 4 x 1000 mg/day and ibuprofen 3 x 600 mg/day was also started. However, marked jaundice ensued (direct bilirubin 313 mcmol/l (normal < 3 mcmol/l), g-GT 16.28 mcmol/l), there was a massive increase in the levels of liver enzymes (ALAT 30.45 mcmol/l, ASAT 22.10 mcmol/l, LDH 20.64 mcmol/l) and a coagulopathy developed (Quick value 29%/International Normalized Ratio (INR) 2.05); the spiking fevers also persisted. Still's disease was suspected and a single dose of 250 mg methylprednisolone was given. After this, the patient was transferred to the authors hospital for management of her acute liver failure, which now dominated the clinical picture.

On admission, the patient was of normal body habitus but in reduced general condition, with marked scleral and cutaneous jaundice and a generalised maculopapular rash. Her left cervical lymph nodes were palpably enlarged but there were no other abnormal findings in the head and neck area. Fine bubbling rales were heard over the bases of both lungs on auscultation. Heart sounds were pure and rhythmical and the pulse rate was 72/min. All peripheral pulses were palpable. The liver was palpable about 3 - 4 cm below the costal arch. The spleen was impalpable. The spine and the extremities were normal on clinical examination. There were no neurological abnormalities.

Initially, the patient had low levels of calcium (1.9 mmol/l; normal 2.2 - 2.65 mmol/l), total protein (34 g/l; normal 69 - 86 g/l) and albumin (16 g/l; normal 35 - 53 g/l). She was mildly hyponatraemic (134 mmol/l; normal 135 - 145 mmol/l) but levels of all other electrolytes were within the normal range. Levels of creatinine and urea were also normal. After the dose of methylprednisolone administered earlier, CRP was only slightly raised at 28.3 mg/l. The ferritin level was extremely high (15,405 mcg/l; normal 10 - 291 mcg/l) and transaminase levels were also significantly elevated (ASAT 15.93 mcmol/l, ALAT 26.21 mcmol/l). Elevations were also observed in the levels of glutamate dehydrogenase (1666 nmol/l; normal < 80 nmol/l), LDH (15.54 mcmol/l) and alkaline phosphatase (8.57 mcmol/l; normal < 2.0 mcmol/l). There was also marked hyperbilirubinaemia (direct bilirubin 228.5 mcmol/l). The level of cholinesterase was 41 mcmol/l (normal 65 - 180 mcmol/l), reflecting impaired hepatic synthesis. Levels of ammonia were raised (133 mcmol/l; normal 11.5 - 38.0 mcmol/l). Microcytic hypochromic anaemia was present, as was leukocytosis ($13.9 \times 10^9/l$). The platelet count was normal. The Quick value was 40% (INR 1.8) and fibrinogen was reduced (1.0 g/l; normal 1.8 - 3.5 g/l). Tests for classical and perinuclear

antineutrophil cytoplasmic antibodies (c-ANCA and p-ANCA), antinuclear antibodies (ANA) and anti-double stranded DNA antibodies were negative. Proteinuria was present (766 mg/l; normal < 150 mg/l) but there were no leukocytes or erythrocytes in the urinary sediment. On the assumption that the patient was suffering from toxic liver damage, a paracetamol assay was carried out. This gave negative results, although admittedly the test was performed several days after the last administration of the drug.

To determine the cause of the liver failure, a percutaneous liver biopsy was carried out after coagulation products had been administered to normalise coagulation parameters and platelet counts.

Still's disease was presumptively diagnosed and the patient was treated with intravenous methylprednisolone 250 mg/day for 3 days followed by oral prednisolone 100 mg/day. For the first time, her fever resolved and she reported a subjective improvement in well-being. However, liver failure persisted and hepatic synthesis continued to be markedly impaired, so evaluation for high-urgency liver transplantation was begun. Conservative treatment with acetylcysteine was started on the suspicion that there might have been additional liver damage due to paracetamol. When 300 mg/kg had been infused over a period of 20 h, however, the lab results showing that paracetamol had not been detected in the blood became available and treatment with acetylcysteine was discontinued. In the absence of specific therapeutic options, it was decided to administer ornithine aspartate 15 g/day by intravenous infusion and ursodeoxycholic acid 750 mg/day orally. High-dose corticosteroid therapy was maintained and the raised levels of liver enzymes fell significantly, thus avoiding the need for a liver transplant.

The patient's liver function eventually returned to normal. As regards her Still's disease, follow-up tests showed complete resolution of inflammatory activity despite a reduction in the prednisolone dosage. Immunosuppressant therapy therefore did not need to be increased and, 14 months after the initial event, the patient is completely symptom-free on a regime of low-dose prednisolone (3 mg/day) and her liver function is normal.

The authors state: "In summary, the cause of the liver failure in our patient is difficult to identify retrospectively. As she had only taken paracetamol at therapeutic dosages, her liver failure is probably most likely to have been due to a toxic effect of the drug superimposed on liver damage arising as a result of the underlying Still's disease. A further rare cause of liver failure, idiosyncratic antibiotic toxicity, must also be considered in view of the various antibiotics she received. However, the fact that her liver function rapidly improved on corticosteroid therapy suggests to us that her Still's disease was a principal factor in the acute liver damage she sustained."

Bayer Vital GmbH, 15.01.2008 DE-BAYER-200718258GPV

Sender diagnosis: not reported

Narrative

This case was derived from the medical literature on 27-Nov-2007.

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degrees C, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 microg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 micromol/l.s, aspartate aminotransferase 1.06 micromol/l.s, gamma-glutamyltransferase 2.73 micromol/l.s, and lactate dehydrogenase 12.48 micromol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 x 1,000 mg/day) and ibuprofen (3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

Authors comment: The etiology of Still's disease is unknown and the disease is characterized by fever episodes, a typical macular-papular exanthema, lymphadenopathy, hepatosplenomegaly, and arthralgias. A mild to moderate increase in liver enzymes is often found as part of this disease. Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen. Still's disease must be considered as part of the differential diagnosis of acute liver failure, because an early diagnosis and consequent therapy with prednisone may prevent the need for liver transplantation.

Bayer Vital GmbH, 18.01.2008 DE-BAYER-200718258GPV

Sender diagnosis: not reported

Narrative

This case was derived from the medical literature on 27-Nov-2007.

A 39-year-old woman was admitted because of a nonitching macular-papular exanthema on both thighs with spreading to the trunk. In addition, the patient complained of dysphagia, symmetrical arthralgias, myalgias, fever of 38 degrees C, and night sweats. An outpatient treatment with nonsteroidal antirheumatics, antihistamines and penicillin was started for 3 days before admission. On admission, a neutrophilic leukocytosis (23.6 Gpt/l), an increase in C-reactive protein (185 mg/l), and a ferritin level of 1,740 microg/l were found. Liver enzymes were increased (alanine aminotransferase 1.03 micromol/l.s, aspartate aminotransferase 1.06 micromol/l.s, gamma-glutamyltransferase 2.73 micromol/l.s, and lactate

dehydrogenase 12.48 micromol/l.s). Sonographic examination showed a mild hepatosplenomegaly, but otherwise normal findings. X-rays of the lungs, hands, and ankles were normal. An echocardiography was within normal limits. Extensive serologic investigations including assays for hepatitis A, B and C as well as repeated blood cultures were negative. Antibiotic therapy was continued without any improvement. In addition, acetaminophen (4 x 1,000 mg/day) and ibuprofen (3 x 600 mg/day) were given. Liver function worsened and an icterus developed. The patient was transferred to the authors' university hospital. Because of the clinical findings of fever episodes, a typical macular exanthema, lymphadenopathy, hepatosplenomegaly, arthralgias, myalgias, dysphagia, and the presence of neutrophilic leukocytosis, fever, an increase in ferritin, but negative serologic titers and no bacteremia, a working diagnosis of Still's disease was made. The patient was treated with high-dose methylprednisone (250 mg/day for 3 days, then 100 mg/day). Liver biopsy revealed subacute hepatitis with necrosis and accompanying cholangitis. The prednisone therapy induced a fast remission and improvement of liver function, liver transplantation was not necessary. The patient is, 16 months after the incident, without symptoms under prednisone 3 mg/day, and the liver function is normal.

Authors comment: The etiology of Still's disease is unknown and the disease is characterized by fever episodes, a typical macular-papular exanthema, lymphadenopathy, hepatosplenomegaly, and arthralgias. A mild to moderate increase in liver enzymes is often found as part of this disease. Rarely, a fulminate liver failure has been described, particularly in the presence of co-administration of nonsteroidal antirheumatics or acetaminophen. Still's disease must be considered as part of the differential diagnosis of acute liver failure, because an early diagnosis and consequent therapy with prednisone may prevent the need for liver transplantation.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07108262

Serious Yes	Death No	Lifethr. Yes	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 39 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure					recovered/resolved
2 Icterus					recovered/resolved
3 Hepatosplenomegaly					Unknown
4 Coagulopathy					recovered/resolved
5 Cholangitis					Unknown
6 Necrosis					Unknown
7 Hepatitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ansaid flurbiprofen	Unknown	/	-	
2 CEFTRIAZONE (NGX) ceftriaxone	Intravenous (not otherwis	/	-	Fever
3 CLARITHROMYCIN (NGX) clarithromycin	Intravenous (not otherwis	/	-	Fever
4 IBUPROFEN (NCH) ibuprofen	Unknown	1800 Mg milligram(s)/Day	-	
5 PARACETAMOL (NGX) paracetamol	Unknown	4000 Mg milligram(s)/Day	-	
6 PHENOXYMETHYLPENICILLIN (NGX) phenoxymethylpenicillin	Oral	/	-	Pyrexia of unknown origin
7 PIPERACILLIN+TAZOBACTAM (NGX) piperacillin; tazobactam	Intravenous (not otherwis	/	-	

Drugs (concomitant)

8 ANTI-HISTAMINES	Unknown	/	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lymphadenopathy			Unknown	associated with Morbus Still
Papular rash circumocular-blepharal			Unknown	on both thighs with spreading to trunk, associated with Morbus Still
Dysphagia			Unknown	for 2 weeks, associated with Morbus Still
Arthralgia			Unknown	symmetrical, associated with Morbus Still
Myalgia			Unknown	symmetrical, associated with Morbus Still
Fever			Unknown	up to 38.5°C, associated with Morbus Still
Night sweats			Unknown	associated with Morbus Still
Still's disease			Unknown	Morbus Still with elevated inflammatory parameters

Primary source

Qualification:	observestudytype:
Literature:	Med Klin (Munich); Linde B, Oelzner P, Katenkamp K, Hein G, Wolf G; 102; 2007; 846-51; Fulminate liver failure in a 39-year-old female patient with leukocytosis, unclear fever, and arthralgic pain

Qualification: Other Health Professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 13.11.2007 DE-SZ09-GXKR2007DE09383

Sender diagnosis: not reported

Narrative

Initial literature report received on 30 Oct 2007. A patient developed fulminant liver failure following treatment with paracetamol, ibuprofen and several antibiotics. The patient presented with a non-itching macular-papular exanthema on both thighs with spreading to the trunk, complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. The patient had already received nonsteroidal antiinflammatories, antihistamines and penicillin per oral as an out-patient treatment due to fever of unknown origin. A parainfectious exanthema was suspected and antibiotic treatment with ceftriaxone IV was started and later changed to piperacillin/tazobactam and clarithromycin due to persistent pyrexia. Paracetamol (1000mg QID) and ibuprofen (600mg TID) were started as additional antiphlogistic therapy, but liver function worsened and the patient developed an icterus of skin and sclerae. Liver values worsened and coagulopathy developed. The diagnosis of an underlying Still's disease was made and the patient was treated with high-dose methylprednisolone IV (250 mg/day) for 3 days, then switched to prednisolone per oral (100 mg/day). Acetylcysteine was started, but stopped when blood tests were negative for paracetamol. Ornithinaspartat and ureol were given. Liver biopsy revealed subacute hepatitis with perivenous necrosis, an accompanying cholangitis and fibrosis. Liver function improved on administration of corrective treatment. At the time of reporting, 16 months after the incident, the patient is without any symptoms under prednisone 3 mg/day. Liver function has turned to normal. The authors state that liver failure was probably induced by a combination of reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol and nonsteroidal antiinflammatories. As the patient did not take high doses of paracetamol, they also consider the use of different antibiotics. adverse event.

Reporter's comment:NOVARTIS, 13.11.2007 DE-NOVNCHHQ-GXKR2007DE09383

Sender diagnosis: not reported

Narrative

Initial literature report received on 30 Oct 2007. A patient developed fulminant liver failure following treatment with paracetamol, ibuprofen and several antibiotics. The patient presented with a non-itching macular-papular exanthema on both thighs with spreading to the trunk, complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. The patient had already received nonsteroidal antiinflammatories, antihistamines and penicillin per oral as an out-patient treatment due to fever of unknown origin. A parainfectious exanthema was suspected and antibiotic treatment with ceftriaxone IV was started and later changed to piperacillin/tazobactam and clarithromycin due to persistent pyrexia. Paracetamol (1000mg QID) and ibuprofen (600mg TID) were started as additional antiphlogistic therapy, but liver function worsened and the patient developed an icterus of skin and sclerae. Liver values worsened and coagulopathy developed. The diagnosis of an underlying Still's disease was made and the patient was treated with high-dose methylprednisolone IV (250 mg/day) for 3 days, then switched to prednisolone per oral (100 mg/day). Acetylcysteine was started, but stopped when blood tests were negative for paracetamol. Ornithinaspartat and ureol were given. Liver biopsy revealed subacute hepatitis with perivenous necrosis, an accompanying cholangitis and fibrosis. Liver function improved on administration of corrective treatment. At the time of reporting, 16 months after the incident, the patient is without any symptoms under prednisone 3 mg/day. Liver function has turned to normal. The authors state that liver failure was probably induced by a combination of reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol and nonsteroidal antiinflammatories. As the patient did not take high doses of paracetamol, they also consider the use of different antibiotics. adverse event.

Reporter's comment:NOVARTIS, 19.11.2007 DE-NOVNCHHQ-GXKR2007DE09383

Sender diagnosis: not reported

Narrative

Initial literature report received on 30 Oct 2007. A patient developed fulminant liver failure following treatment with paracetamol, ibuprofen and several antibiotics. The patient presented with a non-itching macular-papular exanthema on both thighs with spreading to the trunk, complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. The patient had already received nonsteroidal antirheumatics, antihistamines and penicillin per oral as an out-patient treatment due to fever of unknown origin. A parainfectious exanthema was suspected and antibiotic treatment with ceftriaxon IV was started and later changed to piperacillin/tazobactam and clarithromycin due to persistent pyrexia. Paracetamol (1000mg QID) and ibuprofen (600mg TID) were started as additional antiphlogistic therapy, but liver function worsened and the patient developed an icterus of skin and sclerae. Liver values worsened and coagulopathy developed. The diagnosis of an underlying Still's disease was made and the patient was treated with high-dose methylprednisone IV (250 mg/day) for 3 days, then switched to prednisolone per oral (100 mg/day). Acetylcystein was started, but stopped when blood tests were negative for paracetamol. Ornithinaspartat and ureol were given. Liver biopsy revealed subacute hepatitis with perivenous necrosis, an accompanying cholangitis and fibrosis. Liver function improved on administration of corrective treatment. At the time of reporting, 16 months after the incident, the patient is without any symptoms under prednisone 3 mg/day. Liver function has turned to normal. The authors state that liver failure was probably induced by a combination of reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol and nonsteroidal antirheumatics. As the patient did not take high doses of paracetamol, they also consider the use of different antibiotics.

Significant FU received on 09 Nov 2007. Full text translation received. No new information provided. The SPC for piperacillin-tazobactam was available and the listedness was provided.

Serious literature report (life-threatening) assessed as unlisted according to the Core Data Sheet Information due to greater specificity/severity. The following terms (hepatitis, liver enzymes elevated) are already included in the Core Data Sheet Information.

All literature reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 20.11.2007 DE-SZ09-GXKR2007DE09383

Sender diagnosis: not reported

Narrative

Initial literature report received on 30 Oct 2007. A patient developed fulminant liver failure following treatment with paracetamol, ibuprofen and several antibiotics. The patient presented with a non-itching macular-papular exanthema on both thighs with spreading to the trunk, complained of dysphagia, symmetrical arthralgias, myalgias and fever. A neutrophilic leukocytosis was found. Liver enzymes were increased and sonographic examination showed a mild hepatosplenomegaly. The patient had already received nonsteroidal antirheumatics, antihistamines and penicillin per oral as an out-patient treatment due to fever of unknown origin. A parainfectious exanthema was suspected and antibiotic treatment with ceftriaxon IV was started and later changed to piperacillin/tazobactam and clarithromycin due to persistent pyrexia. Paracetamol (1000mg QID) and ibuprofen (600mg TID) were started as additional antiphlogistic therapy, but liver function worsened and the patient developed an icterus of skin and sclerae. Liver values worsened and coagulopathy developed. The diagnosis of an underlying Still's disease was made and the patient was treated with high-dose methylprednisone IV (250 mg/day) for 3 days, then switched to prednisolone per oral (100 mg/day). Acetylcystein was started, but stopped when blood tests were negative for paracetamol. Ornithinaspartat and ureol were given. Liver biopsy revealed subacute hepatitis with perivenous necrosis, an accompanying cholangitis and fibrosis. Liver function improved on administration of corrective treatment. At the time of reporting, 16 months after the incident, the patient is without any symptoms under prednisone 3 mg/day. Liver function has turned to normal. The authors state that liver failure was probably induced by a combination of reduced liver function due to the underlying Still's disease and the toxic effect of paracetamol and nonsteroidal antirheumatics. As the patient did not take high doses of paracetamol, they also consider the use of different antibiotics.

Significant FU received on 09 Nov 2007. Full text translation received. No new information provided. The SPC for piperacillin-tazobactam was available and the listedness was provided.

Serious literature report (life-threatening) assessed as unlisted according to the Core Data Sheet Information due to greater specificity/severity. The following terms (hepatitis, liver enzymes elevated) are already included in the Core Data Sheet Information.

All literature reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08007013

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 64 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus	20071025			3 Month	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen al 800 ibuprofen	Oral	800 Mg milligram(s)/Day Film-coated tablet	200707- 3 Month	Arthrosis multiple

Drugs (concomitant)

2 Delix 2,5 mg Tbl. ramipril	Oral	/	-	Drug use for unknown indication
		Tablet	-	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT		2800	U/L	
GOT		1700	U/L	

Narratives (nur bei elektronischem Eingang)

ALIUD PHARMA GmbH & Co. KG, 22.01.2008 DE-STADA-009218

Sender diagnosis: not reported

Narrative

This report was received by a consumer who is physician. The 64-year-old male patient received ibuprofen beginning from middle of July 2007 for polyarthrosis (at the beginning of ibuprofen therapy it was taken irregularly at doses up to 2400 mg, several weeks before the event occurred ibuprofen was taken regularly at a dose of 400 mg twice daily). Around 2007-Oct-25 he developed an icterus. The event caused hospitalisation on 2007-Oct-29 for 10 days. Neither hepatitis nor occlusion were diagnosed but only an intrahepatic event. The outcome of the event was reported to be recovered. Drug history included lumiracoxib for 2 or 3 weeks (June 2007). Because of ineffectivity this drug was replaced first by diclofenac or ibuprofen and afterwards by ibuprofen only. The liver values had been controlled regularly at rather short intervals. During lumiracoxib therapy and also after starting ibuprofen therapy (June/July 2007) the GOT- and GPT-values were increased only moderately (about 80 U/L). On 2007-Oct-09 both liver values were in the normal range. During hospitalisation GPT was 2800 U/L and GOT 1700 U/L. One week before the icterus occurred, the patient had a severe diarrhoe during one night. At the next morning he was without any complaints. Concomitant medication included ramipril and capsules containing fish oil. The reporter assessed the causal relationship between the suspected drug and the event as unlikely related due to the chronological sequence of liver values. But since other causes for the event are lacking, a causal relationship to ibuprofen cannot be excluded. The hospital report has been requested.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08016252

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 20010123	Age: 6 Year	weight (kg): 17	BMI_int: 13.55	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leukopenia	200703				recovered/resolved
2 Hepatosplenomegaly	200703				recovered/resolved
3 Abscess leg	200703				recovered/resolved
4 Colonic dysfunction	200703				recovered/resolved
5 Gangrenous appendicitis	200703				recovered/resolved
6 Mucosal inflammation	200703				recovered/resolved
7 Decubitus	200703				recovered/resolved
8 Thrombocytopenia	200703				recovered/resolved
9 Agranulocytosis	200703				recovered/resolved
10 Anaemia	200703				recovered/resolved
11 Pancytopenia	200703				recovered/resolved
12 Staphylococcal sepsis	200703				recovered/resolved
13 Streptococcal tonsillitis	200703				recovered/resolved
14 Respiratory insufficiency	200703				recovered/resolved
15 Loss of weight	200703				recovered/resolved
16 Stomatitis	200703				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Nurofen Junior Fiebersaft ibuprofen	Oral	/	200711-20071203 -	Fever
2 Nurofen Junior Fiebersaft ibuprofen	Oral	/	20070312-20070328 17 Day	Febrile infection

Drugs (concomitant)

3 boostrix diphtheria toxoid; pertussis vaccine acellular; tetanus toxoid	Intramuscular	/	20070301- -	Vaccination
4 capval noscaphine resin	Oral	/	20070312- -	
5 contramutan aconite; adonis vernalis glycosides; belladonna alkaloids; echinacea spp; eupatorium perfoliatum; myristica	Oral	/	20070312- -	
6 infectocillin 500 soft phenoxymethylpenicillin potassium	Oral	/	20070320-20070326 7 Day	
7 klacid clarithromycin	Oral	/	20071202-20071203 2 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tonsillitis			Unknown	
Sepsis			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08019428

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19741129	Age: 30 Year	weight (kg): 50	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function test abnormal					recovered/resolved
2 Overdose					Unknown
3 Suicidal ideation					recovered/resolved
4 Sleepiness					recovered/resolved
5 Somnolence					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 gelonida acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/	-	Drug use for unknown indication
2 GELONIDA acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/ Tablet	-	Suicidal ideation
3 ibuprofen ibuprofen	Oral	/	-	Drug use for unknown indication
4 IBUPROFEN ibuprofen	Oral	800 Mg milligram(s)/ Tablet	-	Suicidal ideation
5 imodium loperamide hydrochloride	Oral	/	-	Drug use for unknown indication
6 IMODIUM loperamide	Oral	/ Capsule	-	Suicidal ideation
7 lariam mefloquine	Oral	/	-	Drug use for unknown indication
8 LARIAM mefloquine	Oral	250 Mg milligram(s)/ Tablet	-	Suicidal ideation
9 talvosilen codeine phosphate; paracetamol	Oral	/	-	Drug use for unknown indication
10 TALVOSILEN codeine phosphate; paracetamol	Oral	/ Tablet	-	Suicidal ideation

Primary source

Qualification: Other Health Professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

ROCHE, 25.02.2008 DE-ROCHE-548290

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE WAS RECEIVED VIA THE BUNDESINSTITUT FUER ARZNEIMITTEL UND MEDIZINPRODUKTE (BFARM), GERMANY, AND CONCERNS A 30-YEAR-OLD FEMALE PATIENT WHO EXPERIENCED SUICIDAL IDEATION, OVERDOSE, LIVER FUNCTION TEST ABNORMAL AND SOMNOLENCE DURING/AFTER THE USE OF MEFLOROQUINE (LARIAM), LOPERAMIDE (IMODIUM), ASPIRIN/CODEINE PHOSPHATE/PARACETAMOL (GELONIDA), IBUPROFEN AND CODEINE PHOSPHATE/PARACETAMOL (TALVOSILEN) FOR UNSPECIFIED INDICATIONS.

THE PATIENT STARTED ORAL MEFLOROQUINE, ORAL LOPERAMID, ORAL ASPIRIN/CODEINE PHOSPHATE/PARACETAMOL, ORAL IBUPROFEN AND ORAL CODEINE PHOSPHATE/PARACETAMOL. AFTER AN UNSPECIFIED PERIOD OF TIME, THE PATIENT TOOK AN OVERDOSE OF MEFLOROQUINE (16 DOSE FORMS DAILY), LOPERAMIDE (24 DOSE FORMS DAILY), ASPIRIN/CODEINE

PHOSPHATE/PARACETAMOL (40 DOSE FORMS DAILY), IBUPROFEN (20 DOSE FORMS DAILY) AND CODEINE PHOSPHATE/PARACETAMOL (70 DOSE FORMS DAILY) DUE TO SUICIDAL IDEATION. AT AN UNSPECIFIED TIMEPOINT, THE PATIENT DEVELOPED AN ABNORMAL LIVER FUNCTION TEST AND SOMNOLENCE.

AT THE TIME OF THE REPORT, THE EVENTS OF SUICIDAL IDEATION, LIVER FUNCTION TEST ABNORMAL AND SOMNOLENCE WERE CONSIDERED RESOLVED.

THE REPORTER ASSESSED THE EVENTS OF SUICIDAL IDEATION, OVERDOSE, LIVER FUNCTION TEST ABNORMAL AND SOMNOLENCE AS MEDICALLY SIGNIFICANT.

FOLLOWING MEDICAL ASSESSMENT, FOR THE EVENTS OF SUICIDAL IDEATION, OVERDOSE, LIVER FUNCTION TEST ABNORMAL AND SOMNOLENCE, THE COMPANY UPGRADED THE CAUSALITY ASSESSMENT TO RELATED FOR MEFLOROQUINE, TO ENABLE APPROPRIATE SUBMISSIONS.

NO FURTHER INFORMATION WAS AVAILABLE.

ROCHE, 10.03.2008 DE-ROCHE-548290

Sender diagnosis: not reported

Narrative

THIS SPONTANEOUS CASE WAS RECEIVED VIA THE BUNDESINSTITUT FUER ARZNEIMITTEL UND MEDIZINPRODUKTE (BFARM), GERMANY, AND CONCERNS A 30-YEAR-OLD FEMALE PATIENT WHO EXPERIENCED SUICIDAL IDEATION, OVERDOSE, LIVER FUNCTION TEST ABNORMAL AND SLEEPINESS DURING/AFTER THE USE OF MEFLOROQUINE (LARIAM), LOPERAMIDE (IMODIUM), ASPIRIN/CODEINE PHOSPHATE/PARACETAMOL (GELONIDA), IBUPROFEN AND CODEINE PHOSPHATE/PARACETAMOL (TALVOSILEN) FOR UNSPECIFIED INDICATIONS.

THE PATIENT STARTED ORAL MEFLOROQUINE, ORAL LOPERAMIDE, ORAL ASPIRIN/CODEINE PHOSPHATE/PARACETAMOL, ORAL IBUPROFEN AND ORAL CODEINE PHOSPHATE/PARACETAMOL. AFTER AN UNSPECIFIED PERIOD OF TIME, THE PATIENT WAS HOSPITALISED AFTER TAKING AN OVERDOSE OF MEFLOROQUINE (16 DOSE FORMS DAILY), LOPERAMIDE (24 DOSE FORMS DAILY), ASPIRIN/CODEINE PHOSPHATE/PARACETAMOL (40 DOSE FORMS DAILY), IBUPROFEN (20 DOSE FORMS DAILY) AND CODEINE PHOSPHATE/PARACETAMOL (70 DOSE FORMS DAILY) DUE TO SUICIDAL IDEATION. AT AN UNSPECIFIED TIMEPOINT, THE PATIENT DEVELOPED AN ABNORMAL LIVER FUNCTION TEST AND SLEEPINESS.

AT THE TIME OF THE REPORT, THE EVENTS OF SUICIDAL IDEATION, LIVER FUNCTION TEST ABNORMAL AND SOMNOLENCE WERE CONSIDERED RESOLVED.

THE REPORTER ASSESSED THE EVENTS OF SUICIDAL IDEATION, OVERDOSE, LIVER FUNCTION TEST ABNORMAL AND SLEEPINESSE AS MEDICALLY SIGNIFICANT. THE REPORTER PROVIDED A CAUSALITY ASSESSMENT FOR THE EVENTS OF SUICIDAL IDEATION AND OVERDOSE, AND LOPERAMIDE AS POSSIBLE. THE REPORTER DID NOT PROVIDE A CAUSALITY ASSESSMENT FOR THE REPORTED EVENTS AND MEFLOROQUINE, ASPIRIN/CODEINE PHOSPHATE/PARACETAMOL, IBUPROFEN AND CODEINE PHOSPHATE/PARACETAMOL.

FOLLOWING MEDICAL ASSESSMENT, FOR THE EVENTS OF SUICIDAL IDEATION, OVERDOSE, LIVER FUNCTION TEST ABNORMAL AND SLEEPINESS, THE COMPANY UPGRADED THE CAUSALITY ASSESSMENT TO RELATED FOR MEFLOROQUINE, TO ENABLE APPROPRIATE SUBMISSIONS.

NO FURTHER INFORMATION WAS AVAILABLE.

UPDATE INFORMATION WAS RECEIVED AND THE FOLLOWING DATA WAS ADDED TO THE CASE: PATIENT INITIALS AND WEIGHT, INDICATION TERM, SUSPECT DRUG FORMS, REPORTER'S CAUSALITY ASSESSMENT FOR LOPERAMIDE, DOSING AMOUNT FOR IBUPROFEN AND MEFLOROQUINE, PATIENT HOSPITALISATION. EVENT TERM CHANGED FROM SOMNOLENCE TO SLEEPINESS.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08040959

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No		No						

Patient

Initials:	Sex: Male	Birth: 19670707	Age: 40 Year	weight (kg): 130	BMI_int: 37.98	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20080122				not recovered/not resolved
2 Urine colour abnormal	20080122				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		600 Mg milligram(s)/Day	20080115-	
ibuprofen			-	
2 ibuprofen		2400 Mg milligram(s)/Day	20071217-20080115	
ibuprofen			30 Day	
3 ibuprofen		3200 Mg milligram(s)/Day	20071211-20071217	
ibuprofen			7 Day	
4 lyrica		75 Mg milligram(s)/Day	20070926-	Chronic pain
pregabalin			-	
5 novalgine		8 Df dosage form/Day	20071203-	
metamizole			-	
6 pantozol		40 Mg milligram(s)/Day	20070926-	
pantoprazole sodium			-	
7 saroten		75 Mg milligram(s)/Day	20071107-	
amitriptyline hydrochloride			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Adipositas			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT	20080121	1375	U/l	
Glutamic-oxaloacetic transferase	20080121	543.1	U/l	
Gamma GT	20080121	778.1	U/l	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08051516

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 37 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests raised					Unknown
2 Bilirubin increased					Unknown
3 GGT increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUHEXAL (NGX)	Oral	600 Mg milligram(s)/Day	-	
ibuprofen		Film-coated tablet	-	

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 29.04.2008 DE-SZ09-GXKR2008DE03734

Sender diagnosis: not reported

Narrative

This spontaneous report combines initial information received on 21 Apr 2008 from a pharmacist and on 24 Apr 2008 from the same pharmacist: This patient with an unknown medical history experienced raised liver function tests with increased bilirubin and increased gamma-glutamyltransferase (GGT) under treatment with IbuHEXAL 600 (ibuprofen) taken once only at a single dosage of 1 tablet. Exact therapy dates were not stated. Concomitantly no other drugs were taken. The patient drank no alcohol. The patient consulted a physician for further investigation in case of other unspecified signs and symptoms. A blood analysis was performed four days after intake of IbuHEXAL 600 revealing an increased GGT of about 380 U/l and an increased bilirubin of 4.2 mg/dl. At the time of report the physician was performing a comprehensive workup. The outcome of the adverse event was not reported. The reporter provided no causality assessment.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-08060785

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth:	Age: 51 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic encephalopathy					recovered/resolved
2 Hepatic coma					recovered/resolved
3 Autoimmune hepatitis					recovered/resolved
4 Acute liver failure					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		500 Mg milligram(s)/	-	Drug use for unknown indication
ibuprofen			-	
2 INTERFERON BETA	Unknown	30 µg microgram(s)/Week	200606-8 Week	Relapsing-remitting multiple sclerosis
interferon beta				
3 LORAZEPAM		/	-	Antidepressant therapy
lorazepam			-	
4 sertraline		/	-	Antidepressant therapy
sertraline			-	
5 SIMVASTATIN		/	-	Hyperlipidemia
simvastatin			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Depression			Unknown	
Lipids blood increased			Unknown	
Metabolic disorder			Yes	
Multiple sclerosis	200605		Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Behrbohm J, Neid M, Stoezel U, Wittekind C, Hauss J, Tillmann H. Improvement of multiple sclerosis on tacrolimus plus mycophenolate mofetil after liver transplantation: clinical improvement of multiple sclerosis in a patient requiring liver transplantation for acute liver failure following interferon. Transpl Int 2007;20:12:1077-1079.	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Albumin		28.6 (on hospital admission)	g/L (on admission)	
Bilirubin		serum: 630.2 µmol/L (on hospital admission)		
International normalized ratio		1.73 (on hospital admission)		

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 27.05.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-03028GD

Sender diagnosis: not reported

Narrative

A patient is reported who underwent liver transplantation after development of acute liver failure during interferon (IFN)-beta therapy for multiple sclerosis (MS) with evidence for autoimmune hepatitis. The authors could not completely exclude a toxic component of the co-medication (including ibuprofen and sertraline).

A 51-year-old-Caucasian woman was diagnosed suffering from MS in 05/2006. Treatment with 30 mcg IFN-beta per week for relapsing-remitting MS was initiated in 06/2006. Eight weeks after initiation of IFN treatment the patient was admitted to hospital due to rapidly progressing nonspecific jaundice. Interferon was immediately stopped after a total of 240 mcg IFN-beta. All IFN injections had been combined with 500 mg ibuprofen. Medical treatment also included simvastatin for elevated blood lipids and antidepressants (sertraline, lorazepam).

Because of disease deterioration with elevated ALT and AST and rapidly increasing bilirubin and INR she was admitted to hospital. She also developed hepatic encephalopathy grade II and therefore was transferred to the authors' transplantation unit 8 days later.

At admission to hospital the initial liver enzymes were elevated, serum bilirubin was 630.2 µmol/l (normal <20.5), international normalized ratio was 1.73 (INR) (normal <1.2), albumin was 28.6 g/l (normal 35-45). Viral serology for hepatitis A, B and C, cytomegaly, Epstein-Barr virus and HIV was negative. Antinuclear antibodies were positive at a titer of 1:160 while other autoantibodies (including ANCA, anti-mitochondrial antibodies, liver-kidney-microsomal antibodies) were negative as was copper in urine. CT and sonography showed no signs of cirrhosis. Autoimmune hepatitis (AIH) score was 12, equivalent to probably autoimmune hepatitis, and later reassured by the histological result of the explanted liver.

Hepatic encephalopathy grade II further progressed despite treatment with lactulose to hepatic coma within 24 h and the patient was listed for emergency liver transplantation.

Three days after admission to the transplantation unit, liver transplantation was performed. Pathologic examination of the explanted liver revealed hepatic bridging necrosis, massive inflamed tissue and early cirrhosis indicating an autoimmune hepatitis. AIH score increased to 15.

These findings were considered to indicate an autoimmune hepatitis and acute inflammation leading to an increase in certainty about the autoimmune hepatitis as the score increased to 15.

Post-transplant immunosuppression consisted of tacrolimus (Prograf, Fujisawa), Mycophenolate Mofetil (Cell-CEPT, Hoffman-LaRoche) and tapering prednisolone.

4 weeks after transplantation the patient was discharged. She did not have any relapses of MS during 12 months after transplantation. Her neurological symptoms (i.e. cerebellar ataxia) have improved dramatically.

The authors state: "Our patient had no history for liver diseases and a non-suspicious medical history. Other causes of liver failure were excluded. Consequently, we presume the fulminant liver failure to be related to the IFN-beta even though a toxic component of the co-medication cannot be completely excluded, we suspect that the immunomodulatory properties of IFN-beta induced an acute autoimmune hepatitis, as it has been suggested in some patients before."

Boehringer Ingelheim GmbH, Germany, 28.05.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-03028GD

Sender diagnosis: not reported

Narrative

A patient is reported who underwent liver transplantation after development of acute liver failure during interferon (IFN)-beta therapy for multiple sclerosis (MS) with evidence for autoimmune hepatitis. The authors could not completely exclude a toxic component of the co-medication (including ibuprofen and sertraline).

A 51-year-old-Caucasian woman was diagnosed suffering from MS in 05/2006. Treatment with 30 mcg IFN-beta per week for relapsing-remitting MS was initiated in 06/2006. Eight weeks after initiation of IFN treatment the patient was admitted to hospital due to rapidly progressing nonspecific jaundice. Interferon was immediately stopped after a total of 240 mcg IFN-beta. All IFN injections had been combined with 500 mg ibuprofen. Medical treatment also included simvastatin for elevated blood lipids and antidepressants (sertraline, lorazepam).

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McNeil Germany, Local Safety Officers, 30.06.2008 DE-JNJFOC-20080606661

Sender diagnosis: not reported

Narrative

Report received in the form of a case report from a full literature article via a business partner (Pfizer): Behrbohm J, Neid M, Stotzel U, Wittekind C, Hauss JP, Tillman H. Improvement of multiple sclerosis on tacrolimus plus mycophenolate mofetil after liver transplantation. European Society for Organ Transplantation 2008;20:1077-1079. The article referenced clinical improvement of multiple sclerosis in a patient requiring liver transplantation for acute liver failure following interferon-B therapy.

A 51-year-old Caucasian woman was diagnosed suffering from MS in 05/2006. Treatment with 30 ug IFN-B per week for relapsing-remitting MS was initiated in 06/2006. Eight weeks after initiation of IFN treatment the patient was admitted to the hospital due to rapidly progressing non-specific jaundice. Interferon was immediately stopped after a total of 240 ug IFN-B. All IFN injections had been combined with 500 mg ibuprofen (formulation and indication unspecified). Medical treatment also included simvastatin (formulation and dose unspecified) for elevated blood lipids and antidepressants (sertraline, lorazepam). Because of disease deterioration with elevated ALT and AST and rapidly increasing bilirubin and INR, the patient was admitted to the hospital. The patient also developed hepatic encephalopathy grade II and therefore was transferred to the transplantation unit 8 days later. Upon admission to the hospital, the initial liver enzymes were elevated, serum bilirubin was 630.2 umol/L (normal <20.5), international normalized ratio was 1.73 (INR) (normal <1.2), albumin was 28.6 g/L (normal 35-45). Viral serology for hepatitis A, B, and C, cytomegalie, Epstein-Barr virus and HIV was negative. Antinuclear antibodies were positive at a titer of 1:160 while other autoantibodies (including ANCA, anti-mitochondrial antibodies, liver-kidney-microsome antibodies) were negative as was copper in urine. CT and sonography showed no signs of cirrhosis. AIH score was 12, equivalent to probably autoimmune hepatitis, and later reassured by the histological result of the explanted liver. Hepatic encephalopathy grade II further progressed despite treatment with lactulose to hepatic coma within 24 h and the patient was listed for emergency liver transplantation. Three days after admission to the transplantation unit, a liver transplantation was performed. Pathologic examination of the explanted liver revealed hepatic bridging necrosis, massive inflamed tissue and early cirrhosis indicating an autoimmune hepatitis. AIH score increased to 15. These findings were considered to indicate an autoimmune hepatitis and acute inflammation leading to an increase in certainty about the autoimmune hepatitis as the score increased to 15. Post-transplant immunosuppression consisted of tacrolimus (Prograf, Fujisawa), Mycophenolate Mofetil (CellCept, Hoffman-LaRoche) and tapering prednisolone. Four weeks after transplantation, the patient was discharged. The patient did not have any relapses of MS during 12 months after transplantation. Her neurological symptoms (i.e. cerebellar ataxia) had improved dramatically.

The authors further discussed that the patient had no history for liver diseases and had a non-suspicious medical history. Other causes of liver failure were excluded. The authors presumed the fulminant liver failure to be related to the IFN-B even though a toxic component of the co-medication could not be completely excluded. The authors further suspected that the immunomodulatory properties of IFN-B induced an acute autoimmune hepatitis, as it had been suggested in some patients previously.

This report is serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 10.12.2012 DE-JNJFOC-20080606661

Sender diagnosis: not reported

Narrative

Report received in the form of a case report from a full literature article via a business partner (Pfizer): Behrbohm J, Neid M, Stotzel U, Wittekind C, Hauss JP, Tillman H. Improvement of multiple sclerosis on tacrolimus plus mycophenolate mofetil after liver transplantation. European Society for Organ Transplantation 2008;20:1077-1079. The article referenced clinical improvement of multiple sclerosis in a patient requiring liver transplantation for acute liver failure following interferon-B therapy.

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of the explanted liver revealed hepatic bridging necrosis, massive inflamed tissue and early cirrhosis indicating an autoimmune hepatitis. AIH score increased to 15. These findings were considered to indicate an autoimmune hepatitis and acute inflammation leading to an increase in certainty about the autoimmune hepatitis as the score increased to 15. Post-transplant immunosuppression consisted of tacrolimus (Prograf, Fujisawa), Mycophenolate Mofetil (CellCept, Hoffman-LaRoche) and tapering prednisolone. Four weeks after transplantation, the patient was discharged. The patient did not have any relapses of MS during 12 months after transplantation. Her neurological symptoms (i.e. cerebellar ataxia) had improved dramatically.

The authors further discussed that the patient had no history for liver diseases and had a non-suspicious medical history. Other causes of liver failure were excluded. The authors presumed the fulminant liver failure to be related to the IFN-B even though a toxic component of the co-medication could not be completely excluded. The authors further suspected that the immunomodulatory properties of IFN-B induced an acute autoimmune hepatitis, as it had been suggested in some patients previously.

This report is serious (hospitalization).

Version 1 was created to add local case ID numbers. No new information was added to the case.

Additional information was received on 28-NOV-2012.

It was determined that manufacturer report number 2008045986 was a duplicate of this case. All relevant information regarding this case will be su

<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08103430

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 20021201	Age: 5 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20080401	200804		2 Day	recovered/resolved
2 Bilirubinuria	20080401	200804		2 Day	recovered/resolved
3 Bilirubin increased	20080401	200804		2 Day	recovered/resolved
4 GGT increased	20080401	200804		2 Day	recovered/resolved
5 Brown urine	20080401	200804		2 Day	recovered/resolved
6 Scleral icterus	20080401	200804		2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Cefaclor	Oral	750 Mg milligram(s)/Day	20080330-20080401	Scarlet fever
cefaclor			2 Day	
2 ibuprofen	Unknown	/	20080330-20080401	Pain NOS
ibuprofen			2 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Exanthema			k.A.	exanthema
Streptococcal infection			k.A.	streptococcal infection

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 09.09.2008 DE-BAYER-200825730GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 04-Sep-2008 from a health professional in Germany via an internet-based pool of cases of BfArM, the regulatory health authority in Germany, referring to a male, whose age was not reported.

Patient's medical history includes exanthema and streptococcal infection.
Concomitant drugs were not reported.

Patient received the following drugs reported as 'suspect drugs':
--Cefaclor for scarlet fever,
--Ibuprofen for pain NOS.
Administration dates and total daily doses were not specified.

On 01-Apr-2008, patient experienced BROWN URINE, SCLERAL ICTERUS, TRANSAMINASES INCREASED, BILIRUBIN INCREASED, and BILIRUBINURIA.
The events were resolved at the time of the report.

The reporter assessed the diagnoses BROWN URINE, SCLERAL ICTERUS, TRANSAMINASES INCREASED, BILIRUBIN INCREASED, and BILIRUBINURIA to be possibly related to Cefaclor and Ibuprofen.

NOVARTIS, 19.09.2008 DE-002147023-CHNY2008DE03473

Sender diagnosis: not reported

Narrative

Initial report received from German Health Authority (ref: DE-BFARM-08103430) on 04 Sep 2008: a male patient received Ibuprofen (unknown manufacturer) for pain NOS and Cefaclor (unknown manufacturer) to treat a scarlet fever. On 01 Apr 2008, the patient developed bilirubinuria, scleral icterus, bilirubin increased, transaminases increased and brown urine. The patient completely recovered. The German Health Authority assessed the causality as possible for both ibuprofen and cefaclor.

For ibuprofen:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (specify the most serious criterion). The events Bilirubinuria, Scleral icterus, Bilirubin increased and Transaminases increased were assessed as unlabelled according to the product information due to greater specificity/severity. The term hepatic dysfunction is already included in the product information.

For cefaclor:

Serious spontaneous report (medically significant) already listed in the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 23.09.2008 DE-SZ09-CHNY2008DE03473

Sender diagnosis: not reported

Narrative

Initial report received from German Health Authority (ref: DE-BFARM-08103430) on 04 Sep 2008: a male patient received Ibuprofen (unknown manufacturer) for pain NOS and Cefaclor (unknown manufacturer) to treat a scarlet fever. On 01 Apr 2008, the patient developed bilirubinuria, scleral icterus, bilirubin increased, transaminases increased and brown urine. The patient completely recovered. The German Health Authority assessed the causality as possible for both ibuprofen and cefaclor.

For ibuprofen:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (specify the most serious criterion). The events Bilirubinuria, Scleral icterus, Bilirubin increased and Transaminases increased were assessed as unlabelled according to the product information due to greater specificity/severity. The term hepatic dysfunction is already included in the product information.

For cefaclor:

Serious spontaneous report (medically significant) already listed in the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Bayer Vital GmbH, 26.02.2009 DE-BAYER-200825730GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 04-Sep-2008 from a health professional in Germany via an internet-based pool of cases of BfArM, the regulatory health authority in Germany, referring to a male, whose age was not reported.

Patient's medical history includes exanthema and streptococcal infection.

Concomitant drugs were not reported.

Patient received the following drugs reported as 'suspect drugs':

--Cefaclor for scarlet fever,

--Ibuprofen for pain NOS.

Administration dates and total daily doses were not specified.

On 01-Apr-2008, patient experienced BROWN URINE, SCLERAL ICTERUS, TRANSAMINASES INCREASED, BILIRUBIN INCREASED, and BILIRUBINURIA.

The events were resolved at the time of the report.

The reporter assessed the diagnoses BROWN URINE, SCLERAL ICTERUS, TRANSAMINASES INCREASED, BILIRUBIN INCREASED, and BILIRUBINURIA to be possibly related to Cefaclor and Ibuprofen.

****Follow-up information received on 23-FEB-2009:**

German hospital pediatrician reported via AMKDAE, the Drug Commission of the German Medical Association, that the 5 year-old male Caucasian child with suspected penicillin allergy (urticaria) had contracted group A streptococcus and suffered from scarlet fever with exanthema, relapsing fever and pain. Thus, the child had been treated for the first time with oral Cephaclo 250 mg thrice daily (total daily dose: 750 mg) over two days.

Additionally, Ibuprofen (administered and well tolerated previously, daily dose not reported) had been given over the same time period.

On 01-Apr-2008, two days after the treatment with Cephaclo and Ibuprofen had started, the child developed brown urine, mild scleral icterus and bilirubinuria. Furthermore, an increase in transaminases, gamma-GT and bilirubin was diagnosed. These adverse reactions lasted over 3-4 days.

Cephaclo and Ibuprofen were discontinued. Upon drug withdrawal, the laboratory values normalised rapidly.

During the further treatment with penicillin-V no adverse reactions occurred.

No rechallenge was performed.

The adverse events were assessed not to be life-threatening and to be related to Cephaclo and Ibuprofen.

There were no particularities in the anamnesis.

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08107048

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19511108	Age: 56 Year	weight (kg): 87	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute toxic	20080725			5 Month	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ENBREL etanercept	Subcutaneous	50 Mg milligram(s)/Week Injection	20080424-20080711 79 Day	
2 ENBREL etanercept	Subcutaneous	2*50 Mg milligram(s)/Wee Injection	200802-200804 61 Day	Psoriasis
3 ibuhexal ibuprofen	Unknown	/	199607-2008 -	
4 SIMVASTATIN simvastatin	Oral	/	-2008 -	
5 SPALT LIQUA ibuprofen	Unknown	/ Capsule	-2008 -	

Drugs (concomitant)

6 canesten-hc clotrimazole; hydrocortisone	Topical	/	20070104- -	Psoriasis
7 clonistada clonidine hydrochloride	Oral	2 Df dosage form/Day	199503- -	
8 METHOTREXATE methotrexate	Unknown	10 Mg milligram(s)/Week	199704- -	
9 MICARDIS telmisartan	Oral	1 Df dosage form/Day	199507- -	
10 TRIAMPUR COMPOSITUM hydrochlorothiazide; triamterene	Oral	1 Df dosage form/Day	199201- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Aortic stenosis			Yes	grade II, caused by degeneration
Cigarette smoker			Yes	currently 2-3 daily, in the past 10-15 cigarettes
Hypertension arterial			Yes	
Splenomegaly			Yes	borderline
Diabetes mellitus			Yes	
Psoriatic arthritis			Yes	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Serum ferritin	20080725	379	ug/L	
HbA1C	20080725	6.52	%	
Carcinoembryonic antigen	20080725	6.7	ug/L	

Narratives (nur bei elektronischem Eingang)Whitehall Much GmbH, 11.09.2008 DE-WYE-G02100208

Sender diagnosis: not reported

Narrative

Information was received from an investigator in a post marketing surveillance study regarding a 56-year-old male participant in a study entitled "Observational study with Enbrel (Etanercept) in patients with Psoriasis." who received Enbrel (etanercept injection) therapy and experienced drug-induced acute toxic hepatitis.

MEDICAL HISTORY:

The patient's concurrent illnesses include smoker (currently 2-3 daily, in the past 10-15 cigarettes), hypertension, splenomegaly (borderline) and aortic stenosis (grade II, caused by degeneration). The patient occasionally drinks beer.

PRODUCT DETAILS:

Indication for Enbrel was psoriasis. Therapy began in Feb-2008 and was permanently discontinued on 11-Jul-2008. Dose regimen was 50mg (subcutaneous).

Additional suspect medication included Spalt Liqua (ibuprofen capsule, liquid filled) and Simvastatin.

CONCOMITANT THERAPY:

Concomitant therapy included Triampur Compositum (hydrochlorothiazide/triamterene), Clonistada (clonidine hydrochloride) and Micardis (telmisartan).

EVENT DETAILS:

The patient was hospitalized on 25-Aug-2008 after having experienced increase of transaminases since initiation of Enbrel in Feb-2008. He did not experience asthenia, tiredness or pain. Autoimmune etiology and congenital metabolic disorders could be ruled out. The patient has no regular alcohol consumption, also was viral hepatitis in scope of hepatitis A, B, C or E not found. So the conclusion was that the patient experienced drug-induced acute toxic hepatitis (hepatitis toxic) caused by Enbrel. Differential diagnostically other hepatotoxic drugs as Ibuprofen or Simvastatin had to be taken into consideration as they also might have contributed to increase of hepatic enzymes. It was recommended to resume Simvastatin therapy after normalisation of hepatic enzymes. Patient's stay in hospital was without complications, he was discharged on 05-Aug-2008 and was recovering.

TEST RESULTS:

On 25-Jul-2008 test results were: international normalised ratio (results: 0.92); red blood cell count (results: 4.19); autoantibody test (results: ANA positive: 1:100; negative ANCA, AMA, ASMA, LKM, RNP, RNP-SM-antibody, SM-antibody, LA-antibody, SCL 70 antibody, CENPB-antibody, Jo1-antibody, fluorescence pattern homogenous, nucleosid antibody, double chain-DNA, LA negative); carcinoembryonic antigen (results: 6.7 ug/L); laboratory test (results: cancer antigen 72-4: 15.2); aspartate aminotransferase (results: 9.5 umol/sL); alanine aminotransferase (results: 16.87 umol/sL); gamma-glutamyltransferase (results: 5.2 umol/sL); blood lactate dehydrogenase (results: 4.88 umol/sL); serum ferritin (results: 379 ug/L); and glycosylated haemoglobin (results: 6.52 %). On 05-Aug-2008 test results were: alanine aminotransferase (results: 10.33 umol/sL); aspartate aminotransferase (results: 4.6 umol/sL); and gamma-glutamyltransferase (results: 4.52 umol/sL). Additionally, serology test (results: infection serology: Anti-HBc total: non reactive, HBs-Ag: non reactive, anti-HBs <2.0, anti-HCV: non reactive; hepatitis A: HAV total: non reactive; hepatitis C: RNA pending) was done on an unspecified date.

RELATEDNESS ASSESSMENT:

The investigator considered hepatitis toxic possibly related to the study product.

Wyeth-Pharma GmbH, 16.09.2008 DE-WYE-G02100208

Sender diagnosis: not reported

Narrative

Additional information was received regarding patient demographics, patient medical history, lab results and product details including clarification of Ibuprofen involved. Information was received from an investigator in a post marketing surveillance study regarding a 56-year-old male participant in a study entitled "Observational study with Enbrel (Etanercept) in patients with Psoriasis." who received Enbrel (etanercept injection) therapy and experienced drug-induced acute toxic hepatitis.

MEDICAL HISTORY:

The patient's concurrent illnesses include smoker (currently 2-3 daily, in the past 10-15 cigarettes), hypertension, splenomegaly (borderline) and aortic stenosis (grade II, caused by degeneration), diabetes mellitus and psoriatic arthropathy. Past therapies included Cyclosporine and Humira (adalimumab). The patient occasionally drinks beer. Previous therapies of psoriasis also included fumarates and PUVA. There are other cases of psoriasis in patient's family.

PRODUCT DETAILS:

Indication for Enbrel was psoriasis. The dose regimen was 50 mg 2 times per week (subcutaneous) from Feb-2008 to Apr-2008 and then was 50 mg 1 time per week (subcutaneous) from 24-Apr-2008 to 11-Jul-2008. Therapy was permanently discontinued.

Additional suspect medication included Ibuhexal (ibuprofen) and Simvastatin.

CONCOMITANT THERAPY:

Concomitant therapy included Triampur Compositum (hydrochlorothiazide/triamterene), Clonistada (clonidine hydrochloride), Micardis (telmisartan), Methotrexate and Canesten-Hc (clotrimazole/hydrocortisone).

EVENT DETAILS:

The patient was hospitalized on 25-Aug-2008 after having experienced increase of transaminases since initiation of Enbrel in Feb-2008. He did not experience asthenia, tiredness or pain. Autoimmune etiology and congenital metabolic disorders could be ruled out. The patient has no regular alcohol consumption, also was viral hepatitis in scope of hepatitis A, B, C or E not found. So the conclusion was that the patient experienced drug-induced acute toxic hepatitis (hepatitis toxic) caused by Enbrel. Differential diagnostically other hepatotoxic drugs as Ibuprofen or Simvastatin had to be taken into consideration as they also might have contributed to increase of hepatic enzymes. It was recommended to resume Simvastatin therapy after normalisation of hepatic enzymes. Patient's stay in hospital was without complications, he was discharged on 05-Aug-2008 and was recovering.

TEST RESULTS:

Aspartate aminotransferase (results: 0.43 umol/sL) was done on 28-Jan-2008. Aspartate aminotransferase (results: 1.18 umol/sL) was done on 25-Feb-2008. Aspartate aminotransferase (results: 0.52 umol/sL) was done on 24-Apr-2008. Aspartate aminotransferase (results: 7.2 umol/sL) was done on 10-Jul-2008. On 25-Jul-2008 test results were: blood lactate dehydrogenase (results: 4.88 umol/sL); serum ferritin (results: 379 ug/L); glycosylated haemoglobin (results: 6.52 %); laboratory test (results: cancer antigen 72-4: 15.2); carcinoembryonic antigen (results: 6.7 ug/L); autoantibody test (results: ANA positive: 1:100; negative ANCA, AMA, ASMA, LKM, RNP, RNP-SM-antibody, SM-antibody, LA-antibody, SCL 70 antibody, CENPB-antibody, Jo1-antibody, fluorescence pattern homogenous, nucleosid antibody, double chain-DNA, LA negative); gamma-glutamyltransferase (results: 5.2 umol/sL); alanine aminotransferase (results: 16.87 umol/sL); aspartate aminotransferase (results: 9.5 umol/sL); international normalised ratio (results: 0.92); and red blood cell count (results: 4.19). On 05-Aug-2008 test results were: alanine aminotransferase (results: 10.33 umol/sL); aspartate aminotransferase (results: 4.6 umol/sL); and gamma-glutamyltransferase (results: 4.52 umol/sL). Additionally, serology test (results: infection serology: Anti-HBc total: non reactive, HBs-Ag: non reactive, anti-HBs <2.0, anti-HCV: non reactive; hepatitis A: HAV total: non reactive; hepatitis C: RNA pending) was done on an unspecified date.

RELATEDNESS ASSESSMENT:

The investigator considered hepatitis toxic possibly related to the study product.

Wyeth-Pharma GmbH, 29.09.2008 DE-WYE-G02100208

Sender diagnosis: not reported

Narrative

A new version was created after receipt of information regarding the outcome of the event, start date of hospitalization was corrected. Information was received from an investigator in a post marketing surveillance study regarding a 56-year-old male participant in a study entitled "Observational study with Enbrel (Etanercept) in patients with Psoriasis." who received Enbrel (etanercept injection) therapy and experienced drug-induced acute toxic hepatitis.

MEDICAL HISTORY:

The patient's concurrent illnesses include smoker (currently 2-3 daily, in the past 10-15 cigarettes), hypertension, splenomegaly (borderline) and aortic stenosis (grade II, caused by degeneration), diabetes mellitus and psoriatic arthropathy. Past therapies included Cyclosporine and Humira (adalimumab). The patient occasionally drinks beer. Previous therapies of psoriasis also included fumarates and PUVA. There are other cases of psoriasis in patient's family.

PRODUCT DETAILS:

Indication for Enbrel was psoriasis. The dose regimen was 50 mg 2 times per week (subcutaneous) from Feb-2008 to Apr-2008 and then was 50 mg 1 time per week (subcutaneous) from 24-Apr-2008 to 11-Jul-2008. Therapy was permanently discontinued. Additional suspect medication included Ibuprofen (ibuprofen) and Simvastatin.

CONCOMITANT THERAPY:

Concomitant therapy included Triampur Compositum (hydrochlorothiazide/triamterene), Clonistada (clonidine hydrochloride), Micardis (telmisartan), Methotrexate and Canesten-Hc (clotrimazole/hydrocortisone).

EVENT DETAILS:

The patient was hospitalized on 25-Jul-2008 after having experienced increase of transaminases since initiation of Enbrel in Feb-2008. He did not experience asthenia, tiredness or pain. Autoimmune etiology and congenital metabolic disorders could be ruled out. The patient has no regular alcohol consumption, also was viral hepatitis in scope of hepatitis A, B, C or E not found. So the conclusion was that the patient experienced drug-induced acute toxic hepatitis (hepatitis toxic) caused by Enbrel. Differential diagnostically other hepatotoxic drugs as Ibuprofen or Simvastatin had to be taken into consideration as they also might have contributed to increase of hepatic enzymes. It was recommended to resume Simvastatin therapy after normalisation of hepatic enzymes. Patient's stay in hospital was without complications, he was discharged on 05-Aug-2008. Investigator reported that the outcome was unknown because it was impossible to contact the patient again.

TEST RESULTS:

Aspartate aminotransferase (results: 0.43 umol/sL) was done on 28-Jan-2008. Aspartate aminotransferase (results: 1.18 umol/sL) was done on 25-Feb-2008. Aspartate aminotransferase (results: 0.52 umol/sL) was done on 24-Apr-2008. Aspartate aminotransferase (results: 7.2 umol/sL) was done on 10-Jul-2008. On 25-Jul-2008 test results were: blood lactate dehydrogenase (results: 4.88 umol/sL); serum ferritin (results: 379 ug/L); glycosylated haemoglobin (results: 6.52 %); laboratory test (results: cancer antigen 72-4: 15.2); carcinoembryonic antigen (results: 6.7 ug/L); autoantibody test (results: ANA positive: 1:100; negative ANCA, AMA, ASMA, LKM, RNP, RNP-SM-antibody, SM-antibody, LA-antibody, SCL 70 antibody, CENPB-antibody, Jo1-antibody, fluorescence pattern homogenous, nucleosid antibody, double chain-DNA, LA negative); gamma-glutamyltransferase (results: 5.2 umol/sL); alanine aminotransferase (results: 16.87 umol/sL); aspartate aminotransferase (results: 9.5 umol/sL); international normalised ratio (results: 0.92); and red blood cell count (results: 4.19). On 05-Aug-2008 test results were: alanine aminotransferase (results: 10.33 umol/sL); aspartate aminotransferase (results: 4.6 umol/sL); and gamma-glutamyltransferase (results: 4.52 umol/sL). Additionally, serology test (results: infection serology: Anti-HBc total: non reactive, HBs-Ag: non reactive, anti-HBs <2.0, anti-HCV: non reactive; hepatitis A: HAV total: non reactive; hepatitis C: RNA pending) was done on an unspecified date.

RELATEDNESS ASSESSMENT:

The investigator considered hepatitis toxic possibly related to the study product.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08109268

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth:	Age: 60 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic drug-induced	20080703			1 Day	recovered/resolved
2 Hepatocellular damage	20080703			1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal	Oral	1200 Mg milligram(s)/Day	20080703-20080725	
ibuprofen		Film-coated tablet	23 Day	

Drugs (concomitant)

2 PANTOZOL	Oral	40 Mg milligram(s)/	-	
pantoprazole sodium			-	

Primary source

Qualification: Pharmacist observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 15.09.2008 DE-NOVNCHHQ-GXKR2008DE08678

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 08 Sep 2008 from the drug commission of German pharmacists (AMK no. 140331), reported by a pharmacist: This patient with an unknown medical history developed acute toxic drug-induced hepatitis with hepatocellular damage during treatment with IbuHEXAL 600 (ibuprofen). Concomitantly the patient received Pantozol (pantoprazol). Treatment with IbuHEXAL 600 was discontinued and the condition resolved. The reporter provided no causality assessment.

Serious spontaneous report of hepatitis toxic (medically significant), already listed in the Core Data Sheet Information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:NOVARTIS, 16.09.2008 DE-SZ09-GXKR2008DE08678

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 08 Sep 2008 from the drug commission of German pharmacists (AMK no. 140331), reported by a pharmacist: This patient with an unknown medical history developed acute toxic drug-induced hepatitis with hepatocellular damage during treatment with IbuHEXAL 600 (ibuprofen). Concomitantly the patient received Pantozol (pantoprazol). Treatment with IbuHEXAL 600 was discontinued and the condition resolved. The reporter provided no causality assessment.

Serious spontaneous report of hepatitis toxic (medically significant), already listed in the Core Data Sheet Information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08119197

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19460725	Age: 62 Year	weight (kg): 96	BMI_int: 29.96	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis granulomatous	200808				recovering/resolving
2 Fatty liver	200808				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AROMASIN exemestane	Oral	/ Tablet	2004-	Breast cancer
2 CELEBREX celecoxib		/	-	
3 ibuprofen ibuprofen		/	-	Drug use for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain in extremity			Unknown	
Radiotherapy			No	
Breast cancer	2004		Unknown	pT1521

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Body temperature	200808			
Liver function tests raised	200808			
Aspartate aminotransferase	200808	203	U/L	
Alanine aminotransferase	200808	268	U/L	
Gamma GT	200808	994	U/L	
Alkaline phosphatase	200808	293	U/L	
C-reactive protein	200808	0.7		
Liver biopsy	20080901			

Narratives (nur bei elektronischem Eingang)

PFIZER INC, 10.10.2008 DE-PFIZER INC-2008084364

Sender diagnosis: not reported

Narrative

This physician reported that a 62-year-old female patient started AROMASIN (exemestane) in 2004 due to invasive breast cancer (pT1521), first diagnosed in 2004. She was also taking CELEBREX (celecoxib) and Ibuprofen. The patient has had radiotherapy. In Aug2008, she developed recurrent fever and increased liver function values. She was hospitalised and a diagnosis of florid granulomatous hepatitis and fatty liver (30% parenchymatic volume) with evidence of Mallory bodies was made by liver puncture biopsy. Laboratory test results were GOT 203 U/l, GPT 268 U/l, Gamma GT 994 U/l, AP 293 U/l and CRP 0.7. Histological findings of 1Sep2008 from the pathologist revealed numerous, lacking of developed lobular but also portal granulomas. There was no canalicular cholestasis. There were large degenerative fatty liver cells of 30% parenchymatic volume with

isolated Mallory bodies. There was no siderosis. The patient was diagnosed with unclarified increased liver function values of cholestatic type. No virus marker was seen. AROMASIN was not discontinued due to the adverse events. Action taken with CELEBREX and Ibuprofen was unknown. At the time of reporting, the adverse events have not resolved.

Per the pathologists' comments, firstly, none of the patient's medication is part of the list of drugs that are commonly known to cause liver granulomatosis. Nevertheless, from the patient's drugs, Ibuprofen could be the most likely cause of the adverse events and if so, it is best to discontinue it. Secondly, the combination of fats (30%) with Mallory bodies (in the sense of a slight distinct steatohepatitis, respectively in the pre stage) is very characteristic of a liver damage through anti-oestrogens (Tamoxifen, Aromatase-inhibitors). The pathologists recommended to discontinue AROMASIN. Both the adverse events are probably of the same clinical importance. Histologically, sarcoidosis is unlikely. The reporting physician considered a probable causal relationship of the adverse events with AROMASIN, CELEBREX and Ibuprofen.

Pfizer is not the Marketing Authorization Holder of ibuprofen in the country of origin of the first author of this literature article.

This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen submitted the same report to the regulatory authorities.

Company Clinical Evaluation

Granulomatous hepatitis is a poorly defined term that is generally used to describe an illness characterized by cholestatic liver enzyme abnormalities and granulomatous inflammatory changes as the predominant histologic manifestation. Granuloma formation is the response of the immune system to the presence of foreign substances that are nondigestible by the enzymes of inflammatory cells and macrophages. Such substances are typically derived from infectious organisms (e.g., mycobacteria and schistosomes) or from injected or ingested material (e.g., talc and mineral oil). The response may also be elicited by hypersensitivity reactions to drugs and toxins. As for the fatty liver, this is due to different causes (e.g., metabolic, nutritional, drug and toxin induced).

This patient was diagnosed with granulomatous hepatitis and fatty liver about four years after AROMASIN treatment start. It was reported that she was concomitantly taking CELEBREX and Ibuprofen. It is worth noting that hepatotoxicity has been described with the administration of ibuprofen and hepatitis and liver failure have been reported with the use of CELEBREX. The reporting physician underlined that the patient's clinical picture was characterized by unclarified increased liver function values of cholestatic type, and no virus marker was seen. At the time of reporting no action was taken with AROMASIN in response to the events. As for the other two suspect drugs, this information is not reported.

On the basis of the available information, the possibility that AROMASIN might have played a role in the onset of the reported events cannot be completely excluded at this point in time. However, a possible contribution of CELEBREX and Ibuprofen should not be overlooked.

McNeil Germany, Local Safety Officers, 15.10.2008 DE-JNJFOC-20081002791

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician via a business partner (Pfizer MI390523) and concerns a 62-year-old female from Germany: 02308/08.

The patient's height was 179 cm and weight was 96 kilograms. The patient's medical history and concurrent conditions included: invasive breast cancer diagnosed in 2004 (pT1521) and radiotherapy (for breast cancer). The patient was treated with ibuprofen (unspecified, unspecified route), dosage, frequency, therapy dates, and indication unknown. Non-company suspect drugs included exemestane for breast cancer and celecoxib for an unknown indication. In AUG-2008, the patient developed recurrent fever and increased liver function values. Her symptoms were serious and she was hospitalized, where a diagnosis of florid granulomatous hepatitis and fatty liver (30% parenchymatic volume) with evidence of Mallory bodies was made by liver puncture biopsy. Laboratory testing performed in AUG 2008 revealed an alkaline phosphatase (AP) 293 IU/L, alanine transaminase (ALT) 268 IU/L, aspartate transaminase (AST) 203 IU/L, C-reactive protein 0.7, gamma glutamyl transferase (GGT) 994 IU/L. Histological findings from the liver biopsy on 01-SEP-2008 revealed numerous, undeveloped lobular and portal granulomas. There was no canalicular cholestasis. There were large degenerative fatty liver cells of 30% parenchymatic volume with isolated Mallory bodies. No virus marker was seen. There was no siderosis. The dose of exemestane was not changed and action taken with ibuprofen and celecoxib was unknown. At the time of the report, the patient had not recovered from granulomatous hepatitis and fatty liver. The pathologist commented that none of the patient's medications was commonly known to cause liver granulomatosis, but ibuprofen could be the most likely to cause the events. Also, the combination of fats (30%) with Mallory bodies (in the sense of a slight distinct steatohepatitis, respectively in the pre-stage) was very characteristic of liver damage through anti-estrogens (Tamoxifen, Aromatase-Inhibitors). Histologically, sarcoidosis was unlikely. The pathologist recommended that ibuprofen and exemestane be discontinued. The reporting physician considered a probable causal relationship of the adverse events with exemestane, ibuprofen, and celecoxib. This report was serious (hospitalization).

PFIZERINC, 31.12.2008 DE-PFIZER INC-2008084364

Sender diagnosis: not reported

Narrative

This physician reported that a 62-year-old female patient started AROMASIN (exemestane) in 2004 due to invasive breast cancer (pT1521), first diagnosed in 2004. She was also taking CELEBREX (celecoxib) and Ibuprofen. The patient had radiotherapy. In Aug2008, she developed recurrent fever and increased liver function values. She was hospitalised and a diagnosis of florid granulomatous hepatitis and fatty liver (30% parenchymatic volume) with evidence of Mallory bodies was made by liver puncture biopsy. Laboratory test results were GOT 203 U/l, GPT 268 U/l, Gamma GT 994 U/l, AP 293 U/l and CRP 0.7. Histological findings of 01Sep2008 from the pathologist revealed numerous lacking of developed lobular but also portal granulomas. There was no canalicular cholestasis. There were large degenerative fatty liver cells of 30% parenchymatic volume with isolated Mallory bodies. There was no siderosis. The patient was diagnosed with unclarified increased liver function values of cholestatic type. No virus marker was seen. AROMASIN was not discontinued due to the adverse events. Action taken with CELEBREX and Ibuprofen was unknown. At the time of reporting, the adverse events have not resolved. Per the pathologists' comments, firstly, none of the patient's medication is part of the list of drugs that are commonly known to cause liver granulomatosis. Nevertheless, from the patient's drugs, Ibuprofen could be the most likely cause of the adverse events and if so, it is best to discontinue it. Secondly, the combination of fats (30%) with Mallory bodies (in the sense of a slight distinct steatohepatitis, respectively in the pre stage) is very characteristic of a liver damage through anti-oestrogens (Tamoxifen, Aromatase-inhibitors). The pathologists recommended to discontinue AROMASIN. Both the adverse events are probably of the same clinically importance. Histologically, sarcoidosis is unlikely. The reporting physician considered a probable causal relationship of the adverse events with AROMASIN, CELEBREX and Ibuprofen. The reporting physician additionally stated that other causes such as sarkoidosis or viral infection of the hepatic changes were excluded and therefore the diagnosis of a drug-induced hepatitis was the most likely explanation for the patient's clinical symptoms. This assumption was at least partly confirmed by the above mentioned pathohistological findings. He stated that attacks of fever (the patient had fever up to 39 degree Celsius) are not really typical for a drug-induced hepatitis, but may occur. The reporter admitted that a causality assessment is difficult, since the patient besides AROMASIN additionally and irregularly had taken Ibuprofen and Diclofenac as needed to treat the chronic pain in her left foot. The previously documented intake of CELEBREX was no longer mentioned, and it seems that the patient had taken Diclofenac instead. The reporter stated that the duration of each intake was short, but the dosage was unknown. The reporter further stated that a causal involvement of AROMASIN must be assumed (at least as an important co-factor within a multifactorial origin of hepatitis), since the patient's condition rapidly improved after withdrawal of AROMASIN with normalization of the liver function tests, except for a remaining slightly elevation of the GammaGT until 02Oct2008 and with no recurrence of fever attacks. As documented by the reporting physician, the patient's gynecologist/oncologist did not expect a significant therapeutic benefit of further AROMASIN-treatment for this patient and therefore, no re-challenge was undertaken. AROMASIN remained permanently discontinued.

Pfizer is not the Marketing Authorization Holder of ibuprofen and Diclofenac in the country of origin of the reporter for this case.

This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen and Diclofenac submitted the same report to the regulatory authorities.

Follow-up (02Dec2008):

Further information has been received regarding the most likely cause of the adverse events, causality assessment with suspect drugs, clinical outcome, and action taken with study drug AROMASIN.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08124044

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: 19880207	Age: 19 Year	weight (kg): 70	BMI_int: 23.39	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver enzyme abnormal	20071103			10 Day	recovering/resolving
2 Hepatitis acute	20071103			10 Day	recovering/resolving
3 Hepatitis acute	20071103			10 Day	recovered/resolved
4 Autoimmune hepatitis	20071103			10 Day	recovering/resolving
5 Stomach pain	20071103			10 Day	recovering/resolving
6 Vomiting	20071103			10 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibuprofen, 800 mg, Tabl, NN ibuprofen	Oral	3200 Mg milligram(s)/Day	20071025-20071211 48 Day	Back pain

Drugs (concomitant)

2 Aspirin 100 N, 100 mg, Tabl, Bayer acetylsalicylic acid	Oral	100 Mg milligram(s)/Day 245	20070207-20071025 8 Month	Migraine
3 Humalog, 300 IE, Amp, Lilly insulin lispro	Subcutaneous	60 lu international unit(s)/l 120	2005- -	Type 1 diabetes mellitus
4 Huminsulin Basal, Amp, Lilly insulin human	Subcutaneous	18 lu international unit(s)/l 120	2005-20071223 -	Type 1 diabetes mellitus

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
House dust allergy			Unknown	house dust allergy
Cigarette smoker			Unknown	smoker (since 2000, at least 25 cigarettes daily)
Asthma bronchial			Unknown	bronchial asthma
Hepatomegaly			Unknown	hepatomegaly
Allergy to animals			Unknown	animal hair (cat, hare) allergy
Type 1 diabetes mellitus			Unknown	entgleist
Hyperglycaemia NOS			Unknown	hyperglycemic crisis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20071103	392	U/l	
Aspartate aminotransferase increased	20071103	199	U/l	
Bilirubin normal	20071103	0.8	mg/dl	
Alanine aminotransferase increased	20071214	685	U/l	
Aspartate aminotransferase increased	20071214	1002	U/l	
Bilirubin normal	20071214	0.76	mg/dl	
Alanine aminotransferase increased	20071217	924	U/l	

Aspartate aminotransferase increased	20071217	1378	U/l
Bilirubin normal	20071217	0.76	mg/dl
Alanine aminotransferase increased	20071221	742	U/l
Aspartate aminotransferase increased	20071221	510	U/l
Bilirubin normal	20071221	0.53	mg/dl
Alanine aminotransferase increased	20071224	346	U/l
Aspartate aminotransferase increased	20071224	160	U/l
Bilirubin normal	20071224	0.49	mg/dl
Alanine aminotransferase increased	20080429	108	U/l
Aspartate aminotransferase increased	20080429	60	U/l
Alkaline phosphatase	20071217	369	U/L
Alkaline phosphatase	20071228	195	U/L
Alkaline phosphatase	20071219	405	U/L
Bilirubin total	20071224	0.49	MG/DL
Bilirubin total	20071221	0.53	MG/DL
Bilirubin total	20071228	0.52	MG/DL
Bilirubin total	20071217	0.76	MG/DL
Bilirubin total	20071214	0.76	MG/DL
Bilirubin total	20071103	0.8	MG/DL
Gamma-glutamyltransferase	20071217	766	U/L
Gamma-glutamyltransferase	20071219	741	U/L
Gamma-glutamyltransferase	20071228	390	U/L
Hepatitis A antibody		negative	/1
Hepatitis A antibody		negative	/1
SGOT	20071219	1194	U/L
SGOT	20071103	199	U/L
SGOT	20071228	144	U/L
SGOT	20071214	1002	U/L
SGOT	20071221	510	U/L
SGOT	20080429	60	U/L
SGOT	20071217	1378	U/L
SGOT	20071224	160	U/L
SGPT	20071103	392	U/L
SGPT	20080429	108	U/L
SGPT	20071228	244	U/L
SGPT	20071219	913	U/L
SGPT	20071224	346	U/L
SGPT	20071221	742	U/L
SGPT	20071217	924	U/L
SGPT	20071214	685	U/L
Lipase	20071219	721	U/L
Lipase	20071228	53	U/L
Alanine aminotransferase increased	20071218	926	U/l

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 22.10.2008 DE-FAKOS-170155

Sender diagnosis: not reported

Narrative

Die UAW hat sich ambulant entwickelt. Ende der Einnahme von Ibuprofen am 11.12.2007. Im weiteren Verlauf Rückgang der erhöhten Transaminasen. Die spezifische Therapie der Hepatitis mit hochdosiertem Prednisolon maskiert das Dechallenge. Ibuprofen Tabl, ist spezifisch gelabelt bei Ratiopharm: "Hepatitis", Stand: 11/2007.

Sender's comment:

Reporter's comment:

Bayer Vital GmbH, 28.10.2008 DE-BAYER-200829113GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 23-Oct-2008 from a physician in Germany via FAKOS, a regional pharmacovigilance center in Germany Medical Association, via an internet-based pool of cases of BfArM, the regulatory health authority in Germany, referring to a male (age: unknown, height: 173 cm, weight: 70 kg).

Patient is a smoker (since 2000, at least 25 cigarettes daily), suffers from allergies: house dust, animal hair (cat, hare), bronchial asthma (patient declaration), and insulin-dependent diabetes mellitus.

Patient received concomitant drugs (refer to relevant section in this report for details).

Patient received the following drug reported as 'suspect drug':

--oral Ibuprofen 800 mg tablet (trade name was not provided) four times a day for back pain from 25-Oct-2007 to 11-Dec-2007.

On 03-Nov-2007, patient experienced ACUTE HEPATITIS, considered serious by the reporter due to requiring hospitalization.

The adverse event developed in the outpatient. Ibuprofen was withdrawn on 11-Dec-2007, and following withdrawal raised transaminases decreased.

The event was treated with high-dose Prednisolon, that masked dechallenge.

The event was resolving at the time of the report.

The reporter assessed the diagnosis ACUTE HEPATITIS to be possibly related to Ibuprofen.

NOVARTIS, 29.10.2008 DE-NOVNCHHQ-GXKR2008DE10079

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 24 Oct 2008 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-08124044) provided by the PVZ-FAKOS, Charite Universitätsmedizin Berlin, reported by a physician: This patient with a medical history of type I diabetes mellitus developed acute hepatitis during treatment with ibuprofen (MAH unknown). Concomitantly other drugs were taken. Ibuprofen was withdrawn and the hepatitis was treated with strong prednisolone. The outcome of the adverse event was reported as recovering. The reporter appraised the causality of the adverse event and the administration of ibuprofen as possible.

Serious spontaneous report (hospitalisation) of Hepatitis acute, already listed in the Core Data Sheet information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Bayer Vital GmbH, 05.11.2008 DE-BAYER-200829113GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 23-Oct-2008 from a physician in Germany via FAKOS, a regional pharmacovigilance center in Germany Medical Association, via an internet-based pool of cases of BfArM, the regulatory health authority in Germany, referring to a male (age: unknown, height: 173 cm, weight: 70 kg).

Patient is a smoker (since 2000, at least 25 cigarettes daily), suffers from allergies: house dust, animal hair (cat, hare), bronchial asthma (patient declaration), and insulin-dependent diabetes mellitus.

Patient received concomitant drugs (refer to relevant section in this report for details).

Patient received the following drug reported as 'suspect drug':

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On 03-Nov-2007, patient experienced ACUTE HEPATITIS, considered serious by the reporter due to requiring hospitalization.

The adverse event developed in the outpatient. Ibuprofen was withdrawn on 11-Dec-2007, and following withdrawal raised transaminases decreased.

The event was treated with high-dose Prednisolon, that masked dechallenge.

The event was resolving at the time of the report.

The reporter assessed the diagnosis ACUTE HEPATITIS to be possibly related to Ibuprofen.

**Follow-up information received on 30-OCT-2008:

Hospital report was provided (hospitalization from 14-Dec-2007 to 29-Dec-2007):

--Diagnoses:

-AUTOIMMUNE HEPATITIS, possibly drug-induced (maximum dosage of ibuprofen at 2 400 mg daily taken for four weeks).

-Diabetes mellitus type-1 requiring insulin treatment with hyperglycemic crisis.

The 19 year-old patient had been admitted for further diagnostic measures and therapy for a relapsing emesis, an increasingly worsening general condition, and distinctly increased liver values.

Sonography, performed on 14-Dec-2007, revealed a distinct fatty liver (a maximum size of 21 cm in the mid-clavicular line).

In the sonography, performed on 20-Dec-2007, liver had increased to 22 cm in the mid-clavicular line.

--Course of the disease and therapy:

Patient had been admitted to hospital due to a suspected drug-induced hepatitis. Main symptoms on admission were fatigue, relapsing emesis, and abdominal complaints. Patient was suffering from a well-known diabetes mellitus type-1 which had developed into a hyperglycemic crisis. Laboratory testing revealed severely increased liver values (glutamat pyruvat transaminase 924 U/l, glutamat oxalacetat transaminase 1378 U/l, gamma glutamyl transferase 823, alkaline phosphatase 457 U/l). Specialized analysis suggested an autoimmune hepatitis, possibly induced by intake of ibuprofen for four weeks for treatment of back pain.

Upon high dosage treatment with prednisolon decrease of liver values was observed.

Laboratory findings: hepatitis A-virus IgG-AK - negative, hepatitis A-virus-IgM-AK - negative, for further laboratory findings see relevant section.

PVZ-FAKOS, Klinische Pharmakologie, 21.07.2009 DE-FAKOS-170155

Sender diagnosis: not reported

Narrative

Vorstellung des Patienten in der Rettungsstelle eines Krankenhauses am 03.11.2007 aufgrund von Bauchschmerzen und rezidivierendem Erbrechen. Der Patient ist ein primär insulinabhängiger Diabetiker und hatte sich am Vorabend bei einem Blutzuckerwert von 14,1 mmol/l die übliche Standarddosis gespritzt. Im Krankenhaus wurden eine Hepatomegalie und erhöhte Transaminasenwerte festgestellt (ALAT 392 U/l (Normbereich: <45 U/l), ASAT 199 U/l (Normbereich: <50 U/l)). Der Patient wurde am selben Tag in die ambulante Betreuung entlassen.

Am 14.12.2007 wurde der Patient erneut in der Rettungsstelle vorstellig, allerdings in einem anderen Krankenhaus. Grund waren rezidivierendes Erbrechen und zunehmende Allgemeinzustandsverschlechterung. Bei Aufnahme wurden deutlich erhöhte Leberwerte festgestellt (ALAT: 685 U/l und ASAT 1002 U/l). Die serologische Diagnostik war negativ bezüglich einer viralen Genese. Die Testung auf Autoantikörper war stark positiv für antinukleäre Antikörper und antimitochondriale Antikörper. Im weiteren Verlauf der stationären Behandlung zeigten sich die Maximalwerte von: ALAT 926 U/l (am 18.12.2007) und ASAT 1378 U/l (am 17.12.2007). Unter hochdosierter Applikation von Prednisolon konnte ein Rückgang der Leberwerte registriert werden. Am 29.12.2007 wurde der Patient aus dem Krankenhaus entlassen.

Das als Auslöser der Hepatitis verdächtige Ibuprofen wurde vom 25.10.-11.12.2007 täglich in einer Dosierung von 4x 800 mg eingenommen. Die spezifische Therapie der Hepatitis maskiert ein mögliches Dechallenge. Deshalb wird das Arzneimittel als "möglich" eingestuft.<dbtext>

To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 28.07.2009 DE-NOVNCHHQ-GXKR2008DE10079

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 24 Oct 2008 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-08124044) provided by the PVZ-FAKOS, Charite Universitätsmedizin Berlin, reported by a physician: This patient with a medical history of type I diabetes mellitus developed

acute hepatitis during treatment with ibuprofen (MAH unknown). Concomitantly other drugs were taken. Ibuprofen was withdrawn and the hepatitis was treated with strong prednisolone. The outcome of the adverse event was reported as recovering. The reporter appraised the causality of the adverse event and the administration of ibuprofen as possible.

Significant follow up was received on 22 Jul 2009 from the same reporter with no new relevant information.

reporter's comment:

SANOFI-SYNTHELABO, 05.08.2009 DE-SANOFI-SYNTHELABO-A01200813813

Sender diagnosis: not reported

Narrative

Reaction as reported: hepatitis acute

LLT: HEPATITIS ACUTE / 10019727

PT: HEPATITIS ACUTE / 10019727

Reaction as reported: elevated liver values (ALAT 685 U/l, ASAT 1002 U/l)

LLT: LIVER ENZYME ABNORMAL / 10024677

PT: HEPATIC ENZYME ABNORMAL / 10062685

Reaction as reported: stomach pain

LLT: STOMACH PAIN / 10042112

PT: ABDOMINAL PAIN UPPER / 10000087

Reaction as reported: vomiting

LLT: VOMITING / 10047700

PT: VOMITING / 10047700

Narrative:

Initial information received from the website of the Health Authorities (DE-BfArM-08124044) via PVZ-FAKOS (case number: DE-FAKOS-170155) through our local affiliate (IBU 06/08) on 03-Nov-2008.

A male patient was treated with ibuprofen in a dose of 4x800 mg in oral form (brand name unknown) for back pain from 25-Oct-2007 to 11-Dec-2007. On 03-Nov-2007 an acute hepatitis was diagnosed. On 11-Dec-2007 ibuprofen was withdrawn. Since 21-Dec-2007 aminotransferase levels were falling and becoming normal. Patient was recovering. Other relevant medical history was: type 1 diabetes mellitus. Concomitant drug were insulin lispro, nearly 3x20 IU/d subcutaneous, no change in dosage; insulin (human), 1x18 IU/d subcutaneous, drug withdrawn on 23-Dec-2007; ASS 1x100 mg/d oral, 07-Feb-2007 to 25-Oct-2007. The reporter's causality assessment was possible.

Additional information received on 23-Jul-2009:

On 03-Nov-2009, the patient was admitted to emergency ward due to vomiting and stomach pain. Elevated liver values were detected (ALAT 392 U/l, ASAT 199 U/l) and an acute hepatitis was diagnosed. Patient was discharged the same day.

On 14-Dec-2007, the patient was admitted to emergency again due to vomiting. Elevated liver values were detected (ALAT 685 U/l, ASAT 1002 U/l). Sereological testing of viral genesis was negative. Patient could be discharged on 29-Dec-2007.

At the time of the reporting, the patient was recovering.

According to the reporter, the reactions are possibly related to ibuprofen.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08124782

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19500426	Age: 58 Year	weight (kg): 62	BMI_int: 23.34	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Non-alcoholic steatohepatitis	200805			4 Year	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUPROFEN (NGX) ibuprofen	Oral	/	2004-	Analgesia
2 methylprednisolone methylprednisolone	Oral	12 Mg milligram(s)/Day	2008-	Arthritis
3 METHYLPREDNISOLONE methylprednisolone	Oral	20 Mg milligram(s)/Day	20080115-2008	Arthritis

Drugs (concomitant)

4 Bisoprolol bisoprolol	Oral	2.5 Mg milligram(s)/Day	-	Drug use for unknown indication
5 diclofenac diclofenac	Unknown	/	-	Arthritis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hepatitis A	1992		No	
Hypertension arterial			Unknown	
Nicotine dependence			Unknown	
Fracture of ankle	2004		No	open talus fracture
Arthritis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Gamma-glutamyltransferase	20080522	unk	U/L	
SGPT	200806	860	U/L	
SGPT	20080522	800	U/L	
Gamma-glutamyltransferase	200806	333	U/L	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 28.10.2008 DE-NOVNCHHQ-GXKR2008DE10128

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 27 Oct 2008 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-08124782) provided by the Drug Commission of the German Medical Association, reported by a physician: This patient with a medical history of hepatitis A (1992), arterial hypertension, nicotine dependence and a open talus fracture in 2004 developed a non-alcoholic steatohepatitis during treatment since 2004 with ibuprofen (MAH unknown) at a daily dosage of 300-600 mg for analgesia. Concomitantly the patient received methylprednisolone for arthritis. The adverse event started on May 2008 and was still on-going at the time of the report. The reporter appraised the causality of the adverse event and the

administration of ibuprofen as possible.
considered suspected for reporting purposes.

Reporter's comment:

Bayer Vital GmbH, 29.10.2008 DE-BAYER-200829321GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 24-Oct-2008 from a physician in Germany via AKDAE, the Drug Commission of the German Medical Association, via an internet-based pool of cases of BfArM, the regulatory health authority in Germany, referring to a 58 year-old female (height: 163 cm, weight: 62 kg).

Patient's concomitant conditions include arterial hypertension and nicotine dependence.
Patient's medical history included hepatitis A in 1992 and open talus fracture in 2004.
Patient received concomitant Methylprednisolone for arthritis since 15-Jan-2008.

Patient received the following drug reported as 'suspect drug':
--oral Ibuprofen (trade name was not provided) at a dose of 300 to 600 mg a day for analgesia from 2004.

In May-2008, four years after start of treatment with Ibuprofen, patient experienced NON-ALCOHOLIC STEATOSIS, considered serious by the reporter due to medical significance.
The event was not resolved at the time of the report.

The reporter assessed the diagnosis NON-ALCOHOLIC STEATOSIS to be possibly related to Ibuprofen.

NOVARTIS, 29.10.2008 DE-NOVNCHHQ-CHNY2008DE03902

Sender diagnosis: not reported

Narrative

Initial physician report received on 24 Oct 2008 via the German Health Authority (BfArM ref: DE-BFARM-08124782). This 58-year-old female patient with a history of hepatitis A in 1992, started ibuprofen (unknown manufacturer) in 2004 for analgesia. She was concomitantly receiving methylprednisolone (unknown manufacturer) since 15 Jan 2008 to treat arthritis. In May 2008, she experienced non-alcoholic steatohepatitis. Final outcome was condition unchanged. The causality was assessed as possible for ibuprofen and also for methylprednisolone.

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard). The event Hepatic steatosis was assessed as unlabelled according to the product information. However other alternative causes (history of hepatitis and concomitant use of methylprednisolone) provide a possible explanation for the reported adverse event.

Reporter's comment:

Bayer Vital GmbH, 17.12.2008 DE-BAYER-200829321GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 24-Oct-2008 from a physician in Germany via AKDAE, the Drug Commission of the German Medical Association, via an internet-based pool of cases of BfArM, the regulatory health authority in Germany, referring to a 58 year-old female (height: 163 cm, weight: 62 kg).

Patient's concomitant conditions include arterial hypertension and nicotine dependence.

Patient's medical history included hepatitis A in 1992 and open talus fracture in 2004.

Patient received concomitant Methylprednisolone for arthritis since 15-Jan-2008.

Patient received the following drug reported as 'suspect drug':

--oral Ibuprofen (trade name was not provided) at a dose of 300 to 600 mg a day for analgesia from 2004.

In May-2008, four years after start of treatment with Ibuprofen, patient experienced NON-ALCOHOLIC STEATOSIS, considered serious by the reporter due to medical significance.

The event was not resolved at the time of the report.

The reporter assessed the diagnosis NON-ALCOHOLIC STEATOSIS to be possibly related to Ibuprofen.

****Follow-up information received on 11-DEC-2008:**

Hospital letter of 30-Jul-2008 submitted by reporting clinician (internist) via Drug Commission of the German Medical Association (AKdÄ) - Dates of hospital stay were not provided:

Diagnoses:

- Suspected non-alcoholic steatohepatitis
- Treatment with methylprednisolone (currently 12mg per day) and diclofenac
- No clues suggesting autoimmune hepatitis
- Undifferentiated arthritis with positive ANA
- Arterial hypertension
- Nicotine abuse (currently 10 cigarettes per day)
- Status post open fracture of the right talus (2004)
- Alcohol consumption ("only on New Year's Eve")

Anamnesis:

In 1992, after having recovered from hepatitis A, patient had been informed about the risk to develop autoimmune hepatitis. In the sequel transaminases and gamma GT had constantly been found mildly increased.

In 2004, subsequently to a traumatic fracture of the talus, patient had taken 300 to 600mg of Ibuprofen per day. A concomitant intake of 2,5mg bisoprolol was reported as well.

Since fall 2007, patient had developed arthritic complaints. Upon self-treatment with 20mg of methylprednisolone (started on 15-Jan-2008), patient's complaints had ameliorized. At time of the report, patient took 12 mg of Methylprednisolone a day.

In early May-2008, check of liver values had showed alterations in the known range.

On 22-May-2008, severe increase in SGPT (up to almost 800U/L) and gamma GT was found. Ibuprofen had been discontinued and the values temporarily improved.

However, since mid-Jun-2008 continual increases of SGPT (most recent value: 860U/L) and gamma GT (most recent value: 333U/L) had been ascertained. Treatment with ursodesoxycholic acid was unsuccessful.

Liver biopsy showed significant fatty degeneration corresponding to moderately progressed steatohepatitis with significant necroinflammatory activity.

Patient presented at the hospital for liver-specific consultation. Patient declared her arthritic complaints currently under control. Apart from that, she reported no further complaints. Patient (height: 163 cm, weight: 2 kg) lost 4 kg of weight since May-2008.

Based on both the synopsis and the slightly elevated liver values the reporting clinician considered a long-term non-alcoholic steatosis hepatitis without inflammatory activity. No clues suggesting autoimmune hepatitis could be detected. Regarding the patient's current condition the clinician diagnosed genuine STEATOHEPATITIS, probably based on treatment with methylprednisolone and possibly aggravated by treatment with non-steroidal antirheumatics. Additionally, potential aggravation caused by non-steroidal anti-inflammatory drugs was taken into consideration. With slow dose reduction of the steroids the clinician expected further improvement. Regarding the steroid-dependent arthritis, careful basic treatment with hydrochloroquin was considered as well.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08124748

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials: Sex: Male Birth: 19381001 Age: 70 Year weight (kg): 88 BMI_int: 25.71 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Colonic haemorrhage	20081017	20081020	4 Day	9 Day	recovered/resolved
2 Bloody stool					
3 Hemoglobin decreased					
4 Renal function tests NOS abnormal					recovering/resolving
5 GGT increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal	Oral	600 Mg milligram(s)/Day Tablet	20081013-20081017 5 Day	Drug use for unknown indication
2 Novaminsulfon	Oral	80 Df dosage form/Day Oral drops	20081015-20081017 3 Day	Drug use for unknown indication
3 Xarelto	Oral	10 Mg milligram(s)/Day Tablet	20081013-20081017 5 Day	Knee arthroplasty

Drugs (concomitant)

4 Aprovel	Unknown	150 Mg milligram(s)/Day	-	Drug use for unknown indication
irbesartan			-	
5 eryfer	Unknown	/Day	-	Drug use for unknown indication
ascorbic acid; ferrous sulfate; sodium bicarbonate		Capsule	-	
6 Glucophage	Unknown	600 Mg milligram(s)/Day	-	Drug use for unknown indication
metformin			-	
7 Glucophage	Unknown	1000 Mg milligram(s)/Day	-	Drug use for unknown indication
metformin			-	
8 Heparin	Subcutaneous	15000 Iu international unit	-	Drug use for unknown indication
heparin			-	
9 Pantozol	Unknown	20 Mg milligram(s)/Day	-	Drug use for unknown indication
pantoprazole sodium			-	
10 Valoron	Unknown	150 Mg milligram(s)/Day	20081014-20081017 4 Day	Drug use for unknown indication
tilidine hydrochloride				

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Gonarthrosis			k.A.	gonarthrosis
Weakness generalised			k.A.	general weakness
Type II diabetes mellitus			k.A.	diabetes mellitus type II
Syncope			k.A.	syncope
Sick sinus syndrome			k.A.	sick sinus syndrome
SA block			k.A.	intermittent SA block
Rectal bleeding			k.A.	loss of fresh blood
Pain in leg			k.A.	pain in right leg
Nausea			k.A.	nausea
Hypertension arterial			k.A.	arterial hypertension
Fatty liver			k.A.	developing fatty liver cirrhosis
Dizziness			k.A.	dizziness
Defaecation urgency			k.A.	urge to defecate
Toxicity renal			k.A.	acute toxic renal damage
Total knee replacement			k.A.	knee total endoprosthesis rightsided
Thrombocytopenia			k.A.	slightly thrombocytopenia
Postoperative bleeding			k.A.	acute postoperative bleeding
Infection NOS			k.A.	infect signs
Colon polypectomy			k.A.	colon polypectomy
Colon polypectomy			k.A.	colon polypectomy
Cardiac pacemaker insertion			k.A.	pace maker implantation
Hepatocellular injury			k.A.	liver parenchyma damage

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hemoglobin		9.4	G/DL	
Hemoglobin		7.2	G/DL	
Activated partial thromboplastin time	20081024	17	S	
Activated partial thromboplastin time	20081021	28	S	
Activated partial thromboplastin time	20081017	37.9	S	
Activated partial thromboplastin time	20081018	34.3	S	
Activated partial thromboplastin time	20081019	30.6	S	
Activated partial thromboplastin time	20081017	44	S	
Activated partial thromboplastin time	20081017	47	S	
Activated partial thromboplastin time	20081009	32	S	
Amylase	20081021	35	U/L	
Creatine kinase MB	20081017	70	U/L	
Creatine kinase MB	20081018	40	U/L	
Creatinine	20081020	1.56	MG/DL	
Creatinine	20081018	2.80	MG/DL	
Creatinine	20081017	2.03	MG/DL	

Creatinine	20081009	1.17	MG/DL
Gamma-glutamyltransferase	20081021	298	U/L
Gamma-glutamyltransferase	20081017	147	U/L
Gamma-glutamyltransferase	20081018	121	U/L
Gamma-glutamyltransferase	20081024	364	U/L
Gamma-glutamyltransferase	20081022	382	U/L
Glucose	20081009	109	MG/DL
Glucose	20081021	208	MG/DL
Glucose	20081014	131	MG/DL
Glucose	20081018	171	MG/DL
Glucose	20081017	241	MG/DL
Hematocrit	20081018	24.9	%
Hematocrit	20081013	36	%
Hematocrit	20081014	30	%
Hematocrit	20081017	21	%
Hematocrit	20081024	27	%
Hematocrit	20081017	22.7	%
Hematocrit	20081009	44	%
Hematocrit	20081020	24	%
Hemoglobin	20081013	12.2	G/DL
Hemoglobin	20081009	14.8	G/DL
Hemoglobin	20081014	10.8	G/DL
Hemoglobin	20081022	8.2	G/DL
Hemoglobin	20081024	8.9	G/DL
Hemoglobin	20081017	7.2	G/DL
Hemoglobin	20081017	7.8	G/DL
Hemoglobin	20081018	8.8	G/DL
Hemoglobin	20081020	8.2	G/DL
Lactate dehydrogenase	20081018	141	U/L
Potassium	20081009	5	MMOL/L
Potassium	20081019	3.77	MMOL/L
Potassium	20081017	4.22	MMOL/L
Potassium	20081014	4.35	MMOL/L
Prothrombin level	20081024	100	%
Prothrombin level	20081009	100	%
Prothrombin time ratio	20081009	0.93	/1
Prothrombin time ratio	20081024	0.85	/1
Prothrombin time ratio	20081018	1.10	/1
Prothrombin time ratio	20081017	1.30	/1
Red blood cell count	20081017	2.46	10E3/MM3
Red blood cell count	20081017	2.94	10E3/MM3
Red blood cell count	20081017	2.2	10E3/MM3
Red blood cell count	20081018	2.81	10E3/MM3
Red blood cell count	20081024	2.8	10E3/MM3

Red blood cell count	20081014	3.3	10E3/MM3
Red blood cell count	20081020	2.67	10E3/MM3
Red blood cell count	20081009	4.8	10E3/MM3
Red blood cell count	20081019	2.51	10E3/MM3
SGOT	20081024	40	MG/DL
SGOT	20081022	46	MG/DL
SGOT	20081021	33	MG/DL
SGOT	20081017	25	U/L
SGOT	20081018	19	U/L
SGPT	20081017	24	U/L
SGPT	20081022	41	U/L
SGPT	20081024	55	U/L
SGPT	20081017	33	U/L
SGPT	20081018	26	U/L
Activated partial thromboplastin time	20081018	34.3	S
Activated partial thromboplastin time	20081019	30.6	S
Activated partial thromboplastin time	20081014	47.0	S
Glucose	20081009	109	MG/DL
Glucose	20081021	208	MG/DL
Glucose	20081014	131	MG/DL
Hemoglobin	20081017	7.2	G/DL
Lactate dehydrogenase	20081018	141	U/L

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 23.10.2008 DE-BAYER-200828853GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 21-Oct-2008 via a sales representative by phone from a health professional in Germany referring to a male patient.

It is unknown whether any concomitant drugs have been given.

Patient received Xarelto (rivaroxaban). Administration dates unknown, total daily dose not reported.

On an unknown date the patient experienced GASTROINTESTINAL BLEEDING considered serious as important medical event. Event was treated with 2 to 3 packed red blood cells. Outcome unknown at time of report.

Follow-up information requested:

Missing information for documentation and medical assessment (standard documentation form including patient details, event details, suspect drug details, concomitant drug details, medical history and concomitant disease).

The original reporter's clinical assessment:

No assessment was given.

Bayer Vital GmbH, 19.11.2008 DE-BAYER-200828853GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 21-Oct-2008 via a sales representative by phone from a health professional in Germany referring to a male patient.

It is unknown whether any concomitant drugs have been given.

Patient received Xarelto (rivaroxaban). Administration dates unknown, total daily dose not reported.

On an unknown date the patient experienced GASTROINTESTINAL BLEEDING considered serious as important medical event. Event was treated with 2 to 3 packed red blood cells. Outcome unknown at time of report.

Follow-up information requested:

Missing information for documentation and medical assessment (standard documentation form including patient details, event details, suspect drug details, concomitant drug details, medical history and concomitant disease).

The original reporter's clinical assessment:

No assessment was given.

****Follow-up information**** received on 05-Nov-2008:

Patient has concomitant arterial hypertension, diabetes mellitus type II and gonarthrosis.

Additional concomitant medication has been given.

Patient received:

- Xarelto (Rivaroxaban) 10mg orally from 10-Oct-2008 to 18-Oct-2008, and

- Ibuhexal (Ibuprofen) at a total daily dose of 600mg oral on unknown dates.

The event term was modified from GASTROINTESTINAL BLEEDING to BLEEDING SIGMOID COLON and two additional SAE were provided:

On 18-Oct-2008 patient experienced BLEEDING SIGMOID COLON (bloody stool, hemoglobin dropped) considered serious as leading to hospitalization. Patient recovered on 20-Oct-2008. Xarelto was withdrawn.

On an unknown date patient experienced INCREASED RENAL PARAMETERS considered serious as leading to hospitalization. Patient's condition improved. Xarelto and Ibuhexal were withdrawn.

On an unknown date patient experienced INCREASED GGT considered serious as leading to hospitalization. Patient has not recovered yet.

Further details:

On 10-Oct-2008 a knee total endoprosthesis was performed.

On 18-Oct-2008 bloody stool was found and hemoglobin dropped from 9.4g/dl to 7.2g/dl. Patient was transferred to internal unit and diagnosis of sigmoid bleeding via colonoscopy was revealed. Xarelto was discontinued.

In addition: increase of renal parameters (no lab data provided): Suspicion of toxic renal damage caused by Ibuprofen and Xarelto. After discontinuation of treatment with Xarelto and Ibuprofen improvement of renal lab parameter but increase of GGT since then (no lab data provided).

The reporter considered the diagnoses BLEEDING SIGMOID COLON and INCREASED RENAL PARAMETERS to be possibly related to Xarelto.

The diagnosis INCREASED RENAL PARAMETERS is possibly related to Ibuhexal.

For the diagnosis INCREASED GGT no assessment was given.

*****BAYER Schering GLOBAL COMMENT (part one):**

GASTROINTESTINAL BLEEDING (serious as medically important event) is listed in the international product information of Xarelto. In the present spontaneous case a patient was reported with gastrointestinal bleeding. Since neither the drug administration dates nor the onset date of the event have been reported the temporal relationship between drug and event is unknown. Thus, the case is based on the received spared information not assessable. FU information has been requested (drug administration dates and indication for use, onset date of event, concomitant diseases, concomitant drugs, medical history, patient demographics).

Bayer Vital GmbH, 07.01.2009 DE-BAYER-200828853GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 21-Oct-2008 via a sales representative by phone from a health professional in Germany referring to a male patient.

It is unknown whether any concomitant drugs have been given.

Patient received Xarelto (rivaroxaban). Administration dates unknown, total daily dose not reported.

On an unknown date the patient experienced GASTROINTESTINAL BLEEDING considered serious as important medical event. Event was treated with 2 to 3 packed red blood cells. Outcome unknown at time of report.

Follow-up information requested:

Missing information for documentation and medical assessment (standard documentation form including patient details, event details, suspect drug details, concomitant drug details, medical history and concomitant disease).

The original reporter's clinical assessment:

No assessment was given.

****Follow-up information**** received on 05-Nov-2008:

Patient has concomitant arterial hypertension, diabetes mellitus type II and gonarthrosis.

Additional concomitant medication has been given.

Patient received:

- Xarelto (Rivaroxaban) 10mg orally from 10-Oct-2008 to 18-Oct-2008, and
- Ibuhexal (Ibuprofen) at a total daily dose of 600mg oral on unknown dates.

The event term was modified from GASTROINTESTINAL BLEEDING to BLEEDING SIGMOID COLON and two additional SAE were provided:

On 18-Oct-2008 patient experienced BLEEDING SIGMOID COLON (bloody stool, hemoglobin dropped) considered serious as leading to hospitalization. Patient recovered on 20-Oct-2008. Xarelto was withdrawn.

On an unknown date patient experienced INCREASED RENAL PARAMETERS considered serious as leading to hospitalization. Patient's condition improved. Xarelto and Ibuhexal were withdrawn.

On an unknown date patient experienced INCREASED GGT considered serious as leading to hospitalization. Patient has not recovered yet.

Further details:

On 10-Oct-2008 a knee total endoprosthesis was performed.

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In addition: increase of renal parameters (no lab data provided): Suspicion of toxic renal damage caused by Ibuprofen and Xarelto. After discontinuation of treatment with Xarelto and Ibuprofen improvement of renal lab parameter but increase of GGT since then (no lab data provided).

The reporter considered the diagnoses BLEEDING SIGMOID COLON and INCREASED RENAL PARAMETERS to be possibly related to Xarelto.

The diagnosis INCREASED RENAL PARAMETERS is possibly related to Ibuhexal.

For the diagnosis INCREASED GGT no assessment was given.

****Follow-up information** received on 22-Dec-2008:

Junior orthopedic physician provided an epicrisis on the case and stated a general causal relationship for Rivaroxaban and Ibuprofen.

Hospital discharge letter on 20-OCT-2008: Physician from the internal ward exclusively stated a positive causal relationship between "increased renal parameters" and treatment with Ibuprofen, Rivaroxaban and Novaminsulfon. Explanation given for "bleeding of sigmoid colon" bleeding diverticula and preexisting liver parenchyma damage.

Conflicting data provided concerning Rivaroxaban treatment.

Hospitalization from 17-OCT-2008 till 20-OCT-2008

Diagnosis:

- Acute postoperative bleeding (lower intestinal tract)
- Suspected drug induced (Ibuprofen, Rivaroxaban, Novaminsulfon) acute toxic renal damage
- Infect signs
- State after knee total endoprosthesis rightsided 13-OCT-2008
- State after pace maker implantation (sick-sinus-syndrome, intermittent SA block and syncope) JAN-2007
- Liver parenchyma damage (developing fatty cirrhosis)
- Slightly thrombocytopenia (suspected toxic genesis), confirmed by iliac crest biopsy
- Diabetes mellitus type 2
- arterial hypertension
- State after colon polypectomy 2001 and 2003 (last colonoscopy 2005)

Anamnesis:

Patient was transferred from orthopedic ward to the internal ward due to gastrointestinal bleeding with increased blood loss perianal.

On 13-OCT-2008 patient underwent implantation of knee TEP. Subsequently patient received 3 packed red blood cells (patient's own blood). During following course patient suffered from urge to defecate, loss of fresh blood, pain in leg right, dizziness, nausea and general weakness. Determined lab control during further course revealed haemoglobin value 7,2 g/dl whereupon patient received one additional unit red cells.

Treatment and further course:

On admitting day an oesophagoduodenoscopy was performed followed by a colonoscopy the next day. At this time no bleeding was detectable, even though the source of bleeding had been suspected in lower sigma. Furthermore, no findings of ulceration, inflammatory or neoplastic source of bleeding.

State after diverticula bleeding was assumed.

Xarelto had been discontinued and changed to heparin 2x7.500 IU s.c.

In total patient received 3 x 250 ml red cell packs that led to circulatory stabilization and improvement of general condition. Prior increased serum creatinin (up

to 2,8 %) had dropped during the clinical course. Glucophage had been discontinued due to increasing renal values. Daily blood glucose monitoring was initiated.

On 19-OCT-2008 passive mobilisation on motor splint started. It was recommended to withdraw all drugs with the potential to cause renal damages or

bleedings.

For New Information please refer to Section(s):

CONCOMITANT DRUG

SUSPECTED DRUG

***BAYER Schering GLOBAL COMMENT (part one):

GASTROINTESTINAL BLEEDING (serious as medically important event) is listed in the international product information of Xarelto. In the present spontaneous case a patient was reported with gastrointestinal bleeding. Since neither the drug administration dates nor the onset date of the event have been reported the temporal relationship between drug and event is unknown. Thus, the case is based on the received sparsely information not assessable. FU information has been requested (drug administration dates and indication for use, onset date of event, concomitant diseases, concomitant drugs, medical history, patient demographics).

**FU 05-Nov-2008: Ibuprofen has been reported as an additional suspected drug. The investigator amended the event term GASTROINTESTINAL BLEEDING into BLEEDING SIGMOID COLON (serious, listed for Xarelto and Ibuprofen). The event occurred about one week after onset of medication with Rivaroxaban and during Ibuprofen treatment. The reporting physician assessed the event as possibly related to Rivaroxaban treatment. Due to the mode of action of all anticoagulants the bleeding risk is increased and therefore a relationship is not excluded. Taking into consideration that the patient took the co-suspected drug Ibuprofen for an unknown period and that gastrointestinal ulcer also with bleeding are listed for this substance, this might well have favoured the occurrence of a gastrointestinal mucosal damage with consecutive bleeding. In addition, two new events have been reported. INCREASED RENAL PARAMETERS (serious, listed for both drugs) occurred during medication with Rivaroxaban and Ibuprofen (no lab data provided). The reporting physician assessed the event as possibly related to Rivaroxaban and Ibuprofen treatment. Due to a compatible temporal relationship a causal association cannot be excluded to both drugs. INCREASED GGT (serious, listed for Rivaroxaban, unlisted for Ibuprofen) occurred on an unknown date after discontinuation of the respective drugs (no lab data provided). The reporting physician did not assess the drug-event relationship. Based on the sparse information received the company considers this event as not assessable. FU information has been requested (LFT and renal lab values pre and post start of Xarelto, start and stop date of Ibuprofen, legible AE attachment, confirmation of knee endoprosthesis).

Bayer Vital GmbH, 07.01.2009 DE-BAYER-200828853GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 21-Oct-2008 via a sales representative by phone from a health professional in Germany referring to a male patient.

It is unknown whether any concomitant drugs have been given.

Patient received Xarelto (rivaroxaban). Administration dates unknown, total daily dose not reported.

On an unknown date the patient experienced GASTROINTESTINAL BLEEDING considered serious as important medical event. Event was treated with 2 to 3 packed red blood cells. Outcome unknown at time of report.

Follow-up information requested:

Missing information for documentation and medical assessment (standard documentation form including patient details, event details, suspect drug details, concomitant drug details, medical history and concomitant disease).

The original reporter's clinical assessment:

No assessment was given.

Follow-up information received on 05-Nov-2008:

Patient has concomitant arterial hypertension, diabetes mellitus type II and gonarthrosis.

Additional concomitant medication has been given.

Patient received:

- Xarelto (Rivaroxaban) 10mg orally from 10-Oct-2008 to 18-Oct-2008, and
- Ibufen (Ibuprofen) at a total daily dose of 600mg oral on unknown dates.

The event term was modified from GASTROINTESTINAL BLEEDING to BLEEDING SIGMOID COLON and two additional SAE were provided: On 18-Oct-2008 patient experienced BLEEDING SIGMOID COLON (bloody stool, hemoglobin dropped) considered serious as leading to hospitalization. Patient recovered on 20-Oct-2008. Xarelto was withdrawn.

On an unknown date patient experienced INCREASED RENAL PARAMETERS considered serious as leading to hospitalization. Patient's condition improved. Xarelto and Ibufen were withdrawn.

On an unknown date patient experienced INCREASED GGT considered serious as leading to hospitalization. Patient has not recovered yet.

Further details:

On 10-Oct-2008 a knee total endoprosthesis was performed.

On 18-Oct-2008 bloody stool was found and hemoglobin dropped from 9.4g/dl to 7.2g/dl. Patient was transferred to internal unit and diagnosis of sigmoid bleeding via colonoscopy was revealed. Xarelto was discontinued.

In addition: increase of renal parameters (no lab data provided): Suspicion of toxic renal damage caused by Ibuprofen and Xarelto. After discontinuation of treatment with Xarelto and Ibuprofen improvement of renal lab parameter but increase of GGT since then (no lab data provided).

The reporter considered the diagnoses BLEEDING SIGMOID COLON and INCREASED RENAL PARAMETERS to be possibly related to Xarelto. The diagnosis INCREASED RENAL PARAMETERS is possibly related to Ibuhexal. For the diagnosis INCREASED GGT no assessment was given.

****Follow-up information received on 22-Dec-2008:**

Junior orthopedic physician provided an epicrisis on the case and stated a general causal relationship for Rivaroxaban and Ibuprofen.

Hospital discharge letter on 20-OCT-2008: Physician from the internal ward exclusively stated a positive causal relationship between "increased renal parameters" and treatment with Ibuprofen, Rivaroxaban and Novaminsulfon. Explanation given for "bleeding of sigmoid colon" bleeding diverticula and preexisting liver parenchyma damage.

Conflicting data provided concerning Rivaroxaban treatment.

Hospitalization from 17-OCT-2008 till 20-OCT-2008

Diagnosis:

- Acute postoperative bleeding (lower intestinal tract)
- Suspected drug induced (Ibuprofen, Rivaroxaban, Novaminsulfon) acute toxic renal damage
- Infect signs
- State after knee total endoprosthesis rightsided 13-OCT-2008
- State after pace maker implantation (sick-sinus-syndrome, intermittent SA block and syncope) JAN-2007
- Liver parenchyma damage (developing fatty cirrhosis)
- Slightly thrombocytopenia (suspected toxic genesis), confirmed by iliac crest biopsy
- Diabetes mellitus type 2
- arterial hypertension
- State after colon polypectomy 2001 and 2003 (last colonoscopy 2005)

Anamnesis:

Patient was transferred from orthopedic ward to the internal ward due to gastrointestinal bleeding with increased blood loss perianal.

On 13-OCT-2008 patient underwent implantation of knee TEP. Subsequently patient received 3 packed red blood cells (patient's own blood). During following course patient suffered from urge to defecate, loss of fresh blood, pain in leg right, dizziness, nausea and general weakness. Determined lab control during further course revealed haemoglobin value 7,2 g/dl whereupon patient received one additional unit red cells.

Treatment and further course:

On admitting day an oesophagoduodenoscopy was performed followed by a colonoscopy the next day. At this time no bleeding was detectable, even though the source of bleeding had been suspected in lower sigma. Furthermore, no findings of ulceration, inflammatory or neoplastic source of bleeding.

State after diverticula bleeding was assumed.

Xarelto had been discontinued and changed to heparin 2x7.500 IU s.c.

In total patient received 3 x 250 ml red cell packs that led to circulatory stabilization and improvement of general condition. Prior increased serum creatinin (up

to 2,8 %) had dropped during the clinical course. Glucophage had been discontinued due to increasing renal values. Daily blood glucose monitoring was initiated.

On 19-OCT-2008 passive mobilisation on motor splint started. It was recommended to withdraw all drugs with the potential to cause renal damages or bleedings.

For New Information please refer to Section(s):

CONCOMITANT DRUG

SUSPECTED DRUG

*****BAYER Schering GLOBAL COMMENT (part one):**

GASTROINTESTINAL BLEEDING (serious as medically important event) is listed in the international product information of Xarelto. In the present spontaneous case a patient was reported with gastrointestinal bleeding. Since neither the drug administration dates nor the onset date of the event have been reported the temporal relationship between drug and event is unknown. Thus, the case is based on the received spared information not assessable. FU information has been requested (drug administration dates and indication for use, onset date of event, concomitant diseases, concomitant drugs, medical history, patient demographics).

****FU 05-Nov-2008:** Ibuprofen has been reported as an additional suspected drug. The investigator amended the event term GASTROINTESTINAL BLEEDING into BLEEDING SIGMOID COLON (serious, listed for Xarelto and Ibuprofen). The event occurred about one week after onset of medication with Rivaroxaban and during Ibuprofen treatment. The reporting physician assessed the event as possibly related to Rivaroxaban treatment. Due to the mode of action of all anticoagulants the bleeding risk is increased and therefore a relationship is not excluded. Taking into consideration that the patient took the co-suspected drug Ibuprofen for an unknown period and that gastrointestinal ulcer also with bleeding are listed for this substance, this might well have favoured the occurrence of a gastrointestinal mucosal damage with consecutive bleeding. In addition, two new events have been reported. INCREASED RENAL PARAMETERS (serious, listed for both drugs) occurred during medication with Rivaroxaban and Ibuprofen (no lab data provided). The reporting physician assessed the event as possibly related to Rivaroxaban and Ibuprofen treatment. Due to a compatible temporal relationship a causal association cannot be excluded to both drugs. INCREASED GGT (serious, listed for Rivaroxaban, unlisted for Ibuprofen) occurred on an unknown date after discontinuation of the respective drugs (no lab data provided). The reporting physician did not assess the drug-event relationship. Based on the sparse information received the company considers this event as not assessable. FU information

has been requested (LFT and renal lab values pre and post start of Xarelto, start and stop date of Ibuprofen, legible AE attachment, confirmation of knee endoprosthesis).

Bayer Vital GmbH, 27.01.2009 DE-BAYER-200828853GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 21-Oct-2008 via a sales representative by phone from a health professional in Germany referring to a male patient.

It is unknown whether any concomitant drugs have been given.

Patient received Xarelto (rivaroxaban). Administration dates unknown, total daily dose not reported.

On an unknown date the patient experienced GASTROINTESTINAL BLEEDING considered serious as important medical event. Event was treated with 2 to 3 packed red blood cells. Outcome unknown at time of report.

Follow-up information requested:

Missing information for documentation and medical assessment (standard documentation form including patient details, event details, suspect drug details, concomitant drug details, medical history and concomitant disease).

The original reporter's clinical assessment:

No assessment was given.

****Follow-up information**** received on 05-Nov-2008:

Patient has concomitant arterial hypertension, diabetes mellitus type II and gonarthrosis.

Additional concomitant medication has been given.

Patient received:

- Xarelto (Rivaroxaban) 10mg orally from 10-Oct-2008 to 18-Oct-2008, and
- Ibuhexal (Ibuprofen) at a total daily dose of 600mg oral on unknown dates.

The event term was modified from GASTROINTESTINAL BLEEDING to BLEEDING SIGMOID COLON and two additional SAE were provided:

On 18-Oct-2008 patient experienced BLEEDING SIGMOID COLON (bloody stool, hemoglobin dropped) considered serious as leading to hospitalization. Patient recovered on 20-Oct-2008. Xarelto was withdrawn.

On an unknown date patient experienced INCREASED RENAL PARAMETERS considered serious as leading to hospitalization. Patient's condition improved. Xarelto and Ibuhexal were withdrawn.

On an unknown date patient experienced INCREASED GGT considered serious as leading to hospitalization. Patient has not recovered yet.

Further details:

On 10-Oct-2008 a knee total endoprosthesis was performed.

On 18-Oct-2008 bloody stool was found and hemoglobin dropped from 9.4g/dl to 7.2g/dl. Patient was transferred to internal unit and diagnosis of sigmoid bleeding via colonoscopy was revealed. Xarelto was discontinued.

In addition: increase of renal parameters (no lab data provided): Suspicion of toxic renal damage caused by Ibuprofen and Xarelto. After discontinuation of treatment with Xarelto and Ibuprofen improvement of renal lab parameter but increase of GGT since then (no lab data provided).

The reporter considered the diagnoses BLEEDING SIGMOID COLON and INCREASED RENAL PARAMETERS to be possibly related to Xarelto.

The diagnosis INCREASED RENAL PARAMETERS is possibly related to Ibuhexal.

For the diagnosis INCREASED GGT no assessment was given.

****Follow-up information** received on 22-Dec-2008:

Junior orthopedic physician provided an epicrisis on the case and stated a general causal relationship for Rivaroxaban and Ibuprofen.

Hospital discharge letter on 20-OCT-2008: Physician from the internal ward exclusively stated a positive causal relationship between "increased renal parameters" and treatment with Ibuprofen, Rivaroxaban and Novaminsulfon. Explanation given for "bleeding of sigmoid colon" bleeding diverticula and preexisting liver parenchyma damage.

Conflicting data provided concerning Rivaroxaban treatment.

Hospitalization from 17-OCT-2008 till 20-OCT-2008

Diagnosis:

- Acute postoperative bleeding (lower intestinal tract)
- Suspected drug induced (Ibuprofen, Rivaroxaban, Novaminsulfon) acute toxic renal damage
- Infect signs
- State after knee total endoprosthesis rightsided 13-OCT-2008
- State after pace maker implantation (sick-sinus-syndrome, intermittent SA block and syncope) JAN-2007
- Liver parenchyma damage (developing fatty cirrhosis)
- Slightly thrombocytopenia (suspected toxic genesis), confirmed by iliac crest biopsy

- Diabetes mellitus type 2
- arterial hypertension
- State after colon polypectomy 2001 and 2003 (last colonoscopy 2005)

Anamnesis:

Patient was transferred from orthopedic ward to the internal ward due to gastrointestinal bleeding with increased blood loss perianal.

On 13-OCT-2008 patient underwent implantation of knee TEP. Subsequently patient received 3 packed red blood cells (patient's own blood). During following course patient suffered from urge to defecate, loss of fresh blood, pain in leg right, dizziness, nausea and general weakness. Determined lab control during further course revealed haemoglobin value 7,2 g/dl whereupon patient received one additional unit red cells.

Treatment and further course:

On admitting day an oesophagoduodenoscopy was performed followed by a colonoscopy the next day. At this time no bleeding was detectable, even though the source of bleeding had been suspected in lower sigma. Furthermore, no findings of ulceration, inflammatory or neoplastic source of bleeding.

State after diverticula bleeding was assumed.

Xarelto had been discontinued and changed to heparin 2x7.500 IU s.c.

In total patient received 3 x 250 ml red cell packs that led to circulatory stabilization and improvement of general condition. Prior increased serum creatinin (up

to 2,8 %) had dropped during the clinical course. Glucophage had been discontinued due to increasing renal values. Daily blood glucose monitoring was initiated.

On 19-OCT-2008 passive mobilisation on motor splint started. It was recommended to withdraw all drugs with the potential to cause renal damages or bleedings.

For New Information please refer to Section(s):

CONCOMITANT DRUG

SUSPECTED DRUG

FU 07-Jan-2009:

Concomitant drugs have been added and administration dates of concomitant drugs have been updated.

Additional concomitant conditions/history have been added.

Lab value has been added; see "Lab Data" section.

In a telephone conversation, the reporter gave his view of a number of still unanswered questions:

1. As intended, Xarelto (rivaroxaban) had initially been administered on 13-Oct-2008, ten hours post surgery. Deviating therapy data were probably due to transcription errors.
2. Drug safety had already received all laboratory findings in possession of the orthopedic clinic from tests carried out prior to surgery. No further findings were present to the clinic.
3. During the patient's stay in the orthopedic clinic, no differential diagnostics procedures with regard to liver and kidneys had been performed. For further information, drug safety should refer to the internal medicine clinic.

For the drug Novaminsulfon the reporter considered the diagnosis INCREASED RENAL PARAMETERS to be possibly related. Novaminsulfon was withdrawn.

FU 22-Jan-2009:

The indication of Xarelto has been amended to knee replacement surgery.

***BAYER Schering GLOBAL COMMENT (part one):

GASTROINTESTINAL BLEEDING (serious as medically important event) is listed in the international product information of Xarelto. In the present spontaneous case a patient was reported with gastrointestinal bleeding. Since neither the drug administration dates nor the onset date of the event have been reported the temporal relationship between drug and event is unknown. Thus, the case is based on the received spared information not assessable. FU information has been requested (drug administration dates and indication for use, onset date of event, concomitant diseases, concomitant drugs, medical history, patient demographics).

**FU 05-Nov-2008: Ibuprofen has been reported as an additional suspected drug. The investigator amended the event term GASTROINTESTINAL BLEEDING into BLEEDING SIGMOID COLON (serious, listed for Xarelto and Ibuprofen). The event occurred about one week after onset of medication with Rivaroxaban and during Ibuprofen treatment. The reporting physician assessed the event as possibly related to Rivaroxaban treatment. Due to the mode of action of all anticoagulants the bleeding risk is increased and therefore a relationship is not excluded. Taking into consideration that the patient took the co-suspected drug Ibuprofen for an unknown period and that gastrointestinal ulcer also with bleeding are listed for this substance, this might well have favoured the occurrence of a gastrointestinal mucosal damage with consecutive bleeding. In addition, two new events have been reported. INCREASED RENAL PARAMETERS (serious, listed for both drugs) occurred during medication with Rivaroxaban and Ibuprofen (no lab data provided). The reporting physician assessed the event as possibly related to Rivaroxaban and Ibuprofen treatment. Due to a compatible temporal relationship a causal association cannot be excluded to both drugs. INCREASED GGT (serious, listed for Rivaroxaban, unlisted for Ibuprofen) occurred on an unknown date after discontinuation of the respective drugs (no lab data provided). The reporting physician did not assess the drug-event relationship. Based on the sparse information received the company considers this event as not assessable. FU information has been requested (LFT and renal lab values pre and post start of Xarelto, start and stop date of Ibuprofen, legible AE attachment, confirmation of knee endoprosthesis).

Bayer Vital GmbH, 17.03.2009 DE-BAYER-200828853GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 21-Oct-2008 via a sales representative by phone from a health professional in Germany referring to a male patient.

It is unknown whether any concomitant drugs have been given.

Patient received Xarelto (rivaroxaban). Administration dates unknown, total daily dose not reported.

On an unknown date the patient experienced GASTROINTESTINAL BLEEDING considered serious as important medical event. Event was treated with 2 to 3 packed red blood cells. Outcome unknown at time of report.

Follow-up information requested:

Missing information for documentation and medical assessment (standard documentation form including patient details, event details, suspect drug details, concomitant drug details, medical history and concomitant disease).

The original reporter's clinical assessment:

No assessment was given.

****Follow-up information**** received on 05-Nov-2008:

Patient has concomitant arterial hypertension, diabetes mellitus type II and gonarthrosis.

Additional concomitant medication has been given.

Patient received:

- Xarelto (Rivaroxaban) 10mg orally from 10-Oct-2008 to 18-Oct-2008, and
- Ibuhexal (Ibuprofen) at a total daily dose of 600mg oral on unknown dates.

The event term was modified from GASTROINTESTINAL BLEEDING to BLEEDING SIGMOID COLON and two additional SAE were provided:

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On an unknown date patient experienced INCREASED RENAL PARAMETERS considered serious as leading to hospitalization. Patient's condition improved. Xarelto and Ibuhexal were withdrawn.

On an unknown date patient experienced INCREASED GGT considered serious as leading to hospitalization. Patient has not recovered yet.

Further details:

On 10-Oct-2008 a knee total endoprosthesis was performed.

On 18-Oct-2008 bloody stool was found and hemoglobin dropped from 9.4g/dl to 7.2g/dl. Patient was transferred to internal unit and diagnosis of sigmoid bleeding via colonoscopy was revealed. Xarelto was discontinued.

In addition: increase of renal parameters (no lab data provided): Suspicion of toxic renal damage caused by Ibuprofen and Xarelto. After discontinuation of treatment with Xarelto and Ibuprofen improvement of renal lab parameter but increase of GGT since then (no lab data provided).

The reporter considered the diagnoses BLEEDING SIGMOID COLON and INCREASED RENAL PARAMETERS to be possibly related to Xarelto.

The diagnosis INCREASED RENAL PARAMETERS is possibly related to Ibuhexal.

For the diagnosis INCREASED GGT no assessment was given.

****Follow-up information** received on 22-Dec-2008:

Junior orthopedic physician provided an epicrisis on the case and stated a general causal relationship for Rivaroxaban and Ibuprofen.

Hospital discharge letter on 20-OCT-2008: Physician from the internal ward exclusively stated a positive causal relationship between "increased renal parameters" and treatment with Ibuprofen, Rivaroxaban and Novaminsulfon. Explanation given for "bleeding of sigmoid colon" bleeding diverticula and preexisting liver parenchyma damage.

Conflicting data provided concerning Rivaroxaban treatment.

Hospitalization from 17-OCT-2008 till 20-OCT-2008

Diagnosis:

- Acute postoperative bleeding (lower intestinal tract)
- Suspected drug induced (Ibuprofen, Rivaroxaban, Novaminsulfon) acute toxic renal damage
- Infect signs
- State after knee total endoprosthesis rightsided 13-OCT-2008
- State after pace maker implantation (sick-sinus-syndrome, intermittent SA block and syncope) JAN-2007
- Liver parenchyma damage (developing fatty cirrhosis)
- Slightly thrombocytopenia (suspected toxic genesis), confirmed by iliac crest biopsy
- Diabetes mellitus type 2
- arterial hypertension
- State after colon polypectomy 2001 and 2003 (last colonoscopy 2005)

Anamnesis:

Patient was transferred from orthopedic ward to the internal ward due to gastrointestinal bleeding with increased blood loss perianal.

On 13-OCT-2008 patient underwent implantation of knee TEP. Subsequently patient received 3 packed red blood cells (patient's own blood). During

following course patient suffered from urge to defecate, loss of fresh blood, pain in leg right, dizziness, nausea and general weakness. Determined lab control during further course revealed haemoglobin value 7,2 g/dl whereupon patient received one additional unit red cells.

Treatment and further course:

On admitting day an oesophagoduodenoscopy was performed followed by a colonoscopy the next day. At this time no bleeding was detectable, even though the source of bleeding had been suspected in lower sigma. Furthermore, no findings of ulceration, inflammatory or neoplastic source of bleeding.

State after diverticula bleeding was assumed.

Xarelto had been discontinued and changed to heparin 2x7.500 IU s.c.

In total patient received 3 x 250 ml red cell packs that led to circulatory stabilization and improvement of general condition. Prior increased serum creatinin (up

to 2,8 %) had dropped during the clinical course. Glucophage had been discontinued due to increasing renal values. Daily blood glucose monitoring was initiated.

On 19-OCT-2008 passive mobilisation on motor splint started. It was recommended to withdraw all drugs with the potential to cause renal damages or bleedings.

For New Information please refer to Section(s):

CONCOMITANT DRUG

SUSPECTED DRUG

****FU 07-Jan-2009**:**

Concomitant drugs have been added and administration dates of concomitant drugs have been updated.

Additional concomitant conditions/history have been added.

Lab value has been added; see "Lab Data" section.

In a telephone conversation, the reporter gave his view of a number of still unanswered questions:

1. As intended, Xarelto (rivaroxaban) had initially been administered on 13-Oct-2008, ten hours post surgery. Deviating therapy data were probably due to transcription errors.
2. Drug safety had already received all laboratory findings in possession of the orthopedic clinic from tests carried out prior to surgery. No further findings were present to the clinic.
3. During the patient's stay in the orthopedic clinic, no differential diagnostics procedures with regard to liver and kidneys had been performed. For further information, drug safety should refer to the internal medicine clinic.

For the drug Novaminsulfon the reporter considered the diagnosis INCREASED RENAL PARAMETERS to be possibly related. Novaminsulfon was withdrawn.

****FU 22-Jan-2009**:**

The indication of Xarelto has been amended to knee replacement surgery.

****FU 04-Mar-2009**:**

Despite our repeated requests no further information could be obtained. Case closed.

*****BAYER Schering GLOBAL COMMENT (part one):**

GASTROINTESTINAL BLEEDING (serious as medically important event) is listed in the international product information of Xarelto. In the present spontaneous case a patient was reported with gastrointestinal bleeding. Since neither the drug administration dates nor the onset date of the event have been reported the temporal relationship between drug and event is unknown. Thus, the case is based on the received sparse information not assessable. FU information has been requested (drug administration dates and indication for use, onset date of event, concomitant diseases, concomitant drugs, medical history, patient demographics).

****FU 05-Nov-2008:** Ibuprofen has been reported as an additional suspected drug. The investigator amended the event term GASTROINTESTINAL BLEEDING into BLEEDING SIGMOID COLON (serious, listed for Xarelto and Ibuprofen). The event occurred about one week after onset of medication with Rivaroxaban and during Ibuprofen treatment. The reporting physician assessed the event as possibly related to Rivaroxaban treatment. Due to the mode of action of all anticoagulants the bleeding risk is increased and therefore a relationship is not excluded. Taking into consideration that the patient took the co-suspected drug Ibuprofen for an unknown period and that gastrointestinal ulcer also with bleeding are listed for this substance, this might well have favoured the occurrence of a gastrointestinal mucosal damage with consecutive bleeding. In addition, two new events have been reported. INCREASED RENAL PARAMETERS (serious, listed for both drugs) occurred during medication with Rivaroxaban and Ibuprofen (no lab data provided). The reporting physician assessed the event as possibly related to Rivaroxaban and Ibuprofen treatment. Due to a compatible temporal relationship a causal association cannot be excluded to both drugs. INCREASED GGT (serious, listed for Rivaroxaban, unlisted for Ibuprofen) occurred on an unknown date after discontinuation of the respective drugs (no lab data provided). The reporting physician did not assess the drug-event relationship. Based on the sparse information received the company considers this event as not assessable. FU information has been requested (LFT and renal lab values pre and post start of Xarelto, start and stop date of Ibuprofen, legible AE attachment, confirmation of knee endoprosthesis).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08142949

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19701002	Age: 37 Year	weight (kg): 45	BMI_int: 17.15	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal pain					Unknown
2 Hepatitis acute	20080413	20080826	4 Month		recovered/resolved
3 Bilirubin increased					Unknown
4 Urine colour abnormal					Unknown
5 Transaminases increased					Unknown
6 Nausea					Unknown
7 Icterus					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Carbostesin 0.25%, 20 ml, Amp, Astra Zeneca	Perineural	50 Mg milligram(s)/Day	20080401-20080409	
bupivacaine		120	9 Day	
2 Ibuprofen, 400 mg, Tabl, NN	Oral	1200 Mg milligram(s)/Day	20080326-20080413	Ache
ibuprofen			19 Day	

Drugs (concomitant)

3 Lormetazepam, 1 mg, Tabl, NN	Oral	1 Mg milligram(s)/ Day	20080326-20080407	
lormetazepam		245	13 Day	
4 Mono Embolex, 3000 IE, Amp, Novartis	Subcutaneous	3000 Iu international unit(s)	20080326-20080410	Thrombosis prophylaxis
certoparin		120	16 Day	
5 Novalgine, Trpf, Sanofi-Aventis	Oral	750 Mg milligram(s)/ Day	20080326-20080409	
metamizole		158	15 Day	
6 Pantozol, 40 mg, Tabl, Nycomed	Oral	40 Mg milligram(s)/Day	20080326-20080413	
pantoprazole		94	19 Day	
7 Tramal, Trpf, Grünenthal	Oral	75 Mg milligram(s)/Day	20080409-20080409	
tramadol		158	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hand fracture	20070722		Unknown	Fracture of one or more phalanges of hand
Abscess of Bartholin's gland	2007		Unknown	
Contracture of joint			Unknown	
Fractured finger	20070722		Unknown	
Ex-smoker	1994		Yes	AT LAST 20 CIGARETTES DAILY
Insect sting allergy			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20080422	592	U/l	
Aspartate aminotransferase increased	20080422	309	U/l	
Bilirubin increased	20080422	4.49	mg/dl	

Alanine aminotransferase increased	20080424	710	U/l
Aspartate aminotransferase increased	20080424	369	U/l
Bilirubin increased	20080424	6.3	mg/dl
Alanine aminotransferase increased	20080428	582	U/l
Aspartate aminotransferase increased	20080428	261	U/l
Bilirubin increased	20080428	3.0	mg/dl
Alanine aminotransferase increased	20080506	149	U/l
Aspartate aminotransferase increased	20080506	39	U/l
Bilirubin increased	20080506	1.1	mg/dl
Alanine aminotransferase normal	20080826	18	U/l
Aspartate aminotransferase normal	20080826	23	U/l
Bilirubin normal	20080826	0.5	U/l

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 05.12.2008 DE-FAKOS-170215

Sender diagnosis: not reported

Narrative

Die UAW hat sich im Krankenhaus entwickelt. Ende der Einnahme von Ibuprofen am 13.04.2008 und der Gabe von Carbostesin am 09.04.2008. Im weiteren Verlauf Rückgang der erhöhten Transaminasen und des Bilirubins.

Ibuprofen Tabl, NN, ist spezifisch gelabelt bei Hexal: "akute Hepatitis", Stand: 04/2007.

Carbostesin Amp, Astra Zeneca, ist nicht gelabelt, Stand: 08/2007.

complete

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 15.12.2008 DE-FAKOS-170215

Sender diagnosis: not reported

Narrative

Die UAW hat sich im Krankenhaus entwickelt. Ende der Einnahme von Ibuprofen am 13.04.2008 und der Gabe von Carbostesin am 09.04.2008. Im weiteren Verlauf Rückgang der erhöhten Transaminasen und des Bilirubins.

Ibuprofen Tabl, NN, ist spezifisch gelabelt bei Hexal: "akute Hepatitis", Stand: 04/2007.

Carbostesin Amp, Astra Zeneca, ist nicht gelabelt, Stand: 08/2007.

complete

Reporter's comment:

Bayer Vital GmbH, 17.12.2008 DE-BAYER-200832431GPV

Sender diagnosis: not reported

Narrative

This solicited case report was received on 08-Dec-2008 from the internet page of BfArM, the regulatory health authority of Germany, referring to a

female patient (height: 162 cm, weight: 45 kg, age was not reported). The case derives from FAKOS Project (Case-Control Surveillance Study).

Medical and drug history:

Patient's medical history includes contracture of joint, fracture of one or more phalanges of hand on 22-Jul-2007, and abscess of Bartholin's gland in 2007. Risk factors include smoking up to 1994 (at least 20 cigarettes daily) and allergy to insect poison.

Concomitant drugs have been given for pain NOS and thrombosis prophylaxis (for details refer to 'Concomitant Drugs' section in this report).

Suspect drugs, timing and conditions surrounding the onset of the reaction:

During hospital stay for unknown reason, patient received

- oral Ibuprofen (no trade name provided) 400 mg tablet thrice a day for ache NOS from 26-Mar-2008 until 13-Apr-2008,

- perineural Carbostesin (bupivacaine hydrochloride) 0.25% injection at a dose of 50 mg once daily for an unknown indication from 01-Apr-2008 until 09-Apr-2008.

The progression of the event and its outcome in the patient:

On 13-Apr-2008, patient experienced ACUTE HEPATITIS, which was considered serious due to prolongation of hospitalization and due to medical significance. Ibuprofen was withdrawn.

From 28-Apr-2008, elevated transaminases and bilirubin had been decreasing (see 'Laboratory Test' section in this report).

The event was resolved on 26-Aug-2008.

The original reporter's clinical assessment:

The investigator considered the diagnosis ACUTE HEPATITIS to be possibly associated with Ibuprofen and Carbostesin (bupivacaine hydrochloride).

PVZ-FAKOS, Klinische Pharmakologie, 13.11.2009 DE-FAKOS-170215

Sender diagnosis: not reported

Narrative

Aufnahme der Patientin in der Ersten Hilfe eines Krankenhauses am 24.04.2008. Es bestanden seit ca. 1 Woche diffuser Bauchschmerz, Übelkeit, Ikterus sowie dunkler Urin. Die Werte der Transaminasen und des Bilirubins waren bei Aufnahme erhöht: ALAT: 710 U/l (Normbereich: <34 U/l), ASAT: 369 U/l (Normbereich: <35 U/l), Bilirubin: 6,3 mg/dl (Normbereich: 0-1 mg/dl). Es wurde ein positiver Titer für antinukleäre Antikörper festgestellt (ANA IgG und ANA/Hep 2). Serologisch erfolgte der Ausschluss einer viralen Hepatitis. Patientin wurde im Krankenhaus nur ambulant behandelt.

Bei weiteren ambulanten Kontrollen kam es zu einem Rückgang und schließlich zur Normalisierung der Transaminasen und des Bilirubins.

Laborwerte vom 26.08.2008: ALAT: 18 U/l, ASAT: 23 U/l, Bilirubin: 0,5 mg/dl.

Zuvor war, bei Zustand nach mehrfachen Operationen nach einem Trümmerbruch am linken kleinen Finger, der Patientin am 01.04.2008 ein Plexus axillaris-Katheter gelegt worden. Das darüber mehrfach verabreichte Carbostesin hatte eine schmerzfreie ergotherapeutische Beübung gewährleistet.

Die UAW hat sich im Krankenhaus entwickelt.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Ibuprofen Tabl., Einnahmezeitraum: 22.06.2007-29.02.2008 und 26.03.-13.04.2008 und

Carbostesin Amp., Einnahmezeitraum: 01.-09.04.2008.

Die Arzneimittel werden aufgrund der unklaren Ätiologie unter Hinsicht auf die positiven Antikörper-Titer mit „möglich“ bewertet.

sanofi-aventis, 09.12.2009 DE-SANOFI-AVENTIS-2009SA005238

Sender diagnosis: not reported

Narrative

Initial report: This serious case, extracted from the website of the German Health Authorities (id no. DE-BFARM-08142949) on 27-Nov-2009, is a report from a Regional pharmacovigilance centre (PV centre, id no. DE-FAKOS-170215).

The case involves a 37-year-old female patient (ex-smoker) with a medical history of fractured finger, abscess of Bartholin's gland, contracture of joint, and allergy to insect poison. On 01-Apr-2008, an axillar plexus catheter had been established in the patient, due to several operations after a comminuted fracture of the left small finger. The patient had been pre-treated with ibuprofen from 2-Jun-2007 to 29-Feb-2008 for ache. She started ibuprofen (trade name not provided) 3x400mg/day again on 26-Mar-2008 for ache; additionally, she started treatment with bupivacaine on 01-Apr-2008 in differing doses (via catheter). The following drugs are reported to have been taken concomitantly (not suspected): pantoprazole, sodium salt heparin fraction, metamizole, lorazepam, and tramadol.

On 13-Apr-2008, the patient developed acute hepatitis. Bupivacaine (last intake 09-Apr-2008) and ibuprofen (last intake 13-Apr-2008) were discontinued.

The patient went to the first aid department of the hospital on 24-Apr-2008 after one week of suffering from diffuse abdominal pain, nausea, icterus, and dark urine. At time of admission, transaminases and bilirubin were increased. A positive titre for antinuclear antibodies was determined (ANA IgG and ANA/Hep 2). Viral hepatitis was excluded by serology. The patient was not hospitalised, but only treated as out-patient. Further controls showed a regression of transaminases and bilirubin; by 26-Aug-2008, the values were within normal ranges.

According to information provided by PV centre, the adverse reaction started during hospitalisation.

The case was assessed as serious by German Health Authority with the criterion hospitalization and medically important condition (both fields ticked by German Health Authority).

Causality assessment was considered "possible" by the PV centre for ibuprofen as well as for bupivacaine, based on the unclear aetiology and under consideration of the positive antibody titres.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08145025

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 15 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute intermittent porphyria	20081201				recovered/resolved
2 Red urine	20081201				recovered/resolved
3 Depression					Unknown
4 Abdominal pain					not recovered/not resolved
5 Lack of drug effect					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DOLORMIN FUR FRAUEN BEI MENSTRUATI ibuprofen lysinate	Oral	/	-	Nausea
2 DOLORMIN FUR FRAUEN BEI MENSTRUATI ibuprofen lysinate	Oral	/	-	Dizziness
3 DOLORMIN FUR FRAUEN BEI MENSTRUATI ibuprofen lysinate	Oral	250 Mg milligram(s)/	20081201-20081201	Abdominal pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Abstains from alcohol			Unknown	
Non-smoker			Unknown	
Abdominal pain			Unknown	periodic for several years
Appendectomy			Unknown	
Colonoscopy			Unknown	
Blood pressure high			Unknown	since age 13 to 14 years old
Nausea	20081127		Unknown	
Dizziness	20081127		Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

McNeil Germany, Local Safety Officers, 11.12.2008 DE-JNJFOC-20081201218

Sender diagnosis: not reported

Narrative

This was a spontaneous report from a mother who is a pharmacist reporting on her daughter, a 15 year old female from Germany (00978/08) Initial case processed with additional information received on 04-DEC-2008.

The patient's weight was not reported. The patient's medical history and concurrent conditions included no alcohol, non smoker, abdominal pain (periodic for several years), appendectomy, colonoscopy, dizziness and nausea beginning approximately 27-NOV-2008, elevated blood pressure (since age 13 to 14 years old) and no known allergies or drug allergies. The patient previously treated her abdominal pain successfully with naproxen and ibuprofen (AER 20081202018), which did not improve the periodic abdominal pain. The patient was treated with naproxen (250mg/tablet, oral) 250 mg once total, on 01-DEC-2008 for periodic abdominal pain, nausea, and dizziness. On 01-DEC-2008, after taking the product, the patient developed abdominal pain and red colored urine. On 01-DEC-2008, a physician was consulted and an unspecified urine test showed leukocytes and bacteria but no hemoglobin to explain the red color. An unspecified test for acute intermittent (hereditary) porphyria was sent to a lab on 01-DEC-2008 (results pending). The patient had no psychotic symptoms and no tachycardia. During the episodes of abdominal pain, the patient was withdrawn, melancholic and had depressive tendencies. The patient's mother reported that as of 04-DEC-2008, the patient's initial complaints, why she took the naproxen in the first place, abdominal pain, nausea and dizziness were still present for one week. A 24 hour urine collection result was

pending. Treatment with naproxen was withdrawn on 01-DEC-2008. At the time of the report, the patient had not recovered from the abdominal pain and the outcome was unspecified for depressive tendencies with feeling withdrawn and melancholic episodes. The outcome for the red colored urine and possible porphyria was reported as recovered.

This report was serious (medically significant).

This case, involving the same patient is linked to 20081202018.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003331

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19660612	Age: 42 Year	weight (kg): 86	BMI_int: 27.14	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzyme increased	20081003	20081105	34 Day	4 Month	recovering/resolving
2 Drug interaction					recovering/resolving
3 Lassitude					recovering/resolving
4 Nausea					recovering/resolving
5 Vomiting					recovering/resolving
6 Jaundice	20081003				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac 50 diclofenac sodium	Oral	100 Mg milligram(s)/Day 245	20081017-20081018	Ache
2 ibuprofen ibuprofen	Oral	800 Mg milligram(s)/ 245	20081019-20081021	Ache
3 IBUPROFEN ibuprofen	Oral	800 Mg milligram(s)/ 245	20081019-20081019	Pain in spine
4 IBUPROFEN ibuprofen	Oral	800 Mg milligram(s)/ 245	20081021-20081021	Pain in spine
5 JANUVIA sitagliptin phosphate	Oral	100 Mg milligram(s)/Day 82	20070101-20081023	Type 2 diabetes mellitus
6 KATADOLON flupirtine maleate	Oral	200 Mg milligram(s)/Day 245	20080717-20081016	Pain
7 SIMVASTATIN simvastatin	Oral	20 Mg milligram(s)/Day 82	20080701-20081023	Hypercholesterolaemia

Drugs (concomitant)

8 METFORMIN metformin	Oral	200 Mg milligram(s)/Day 82	2003-20081023	Type 2 diabetes mellitus
9 PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/ 245	20081022-20081023	Nausea
10 VIAGRA sildenafil citrate	Oral	25 Mg milligram(s)/ 82	2005-	Potency disturbance

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ache			Yes	Stop Date: Continuing
Bone pain	20060101		Yes	
Hypercholesterolaemia	20080701		Yes	
Low back pain			Yes	Stop Date: Continuing
Spinal pain	2006		Yes	
Pain			Yes	
Potency disturbance			Yes	Stop Date: Continuing
Erectile dysfunction			Yes	
Type 2 diabetes mellitus	20030101		Yes	
Nausea			Yes	Stop Date: Continuing

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Other Health Professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20081024	38.72	µmol/s.l	
Alanine aminotransferase	20081105	18.3	µmol/s.l	
Alkaline phosphatase	20081105	2.76	µmol/l	
Alkaline phosphatase	20081024	3.14	µmol/l	
Aspartate aminotransferase	20081105	5.25	µmol/s.l	
Aspartate aminotransferase	20081024	15.07	µmol/s.l	
Bilirubin direct	20081024	31.4	µmol/l	
Bilirubin direct	20081105	17.9	µmol/l	
Bilirubin total	20081024	49.7	µmol/l	
Bilirubin total	20081105	33.3	µmol/l	
Calcium	20081024	2.39	mmol/l	
Creatinine	20081024	73.0	µmol/l	
Fibrinogen	20081024	2.21	g/l	
Gamma GT	20081024	12.21	mmol/s.l	
Gamma GT	20081105	9.97	mmol/s.l	
Haematocrit	20081024	0.47		
Haemoglobin	20081024	10.30	mmol/l	
HbA1C	20081030	9.20	Prozent	
INR	20081105	0.98		
INR	20081024	1.23		
Lactate dehydrogenase	20081105	4.16	µmol/l	
Lactate dehydrogenase	20081024	8.13	µmol/l	
Mean corpuscular haemoglobin	20081024	88.9	fl	
Partial prothrombin time	20081024	33.6	sec	
Potassium	20081024	3.85	mmol/l	
Sodium	20081024	134	mmol/l	
Thrombin time	20081025	23.5	sec	
Thrombocyte count	20081024	163	Gpt/l	
Urea	20081024	3.3	mmol/l	
Ammonia	20081025	58.9	µmol/l	
Antithrombin III	20081025	59	Prozent	
Quick's test	20081105	102	Prozent	
Quick's test	20081024	63.0	Prozent	
Urine microalbumin	20081030	151	mg/l	
Total white blood cell count	20081024	9.1	Gpt/l	

Narratives (nur bei elektronischem Eingang)Universitätsklinikum Jena, 12.01.2009 DE-PVZIBET-WEIMAR-10104

Sender diagnosis: not reported

Narrative

Sender's comment:

Initialmeldung - Zusatzinformationen werden erwartet

Reporter's comment:

NOVARTIS, 20.01.2009 DE-NOVNCHHQ-GXKR2009DE00717

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 14 Jan 2009 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-09003331), provided by a regional pharmacovigilance center, reported by a physician: This patient with a medical history of type II diabetes mellitus and pain in spine developed hepatic enzyme increased during treatment with simvastatin (MAH unknown) for hypercholesterolaemia, Diclac (diclofenac) for ache and ibuprofen (MAH unknown) for ache. Katadolon (flupirtine maleate) was also suspected. Several other drugs were taken concomitantly. The outcome of the adverse events was reported as not resolved. The reporter appraised the causality of the intake of simvastatine and the adverse event as possible, the intake of Diclac and ibuprofen and the adverse event as dubious.

information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

009507513, 21.01.2009 DE-MERCK-0901DEU00160

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 42-year-old male patient with type 2 diabetes mellitus, nausea, potency disturbance and pain in spine who, in July 2008, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hypercholesterolaemia (duration not reported).

Concomitant therapy included flupirtine maleate (KATADOLON), 200 mg tablet, 200 mg once a day for the treatment of pain (started on 17-JUL-2008).

Other prior and concomitant therapy included sitagliptin phosphate (JANUVIA) (MDS), metformin, pantoprazole sodium (PANTOZOL), sildenafil citrate (VIAGRA), diclofenac Na (DICLAC) and ibuprofen.

On 16-OCT-2008, therapy with flupirtine was stopped.

On 24-OCT-2008, therapy with simvastatin was discontinued (last dose taken on 23-OCT-2008) because that day laboratory analysis revealed increased values for AST, ALT, GGT, AP, direct and total bilirubin and LDH (values not provided). However, onset date for "increase of liver enzymes" was explicitly reported as 03-NOV-2008. The patient was hospitalized (date not reported).

On 05-NOV-2008, AST, ALT, GGT, AP, direct and total bilirubin and LDH were still increased. The patient was reported as not recovered from increase of liver enzymes.

The regional pharmacovigilance center felt that increase of liver enzymes was possibly related to therapy with simvastatin and very likely related to therapy with flupirtine maleate.

Additional information has been requested.

PLIVA Research and Development Ltd., 04.02.2009 DE-PLIVA Research and Development Ltd.-107#03#2009-00296

Sender diagnosis: not reported

Narrative

This case was received from German Health Authority-the BFARM (DE-BFARM-09003331) on 27-Jan-2009.

A 42-year-old male patient with pain in spine, hypercholesterolaemia and type 2 diabetes mellitus developed hepatic enzyme increased during treatment with Katadolon (flupirtine maleate-200 mg daily, oral) for ache from 17-Jul-2008 till 16-Oct-2008 and simvastatin (20 mg daily, film-coated tablet, oral) for hypercholesterolaemia from Jul-2008 till 23-Oct-2008.

Concomitant medications included Januvia (sitagliptin-100 mg daily, film-coated tablet, oral) for type 2 diabetes mellitus from 2007 till 23-Oct-2008, metformin (100 mg twice daily, film-coated tablet, oral) for type 2 diabetes mellitus from 2003 till 23-Oct-2008, Pantozol (pantoprazole sodium-40 mg daily, oral) for nausea from 22-Oct-2008 till 23-Oct-2008 and Viagra (sildenafil citrate-25 mg daily, film-coated tablet, oral) for potency disturbance since 2005.

Interacting drugs included Diclac (diclofenac sodium-50 mg twice daily, oral) for ache from 17-Oct-2008 till 18-Oct-2008 and ibuprofen (800 mg, oral) for ache on 19th and 21th Oct 2008.

Patient past drug therapy: Actos for type 2 diabetes mellitus from 2003 till 2007.

Laboratory investigations revealed decreased thrombocyte count, quick's test and sodium level on 24-Oct-2008. International normalised ratio, ASAT, ALAT, AP, GGT, Bilirubin total, Bilirubin direct and lactate dehydrogenase were increased on 24-Oct-2008.

On 25-Oct-2008 thrombin time and ammonia were increased and antithrombin III was decreased.

HbA1C and Urine microalbumin were increased on 30-Oct-2008.

On 05-Nov-2008 ASAT, ALAT, AP, GGT, Bilirubin total, Bilirubin direct and lactate dehydrogenase were increased.

The reaction onset was on 03-Nov-2008.

The outcome was stated as not recovered and the causality was reported as highly probable for Katadolon and possible for simvastatin.

HELIOS Klinikum Wuppertal, 04.10.2010 DE-PVZIBE-WEIMAR-10104

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberenzymanstieg

KURZZUSAMMENFASSUNG:

Zunehmende Abgeschlagenheit bei Leberenzymerrhöhung nach Katadoloneinnahme und unter Simvastatin

BESCHREIBUNG:

Patient wurde wegen einer seit 3 Wochen zunehmenden Abgeschlagenheit stationär aufgenommen. Als führendes Symptom zeigte sich laborchemisch ein beginnendes Leberversagen mit deutlicher Erhöhung von Bilirubin und Transaminasen, Abfall der Gerinnungsfunktion und Anstieg des Ammoniakspiegels. Patient hatte aufgrund von Kreuzschmerzen vom 17.07 - 16.10.08 Flupirtin eingenommen. Da die schmerzensenkende Wirkung aussetzte, wurde es abgesetzt und durch Diclofenac ersetzt. Aufgrund einer möglichen Fahrtauglichkeitseinschränkung wurde dieses nach 2 Tagen von Ibuprofen ersetzt. Seit dem 22.10.08 traten zusätzlich Übelkeit und ein einmaliges Erbrechen auf.

THERAPIE:

- vom 24.10 - 27.10.08: Sterofundin 1000 tgl. als Infusion

- vom 24.10. - 03.11.08: Konaktion 10 Trf. tgl.

- ab 24.10.08: Absetzen aller Medikamente

VERLAUF:

- Hepatitis-Serologie:

Anti-HAV-IgM, HBs-AG, Anti-HBc, Anti-HBc-IgM, Anti-HCV negativ

- Infektionsdiagnostik:

EBV- IgM negativ, EBV-EA-AK negativ, EBV-IgG 1:960, Rotaviren und Adenoviren negativ

- Autoimmundiagnostik:

ANA, Doppelstrang-DNA, AMA, p-, c-, ANCA, LKM, SLA sämtliche negativ

- 24.10.08 Oberbauchsonographie:

Leber grenzwertig groß, gering strukturverdichtet und konturabgerundet im Sinne einer Steatosis hepatis Grad 1.

- 30.10.08 Leberpunktion:

Das histomorphologische Bild zeigt Veränderungen wie sie im Rahmen einer medikamentös-toxischen Genese zu sehen sein können. Darüber hinaus zeigen sich stoffwechselbedingte Veränderungen bei Diabetes mellitus.

Nach Absetzen aller Medikamente langsame Besserung der pathologischen Laborparameter.

CDT vom 24.10.08: 2,08% (Der Wertebereich von 1,8-2,2% ist als Grauzone definiert. Die Interpretation der Meßergebnisse in diesem Bereich sollte mit der entsprechenden Vorsicht erfolgen.) Bei dem Patienten wurde Alkohol als toxisches Agens ausgeschlossen.

ERLÄUTERUNGEN ZUR UAW:

Lt. Fachinformation sind unter Simvastatin und Katadolon Leberenzym erhöhungen möglich. Lt. Entlassungsbrief liegt ein medikamentös-toxisches Lebersversagen nach Katadolon-/Simvastatin-Einnahme vor.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: KATADOLON

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: - Challenge: very suggestive

- Dechallenge: suggestive

Katadolon wird aufgrund des längeren Einnahmezeitraumes am stärksten verdächtigt

MEDIKAMENT: diclac 50

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: Lt. Fachinformation kann Diclofenac zur Erhöhung der Serumtransaminasen führen und bei Langzeittherapie zu Leberschäden. Da die (als Symptom der Leberschädigung interpretierte) Abgeschlagenheit schon vor Tabletteneinnahme eintrat, und eine sehr kurze Einnahmedauer vorliegt, wird es als ursächliches Medikament nicht verdächtigt, nur im Sinne einer Verstärkung der Symptome.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: Lt. Fachinformation kann Ibuprofen selten zu Leberschäden und Leberfunktionsstörungen führen, insbesondere bei Langzeitbehandlung. Da die (als Symptom der Leberschädigung interpretierte) Abgeschlagenheit schon vor Tabletteneinnahme eintrat, wird es nicht verdächtigt als ursächliches Medikament, sondern nur im Sinne einer Symptomverstärkung.

MEDIKAMENT: SIMVASTATIN

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: Lt. Fachinformation ist eine Transaminasenerhöhung bei Einnahme von 40 mg dokumentiert als seltenes Ereignis. Hier lag die Tagesdosis bei 20 mg, daher unwahrscheinlicher, aber nicht ausgeschlossen. Ebenfalls sind selten Hepatitis und Ikterus beschrieben.

MEDIKAMENT: JANUVIA

BETREFFENDE UAW: Leberenzym erhoeht

ERLÄUTERUNG: Januvia wird verdächtigt besonders hinsichtlich Chronologie.<dbtext>To_Char_NARRATIVE_2</dbtext>

009507513, 07.10.2010 DE-MERCK-0901DEU00160

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency concerning a 42-year-old male patient with type 2 diabetes mellitus, nausea, potency disturbance, low back pain, pain (unspecified) and pain in spine who, in July 2008, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hypercholesterolaemia (duration not reported).

Suspect therapy included flupirtine maleate (KATADOLON), 200 mg tablet, 200 mg once a day for the treatment of back pain (started on 17-JUL-2008) and sitagliptin phosphate (MSD), 100 mg tablet, 100 mg once a day for the treatment of type II diabetes mellitus.

Concomitant therapy included metformin, pantoprazole sodium (PANTOZOL) and sildenafil citrate (VIAGRA).

In the beginning of October 2008, the patient developed fatigue. On 16-OCT-2008, therapy with flupirtine was stopped due to missing analgesic effect. On 17-OCT-2008, suspect therapy with diclofenac Na (DICALAC), 50 mg tablet, 50 mg twice a day for the treatment of ache was initiated. On 18-OCT-2008, therapy with diclofenac Na (DICALAC) was discontinued and replaced with suspect therapy ibuprofen, 800 mg tablet, 800 mg once a day for the treatment of ache (taken on 19-OCT-2008 and 21-OCT-2008) due to possible reduced fitness to drive. On 21-OCT-2010, therapy with ibuprofen was discontinued.

On approximately 24-OCT-2008, the patient was hospitalized due to persisting fatigue. On 24-OCT-2008, therapy with simvastatin was discontinued (last dose taken on 23-OCT-2008) because that day laboratory analysis revealed increased liver enzymes. However, the patient was diagnosed with "increase of liver enzymes" and onset date was reported as 03-OCT-2008 in follow-up (the only by the agency coded adverse event). Blood analysis revealed INR 1.23/ units unknown, direct serum bilirubin 31.4 mcml/l, blood lactate dehydrogenase 8.13 mcml/l, serum alanine aminotransferase 38.72 mcml/s.l, total serum bilirubin 49.7 mcml/l, serum gamma-glutamyl transferase 12.21 mmol/s.l, serum aspartate aminotransferase 15.07 mcml/s.l and serum alkaline phosphatase 3.14 mcml/s.l. That same day, abdominal ultrasound revealed grade 1 hepatic steatosis and serum transferrin test (CDT) showed 2.08 %, however, alcohol was excluded as toxic agent.

As of 24-OCT-2008, the patient was treated with electrolytes (unspecified) (+) sodium lactate (STEROFUNDIN) i. v. and phytonadione (KONAKION), 10 drops daily.

On approximately 24-OCT-2008, laboratory analysis revealed negative deoxyribonucleic acid antibodies test, negative rotavirus and adenovirus test, negative serum Epstein-Barr viral capsid antigen immunoglobulin G antibody test (1:960/ units unknown), negative serum Epstein-Barr viral capsid antigen immunoglobulin M antibody test, negative serum Epstein-Barr virus early antigen immunoglobulin M antibody test, negative serum soluble liver antigen autoantibody test, negative serum perinuclear antineutrophil cytoplasmic antibody test (p-ANCA), negative serum cytoplasmic antineutrophil cytoplasmic antibody test (c-ANCA), negative serum liver and kidney microsomal antibody test, negative serum hepatitis C antibody test, negative serum hepatitis B surface antigen test, negative serum hepatitis B core immunoglobulin M antibody test, negative serum hepatitis B core immunoglobulin G antibody test, negative serum hepatitis A immunoglobulin M antibody test, negative serum antinuclear antibodies test (ANA) and negative serum antimitochondrial antibody test (AMA).

On 30-Oct-2008, hepatic biopsy showed medicamentous-toxic liver changes as well as metabolism-induced changes in the context of the patient's diabetes mellitus. That same day, whole blood glycosylated hemoglobin was 9.20 %.

In the further course, pathological laboratory parameters slightly decreased.

On 05-Nov-2008, blood analysis revealed INR 0.98/ units unknown, serum alkaline phosphatase 2.76 mcmol/s.l, serum alanine aminotransferase 18.3 mcmol/s.l, direct serum bilirubin 17.9 mcmol/l, total serum bilirubin 33.3 mcmol/l, serum gamma-glutamyl transferase 9.97 mmol/s.l, serum aspartate aminotransferase 5.25 mcmol/s.l, prothrombin time 102 % and blood lactate dehydrogenase 4.16 mcmol/l.

As of 05-NOV-2008, the patient was recovering from increase of liver enzymes, however the agency indicated 05-NOV-2008 as stopp date. According to the discharge letter, a medicamentous-toxic liver failure after flupirtine maleate (KATADOLON)/ simvastatin intake has taken place (not coded as AE by the agency).

The regional pharmacovigilance center felt that increase of liver enzymes were possibly related to therapy with simvastatin and/or sitagliptin phosphate and probably related to therapy with flupirtine maleate. Diclofenac Na (DICALAC) and ibuprofen were listed as possible interacting drugs.

Additional information is not expected.

sanofi-aventis, 18.10.2010 DE-SANOFI-AVENTIS-2010SA060395

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09003331) on 06-Oct-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

It's involved a 42-year-old male patient suffered from diabetes mellitus type II treated with metformin hydrochloride 100 mg/day starting in 2003 as well as with sitagliptin phosphate starting in 2007. The hypercholesterolemia was treated with simvastatin 20 mg/day from July 2008. For potency disturbance the patient took sildenafil citrate 25 mg as needed since 2005.

Due to pain in spine the patient following drugs were used: flupirtine maleate 200 mg/day was started on 17-Jul-2008. After decreasing of drug effect the patient switched to diclofenac, starting on 17-Oct-2008. Two days later the medication was again switched to ibuprofen (due to possible disability to drive a car), which was given until 21-Oct-2008. Simvastatin and sitagliptin phosphate were discontinued on 23-Oct-2008.

Since 03-Oct-2008 the patient noticed an increasing lassitude. Additionally he complained about nausea and one-time vomiting since 22-Oct-2008.

Two days later the patient was hospitalised and increased hepatic enzymes were revealed by lab tests.

Corrective treatment with pantoprazole 40 mg/day against nausea was given from 22-Oct-2008 to the next day, sterofundin 1000 ml from 24-Oct-2008 to 27-Oct-2008, phytomenadione 10 drops/day from 24-Oct-2008 to 03-Nov-2008. On 05-Nov-2008 the patient was recovering.

Reporter causality:

Possible for ibuprofen as interactive drug (aggravation of the existing symptoms)

Possible for simvastatin

sanofi-aventis, 14.01.2011 DE-SANOFI-AVENTIS-2010SA060395

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09003331) on 06-Oct-2010.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

It's involved a 42-year-old male patient suffered from diabetes mellitus type II treated with metformin hydrochloride 100 mg/day starting in 2003 as well

as with sitagliptin phosphate starting in 2007. The hypercholesterolemia was treated with simvastatin 20 mg/day from July 2008. For potency disturbance the patient took sildenafil citrate 25 mg as needed since 2005.

Due to pain in spine the patient following drugs were used: flupirtine maleate 200 mg/day was started on 17-Jul-2008. After decreasing of drug effect the patient switched to diclofenac, starting on 17-Oct-2008. Two days later the medication was again switched to ibuprofen (due to possible disability to drive a car), which was given until 21-Oct-2008. Simvastatin and sitagliptin phosphate were discontinued on 23-Oct-2008.

Since 03-Oct-2008 the patient noticed an increasing lassitude. Additionally he complained about nausea and one-time vomiting since 22-Oct-2008. Two days later the patient was hospitalised and increased hepatic enzymes were revealed by lab tests.

Corrective treatment with pantoprazole 40 mg/day against nausea was given from 22-Oct-2008 to the next day, sterofundin 1000 ml from 24-Oct-2008 to 27-Oct-2008, phytomenadione 10 drops/day from 24-Oct-2008 to 03-Nov-2008. On 05-Nov-2008 the patient was recovering.

Reporter causality:

Possible for ibuprofen as interactive drug (aggravation of the existing symptoms)

Possible for simvastatin

Significant correction was performed on 12-JAN-2011: hepatocellular damage in context of a drug interaction between ibuprofen, simvastatin, flupirtine, diclofenac and sitagliptin is now considered unlisted for ibuprofen and simvastatin.

009507513, 17.12.2012 DE-MERCK-0901DEU00160

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency via a regional Pharmacovigilance center, concerning a 42-year-old male patient with type 2 diabetes mellitus, nausea, potency disturbance, low back pain, pain (unspecified) and pain in spine who, in July 2008, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day, orally for the treatment of hypercholesterolemia (duration not reported).

Suspect therapy included flupirtine maleate (KATADOLON), 200 mg tablet, 200 mg once a day for the treatment of back pain (started on 17-JUL-2008) and sitagliptin phosphate (JANUVIA), 100 mg tablet, 100 mg once a day for the treatment of type II diabetes mellitus. Concomitant therapy included metformin, pantoprazole sodium (PANTOZOL) and sildenafil citrate (VIAGRA).

In the beginning of October 2008, the patient developed fatigue. On 16-OCT-2008, therapy with flupirtine was stopped due to missing analgesic effect. On 17-OCT-2008, suspect therapy with diclofenac Na (DICAL), 50 mg tablet, 50 mg twice a day for the treatment of ache was initiated. On 18-OCT-2008, therapy with diclofenac Na (DICAL) was discontinued and replaced with suspect therapy ibuprofen, 800 mg tablet, 800 mg once a day for the treatment of ache (taken on 19-OCT-2008 and 21-OCT-2008) due to possible reduced fitness to drive. On 21-OCT-2010, therapy with ibuprofen was discontinued.

On approximately 24-OCT-2008, the patient was hospitalized due to persisting fatigue. On 24-OCT-2008, therapy with simvastatin was discontinued (last dose taken on 23-OCT-2008) because that day laboratory analysis revealed increased liver enzymes. However, the patient was diagnosed with "increase of liver enzymes" and onset date was reported as 03-OCT-2008 in follow-up (the only by the agency coded adverse event). Blood analysis revealed INR 1.23/ units unknown, direct serum bilirubin 31.4 mcml/l, blood lactate dehydrogenase 8.13 mcml/l, serum alanine aminotransferase 38.72 mcml/s.l, total serum bilirubin 49.7 mcml/l, serum gamma-glutamyl transferase 12.21 mmol/s.l, serum aspartate aminotransferase 15.07 mcml/s.l and serum alkaline phosphatase 3.14 mcml/s.l. That same day, abdominal ultrasound revealed grade 1 hepatic steatosis and serum transferrin test (CDT) showed 2.08 %, however, alcohol was excluded as toxic agent.

As of 24-OCT-2008, the patient was treated with electrolytes (unspecified) (+) sodium lactate (STEROFUNDIN) i. v. and phytonadione (KONAKION), 10 drops daily.

On approximately 24-OCT-2008, laboratory analysis revealed negative deoxyribonucleic acid antibodies test, negative rotavirus and adenovirus test, negative serum Epstein-Barr viral capsid antigen immunoglobulin G antibody test (1:960/ units unknown), negative serum Epstein-Barr viral capsid antigen immunoglobulin M antibody test, negative serum Epstein-Barr virus early antigen immunoglobulin M antibody test, negative serum soluble liver antigen autoantibody test, negative serum perinuclear antineutrophil cytoplasmic antibody test (p-ANCA), negative serum cytoplasmic antineutrophil cytoplasmic antibody test (c-ANCA), negative serum liver and kidney microsomal antibody test, negative serum hepatitis C antibody test, negative serum hepatitis B surface antigen test, negative serum hepatitis B core immunoglobulin M antibody test, negative serum hepatitis B core immunoglobulin G antibody test, negative serum hepatitis A immunoglobulin M antibody test, negative serum antinuclear antibodies test (ANA) and negative serum antimitochondrial antibody test (AMA).

On 30-Oct-2008, hepatic biopsy showed medicamentous-toxic liver changes as well as metabolism-induced changes in the context of the patient's diabetes mellitus. That same day, whole blood glycosylated hemoglobin was 9.20 %.

In the further course, pathological laboratory parameters slightly decreased.

On 05-Nov-2008, blood analysis revealed INR 0.98/ units unknown, serum alkaline phosphatase 2.76 mcml/s.l, serum alanine aminotransferase 18.3 mcml/s.l, direct serum bilirubin 17.9 mcml/l, total serum bilirubin 33.3 mcml/l, serum gamma-glutamyl transferase 9.97 mmol/s.l, serum aspartate aminotransferase 5.25 mcml/s.l, prothrombin time 102 % and blood lactate dehydrogenase 4.16 mcml/l.

As of 05-NOV-2008, the patient was recovering from increase of liver enzymes, however the agency indicated 05-NOV-2008 as stop date. According to the discharge letter, a medicamentous-toxic liver failure after flupirtine maleate (KATADOLON)/ simvastatin intake has taken place (not coded as AE by the agency).

The regional Pharmacovigilance center felt that increase of liver enzymes were possibly related to therapy with simvastatin and/or sitagliptin phosphate and probably related to therapy with flupirtine maleate. Diclofenac Na (DICALAC) and ibuprofen were listed as possible interacting drugs.

This report was identified from a line listing obtained on request by the company from the FDA under the freedom of information act on 06-OCT-2010. It was reported that the patient's concurrent conditions included bone pain. It was reported that the patient's onset date for hepatic enzyme increased was 03-OCT-2008 (previously reported as 24-oct-2008), and it was reported that on the same date, the patient experienced Drug interaction. It was also stated that in 2007, the patient started therapy with sitagliptin phosphate (JANUVIA), 100 mg once a day, orally for type 2 diabetes mellitus. It was reported that the start date for flupirtine maleate (KATADOLON) was 17-JUL-2008 (also reported as 14-JUL-2008). On unspecified dates laboratory tests such as coagulating function, ammonia level, retrovirus test and adenovirus test were performed with results of: decreased, increased, negative and negative, respectively.

At the time of the report, the patient's outcome for both events was updated from recovering to not recovered.

Additional information is not expected.

009507513, 27.12.2012 DE-MERCK-0901DEU00160

Sender diagnosis: not reported

Narrative

Information has been received from a physician via a regional pharmacovigilance center and the German agency via a regional Pharmacovigilance center, concerning a 42-year-old male patient with type 2 diabetes mellitus, nausea, potency disturbance, low back pain, pain (unspecified) and pain in spine since 2006 who, in July 2008, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day, orally for the treatment of hypercholesterolemia (duration not reported).

Suspect therapy included flupirtine maleate (KATADOLON), 200 mg tablet, 200 mg once a day for the treatment of back pain (started on 17-JUL-2008) and sitagliptin phosphate (JANUVIA), 100 mg tablet, 100 mg once a day for the treatment of type II diabetes mellitus. Concomitant therapy included metformin, pantoprazole sodium (PANTOZOL) and sildenafil citrate (VIAGRA).

In the beginning of October 2008, the patient developed fatigue. On 16-OCT-2008, therapy with flupirtine was stopped due to missing analgesic effect. On 17-OCT-2008, suspect therapy with diclofenac Na (DICALAC), 50 mg tablet, 50 mg twice a day for the treatment of ache was initiated. On 18-OCT-2008, therapy with diclofenac Na (DICALAC) was discontinued and replaced with suspect therapy ibuprofen, 800 mg tablet, 800 mg once a day for the treatment of ache (taken on 19-OCT-2008 and 21-OCT-2008) due to possible reduced fitness to drive. On 21-OCT-2010, therapy with ibuprofen was discontinued.

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As of 24-OCT-2008, the patient was treated with electrolytes (unspecified) (+) sodium lactate (STEROFUNDIN) i. v. and phytonadione (KONAKION), 10 drops daily.

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On 30-OCT-2008, hepatic biopsy showed medicamentous-toxic liver changes as well as metabolism-induced changes in the context of the patient's diabetes mellitus. That same day, whole blood glycosylated hemoglobin was 9.20 %.

In the further course, pathological laboratory parameters slightly decreased.

On 05-NOV-2008, blood analysis revealed INR 0.98/ units unknown, serum alkaline phosphatase 2.76 mcml/s.l, serum alanine aminotransferase 18.3 mcml/s.l, direct serum bilirubin 17.9 mcml/l, total serum bilirubin 33.3 mcml/l, serum gamma-glutamyl transferase 9.97 mmol/s.l, serum aspartate aminotransferase 5.25 mcml/s.l, prothrombin time 102 % and blood lactate dehydrogenase 4.16 mcml/l.

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The regional Pharmacovigilance center felt that increase of liver enzymes were possibly related to therapy with simvastatin and/or sitagliptin phosphate and probably related to therapy with flupirtine maleate. Diclofenac Na (DICALAC) and ibuprofen were listed as possible interacting drugs.

This report was identified from a line listing obtained on request by the company from the FDA under the freedom of information act on 06-OCT-2010. It was reported that the patient's concurrent conditions included bone pain. It was reported that the patient's onset date for hepatic enzyme increased was 03-OCT-2008 (previously reported as 24-OCT-2008), and it was reported that on the same date, the patient experienced Drug interaction. It was also stated that in 2007, the patient started therapy with sitagliptin phosphate (JANUVIA), 100 mg once a day, orally for type 2 diabetes mellitus. It was reported that the start date for flupirtine maleate (KATADOLON) was 17-JUL-2008 (also reported as 14-JUL-2008). On unspecified dates laboratory tests such as coagulating function, ammonia level, retrovirus test and adenovirus test were performed with results of: decreased, increased, negative and negative, respectively.

At the time of the report, the patient's outcome for both events was updated from recovering to not recovered.

Follow up information has been received from a line listing obtained on request by the company from the FDA under the freedom of information act on 19-AUG-2011. It was reported that on 05-NOV-2008, the patient was recovering from hepatic enzymes increased and that drug interaction was not an adverse reaction.

Additional information is not expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09016274

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hyperbilirubinaemia					Unknown
2 Neurodevelopmental disorder					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		/	-	Patent ductus arteriosus
ibuprofen			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Patent ductus arteriosus			Unknown	hemodynamically relevant
Birth weight low			Unknown	
Premature baby			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Rheinlaender C, Helfenstein D, Walch E, Berns M, Obladen M, Koehne P. Total serum bilirubin levels during cyclooxygenase Inhibitor treatment for patent ductus arteriosus in preterm infants. Acta Pediatrics 2009;98:36-42.	
Qualification:	Other Health Professional	observestudytype:
Literature:	Rheinlaender C, Helfenstein D, Walch E, Berns M, Obladen M, Koehne P. Total serum bilirubin levels during cyclooxygenase Inhibitor treatment for patent ductus arteriosus in preterm infants. Acta Pediatrics 2009;98:36-42.	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin total		elevated total serum bilirubin (TBC)		

Narratives (nur bei elektronischem Eingang)

McNeil Germany, Local Safety Officers, 06.02.2009 DE-JNJFOC-20090104673

Sender diagnosis: not reported

Narrative

Report received from literature: Rheinlaender C, Helfenstein D, Walch E, Berns M, Obladen M, Koehne P. Total serum bilirubin levels during cyclooxygenase Inhibitor treatment for patent ductus arteriosus in preterm infants. Acta Pediatrics 2009; 98: 36-42.

This literature does not meet report criteria as there were no specific patient identifiers provided.

Fifty two preterm infants (demographics unspecified) with a medical history of premature low birth weight and hemodynamically relevant patent ductus arteriosus were treated with Ibuprofen (unspecified) a regime of 10-5-5 mg/kg at 24-hour intervals, on unspecified dates for the treatment of patent ductus arteriosus. The infants experienced elevated total serum bilirubin (TBC) and neurodevelopmental disorder.

This report is serious (hospitalization).

Case invalidated as non-company drug given based on additional information received from a physician on 03-FEB-2009<dbtext>
To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09023306

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19620614	Age: 46 Year	weight (kg): 74.30	BMI_int: 24.83	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20080715	20080723	9 Day	4 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	2*600 Mg milligram(s)/ Da 82	20080706-20080717 12 Day	Pain NOS
2 Novalgin metamizole	Oral	3*20 Gtt drop(s)/ Day 169	20080712-20080713 2 Day	Fever

Drugs (concomitant)

3 DOXYCYCLINE doxycycline	Oral	200 Mg milligram(s)/ Day 5	20080717-20080717 -	Tick bite
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Infection respiratory	20080706	20080718	No	
Knee injury	200807		Unknown	
Lyme borreliosis	200806		No	V.a., nicht bestätigt

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20080718	3.79	µmol/l*s	
Alanine aminotransferase	20080718	5.6	µmol/l*s	
Gamma GT	20080718	6.7	µmol/l*s	
Alkaline phosphatase	20080718	4.55	µmol/l*s	
LDH	20080718	6.8	µmol/l*s	
Aspartate aminotransferase	20080719	2.63	µmol/l*s	
Alanine aminotransferase	20080719	4.6	µmol/l*s	
Gamma GT	20080719	5.3	µmol/l*s	
Aspartate aminotransferase	20080720	0.63	µmol/l*s	
Alanine aminotransferase	20080720	2.0	µmol/l*s	
Gamma GT	20080720	4.3	µmol/l*s	
Alkaline phosphatase	20080720	3.0	µmol/l*s	
Aspartate aminotransferase	20080723	0.5	µmol/l*s	
Alanine aminotransferase	20080723	1.53	µmol/l*s	
Gamma GT	20080723	3.99	µmol/l*s	

Alkaline phosphatase	20080723	2.76	μmol/l*s
Bilirubin total	20080718	6	μmol/l
Creatinine	20080718	76	μmol/l

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 24.02.2009 DE-PVZIBE-JENA-10002

Sender diagnosis: not reported

Narrative

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Leberenzyme erhoeht

; Nebenwirkung: Leberwerterhöhung (Micromedex; RL). Junge Patientin kommt wegen Leberwerterhöhung nach Ibuprofen und Novalgineinnahme und grippalem Infekt zur Abklärung. Stationär konnte eine Infektion mit EBV, CMV und Hepatitis A/B/C-Viren ausgeschlossen werden. Für eine autoimmune Genese fand sich kein Anhalt. Klinisch wurde die passagere Leberwerterhöhung am ehesten Ibuprofen zugeschrieben. Klin.-pharmakologisch wird der Zusammenhang nicht so sicher gesehen, da auch ein Grippe-Virus Auswirkungen auf die Leberfunktion haben kann.

MEDIKAMENT: NOVALGIN

/SCH/

BETREFFENDE UAW: Leberenzyme erhoeht

; Junge Patientin kommt wegen Leberwerterhöhung nach Ibuprofen und Novalgineinnahme und grippalem Infekt zur Abklärung. Stationär konnte eine Infektion mit EBV, CMV und Hepatitis A/B/C-Viren ausgeschlossen werden. Für eine autoimmune Genese fand sich kein Anhalt. Klinisch wurde die passagere Leberwerterhöhung am ehesten Ibuprofen zugeschrieben. Klin.-pharmakologisch wird der Zusammenhang nicht so sicher gesehen, da auch ein Grippe-Virus Auswirkungen auf die Leberfunktion haben kann.

MEDIKAMENT: DOXYCYCLINE

BETREFFENDE UAW: Leberenzyme erhoeht

; in ZNA abgesetzt, kein zeitlicher Zusammenhang

ANMERKUNGEN ZU FOLGENDER UAW:

Leberwerterhöhung unter Ibuprofen, Novalgin

KURZZUSAMMENFASSUNG:

Leberwerterhöhung unter Ibuprofen, Novalgin

BESCHREIBUNG:

Patientin kommt mit Einweisung durch Hausarzt wegen unklarer Leberwerterhöhung und allgemeiner Schwäche.

06.07.08 - 17.07.08 Einnahme von Ibuprofen 600 nach Knieverletzung.

12./13.07. Fieber bis 39°C und Einnahme von Novalgin-Tropfen, verordnet vom Bereitschaftsarzt (keine Wirkung). Am 15.07.08 kein Fieber mehr, aber noch Schweißausbrüche, allgemeine Schwäche, Kräfteverfall.

17.07.2008 Hausärztin konsultiert, Doxycyclin wegen Zeckenbiß verordnet, Einweisung in ZNA nach Blutwertbefund.

THERAPIE:

- Klinikeinweisung durch Hausärztin am 18.07.2008
- Ibuprofen abgesetzt am 17.07.08 (selbstständig)
- Überwachung in Klinik und Ursachendiagnostik

VERLAUF:

- in Klinik Sono-Abdomen,
- EBV, CMV, Hepatitisserologie, Autoimmunhepatitis-Serologie untersucht, 21.07.08 EBV-Befund spricht für länger zurückliegende Infektion, CMV + Hepatitis-Serologie neg., kein Anhalt für Autoimmunhepatitis, Patientin fühlt sich aktuell wohl,
- ab Aufenthalt Enzymerhöhung rückläufig,

ERLÄUTERUNGEN ZUR UAW:

sicher erfolgte die Einweisung bei erhöhten Leberenzymen zur Abklärung auch hinsichtlich eines Infekts<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 27.02.2009 DE-NOVNCHHQ-GXKR2009DE02367

Sender diagnosis: not reported

Narrative

Initial report received from health authority (DE-BFARM-09023306) on 26 Feb 2009: This multimorbid patient developed an increase of hepatic enzymes during the therapy with ibuprofen (MAH unknown) and Novalgin (metamizole). Outcome was provided as recovered. Causality was

assessed as likely (for ibuprofen) and possible (for Novalgin).

Serious spontaneous report of hepatic enzymes increased (hospitalisation) already listed in the Core Data Sheet Information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Bayer Vital GmbH, 06.03.2009 DE-BAYER-200914971GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was identified on 25-Feb-2009 in an internet-based pool of cases of BfArM, the regulatory health authority in Germany, reported from a physician in Germany, and referring to a 46 year-old female (height: 173 cm, weight: 74.3 kg).

Patient suffered from concomitant flu and respiratory infection from 06-Jul-2008 until 18-Jul-2008.

Patient received concomitant oral Doxycycline 200 mg once for tick bite on 17-Jul-2008.

Patient received the following drugs reported as 'suspect drugs':

--oral Ibuprofen 600 mg film-coated tablet was given twice daily for pain NOS after a knee injury also during fever period from 06-Jul-2008 until 17-Jul-2008,

--Novalgin (metamizole sodium monohydrate) 20 drops oral solution was prescribed by a doctor on duty to be taken thrice daily for fever up to 39°C from 12-Jul-2008 to 13-Jul-2008. Drug showed lack of efficacy, therefore was withdrawn by patient.

On 15-Jul-2008, fever had subsided, patient, however, still experienced sweating, general weakness, and strength decline.

On 17-Jul-2008, patient visited her general practitioner (GP) and was prescribed Doxycycline for a tick bite.

On 18-Jul-2008, patient was admitted to hospital in emergency unit by her GP due to an unclear HEPATIC ENZYMES INCREASED and to general weakness.

During stay in hospital starting, infection with epstein-barr-virus (EBV), cytomegalovirus, and hepatitis A, B, and C could be excluded by hepatitis/autoimmun-serology. On 21-Jul-2008, EBV result suggested EBV infection long time back in the past. Cytomegalovirus, and hepatitis A, B, and C serology were negative.

There was nothing to suggest an autoimmune reaction.

At time of report, the patient was feeling well. During the patient's stay in hospital, the enzymes decreased.

The event was resolved on 23-Jul-2008.

Causality assessment (reporter) according to Begaud algorithm for Ibuprofen: 'likely' and Novalgin (metamizol): 'possible'.

Sanofi-Aventis Deutschland GmbH, 09.03.2009 DE-AVENTIS-200910525DE

Sender diagnosis: not reported

Narrative

Initial report: This serious case was extracted from the website of the German health authority (authority number DE-BFARM-09023306). It was reported by a physician via a regional pharmacovigilance centre (identification no. DE-PVZIBE-JENA-10002). The case involves a 46-year-old female patient with respiratory infection and knee injury. No concomitant drug was reported to have been taken at time of onset of event. The patient started treatment with metamizole sodium (Novalgin) "20 drops, 3 in day", from 12- to 13-Jul-2008 for fever and with ibuprofen "600 mg, 2 in day" from 06- to 17-Jul-2008 for unspecified pain after knee infection. Metamizole was discontinued by the patient because of lack of drug effect on 13-Jul-2008. Two days later, on 15-Jul-2008, the patient suffered from outbreak of sweat, general weakness and decomposition of strength. On 17-Jul-2008 she consulted her family

doctor who prescribed doxycycline for tick bite, after laboratory results were available, the patient was referred to hospital; ibuprofen had already been discontinued on 17-Jul-2008 by the patient herself. On 18-Jul-2008, laboratory tests showed increased liver test values with alanine aminotransferase at 10 ULN, aspartate aminotransferase at 7.3 ULN, alkaline phosphatase at 2.3 ULN and gamma glutamyltransferase at 10.6 ULN. Two days later, the values had decreased with alanine aminotransferase at 3.5 ULN, aspartate aminotransferase at 1.2 ULN, alkaline phosphatase at 1.5 ULN and gamma glutamyltransferase at 6.8 ULN.

Abdominal sonography, EBV, CMV, hepatitis and autoimmune-hepatitis serology were performed. EBV results spoke for an infection in the past, CMV hepatitis, and autoimmune-hepatitis serology were negative, abdominal sonography was performed, but results were not provided in this report.

During hospitalisation, liver values decreased and normalised on 23-Jul-2008. The patient recovered completely.

Causality assessment according to Begaud algorithm of pharmacovigilance centre:

"likely" for ibuprofen

"possible" for metamizole

"dubious" for doxycycline

Elucidation of causality assessments:

For ibuprofen and metamizole: in hospital an infection with EBV, CMV and hepatitis A/B/C viruses could be excluded. There was no hint for an autoimmune etiology. Clinically, a transient increase in liver values was assumed as most likely related to ibuprofen. Clinical-pharmacologically, the relationship with ibuprofen was not seen as thus likely, because an influenza virus could have an impact on the liver function, as well.

For doxycycline: no temporal relationship.

Laboratory values:

18-Jul-2008: aspartate aminotransferase 3.79 umol/L*s (normal up to 0.52), alanine aminotransferase 5.6 umol/L*s (up to 0.56), gamma-GT 6.7 umol/L*s (up to 0.63), alkaline phosphatase 4.55 umol/L*s (up to 2), LDH 6.8 umol/L*s (up to 4.12)

19-Jul-2008: aspartate aminotransferase 2.63 umol/L*s, alanine aminotransferase 4.6 umol/L*s, gamma-GT 4.3 umol/L*s

20-Jul-2008: aspartate aminotransferase 0.63 umol/L*s, alanine aminotransferase 2.0 umol/L*s, gamma-GT 4.3 umol/L*s, alkaline phosphatase 3.0 umol/L*s

23-Jul-2008: aspartate aminotransferase 0.5 umol/L*s, alanine aminotransferase 1.53 umol/L*s, gamma-GT 3.99 umol/L*s, alkaline phosphatase 2.76 umol/L*s

HELIOS Klinikum Wuppertal, 29.01.2010 DE-PVZIBE-JENA-10002

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberwerterhöhung

KURZZUSAMMENFASSUNG:

Leberwerterhöhung unter Ibuprofen, Novalgin

BESCHREIBUNG:

Patientin kommt mit Einweisung durch Hausarzt wegen unklarer Leberwerterhöhung und allgemeiner Schwäche.

06.07.08 - 17.07.08 Einnahme von Ibuprofen 600 nach Knieverletzung.

12./13.07. Fieber bis 39°C und Einnahme von Novalgin-Tropfen, verordnet vom Bereitschaftsarzt (keine Wirkung). Am 15.07.08 kein Fieber mehr, aber noch Schweißausbrüche, allgemeine Schwäche, Kräfteverfall.

17.07.2008 Hausärztin konsultiert, Doxycyclin wegen Zeckenbiß verordnet, Einweisung in ZNA nach Blutwertbefund.

THERAPIE:

- Klinikeinweisung durch Hausärztin am 18.07.2008
- Ibuprofen abgesetzt am 17.07.08 (selbstständig)
- Überwachung in Klinik und Ursachendiagnostik

VERLAUF:

- in Klinik Sono-Abdomen,
- EBV, CMV, Hepatitisserologie, Autoimmunhepatitis-Serologie untersucht, 21.07.08 EBV-Befund spricht für länger zurückliegende Infektion, CMV + Hepatitis-Serologie neg., kein Anhalt für Autoimmunhepatitis, Patientin fühlt sich aktuell wohl, ab Aufenthalt Enzymerhöhung rückläufig

ERLÄUTERUNGEN ZUR UAW:

Aufnahme erfolgte zur Abklärung der unklaren Leberwerterhöhung

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Leberenzyme erhoeht

ERLÄUTERUNG: Nebenwirkung: Leberwerterhöhung (Micromedex; RL). Junge Patientin kommt wegen Leberwerterhöhung nach Ibuprofen und Novalgineinnahme und grippalem Infekt zur Abklärung. Stationär konnte eine Infektion mit EBV, CMV und Hepatitis A/B/C-Viren ausgeschlossen werden. Für eine autoimmune Genese fand sich kein Anhalt. Klinisch wurde die passagere Leberwerterhöhung am ehesten Ibuprofen zugeschrieben. Klin.-pharmakologisch wird der Zusammenhang nicht so sicher gesehen, da auch ein Grippe-Virus Auswirkungen auf die Leberfunktion haben kann.

MEDIKAMENT: NOVALGIN /SCH/

BETREFFENDE UAW: Leberenzyme erhoeht

ERLÄUTERUNG: Junge Patientin kommt wegen Leberwerterhöhung nach Ibuprofen und Novalgineinnahme und grippalem Infekt zur Abklärung. Stationär konnte eine Infektion mit EBV, CMV und Hepatitis A/B/C-Viren ausgeschlossen werden. Für eine autoimmune Genese fand sich kein Anhalt. Klinisch wurde die passagere Leberwerterhöhung am ehesten Ibuprofen zugeschrieben. Klin.-pharmakologisch wird der Zusammenhang nicht so sicher gesehen, da auch ein Grippe-Virus Auswirkungen auf die Leberfunktion haben kann.

MEDIKAMENT: DOXYCYCLINE

BETREFFENDE UAW: Leberenzyme erhoeht

ERLÄUTERUNG: in Notaufnahme abgesetzt, kein zeitlicher Zusammenhang, da sich UAW vor Medikamenteneinnahme entwickelte.<dbtext

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sanofi-aventis, 11.02.2010 DE-SANOFI-AVENTIS-200910525DE

Sender diagnosis: not reported

Narrative

Initial report: This serious case was extracted from the website of the German health authority (authority number DE-BFARM-09023306). It was reported by a physician via a regional pharmacovigilance centre (identification no. DE-PVZIBE-JENA-10002). The case involves a 46-year-old female patient with respiratory infection and knee injury. No concomitant drug was reported to have been taken at time of onset of event. The patient started treatment with metamizole sodium (Novalgine) "20 drops, 3 in day", from 12- to 13-Jul-2008 for fever and with ibuprofen "600 mg, 2 in day" from 06- to 17-Jul-2008 for unspecified pain after knee infection. Metamizole was discontinued by the patient because of lack of drug effect on 13-Jul-2008. Two days later, on 15-Jul-2008, the patient suffered from outbreak of sweat, general weakness and decomposition of strength. On 17-Jul-2008 she consulted her family doctor who prescribed doxycycline for tick bite, after laboratory results were available, the patient was referred to hospital; ibuprofen had already been discontinued on 17-Jul-2008 by the patient herself. On 18-Jul-2008, laboratory tests showed increased liver test values with alanine aminotransferase at 10 ULN, aspartate aminotransferase at 7.3 ULN, alkaline phosphatase at 2.3 ULN and gamma glutamyltransferase at 10.6 ULN. Two days later, the values had decreased with alanine aminotransferase at 3.5 ULN, aspartate aminotransferase at 1.2 ULN, alkaline phosphatase at 1.5 ULN and gamma glutamyltransferase at 6.8 ULN.

Abdominal sonography, EBV, CMV, hepatitis and autoimmune-hepatitis serology were performed. EBV results spoke for an infection in the past, CMV hepatitis, and autoimmune-hepatitis serology were negative, abdominal sonography was performed, but results were not provided in this report.

During hospitalisation, liver values decreased and normalised on 23-Jul-2008. The patient recovered completely.

Causality assessment according to Begaud algorithm of pharmacovigilance centre:

"likely" for ibuprofen

"possible" for metamizole

"dubious" for doxycycline

Elucidation of causality assessments:

For ibuprofen and metamizole: in hospital an infection with EBV, CMV and hepatitis A/B/C viruses could be excluded. There was no hint for an autoimmune etiology. Clinically, a transient increase in liver values was assumed as most likely related to ibuprofen. Clinical-pharmacologically, the relationship with ibuprofen was not seen as thus likely, because an influenza virus could have an impact on the liver function, as well.

For doxycycline: no temporal relationship.

Laboratory values:

18-Jul-2008: aspartate aminotransferase 3.79 umol/L*s (normal up to 0.52), alanine aminotransferase 5.6 umol/L*s (up to 0.56), gamma-GT 6.7 umol/L*s (up to 0.63), alkaline phosphatase 4.55 umol/L*s (up to 2), LDH 6.8 umol/L*s (up to 4.12)

19-Jul-2008: aspartate aminotransferase 2.63 umol/L*s, alanine aminotransferase 4.6 umol/L*s, gamma-GT 4.3 umol/L*s

20-Jul-2008: aspartate aminotransferase 0.63 umol/L*s, alanine aminotransferase 2.0 umol/L*s, gamma-GT 4.3 umol/L*s, alkaline phosphatase 3.0 umol/L*s

23-Jul-2008: aspartate aminotransferase 0.5 umol/L*s, alanine aminotransferase 1.53 umol/L*s, gamma-GT 3.99 umol/L*s, alkaline phosphatase 2.76 umol/L*s

Additional information received on 03-Feb-2010:

This follow up information was also extracted from the website of the German health authority and had been created by pharmacovigilance centre. The suspicion of lyme borreliosis was not confirmed (bilirubin and creatinine were within normal ranges on 18-Jul-2008). Ibuprofen (no tradename provided) is identified as a potential company drug. No further information was received so far.

Reporter causality (WHO Score): possible for ibuprofen and metamizole sodium

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09030615

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 19890525	Age: 19 Year	weight (kg): 78	BMI_int: 22.79	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Aspartate aminotransferase increase	20081128				recovered/resolved with sequel
2 Mycoplasma infection NOS	20081125				recovered/resolved with sequel
3 Transaminases increased	20081125	20081212	18 Day	15 Day	recovered/resolved
4 Gamma GT raised	20081128				recovered/resolved with sequel
5 Adynamia					Unknown
6 Blood sodium decreased					Unknown
7 C-reactive protein increased	200811				recovering/resolving
8 Exanthema	20081125				recovering/resolving
9 Fever	20081125				recovering/resolving
10 Joint pain					Unknown
11 Reduced general condition	200808				Unknown
12 Exanthema	20081125				recovered/resolved with sequel
13 Fever	20081125				recovered/resolved with sequel
14 Transaminases increased	20081125				recovered/resolved with sequel
15 Alanine aminotransferase increase	20081128				recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprobay ciprofloxacin hydrochloride	Unknown	/	20081128-20081207 10 Day	Mycoplasma infection
2 ERGENYL CHRONO valproate sodium; valproic acid	Oral	500 Mg milligram(s)/Day 245	200705- -	Epilepsy
3 ibuprofen ibuprofen	Oral	400 Mg milligram(s)/Day 245	20081206-20081208 3 Day	Migraine
4 Lamotrigin lamotrigine	Oral	200 Mg milligram(s)/Day 245	20081110-20081210 31 Day	Epilepsy
5 PANTOZOL pantoprazole sodium	Oral	20 Mg milligram(s)/Day 245	2001-20081210 7 Year	Reflux oesophagitis
6 RANITIDINE ranitidine	Oral	150 Mg milligram(s)/Day 245	2001-20081210 7 Year	Reflux oesophagitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Epilepsy		200712	Yes	letzter Anfall 12/2007
Reflux oesophagitis	2001		Yes	
Weakness	200808		Yes	
Migraine			Yes	
Joint pain	200808		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Haemoglobin	20081128	8.63	mmol/l	
Haematocrit	20081128	0.42	NA	
Mean corpuscular haemoglobin	20081128	96.4	fl	
Total white blood cell count	20081128	5.8	Gpt/l	
Thrombocyte count	20081128	137	Gpt/l	
Quick's test	20081128	90.8	Prozent	
International normalised ratio	20081128	1.05	NA	
Partial prothrombin time	20081128	35.1	sec	
Fibrinogen	20081128	3.4	g/l	
Sodium	20081128	133	mmol/l	
Potassium	20081128	4.05	mmol/l	
Calcium	20081128	2.29	mmol/l	
Creatinine	20081128	63	µmol/l	
Urea	20081128	3.3	mmol/l	
Aspartate aminotransferase	20081128	1.4	µmol/s.l	
Alanine aminotransferase	20081128	2.02	µmol/s.l	
Gamma GT	20081128	1.34	µmol/s.l	
C-reactive protein	20081128	23.3	mg/l	
TSH	20081128	3.96	µIU/ml	
Aspartate aminotransferase	20081204	2.89	µmol/s.l	
Alanine aminotransferase	20081204	4.28	µmol/s.l	
Gamma GT	20081204	0.92	µmol/s.l	
Haemoglobin	20081204	8.44	mmol/l	
Haematocrit	20081204	5.2	NA	
Aspartate aminotransferase	20081210	9.53	µmol/s.l	
Alanine aminotransferase	20081210	17.54	µmol/s.l	
Gamma GT	20081210	1.76	µmol/s.l	
Aspartate aminotransferase	20081212	2.92	µmol/s.l	
Alanine aminotransferase	20081212	10.44	µmol/s.l	
Gamma GT	20081212	2.08	µmol/s.l	
C-reactive protein	20081128	23.3	mg/l	
TSH	20081128	3.96	µIU/ml	
Total white blood cell count	20081211	5.2	Gpt/l	

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 10.03.2009 DE-PVZIBE-WEIMAR-10149

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Fieber (mit Exanthem und Transaminasenanstieg)

KURZZUSAMMENFASSUNG:

Bei bekannter Epilepsie Therapie mit Ergenyl chrono seit 2007, was aber wegen Verschlechterung des Allgemeinzustandes (Kraftlosigkeit + Gelenkschmerzen seit ca. 08/2008) seit 10.11.08 "ausgeschlichen" wurde. Deshalb Therapiebeginn mit Lamotrigin am 10.11.08. Am 25.11.08

Entwicklung von Fieber und eines stammbetonten kleinfleckigen konfluierenden Exanthems, was zur stationären Aufnahme führte. Transaminasen bei stat. Aufnahme erhöht.

THERAPIE:

Bzgl. Fieber bestand V.a. Infektion (Mycoplasmen), darum Therapie mit Ciprobay vom 28.11. - 07.12.08 sowie Ibuprofen 400 mg vom 06.12.08 - 08.12.08.

VERLAUF:

Unter Therapie mit Ciprobay und Ibuprofen waren Entzündungszeichen und Exanthem rückläufig, Transaminasen stiegen aber weiter an. Follow up: aber am 10.12.08 zeigte sich ein weiterer exzessiver Transaminasenanstieg. Am 10.12.08 wurde diesbezüglich Lamotrigin abgesetzt, ebenso Ranitidin und Pantozol. Seit 10.12.08 nur noch Einnahme von Ergenyl chrono und Beginn mit Frisium 10 mg Tbl. zur Epilepsieprophylaxe. Rückläufige Transaminasen seit 12.12.08.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: RANITIDINE

BETREFFENDE UAW: Fieber

ERLÄUTERUNG: Ranitidin wurde entgegen der Fachinformation-Empfehlung relativ lange (ca. 7 Jahre) gegeben. Auch für Ranitidin ist wie im Fall von Pantozol trotz langer guter Verträglichkeit eine Symptomursache nicht komplett auszuschließen.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Fieber

ERLÄUTERUNG: Ibuprofen bleibt trotz "nur" Bedarfsedikation verdächtig, allerdings eher unwahrscheinlich, da lt. Patientenanamnese wirklich nur selten eingenommen.

MEDIKAMENT: LAMOTRIGIN

BETREFFENDE UAW: Fieber

ERLÄUTERUNG: C2, denn nach dem Absetzen des Medikamentes wurden Transaminasen rückläufig. Der zwischenzeitlich extreme Anstieg der Transaminasen ist auf einen synergistischen Effekt von Ciprobay zurückzuführen. Medikament sehr verdächtig wegen Therapiebeginn 10.11.08.

MEDIKAMENT: ERGENYL CHRONO

BETREFFENDE UAW: Fieber

ERLÄUTERUNG: Fieberreaktion noch nicht in Fachinfo beschrieben.

MEDIKAMENT: PANTOZOL

BETREFFENDE UAW: Fieber

ERLÄUTERUNG: Fieberreaktion in Fachinfo nicht erwähnt aber Exanthem und Transaminasenanstieg.

NOVARTIS, 17.03.2009 DE-NOVNCHHQ-GXKR2009DE03223

Sender diagnosis: not reported

Narrative

Initial report received from health authority (DE-BFARM-09030615) on 16 Mar 2009: This patient developed fever during the therapy with Ergenyl chrono (valproic acid), ibuprofen (MAH unknown), lamotrigine (MAH unknown), Pantozol (pantoprazole) and ranitidine (MAH unknown). Outcome was provided as recovered with sequel. Causality was assessed as possible (for Ergenyl chrono), likely (for ibuprofen, ranitidine and Pantozol) and very likely (for lamotrigine).

For lamotrigine and ranitidine: Serious spontaneous report of fever (hospitalisation) already listed in the Core Data Sheet Information.

For ibuprofen: Serious spontaneous report of fever (hospitalisation) assessed as unlisted according to the Core Data Sheet Information. However other alternative causes (e.g. other drugs) provide a possible explanation for the reported adverse event.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Bayer Vital GmbH, 18.03.2009 DE-BAYER-200916196GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was identified on 11-Mar-2009 in an internet-based pool of cases of BfArM, the regulatory health authority in Germany, reported from a physician in Germany, and referring to a 19 year-old male (height: 185 cm, weight: 78 kg).

Patient's medical history included continuing epilepsy (last seizure in Dec-2007), reflux oesophagitis since 2001, and migraine.

Patient received the following drugs reported as 'suspect drugs':

- oral Ibuprofen 400 mg tablet was taken as needed for migraine for many years until 08-Dec-2008,
- oral Ergenyl Chrono (valproate) 500 mg once daily for epilepsy since May-2007
- oral Lamotrigine 200 mg per day for epilepsy from 10-Nov-2008 until 10-Dec-2008,
- oral Pantozol (pantoprazole) 20 mg per day for reflux oesophagitis from 2001 until 10-Dec-2008,
- oral Ranitidine 150 mg once daily for reflux oesophagitis from 2001 until 10-Dec-2008.

Since 10-Nov-2008, Ergenyl Chrono (valproic acid) was tapered due to impairment of physical condition (feebleness and joint pain), which existed since Aug-2008.

On 10-Nov-2008, treatment with Lamotrigine was initiated.

On 25-Nov-2008, patient was admitted to hospital due to FEVER and EXANTHEMA (truncal region, small spotted, confluent). TRANSAMINASES ELEVATED on admission.

Due to suspected mycoplasma infection treatment was commenced with Ciprobay (ciprofloxacin) from 28-Nov-2008 until 07-Dec-2008, and Ibuprofen 400 mg from 06-Dec-2008 until 08-Dec-2008.

Inflammatory signs and EXANTHEMA ameliorated but transaminases remained increasing.

10-Dec-2008: further excessive elevation of transaminases. Treatment with Lamotrigine, Ranitidine and Pantozol (pantoprazole) discontinued.

Since 10-Dec-2008, Ergenyl Chrono (valproic acid) therapy was continued and prophylactic treatment (epilepsy) with Frisium (Clobazam) 10 mg was initiated.

Since 12-Dec-2008, transaminases were lowering.

Outcome of the events: recovered/resolved with sequelae.

Reporter's clinical assessment:

The reporter assessed the event

- FEVER to be "more unlikely" related to Ibuprofen, since according to patient's medical history intake had only been on a rare base, however, relationship remains "suspected",
- FEVER to be "likely" related to Ranitidine and Pantozol (pantoprazole), due to out of label use for approximately 7 years,
- FEVER to be "very likely" related to Lamotrigine (due temporal relationship) and lowering of transaminases after withdrawal, temporary TRANSAMINASES ELEVATED due to synergistic effect of treatment with Ciprobay (ciprofloxacin).

SANOFI-SYNTHELABO, 24.03.2009 DE-SANOFI-SYNTHELABO-A04200900315

Sender diagnosis: not reported

Narrative

Reaction as reported: fever (with exanthema with increase of transaminases)

LLT: FEVER / 10016558

PT: PYREXIA / 10037660

Sender's diagnosis: transaminase increased (ASAT 18.3 ULN and ALAT 33.7 ULN)

LLT: HEPATOCELLULAR DAMAGE / 10019831

PT: HEPATOCELLULAR INJURY / 10019837

Sender's diagnosis: exanthema

LLT: EXANTHEM / 10015585

PT: RASH / 10037844

Sender's diagnosis: C-reactive protein increased (2.3 ULN)

LLT: C-REACTIVE PROTEIN INCREASED / 10006825

PT: C-REACTIVE PROTEIN INCREASED / 10006825

Sender's diagnosis: blood sodium decreased (133 mmol/L)

LLT: BLOOD SODIUM DECREASED / 10005802

PT: BLOOD SODIUM DECREASED / 10005802

Sender's diagnosis: adynamia

LLT: ADYNAMIA / 10001425

PT: ASTHENIA / 10003549

Sender's diagnosis: joint pain
 LLT: JOINT PAIN / 10023222
 PT: ARTHRALGIA / 10003239

Narrative:
 Initial report:

This case was extracted from the website of the German health authority (authority number DE-BFARM-09030615); it was reported by a physician via a regional pharmacovigilance centre (DE-PVZIBE-WEIMAR-10149); data have been captured and coded as originally assessed and reported by the PV Centre.

The case involves a 19-year-old male who started treatment with sodium valproate + valproic acid (Ergenyl chrono) at a dose of 500 mg daily in May 2007 for epilepsy, ibuprofen at a dose of 400 mg if necessary (no trade name provided) on 08-Dec-2008 because of migraine, lamotrigine at a dose of 100 mg 2 in 1 Day (no trade name provided) from 10-Nov-2008 to 10-Dec-2008 because of epilepsy, ranitidine at a dose of 150 mg daily (no trade name provided) from 2001 to 10-Dec-2008 and pantozol at a dose of 20 mg daily from 2001 to 10-Dec-2008.

The patient was treated with sodium valproate + valproic acid since 2007, but due to decreasing of his general condition (adynamia and joint pain) since approximately August 2008 the dose was reduced slowly and was finally withdrawn and consequently lamotrigine was started on 10-Nov-2008. Since 25-Nov-2008 the patient suffered from fever with trunk excentuated, small spottet, confluent exanthema. Hospitalisation became necessary. On admission blood sodium was 133 mmol/l, ASAT was 2.7 ULN and ALAT was 33.7 ULN. Due to fever an infection (mycoplasmas) was suspected. Ciprofloxacin from 28-Nov-2008 to 07-Dec-2008 as well as ibuprofen at dose of 400 mg daily from 06-Dec-2008 to 08-Dec-2008 were given. Due to this therapy signs of inflammation and exanthema improved.

On 10-DEC-2008, the patient presented another excessive increase in transaminases: ASAT was 18.3 ULN, ALAT was 33.7 ULN.

On 10-Dec-2008 lamotrigine was discontinued, as well as ranitidine and pantozol. Clobazam was added for epilepsy prophylaxis.

Outcome: recovered/resolved with sequelae.

Reporters causality assessment according to Begaud Algorithm:

Ergenyl chrono - "possible"

Ibuprofen - "likely"

Lamotrigine - "very likely"

Ranitidine - "likely"

Pantozol - "likely"

NOVARTIS, 25.03.2009 DE-002147023-PHHY2009DE09357

Sender diagnosis: not reported

Narrative

Initial German Health Authority (ref. no. DE-BFARM-09030615) report received from Nycomed (Mfr. control no. 0204468) on 16 Mar 2009 : This patient who started therapy with Pantozol (pantoprazole), 20 mg once daily, orally, in 2001, due to reflux oesophagitis. He was also started on Ibuprofen 400 mg as needed on an unspecified date due to migraine, on an unspecified date. Ergenyl Chrono (valproic acid/valproate sodium), Lamotrigine and Ranitidine were additionally reported as co-suspect medications in this case. The patient had a medical history of epilepsy, and no concomitant medications were reported. On 25 Nov 2008 (about 7 years after the first dose of Pantozol and an unspecified time after the first dose of Ibuprofen), the patient experienced fever (NOS) with confluent trunk exanthema. the patient was hospitalized. On 28 Nov 2008, lab value revealed transaminases increased (ALAT: 2.02 mcmol/sl - normal range (NR): <0.52; ASAT: 1.4 mcmol/sl - NR: <0.52; Gamma GT: 1.34 mg/L - NR: < 0.53). C-reactive protein was measured at 23.3 mg/L (NR: <10). Infection (mycoplasma) was suspected and a treatment with Ciprofloxacin was given from 28 Nov 2008 to 07 Dec 2008 in combination with Ibuprofen (from 06 Dec 2008 to 08 Dec 2008). Under treatment, signs of inflammation and exanthema abated. On 10 Dec 2008, transaminases increased (ALAT: 17.54 mcmol/sl; ASAT: 9.53 mcmol/sl and Gamma GT: 1.76 mcmol/sl). All suspect drugs except Ergenyl Chrono discontinued on 10 Dec 2008 and a treatment with Clobazam was started for epilepsy prophylaxis. On 12 Dec 2008, transaminases values were: ALAT: 10.44 mcmol/sl, ASAT: 2.92 and Gamma GT: 2.08 mcmol/sl. The therapy with pantoprazole was discontinued on 10-Dec-2008 and with ibuprofen on 08-Dec-2008. The patient ?? was reported as recovered with sequel on an unspecified date.

NOVARTIS COMMENT 25.03.2009:

Serious spontaneous report, mycoplasma infection (hospitalization) assessed as unlisted according to the Core Data Sheet. Serious spontaneous report, transaminase level increased and rash (hospitalization), assessed as listed according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes, or in accordance with national regulatory requirements.<dbtext>To_Char_NARRATIVE_2</dbtext>

HELIOS Klinikum Wuppertal, 01.06.2010 DE-PVZIBE-WEIMAR-10149

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Transaminasenanstieg

KURZZUSAMMENFASSUNG:

Transaminasenanstieg unter einer Therapie mit Lamotrigin.

BESCHREIBUNG:

Bei bekannter Epilepsie Therapie mit Ergenyl chrono seit 2007, was aber wegen Verschlechterung des Allgemeinzustandes (Kraftlosigkeit +

Gelenkschmerzen seit ca. 08/2008) seit 10.11.08 "ausgeschlichen" wurde. Deshalb Therapiebeginn mit Lamotrigin am 10.11.08. Am 25.11.08 Entwicklung von Fieber und eines stammbetonten kleinfleckigen konfluierenden Exanthems, was zur stationären Aufnahme am 28.11. führte. Transaminasen bei stat. Aufnahme erhöht, was bereits dem Hausarzt am 25.11.2008 auffiel (Werte nicht mitgeteilt).

THERAPIE:

Lamotrigin wurde abgesetzt - allerdings erst am 10.12.08. Zuvor Therapie mit Ciprofloxacin 250 mg/d (28.11.08-7.12.08) wegen V.a. Atemwegsinfekt. VERLAUF:

Am 10.12.08 wurde Lamotrigin abgesetzt. Nach Absetzen von Lamotrigin rückläufige Transaminasen. Zunächst jedoch nochmaliger Anstieg dieser, was evtl. im Zusammenhang mit der Gabe von Ciprofloxacin zu sehen ist. Entzündungszeichen und Exanthem ab 5.12.08 rückläufig. Rückläufige Transaminasen seit 12.12.08. Entlassung bei subjektivem Wohlbefinden, wobei der Fall nicht eindeutig geklärt werden konnte hinsichtlich Ursachen.

ERLÄUTERUNGEN ZUR UAW:

Es wurde der Verdacht auf eine komplexe Nebenwirkung durch Lamotrigin geäußert aber auch ein infektassoziertes Fieber/Exanthem mit Transaminasenanstieg wurde differentialdiagnostisch im Entlassungsbrief diskutiert.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: LAMOTRIGIN

BETREFFENDE UAW: Transaminasen erhoeht

ERLÄUTERUNG: C2, denn nach dem Absetzen des Medikamentes wurden Transaminasen rückläufig. Der zwischenzeitlich extreme Anstieg der Transaminasen ist auf einen synergistischen Effekt von Ciprobay zurückzuführen. Medikament sehr verdächtig wegen Therapiebeginn 10.11.08.

sanofi-aventis, 11.06.2010 DE-SANOFI-AVENTIS-A04200900315

Sender diagnosis: not reported

Narrative

Initial report:

This case was extracted from the website of the German health authority (authority number DE-BFARM-09030615); it was reported by a physician via a regional pharmacovigilance centre (DE-PVZIBE-WEIMAR-10149); data have been captured and coded as originally assessed and reported by the PV Centre.

The case involves a 19-year-old male who started treatment with sodium valproate + valproic acid (Ergenyl chrono) at a dose of 500 mg daily in May 2007 for epilepsy, ibuprofen at a dose of 400 mg if necessary (no trade name provided) on 08-Dec-2008 because of migraine, lamotrigin at a dose of 100 mg 2 in 1 Day (no trade name provided) from 10-Nov-2008 to 10-Dec-2008 because of epilepsy, ranitidine at a dose of 150 mg daily (no trade name provided) from 2001 to 10-Dec-2008 and pantozol at a dose of 20 mg daily from 2001 to 10-Dec-2008.

The patient was treated with sodium valproate + valproic acid since 2007, but due to decreasing of his general condition (adynamia and joint pain) since approximately August 2008 the dose was reduced slowly and was finally withdrawn and consequently lamotrigin was started on 10-Nov-2008. Since 25-Nov-2008 the patient suffered from fever with trunk excentuated, small spottet, confluent exanthema. Hospitalisation became necessary. On admission blood sodium was 133 mmol/l, ASAT was 2.7 ULN and ALAT was 33.7 ULN. Due to fever an infection (mycoplasmas) was suspected. Ciprofloxacin from 28-Nov-2008 to 07-Dec-2008 as well as ibuprofen at dose of 400 mg daily from 06-Dec-2008 to 08-Dec-2008 were given. Due to this therapy signs of inflammation and exanthema improved.

On 10-DEC-2008, the patient presented another excessive increase in transaminases: ASAT was 18.3 ULN, ALAT was 33.7 ULN.

On 10-Dec-2008 lamotrigin was discontinued, as well as ranitidine and pantozol. Clobazam was added for epilepsy prophylaxis.

Outcome: recovered/resolved with sequelae.

Reporters causality assessment according to Begaud Algorithm:

Ergenyl chrono - "possible"

Ibuprofen - "likely"

Lamotrigin - "very likely"

Ranitidine - "likely"

Pantozol - "likely"

01-Sep-2009: Upon internal review for PSUR preparation, "transaminase increased (ASAT 18.3 ULN and ALAT 33.7 ULN)" coded as hepatocellular injury was moved to the first clinical event.

On 07-Jun-2010 the additional information was extracted from the website of the German health authority (authority number DE-BFARM-09030615) documenting a case reported by a physician via pharmacovigilance center.

This case was re-assessed by the pharmacovigilance center so that following changes were detected:

Lamotrigin was considered as single suspected drug. Ranitidine, ibuprofen, valproate sodium/valproic acid and pantoprazole sodium were reclassified as concomitant drugs.

However; in connection with intake of valproate sodium/valproic acid the events of adynamia and joint pain were reported.

Furthermore the reporter specified increased transaminases as leading event, which was resolving on 12-Dec-2008. The patient could be discharged.

Reporter causality to increased transaminases: probable (WHO score) to lamotrigine,

GlaxoSmithKline, 11.06.2010 DE-GlaxoSmithKline-D0060900A

Sender diagnosis: not reported

Narrative

This case was reported by a regulatory authority and described the occurrence of transaminases increased in a 19-year-old male patient who received Lamotrigine tablet over a period of 31 days for epilepsy.

Concurrent medical conditions included epilepsy with the last convulsion in December 2007, migraine, and reflux esophagitis. Co-suspect medication included sodium valproate + valproic acid (Ergenyl chrono), Ranitidine hydrochloride (Ranitidin), Ibuprofen (Ibuprofen) and Pantoprazole (Pantozol). Since August 2008 the subject had joint pain and weakness under treatment with Ergenyl chrono. The drug was slowly reduced to be stopped.

On 10 November 2008 the patient started Lamotrigine (oral) at 100 mg twice per day.

Approximately 15 days later, on 25 November 2008, the patient experienced fever and confluent macular exanthema, especially on the trunk.

The patient was hospitalised.

When admitted transaminases were increased and mycoplasma infection was suspected.

The patient was treated with ciprofloxacin hydrochloride (Ciprobay) and ibuprofen. Inflammatory parameters and exanthema improved, but transaminases increased further (excessively on 10 December 2008).

Treatment with Lamotrigine was discontinued on 10 December 2008, as well as ranitidine and pantoprazole. Since 10 December 2008 the patient only received Ergenyl chrono and additional treatment with clobazam (Frisium) was started.

Transaminases improved on 12 December 2008.

The regulatory authority reported that fever was very likely related to treatment with Lamotrigine, possibly related to Ergenyl chrono and likely related to Ibuprofen, pantoprazole and ranitidin. The causality for the other events was not reported.

Follow-up information was received on 07 June 2010 via the German regulatory authority (BfArM).

The drugs formerly reported as co-suspect were now considered concurrent (sodium valproate + valproic acid, ibuprofen, ranitidine hydrochloride and pantoprazole).

The patient was hospitalised on 28 November 2008. At that time transaminases were increased, which had already been found by the family physician on 25 November 2008 (no values provided).

After discontinuation of lamotrigine the transaminases decreased. There was another increase, which was suspected to be caused by ciprofloxacin treatment for suspected airways infection (28 November to 07 December 2008).

Since 12 December 2008 transaminases continuously decreased. Inflammatory signs and exanthema improved on 08 December 2008.

The patient was discharged with subjective well-being.

The reporter suspected a probable relation of increased transaminases to treatment with lamotrigine, but an infection-associated febrile exanthema with increased transaminases was also discussed in the hospital report (report not provided).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09034464

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19420910	Age: 66 Year	weight (kg): 92	BMI_int: 26.59	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute post hemorrhagic anemia	20090210			2 Day	Unknown
2 Postoperative bleeding	20090210	20090211	2 Day	2 Day	Unknown
3 Wound secretion					
4 Quick's value decreased	20090210	20090211	2 Day	2 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		1800 Mg milligram(s)/Day	-	
ibuprofen			-	
2 Xarelto	Unknown	10 Mg milligram(s)/Day	20090209-20090210	Thrombosis prophylaxis
rivaroxaban		245	2 Day	

Drugs (concomitant)

3 Aniflazym		/	-	
serrapeptase			-	
4 Antibiotika	Unknown	/	-	Preoperative care
			-	
5 Antithrombotische Mittel	Unknown	/	-	Preoperative care
			-	
6 Pantozol		40 Mg milligram(s)/Day	-	
pantoprazole sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Osteoarthritis			k.A.	gonarthrosis on both
Total knee replacement	20090209		k.A.	total knee endoprosthesis both sides

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Prothrombin level	20090210	47	%	
Prothrombin level	20090211	67	%	
Activated partial thromboplastin time	20090204	37	S	
Activated partial thromboplastin time	20090210	49	S	
Activated partial thromboplastin time	20090211	44	S	
C-reactive protein	20090216	63.1	MG/L	
C-reactive protein	20090214	139.4	MG/L	
C-reactive protein	20090212	134.3	MG/L	

C-reactive protein	20090211	177.0	MG/L
Hemoglobin	20090215	6.4	MMOL/L
Hemoglobin	20090214	5.8	MMOL/L
Hemoglobin	20090214	5.5	MMOL/L
Hemoglobin	20090216	7.3	MMOL/L
Hemoglobin	20090127	14.7	G/DL
Hemoglobin	20090212	5.2	MMOL/L
Hemoglobin	20090210	4.9	MMOL/L
Hemoglobin	20090204	9.2	MMOL/L
Prothrombin level	20090127	91	%
Prothrombin level	20090211	67	%
Prothrombin level	20090210	48	%
Prothrombin level	20090204	102	%
White blood cells	20090204	10.63	GIGA/L
White blood cells	20090204	10.54	GIGA/L

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 17.03.2009 DE-BAYER-200915785GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 05-Mar-2009 from a orthopedics in Germany, referring to a 66 year-old Caucasian male. Case cluster to 200915593GPV and 200915105GPV.

Medical and drug history:

The patient with a condition of gonarthrosis on both sides underwent total knee endoprosthesis on 09-Feb-2009. It is unknown whether any concomitant drugs have been given.

Patient received:

-Xarelto (rivaroxaban) for postoperative thrombosis prophylaxis from 09-Feb-2009 until 10-Feb-2009 (at a total daily dose of 10 MG).

On 10-Feb-2009, after hemostasis, recurrence of bloody wound secretion (POSTOPERATIVE BLEEDING, considered serious due to medical importance by company) accompanied by QUICK DECREASE (<50%) (considered non-serious by company) was observed in a patient. On that day exact Quick's value was 47%. The next day, on 11-Feb-2009, the Quick's value had returned to 67% and the wound was dry. Xarelto was withdrawn and replaced by Mono-Embolex (certoparin sodium).

The original reporter's clinical assessment:

The reporter considered the diagnoses POSTOPERATIVE BLEEDING (bloody wound secretion) and QUICK DECREASE (<50%) to be probably related.

Bayer Vital GmbH, 18.03.2009 DE-BAYER-200915785GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 05-Mar-2009 from a orthopedics in Germany, referring to a 66 year-old Caucasian male. Case cluster to 200915593GPV and 200915105GPV.

Medical and drug history:

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and replaced by Mono-Embolex (certoparin sodium).

The original reporter's clinical assessment:

The reporter considered the diagnoses POSTOPERATIVE BLEEDING (with bloody wound secretion) and QUICK DECREASE (<50%) to be probably related.

Bayer Vital GmbH, 01.05.2009 DE-BAYER-200915785GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 05-Mar-2009 from a orthopedics in Germany, referring to a 66 year-old Caucasian male. Case cluster to 200915593GPV and 200915105GPV.

Medical and drug history:

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The original reporter's clinical assessment:

The reporter considered the diagnoses POSTOPERATIVE BLEEDING (with bloody wound secretion) and QUICK DECREASE (<50%) to be probably related.

**FU information received on 16-Apr-2009:

POSTOPERATIVE BLEEDING considered serious due to medical importance.

Hospital summary from discharge letter on 15-FEB-2009:

Hospitalization from 08-Feb-2009 till 17-Feb-2009.

Diagnosis:

- Gonarthrosis both sides

Surgery:

Implantation of knee-TEP both sides on 09-FEB-2009.

Further diagnosis:

- Acute bleeding anemia

Clinical course:

Scheduled hospital stay for knee-TEP implantation both sides. Prior surgery intervention prophylactic administration of antibiotic and antithrombotic medication. Wound healing, per primam intentionem. Due to bleeding anemia patient had been treated with 5 packs red cells. The clinical course was uneventful. During the prophylactic treatment with Xarelto (Rivaroxaban) worsening of coagulation values was noted therefore the further anticoagulation with Monoembolex (certoparin-sodium) 0,3 1xQD was initiated.

Results on discharge:

Wound without irritations (dry)

Recommendation:

Further thrombosis prophylaxis with Monoembolex (certoparin-sodium) 0,3 1xQD s.c.

Bayer Vital GmbH, 07.08.2009 DE-BAYER-200915785GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 05-Mar-2009 from a orthopedics in Germany, referring to a 66 year-old Caucasian male. Case cluster to 200915593GPV and 200915105GPV.

Medical and drug history:

The patient with a condition of gonarthrosis on both sides underwent total knee endoprosthesis on 09-Feb-2009.

It is unknown whether any concomitant drugs have been given.

Patient received:

-Xarelto (rivaroxaban) for postoperative thrombosis prophylaxis from 09-Feb-2009 until 10-Feb-2009 (at a total daily dose of 10 MG).

On 10-Feb-2009, after hemostasis, POSTOPERATIVE BLEEDING (considered serious due to medical importance by company) with recurrence of bloody wound secretion, accompanied by QUICK DECREASE (<50%) (considered non-serious by company) was observed in a patient. On that day exact Quick's value was 47%. The next day, on 11-Feb-2009, the Quick's value had returned to 67% and the wound was dry. Xarelto was withdrawn and replaced by Mono-Embolex (certoparin sodium).

The original reporter's clinical assessment:

The reporter considered the diagnoses POSTOPERATIVE BLEEDING (with bloody wound secretion) and QUICK DECREASE (<50%) to be probably related.

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Results on discharge:

Wound without irritations (dry)

Recommendation:

Further thrombosis prophylaxis with Monoembolex (certoparin-sodium) 0,3 1xQD s.c.

**FU 28-Jul-2009 submitted by a clinician:

Concomitant drugs have been reported: Ibuprofen, Pantozol, Aniflazym.

Duration of surgery (total knee endoprosthesis): 89 minutes. Stop time of surgery: 12:54 a.m.

First Xarelto tablet was taken on 09-Feb-2009 at 09:00 p.m..

Patient's anamnesis included no preoperative bleeding history.

FIRST LOOK

Country: Deutschland

Type: Not available to sender (unknown)

case: DE-BFARM-09040167

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19240219	Age: 84 Year	weight (kg): 70	BMI_int: 26.35	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Potassium blood decreased	200807	20080807	1 Month	2 Year	recovered/resolved
2 Headache	200807	20080807			recovered/resolved
3 Nausea	200807	20080807		2 Year	recovered/resolved
4 Hyponatraemia	200807	20080807	1 Month	2 Year	recovered/resolved
5 Gastritis erosive	200807	20080807		2 Year	recovered/resolved
6 Potassium blood decreased	200807	20080807			recovered/resolved
7 Blood sodium decreased	200807	20080807			recovered/resolved
8 Vomiting	200807	20080807			recovered/resolved
9 Aortic sclerosis	20080807			2 Year	Unknown
10 Disorder renal	20080807			2 Year	Unknown
11 Liver damage	20080807			2 Year	Unknown
12 Pain groin					Unknown
13 Occult blood positive	20080807			2 Year	Unknown
14 Exsiccosis					Unknown
15 Urinary tract infection					Unknown
16 Hypokalaemia	200807	20080807	1 Month	2 Year	recovered/resolved
17 Hypokalemia	200807	20080807			recovered/resolved
18 Hyponatremia	200807	20080807	1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AGGRENOX acetylsalicylic acid; dipyridamole	Oral	450 Df dosage form/Day 208	200605-20080805 27 Month	Thrombosis prophylaxis
2 ENABETA comp enalapril maleate; hydrochlorothiazide	Oral	1 Df dosage form/Day 245	200605-20080805 27 Month	Hypertension arterial
3 Hydrochlorothiazide hydrochlorothiazide	Oral	25 Mg milligram(s)/Day 245	200605-20080805 27 Month	Hypertension arterial
4 IBUPROFEN ibuprofen	Oral	2*600 Mg milligram(s)/ Day 245	200806-20080805 2 Month	Coxarthrosis
5 indometacin indometacin	Oral	2*50 Mg milligram(s)/ Day 245	200806-200807 31 Day	Coxarthrosis

Drugs (concomitant)

6 FELOCOR felodipine	Oral	5 Mg milligram(s)/Day 245	2000- -	Hypertension arterial
7 L-THYROXIN HEXAL levothyroxine	Oral	50 Mg milligram(s)/Day 245	1978- -	Hypothyroidism
8 METOCLOPRAMIDE metoclopramide	Oral	36 Mg milligram(s)/Day 158	200807-20080808 1 Month	Nausea

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Coxarthrosis	2005		Yes	
Hypertension arterial	1998		Yes	
Hypertensive heart disease	2000		Yes	
Goiter nodular		1978	No	Beginn unbekannt, Strumaektomie 1978
Hypothyroidism			Yes	Beginn unbekannt
Nausea	200807	200808	No	
Strumectomy		1978	k.A.	
Aortic sclerosis			Yes	
Hepatocellular damage			Yes	
Urinary infection			Yes	
Chronic kidney disease			Yes	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Other Health Professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20080805	78	µmol/l	
Creatinine	20080807	76	µmol/l	
Creatinine clearance	20080805	>1	ml/s	
Creatinine clearance	20080807	>1	ml/s	
Quick's test	20080805	110	Prozent	
Partial prothrombin time	20080805	27.5	sec	
INR	20080805	0.93	therapeutis	
Thrombocyte count	20080805	236	Gpt/l	
Haemoglobin	20080805	8.6	mmol/l	
Haematocrit	20080805	0.38	SI-Einheit	
Total white blood cell count	20080805	7.78	Gpt/l	
C-reactive protein	20080805	2.49	mg/l	
Potassium	20080805	3.3	mmol/l	
Potassium	20080807	3.8	mmol/l	
Sodium	20080805	123	mmol/l	
Sodium	20080807	132	mmol/l	
Calcium	20080805	2.33	mmol/l	
Calcium	20080807	2.45	mmol/l	
Albumin	20080807	35.8	g/l	
White blood cells urine	20080805	500	µl	
Red blood cells urine	20080805	<10	µl	
Protein urine	20080805	<0.05	g/l	

Narratives (nur bei elektronischem Eingang)Medizinische Fakultät der Universität Rostock, 27.03.2009 DE-PVZIBE-ROSTOCK-10121

Sender diagnosis: not reported

Narrative**ANMERKUNGEN ZU FOLGENDER UAW:**

Hyponatriämie

KURZZUSAMMENFASSUNG:

Hyponatriämie unter Ibuprofen, Indometacin, Enalapril, HCT

BESCHREIBUNG:

seit Tagen Übelkeit, Erbrechen, Kopfschmerzen;

Einweisungsdiagnose: Dyselektrolytämie (deutliche Hyponatriämie, geringe Hypokaliämie);

Krankenhausaufenthalt: 05.-13.08.2008;

anamnestisch: wegen Hüftschmerzen Indometacin- und Ibuprofen-Einnahme;

bei Aufnahme: Blutdruck 160/70mmHg, Zeichen der Exikose, Druckschmerz über rechter Leiste

THERAPIE:

Infusionstherapie;

Diuretika abgesetzt;

Protonenpumpeninhibitor-Gabe;

Schmerzmedikation mit Metamizol;

Ciprofloxacin-Gabe wegen Harwegsinfekt

VERLAUF:

07.08.08 Labor: Protein- Elektrolyt- Phorese: ohne pathologischen Befund, okkultes Blut 2x positiver Befund;

07.08.08 Sonografie Abdomen: chronische Nierenveränderungen, Leberparenchymschaden, geringe Aortensklerose;

08.08.08 Gastroskopie: Antrumgastritis mit erosiver Komponente;

Entlassungsmedikation: Enalapril, L-Thyroxin, Omeprazol, Metamizol (bei Bedarf)

Besserung der Symptomatik

ERLÄUTERUNGEN ZUR UAW:

Einweisungsdiagnose: Dyselektrolytämie (deutliche Hyponatriämie, geringe Hypokaliämie);

Krankenhausaufenthalt: 05.-13.08.2008; Kopfschmerzen, Übelkeit und Erbrechen sind typische Symptome für eine Hyponatriämie. Ursache könnte ein Natrium- und Kaliumverlust durch HCT sein. Für Enalapril ist ebenfalls eine Hyponatriämie beschrieben. Die Einnahme von NSAR kann eine erhöhte Sekretion des antidiuretischen Hormons bewirken (Syndrom der inadäquaten ADH-Sekretion = SIADH), woraus ebenfalls ein Natriumverlust resultiert (Otto, G: Polypharmazie: Medikamentenassoziierte Elektrolytstörungen; MMA Geriatrie Praxis 06/2008, Siegenthaler's Differentialdiagnose internistischer Krankheiten).

Als ALternativursachen für die Auslösung der Hyponatriämie käme die bei dem Patienten bestehende Hypothyreose in Frage, die ebenfalls über ein SIADH eine Hyponatriämie bedingen kann. Es wurden leider während des stationären Aufenthaltes keine Schilddrüsenhormone (zur Güte der Einstellung mit L-Thyroxin) überprüft. Eine Diarrhoe als Alternativursache lag nicht vor. Ein Aufnahmesymptom war allerdings Erbrechen. Dieses kann infolge der Hyponatriämie aufgetreten sein, könnte, wenn es zeitlich vorher bestand (infolge der begleitenden Gastritis bzw. des begleitenden Harnwegsinfektes) auch die Hyponatriämie bedingt haben.

ANMERKUNGEN ZU FOLGENDER UAW:

Hypokaliämie

KURZZUSAMMENFASSUNG:

Hypokaliämie unter Hydrochlorothiazid

THERAPIE:

Infusionstherapie;

Diuretika abgesetzt;

Protonenpumpeninhibitor-Gabe;

Schmerzmedikation mit Metamizol;

Ciprofloxacin-Gabe wegen Harwegsinfekt

ERLÄUTERUNGEN ZUR UAW:

Einweisungsdiagnose: Dyselektrolytämie (deutliche Hyponatriämie, geringe Hypokaliämie);

Krankenhausaufenthalt: 05.-13.08.2008;

Von den bestehenden Symptomen Kopfschmerzen, Übelkeit, Erbrechen sind Übelkeit und Erbrechen auch für eine Hypokaliämie beschrieben.

Insgesamt führend scheint bei diesem Patienten aber die Hyponatriämie zu sein.

ANMERKUNGEN ZU FOLGENDER UAW:

erosive Gastritis

KURZZUSAMMENFASSUNG:

erosive Gastritis unter ASS, Ibuprofen, Indometacin

THERAPIE:

Infusionstherapie;

Diuretika abgesetzt;

Protonenpumpeninhibitor-Gabe;

Schmerzmedikation mit Metamizol;

Ciprofloxacin-Gabe wegen Harwegsinfekt

ERLÄUTERUNGEN ZUR UAW:

Einweisungsdiagnose: Dyselektrolytämie (deutliche Hyponatriämie, geringe Hypokaliämie;
Krankenhausaufenthalt: 05.-13.08.2008; von den bestehenden Symptomen Kopfschmerzen Übelkeit und Erbrechen sind die Symptome Übelkeit und Erbrechen auch mit der während des stationären Aufenthaltes diagnostizierten erosiven Gastritis vereinbar. Diese könnte zu Erbrechen geführt haben, welches eine Dyselektrolytämie bewirkte. Umgekehrt könnte die Dyselektrolytämie ein Übelkeit und Erbrechen gefördert haben, welches die Entwicklung der Gastritis unterstützt hat.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: AGGRENEX

BETREFFENDE UAW: erosive Gas<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 01.04.2009 DE-NOVNCHHQ-GXKR2009DE03954

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 31 Mar 2009 from the German Health Authority (DE-BfArM-09040167), provided by a pharmacovigilance center, reported by a physician: This multimorbid patient developed blood sodium decreased, blood potassium decreased and gastritis erosive during treatment with ibuprofen (MAH unknown), indometacin (MAH unknown), Aggrenox (acetylsalicylic acid and dipyridamole) and Enabeta comp (enalapril maleate and hydrochlorothiazide). Several other drugs were taken concomitantly. The outcome of the adverse events was reported as recovered. Causality was assessed for ibuprofen and indometacin as dubious for blood potassium decreased and as possible for blood sodium decreased and for gastritis erosive.

Information: The information provided in this individual case does not warrant a change to the Core Data Sheet. The topic will be monitored closely.

Serious spontaneous report (hospitalisation) of gastritis erosive, already listed in the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 14.04.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-02747DE

Sender diagnosis: not reported

Narrative

This is a spontaneous report (health professional confirmed) from the German Health Authority BfArM.

On 31 March 2009 information has been received concerning a 84-year old female patient. She was treated with Aggrenox (200 mg dipyridamole + /25 mg ASS), 2 capsules daily orally, from May-2006 until 05-Aug-2008 for thrombosis prophylaxis; and the non BI suspected drugs: Enabeta comp. (10mg enalapril + 25mg HCT) tablets daily once orally, from May-2006 until 05-Aug-2008 for arterial hypertension; Ibuprofen 600 mg tablets daily twice orally from Jun-2008 until 05-Aug-2008 for coxarthrosis and Indometacin 50 mg tablets orally daily twice for coxarthrosis treatment also. All those are considered suspected drugs by the reporting source. While on therapy with the suspected drugs the patient suffered from the following events and was hospitalised from 05-Aug 2008 until 13-Aug-2008:

Events:

significant hyponatraemia from Jul-2008 until 07-Aug-2008 treated with infusion therapy and withdrawal of diuretic therapy

mild hypokalaemia from -Jul-2008 until 07-Aug-2008 treated with infusion therapy and withdrawal of diuretic therapy

erosive gastritis from Jul-2008

nausea treated with infusion therapy, withdrawal of diuretic therapy and oral administration of metoclopramide (Jul-2008 until Aug-2008)

vomiting treated with infusion therapy and withdrawal of diuretic therapy

headache treated with infusion therapy and withdrawal of diuretic therapy

exsiccosis treated with infusion therapy and withdrawal of diuretic therapy

urinary tract infection treated with antibiotics (ciprofloxacin)

occult bleed positive findings twice diagnosed on 07-Aug 2008

chronic renal alteration diagnosed on 07-Aug 2008 (abdominal sonographic imaging)

damage of liver parenchyma diagnosed on 07-Aug-2008 (abdominal sonographic imaging)

mild aortic sclerosis diagnosed on 07-Aug 2008 (abdominal sonographic imaging)

pain upon pressure right groin

The patient recovered from the events hyponatraemia, hypokalaemia and erosive gastritis. For all other events the outcome was not reported.

The reporting source saw a possible reasonable relationship with the suspected drugs and the events as follows:

Aggrenox and the event erosive gastritis, but dubious causal relationship with the hyponatraemia and the hypokalaemia. For all other events causal relationship was not reported.

Enabeta comp and the events hyponatraemia and hypokalaemia, but dubious causal relationship with the erosive gastritis. For all other events causal relationship was not reported.

Ibuprofen and the events hyponatraemia and erosive gastritis, but dubious causal relationship with the hypokalaemia. For all other events causal relationship was not reported.

Ondometacin and the events hyponatraemia and erosive gastritis, but dubious causal relationship with the hypokalaemia. For all other events causal relationship was not reported.

As concomitant disease are reported coxarthrosis diagnosed 2005, arterial hypertension diagnosed 1998, hypertensive heart disease diagnosed in 2000 and hypothyroidism. Nodular goiter treated ended with strumectomy in 1978 is considered as past disease. Other relevant medical history is not reported.

As concomitant medication are reported Felacor(felodipine) for treatment of arterial hypertension from the year 2000 on and L-Thyroxin Hexal from 1978 on for treatment of hypothyroidism.

Laboratory test(s) show the following results:

Results of tests:	normal ranges:
	high and low
05-AUG-2008 Creatinine 78	µmol/l 35 88
07-AUG-2008 Creatinine 76	µmol/l 35 88
05-AUG-2008 Creatinine clearance >1	ml/s 1.58 2.66
07-AUG-2008 Creatinine clearance >1	ml/s 1.58 2.66
05-AUG-2008 Quick's test 110	Prozent 70 130
05-AUG-2008 Partial prothrombin time 27.5	sec 27 37
05-AUG-2008 INR 0.93	therapeutischer Bereich 1.25
05-AUG-2008 Thrombocyte count 236	Gpt/l 150 450
05-AUG-2008 Haemoglobin 8.6	mmol/l 7.4 9.9
05-AUG-2008 Haematocrit 0.38	SI-Einheit 0.35 0.47
05-AUG-2008 Total white blood cell count 7.78	Gpt/l 4.0 9.0
05-AUG-2008 C-reactive protein 2.49	mg/l 5.0
05-AUG-2008 Potassium 3.3	mmol/l 3.6 5.1
07-AUG-2008 Potassium 3.8	mmol/l 3.6 5.1
05-AUG-2008 Sodium 123	mmol/l 136 144
07-AUG-2008 Sodium 132	mmol/l 136 144
05-AUG-2008 Calcium 2.33	mmol/l 2.2 2.55
07-AUG-2008 Calcium 2.45	mmol/l 2.2 2.55
07-AUG-2008 Albumin 35.8	g/l 35 48
05-AUG-2008 White blood cells urine 500	µl 0 25
05-AUG-2008 Red blood cells urine <10	µl 0 10
05-AUG-2008 Protein urine <0.05	g/l < 0.25

Tracking:

31March2009 Initial received from authority

SANOFI-SYNTHELABO, 14.04.2009 DE-SANOFI-SYNTHELABO-A04200900417

Sender diagnosis: not reported

Narrative

Reaction as reported: erosive gastritis
LLT: GASTRITIS EROSIVE / 10017865
PT: GASTRITIS EROSIVE / 10017865

Reaction as reported: hyponatremia
LLT: HYPONATREMIA / 10021038
PT: HYPONATRAEMIA / 10021036

Reaction as reported: nausea
LLT: NAUSEA / 10028813
PT: NAUSEA / 10028813

Reaction as reported: vomiting
LLT: VOMITING / 10047700
PT: VOMITING / 10047700

Reaction as reported: headache
LLT: HEADACHE / 10019211
PT: HEADACHE / 10019211

Reaction as reported: hypokalemia
LLT: HYPOKALEMIA / 10021018
PT: HYPOKALAEMIA / 10021015

Narrative:

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09040167)

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

This case involves an 84-year-old female with a medical history of strumectomy in 1978. The following drugs are known to have been taken by the patient prior to the event: felodipine for arterial hypertension and levothyroxine sodium for hypothyroidism. Additionally hypertensive heart disease and goiter nodular were known.

Furthermore the patient suffered from urinary infection, the according therapy with ciprofloxacin was started after occurrence of the case events.

In June 2008 the patient had started with ibuprofen 600 mg twice as needed and indometacin 50 mg twice as needed (trade names not provided) for coxarthrosis. The further medication included dipyridamole+acetylsalicylic acid (225 mg twice daily) for thrombosis prophylaxis and enalapril maleate (10 mg daily) with hydrochlorothiazide (25 mg daily) for arterial hypertension, both started in May 2006.

In July 2008 the patient began to complain about nausea, vomiting and headache. Indometacin was discontinued in the same month. The patient was hospitalised due to pronounced hyponatremia and mild hypokalemia on 05-Aug-2008. Corrective treatment was started with infusions NOS, proton pump inhibitors and metamizole for pain caused by coxarthrosis. Ibuprofen, dipyridamole+acetylsalicylic acid and hydrochlorothiazide was stopped on day of admission. The performed gastroscopy displayed antrum gastritis with erosive component on 08-Aug-2008.

On day before the abdominal sonography disclosed concomitantly chronic renal changes, hepatocellular damage and mild aortic sclerosis.

The patient recovered from hyponatremia and hypokalemia by 07-Aug-2008 and from erosive gastritis in the same months. The patient could be discharged on 13-Aug-2008.

Causality assessment for ibuprofen determined by PV centre:

hyponatremia: possible
hypokalemia: dubious
erosive gastritis: possible

alternative explanation by PV centre:

- hypothyroidism for hyponatremia,
- non-company suspected drugs: hydrochlorothiazide for hyponatremia and hypokalemia, enalapril maleate for hyponatremia

ASTELLAS, 14.04.2009 DE-ASTELLAS-2009EU001388

Sender diagnosis: not reported

Narrative

Authority report, reported by a physician, retrieved from the BfArM homepage on 31MAR2009. DE-BFARM-09040167.

A 84 year-old female patient started ibuprofen 600 mg twice daily on an unknown day in June 2008 for coxarthrosis. On an unknown day in July 2008 decreased blood sodium, hypokalemia, erosive gastritis occurred. The patient was hospitalized with the diagnosis dyselektrolytemia. Hospital stay from 05AUG2008 until 13AUG2008. Nausea, vomiting, headache since days was reported at admission. Anamnestic findings: Intake of indometacin and ibuprofen due to pain in the hip. At admission: blood pressure 160/70 mmHg, signs of exsiccation, pressure pain over right inguinal region. Concomitant drugs: felodipine (Felacor), levothyroxine sodium (L-thyroxin) and metoclopramide hydrochloride (Metoclopramide). Medical history: from 1998 to continuing arterial hypertension, from 2000 to continuing hypertensive heart disease, from unknown date to 1978 nodular goiter, hypothyroidism start date unknown is ongoing, from July 2008 to August 2008 nausea.

Lab data:

05AUG2008 Creatinine 78 umol/l.
 07AUG2008 Creatinine 76 umol/l
 05AUG2008 Creatinine renal clearance >1 ml/s.
 07AUG2008 Creatinine renal clearance >1 ml/s.
 05AUG2008 Prothrombin time (Quik's test) 110%.
 05AUG2008 Partial prothrombin time 27.5 sec.
 05AUG2008 International normalised ratio 0.93 therapeutisch.
 05AUG2008 Platelet count 236 Gpt/l.
 05AUG2008 Haemoglobin 8.6 mmol/l.
 05AUG2008 Haematocrit 0.38 Si-Einheit.
 05AUG2008 White blood cell count 7.78 Gpt/l.
 05AUG2008 C-reactive protein 2.49 mg/l.
 05AUG2008 Blood potassium 3.3 mmol/l.
 07AUG2008 Blood potassium 3.8 mmol/l.
 05AUG2008 Blood sodium 123mmol/l.
 07AUG2008 Blood sodium 132 mmol/l.
 05AUG2008 Blood calcium 2.33 mmol/l.
 07AUG2008 Blood calcium 2.45 mmol/l.
 07AUG2008 Blood albumin 35.8 g/l.
 05AUG2008 White blood cells urine 500 ul.
 05AUG2008 Red blood cells urine <10 ul.
 05AUG2008 Protein urine <0.05 g/l.
 07AUG2008 Sonography abdomen showed: chronic kidney changes, hepatic parenchymal damage, mild aortic sclerosis.
 08AUG2008 Gastroscopy showed: andrum gastritis with erosive components.

Therapy: Infusion therapy, diuretics withdrawn, administration of proton pump inhibitors, pain medication with metamizol, ciprofloxacin due to urinary tract infection.

Outcome: Hyponatramie is recovered, hypokalaemie is recovered and erosive gastritis is recovered. All on 07AUG2008.

The reporter assessed all three events as possibly related to ibuprofen.

Company note: From 1 November 2007 Astellas will use 4 causality categories, probable, possible, not related and unassessable, and not use unlikely for any cases received on or after this date except for cases from ongoing studies that are using the unlikely category. Following this rule, the reporter assessment unlikely will be assigned to the Astellas category possible.

HELIOS Klinikum Wuppertal, 04.06.2010 DE-PVZIBE-ROSTOCK-10121

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
 Hyponatriämie

KURZZUSAMMENFASSUNG:

Hyponatriämie unter Ibuprofen, Indometacin, Enalapril, HCT

BESCHREIBUNG:

seit Tagen Übelkeit, Erbrechen, Kopfschmerzen;

Einweisungsdiagnose: Dyselektrolytämie (deutliche Hyponatriämie, geringe Hypokaliämie;

Krankenhausaufenthalt: 05.-13.08.2008;

anamnestisch: wegen Hüftschmerzen Indometacin- und Ibuprofen-Einnahme;

bei Aufnahme: Blutdruck 160/70mmHg, Zeichen der Exikkose, Druckschmerz über rechter Leiste

THERAPIE:

Infusionstherapie;

Diuretika abgesetzt;

Protonenpumpeninhibitor-Gabe;

Schmerzmedikation mit Metamizol;

Ciprofloxacin-Gabe wegen Harwegsinfekt

VERLAUF:

07.08.08 Labor: Protein- Elektrot- Phorese: ohne pathologischen Befund, okkultes Blut 2x positiver Befund;
 07.08.08 Sonografie Abdomen: chronische Nierenveränderungen, Leberparenchymschaden, geringe Aortensklerose;
 08.08.08 Gastroskopie: Antrumgastritis mit erosiver Komponente;
 Entlassungsmedikation: Enalapril, L-Thyroxin, Omeprazol, Metamizol (bei Bedarf)

Besserung der Symptomatik

ERLÄUTERUNGEN ZUR UAW:

Einweisungsdiagnose: Dyselektrolytämie (deutliche Hyponatriämie, geringe Hypokaliämie;
 Krankenhausaufenthalt: 05.-13.08.2008; Kopfschmerzen, Übelkeit und Erbrechen sind typische Symptome für eine Hyponatriämie. Ursache könnte ein Natrium- und Kaliumverlust durch HCT sein. Für Enalapril ist ebenfalls eine Hyponatriämie beschrieben. Die Einnahme von NSAR (hier Ibuprofen, Indometacin) kann eine erhöhte Sekretion des antidiuretischen Hormons bewirken (Syndrom der inadäquaten ADH-Sekretion = SIADH), woraus ebenfalls ein Natriumverlust resultiert (Otto, G: Polypharmazie: Medikamentenassoziierte Elektrolytstörungen; MMA Geriatrie Praxis 06/2008, Siegenthaler's Differentialdiagnose internistischer Krankheiten).

Als ALternativursachen für die Auslösung der Hyponatriämie käme die bei dem Patienten bestehende Hypothyreose in Frage, die ebenfalls über ein SIADH eine Hyponatriämie bedingen kann. Es wurden leider während des stationären Aufenthaltes keine Schilddrüsenhormone (zur Güte der Einstellung mit L-Thyroxin) überprüft. Eine Diarrhoe als Alternativursache lag nicht vor. Ein Aufnahmesymptom war allerdings Übelkeit/Erbrechen. Dieses kann infolge der Hyponatriämie aufgetreten sein, könnte, wenn es zeitlich vorher bestand (infolge der begleitenden Gastritis bzw. des stationär diagnostizierten, begleitenden Harnwegsinfektes) auch die Hyponatriämie bedingt haben.

ANMERKUNGEN ZU FOLGENDER UAW:

Hypokaliaemie

KURZZUSAMMENFASSUNG:

Hypokaliämie unter Hydrochlorothiazid

BESCHREIBUNG:

seit Tagen Übelkeit, Erbrechen, Kopfschmerzen;

Einweisungsdiagnose: Dyselektrolytämie (deutliche Hyponatriämie, geringe Hypokaliämie;

Krankenhausaufenthalt: 05.-13.08.2008;

anamnestisch: wegen Hüftschmerzen Indometacin- und Ibuprofen-Einnahme;

bei Aufnahme: Blutdruck 160/70mmHg, Zeichen der Exikkose, Druckschmerz über rechter Leiste

THERAPIE:

Infusionstherapie;

Diuretika abgesetzt;

Protonenpumpeninhibitor-Gabe;

Schmerzmedikation mit Metamizol;

Ciprofloxacin-Gabe wegen Harwegsinfekt

VERLAUF:

07.08.08 Labor: Protein- Elektro- Phorese: ohne pathologischen Befund, okkultes Blut 2x positiver Befund;

07.08.08 Sonografie Abdomen: chronische Nierenveränderungen, Leberparenchymschaden, geringe Aortensklerose;

08.08.08 Gastroskopie: Antrumgastritis mit erosiver Komponente;

Entlassungsmedikation: Enalapril, L-Thyroxin, Omeprazol, Metamizol (bei Bedarf)

Besserung der Symptomatik

ERLÄUTERUNGEN ZUR UAW:

Einweisungsdiagnose: Dyselektrolytämie (deutliche Hyponatriämie, geringe Hypokaliämie);

Krankenhausaufenthalt: 05.-13.08.2008;

Von den bestehenden Symptomen Kopfschmerzen, Übelkeit, Erbrechen sind Übelkeit und Erbrechen auch für eine Hypokaliämie beschrieben.

Insgesamt führend scheint bei diesem Patienten aber die Hyponatriämie unter HCT-Einnahme zu sein. Erbrechen kann zusätzlich eine Dyselektrolytämie verstärken.

ANMERKUNGEN ZU FOLGENDER UAW:

erosive Gastritis

KURZZUSAMMENFASSUNG:

erosive<dbtext>To_Char_NARRATIVE_2</dbtext>

Bayer Vital GmbH, 11.06.2010 DE-BAYER-201028145GPV

Sender diagnosis: not reported

Narrative

Source of report and patient demography: This spontaneous case report was and identified on the internet web page on 30-Mar-2010 of the German health authority (Federal Institute for Drugs and Medical Devices - BfArM), referring to an 84 year-old female. Further information received on 07-Jun-2010 is included.

Medical and drug history: Patient has the concomitant conditions of coxarthrosis, arterial hypertension, hypertensive heart disease, and hypothyroidism. Patient's relevant medical history includes nodular goiter until 1978, strumectomy in 1978, and nausea from July 2008 to August 2008. Concomitant drugs have been given for arterial hypertension, hypothyroidism, and nausea. For details see section "Concomitant Drugs" at the end of this report.

Suspect drugs, timing and conditions surrounding the onset of the reactions:

Patient received Ibuprofen for coxarthrosis. Administration began in Jun-2008 as needed until 05-Aug-2008.

Patient also received Indometacin for coxarthrosis. Administration began in Jun-2008, for a duration of 4 weeks and 3 days at a total daily dose of 100 mg oral.

Co-suspected drugs are:

Aggrenox (acetylsalicylic acid + dipyridamole) for thrombosis prophylaxis and Enabeta comp (enalapril maleate + hydrochlorothiazide) for arterial hypertension.

Administration began in May-2006, for a duration of 2 years 3 months, at a dose of 200/25 mg twice a day of Aggrenox and 10/25 mg oral of Enabeta comp.

The progression of the events and their outcome in the patient: The patient's anamnesis included a previous treatment with indometacin and ibuprofen for coxalgia. Since several days the patient had suffered from nausea, vomiting and headache. Thus, after being diagnosed with dyselektrolytemia (significant HYPONATREMIA with mild HYPOKALEMIA) and EROSIVE GASTRITIS the patient was hospitalised from 05-Aug-2008 to 13-Aug-2008. On admission the blood pressure was measured 160/70mmHg.

Apart from that the patient showed signs of exsiccosis and complained about pain on pressure affecting the right groin area. Subsequently the patient underwent infusion therapy and received proton pump inhibitors (NOS). Furthermore metamizole was given for analgetic treatment while a urinary tract infection was treated with ciprofloxacin. The diuretics were discontinued.

Further course: On 07-Aug-2008 protein electrophoresis did not yield any pathological findings while occult blood test showed 2 positive results.

Abdominal sonography revealed chronic renal alterations and damages of the liver parenchyma. Furthermore more aortic sclerosis was detected. On 08-Aug-2008 gastroscopy showed antral gastritis with erosion.

On 13-Aug-2008 the patient was discharged in ameliorated condition.

Medication on discharge:

- Enalapril
- L-Thyroxin (levothyroxin)
- Omeprazole
- Metamizole (as needed)

The original reporter's clinical assessment: The reporter considered the diagnosis EROSIVE GASTRITIS related to Aggrenox, Indometacin and Ibuprofen. The reporter assessed the HYPONATREMIA related to Indometacin, Ibuprofen, Enabeta comp. The event HYPOKALEMIA was assessed as related to Enabeta comp by the reporter.

For all other drug-event relationships assessment was given as dubious.

Specification on causalities:

Hyponatremia:

Headache, nausea and vomiting - potentially caused by losses of sodium and potassium after HTC intake - were declared typical symptoms for hyponatremia. Furthermore enalapril intake and NSAID (non steroidal anti inflammatory drug) intake with subsequent release of ADH were considered.

Hypokalemia:

Nausea and vomiting were declared associated with hypokalemia but hyponatremia was considered the major problem.

Erosive gastritis:

Nausea and vomiting were considered associated with erosive gastritis due to a potential dyselektrolytemia following vomiting. On the other hand dyselektrolytemia was estimated a potential cause of nausea and vomiting, in this way leading on to gastritis.

Drug: Aggrenox - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with acetylsalicylic acid.

Drug: Enabeta comp - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) based on enalapril intake. Both hyponatremia and hypokalemia are known side effects during treatments with hydrochlorothiazide. Upon discontinuation and following infusion therapy the sodium values returned to normal.

Alternative causes:

- Existing and medicated hypothyroidism
- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Enabeta comp - Event: HYPOKALEMIA

Specification: Hypokalemia is a known side effect during treatments with hydrochlorothiazide. Upon discontinuation the potassium values returned to normal.

Drug: Ibuprofen - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) during non steroidal anti inflammatory drugs (NSAID) intake.

Alternative causes:

- Further suspected drugs
- Hypothyroidism (medically substituted)

- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Ibuprofen - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with ibuprofen.

Drug: Indometacin - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) during non steroidal anti inflammatory drugs (NSAID) intake

Alternative causes:

- Further suspected drugs
- Hypothyroidism (medically substituted)
- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Indometacin - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with indometacin.

Bayer Vital GmbH, 14.06.2010 DE-BAYER-201028145GPV

Sender diagnosis: not reported

Narrative

Source of report and patient demography: This spontaneous case report was identified on the internet web page on 30-Mar-2010 of the German health authority (Federal Institute for Drugs and Medical Devices - BfArM), referring to an 84 year-old female. Further information received on 07-Jun-2010 is included.

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Administration began in May-2006, for a duration of 2 years 3 months, at a dose of 200/25 mg twice a day of Aggrenox and 10/25 mg oral of Enabeta comp.

The progression of the events and their outcome in the patient: The patient's anamnesis included a previous treatment with indometacin and ibuprofen for coxalgia. Since several days the patient had suffered from nausea, vomiting and headache. Thus, after being diagnosed with dyselektrolytemia (significant HYPONATREMIA with mild HYPOKALEMIA) and EROSIVE GASTRITIS the patient was hospitalised from 05-Aug-2008 to 13-Aug-2008. On admission the blood pressure was measured 160/70mmHg.

Apart from that the patient showed signs of exsiccosis and complained about pain on pressure affecting the right groin area. Subsequently the patient underwent infusion therapy and received proton pump inhibitors (NOS). Furthermore metemazole was given for analgetic treatment while a urinary tract infection was treated with ciprofloxacin. The diuretics were discontinued.

Further course: On 07-Aug-2008 protein electrophoresis did not yield any pathological findings while occult blood test showed 2 positive results. Abdominal sonography revealed chronic renal alterations and damages of the liver parenchyma. Furthermore more aortic sclerosis was detected. On 08-Aug-2008 gastroscopy showed antral gastritis with erosion. On 13-Aug-2008 the patient was discharged in ameliorated condition.

Medication on discharge:

- Enalapril
- L-Thyroxin (levothyroxin)
- Omeprazole
- Metemazole (as needed)

The original reporter's clinical assessment: The reporter considered the diagnosis EROSIVE GASTRITIS related to Aggrenox, Indometacin and Ibuprofen. The reporter assessed the HYPONATREMIA related to Indometacin, Ibuprofen, Enabeta comp. The event HYPOKALEMIA was assessed as related to Enabeta comp by the reporter.

For all other drug-event relationships assessment was given as dubious.

Specification on causalities:

Hyponatremia:

Headache, nausea and vomiting - potentially caused by losses of sodium and potassium after HTC intake - were declared typical symptoms for

hyponatremia. Furthermore enalapril intake and NSAID (non steroidal anti inflammatory drug) intake with subsequent release of ADH were considered.

Hypokalemia:

Nausea and vomiting were declared associated with hypokalemia but hyponatremia was considered the major problem.

Erosive gastritis:

Nausea and vomiting were considered associated with erosive gastritis due to a potential dyselectrolytemia following vomiting. On the other hand dyselectrolytemia was estimated a potential cause of nausea and vomiting, in this way leading on to gastritis.

Drug: Aggrenox - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with acetylsalicylic acid.

Drug: Enabeta comp - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) based on enalapril intake. Both hyponatremia and hypokalemia are known side effects during treatments with hydrochlorothiazide. Upon discontinuation and following infusion therapy the sodium values returned to normal.

Alternative causes:

- Existing and medicated hypothyroidism
- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Enabeta comp - Event: HYPOKALEMIA

Specification: Hypokalemia is a known side effect during treatments with hydrochlorothiazide. Upon discontinuation the potassium values returned to normal.

Drug: Ibuprofen - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) during non steroidal anti inflammatory drugs (NSAID) intake.

Alternative causes:

- Further suspected drugs
- Hypothyroidism (medically substituted)
- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Ibuprofen - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with ibuprofen.

Drug: Indometacin - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) during non steroidal anti inflammatory drugs (NSAID) intake

Alternative causes:

- Further suspected drugs
- Hypothyroidism (medically substituted)
- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Indometacin - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with indometacin.

Boehringer Ingelheim GmbH, Germany, 16.06.2010 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-02747DE

Sender diagnosis: not reported

Narrative

This is a spontaneous report (health professional confirmed) from the German Health Authority BfArM.

On 08Jun2006 follow-up information has been received concerning a 84-year old female patient. She was treated with Aggrenox (200 mg dipyridamole + 25 mg ASS), 2 capsules daily orally, from May-2006 until 05-Aug-2008 for thrombosis prophylaxis; and the non BI suspected drugs: Enabeta comp. (10mg enalapril + 25mg HCT) tablets daily once orally, from May-2006 until 05-Aug-2008 for thrombosis prophylaxis, Enabeta Comp (10 mg enalapril + 25 mg HCT) tablets daily once orally for arterial hypertension from May 2006 to 05Aug2008, ibuprofen 600 mg tablets daily twice orally from Jun-2008 until 05-Aug-2008 for coxarthrosis and Indometacin 50 mg tablets orally daily twice for coxarthrosis treatment also from Jun2008 to Jul2008. All those are considered suspected drugs by the reporting source. While on therapy with the suspected drugs the patient suffered from the following events and was hospitalised from 05Aug2008 until 13Aug2008.

Events:

significant hyponatraemia from Jul2008 until 07Aug2008 treated with infusion therapy and withdrawal of diuretic therapy
mild hypokalaemia from -Jul-2008 until 07-Aug-2008 treated with infusion therapy and withdrawal of diuretic therapy
erosive gastritis from Jul-2008 until 07Aug2008 treated with infusion therapy and withdrawal of diuretic therapy
nausea treated with infusion therapy, withdrawal of diuretic therapy and oral administration of metoclopramide (Jul-2008 until Aug-2008)
vomiting treated with infusion therapy and withdrawal of diuretic therapy

headache treated with infusion therapy and withdrawal of diuretic therapy
 exsiccosis treated with infusion therapy and withdrawal of diuretic therapy
 urinary tract infection treated with antibiotics (ciprofloxacin)
 occult bleed positive findings twice diagnosed on 07-Aug 2008
 chronic renal alteration diagnosed on 07-Aug 2008 (abdominal sonographic imaging)
 damage of liver parenchyma diagnosed on 07-Aug-2008 (abdominal sonographic imaging)
 mild aortic sclerosis diagnosed on 07-Aug 2008 (abdominal sonographic imaging)
 pain upon pressure right groin

The patient recovered from the events hyponatraemia, hypokalaemia and erosive gastritis. For all other events the outcome was not reported.

The reporting source saw a possible reasonable relationship with the suspected drugs and the events as follows:

Aggrenox and the event erosive gastritis, but dubious causal relationship with the hyponatraemia and the hypokalaemia. For all other events causal relationship was not reported.

Enabeta comp and the events hyponatraemia and hypokalaemia, but dubious causal relationship with the erosive gastritis. For all other events causal relationship was not reported.

Ibuprofen and the events hyponatraemia and erosive gastritis, but dubious causal relationship with the hypokalaemia. For all other events causal relationship was not reported.

Indometacin and the events hyponatraemia and erosive gastritis, but dubious causal relationship with the hypokalaemia. For all other events causal relationship was not reported.

Concomitant diseases included coxarthrosis diagnosed 2005, arterial hypertension diagnosed 1998, hypertensive heart disease diagnosed in 2000 and hypothyroidism.

Nodular goiter treated ended with strumectomy in 1978 is considered as past disease. Other relevant medical history is not reported.

Concomitant medications included Felacor(felodipine) for treatment of arterial hypertension from the year 2000 on; L-Thyroxin Hexal from 1978 on for treatment of hypothyroidism and Metoclopramide for treatment of nausea between Jul-2008 to 08-Aug2008.

Laboratory test(s) show the following results:

Results of tests:	normal ranges:
	high and low
05-AUG-2008 Creatinine 78	mmol/l 35 88
07-AUG-2008 Creatinine 76	mmol/l 35 88
05-AUG-2008 Creatinine clearance >1	ml/s 1.58 2.66
07-AUG-2008 Creatinine clearance >1	ml/s 1.58 2.66
05-AUG-2008 Quick's test 110	Prozent 70 130
05-AUG-2008 Partial prothrombin time 27.5	sec 27 37
05-AUG-2008 INR 0.93	therapeutic area 1.25
05-AUG-2008 Thrombocyte count 236	Gpt/l 150 450
05-AUG-2008 Haemoglobin 8.6	mmol/l 7.4 9.9
05-AUG-2008 Haematocrit 0.38	SI-Einheit 0.35 0.47
05-AUG-2008 Total white blood cell count 7.78	Gpt/l 4.0 9.0
05-AUG-2008 C-reactive protein 2.49	mg/l 5.0
05-AUG-2008 Potassium 3.3	mmol/l 3.6 5.1
07-AUG-2008 Potassium 3.8	mmol/l 3.6 5.1
05-AUG-2008 Sodium 123	mmol/l 136 144
07-AUG-2008 Sodium 132	mmol/l 136 144
05-AUG-2008 Calcium 2.33	mmol/l 2.2 2.55
07-AUG-2008 Calcium 2.45	mmol/l 2.2 2.55
07-AUG-2008 Albumin 35.8	g/l 35 48
05-AUG-2008 White blood cells urine 500	mcl 0 25
05-AUG-2008 Red blood cells urine <10	mcl 0 10
05-AUG-2008 Protein urine <0.05	g/l < 0.25

Bayer Case ID: 201028145GPV

Tracking:

31March2009 Initial received from authority

08Jun2010 follow up information received from the german health authority - concomitant medication: Metoclopramide

14Jun2010 - Bayer Case ID added to narrative

Boehringer Ingelheim GmbH, Germany, 18.06.2010 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-02747DE

Sender diagnosis: not reported

Narrative

This is a spontaneous report (health professional confirmed) from the German Health Authority BfArM.

On 08Jun2006 follow-up information has been received concerning a 84-year old female patient. She was treated with Aggrenox (200 mg dipyridamole + 25 mg ASS), 2 capsules daily orally, from May-2006 until 05-Aug-2008 for thrombosis prophylaxis; and the non BI suspected drugs: Enabeta comp. (10mg enalapril + 25mg HCT) tablets daily once orally, from May-2006 until 05-Aug-2008 for thrombosis prophylaxis, Enabeta Comp (10 mg enalapril + 25 mg HCT) tablets daily once orally for arterial hypertension from May 2006 to 05Aug2008, ibuprofen 600 mg tablets daily twice orally from Jun-2008 until 05-Aug-2008 for coxarthrosis and Indometacin 50 mg tablets orally daily twice for coxarthrosis treatment also from Jun2008 to Jul2008. All those are considered suspected drugs by the reporting source. While on therapy with the suspected drugs the patient suffered from the following events and was hospitalised from 05Aug2008 until 13Aug2008.

Events:

significant hyponatraemia from Jul2008 until 07Aug2008 treated with infusion therapy and withdrawal of diuretic therapy
 mild hypokalaemia from -Jul-2008 until 07-Aug-2008 treated with infusion therapy and withdrawal of diuretic therapy
 erosive gastritis from Jul-2008 until 07Aug2008 treated with infusion therapy and withdrawal of diuretic therapy
 nausea treated with infusion therapy, withdrawal of diuretic therapy and oral administration of metoclopramide (Jul-2008 until Aug-2008)
 vomiting treated with infusion therapy and withdrawal of diuretic therapy
 headache treated with infusion therapy and withdrawal of diuretic therapy
 exsiccosis treated with infusion therapy and withdrawal of diuretic therapy
 urinary tract infection treated with antibiotics (ciprofloxacin)
 occult bleed positive findings twice diagnosed on 07-Aug 2008
 chronic renal alteration diagnosed on 07-Aug 2008 (abdominal sonographic imaging)
 damage of liver parenchyma diagnosed on 07-Aug-2008 (abdominal sonographic imaging)
 mild aortic sclerosis diagnosed on 07-Aug 2008 (abdominal sonographic imaging)
 pain upon pressure right groin

The patient recovered from the events hyponatraemia, hypokalaemia and erosive gastritis. For all other events the outcome was not reported.

The reporting source saw a possible reasonable relationship with the suspected drugs and the events as follows:

Aggrenox and the event erosive gastritis, but dubious causal relationship with the hyponatraemia and the hypokalaemia. For all other events causal relationship was not reported.

Enabeta comp and the events hyponatraemia and hypokalaemia, but dubious causal relationship with the erosive gastritis. For all other events causal relationship was not reported.

Ibuprofen and the events hyponatraemia and erosive gastritis, but dubious causal relationship with the hypokalaemia. For all other events causal relationship was not reported.

Indometacin and the events hyponatraemia and erosive gastritis, but dubious causal relationship with the hypokalaemia. For all other events causal relationship was not reported.

Concomitant diseases included coxarthrosis diagnosed 2005, arterial hypertension diagnosed 1998, hypertensive heart disease diagnosed in 2000 and hypothyroidism.

Nodular goiter treated ended with strumectomy in 1978 is considered as past disease. Other relevant medical history is not reported.

Concomitant medications included Felacor(felodipine) for treatment of arterial hypertension from the year 2000 on; L-Thyroxin Hexal from 1978 on for treatment of hypothyroidism and Metoclopramide for treatment of nausea between Jul-2008 to 08-Aug2008.

Laboratory test(s) show the following results:

Results of tests:	normal ranges:
	high and low
05-AUG-2008 Creatinine 78	mmol/l 35 88
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05-AUG-2008 Creatinine clearance >1	ml/s 1.58 2.66
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05-AUG-2008 Quick's test 110	Prozent 70 130
05-AUG-2008 Partial prothrombin time 27.5	sec 27 37
05-AUG-2008 INR 0.93	therapeutic area 1.25
05-AUG-2008 Thrombocyte count 236	Gpt/l 150 450
05-AUG-2008 Haemoglobin 8.6	mmol/l 7.4 9.9
05-AUG-2008 Haematocrit 0.38	SI-Einheit 0.35 0.47
05-AUG-2008 Total white blood cell count 7.78	Gpt/l 4.0 9.0
05-AUG-2008 C-reactive protein 2.49	mg/l 5.0
05-AUG-2008 Potassium 3.3	mmol/l 3.6 5.1
07-AUG-2008 Potassium 3.8	mmol/l 3.6 5.1
05-AUG-2008 Sodium 123	mmol/l 136 144
07-AUG-2008 Sodium 132	mmol/l 136 144
05-AUG-2008 Calcium 2.33	mmol/l 2.2 2.55
07-AUG-2008 Calcium 2.45	mmol/l 2.2 2.55
07-AUG-2008 Albumin 35.8	g/l 35 48

05-AUG-2008 White blood cells urine 500	mcl 0 25
05-AUG-2008 Red blood cells urine <10	mcl 0 10
05-AUG-2008 Protein urine <0.05	g/l < 0.25

Bayer Case ID: 201028145GPV

Tracking:

31March2009 Initial received from authority

08Jun2010 follow up information received from the german health authority - concomitant medication: Metoclopramide, listedness for event exsiccosis updated

14Jun2010 - Bayer Case ID added to narrative

sanofi-aventis, 18.06.2010 DE-SANOFI-AVENTIS-A04200900417

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09040167)

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

This case involves an 84-year-old female with a medical history of strumectomy in 1978. The following drugs are known to have been taken by the patient prior to the event: felodipine for arterial hypertension and levothyroxine sodium for hypothyroidism. Additionally hypertensive heart disease and goiter nodular were known.

Furthermore the patient suffered from urinary infection, the according therapy with ciprofloxacin was started after occurrence of the case events.

In June 2008 the patient had started with ibuprofen 600 mg twice as needed and indometacin 50 mg twice as needed (trade names not provided) for coxarthrosis. The further medication included dipyridamole+acetylsalicylic acid (225 mg twice daily) for thrombosis prophylaxis and enalapril maleate (10 mg daily) with hydrochlorothiazide (25 mg daily) for arterial hypertension, both started in May 2006.

In July 2008 the patient began to complain about nausea, vomiting and headache. Indometacin was discontinued in the same month. The patient was hospitalised due to pronounced hyponatremia and mild hypokalemia on 05-Aug-2008. Corrective treatment was started with infusions NOS, proton pump inhibitors and metamizole for pain caused by coxarthrosis. Ibuprofen, dipyridamole+acetylsalicylic acid and hydrochlorothiazide was stopped on day of admission. The performed gastroscopy displayed antrum gastritis with erosive component on 08-Aug-2008.

On day before the abdominal sonography disclosed concomitantly chronic renal changes, hepatocellular damage and mild aortic sclerosis.

The patient recovered from hyponatremia and hypokalemia by 07-Aug-2008 and from erosive gastritis in the same months. The patient could be discharged on 13-Aug-2008.

Causality assessment for ibuprofen determined by PV centre:

hyponatremia: possible

hypokalemia: dubious

erosive gastritis: possible

alternative explanation by PV centre:

- hypothyroidism for hyponatremia,

- non-company suspected drugs: hydrochlorothiazide for hyponatremia and hypokalemia,

enalapril maleate for hyponatremia

Additional information received from a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09040167) on 08-JUN-2010:

Due to erosive gastritis, hypokalemia and hyponatremia with symptoms of nausea, vomiting and headache, the patient was hospitalised from 05-AUG-2008 to 13-AUG-2008. At time of hospitalisation: blood pressure of 160/70 mmHg, signs of exsiccosis, pressure pain above right groin.

Course:

07-AUG-2008: laboratory: protein- and electrophoresis: without any pathologic findings; occult blood: 2 times positive findings

07-AUG-2008: abdominal sonography: chronic renal changes, liver parenchym damage, slight aortic sclerosis.

08-AUG-2008: gastroscopy: antrum gastritis with erosive components.

Medication when the patient was discharged from hospital: enalapril, levothyroxine, omeprazole, metamizole.

Improvement of symptoms.

Moreover the reporter stated that indometacin was given as tablet. Therefore indometacin is no longer considered as company product in this case as it is only available as suppository.

Causality assessment for ibuprofen determined by PV centre:

hyponatremia: possible

erosive gastritis: possible

For hypokalemia no assessment was reported.

Sender diagnosis: not reported

Narrative

Source of report and patient demography: This spontaneous case report was identified on the internet web page on 30-Mar-2010 of the German health authority (Federal Institute for Drugs and Medical Devices - BfArM), referring to an 84 year-old female. Further information received on 07-Jun-2010 is included.

Medical and drug history: Patient has the concomitant conditions of coxarthrosis, arterial hypertension, hypertensive heart disease, and hypothyroidism. Patient's relevant medical history includes nodular goiter until 1978, strumectomy in 1978, and nausea from July 2008 to August 2008. Concomitant drugs have been given for arterial hypertension, hypothyroidism, and nausea. For details see section "Concomitant Drugs" at the end of this report.

Suspect drugs, timing and conditions surrounding the onset of the reactions:

Patient received Ibuprofen for coxarthrosis. Administration began in Jun-2008 as needed until 05-Aug-2008.

Patient also received Indometacin for coxarthrosis. Administration began in Jun-2008, for a duration of 4 weeks and 3 days at a total daily dose of 100 mg oral.

Co-suspected drugs are:

Aggrenox (acetylsalicylic acid + dipyridamole) for thrombosis prophylaxis and Enabeta comp (enalapril maleate + hydrochlorothiazide) for arterial hypertension.

Administration began in May-2006, for a duration of 2 years 3 months, at a dose of 200/25 mg twice a day of Aggrenox and 10/25 mg oral of Enabeta comp.

The progression of the events and their outcome in the patient: The patient's anamnesis included a previous treatment with indometacin and ibuprofen for coxalgia. Since several days the patient had suffered from nausea, vomiting and headache. Thus, after being diagnosed with dyselectrolytemia (significant HYPONATREMIA with mild HYPOKALEMIA) and EROSIVE GASTRITIS the patient was hospitalised from 05-Aug-2008 to 13-Aug-2008. On admission the blood pressure was measured 160/70mmHg.

Apart from that the patient showed signs of exsiccosis and complained about pain on pressure affecting the right groin area. Subsequently the patient underwent infusion therapy and received proton pump inhibitors (NOS). Furthermore metamizole was given for analgetic treatment while a urinary tract infection was treated with ciprofloxacin. The diuretics were discontinued.

Further course: On 07-Aug-2008 protein electrophoresis did not yield any pathological findings while occult blood test showed 2 positive results. Abdominal sonography revealed chronic renal alterations and damages of the liver parenchyma. Furthermore more aortic sclerosis was detected. On 08-Aug-2008 gastroscopy showed antral gastritis with erosion. On 13-Aug-2008 the patient was discharged in ameliorated condition.

Medication on discharge:

- Enalapril
- L-Thyroxin (levothyroxin)
- Omeprazole
- Metamizole (as needed)

The original reporter's clinical assessment: The reporter considered the diagnosis EROSIVE GASTRITIS related to Aggrenox, Indometacin and Ibuprofen. The reporter assessed the HYPONATREMIA related to Indometacin, Ibuprofen, Enabeta comp. The event HYPOKALEMIA was assessed as related to Enabeta comp by the reporter.

For all other drug-event relationships assessment was given as dubious.

Specification on causalities:

Hyponatremia:

Headache, nausea and vomiting - potentially caused by losses of sodium and potassium after HTC intake - were declared typical symptoms for hyponatremia. Furthermore enalapril intake and NSAID (non steroidal anti inflammatory drug) intake with subsequent release of ADH were considered.

Hypokalemia:

Nausea and vomiting were declared associated with hypokalemia but hyponatremia was considered the major problem.

Erosive gastritis:

Nausea and vomiting were considered associated with erosive gastritis due to a potential dyselectrolytemia following vomiting. On the other hand dyselectrolytemia was estimated a potential cause of nausea and vomiting, in this way leading on to gastritis.

Drug: Aggrenox - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with acetylsalicylic acid.

Drug: Enabeta comp - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) based on enalapril intake. Both hyponatremia and hypokalemia are known side effects during treatments with hydrochlorothiazide. Upon discontinuation and following infusion therapy the sodium values returned to normal.

Alternative causes:

- Existing and medicated hypothyroidism
- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Enabeta comp - Event: HYPOKALEMIA

Specification: Hypokalemia is a known side effect during treatments with hydrochlorothiazide. Upon discontinuation the potassium values returned to normal.

Drug: Ibuprofen - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) during non steroidal anti inflammatory drugs (NSAID) intake.

Alternative causes:

- Further suspected drugs
- Hypothyroidism (medically substituted)
- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Ibuprofen - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with ibuprofen.

Drug: Indometacin - Event: HYPONATREMIA

Specification: Hyponatremia initiated by an increased release of antidiuretic hormone (ADH) during non steroidal anti inflammatory drugs (NSAID) intake

Alternative causes:

- Further suspected drugs
- Hypothyroidism (medically substituted)
- Vomiting (e.g. caused by gastritis or urinary tract infection)

Drug: Indometacin - Event: EROSIVE GASTRITIS

Specification: Mucosal damages are known side effects during treatments with indometacin.

****Addendum:

Correction on 15-Jun-2010: The initial report received on 30-Mar-2009 at local drug safety and not in 2010 as mistakenly reported (typo error).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09041058

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	200903				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Unknown	/	-	Analgesia
ibuprofen			-	
2 Xarelto	Oral	/	-	Thrombosis prophylaxis
rivaroxaban		245	-	

Drugs (concomitant)

3 indometacin	Unknown	/	-	Prophylaxis
indometacin			-	
4 Valoron	Unknown	/	-	Analgesia
tilidine hydrochloride			-	
5 voltaren	Unknown	/	-	Analgesia
diclofenac sodium			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 31.03.2009 DE-BAYER-200917097GPV

Sender diagnosis: not reported

Narrative

This initial spontaneous case report was received on 20-Mar-2009 from a health professional in Germany, via company's sales representative, referring to a patient of unknown gender.

Suspect drug, timing and conditions surrounding the onset of the reaction:

Patient received Xarelto for thrombosis prophylaxis. Administration dates unknown, total daily dose not reported.

Concomitant drugs have been given.

In Mar-2009, patient experienced INCREASED TRANSAMINASES considered serious as important medical event. Outcome unknown at time of report.

The physician reported about a patient who was treated with Xarelto (rivaroxaban) for thrombosis prophylaxis due to surgery of total joint endoprosthesis. Prior to surgery liver parameters were screened routinely. Post-operative laboratory values detected significant increased transaminases. Concomitant medication included Indometacin for ossification prophylaxis. For analgesia patient received Voltaren (diclofenac), Ibuprofen and Valoron drops (naloxon/tilidin) as needed. Withdrawal of acetylsalicylic acid one week prior to surgery is done on a routine basis. Reporters comment: 3rd case of this particular hospital reporting about 'increased liver values' (NOS). Other cases: 200832128GPV and 200832141GPV. Despite several contacts, no lab data for any of the cases received yet.

The original reporter's clinical assessment:

No assessment was given.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09042861

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19870122	Age: 22 Year	weight (kg): 57	BMI_int: 20.2	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal pain	20090207			9 Day	recovering/resolving
2 Acute fulminant hepatitis	20090207			9 Day	recovering/resolving
3 Ascites					recovering/resolving
4 Liver damage	20090207				recovering/resolving
5 Hepatic lesion	20090207				recovering/resolving
6 Urine color abnormal		20090218			recovered/resolved
7 Explorative laparotomy	20090211			13 Day	Unknown
8 Weakness					not recovered/not resolved
9 Vomiting					recovering/resolving
10 Liver failure					recovering/resolving
11 Hepatosplenomegaly					recovering/resolving
12 Hepatic function decreased					recovering/resolving
13 Headache					recovering/resolving
14 Chills	20090207			9 Day	recovering/resolving
15 Body temperature increased	20090207			9 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CLEXANE enoxaparin sodium	Subcutaneous	0.4 Mg milligram(s)/Day 120	20090121- -	Thrombosis prophylaxis
2 ibuprofen ibuprofen	Oral	400 Mg milligram(s)/Day 245	20090121-20090130 10 Day	Pain NOS
3 OXYGESIC oxycodone hydrochloride	Oral	30 Mg milligram(s)/Day 212	20090130-20090210 12 Day	Pain NOS
4 PANTOZOL pantoprazole sodium	Oral	40 Mg milligram(s)/ Day 245	20090121-20090210 21 Day	Gastric ulcer prophylaxis
5 voltaren diclofenac sodium	Rectal	100 Mg milligram(s)/Day 242	20090130-20090210 12 Day	Pain NOS

Drugs (concomitant)

6 BERLOSIN metamizole	Oral	500 Mg milligram(s)/Day 245	20090121-20090130 10 Day	Pain NOS
7 sex hormones and modulators of the geni. sys.		/	2002-20090121 7 Year	Contraception

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lumbar vertebral fracture	20090121		Yes	lumbar vertebral fracture
Road traffic accident	20090121	20090121	No	Z.n.
Intervertebral disc replacement	20090127		Unknown	
Abdominal injury			Unknown	blunt abdominal trauma
Pain NOS	20090121		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Qualification: Other Health Professional

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20090223	0.62	µmol/l*s	
Aspartate aminotransferase	20090226	0.23	µmol/l*s	
Alanine aminotransferase	20090223	0.87	µmol/l*s	
Alanine aminotransferase	20090226	0.72	µmol/l*s	
Bilirubin total	20090223	24	µmol/l	
Bilirubin direct	20090226	19.9	µmol/l	
Alkaline phosphatase	20090226	1.62	µmol/l*s	
Gamma GT	20090223	2.05	µmol/l*s	
Gamma GT	20090226	1.76	µmol/l*s	
Quick's test	20090210	64	Prozent	
Quick's test	20090223	80	Prozent	
Quick's test	20090226	99	Prozent	
Albumin	20090223	60	g/l	
C-reactive protein	20090210	192	mg/l	
C-reactive protein	20090223	3.3	mg/l	
C-reactive protein	20090226	2	mg/l	
Creatinine	20090223	72	µmol/l	
Gamma GT	20090226	1.76	µmol/l*s	

Narratives (nur bei elektronischem Eingang)

Universitätsklinikum Jena, 03.04.2009 DE-PVZIBE-JENA-10072

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

fulminante Hepatitis unter Diclofenac, Pantoprazol, Oxycodon

KURZZUSAMMENFASSUNG:

fulminante Hepatitis mit Synthesestörung

BESCHREIBUNG:

Überweisung der jungen Patientin aus Heimatkrankenhaus zur Exploration Lebertransplantation bei Leberversagen infolge fulminanter Hepatitis. Ab 07.02.09 intermittierende Temperaturanstiege, Schüttelfrost, Bauch- und Kopfschmerzen. Aufnahme im Heimatkrankenhaus am 10.02.2009. Dort am 11.02.2009 explorative Laparotomie bei V.a. Milzruptur bei freier Flüssigkeit im Abdomen und Z.n. Verkehrsunfall 21.01.09 mit Fraktur eines Lendenwirbels und operativer Versorgung (27.01.09 Wirbelkörper-Ersatz), sowie Z.n. stumpfem Bauchtrauma. Bei freier Flüssigkeit handelte es sich um Aszites. Erbrechen bis 18.02.09, Urin dunkel (braun). Bei Eintreffen in unserer Klinik geht es Patientin schon besser, subjektiv noch geschwächt, aber sonst beschwerdefrei.

THERAPIE:

Heimatkrankenhaus: 10.02.-23.02.09 Intensivstation, Diagnostik. 10.02.09 Pantozol, Voltaren, Oxygesic abgesetzt. 23.02.-27.02.09 Clexane 0.4 mg/d, Diagnostik.

VERLAUF:

Überweisungsdiagnose: extreme Hepatosplenomegalie und Lebersynthesestörung unklarer Genese. In Heimatkrankenhaus bereits autoimmune Hepatitis, primäre biliäre Zirrhose, frische virale Infektion (CMV, HSV, EBV, Adenovirus, Parvo-Virus, Paramyxovirus, Mumps-, Masern-, RSV, Coxsackie-Virus, HIV, Hepatitis A,B,C, etc) ausgeschlossen. 13.02.09 MEGX-Test (Leberfunktionstest) < 20 µg/ml --> stark erniedrigt. 20.02.09 MEGX-Test nach 15 min 81,5 µg/ml, 30 min 71,5 µg/ml --> immer noch stark erniedrigt. Leider sind aus dem Aufenthalt im Heimatkrankenhaus keine Leberwerte übermittelt. Temperatur 23.02. 37,6°C; 24.02. 36,8°C. 24.02.09 SLA-AAK, LKM-AAK negativ --> keine autoimmune Hepatitis, AMA-AAK negativ --> keine primäre biliäre Zirrhose, ANCA - Titer negativ --> keine primäre sklerosierende Cholangitis.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PANTOZOL**BETREFFENDE UAW:** akute fulminante Hepatitis

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: OXYGESIC**BETREFFENDE UAW:** akute fulminante Hepatitis

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: VOLTAREN**BETREFFENDE UAW:** akute fulminante Hepatitis

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

NOVARTIS, 14.04.2009 DE-002147023-PHHY2009DE12954

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (HA reference number: DE-BFARM-09042861), and also via Nycomed (0204946) on 03 Apr 2009: The patient had road traffic accident on 21 Jan 2009 with blunt abdominal trauma, which resulted in lumbar vertebral fracture and the patient underwent lumbar vertebral replacement on 27 Jan 2009. The polymedicated patient was on Voltaren (diclofenac sodium) suppository from 30 Jan 2009 for pain. The patient was also on Oxygesic (oxycodone hydrochloride) for pain and Pantozol (pantoprazole) for gastric ulcer prophylaxis, which were also considered suspect. Since 07 Feb 2009 the patient had intermittent temperature increases, chills, abdominal pain and headache. The patient was admitted to hospital on 10 Feb 2009. On 11 Feb 2009 explorative laparotomy was done for the suspicion of spleen rupture with free fluid in abdomen (due to blunt abdominal trauma suffered during the road traffic accident). The free fluid was assessed as ascitis, possible spleen rupture was excluded. Extreme hepatosplenomegaly and liver synthesis impairment was found. Laboratory tests done on 10 Feb 2009 revealed increased C-reactive protein 192 mg/l (normal 0-7.5). On 13 Feb 2009, revealed severely decreased Monoethyl glycine xylidide (MEGX) test (liver function deterioration; Liver function test values were not reported), on 23 Feb 2009 revealed aspartate aminotransferase 0.62 umol/l (normal less than 0.52), Alanine aminotransferase 0.87 umol/l (normal less than 0.56), bilirubin total 24 umol/l (normal less than 21), gamma GT 2.05 umol/l (normal 0-0.63), albumin 60 g/l (normal 35-53), normal C reactive protein (3.3), and on 26 Feb 2009, bilirubin direct 19.9 umol/l (normal 0-3), normal aspartate aminotransferase (0.23). A diagnosis of acute fulminant hepatitis with liver failure was made. This event was considered life threatening. The onset for acute fulminant hepatitis was reported to be 07 Feb 2009. The patient complained of vomiting and dark urine (brown) until 18 Feb 2009. The clinicians regarded causal relationship as most probable for diclofenac, Oxygesic and Pantozol were also suspected. The therapy with all three drugs was discontinued on 10 Feb 2009, and the outcome of the patient was reported to be condition improving for all the symptoms except for weakness.

HELIOS Klinikum Wuppertal, 21.04.2010 DE-PVZIBE-JENA-10072

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberschädigung

KURZZUSAMMENFASSUNG:

Leberschädigung mit Synthesestörung unter Diclofenac, Pantoprazol, Oxycodon und Clexane

BESCHREIBUNG:

Überweisung der jungen Patientin aus Heimatkrankenhaus zur Exploration Lebertransplantation bei Leberversagen infolge fulminanter Hepatitis. Ab 07.02.09 intermittierende Temperaturanstiege, Schüttelfrost, Bauch- und Kopfschmerzen. Aufnahme im Heimatkrankenhaus am 10.02.2009. Dort am 11.02.2009 explorative Laparotomie bei V.a. zweizeitige Milzruptur bei freier Flüssigkeit im Abdomen und Z.n. Verkehrsunfall 21.01.09 mit Fraktur eines Lendenwirbels und operativer Versorgung (27.01.09 Wirbelkörper-Ersatz), sowie Z.n. stumpfem Bauchtrauma. Bei freier Flüssigkeit handelte es sich um Aszites. Erbrechen bis 18.02.09, Urin dunkel (braun). Bei Eintreffen in unserer Klinik geht es Patientin schon besser, subjektiv noch

geschwächt, aber sonst beschwerdefrei.

THERAPIE:

Heimatkrankenhaus: 10.02.-23.02.09 Intensivstation, Diagnostik. 10.02.09 Pantozol, Voltaren, Oxygesic abgesetzt. 23.02.-27.02.09 Clexane 0.4 mg/d, Diagnostik.

VERLAUF:

Überweisungsdiagnose: extreme Hepatosplenomegalie und Lebersynthesestörung unklarer Genese. In Heimatkrankenhaus bereits autoimmune Hepatitis, primäre biliäre Zirrhose, frische virale Infektion (CMV, HSV, EBV, Adenovirus, Parvo-Virus, Paramyxovirus, Mumps-, Masern-, RSV, Coxsackie-Virus, HIV, Hepatitis A,B,C, etc) ausgeschlossen. 13.02.09 MEGX-Test (Leberfunktionstest) < 20 µg/ml --> stark erniedrigt. 20.02.09 MEGX-Test nach 15 min 81,5 µg/ml, 30 min 71,5 µg/ml --> immer noch stark erniedrigt. Leider sind aus dem Aufenthalt im Heimatkrankenhaus keine Leberwerte übermittelt. Temperatur 23.02. 37,6°C; 24.02. 36,8°C. 24.02.09 SLA-AAK, LKM-AAK negativ -->keine autoimmune Hepatitis, AMA-AAK negativ --> keine primäre biliäre Zirrhose, ANCA - Titer negativ --> keine primäre sklerosierende Cholangitis.

ERLÄUTERUNGEN ZUR UAW:

10.2. - 23.2 intensivmedizinische Betreuung in Heimat-KH wg fulminanter Hepatitis (dortige Diagnose). Schließlich bei Unsicherheit über den zukünftigen Verlauf und evtl Notwendigkeit einer Leber-Transplantation Verlegung zu uns. Hier bereits bei Vorstellung wesentlich besserer Zustand als erwartet mit weiterer Stabilisierung im Verlauf.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PANTOZOL

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: OXYGESIC

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: VOLTAREN

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: CLEXANE

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: It Fachinfo wird ein Anstieg der Leberenzyme häufig beschrieben. In diesem Fall negatives Dechallenge, da trotz Weitergabe des Medikaments Rückbildung der hepatischen Dysfunktion. Zusammenhang unwahrscheinlich.

NOVARTIS, 27.04.2010 DE-002147023-PHHY2009DE12954

Sender diagnosis: not reported

Narrative

Initial report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (HA reference number: DE-BFARM-09042861), and also via Nycomed (0204946) on 03 Apr 2009: The patient had road traffic accident on 21 Jan 2009 with blunt abdominal trauma, which resulted in lumbar vertebral fracture and the patient underwent lumbar vertebral replacement on 27 Jan 2009. The polymedicated patient was on Voltaren (diclofenac sodium) suppository from 30 Jan 2009 for pain. The patient was also on Oxygesic (oxycodone hydrochloride) for pain and Pantozol (pantoprazole) for gastric ulcer prophylaxis, which were also considered suspect. Since 07 Feb 2009 the patient had intermittent temperature increases, chills, abdominal pain and headache. The patient was admitted to hospital on 10 Feb 2009. On 11 Feb 2009 explorative laparotomy was done for the suspicion of spleen rupture with free fluid in abdomen (due to blunt abdominal trauma suffered during the road traffic accident). The free fluid was assessed as ascitis, possible spleen rupture was excluded. Extreme hepatosplenomegaly and liver synthesis impairment was found. Laboratory tests done on 10 Feb 2009 revealed increased C-reactive protein 192 mg/l (normal 0-7.5). On 13 Feb 2009, revealed severely decreased Monoethyl glycine xylylidide (MEGX) test (liver function deterioration; Liver function test values were not reported), on 23 Feb 2009 revealed aspartate

aminotransferase 0.62 umol/l (normal less than 0.52), Alanine aminotransferase 0.87 umol/l (normal less than 0.56), bilirubin total 24 umol/l (normal less than 21), gamma GT 2.05 umol/l (normal 0-0.63), albumin 60 g/l (normal 35-53), normal C reactive protein (3.3), and on 26 Feb 2009, bilirubin direct 19.9 umol/l (normal 0-3), normal aspartate aminotransferase (0.23). A diagnosis of acute fulminant hepatitis with liver failure was made. This event was considered life threatening. The onset for acute fulminant hepatitis was reported to be 07 Feb 2009. The patient complained of vomiting and dark urine (brown) until 18 Feb 2009. The clinicians regarded causal relationship as most probable for diclofenac, Oxygesic and Pantozol were also suspected. The therapy with all three drugs was discontinued on 10 Feb 2009, and the outcome of the patient was reported to be condition improving for all the symptoms except for weakness.

Follow-up report received from a regional Pharmacovigilance center (intensive monitoring project) via Health Authority on 22 Apr 2010: The patient was in intensive care unit due to fulminant hepatitis from 10 Feb 2010 to 23 Feb 2010 (diagnosis of home hospital) and transferred to Helios clinical. Patient's condition was stabilized. Clexane was continued, but the hepatic dysfunction was dissolved. The reporter assessed the causality as unlikely for Clexane and Ibuprofen, possible for Pantozol, Oxycodone and Diclofenac sodium.

sanofi-aventis, 27.04.2010 DE-SANOFI-AVENTIS-2010SA023580

Sender diagnosis: not reported

Narrative

Initial Report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-JENA-10072) and was extracted from the website of the German health authority (authority number DE-BFARM-09042861) on 23-Apr-2010

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This serious case involves a 22-year-old female patient. On 21-Jan-2009 she experienced a road traffic accident with fracture of a lumbar vertebrae and a blunt abdominal trauma. Six days later a vertebral body replacement was performed. Since 07-Feb-2009 she suffered from intermittent increases in temperature, chill, abdominal pain and headache. Three days later the patient was hospitalised. From 10-Feb-2009 until 23-Feb-2009 she was in an intensive-care unit due to a fulminant hepatitis. Pantoprazole sodium, diclofenac potassium and oxycodone hydrochloride were withdrawn on 10-Feb-2009. On 11-Feb-2009 exploratory laparotomy was performed with suspicion of a staged splenic rupture at free fluid in the abdomen. Ascites was diagnosed. On 13-Feb-2009 a liver function test was performed with the following finding: monoethylglycine xylidide was greatly decreased (<20 µg/ml). The patient suffered from vomiting until 18-Feb-2009, urine was dark (brown). On 20-Feb-2009 a further liver function test was performed with the following finding: monoethylglycine xylidide was still greatly decreased with 81.5 µg/ml after 15 min and 71.5 µg/ml after 30 min.

The patient was transferred to university hospital for evaluation of a liver transplantation because of hepatic failure due to fulminant hepatitis. Referral diagnosis: extreme hepatosplenomegaly and hepatic synthesis disorder of unknown pathogenesis. On admission the patient was nearly free of symptoms, but subjectively weakened. Primary biliary cirrhosis, fresh viral infections, autoimmune hepatitis and primary sclerosing cholangitis were excluded. The patient was recovering.

The reporter assessed the causal relationship according to WHO score for pantoprazole sodium and oxycodone hydrochloride as "possible", for diclofenac potassium as "probable" and for enoxaparin sodium and ibuprofen as "unlikely".

Explanatory notes on causality:

For pantoprazole sodium, oxycodone hydrochloride and diclofenac potassium: Liver damage was most likely due to drug-related toxicity by the intake of diclofenac. Pantoprazole sodium and oxycodone hydrochloride were also suspected. In addition, viral or operation related causes could not be ruled out with certainty. The liver function spontaneously recovered after withdrawal of the suspected medication.

For enoxaparin sodium: Increase in liver enzymes is frequently described in the Physician's information leaflet. In this case, however, dechallenge was negative. Despite continuation of treatment with the drug hepatic dysfunction regressed. Therefore, a causal relationship is unlikely.

Causality explanation concerning ibuprofen was not provided.

Bayer Vital GmbH, 28.04.2010 DE-BAYER-201023009GPV

Sender diagnosis: not reported

Narrative

This solicited case report was identified on 22-Apr-2010 in an internet-based pool of cases of BfArM, the regulatory health authority in Germany, reported from a German physician via local pharmacovigilance centre, which systematically screens for adverse drug reactions in selected hospitals. The report refers to a 22 year-old female (height: 168 cm, weight: 57 kg), who experienced hepatic lesion with synthesis disorder upon treatment with ibuprofen, diclofenac, pantoprazol, oxycodon, and enoxaparin.

Patient's concomitant conditions/risk factors included a road traffic accident on 21-Jan-2009, with lumbar vertebral fracture and pain ongoing since 21-Jan-2009. Concomitant drugs were given for pain. Contraceptives have been ceased on 21-Jan-2009 (see '*Concomitant Drugs' section).

Patient received the following drugs, reported as 'suspect drugs':

- oral Ibuprofen 400 mg tablet was given once daily from 21-Jan-2009 until 30-Jan-2009 for pain;
- oral Pantozol (pantoprazole) 40 mg tablet was given once daily from 21-Jan-2009 until 10-Feb-2009 for gastric ulcer prophylaxis;
- rectal Voltaren (diclofenac) 50 mg suppository was given twice daily from 30-Jan-2009 until 10-Feb-2009 for pain;
- oral Oxygesic (oxycodone) 10 mg prolonged-release tablet was given thrice daily from 30-Jan-2009 until 10-Feb-2009 for pain;
- subcutaneous Clexane (enoxaparin) 0.4 mg injection was given once daily from 21-Jan-2009 from thrombosis prophylaxis.

Onset, progression and outcome of the adverse reaction:

From 07-Feb-2009 onward, the 22 year-old female patient experienced intermittent rise in temperature, shivering, abdominal pain and headache.

On 10-Feb-2009, patient was admitted to hospital in her hometown.

On 11-Feb-2009, explorative laparotomy was performed for suspected two-timed splenic rupture with free abdominal fluid. Patient had experienced a traffic accident on 21-Jan-2009 with lumbar vertebra fracture which had to be treated surgically (replacement of vertebral body on 27-Jan-2009), and blunt abdominal trauma. Free fluid was identified as ascites.

Until 18-Feb-2009, the patient suffered from vomiting, her urine was found to be dark brown.

Patient was transferred to reporting clinic for the exploration of a liver transplantation for liver failure due to fulminant hepatitis.

On admission at this hospital, patient's condition had improved, patient felt that she was still weak, but was otherwise free from complaints.

Therapy:

Hospital in the patient's hometown: From 10-Feb-2009 to 23-Feb-2009, treatment at intensive care unit, diagnostics. On 10-Feb-2009: pantoprazol, diclofenac, oxycodon

withdrawn. From 23-Feb-2009 to 27-Feb-2009, enoxaparin 0.4 mg daily, diagnostics.

Course:

Diagnosis provided by the admitting hospital: extreme hepatosplenomegaly and liver synthesis disorder of unknown origin. In the hospital of patient's hometown already autoimmune hepatitis, primary biliary cirrhosis. Fresh viral infection was excluded (no cytomegaly virus, herpes simplex virus, Epstein-Barr virus, adenovirus, parvovirus, mumps, measles, respiratory syncytial virus, Coxsackie virus, human immunodeficiency virus, hepatitis A, B, C etc.). Monoethylglycine xylylidide testing (liver function test) < 20 mcg/ml severely decreased. On 20-Feb-2009, the results of the MEGX test were still severely decreased: 81.5 mcg/ml after 15 m, 71.5 mcg/ml after 30 m. Unfortunately, no liver values were reported by the hospital in patient's hometown.

Temperature: 23-Feb-2009: 37.6°C, 24 Feb-2009: 36.8 °C. On 24-Feb-2009: soluble liver antigen auto-antibodies and liver kidney microsome auto-antibodies: negative, that meant (i.e.) there was no auto-immune hepatitis, anti-mitochondrial antibodies: negative, i.e. no primary biliary cirrhosis, anti-neutrophil cytoplasmic antibodies titre: negative, i.e. no primary sclerotic cholangitis.

Comments on the serious adverse event:

From 10-Feb-2009 to 23-Feb-2009, intensive care at hospital of patient's hometown due to fulminant hepatitis (diagnosis made there). Finally, due to uncertainty as to future course and possible necessity of a liver transplantation, patient was transferred to the reporting hospital. There, patient was found to be in an essentially better condition as had been expected. In the course, her condition stabilised further. At the time of the report, patient was recovering.

Event was considered serious due to its life-threatening nature and due to requiring hospitalization/prolongation of existing hospitalization.

Causality assessment:

Reporter assessed the diagnosis HEPATIC LESION to be probably related to Voltaren (diclofenac), to be possibly related to Pantozol (pantoprazole) and Oxygesic (oxycodone), and to be unlikely related to Ibuprofen and Clexane (enoxaparin) using method 'WHO score' for evaluation.

HELIOS Klinikum Wuppertal, 01.06.2010 DE-PVZIBE-JENA-10072

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberschädigung

KURZZUSAMMENFASSUNG:

Leberschädigung mit Synthesestörung unter Diclofenac, Pantoprazol, Oxycodon und Clexane

BESCHREIBUNG:

Überweisung der jungen Patientin aus Heimatkrankenhaus zur Exploration Lebertransplantation bei Leberversagen infolge fulminanter Hepatitis. Ab 07.02.09 intermittierende Temperaturanstiege, Schüttelfrost, Bauch- und Kopfschmerzen. Aufnahme im Heimatkrankenhaus am 10.02.2009. Dort am 11.02.2009 explorative Laparotomie bei V.a. zweizeitige Milzruptur bei freier Flüssigkeit im Abdomen und Z.n. Verkehrsunfall 21.01.09 mit Fraktur eines Lendenwirbels und operativer Versorgung (27.01.09 Wirbelkörper-Ersatz), sowie Z.n. stumpfem Bauchtrauma. Bei freier Flüssigkeit handelte es sich um Aszites. Erbrechen bis 18.02.09, Urin dunkel (braun). Bei Eintreffen in unserer Klinik geht es Patientin schon besser, subjektiv noch geschwächt, aber sonst beschwerdefrei.

THERAPIE:

Heimatkrankenhaus: 10.02.-23.02.09 Intensivstation, Diagnostik. 10.02.09 Pantozol, Voltaren, Oxygesic abgesetzt. 23.02.-27.02.09 Clexane 0.4 mg/d, Diagnostik.

VERLAUF:

Überweisungsdiagnose: extreme Hepatosplenomegalie und Lebersynthesestörung unklarer Genese. In Heimatkrankenhaus bereits autoimmune Hepatitis, primäre biliäre Zirrhose, frische virale Infektion (CMV, HSV, EBV, Adenovirus, Parvo-Virus, Paramyxovirus, Mumps-, Masern-, RSV, Coxsackie-Virus, HIV, Hepatitis A,B,C, etc) ausgeschlossen. 13.02.09 MEGX-Test (Leberfunktionstest) < 20 µg/ml --> stark erniedrigt. 20.02.09 MEGX-Test nach 15 min 81,5 µg/ml, 30 min 71,5 µg/ml --> immer noch stark erniedrigt. Leider sind aus dem Aufenthalt im Heimatkrankenhaus keine Leberwerte übermittelt. Temperatur 23.02. 37,6°C; 24.02. 36,8°C. 24.02.09 SLA-AAK, LKM-AAK negativ -->keine autoimmune Hepatitis, AMA-AAK negativ --> keine primäre biliäre Zirrhose, ANCA - Titer negativ --> keine primäre sklerosierende Cholangitis.

ERLÄUTERUNGEN ZUR UAW:

10.2. - 23.2 intensivmedizinische Betreuung in Heimat-KH wg fulminanter Hepatitis (dortige Diagnose). Schließlich bei Unsicherheit über den zukünftigen Verlauf und evtl Notwendigkeit einer Leber-Transplantation Verlegung zu uns. Hier bereits bei Vorstellung wesentlich besserer Zustand als erwartet mit weiterer Stabilisierung im Verlauf.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: PANTOZOL

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: OXYGESIC

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: VOLTAREN

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: Junge Patientin ohne Vorerkrankungen mit Z.n. Verkehrsunfall entwickelt nach Entlassung schwere Krankheitssymptome und Aszites. Im Verlauf kommt es im Heimatkrankenhaus zum V.a. akutes Leberversagen mit Synthesestörung und deshalb Verlegung an Uni-Klinikum zur Exploration Lebertransplantation. Die Kliniker sahen den Zusammenhang am ehesten medikamentös-toxisch mit der Einnahme von Diclofenac. Es müssen auch die anderen zeitgleich verordneten Medikamente (Oxygesic, Pantozol) verdächtigt werden. S 2, da virale oder OP-bedingte Ursache nicht ganz sicher auszuschließen ist. Die Leberfunktion erholte sich nach Absetzen der Medikation spontan.

MEDIKAMENT: CLEXANE

BETREFFENDE UAW: Leberschaedigung

ERLÄUTERUNG: lt Fachinfo wird ein Anstieg der Leberenzyme häufig beschrieben. In diesem Fall negatives Dechallenge, da trotz Weitergabe des Medikaments Rückbildung der hepatischen Dysfunktion. Zusammenhang unwahrscheinlich.

sanofi-aventis, 18.06.2010 DE-SANOFI-AVENTIS-2010SA023580

Sender diagnosis: not reported

Narrative

Initial Report:

This ICSR had been created by a regional pharmacovigilance centre (DE-PVZIBE-JENA-10072) and was extracted from the website of the German health authority (authority number DE-BFARM-09042861) on 23-Apr-2010

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

This serious case involves a 22-year-old female patient. On 21-Jan-2009 she experienced a road traffic accident with fracture of a lumbar vertebrae and a blunt abdominal trauma. Six days later a vertebral body replacement was performed. Since 07-Feb-2009 she suffered from intermittent increases in temperature, chill, abdominal pain and headache. Three days later the patient was hospitalised. From 10-Feb-2009 until 23-Feb-2009 she was in an intensive-care unit due to a fulminant hepatitis. Pantoprazole sodium, diclofenac potassium and oxycodone hydrochloride were withdrawn on 10-Feb-2009. On 11-Feb-2009 exploratory laparotomy was performed with suspicion of a staged splenic rupture at free fluid in the abdomen. Ascites was diagnosed. On 13-Feb-2009 a liver function test was performed with the following finding: monoethylglycine xylidide was greatly decreased (<20 µg/ml). The patient suffered from vomiting until 18-Feb-2009, urine was dark (brown). On 20-Feb-2009 a further liver function test was performed with the following finding: monoethylglycine xylidide was still greatly decreased with 81.5 µg/ml after 15 min and 71.5 µg/ml after 30 min.

The patient was transferred to university hospital for evaluation of a liver transplantation because of hepatic failure due to fulminant hepatitis. Referral diagnosis: extreme hepatosplenomegaly and hepatic synthesis disorder of unknown pathogenesis. On admission the patient was nearly free of symptoms, but subjectively weakened. Primary biliary cirrhosis, fresh viral infections, autoimmune hepatitis and primary sclerosing cholangitis were excluded. The patient was recovering.

The reporter assessed the causal relationship according to WHO score for pantoprazole sodium and oxycodone hydrochloride as "possible", for diclofenac potassium as "probable" and for enoxaparin sodium and ibuprofen as "unlikely".

Explanatory notes on causality:

For pantoprazole sodium, oxycodone hydrochloride and diclofenac potassium: Liver damage was most likely due to drug-related toxicity by the intake of diclofenac. Pantoprazole sodium and oxycodone hydrochloride were also suspected. In addition, viral or operation related causes could not be ruled out with certainty. The liver function spontaneously recovered after withdrawal of the suspected medication.

For enoxaparin sodium: Increase in liver enzymes is frequently described in the Physician's information leaflet. In this case, however, dechallenge was

negative (?). Despite continuation of treatment with the drug hepatic dysfunction regressed. Therefore, a causal relationship is unlikely.

Causality explanation concerning ibuprofen was not provided.

Additional Information received on 07-Jun-2010:

We received additional information from the regional pharmacovigilance centre via the German Health Authority on 07-Jun-2010.

The ibuprofen drug was not further suspected to cause the liver damage and was reclassified to concomitant drug. Therefore, no causality assessment was necessary for ibuprofen.

The causal relationship to enoxaparin sodium was assessed as unlikely.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09054567

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19621113	Age: 46 Year	weight (kg): 65	BMI_int: 21.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pruritus	20090413				not recovered/not resolved
2 Icterus	20090413				not recovered/not resolved
3 Decreased activity	20090413				not recovered/not resolved
4 Hepatitis	20090413			15 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprofloxacin	Oral	500 Mg milligram(s)/Day	20090330-20090405	Injury to finger NOS
ciprofloxacin		90	7 Day	
2 ibuprofen	Oral	600 Mg milligram(s)/Day	20090330-20090405	Injury to finger NOS
ibuprofen		245	7 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		6.7	mg/dl	
Glutamic-oxaloacetic transferase		1331	U/l	
GPT		1973	U/l	
Alkaline phosphatase		474	U/l	
GGT		407	U/l	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 08.05.2009 DE-NOVNCHHQ-GXKR2009DE05277

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 29 Apr 2009 from the German Health Authority (DE-BfArM-09054567), provided by the Drug Commission of the German medical Association, reported by a physician: This patient with an unknown medical history developed hepatitis during treatment with ciprofloxacin and ibuprofen (MAH unknown). No information about concomitant medication was provided. The outcome of the adverse event was reported as not recovered. Causality was assessed as possible for both drugs.

Serious spontaneous report (medically significant) of hepatitis, already listed in the Core Data Sheet Information.
All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Bayer Vital GmbH, 08.05.2009 DE-BAYER-200919948GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 29-Apr-2009 by BFARM, the Regulatory Authority of Germany, referring to a 46 year-old male.

It is unknown whether any concomitant drugs have been given.

Patient received Ciprofloxacin (ciprofloxacin; 500 mg daily) and Ibuprofen (ibuprofen; 600 mg daily) for injury to finger (nos). Administrations began on 30-Mar-2009, for a duration of 7 days.

On 13-Apr-2009, 14 days after starting treatment with Ciprofloxacin and Ibuprofen patient experienced HEPATITIS considered serious due to important medical event. Patient had not recovered at time of report.

For the drugs Ciprofloxacin and Ibuprofen the reporter considered the diagnosis HEPATITIS to be possibly related.

reporter's comment:

SANOFI-SYNTHELABO, 15.05.2009 DE-SANOFI-SYNTHELABO-A04200900602

Sender diagnosis: not reported

Narrative

Reaction as reported: hepatitis (bilirubin 6.7 mg/dl, GOT 1331 U/l, GPT 1973 U/l, Alkaline phosphatase 474 U/l, gamma-GT 407 U/l)

LLT: HEPATITIS / 10019717

PT: HEPATITIS / 10019717

Narrative:

Initial report: This case was extracted from the website of the German health authority (authority number DE-BFARM-09054567); it was reported by a physician via the Drug Commission of the German Medical Association (DE-DCGMA-09149751).

The case involves a 46-year-old male patient who had been treated with ibuprofen (no trade name provided) 1x600mg/day and ciprofloxacin 1x500mg/day from 30-Mar-2009 to 05-Apr-2009 for injury to finger (NOS). On 13-Apr-2009 the patient was diagnosed to suffer from hepatitis. The outcome was reported as "not recovered/not resolved". Further information was not provided.

Causality assessment according to WHO Global Introspection for Drug Commission of the German Medical Association: possible for ibuprofen and ciprofloxacin. The event was considered medically important either by the German health authority or by the Drug Commission of the German Medical Association.

SANOFI-SYNTHELABO, 10.06.2009 DE-SANOFI-SYNTHELABO-A04200900602

Sender diagnosis: not reported

Narrative

Reaction as reported: hepatitis (bilirubin 6.7 mg/dl, GOT 1331 U/l, GPT 1973 U/l, Alkaline phosphatase 474 U/l, gamma-GT 407 U/l)

LLT: HEPATITIS / 10019717

PT: HEPATITIS / 10019717

Reaction as reported: icterus

LLT: ICTERUS / 10021207

PT: JAUNDICE / 10023126

Reaction as reported: pruritus

LLT: PRURITUS / 10037087

PT: PRURITUS / 10037087

Reaction as reported: decreased activity

LLT: DECREASED ACTIVITY / 10011953

PT: DECREASED ACTIVITY / 10011953

Narrative:

Initial report: This case was extracted from the website of the German health authority (authority number DE-BFARM-09054567); it was reported by a physician via the Drug Commission of the German Medical Association (DE-DCGMA-09149751).

The case involves a 46-year-old male patient who had been treated with ibuprofen (no trade name provided) 1x600mg/day and ciprofloxacin 1x500mg/day from 30-Mar-2009 to 05-Apr-2009 for injury to finger (NOS). On 13-Apr-2009 the patient was diagnosed to suffer from hepatitis. The outcome was reported as "not recovered/not resolved". Further information was not provided.

Causality assessment according to WHO Global introspection for Drug Commission of the German Medical Association: possible for ibuprofen and ciprofloxacin. The event was considered medically important either by the German health authority or by the Drug Commission of the German Medical Association.

Additional information received on 02-Jun-2009 concerning the summarised hospital report.

Following symptoms for hepatitis were listed: icterus, pruritus and decreased activity. The symptomatic treatment for pruritus included colestyramine.

No further information expected.

Bayer Vital GmbH, 02.07.2009 DE-BAYER-200919948GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 29-Apr-2009 by BFARM, the Regulatory Authority of Germany, referring to a 46 year-old male.

It is unknown whether any concomitant drugs have been given.

Patient received Ciprofloxacin (ciprofloxacin; 500 mg daily) and Ibuprofen (ibuprofen; 600 mg daily) for injury to finger (nos). Administrations began on 30-Mar-2009, for a duration of 7 days.

On 13-Apr-2009, 14 days after starting treatment with Ciprofloxacin and Ibuprofen patient experienced HEPATITIS considered serious due to important medical event. Patient had not recovered at time of report.

For the drugs Ciprofloxacin and Ibuprofen the reporter considered the diagnosis HEPATITIS to be possibly related.

****Follow up information received on 22-Jun-2009:**

Patient was treated with ciprofloxacin and ibuprofen after injury of finger from 30-Mar to 5-Apr-2009. Since 13-Apr-2009 patient suffered from HEPATITIS with icterus, pruritus and lack of performance.

Liver values were initially rising, but declined in further clinical course.

Symptomatic therapy: with Quantalan/colestyramin was given due to pruritus; no other therapy

Further details: Visit abroad, patient had recently been in turkey for one week.

No contact with animals (sheep, cows, rats).

He was not treated with ciprofloxacin before, no further underlying diseases.

Bayer Vital GmbH, 29.10.2009 DE-BAYER-200919948GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 29-Apr-2009 by BFARM, the Regulatory Authority of Germany, referring to a 46 year-old male.

It is unknown whether any concomitant drugs have been given.

Patient received Ciprofloxacin (ciprofloxacin; 500 mg daily) and Ibuprofen (ibuprofen; 600 mg daily) for injury to finger (nos). Administrations began on 30-Mar-2009, for a duration of 7 days.

On 13-Apr-2009, 14 days after starting treatment with Ciprofloxacin and Ibuprofen patient experienced HEPATITIS considered serious due to important medical event. Patient had not recovered at time of report.

For the drugs Ciprofloxacin and Ibuprofen the reporter considered the diagnosis HEPATITIS to be possibly related.

****Follow up information received on 22-Jun-2009:**

Patient was treated with ciprofloxacin and ibuprofen after injury of finger from 30-Mar to 5-Apr-2009. Since 13-Apr-2009 patient suffered from HEPATITIS with icterus, pruritus and lack of performance.

Liver values were initially rising, but declined in further clinical course.

Symptomatic therapy: with Quantalan/colestyramin was given due to pruritus; no other therapy

Further details: Visit abroad, patient had recently been in turkey for one week.

No contact with animals (sheep, cows, rats).

He was not treated with ciprofloxacin before, no further underlying diseases.

Follow-up received on 26-OCT-2009:

Patient condition has improved and he has been discharged with decreasing hepatic parameters. This has not occurred so far.
No further information available.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-09061587

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 46 Year	weight (kg): 65	BMI_int: 21.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis	20090413			14 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprofloxacin ciprofloxacin	Oral	/	20090330-20090405 7 Day	Injury to finger NOS
2 ibuprofen ibuprofen	Oral	/	20090330-20090405 7 Day	Injury to finger NOS

Primary source

Qualification: Other Health Professional

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		6.7	mg/dl	
Glutamic-oxaloacetic transferase		1331	U/l	
GPT		1973	U/l	
Alkaline phosphatase		474	U/l	
GGT		407	U/l	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 08.05.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-03552DE

Sender diagnosis: not reported

Narrative

This is a spontaneous report (health professional confirmed) from the German Health Authority.

On 04May2009 information has been received concerning a 46-year-old male patient who hepatitis on 13Apr2009 after therapy with daily 600 mg ibuprofen and 500 mg ciprofloxacin for the treatment of injury to finger NOS from 30Mar2009 to 05Apr2009. Both drugs were withdrawn. The patient has not yet recovered from the event.

Laboratory values were:

Bilirubin	6.7	mg/dl
GPT	1331	U/l
GPT	1973	U/l
Alkaline phosphatase	474	U/l
GGT	407	U/l

Information on the treatment of the event, on co-diseases and co-medication was not provided. The physician rated the causal relationship between the event and the treatment with ibuprofen and ciprofloxacin as possible and the event as not to be life-threatening.

Tracking:

04May2009: initial information via BfArM Web Tool

reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09063366

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19500807	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Thrombocytopenia	200905				recovered/resolved
2 Glucose tolerance decreased	20090511				recovered/resolved
3 Steatosis hepatic					recovered/resolved
4 Type II diabetes mellitus					Unknown
5 Blood sugar increased	200905				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUPROFEN UNKNOWN MANUFACTURER ibuprofen		/	2007- -	
2 VOLTAFLEX GLUCOSAMINE glucosamine hydrochloride	Oral	/ 82	2007-2009 2 Year	Joint pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pollakiuria			Unknown	
Infection			Unknown	Unspecified infection at the beginning of 2009
Arthrosis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Blood glucose	200905	20	mmol/l	
HbA1C		10.2	NA	
Thrombocyte count		43	NA	
Thrombocyte count	200905	41	NA	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 18.05.2009 DE-NOVNCHHQ-CHNY2009DE02010

Sender diagnosis: not reported

Narrative

This physician report combines initial and follow-up information both received on 13 May 2009. This male patient was diagnosed with new elevated blood sugar levels (exact value unknown) and thrombocytopenia prior to pre-planned arthroscopic diagnostics and was admitted to internist department of hospital for further diagnostics with regard to elevated blood sugar levels and thrombocytopenia. The clinician reported that the patient was under regular treatment with Voltaflex (glucosamine hydrochloride) 750 mg and unspecified Ibuprofen compound (probably Ibu-Hexal) to treat joint pain since approximately 2008. The therapy with Voltaflex and Ibuprofen-containing compound was still ongoing at date of report. Further diagnostics planned with regard to elevated blood sugar levels and thrombocytopenia. Outcome was not reported. Follow-up information was requested.

glucose increased and thrombocytopenia were assessed as unlabeled according to the product information. The available information was considered inadequate to fully assess the case.

For Ibuprofen unknown manufacturer:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (hospitalization). The event Blood glucose increased was assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The event Thrombocytopenia was assessed as listed according to the product information.

Reporter's comment:

NOVARTIS, 07.07.2009 DE-NOVNCHHQ-CHNY2009DE02010

Sender diagnosis: not reported

Narrative

This physician report combines initial and follow-up information both received on 13 May 2009. This male patient was diagnosed with new elevated blood sugar levels (exact value unknown) and thrombocytopenia prior to pre-planned arthroscopic diagnostics and was admitted to internist department of hospital for further diagnostics with regard to elevated blood sugar levels and thrombocytopenia. The clinician reported that the patient was under regular treatment with Voltalex (glucosamine hydrochloride) 750 mg and unspecified Ibuprofen compound (probably Ibu-Hexal) to treat joint pain since approximately 2008. The therapy with Voltalex and Ibuprofen-containing compound was still ongoing at date of report. Further diagnostics planned with regard to elevated blood sugar levels and thrombocytopenia. Outcome was not reported. Follow-up information was requested.

Follow-up report received from physician on 30 Jun 2009. Voltalex Glucosaminhydrochlorid treatment was started in 2007 and ibuprofen was started since 2007. The physician specified that the patient developed reduced glucose tolerance diagnosed on 11 May 2009. Following drug treatment with insulin (unknown manufacturer), adverse symptoms lasted all together for several months. Final outcome was complete recovery. The reporting physician assessed the causality as possible.

tolerance decreased and related symptom were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The event Thrombocytopenia was assessed as listed according to the product information.

Reporter's comment:

NOVARTIS, 10.08.2009 DE-NOVNCHHQ-CHNY2009DE02010

Sender diagnosis: not reported

Narrative

This physician report combines initial and follow-up information both received on 13 May 2009. This male patient was diagnosed with new elevated blood sugar levels (exact value unknown) and thrombocytopenia prior to pre-planned arthroscopic diagnostics and was admitted to internist department of hospital for further diagnostics with regard to elevated blood sugar levels and thrombocytopenia. The clinician reported that the patient was under regular treatment with Voltalex (glucosamine hydrochloride) 750 mg and unspecified Ibuprofen compound (probably Ibu-Hexal) to treat joint pain since approximately 2008. The therapy with Voltalex and Ibuprofen-containing compound was still ongoing at date of report. Further diagnostics planned with regard to elevated blood sugar levels and thrombocytopenia. Outcome was not reported. Follow-up information was requested.

Follow-up report received from physician on 30 Jun 2009. Voltalex Glucosaminhydrochlorid treatment was started in 2007 and ibuprofen was started since 2007. The physician specified that the patient developed reduced glucose tolerance diagnosed on 11 May 2009. Following drug treatment with insulin (unknown manufacturer), adverse symptoms lasted all together for several months. Final outcome was complete recovery. The reporting physician assessed the causality as possible.

Internal review received on 04 Aug 2009 of the data received on 30 Jun 2009: A clinical report was also included in the initial report. The patient was hospitalized from 11 May 2009 to 20 May 2009. The patient was diagnosed with elevated blood sugar level with approximately 20 mmol/l and thrombocytopenia with 41 Gpt/l prior to pre-planned elective arthroscopic diagnostics in May 2009. The patient was newly diagnosed with type II diabetes mellitus, tentative diagnosis of Morbus Werlhof ED and tentative diagnosis of internal knee damage. Lab test revealed HbA1c at 10.2, normal folic acid and vitamin B12, last thrombocyte value at 43 (maximum value of 87 after intravenous prednisolone injection and minimum value of 23) and no antibodies against thrombocytes were found. On 20 May 2009, bone marrow punch biopsy was performed and normal bone marrow lead to tentative diagnosis of loss of thrombocytes. Besides steatosis hepatis diagnosed on 13 May 2009, all other diagnostics revealed no abnormal findings. The clinicians stated that is had to be discussed if in predisposed patients the reduced glucose tolerance might be caused due to intake of

glucosamine preparations. Final outcome was complete recovery. The reporting physician assessed the causality as possibly related to Glucosaminhydrochlorid.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09083682

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19650812	Age: 43 Year	weight (kg): 83	BMI_int: 25.9	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	200904				Unknown
2 Hepatitis					Unknown
3 Hepatitis toxic drug-induced	20090519				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		400 Mg milligram(s)/Hour	-	Neck pain
ibuprofen			-	
2 SIMVASTATIN	Oral	40 Mg milligram(s)/Day	-	Hyperlipemia
simvastatin			-	
3 Tramal		/	-	Neck pain
tramadol			-	
4 Trancolong	Oral	200 Mg milligram(s)/Day	20090414-	Neck pain
flupirtine maleate		212	-	

Drugs (concomitant)

5 pilocarpine	Intraocular	/	-	Glaucoma
pilocarpine		71	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20090519	213	U/L	
Alkaline phosphatase	20090522	219	U/L	
Bilirubin total	20090519	7.3	mg/dl	
Bilirubin total	20090522	11.4	mg/dl	
Blood iron	20090520	56.1	Î¼mol/L	
Blood triglycerides	20090519	216	mg/dl	
Cholesterol	20090519	132	mg/dl	
Gamma GT	20090519	422	U/L	
Gamma GT	20090522	335	U/L	
Gamma GT	20090630	124	U/L	
GPT	20090519	2410	U/L	
GPT	20090520	2222	U/L	
GPT	20090630	52	U/L	
HDL	20090519	6	mg/dl	
Plasma glucose	20090519	120	mg/dl	
Plasma glucose	20090522	126	mg/dl	

	DE-BFARM-09083682		
Serum ferritin	20090520	1462	Î¼g/L
Serum immunoglobulin G	20090520	22.2	g/L
Serum immunoglobulin M	20090520	2.98	g/L
SGOT	20090630	35	U/L
SGOT	20090519	997	U/L
Lipase	20090519	64	U/L
Transferrin saturation	20090520	68	%

Narratives (nur bei elektronischem Eingang)

Dr. Kade Pharmazeutische Fabrik GmbH, 01.07.2009 DE-DRKADE-TrnCL-09-04

Sender diagnosis: not reported

Narrative

Spontaneous serious report. This case was received by Dr. Kade on 23.06.2009

A sales representative forwarded a physician's report on the case of a male patient (no further information on patient provided) who experienced increase of transaminases during treatment with Trancolong in April 2009. The physician claimed that laboratory tests prior to Trancolong treatment (end of March 2009) showed liver parameters within the normal range.

The patient was admitted to hospital where a hepatitis had been diagnosed.

This case was considered serious based on the following criteria: hepatitis, hospitalization required.

Dr. Kade Pharmazeutische Fabrik GmbH, 21.08.2009 DE-DRKADE-TrnCL-09-04

Sender diagnosis: not reported

Narrative

Spontaneous serious report.

This case was received by Dr. Kade on 23.06.2009

A sales representative forwarded a physician's report on the case of a male patient (no further information on patient provided) who experienced increase of transaminases during treatment with Trancolong in April 2009. The physician claimed that laboratory tests prior to Trancolong treatment (end of March 2009) showed liver parameters within the normal range.

The patient was admitted to hospital where a hepatitis had been diagnosed.

This case was considered serious based on the following criteria: hepatitis, hospitalization required.

At the time of this report the case is not assessable due to insufficient information. Follow-up information has been requested.

17-Aug-2009:

Upon request the reporting physician provided additional information on the patient (initials: LR, date of birth: 12.08.65, height: 179 cm, weight: 83 kg, ethnic origin: Caucasian) and the event. The hospital reports of diagnostic findings and the discharge letter were provided. Summary of relevant information:

The patient had been admitted to hospital on 19.05.2009 with the diagnosis of a non-distinctive hepatopathy. He indicated feeling sickish for approximately 2 months and reported that he experienced nausea and unspecific upper abdominal pain since about 1 week. During the last days before admission to hospital dark urine and icterus had developed. According to the hospital admission report dated 20.05.09 patient's medication comprised ibuprofen and tramadol (Tramal) which were taken if needed for the treatment of neck pain. By contrast, in the discharge letter dated 08.06.2009 flupirtine (no brand name given, Dr. Kade product cannot be excluded) is stated as second analgesic besides ibuprofen. In the admission report additionally simvastatin at a dose of 40 mg (once in the evening for a period of 3 to 4 years) and Pilocarpin eye drops are noted as concomitant medication. Because of suspicion of a toxic hepatopathy all drugs were discontinued upon admission to hospital with the exemption of Pilocarpin eye drops for the treatment of glaucoma.

The patient presented with a distinct icterus of skin and sclera. Laboratory tests upon admission revealed increased transaminase values up to 2400 U/l indicating an acute hepatitis. Due to findings of increased Gammaglobuline and increased IgG an auto-immune hepatitis was considered as differential diagnosis to a toxic drug induced hepatitis. Sonography of the abdomen did not reveal conspicuous clinical findings. For further diagnosis a liver biopsy had been performed which presented a finding that most likely resembled a toxic drug-induced hepatic damage (intra-lobular single cell necrosis with mixed inflammatory infiltrate and minor cholestasis, inflammation activity score 2, fibrosis stage 0, inflammatory infiltrate relatively weakly pronounced). In the discharge letter "acute drug induced hepatitis, e.g. by flupirtine" is stated.

Follow-up laboratory tests showed significantly decreasing values of transaminases and bilirubin. The patient was discharged home on 04.06.2009 (hospitalization periods: 19.05.-20.05., 25.05.-26.05., 03.06.-04.06.) Regular controls of the liver parameters were recommended in the discharge letter.

According to the information of the reporting physician on 17.08.09 the patient's medication comprised Trancolong (400 mg flupirtine maleate / prolonged release tablet, 189; tablet if required, start of treatment 14.04.09) Ibuprofen 400 (1 tablet, if required), Simvastatin 40 (1 x 1 tablet daily). He stated that Trancolong had been taken since 14.04., no further information with respect of treatment duration had been provided for simvastatin 40 mg and ibuprofen 400 mg. The physician provided the last values of the laboratory results for GOT, GPT and γ -GT (Test date: 30.06, values nearly within normal range). He stated that the patient was in good health again. No causality assessment was provided by the reporting physician.

Outcome is recovered / resolved

Additional information:

Test date 25.05.09: antinuclear antibodies: negative; antimitochondrial antibodies: negative; Hepatitis A Virus IgG antibodies: positive; Hepatitis A Virus IgM antibodies: negative (Condition after Hepatitis A infection or active immunization against Hepatitis A)
Hepatitis B virus surface antigen: negative; Hepatitis B virus surface antibodies: 191 U/L; Hepatitis B virus core antibodies: negative. Isolated Anti-HBs. This finding may indicate a previous hepatitis B infection which had developed a long time ago or an immunization against hepatitis B. Can also be transferred with blood transfusions
Hepatitis C virus antibodies: negative

Original hospital reports of Asklepios Klinik Barmbek, Hamburg (all first and last names depersonalized by using initials):

Report dated 20-May-2009:

Patient: L. R., 12.08.1965
Stationärer Aufenthalt: 19.05. - 20.05.2009

Sehr geehrte Frau Kollegin, sehr geehrter Herr Kollege,

wir berichten Ihnen abschliessend ueber obige(n) Patientin/en

Diagnosen:
1. unklare Hepatopathie

Anamnese:
Seit 1 Woche vor Aufnahme Uebelkeit, Brennen der Augen; im Verlauf Erbrechen, Ikterus, Dunkelfaerbung des Urins, rechtsseitiges Oberbauchdruckgefuehl. Die haeusliche Medikation umfasste Ibuprofen und Tramal bei Bedarf (Nackenschmerzen), Simvastatin seit 3-4 Jahren 40 mg abends (alles jetzt pausiert) und Pilacarpin-Augentropfen.

Befund bei Aufnahme:
179 cm, 83 kg, Gewichtsabnahme von 2 kg in der vergangenen Woche, Sklerenikterus, RR 147/95 mm Hg, Puls regelmaessig, 79 bpm, Leber 2 cm unter dem Rippenbogen tastbar, induriert, AZ und EZ gut, sonstiger koerperlicher Befund nicht wegweisend.

Labor: (s. u.):
Befunde wie die Autoimmunhepatitisserologie stehen noch aus.

Sonographie:
Keine intra- oder extrahepatische Erweiterung der Gallenwege, Gallenblase mit verdickter Wand, keine hepatischen Herdbefunde.
Verlauf:
Die Genese der Hepatopathie ist unklar, Teile der Labordiagnostik stehen noch aus. Wir entlassen den Patienten bei gutem AZ und vereinbarten eine Wiedervorstellung in 2 Tagen zur Leberwertkontrolle in der ZNA (bei akuter Verschlechterung frueher). Sobald alle Laborwerte vorliegen, ist eine Vorstellung in der Lebersprechstunde unserer gastroenterologischen Abteilung geplant.

Medikation:
Augentropfen weiter, uebrige Medikation pausiert wegen der Moeglichkeit des Vorliegens einer toxischen Hepathopathie.

Mit freundlichen kollegialen Gruessen

U. T.
Chefarzt

Report dated 22-May-2009:

€R., L. *12.08.1965 (43 Jahre)

Sehr geehrte Frau Kollegin, sehr geehrter Herr Kollege,

wir berichten Ihnen ueber Ihren Patienten, der sich heute in unserer ambulanten Krankenhausbehandlung befand.

Diagnosen

K76.9 Leberkrankheit, nicht naeher bezeichnet

Vorgeschichte / Befund

Bis vor 2 Tagen stationaer auf Station IMC A mit unklarer Hepatopathie, zu dem Zeitpunkt unauffaellige Sonographie, Laborparameter noch nicht vollstaendig, aktuell wie vereinbart WV zur Kontrolle der Laborparameter. Patient fuehlt sich schlapp, keinen Appetit, unveraendert Ikterus, wach, voll orientiert.

Diagnostik

AP: 219 U/L (Normalwert: 40-130), Bilirubin gesamt: 11,4 mg/dl (Normalwert: <1.2), Calcium: 2.25 mmol/l (Normalwert: 2.15-2.58), CK: 55 U/l (Normalwert: <190), CRP: 2.5 mg/l (Normalwert: <5.0), Erythrozyten: 5.1 /pl (Normalwert: 4.5-5.9), GFR (MDRD abbr.): >90 ml/min (Normalwert:>60), j3-GT: 335 U/l (Normalwert: <60), Glucose (Hep.-Plasma): 126 mg/dl (74-109), ALAT (GPT): 2222 U/l (Normalwert:<50), Hb: 15.5 g/dl (Normalwert:13.5-17.5), Hk: 0.45 l/l (Normalwert: 0.40-0.53), INR: 1.06 l/l, Kalium: 4.04 mmol/l (Normalwert: 3.40-4.50), Kreatinin: 0.7 mg/dl (Normalwert: 0.7-1.2), Leukozyten: 4.9 /nl (Normalwert: 3.5-9.8), Lipase: 53 U/l (Normalwert:<60), MCH (HbE): 30.5 pg (Normalwert: 28.0-33.0), MCHC: 34.1 g/dl (Normalwert: 33.0-36.0), MCV: 89 fl (Normalwert: 80-96), Natrium: 136 mmol/l (Normalwert: 135-145), PTT: 36.6 Sekunden (Normalwert: 28.0-40.0), Quick: 90 % (Normalwert: 70-130), RDW (EVB): 16.6 % (Normalwert:11-16), Thrombozyten: 214 /nl (Normalwert: 140-360). CHE und Albumin steht noch aus, PCR auf Hepatitis C, Coeruloplasmin nachgefordert, Patient wird Urin sammeln auf Kupfer

Behandlungsverlauf / Empfehlung

Morgen WV bei Dr. B. zur Kontrolle der Lebersyntheseparameter, bei klinischer Verschlechterung sofort WV. Moeglicherweise wird in der folgenden Woche eine Leberpunktion notwendig sein, vorerst keine Medikation.

Dr. A. G.
Oberarzt

Report dated 08-June-2009 (discharge letter):

€Patient: R. L., geboren am 12.08.1965
wohnhaft: xxxxx-street Hamburg

Sehr geehrte Frau Kollegin, sehr geehrter
Herr Kollege,

wir berichten ueber Ihren o. g. Patienten, der sich vom 25.05. bis zum 26.05. und vom 03.06. bis zum 04.06.09 in unserer stationaeren Behandlung befand.

Diagnosen:

1. Akute medikamentös toxische Hepatitis, z. B. unter Flupirtin
2. Glaukom
3. Hyperlipidaemie

Anamnese:

Herr R. berichtet bei Aufnahme, dass er sich seit ca. zwei Monaten nicht mehr richtig wohl fuehle, er sei nicht mehr so belastbar wie sonst und habe zudem seit anderthalb Wochen Oberbauchschmerzen und Uebelkeit. In den letzten Tagen habe er dann dunklen Urin und einen Ikterus entwickelt.

Aufnahmebefund:

Wacher und orientierter Patient in leicht reduziertem Allgemein- und schlankem Ernaehrungszustand, deutlicher Haut- und Slerenikterus, Pulmo bds. belueftet, vesikulaeres Atemgeraesch, sonorer Klopfschall, Cor: Herzaktion rein und rhythmisch, Abdomen: weich, regelrechte Darmgerauesche, leichter Druckschmerz im rechten Oberbauch, keine Abwehrspannung; kein Klopfschmerz ueber den Nierenlagern oder der Wirbelsaeule.

Labor:
In Kopie anbei

Abdominelle Sonographie:

Biopsie in typischer Weise nach Menghini durchgefuehrt; Zylinder zusammenhaengend, knapp ueber 1 cm lang; nach Biographie keine sonographische Auffaelligkeit im Sinne einer Komplikation.

Histologie:

Leberstanzylinder mit kleinen zentralen Kollapsfeldern und intralobulaeren Einzelzellnekrosen bei gemischtem Entzuendungsinfiltrat und geringer Cholestase. Entzuendungsaktivitaetsgrad 2, Fibrorestadium 0. Das histolgoische Bild spricht fuer eine (medikamentös-)toxische Leberschaedigung. Das Entzuendungsinfiltrat ist insgesamt relativ gering ausgepraegt.

Abdominelle Sonographie:

Leber unauffaellig, kein intra-extrahepatischer Aufstau. GB mit verdickter Wand. Pankreas, Nieren und Milz unauffaellig. zB Hepatitis.

Therapie und Verlauf:

Die Aufnahme des Patienten erfolgte bei Unwohlsein, Ikterus und unspezifischen Oberbauchschmerzen. Bereits im Aufnahmelaor imponierte das Bild einer akuten Hepatitis mit Transaminasen bis in 2400 U/l. Herr R. berichtet, dass er wegen Nackenschmerzen ueber laengere Zeit Ibuprofen und Flupirtin eingenommen habe.

Die Hepatitisserologie blieb unauffaellig, in den ausfuehrlichen laborchemischen Untersuchungen zeigten sich erhöhte Î³-Globuline sowie eine IgG-Fraktion, so dass neben einer medikamentös-toxischen Hepatitis die Differentialdiagnose Autoimmunhepatitis besteht. Zur weiteren Diagnosesicherung fuehrten wir nun eine Biopsie der Leber durch, hier zeigte sich das Bild einer am ehesten medikamentös-toxischen Leberschaedigung. In mehreren Verlaufskontrollen waren die Transaminasen sowie das Bilirubin deutlich ruecklaeufig, die Syntheseparameter CHE, Albumin sowie Quick waren nie beeintraehtigt. Wir bitten um regelmaessige Laborkontrollen der Leberparameter.

Am 04.06.09 entlassen wir den Patienten wieder in Ihre ambulante Behandlung und stehen Ihnen bei Rueckfragen gerne zur Verfuegung.

Medikation:

Augentropfen bei Glaukom

Mit freundlichem Gruss

PD Dr. S. F. Dr. N. B. Dr. C. D.
 Chefarzt Oberarzt Stationsaerztin
 Internist/Gastroenterologe Internistö

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09090323

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials: Sex: Male Birth: 19400222 Age: 69 Year weight (kg): 84 BMI_int: 27.12 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pancreatitis	20090227	20090304	6 Day	2 Day	recovered/resolved
2 Abdominal pain NOS	20090301			5 Day	recovering/resolving
3 ALT increased					Unknown
4 Amylase increased	20090302			6 Day	Unknown
5 AST increased	20090302			6 Day	Unknown
6 Epigastric pain	20090227			3 Day	recovering/resolving
7 Lipase increased	20090302			6 Day	Unknown
8 Abdominal pain					Unknown
9 Edematous pancreatitis	20090227	20090304	6 Day	6 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 buprenorphine buprenorphine	Transdermal	/	20090224-20090302 7 Day	Postoperative pain
2 IBUPROFEN ibuprofen	Oral	600 Mg milligram(s)/Day 82	2007-20090220 -	Coxarthrosis
3 OMEPR omeprazole	Oral	40 Mg milligram(s)/Day 245	20090225- -	Gastric ulcer prophylaxis
4 PARACETAMOL paracetamol	Oral	1500 Mg milligram(s)/Day 245	20090226-20090227 2 Day	Postoperative pain
5 RAMIPRIL ramipril	Oral	2* /Day 245	2000- -	Hypertension arterial
6 VOLTAREN RESINAT diclofenac resinate	Oral	150 Mg milligram(s)/Day 6	20090225-20090227 3 Day	Postoperative pain

Drugs (concomitant)

7 BERLOSIN metamizole	Oral	1500 Mg milligram(s)/Day 245	20090226-20090227 2 Day	Postoperative pain
8 CEFUROXIME cefuroxime	Intravenous drip	1.5 Mg milligram(s)/Day 293	20090224-20090224 1 Day	Infection prophylaxis
9 FLOTRIN terazosin hydrochloride	Oral	2 Mg milligram(s)/Day 245	2005- -	Benign prostatic hyperplasia
10 norspan buprenorphine	Transdermal	/Day 250	20090224-20090302 7 Day	Postoperative pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Total hip replacement	20090224		No	
Prosthesis implantation	200807		Unknown	
Total hip replacement	200807		No	
Alcohol abuse, unspecified drinking behavior			Yes	
Hypertension arterial	1990		Yes	
Benign prostatic hyperplasia	200807		Yes	Beginn unbekannt
Pancreatitis	200807	200807	No	
Coxarthrosis			Yes	Beginn unbekannt, TEP links 07/2008, rechts 24.02.2009
Prosthesis implantation	20090224		Unknown	
Ex-smoker	1980		Yes	1955-1980 1 Packung/Woche
Alcohol use			Yes	Beginn unbekannt, 3 Bier+1 Flasche Rotwein in der Woche

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20090302	89	µmol/l	
Creatinine clearance	20090302	>1.0	ml/s	
Aspartate aminotransferase	20090223	26.0	U/l	
Aspartate aminotransferase	20090302	52.4	U/l	
Alanine aminotransferase	20090223	21.0	U/l	
Alanine aminotransferase	20090302	55.1	U/l	
Alkaline phosphatase	20090223	57.0	U/l	
Gamma GT	20090223	14.0	U/l	
Gamma GT	20090302	32.0	U/l	
C-reactive protein	20090302	24.9	mg/l	
Amylase	20090223	57.1	U/l	
Amylase	20090302	1495	U/l	
Lipase	20090302	1429	U/l	
Potassium	20090302	4.1	mmol/l	
Sodium	20090302	138	mmol/l	
Alanine aminotransferase	20090223	21	NA	
Alanine aminotransferase	20090302	55.1	NA	
Amylase	20090223	57.1	NA	
Amylase	20090302	1495	NA	
Aspartate aminotransferase	20090223	26	NA	
Aspartate aminotransferase	20090302	52.4	NA	
C-reactive protein	20090302	24.9	mg/l	
Lipase	20090302	1429	NA	
Ultrasonography	20090223	nicht vorhanden	NA	absent

Ultrasonography	20090302	nicht vorhanden	absent
Calcium	20090223	2.31	mmol/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal / Universität Witten, 14.07.2009 DE-PVZIBE-02-10154

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Pankreatitis

KURZZUSAMMENFASSUNG:

akute Pankreatitis infolge von Diclofenac (Paracetamol)

BESCHREIBUNG:

seit 27.02.2009 leichte epigastrale Schmerzen;

ab 01.03.2009 heftig stechende Bauchschmerzen, in den Unterbauch ausstrahlend, Zustand nach Hüft-TEP-OP am 24.02.2009;

bei Aufnahme 12-fach erhöhter Lipase-Wert;

anamnestisch: 07/2008 identisches Ereignis nach 1. Hüft-TEP-OP (Arzneimittelanamnese nicht bekannt, OP war in Sachsen)

THERAPIE:

initiale Nahrungskarenz, Flüssigkeitssubstitution (Sterofundin);

Schmerzmittel abgesetzt;

Würzburger Perfusor oder Haldol;

Clexane 40mg s.c.

VERLAUF:

kein typisches Vorliegen von Risikofaktoren für eine Pankreatitis;

schnelle Besserung der Symptomatik (keine weitere Laborkontrolle erfolgt);

02.03.2009 Sonographie Abdomen: ödematöse Pankreatitis, Pankreasgang nicht erweitert, Splenoportales Gefäßsystem unauffällig, Leber, Gallenblase und Gallenwege unauffällig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 02.-04.03.2009;

Einweisungsgrund: akute Pankreatitis, Zustand nach Totaler Endoprothese des Hüftgelenks am 24.02.2009;

Im zeitlichen Verlauf wären Diclofenac, Paracetamol, Buprenorphin und Cefuroxim verdächtig. Für NSAR sind Pankreatiden beschrieben. Für Paracetamol, Buprenorphin und Cefuroxim gibt es Hinweise in der Literatur.

Nach der ersten Hüft-TEP im Juli 2008 trat ebenfalls eine Pankreatitis auf. Die damalige Arzneimittelanamnese ist leider nicht ermittelbar. Als Alternativursache für die Pankreatitis könnte eine Reaktion auf die Narkose diskutiert werden, ebenso auf den chirurgischen Eingriff. Allerdings sind bisher in der Literatur nur abdominalchirurgische Eingriffe mit Pankreatitiden assoziiert worden. Die Gallenwege waren sonographisch unauffällig. Gegen akuten Alkoholkonsum als Ursache spricht die normalwertige Gamma-GT. Triglyzeride wurden nicht bestimmt.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:**MEDIKAMENT: RAMIPRIL**

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist in der Fachinfo als sehr selten beschrieben. Medikament wird schon seit mehreren Jahren eingenommen und wurde nicht abgesetzt. Trotz Weitergabe erfolgte eine Symptombesserung, daher wird Ramipril hier nicht verdächtig.

MEDIKAMENT: NORSPAN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Nur geringe bis gar kein Einfluss auf Sphincter Oddi (Staritz et al, 1986, Effect of modern analgesic on the bile duct sphincter in man. Gut 27:567);

In der Fachinfo sind keine Pankreatiden beschrieben, aber abdominale Schmerzen.

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Der Arzneimittelbrief (Jahrgang 36 Nr.7, Juli 2002): Paracetamol-assoziierte Pankreatiden beschrieben; in der Fachinfo sind Pankreatiden bei Überdosierung beschrieben.

MEDIKAMENT: VOLTAREN RESINAT

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist eine bekannte UAW, ein zeitlicher Zusammenhang besteht.

MEDIKAMENT: OMEP

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist für Omeprazol beschrieben, Medikament wurde weitergegeben, es besteht daher kein zeitlicher Zusammenhang.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: In der Fachinfo sind Pankreatiden als sehr selten beschrieben.<dbtext>To_Char_NARRATIVE_2</dbtext>

Boehringer Ingelheim GmbH, Germany, 22.07.2009 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04909DE

Sender diagnosis: not reported

Narrative

This is a spontaneous (health professional confirmed) report from the German Health Authority via Web-Tool.

On 16Jul09 information has been received concerning a 69-years old male patient who suffered from pancreatitis from 27Feb09 to 04Mar09 while on therapy with paracetamol 3 tablets per day from 26Feb09 to 27Feb09. The reporting physician assumed the relationship between paracetamol and the event as likely.

Relevant medical history included coxarthrosis, benign prostatic hyperplasia and arterial hypertension. The patient is alcohol user (3 bottles of beer and 1 bottle of red wine in the week) and is ex-smoker.

The patient suffered from pancreatitis in Jul08. Concomitant medication included Berlosin 500mg (metamizole sodium), 3 tablets daily for postoperative pain; Cefuroxime (cefuroxime) 1,5 mg daily for infection prophylaxis; Voltaren Resinat 75 mg (diclofenac resinate), daily 2 capsules for postoperative pain; Diclofenac Resinate (diclofenac resinate) for an unknown indication; Flotrin 2 mg (terazosin hydrochloride), daily 1 tablet for benign prostatic hyperplasia; Ibuprofen 600 mg (ibuprofen) daily 1 film-coated tablet for coxarthrosis; Norspan (buprenorphine) daily 1 transdermal patch for postoperative pain; Omeprazol 40 mg (omeprazole) daily 1 tablet for gastric ulcer prophylaxis and Ramipril (ramipril) daily 2 tablets (2,5-0-5 mg) for hypertension arterial.

Tracking:

16Jul09 - initial information via Web-Tool

SANOFI-SYNTHELABO, 28.07.2009 DE-SANOFI-SYNTHELABO-A04200901023

Sender diagnosis: not reported

Narrative

Reaction as reported: pancreatitis (lipase 25 xULN, amylase 14 xULN)

LLT: PANCREATITIS / 10033645

PT: PANCREATITIS / 10033645

Narrative:

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute pancreatitis due to diclofenac (paracetamol).

The case involves a 69-year-old male patient with a past history of total endoprosthetic replacement of left hip and pancreatitis in Jul. 2008 (medication administered at that time unknown to reporter). At time of event the patient (ex-smoker) was suffering from coxarthrosis, benign prostatic hyperplasia, and arterial hypertension; the patient stated that he drank three beer and one bottle of red wine weekly. The following drugs are reported to have been taken concomitantly (not suspected): metamizole, cefuroxime, terazosin, ibuprofen, buprenorphine, omeprazole, and ramipril. On 24-Feb-2009, the patient underwent total endoprosthetic replacement of his right hip. On 25-Feb-2009, he started treatment with diclofenac 2x75mg/day for postoperative pain. On 26-Feb-2009, additional analgesic treatment with paracetamol (no trade name provided) 3x500mg/day was initiated. Two days later, the patient experience mild epigastric pain. Diclofenac, paracetamol, and metamizole were discontinued. Another two days later, the patient developed severe stabbing pain in abdomen, spreading into the lower abdomen. The following day (02-Mar-2009), the patient was hospitalised. Lipase on admission was 12fold the normal upper value. Abdominal sonography on 02-Mar-2009 revealed an oedematous pancreatitis. Buprenorphine was discontinued, and analgesic therapy was changed to Wuerzburger's perfusion apparatus or haloperidol. The patient also received enoxaparin 40 mg subcutaneously. Under initial absence of food, and fluid substitution, the symptoms resolved rapidly (no further laboratory tests performed), and the patient was discharged from hospital on 04-Mar-2009. Outcome was reported as recovered / resolved.

Causality determined by PV centre: "likely" for paracetamol and diclofenac.

Explanatory notes on causality: No typical risk factors for pancreatitis were present. Alternative explanation could have been a reaction to narcosis or the surgery itself (the patient suffered from a similar reaction in July 2008 after the first total endoprosthetic replacement of left hip), but such association has only been discussed in literature for abdominal operations. Acute alcohol use as cause was ruled out due to normal gamma-GT values (triglycerides were not measured). Paracetamol-associated pancreatitides were described in literature, and pancreatitis after overdose of paracetamol is described in physician's information. Pancreatitis is a known adverse effect of diclofenac, and there was a temporal relationship in this case. All other drugs were considered "not suspected".

NOVARTIS, 29.07.2009 DE-002147023-PHHY2009DE29503

Sender diagnosis: not reported

Narrative

Initial report was received from a physician via regional Pharmacovigilance Center (intensive monitoring project) via Health Authority (DE- BFARM - 09090323) on 15 Jul 2009: This patient with the history of pancreatitis in Jul 2008, smoking, drinking, coxarthrosis, benign prostatic hyperplasia and hypertension was on multi drug treatment including Voltaren resinat (diclofenac resinate) capsule, 75 mg bid for postoperative pain from 25 Feb 2009 to 27 Feb 2009. The co-suspects included Paracetamol, Norspan (buprenorphine) and Cefuroxim. The patient had hip total endoprosthesis (TEP) surgery on 24 Feb 2009. The patient experienced acute pancreatitis, mild epigastric pain 27 Feb 2009 and severe prickling abdominal pain on 01 Mar 2009. The patient was hospitalized from 02 Mar 2009 to 04 Mar 2009. On 02 Mar 2009, lipase increased to 1429 U/L (normal high range 57 U/L) and amylase increased to 1495 U/L (normal range 28-100 U/L). There was a mild increase in the AST (52.4 U/L) and ALT (55.1 U/L) ON 02 Mar 2009. The patient had experienced similar event after first hip TEP surgery in Jul 2008. The pain medications were stopped. The events were treated with initial food abstention, fluid substitution (Sterofundin); Wuerzburger perfusor or Haldol and Clexane 40 mg subcutaneous. On 02 Mar 2009, sonography abdomen showed edematous pancreatitis and pancreas duct was not enlarged with no other findings. The symptoms improved fast and pancreatitis resolved on 04 Mar 2009. Due to temporal relationship, Voltaren Resinat, Paracetamol, Norspan and Cefuroxim were suspected drugs.

HELIOS Klinikum Wuppertal, 26.01.2010 DE-PVZIBE-02-10154

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Pankreatitis

KURZZUSAMMENFASSUNG:

akute Pankreatitis infolge von Diclofenac (Paracetamol, Buprenorphin, Cefuroxim)

BESCHREIBUNG:

seit 27.02.2009 leichte epigastrale Schmerzen;

ab 01.03.2009 heftig stechende Bauchschmerzen, in den Unterbauch ausstrahlend, Zustand nach Hüft-TEP-OP am 24.02.2009;

bei Aufnahme 12-fach erhöhter Lipase-Wert;

anamnestisch: 07/2008 identisches Ereignis nach 1. Hüft-TEP-OP (Arzneimittelanamnese nicht bekannt, OP war in Sachsen)

THERAPIE:

initiale Nahrungskarenz, Flüssigkeitssubstitution (Sterofundin);

Schmerzmittel abgesetzt;

Würzburger Perfusor (Tramadol, Metamizol) und Haldol;

Clexane 40mg s.c.

VERLAUF:

kein typisches Vorliegen von Risikofaktoren für eine Pankreatitis (es wurde ein regelmäßiger Alkoholkonsum angegeben, allerdings zur Zeit wegen OP kein Alkohol);

schnelle Besserung der Symptomatik (keine weitere Laborkontrolle erfolgt);

02.03.2009 Sonographie Abdomen: ödematöse Pankreatitis, Pankreasgang nicht erweitert, Splenoportales Gefäßsystem unauffällig, Leber,

Gallenblase und Gallenwege unauffällig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 02.-04.03.2009;

Einweisungsgrund: akute Pankreatitis, Zustand nach Totaler Endoprothese des Hüftgelenks am 24.02.2009;

Im zeitlichen Verlauf wären Diclofenac, Paracetamol, Buprenorphin und Cefuroxim verdächtig. Für NSAR sind Pankreatiden auch in der Fachinfo beschrieben. Für Paracetamol, Buprenorphin und Cefuroxim gibt es Hinweise in der Literatur (aber nicht eindeutig in der Fachinfo).

Nach der ersten Hüft-TEP im Juli 2008 trat ebenfalls eine Pankreatitis auf. Die damalige Arzneimittelanamnese ist leider nicht ermittelbar. Als Alternativursache für die Pankreatitis könnte eine Reaktion auf die Narkose diskutiert werden, ebenso auf den chirurgischen Eingriff. Allerdings sind bisher in der Literatur nur abdominalchirurgische Eingriffe mit Pankreatitiden assoziiert worden. Die Gallenwege waren sonographisch unauffällig. Gegen akuten Alkoholkonsum als Ursache spricht die normalwertige Gamma-GT. Triglyzeride wurden nicht bestimmt.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: RAMIPRIL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist in der Fachinfo als sehr selten beschrieben. Medikament wird schon seit mehreren Jahren eingenommen und wurde nicht abgesetzt. Trotz Weitergabe erfolgte eine Symptombesserung, daher ist Ramipril hier unwahrscheinlich.

MEDIKAMENT: NORSPAN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Nur geringe bis gar kein Einfluss auf Sphincter Oddi (Staritz et al, 1986, Effect of modern analgesic on the bile duct sphincter in man. Gut 27:567);

In der Fachinfo sind keine Pankreatiden beschrieben, aber abdominale Schmerzen.

MEDIKAMENT: PARACETAMOL

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Der Arzneimittelbrief (Jahrgang 36 Nr.7, Juli 2002): Paracetamol-assoziierte Pankreatiden beschrieben; in der Fachinfo sind Pankreatiden bei Überdosierung beschrieben.

MEDIKAMENT: VOLTAREN RESINAT

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist eine bekannte UAW, ein zeitlicher Zusammenhang besteht.

MEDIKAMENT: OMEP

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: Pankreatitis ist für Omeprazol beschrieben, Medikament wurde weitergegeben, darunter Symptombesserung - es besteht daher kein zeitlicher Zusammenhang.

MEDIKAMENT: IBUPROFEN

BETREFFENDE UAW: Pankreatitis

ERLÄUTERUNG: In der Fachinfo sind Pankreatiden als sehr selten beschrieben. Die Einnahme wurde 1 Woche vor Auftreten der Symptome beendet und über einen mehrjährigen Zeitraum gut vertragen.

NOVARTIS, 03.02.2010 DE-NOVNCHHQ-GXKR2009DE08058

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 15 Jul 2009 from the German Health Authority (DE-BfArM-09090323), provided by a pharmacovigilance center, reported by a physician: This multimorbid patient developed pancreatitis during treatment with paracetamol tablets (MAH unknown) for postoperative pain. Voltaren resinate (diclofenac resinate) was also suspected. Several other drugs were taken concomitantly. The outcome of the adverse event was reported as resolved. For paracetamol causality was assessed as likely.

Significant follow-up received from health authority (BfArM homepage, DE-BFARM-09090323) on 27 Jan 2010 changing several concomitant drugs into suspected drugs and changing causality assessment: Ramipril (MAH unknown), Omeprazole (omeprazole) and ibuprofen (MAH unknown) were also suspected. Causality for ramipril, Omeprazole and ibuprofen was assessed as unlikely. For paracetamol causality was assessed as possible. Spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 04.02.2010 DE-Boehringer Ingelheim GmbH, Germany-2009-DE-04909DE

Sender diagnosis: not reported

Narrative

This is a spontaneous report health care professional confirmed report from the German Health Authority BfArM.

18.03.2013

BfArM - Pharmacovigilance

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On 28Jan2010 follow-up information was received from a physician in Germany via the German Health Authority BfArM concerning a 69-year-old male patient, who was on paracetamol 3x500 mg/day from 26Feb2009 to 27Feb2009 for postoperative pain, on ramipril 2,5-0-5 mg/day since 200 and continuing for arterial hypertension, on Voltaren Resinat (diclofenac resinate) 2x75 mg/day from 25Feb2009 to 27Feb2009 for postoperative pain, Omep (omeprazole) 1x40 mg/day since 25Feb2009 and continuing for gastric ulcer prophylaxis and on ibuprofen 1x600 mg/day from 2007 to 20Feb2009 for coxarthrosis. The event was considered as serious due to the statement of the reporter. Ramipril and Omep were continued. Paracetamol, Voltaren Resinat and ibuprofen were discontinued. The patient has recovered.

From 27Feb2009 to 04Mar2009 the patient suffered from pancreatitis.

Comments to the following event:

Pancreatitis

Short summary:

acute pancreatitis due to diclofenac (Paracetamol, Buprenorphin, Cefuroxim)

Description:

since 27Feb2009 mild epigastric pain,

since 01Mar2009 severe stabbing stomach pain, emissive into the lower abdomen, condition after hip-TEP-operation on 24Feb2009,

during accommodation 12-point increased lipase value,

anamnesic: Jul2008 identical event after first hip-TEP-operation (medicament anamnesis not known, operation was in Saxony)

Therapy:

initial abstinence from food, liquid substitution (Sterofundin)

analgesics discontinued

Würzburger perfusion (tramadol, metamizol) and Haldol,

Clexane 40 mg sc

Course:

no typical existent of risk factors for a pancreatitis (regular intake of alcohol was stated, but at the time of the operation no alcohol),

fast improvement of the symptomatology (no further laboratory test has been carried out)

02Mar2009 sonography abdomen: oedematous pancreatitis, pancreatic duct not expanded, splenoportal vascular system without pathological findings,

liver, gallbladder and bile duct without pathological findings

explanation of the event:

hospitalisation from 02Mar2009 - 04Mar2009

reason for hospitalisation: acute pancreatitis, condition after total endoprosthesis of hip joint on 24Feb2009,

in chronological sequence Diclofenac, Paracetamol, Buprenorphin and Cefuroxim are suspected. For NSAR are pancreatitis also described in the

product information. For Paracetamol, Buprenorphin and Cefuroxim there are references in the literature (but not definite in the product information)

After the first hip-TEP in Jul2008 a pancreatitis also occurred. At that time a medicament anamnesis is not ascertainable. As alternative reason for the pancreatitis could be discussed a reaction due to the anaesthesia, as well due to the surgical intervention. However until now only abdominal surgical interventions are associated with pancreatitis in literature.

The bile duct were sonographic without pathological findings. Against acute intake of alcohol as reason speaks the normal value of Gamma-GT (glutamyl transpeptidase).

Triglycerides were not calculated.

Concomitant medications included Flotrin (terazosin hydrochloride) 1x2 mg/day since 2005 and continuing for benign prostatic hyperplasia, Cefuroxime (cefuroximaxetil) 1x1,5 mg/day on 24Feb2009 for infection prophylaxis, Norspan (buprenorphine) 1x1 plaster/day from 24Feb2009 to 02Mar2009 for postoperative pain and Berlosin (metamizole sodium) 3x500 mg/day from 26Feb2009 to 27Feb2009 for postoperative pain.

Concomitant diseases included ex-smoker since -1980 (1955-1980 1 package/week) and continuing, alcohol use (beginning unknown, 3 beer + 1 bottle wine a week) and continuing, coxarthrosis (beginning unknown) and continuing, benign prostatic hyperplasia (beginning unknown) and continuing and arterial hypertension sine 1990 and continuing.

Past diseases included TEP (total endoprosthesis) left in Jul2008, TEP (total endoprosthesis) right on 24Feb2009 and pancreatitis in Jul2008.

The reporter rated the causal relationship between the event and Paracetamol and Voltaren Resinat as possible and rated the causal relationship between the event and Ramipril, Omep and Ibuprofen as unlikely.

Tracking of Changes:

16Jul2009 - initial information from physician via the German Health Authority BfArM

28Jan2010 - follow-up information from physician via the German Health Authority BfArM

sanofi-aventis, 04.02.2010 DE-SANOFI-AVENTIS-A04200901023

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority

(authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute pancreatitis due to diclofenac (paracetamol).

The case involves a 69-year-old male patient with a past history of total endoprosthetic replacement of left hip and pancreatitis in Jul. 2008 (medication administered at that time unknown to reporter). At time of event the patient (ex-smoker) was suffering from coxarthrosis, benign prostatic hyperplasia, and arterial hypertension; the patient stated that he drank three beer and one bottle of red wine weekly. The following drugs are reported to have been taken concomitantly (not suspected): metamizole, cefuroxime, terazosin, ibuprofen, buprenorphine, omeprazole, and ramipril. On 24-Feb-2009, the patient underwent total endoprosthetic replacement of his right hip. On 25-Feb-2009, he started treatment with diclofenac 2x75mg/day for postoperative pain. On 26-Feb-2009, additional analgesic treatment with paracetamol (no trade name provided) 3x500mg/day was initiated. Two days later, the patient experience mild epigastric pain. Diclofenac, paracetamol, and metamizole were discontinued. Another two days later, the patient developed severe stabbing pain in abdomen, spreading into the lower abdomen. The following day (02-Mar-2009), the patient was hospitalised. Lipase on admission was 12fold the normal upper value. Abdominal sonography on 02-Mar-2009 revealed an oedematous pancreatitis. Buprenorphine was discontinued, and analgesic therapy was changed to Wuerzburger's perfusion apparatus or haloperidol. The patient also received enoxaparin 40 mg subcutaneously. Under initial absence of food, and fluid substitution, the symptoms resolved rapidly (no further laboratory tests performed), and the patient was discharged from hospital on 04-Mar-2009. Outcome was reported as recovered / resolved.

Causality determined by PV centre: "likely" for paracetamol and diclofenac.

Explanatory notes on causality: No typical risk factors for pancreatitis were present. Alternative explanation could have been a reaction to narcosis or the surgery itself (the patient suffered from a similar reaction in July 2008 after the first total endoprosthetic replacement of left hip), but such association has only been discussed in literature for abdominal operations. Acute alcohol use as cause was ruled out due to normal gamma-GT values (triglycerides were not measured). Paracetamol-associated pancreatitides were described in literature, and pancreatitis after overdose of paracetamol is described in physician's information. Pancreatitis is a known adverse effect of diclofenac, and there was a temporal relationship in this case. All other drugs were considered "not suspected".

Additional information extracted from the website of the German health authority on 28-Jan-2010. Information by PV centre: The patient had reported regular alcohol consumption, but around the time of operation the patient did not consume alcohol.

The following drugs were switched from concomitant to suspect medication: ramipril (trade name not provided) 7.5mg/day from 2000 onwards for arterial hypertension, ibuprofen (trade name not provided) 600mg/day from 2007 to 20-Feb-2009 for coxarthrosis, and omeprazole 1x40mg/day from 25-Feb-2009 onwards as gastric ulcer prophylaxis.

Causality assessment was changed by reporter to WHO score:

"possible" for paracetamol, diclofenac,

"unlikely" for ramipril, ibuprofen, omeprazole.

Comments regarding causality of new suspect drugs:

Pancreatitis is described in physicians' information for ramipril as very rare; the drug had been taken for several years, the symptoms improved despite continuation of treatment, therefore ramipril is rather unlikely in this case. Pancreatitis is described in physicians' information for ibuprofen as very rare; the drug was stopped one week before onset of symptoms and had been tolerated over a longer period of time. Pancreatitis is described for omeprazole; symptoms improved despite continuation of treatment with omeprazole, therefore no temporal relationship exists in this case.

NOVARTIS, 05.02.2010 DE-002147023-PHHY2009DE29503

Sender diagnosis: not reported

Narrative

Initial report was received from a physician via regional Pharmacovigilance Center (intensive monitoring project) via Health Authority (DE- BFARM - 09090323) on 15 Jul 2009: This patient with the history of pancreatitis in Jul 2008, smoking, drinking, coxarthrosis, benign prostatic hyperplasia and hypertension was on multi drug treatment including Voltaren resinat (diclofenac resinate) capsule, 75 mg bid for postoperative pain from 25 Feb 2009 to 27 Feb 2009. The co-suspects included Paracetamol, Norspan (buprenorphine) and Cefuroxim. The patient had hip total endoprosthesis (TEP) surgery on 24 Feb 2009. The patient experienced acute pancreatitis, mild epigastric pain 27 Feb 2009 and severe prickling abdominal pain on 01 Mar 2009. The patient was hospitalized from 02 Mar 2009 to 04 Mar 2009. On 02 Mar 2009, lipase increased to 1429 U/L (normal high range 57 U/L) and amylase increased to 1495 U/L (normal range 28-100 U/L). There was a mild increase in the AST (52.4 U/L) and ALT (55.1 U/L) ON 02 Mar 2009. The patient had experienced similar event after first hip TEP surgery in Jul 2008. The pain medications were stopped. The events were treated with initial food abstention, fluid substitution (Sterofundin); Wuerzburger perfusor or Haldol and Clexane 40 mg subcutaneous. On 02 Mar 2009, sonography abdomen showed edematous pancreatitis and pancreas duct was not enlarged with no other findings. The symptoms improved fast and pancreatitis resolved on 04 Mar 2009. Due to temporal relationship, Voltaren Resinat, Paracetamol, Norspan and Cefuroxim were suspected drugs.

Follow-up information received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority on 27 Jan 2010: Ramipril (manufacturer unknown) 2.5 mg and 5 mg two times a day for hypertension arterial from 2000 and Ibuprofen (manufacturer unknown) 600 mg per day for coxarthrosis from 2007 to 20 Feb 2009 were added as co-suspect in this case. Norspan (buprenorphine) and Cefuroxim was changed from suspect drug to concomitant medications. The onset date for pancreatitis was reported as 27 Feb 2009.

sanofi-aventis, 07.04.2010 DE-SANOFI-AVENTIS-A04200901023

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority

18.03.2013

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(authority number DE-BFARM-09090323).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

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FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09090343

Serious No	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 19180430	Age: 90 Year	weight (kg): 53	BMI_int: 20.7	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute renal failure	20090425	20090501	7 Day	2 Day	recovered/resolved
2 Abdominal pain	20090425				Unknown
3 Serum creatinine increased	20090425	20090501	7 Day		recovered/resolved
4 White blood cell count increased	20090426			3 Day	Unknown
5 Confused	20090425				Unknown
6 Disorientated	20090425				Unknown
7 Aspartate aminotransferase increased	20090426			3 Day	Unknown
8 Bladder catheterisation					Unknown
9 Blood glucose increased	20090426			3 Day	Unknown
10 Blood pressure increased	20090425			2 Day	Unknown
11 Hydronephrosis					Unknown
12 Kidney pain					Unknown
13 Platelet count decreased	20090426			3 Day	Unknown
14 Pulse increased	20090425			2 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUN ibuprofen lysinate	Oral	2400 Mg milligram(s)/Day 212	20090424-20090426 3 Day	Pain
2 NOVALGIN metamizole	Oral	3 Df dosage form/Day 158	20090424-20090426 3 Day	Pain
3 PANTOZOL pantoprazole	Oral	40 Mg milligram(s)/Day 245	20090424-20090426 3 Day	Gastric ulcer prophylaxis

Drugs (concomitant)

4 ALNA tamsulosin hydrochloride	Oral	0.4 Mg milligram(s)/ Day 245	2006-20090424 1095 Day	Benign prostatic hyperplasia
5 ASS acetylsalicylic acid	Oral	100 Mg milligram(s)/ Day 245	2005-20090424 -	Coronary artery disease
6 ISMN "GENERICON" isosorbide mononitrate	Oral	20 Mg milligram(s)/ Day 245	2006- -	Coronary artery disease NOS

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Coronary artery disease	2005		Yes	
Cholecystolithiasis	2006	2006	No	endoskopisch retrograde Cholangiopankreatikographie (ERCP)
Endoscopic retrograde cholangiopancreatography	2006		No	
Fall	20090424		Unknown	
Benign prostatic hyperplasia	2006		Yes	
Contusion of hip	20090424		Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20090426	187	µmol/l	
Creatinine	20090430	73	µmol/l	
Urea	20090426	10.4	mmol/l	
Urea	20090410	5.0	mmol/l	
Aspartate aminotransferase	20090426	1.72	µkat/l	
Alanine aminotransferase	20090426	0.61	µkat/l	
Alkaline phosphatase	20090426	1.57	µkat/l	
Thrombocyte count	20090426	110	Gpt/l	
Haemoglobin	20090426	10.3	mmol/l	
Haematocrit	20090426	0.47	SI-Einheit	
Total white blood cell count	20090426	13.4	10E9/l	
C-reactive protein	20090426	90.2	mg/l	
C-reactive protein	20090429	43	mg/l	
C-reactive protein	20090430	34.9	mg/l	
Lipase	20090426	0.61	µkat/l	
Glucose	20090426	13.6	mmol/l	
Blood potassium	20090426	4.2	mmol/l	
Blood potassium	20090430	3.8	mmol/l	
Sodium	20090426	136	mmol/l	
Sodium	20090430	137	mmol/l	
Troponin T	20090426	<0.1	ng/ml	
TSH	20090426	0.36	µU/ml	
Free T3	20090426	2.9	pmol/l	
Free T4	20090426	16.3	pmol/l	
Aspartate aminotransferase	20090426	1.72 ukat/l	NA	
Blood pressure	20090425	160/95 mmHg	NA	
C-reactive protein	20090426	90.2	mg/l	
C-reactive protein	20090430	34.9	mg/l	
C-reactive protein	20090429	43	mg/l	
Creatinine	20090426	187	umol/l	
Glucose	20090426	13.6	mmol/l	
Pulse rate	20090425	92/min	NA	
Thrombocyte count	20090426	110 Gpt/l	NA	
Troponin T	20090426	0.36 uU/mL	NA	
Ultrasound scan	20090426	nicht vorhanden	NA Absent	
Urea	20090426	10.4	mmol/l	
White blood cell count	20090426	13.4x10^9/	NA	

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal / Universität Witten, 14.07.2009 DE-PVZIBE-02-10172

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

akutes Nierenversagen

KURZZUSAMMENFASSUNG:

akutes Nierenversagen (Verwirrtheit, Bauchschmerzen, Desorientiertheit) unter Ibuprofen-, Metamizol- und Pantoprazol- Einnahme (Tamsulosin 1 Tag vor UAW abgesetzt, die Nicht-Einnahme kann Harnstauungsniere bedingen)

BESCHREIBUNG:

Patient seit 25.04.2009 Verwirrt, Bauchschmerzen, örtlich und zeitlich nicht orientiert;

anamnestisch: Zustand nach Sturz am 24.04.2009: Hüftprellung, daraufhin Schmerztherapie;

laut Ehefrau war der Patient vor dem Sturz nicht verwirrt, er führte den Haushalt selbstständig;

bei Aufnahme: Blutdruck 160/95mmHg, Puls 92/min, keine Abwehrspannung, rechtes Nierenlager Klopfeschmerz, trockene Zunge, keine Ödeme

THERAPIE:

intravenöse Flüssigkeitssubstitution;

Dauerkatheter (Spontanentleerung);

1x20mg ISMN (sonst keine Dauermedikation);

27.04.-01.05.2009 Ciprobay

VERLAUF:

26.04.2009 CCT: keine Blutung oder Ischämie, keine Fraktur, Zeichen der vaskulären Enzephalopathie (mikroangiopathisch);

26.04.2009 Sonographie Abdomen: Cholecystolithiasis oder Anzeichen von Entzündung, beidseits Harnstauungsniere Grad 1, prallgefüllte Harnblase (950ml);

im Verlauf Besserung des Allgemeinzustandes (und der Vigilanz), benötigt aber Hilfe bei allen Tätigkeiten, bis zur Entlassung erholen sich die Nierenwerte vollständig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 26.04.-01.05.2009;

Einweisung aufgrund eines akuten Nierenversagens (bei einem Creatinin-Wert von 187µmol/l); eine Nierenerkrankung ist anamnestisch nicht bekannt;

Differentialdiagnostisch: CCT: keine Blutung, Ischämie oder Fraktur;

Tamsulosin zur Therapie der benignen Prostatahyperplasie wurde 1 Tag vorher abgesetzt, Harnstauungsniere ist durch das Weglassen der Therapie möglich;

es liegen keine Nierenwerte vor dem 26.04.2009 vor;

unter der Therapie erholen sich die Nierenwerte vollständig

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: IMBUN - SLOW RELEASE

BETREFFENDE UAW: akutes Nierenversagen

ERLÄUTERUNG: Als Nebenwirkung ist eine interstitielle Nephritis, die mit einer akuten Niereninsuffizienz einhergehen kann, beschrieben. Alternativ kann das Absetzen von Tamsulosin zur Symptomatik geführt haben (Harnstauungsniere).

MEDIKAMENT: ASS

BETREFFENDE UAW: akutes Nierenversagen

ERLÄUTERUNG: Nierenfunktionsstörungen sind als sehr selten beschrieben, gegen einen Zusammenhang spricht jedoch die jahrelange Einnahme von ASS.

MEDIKAMENT: PANTOZOL

BETREFFENDE UAW: akutes Nierenversagen

ERLÄUTERUNG: Als Nebenwirkung ist eine interstitielle Nephritis beschrieben. Alternativ kann das Absetzen von Tamsulosin zur Symptomatik geführt haben (Harnstauungsniere).

MEDIKAMENT: NOVALGIN

BETREFFENDE UAW: akutes Nierenversagen

ERLÄUTERUNG: Vorübergehende Nierenstörung ist in Einzelfällen beschrieben. Alternativ kann das Absetzen von Tamsulosin zur Symptomatik geführt haben (Harnstauungsniere).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sanofi-Aventis Deutschland GmbH, 22.07.2009 DE-AVENTIS-200911813DE

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09090343).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute renal failure (confusion, abdominal pain, disorientation) under intake of ibuprofen, metamizole and pantoprazole (tamsulosin discontinued one day before event, absence of drug can cause hydronephrosis).

The case involves a 90-year-old male patient with a past medical history of endoscopic retrograde cholangiopancreatography in 2006 due to cholecystolithiasis. At time of event the patient was suffering from coronary artery disease, benign prostatic hyperplasia, and contusion of hip. The following drugs are reported to have been taken concomitantly (not suspected): tamsulosin, acetylsalicylic acid, and isosorbide mononitrate. On 24-Apr-2009, the patient started treatment with metamizole sodium (Novalgine) 3x750 mg/day and ibuprofen (Imbun - slow release) 3x800mg/day as analgesic therapy, and treatment with pantoprazole 1x40mg/day for gastric ulcer prophylaxis. On the second day of treatment, the patient developed confusion, abdominal pain, and disorientation to place and time. The patient was hospitalised the following day due to acute renal failure (creatinine 187 µmol/l) without

medical history of renal disease. Examination on admission: blood pressure 160/95 mmHg, pulse 92/min, no guarding, right renal angle tenderness, dry tongue, no oedemas. Cranial computerized tomography on 26-Apr-2009 revealed no bleeding of ischemia, no fracture, but signs of vascular encephalopathy (microangiopathic). Abdominal sonography on 26-Apr-2009 revealed cholecystolithiasis or signs of inflammation, bilateral hydronephrosis grade I, tightly filled urinary bladder (950 ml). All medication except isosorbide mononitrate was discontinued. The patient was treated with intravenous fluid substitution, permanent catheter, and ciprofloxacin (from 27-Apr-2009 to 01-May-2009). Under treatment, renal values normalised, and general health condition and vigilance improved. The patient was discharged from hospital on 01-May-2009. Outcome was reported as "not recovered / not resolved" on

01-May-2009.

Causality determined by PV centre: "likely" for all metamizole, ibuprofen, and pantoprazole

Explanatory notes on causality for metamizole: transient disturbance of renal function is prescribed in single cases. Alternative explanation could be the stop of treatment with tamsulosin, that could have caused the event (hydronephrosis).

Laboratory:

26-APR-2009 Creatinine 187µmol/l (norm. high range 127)
 30-APR-2009 Creatinine 73µmol/l
 26-APR-2009 Urea 10.4 mmol/l (9.2)
 10-APR-2009 Urea 5.0 mmol/l
 26-APR-2009 Aspartat aminotranspharase 1.72 ukat /l (0.05-0.83)
 26-APR-2009 Alanine aminotranspharase 0.61 ukat /l (0.83)
 26-APR-2009 Alkaline phosphatase 1.57 ukat/l (1.98)
 26-APR-2009 Thrombocytes 110 Gpt/l (150-400)
 26-APR-2009 Haemoglobin 10.3 mmol/l (8.5-10.9)
 26-APR-2009 Hematocrit 0.47 (0.4-0.51)
 26-APR-2009 Total white blood cell count 13.4x10⁹/l (4 - 10)
 26-APR-2009: C-reactive protein 90.2 mg/l (5.0)
 29-APR-2009: C-reactive protein 43 mg/l
 30-APR-2009: C-reactive protein 34.9 mg/l
 26-APR-2009: Lipase 0.61 ukat/l (1)
 26-APR-2009: Glucose 13.6 mmol/l (3.3-5.8)
 26-APR-2009: Blood potassium 4.2 mmol/l (3.5-5.-3)
 30-APR-2009: Blood potassium 3.8
 26-APR-2009: Sodium 136 mmol/l (136-152)
 30-APR-2009: Sodium 137 mmol/l
 26-APR-2009: Troponin-T <0.1 ng/ml (0.1-1)
 26-APR-2009: TSH 0.36 uU/ml (0.40-4.0)
 26-APR-2009: Free T3 2.9 pmol/l (2.3-6.3)
 26-APR-2009: Fre T4 16.3 pmol/l (10.3-23.3)

NOVARTIS, 30.07.2009 DE-002147023-PHHY2009DE30030

Sender diagnosis: not reported

Narrative

Initial Health Authority report from the BFARM website (Ref no: DE-BFARM-09090343) received on 15 Jul 2009 via Nycomed (0206562): This patient started therapy with Pantozol (pantoprazole tablet), 40 mg once daily, oral, on 24 Apr 2009, for gastric ulcer prophylaxis. Co-suspect medications were Imbun (ibuprofen) 800 mg and Novalgine (metamizole), both initiated on 24 Apr 2009, for pain relief. The patient had a medical history of coronary artery disease, cholecystolithiasis, contusion of hip and benign prostatic hyperplasia. He was concomitantly treated with Alna (tamsulosin hydrochloride tablet, stopped on 24 Apr 2009), ASS (acetylsalicylic acid), and ISMN (isosorbide mononitrate tablet). On 24 Apr 2009, the patient fell and was started on the suspected drugs due to hip contusion. On 25 Apr 2009, i.e. one day after the first dose of Pantozol and other suspect medications, the patient was confused, not oriented (time and place), and had abdominal pain. At admission to hospital, the patient had a blood pressure of 160/95 mmHg, pulse 92/min, no abdominal tenderness, no oedema, but he had right kidney pain. He was diagnosed with acute renal failure (creatinine level was 187 micromol/l, no normal values provided) and treated with a permanent bladder catheter, intravenous fluid replacement and ciprofloxacin. Ultrasound on 26 Apr 2009 showed hydronephrosis on both sides and a full bladder (950 ml). Cranial CT showed no bleeding, ischemia or fracture. Renal lab values completely normalized under ongoing therapy. On 26 Apr 2009 Aspartate aminotransferase was 1.72 ukat/l (normal range: 0.05-0.83 ukat/l), blood glucose was 13.6 mmol/l (normal range: 3.3 - 5.8 mmol/l), platelet count was 110 Gpt/l (normal range: 150-400

Gpt/l) and white blood cell count was $13.4 \times 10^9/l$ (normal range: $4 \times 10^9/l$ - $10 \times 10^9/l$). Creatinine level was 73 micromol/l on 30 Apr 2009. Therapy with Pantozol and all suspect medications was discontinued on 26 Apr 2009. The patient had recovered at the time of reporting. The German Health Authority assessed causality as likely for Pantozol, Imbun and Novalgin, but favoured the withdrawal of Alna as the origin of the event.

NOVARTIS, 06.08.2009 DE-002147023-PHHY2009DE30030

Sender diagnosis: not reported

Narrative

Initial Health Authority report from the BFARM website (Ref no: DE-BFARM-09090343) received on 15 Jul 2009 via Nycomed (0206562): This patient started therapy with Pantozol (pantoprazole tablet), 40 mg once daily, oral, on 24 Apr 2009, for gastric ulcer prophylaxis. Co-suspect medications were Imbun (ibuprofen) 800 mg and Novalgin (metamizole), both initiated on 24 Apr 2009, for pain relief. The patient had a medical history of coronary artery disease, cholecystolithiasis, contusion of hip and benign prostatic hyperplasia. He was concomitantly treated with Alna (tamsulosin hydrochloride tablet, stopped on 24 Apr 2009), ASS (acetylsalicylic acid), and ISMN (isosorbide mononitrate tablet). On 24 Apr 2009, the patient fell and was started on the suspected drugs due to hip contusion. On 25 Apr 2009, i.e. one day after the first dose of Pantozol and other suspect medications, the patient was confused, not oriented (time and place), and had abdominal pain. At admission to hospital, the patient had a blood pressure of 160/95 mmHg, pulse 92/min, no abdominal tenderness, no oedema, but he had right kidney pain. He was diagnosed with acute renal failure (creatinine level was 187 micromol/l, no normal values provided) and treated with a permanent bladder catheter, intravenous fluid replacement and ciprofloxacin. Ultrasound on 26 Apr 2009 showed hydronephrosis on both sides and a full bladder (950 ml). Cranial CT showed no bleeding, ischemia or fracture. Renal lab values completely normalized under ongoing therapy. On 26 Apr 2009 Aspartate aminotransferase was 1.72 ukat/l (normal range: 0.05-0.83 ukat/l), blood glucose was 13.6 mmol/l (normal range: 3.3 - 5.8 mmol/l), platelet count was 110 Gpt/l (normal range: 150-400 Gpt/l) and white blood cell count was $13.4 \times 10^9/l$ (normal range: $4 \times 10^9/l$ - $10 \times 10^9/l$). Creatinine level was 73 micromol/l on 30 Apr 2009. Therapy with Pantozol and all suspect medications was discontinued on 26 Apr 2009. The patient had recovered at the time of reporting. The German Health Authority assessed causality as likely for Pantozol, Imbun and Novalgin, but favoured the withdrawal of Alna as the origin of the event.

Follow-up information was received on 24 Jul 2009 and forwarded by Nycomed on 27 Jul 2009 : The onset date of the event acute renal failure was amended from 24 May 2009 to 25 Apr 2009.

HELIOS Klinikum Wuppertal, 28.05.2010 DE-PVZIBE-02-10172

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Kreatinin im Serum erhöht

KURZZUSAMMENFASSUNG:

Kreatininanstieg (Verwirrtheit, Bauchschmerzen, Desorientiertheit) unter Ibuprofen-, Metamizol- Einnahme; eine Harnstauungsniere durch die Nicht-Einnahme von Tamsulosin (1 Tag vor UAW abgesetzt) ist differentialdiagnostisch zu bedenken)

BESCHREIBUNG:

Patient seit 25.04.2009 verwirrt, Bauchschmerzen, örtlich und zeitlich nicht orientiert;
anamnestisch: Zustand nach Sturz am 24.04.2009: Hüftprellung, daraufhin Schmerztherapie; laut Ehefrau war der Patient vor dem Sturz nicht verwirrt, er führte den Haushalt selbstständig;
bei Aufnahme: Blutdruck 160/95mmHg, Puls 92/min, keine Abwehrspannung, rechtes Nierenlager Klopfeschmerz, trockene Zunge, keine Ödeme

THERAPIE:

intravenöse Flüssigkeitssubstitution;
Dauerkatheter (Spontanentleerung);
1x20mg ISMN (sonst keine Dauermedikation);
27.04.-01.05.2009 Ciprobay

VERLAUF:

26.04.2009 CCT: keine Blutung oder Ischämie, keine Fraktur, Zeichen der vaskulären Enzephalopathie (mikroangiopathisch);
26.04.2009 Sonographie Abdomen: Cholecystolithiasis oder Anzeichen von Entzündung, beidseits Harnstauungsniere Grad 1, prallgefüllte Harnblase (950ml);
im Verlauf Besserung des Allgemeinzustandes (und der Vigilanz), benötigt aber Hilfe bei allen Tätigkeiten, bis zur Entlassung erholen sich die Nierenwerte vollständig

ERLÄUTERUNGEN ZUR UAW:

#kontemporäre UAW#

Krankenhausaufenthalt: 26.04.-01.05.2009;

Die Krankenhauseinweisung erfolgte infolge des Symptomenkomplexes z.n. Sturz auf die Hüfte (Hüftprellung) und anschließende zunehmende Verwirrtheit sowie Bauchschmerzen und begleitendem Kreatininanstieg. Die Bauchschmerzen sind mit dem stationär diagnostizierten Harnwegsinfekt erklärbar. Verwirrtheit läßt sich auf den z.n. Sturz (zerebral keine Blutung oder Ischämie in der Bildgebung nachweisbar), den Harnwegsinfekt

verbunden mit vermutlich geringerer Nahrungs- und Flüssigkeitsaufnahme (klinisch trockene Zunge) zurückführen. Es bleibt zweifelhaft, inwieweit der geringgradige Kreatininanstieg hier zu einer klinischen Korrelat geführt haben könnte. Bei dem bisher nierengesunden Patienten kann jedoch die kurzfristige Gabe von insbesondere Ibuprofen (Metamizol) in Zusammenhang mit einem Flüssigkeitsdefizit zu dem akuten, prärenal bedingten, Kreatininanstieg geführt haben. Im Gesamtkontext ist dieser als begleitend anzusehen. Unter der Therapie erholen sich die Nierenwerte vollständig

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: IMBUN - SLOW RELEASE

BETREFFENDE UAW: Kreatinin im Serum erhöht

ERLÄUTERUNG: NSAR greifen in die Kompensationsmechanismen der renalen Mikrozirkulation ein und können damit eine noch kompensierte renale Minderperfusion in ein manifestes prärenales Nierenversagen umwandeln. Insbesondere trifft dies auf Patienten mit Volumenmangel oder chron. Niereninsuffizienz zu, da bei diesen die glomeruläre Filtrationsrate teilweise durch eine Prostaglandin-vermittelte Hyperfiltration aufrechterhalten wird.

Infolge des Sturzes hatte der Patient vermindert Nahrung/Flüssigkeit zu sich genommen und gleichzeitig Ibuprofen erhalten, so dass ein pathophysiologischer und zeitlicher Zusammenhang für einen akuten prärenal bedingten Kreatininanstieg denkbar ist. Alternativ kann das Absetzen von Tamsulosin zur Symptomatik geführt haben (Harnstauungsniere).

MEDIKAMENT: NOVALGIN

BETREFFENDE UAW: Kreatinin im Serum erhöht

ERLÄUTERUNG: Vorübergehende Nierenstörung ist in Einzelfällen beschrieben. Metamizol wurde erst wenige Tage zuvor in die Therapie eingeführt. Alternativ kann das Absetzen von Tamsulosin zur Symptomatik geführt haben (Harnstauungsniere).<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 07.06.2010 DE-SANOFI-AVENTIS-200911813DE

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-09090343).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the Begaud algorithm.

Short summary: Acute renal failure (confusion, abdominal pain, disorientation) under intake of ibuprofen, metamizole and pantoprazole (tamsulosin discontinued one day before event, absence of drug can cause hydronephrosis).

The case involves a 90-year-old male patient with a past medical history of endoscopic retrograde cholangiopancreatography in 2006 due to cholecystolithiasis. At time of event the patient was suffering from coronary artery disease, benign prostatic hyperplasia, and contusion of hip. The following drugs are reported to have been taken concomitantly (not suspected): tamsulosin, acetylsalicylic acid, and isosorbide mononitrate. On 24-Apr-2009, the patient started treatment with metamizole sodium (Novalgin) 3x750 ml/day and ibuprofen (Imbun - slow release) 3x800mg/day as analgesic therapy, and treatment with pantoprazole 1x40mg/day for gastric ulcer prophylaxis. On the second day of treatment, the patient developed confusion, abdominal pain, and disorientation to place and time. The patient was hospitalised the following day due to acute renal failure (creatinine 187 µmol/l) without medical history of renal disease. Examination on admission: blood pressure 160/95 mmHg, pulse 92/min, no guarding, right renal angle tenderness, dry tongue, no oedemas. Cranial computerized tomography on 26-Apr-2009 revealed no bleeding of ischemia, no fracture, but signs of vascular encephalopathy (microangiopathic). Abdominal sonography on 26-Apr-2009 revealed cholecystolithiasis or signs of inflammation, bilateral hydronephrosis grade I, tightly filled urinary bladder (950 ml). All medication except isosorbide mononitrate was discontinued. The patient was treated with intravenous fluid substitution, permanent catheter, and ciprofloxacin (from 27-Apr-2009 to 01-May-2009). Under treatment, renal values normalised, and general health condition and vigilance improved. The patient was discharged from hospital on 01-May-2009. Outcome was reported as "not recovered / not resolved" on 01-May-2009.

Causality determined by PV centre: "likely" for all metamizole, ibuprofen, and pantoprazole

Explanatory notes on causality for metamizole: transient disturbance of renal function is prescribed in single cases. Alternative explanation could be the stop of treatment with tamsulosin, that could have caused the event (hydronephrosis).

Laboratory:

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30-APR-2009 Creatinine 73µmol/l

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26-APR-2009 Alanine aminotransferase 0.61 ukat /l (0.83)

26-APR-2009 Alkaline phosphatase 1.57 ukat/l (1.98)

26-APR-2009 Thrombocytes 110 Gpt/l (150-400)

26-APR-2009 Haemoglobin 10.3 mmol/l (8.5-10.9)

26-APR-2009 Hematocrit 0.47 (0.4-0.51)

26-APR-2009 Total white blood cell count $13.4 \times 10^9/l$ (4 - 10)
26-APR-2009: C-reactive protein 90.2 mg/l (5.0)
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26-APR-2009: TSH 0.36 uU/ml (0.40-4.0)
26-APR-2009: Free T3 2.9 pmol/l (2.3-6.3)
26-APR-2009: Free T4 16.3 pmol/l (10.3-23.3)

Additional information extracted from the website of the German health authority on 01-Jun-2010. Information by PV centre: Event was changed to "serum creatinine increased". Pantozol was changed from suspected to concomitant medication.

PV centre's comment concerning adverse drug reaction: "contemporary adverse drug effect"; hospitalisation from 26-Apr-2009 to 01-May-2009.

The patient was hospitalised due to symptom complex of previous fall on hip (hip contusion) followed by progressing confusion, as well as abdominal pain, and accompanying increase in creatinine. The abdominal pain was related to the urinary tract infection (diagnosed during hospitalisation). Confusion was attributed to the fall (no cerebral bleeding or ischemia detectable in imaging) and to the urinary tract infection associated with probably reduced food and fluid intake (clinically: dry tongue). Clinical correlation of the low-grade creatinine increase remained doubtful. The short-term administration of especially ibuprofen (metamizole) in connection with a fluid deficit, could have led to an acute, pre-renal, creatinine increase in this patient (up to that time normal renal function). In the overall context, the creatinine increase had to be considered as "accompanying" according to the reporter. Renal values normalized completely under treatment.

Causality assessment by PV centre (method: WHO score) was changed to "unlikely" for metamizole and "possible" for ibuprofen.

Case was closed by the PV centre, as no further information was expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09102044

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg): 65	BMI_int: 23.59	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20090417	20090709	84 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Dolormin mit Naproxen Tabl, 250 mg ibuprofen lysinate	Oral	/	2002-20090322	Menstrual discomfort
2 Metypred forte 1000mg GALEN methylprednisolone sodium succinate	Intravenous (not otherwis	1000 Mg milligram(s)/Day	20090318-20090320	Multiple sclerosis
3 Schuesslersalze homeopatics nos	Oral	/	3 Day 200902-200903 29 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Multiple sclerosis	200703		k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20090417	1668	U/l	
Alanine aminotransferase increased	20090420	1422	U/l	
Alanine aminotransferase increased	20090430	894	U/l	
Alanine aminotransferase normal	20090709	34	U/l	
Aspartate aminotransferase increased	20090417	1110	U/l	
Aspartate aminotransferase increased	20090419	1212	U/l	
Aspartate aminotransferase increased	20090420	1056	U/l	
Aspartate aminotransferase increased	20090430	672	U/l	
Aspartate aminotransferase normal	20090709	34	U/l	
Bilirubin increased	20090417	9.0	mg/dl	
Bilirubin increased	20090424	21.36	mg/dl	
Bilirubin increased	20090430	16.98	mg/dl	
Bilirubin normal	20090709	1.05	mg/dl	

Narratives (nur bei elektronischem Eingang)

Galenpharma GmbH, 05.08.2009 DE-HRB2237KIP-000041

Sender diagnosis: not reported

Narrative

A 44-year-old woman with multiple sclerosis (MS) suffered from severe acute hepatitis (icterus; ALAT, ASAT, bilirubine strongly increased, "Quick-Wert" significantly decreased) 4 weeks after a pulse therapy with 3 x 1000 mg/3 days methylprednisolone i.v. for acute MS-episode; almost 2 weeks

after the pulse therapy she already suffered from adynamia. Simultaneously she took naproxene (1250 mg on the first day, 750 mg on the second day) because of discomfort with her menstruation.

Two years ago the patient suffered from a similar clinical episode, which was treated as an inpatient for toxic hepatitis.

After a few days the laboratory data improved. Outcome: obviously resolved.<dbtext>To_Char_NARRATIVE_2</dbtext>

- seriousness: serious (medical important, life threatening, prolonged hospitalisation)

- risk-benefit-ratio: unchanged due to this singular case. Relevant papers from the international literature are describing pericentral liver cell necrosis (Lee HM et al., 2007), recurrent acute hepatitis (Das D et al., 2006), toxic hepatitis (Topal F et al., 2006) as isolated, unlisted events due to treatment with methylprednisolone.

GALENpharma will be aware of further comparable cases and looks out for further evidence.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09119854

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19611027	Age: 47 Year	weight (kg): 70	BMI_int: 28.04	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20090625			3 Month	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen AL, 600 mg, Filmtabl, Aliud ibuprofen	Oral	/ 82	20090316-20090619 3 Month	Pain in hand
2 Ibu-ratiopharm, 400 mg, Filmtabl ibuprofen	Oral	1200 Mg milligram(s)/Day 82	20090619-20090629 11 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Complex regional pain syndrome			k.A.	Komplexes regionales Schmerzsyndrom der rechten Hand
Hypertension arterial			k.A.	
Radius fracture	20090301		k.A.	Zustand nach distaler Radiusfraktur
Fractured metacarpal	20090301		k.A.	Zustand nach Fraktur der Ossa metacarpalia IV und V

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase normal	20090619	17	U/l	
Alanine aminotransferase normal	20090629	571	U/l	
Alanine aminotransferase normal	20090701	331	U/l	
Alanine aminotransferase normal	20090702	267	U/l	
Alanine aminotransferase normal	20090706	343	U/l	
Alanine aminotransferase normal	20090708	449	U/l	
Alanine aminotransferase normal	20090714	360	U/l	
Alanine aminotransferase normal	20090716	305	U/l	
Aspartate aminotransferase normal	20090619	23	U/l	
Aspartate aminotransferase increased	20090629	346	U/l	
Aspartate aminotransferase increased	20090701	99	U/l	
Aspartate aminotransferase increased	20090702	92	U/l	
Aspartate aminotransferase increased	20090706	200	U/l	
Aspartate aminotransferase increased	20090708	235	U/l	
Aspartate aminotransferase increased	20090714	140	U/l	
Aspartate aminotransferase increased	20090716	126	U/l	
Bilirubin increased	20090702	2.84	mg/dl	

Bilirubin increased	20090706	2.39	mg/dl
Blood bilirubin normal	20090716	0.93	mg/dl
Blood alkaline phosphatase normal	20090619	83	U/l
Alkaline phosphatase increased	20090701	207	U/l
Alkaline phosphatase increased	20090702	217	U/l
Alkaline phosphatase increased	20090708	189	U/l
Alkaline phosphatase increased	20090716	165	U/l

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 10.09.2009 DE-FAKOS-170287

Sender diagnosis: not reported

Narrative**Sender's comment:**

initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 10.09.2009 DE-FAKOS-170287

Sender diagnosis: not reported

Narrative

Aufnahme der Patientin in ein Krankenhaus am 19.06.2009. Grund waren starke Schmerzen in der rechten Hand bei Zustand nach distaler Radius- und Metacarpale-IV und V-Fraktur nach Unfall und operativer Versorgung in einem anderen Krankenhaus am 01.03.2009. Die Leberwerte bei Aufnahme waren normal: ALAT 17 U/l (Normbereich: <36 U/l), ASAT 23 U/l (Normbereich: <36 U/l), Alkalische Phosphatase (AP) 83 U/l (Normbereich: 36-132 U/l).

Während des Krankenhausaufenthaltes kam es zum Haut- und Sklerenikterus am 25.06.2009 sowie Erhöhung der Leberwerte. Transaminasenwerte: vom 29.06.2009: ALAT 571 U/l, ASAT 346 U/l.

vom 01.07.2009: ALAT 331 U/l, ASAT 99 U/l, AP: 207 U/l,

vom 02.07.2009: ALAT 267 U/l, ASAT 92 U/l, AP: 217 U/l,

vom 16.07.2009: ALAT 305 U/l, ASAT 126 U/l, AP 165 U/l.

Entlassung der Patientin aus dem Krankenhaus am 18.07.2009 in die ambulante Weiterbehandlung. Die UAW hat sich im Krankenhaus entwickelt.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Ibuprofen AL 600 Tabl. (Aliud Pharma) Einnahmezeitraum: 16.03.-18.06.2009 (3x 1 täglich), 19.06.2009: 1 Tabl. und

Ibuprofen-ratiopharm 400 Tabl. (Ratiopharm) Einnahmezeitraum: 19.06.-29.06.2009 (3x 1 täglich).

Die Arzneimittel werden mit "möglich" bewertet, da die Dechallenge-Reaktion keinen stetigen Abfall gezeigt hat und die Werte zuletzt noch deutlich erhöht waren.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09125126

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		

Patient

Initials:	Sex: Male	Birth: 19531124	Age: 55 Year	weight (kg): 70	BMI_int: 21.85	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20090724	20090901	40 Day	4 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibu, 400 mg, Filmtabl, 1A-Pharma ibuprofen	Oral	/Day 82	20090721-20090730 10 Day	Head pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hepatosplenomegaly			Unknown	
Cholecystolithiasis			Unknown	
Allergy			Unknown	Pollinosis (birch, grass), fruit (apples, cherries)

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20090729	361	U/l	
Alanine aminotransferase increased	20090730	356	U/l	
Alanine aminotransferase increased	20090803	128	U/l	
Alanine aminotransferase normal	20090901	22	U/l	
Aspartate aminotransferase increased	20090729	232	U/l	
Aspartate aminotransferase normal	20090901	23	U/l	
Alkaline phosphatase increased	20090729	487	U/l	
Alkaline phosphatase increased	20090730	398	U/l	
Alkaline phosphatase normal	20090901	74	U/l	
Bilirubin normal	20090730	0.53	mg/dl	
Bilirubin normal	20090901	0.42	mg/dl	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 22.09.2009 DE-FAKOS-170296

Sender diagnosis: not reported

Narrative

Aufnahme des Patienten in ein Krankenhaus am 30.07.2009. Grund war ein durch den Hausarzt festgestellter Anstieg der Transaminasen. Laborwerte vom 30.07.2009: ALAT: 356 U/l (Normbereich: <51 U/l), ASAT vom 29.07.2009: 232 U/l (Normbereich: <50 U/l), Alkalische Phosphatase (AP): 398 U/l (Normbereich: 40-129 U/l), Bilirubin: 0,53 mg/dl (Normbereich: <1 mg/dl). Der Patient hatte seit einer Woche bestehende starke Kopfschmerzen, mit Übelkeit einhergehend. Es erfolgte eine Einnahme von Ibuprofen 400 1A-Pharma. Zusätzlich waren im Verlauf wässrige Diarrhoen 3-4-mal pro Tag sowie 2-maliges Erbrechen nach Nahrungsaufnahme aufgetreten.

Serologisch erfolgte der Ausschluss einer viralen Hepatitis. Es wurden LKM-Antikörper vom Nicht-LKM1-Typ festgestellt. Die Leberwerte zeigten sich im Verlauf deutlich fallend. Am 04.08.2009 konnte der Patient in gutem Befinden aus dem Krankenhaus entlassen werden. Laborwerte vom 03.08.2009: ALAT 128 U/l, AP 148 U/l. Am 01.09.2009 konnte vom Hausarzt eine Normalisierung der Laborparameter festgestellt werden: ALAT 22

U/l, ASAT 23 U/l, Bilirubin 0,42 mg/dl, AP 74 U/l.

Die UAW hat sich ambulant entwickelt.

Als Auslöser der Hepatitis wird Ibuprofen verdächtigt: Ibu 400 1A-Pharma Filmtabl (Einnahmezeitraum: 21.- 30.07.2009). Das Arzneimittel wird mit "wahrscheinlich" bewertet.<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 25.09.2009 DE-NOVNCHHQ-GXKR2009DE10518

Sender diagnosis: not reported

Narrative

Initial report received from health authority (BfArM homepage, DE-BFARM-09125126) on 23 Sep 2009: This patient developed acute hepatitis during therapy with Ibu 400 akut 1A Pharma (ibuprofen). Outcome was provided as recovered. Causality was assessed as probable/likely.

Sender's comment:

Serious spontaneous report (hospitalisation) of hepatitis acute, already listed in the Core Data Sheet Information. All spontaneous reports are considered suspected for reporting purposes

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09133958

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: 19711021	Age: 38 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hypertension	2009			6 Month	Unknown
2 Granulocytes abnormal	2009			6 Month	Unknown
3 White blood cell count increased	2009			6 Month	Unknown
4 Thrombocytopenia	2009			6 Month	Unknown
5 Gamma GT increased	2009			6 Month	Unknown
6 Dizziness	2009			6 Month	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ENBREL etanercept	Subcutaneous	2*25 Mg milligram(s)/Week 120	20080627-	Rheumatoid arthritis
2 ibuprofen ibuprofen	Oral	/	-	Rheumatoid arthritis

Drugs (concomitant)

3 MTX methotrexate sodium	Subcutaneous	15 Mg milligram(s)/Week	-	Rheumatoid arthritis
4 PREDNISOLONE prednisolone	Oral	7.5 Mg milligram(s)/Day	-	Rheumatoid arthritis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Baker's cyst			No	right knee (Dec-2007)
Transaminases increased			No	under MTX Hexal
Synoviorthesis			No	chemical synoviorthesis left knee joint (Dec-2007)
Synovectomy			No	Dec-2007

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Blood pressure systolic	2009	220	mmHg	
Neutrophil count	20080317	84.5	%	
Neutrophil count	20080708	93.1	%	
Neutrophil count	20080806	66.6	%	

Narratives (nur bei elektronischem Eingang)

Wyeth-Pharma GmbH, 12.10.2009 DE-WYE-G04541709

Sender diagnosis: not reported

Narrative

This case was considered medically important regarding the events hypertension, dizziness and increased gamma GT. Information was received primarily from a consumer via a healthcare professional regarding a male patient in an Enbrel compliance program (RUDI) who received Enbrel (etanercept injection) therapy and experienced hypertension with systolic blood pressure above 200 mmHg, dizziness, granulocyte count changed, thrombocytopenia, white blood cell count increased and gamma GT increased. The healthcare professional has never seen the patient and received all information by phone only, therefore this case was classified as not medically confirmed.

MEDICAL HISTORY:

Relevant medical history was not provided.

PRODUCT DETAILS:

Indication for Enbrel was rheumatoid arthritis. Therapy began on 27-Jun-2008 and continued. Dose regimen was 25 mg 2 times per week (subcutaneous).

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

Beginning a couple of weeks prior to date of this report the patient experienced hypertension with systolic blood pressure above 220 mmHg (hypertension) accompanied by dizziness (dizziness). He also experienced profound gamma GT increased (gamma-glutamyltransferase increased) with levels above 200 U/L. These events were considered medically important. It was also reported that the patient experienced blood picture abnormalities including thrombocytopenia (thrombocytopenia), white blood cell count increased (white blood cell count increased) and granulocyte count changed (granulocytes abnormal). No specific therapy was initiated. Visits at medical specialists are planned. The outcome was unknown. According to the patient his rheumatologist considered the events not related to Enbrel.

TEST RESULTS:

In 2009 test results were: white blood cell count (results: increased); gamma-glutamyltransferase (results: > 200 U/L); and blood pressure systolic (results: 220 mmHg).

Reporter's comment:

Wyeth-Pharma GmbH, 22.12.2009 DE-WYE-G04541709

Sender diagnosis: not reported

Narrative

Additional information was received from a healthcare professional regarding patient demographics, medical history, product details and lab results. Information was received from a healthcare professional regarding a 38-year-old male patient who received Enbrel (etanercept injection) therapy and experienced hypertension with systolic blood pressure above 200 mmHg, dizziness, granulocyte count changed, thrombocytopenia, white blood cell count increased and gamma GT increased. Initial information had been received from a patient who is participant in an Enbrel compliance program entitled RUDI. Most of event-related information which had been reported by the patient was not confirmed by the physician.

MEDICAL HISTORY:

The patient has a past history of synovial cyst (right knee (Dec-2007)), synovectomy (Dec-2007), synoviorthesis (chemical synoviorthesis left knee joint (Dec-2007)) and transaminases increased (under MTX Hexal). Past therapies included Arava (leflunomide tablet).

PRODUCT DETAILS:

Indication for Enbrel was rheumatoid arthritis. Therapy began on 27-Jun-2008 and continued. Dose regimen was 25 mg 2 times per week (subcutaneous).

Additional suspect medication included Ibuprofen (no Wyeth brand).

CONCOMITANT THERAPY:

Concomitant therapy included Mtx (methotrexate sodium) and Prednisolone.

EVENT DETAILS:

According to the patient he experienced hypertension with systolic blood pressure above 220 mmHg (hypertension) accompanied by dizziness (dizziness) a couple of weeks prior to date of this report. He also experienced profound gamma GT increased (gamma-glutamyltransferase increased) with levels above 200 U/L. These events were considered medically important. It was also reported that the patient experienced blood picture abnormalities including thrombocytopenia (thrombocytopenia), white blood cell count increased (white blood cell count increased) and granulocyte count changed (granulocytes abnormal). No specific therapy was initiated. Visits at medical specialists were planned. The outcome was unknown. According to the patient his rheumatologist considered the events not related to Enbrel. According to reporting rheumatologist symptoms existed in same or greater activity prior to onset of Enbrel therapy, in his opinion they might be related to Ibuprofen therapy (no Wyeth brand) which was most probably discontinued in mid Sep-2009.

TEST RESULTS:

Gamma-glutamyltransferase (results: 199 U/L (normal 12 - 64 U/L)) was done on 28-Jan-2008. Neutrophil count (results: 84.5 %) was done on 17-Mar-2008. White blood cell count (results: 16.9 G/L) was done on 14-May-2008. On 08-Jul-2008 test results were: neutrophil count (results: 93.1 %); gamma-glutamyltransferase (results: 87 U/L); and white blood cell count (results: 16.5 G/L). Neutrophil count (results: 66.6 %) and gamma-glutamyltransferase (results: 43 U/L) were done on 06-Aug-2008. In 2009 test results were: gamma-glutamyltransferase (results: > 200 U/L); white blood cell count (results: increased); and blood pressure systolic (results: 220 mmHg). Gamma-glutamyltransferase (results: 70 U/L) and white blood cell count (results: 13.2 G/L) were done on 21-Aug-2009. Additionally, white blood cell count (results: elevated prior to Enbrel therapy) and platelet

count (results: within normal limits from Dec-2007 to Jun-2008 and Jul-20089 to 21-Aug-2008) were done on an unspecified date.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09174811

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19801029	Age: 28 Year	weight (kg): 77	BMI_int: 21.56	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20090922			30 Day	recovered/resolved
2 Abdominal pain	20090922			30 Day	recovered/resolved
3 Allergic reaction	20090919			26 Day	Unknown
4 Nausea	20090922			30 Day	recovered/resolved
5 Vomiting	20090922			30 Day	recovered/resolved
6 Gastritis	20090922			3 Day	Unknown
7 Elevated liver enzymes	20090922			3 Day	Unknown
8 GGT increased	20090922			30 Day	recovered/resolved
9 Transaminases increased	20090922			30 Day	recovered/resolved
10 Aspartate aminotransferase increased	20090922			30 Day	recovered/resolved
11 Alanine aminotransferase increased	20090922			30 Day	recovered/resolved
12 General physical health deterioration	20090922			30 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Grippostad C, Hartkps, STADA	Oral	6 Df dosage form/Day	20090902-20090905	Flu symptoms
caffeine; chlorphenamine maleate; paracetamol		6	4 Day	
2 IBUPROFEN	Oral	600 Mg milligram(s)/Day	20090824-20090831	Flu symptoms
ibuprofen		245	8 Day	
3 Ibuprofen, 200 mg, Tabl, NN in Spanien	Oral	600 Mg milligram(s)/Day	20090824-20090831	
ibuprofen			8 Day	
4 PANTOZOL		/	-	Gastritis
pantoprazole			-	
5 Paracetamol, Tabl, NN in Spanien	Oral	/	20090824-20090831	Flu symptoms
paracetamol			8 Day	
6 Reactine, 10 mg, Filmtabl, Mc Neil	Oral	30 Mg milligram(s)/Day	20090919-20090921	Allergic to cats
cetirizine		82	3 Day	

Drugs (concomitant)

7 SYMBICORT TURBUHALER "DRACO"	Respiratory (inhalation)	2 Df dosage form/Day	20090701-	Allergic asthma
budesonide; formoterol		110	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pollinosis			k.A.	
Allergic asthma			Unknown	
Allergy			Unknown	animal hair
Gastritis erosive			k.A.	
House dust allergy			k.A.	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Alanine aminotransferase increased	20090922	305	U/l
Alanine aminotransferase increased	20090923	264	U/l
Alanine aminotransferase increased	20091001	241	U/l
Alanine aminotransferase increased	20091029	337	U/l
Alanine aminotransferase increased	20091118	59	U/l
Aspartate aminotransferase increased	20090922	99	U/l
Aspartate aminotransferase increased	20090923	83	U/l
Aspartate aminotransferase increased	20091001	60	U/l
Aspartate aminotransferase increased	20091029	463	U/l
Aspartate aminotransferase normal	20091118	41	U/l
Bilirubin normal	20090922	0.53	mg/dl
Bilirubin normal	20090923	0.62	mg/dl
Alkaline phosphatase normal	20090922	116	U/l
Alkaline phosphatase normal	20090923	102	U/l
Alkaline phosphatase normal	20091001	77	U/l
Alkaline phosphatase normal	20091118	84	U/l
Alanine aminotransferase	20091001	241	U/L
Aspartate aminotransferase	20091118	41	U/L
GGT	20090922	63	U/L
Laboratory test	20090922	116	U/L
Laboratory test	20091118	84	U/L

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 30.12.2009 DE-FAKOS-170273

Sender diagnosis: not reported

Narrative**Sender's comment:**

initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 09.02.2010 DE-FAKOS-170273

Sender diagnosis: not reported

Narrative

Während eines Aufenthaltes in Spanien (bis Ende August 2009) waren am 20.08.2009 grippale Symptome mit subfebrilen Temperaturen und Gliederschmerzen, sowie eine allgemeine Schwäche aufgetreten. Es erfolgte eine Selbstmedikation mit Paracetamol, Ibuprofen sowie Grippostad C. Die grippalen Erscheinungen waren Anfang September 2009 wieder abgeklungen. Dann traten am 19.09.2009 allergische Symptome in den oberen Luftwegen auf, weswegen der Patient Reactine für 3 Tage einnahm. Am 22.09.2009 wurde er stationär aufgenommen wegen Verschlechterung des Allgemeinzustandes mit Übelkeit, Erbrechen und Bauchschmerzen. Laborchemisch konnten deutlich erhöhte Transaminasen festgestellt werden (ALAT 305 U/l (Normbereich: <41 U/l), ASAT 99 U/l (Normbereich: <37 U/l)). Weitere Laborwerte vom 22.09.2009: GGT (Gamma-Glutamyl-Transferease) 63 U/l (Normbereich: 8-61 U/l), AP (Alkalische Phosphatase) 116 U/l (Normbereich: 40-260 U/l), Bilirubin gesamt 0,53 mg/dl (Normbereich: <1,2 mg/dl). Am 23.09.2009 wurde der Patient bei Wohlbefinden aus dem Krankenhaus entlassen. Im weiteren Verlauf fielen die

Transaminasen zunächst wieder ab (Laborwerte vom 01.10.2009: ALAT 241 U/l, ASAT 60 U/l). Unter einer ambulanten Medikation mit Pantozol zur Behandlung einer gastroscopisch nachgewiesenen erosiven Gastritis kam es zu einem erneuten Anstieg der Aminotransferasen (Laborwerte vom 29.10.2009: ALAT 337 U/l, ASAT 463 U/l, außerdem GGT 44 U/l. Nach dem Absetzen von Pantozol besserten sich die Werte wieder (18.11.2009: ALAT 59 U/l, ASAT 41 U/l, GGT 30 U/l, AP 84 U/l). Patient war beschwerdefrei und in gutem Allgemeinzustand.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Reactine 10 mg Filmtabl, Mc Neil, Einnahmezeitraum: 19.-21.09.2009, Grippostad C Kps, STADA, Einnahmezeitraum: 02.-05.09.2009 sowie vorher Paracetamol Tabl NN (in Spanien), Einnahmezeitraum: 24.-31.08.2009 und Ibuprofen 200 mg Tabl, NN (in Spanien), Einnahmezeitraum: 24.-31.08.2009.

Gesamtbeurteilung:

Eine medikamenteninduzierte Genese der Hepatitis ist möglich.

Möglicherweise erlitt der Patient Ende August 2009 einen viralen Infekt mit den oben genannten Symptomen, in dessen Rahmen es auch zu einer Leberbeteiligung gekommen sein könnte. Entsprechende Infektionsserologie wurde aktuell nicht durchgeführt, eine Leberpunktion ebenfalls nicht. Sonographisch war die Leber unauffällig. Dennoch könnte auch eine medikamentenverursachte Hepatitis vorgelegen haben. Hier kommt am ehesten Reactine in Frage, aber aufgrund des zeitlichen Zusammenhangs auch Ibuprofen und Paracetamol bzw. Grippostad C.

Die Arzneimittelkausalität der genannten Arzneistoffe wird mit "möglich" bewertet.

Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 19.02.2010 DE-SANOFI-AVENTIS-2010SA009043

Sender diagnosis: not reported

Narrative

Initial report:

This serious case was extracted from the website of the German Health Authorities (id no. DE-BFARM-09174811). Initially the case was reported from a German physician via a Regional pharmacovigilance centre (id no. DE-FAKOS-170273). The case involves a 28-year-old male patient with an allergy due to house dust, animal hair and pollinosis (gras) as well as allergic asthma. On 20-Aug-2009 the patient suffered from influenza-like symptoms with subfebrile temperature and joint pain as well as general malaise during a detention in Spain. Therefore he started self-medication on 24-Aug-2009 with ibuprofen (no trade name provided) 600 mg daily and paracetamol (no trade name provided). Therapy with ibuprofen and paracetamol was stopped on 31-Aug-2009 and the patient started therapy with caffeine/chlorphenamine maleate/paracetamol six dosage forms daily on 02-Sep-2009. Influenza-like symptoms were recovered at the beginning of September 2009. Therefore therapy with caffeine/chlorphenamine maleate/paracetamol was stopped on 05-Sep-2009. On 19-Sep-2009 the patient developed an allergic reaction of the upper respiratory tract. He received cetirizine 30 mg daily from 19-Sep-2009 to 21-Sep-2009. On 22-Sep-2009 he was hospitalized due to aggravation of general condition with nausea and vomiting as well as abdominal pain. During hospitalization increased values for transaminases were mentioned. Diagnosis: acute hepatitis. On 23-Sep-2009 the patient was discharged from the hospital in good general condition. In the further course transaminases decreased. During an ambulant treatment with pantoprazole sodium (from another company) due to erosive gastritis (proven in a gastroscopy) aminotransferases increased again. Therefore pantoprazole sodium was withdrawn and values decreased. Outcome of acute hepatitis: recovering/resolving.

Causality assessment according Sender (method: Global introspection (WHO GI)): possible for all suspected products.

NOVARTIS, 25.02.2010 DE-002147023-CHPA2010DE03101

Sender diagnosis: not reported

Narrative

Initial health authority report received 12 Feb 2010 from a physician, which was forwarded to the Regional Pharmacovigilance Center (case number: DE-BFARM-09174811). This 28 year-old male patient, with a history of allergies to house dust, animal hair, and grass, developed flu-like symptoms on 20 Aug 2009, with subfebrile temperature, limb pain and general impairment. He received treatment with the following medications: Paracetamol (acetaminophen) from 24 to 31 Aug 2009 (unknown dose), Ibuprofen (ibuprofen) from 24 to 31 Aug 2009, 200mg, orally, 1 tablet 3 times daily, and Grippostad C (paracetamol, caffeine, chlorphenamine maleate) 02 to 05 Sep 2009, orally, 2 DF three times daily. The flu-like symptoms abated completely at the beginning of Sep 2009. On 19 Sep 2009, the patient developed allergic symptoms of the upper respiratory tract, due to an allergy to cats, and therefore used Reactine (cetirizine), 10mg, 1 DF, three times daily, orally, for 3 days. On 22 Sep 2009, the patient was hospitalized due to aggravation of general condition, with nausea, vomiting, and abdominal pain, and acute hepatitis. The clinical chemistry showed significantly increased transaminases (ALAT: 305 U/l, normal range: < 41 U/l; ASAT 99 U/l, normal range: <37 U/l). Additional lab findings on 22 Sep 2009: GGT (gamma-glutamyltransferase) 63 U/l, normal range: 8-61 U/l; AP (Alkaline phosphatase) 116 U/l, normal range: 40-260 U/l; Bilirubin total 0.53 mg/dl, normal range: <1.2 mg/dl. On 23 Sep 2009, the patient was discharged from the hospital in good condition. The transaminases dropped down (lab values from 01 Oct 2009: ALAT 241 U/l, ASAT 60 U/l). During an ambulant treatment with Pantozol (pantoprazole) (date unknown), dosage unspecified, to treat erosive gastritis, as diagnosed by a gastroscopy (date unspecified), the patient again developed increased transaminase (lab values from 29 Oct 2009: ALAT 337 U/l, ASAT 463 U/l, additionally GGT 44 U/l). Lab values resolved (lab values from 18 Nov 2009: ALAT 59 U/l, ASAT 41 U/l, GGT 30 U/l, AP 84 U/l) after treatment with Pantozol was discontinued. The patient was symptom free and in good general condition. As trigger for the hepatitis, the following medicinal products were suspected: Reactine, Grippostad C, Paracetamol, and Ibuprofen.

The available information was considered inadequate to fully assess the case.

Reporter's comment:GlaxoSmithKline, 26.02.2010 DE-GlaxoSmithKline-B0636094A

Sender diagnosis: not reported

Narrative

This case was reported by a regulatory authority, via UCB and described the occurrence of acute hepatitis in a 28-year-old male patient who received Cetirizine hydrochloride (Reactine) over a period of 4 days for allergy.

Co-suspect medication included Paracetamol, Ibuprofen and Grippostad.

On 19 September 2009 the patient started Cetirizine hydrochloride (oral). Approximately 3 days later, on 22 September 2009, the patient was hospitalized due to nausea, vomiting, abdominal pain and decreased general health. On the same day, increased liver enzyme activity was found: Alanine transaminase (ALAT) 305 U/L (normal range less than 41 U/L), aspartate aminotransferase (ASAT) 99 U/L (normal range less than 37 U/L) and additional laboratory results were also noted as: gamma-glutamyl transpeptidase (gGT) 63 U/L (normal range 8-61 U/L), alkaline phosphatase (aP) 116 U/L (normal range less than 40-260 U/L), total bilirubin 0.53 mg/dl (normal range less than 1.2 mg/dl). Treatment with Cetirizine hydrochloride was discontinued on 22 September 2009. It was unspecified whether cetirizine hydrochloride was discontinued due to the events. On 23 September 2009, the patient was discharged in good health. On 01 October 2009, laboratory results noted ALAT 241 U/L, ASAT 60 U/L. The patient received outpatient treatment with pantoprazol due to gastritis. Liver enzymes again increased. On 29 October 2009, laboratory values were reported as ALAT 337 U/L, ASAT 463 U/L, gGT 44 U/L. After pantoprazol was stopped, the liver enzyme activity decreased again. On 18 November 2009, laboratory results noted: ALAT 59 U/L, ASAT 41 U/L, gGT 30 U/L, aP 84 U/L. The patient recovered on an unknown date and was in good health. On an unknown date, a liver ultrasound had a normal appearance. The regulatory authority reported that the events were clinically significant (or requiring intervention). At the time of reporting, the events were resolved. The regulatory authority reported that the events were possibly related to treatment with Cetirizine hydrochloride. It was reported that the causal relationship of the event to the four suspect drugs was determined to be possibly.

GlaxoSmithKline, 01.03.2010 DE-GlaxoSmithKline-B0636094A

Sender diagnosis: not reported

Narrative

This case was reported by a regulatory authority, via UCB and described the occurrence of acute hepatitis in a 28-year-old male patient who received Cetirizine hydrochloride (Reactine) over a period of 4 days for allergy.

Co-suspect medication included Paracetamol, Ibuprofen and Grippostad.

On 19 September 2009 the patient started Cetirizine hydrochloride (oral). Approximately 3 days later, on 22 September 2009, the patient was hospitalized due to nausea, vomiting, abdominal pain and decreased general health. On the same day, increased liver enzyme activity was found: Alanine transaminase (ALAT) 305 U/L (normal range less than 41 U/L), aspartate aminotransferase (ASAT) 99 U/L (normal range less than 37 U/L) and additional laboratory results were also noted as: gamma-glutamyl transpeptidase (gGT) 63 U/l (normal range 8-61 U/l), alkaline phosphatase (aP) 116 U/l (normal range less than 40-260 U/l), total bilirubin 0.53 mg/dl (normal range less than 1.2 mg/dl). Treatment with Cetirizine hydrochloride was discontinued on 22 September 2009. It was unspecified whether cetirizine hydrochloride was discontinued due to the events. On 23 September 2009, the patient was discharged in good health. On 01 October 2009, laboratory results noted ALAT 241 U/l, ASAT 60 U/l. The patient received outpatient treatment with pantoprazol due to gastritis. Liver enzymes again increased. On 29 October 2009, laboratory values were reported as ALAT 337 U/l, ASAT 463 U/l, gGT 44 U/l. After pantoprazol was stopped, the liver enzyme activity decreased again. On 18 November 2009, laboratory results noted: ALAT 59 U/l, ASAT 41 U/l, gGT 30 U/l, aP 84 U/l. The patient recovered on an unknown date and was in good health. On an unknown date, a liver ultrasound had a normal appearance. The regulatory authority reported that the events were clinically significant (or requiring intervention). At the time of reporting, the events were resolved. The regulatory authority reported that the events were possibly related to treatment with Cetirizine hydrochloride. It was reported that the causal relationship of the event to the four suspect drugs was determined to be possibly.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10002875

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials: Sex: Female Birth: 19580203 Age: 51 Year weight (kg): 75 BMI_int: 26.57 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis cholestatic	20090930	20091022	23 Day		not recovered/not resolved
2 Drug abuse					Unknown
3 Drug interaction					Unknown
4 Icterus					Unknown
5 Pruritus					Unknown
6 Reduced general condition	20090930				recovered/resolved with sequel
7 Scleral icterus					Unknown
8 Liver cirrhosis					Unknown
9 Cirrhosis of liver					recovered/resolved with sequel
10 Drug interaction					Unknown
11 Fatty liver					recovered/resolved with sequel
12 Hepatitis cholestatic	20090930	20091022	23 Day		recovered/resolved with sequel
13 Hepatomegaly					recovered/resolved with sequel
14 Skin coloring yellow	20090930				recovered/resolved with sequel
15 Scleral icterus	20090930				recovered/resolved with sequel
16 Metabolic disorder					recovered/resolved with sequel
17 Gallbladder enlargement					recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibu-ratiopharm 500 lysinat schmerztabletten ibuprofen	Oral	3000 Mg milligram(s)/Day 82	20090924-20091001 8 Day	Intervertebral disc disorder
2 PARACETAMOL paracetamol	Oral	6*500 Mg milligram(s)/4 D 245	1999-20090923 10 Year	Intervertebral disc disorder
3 RASILEZ HCT aliskiren fumarate; hydrochlorothiazide	Oral	1 Df dosage form/Day 245	-20091001 -	Hypertension arterial

Drugs (concomitant)

4 PANTOZOL pantoprazole sodium	Oral	80 Mg milligram(s)/Day 94	2000- -	Gastric ulcer prophylaxis
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Yes	Beginn unbekannt
Polydrug abuse	1999		Yes	Paracetamol (seit 1999 alle 4h 1 Tbl. á 500mg), Ibuprofen (seit 09/09: 40 Tbl. in 7 Tagen á 500mg)
Alcohol abuse	1991	2002	Yes	alcohol abuse
Alcohol use	2002		Yes	gelegentlich (zu Feiern) 1 Glas Wein
Helicobacter gastritis			Unknown	chronic active helicobacter-pylori-gastritis
Duodenal ulcer	2000	2000	Unknown	Beginn unbekannt
Intervertebral disc disorder	1999		Yes	
Axial hiatal hernia			Unknown	axial hiatal hernia
Reflux esophagitis			Unknown	peptic esophagitis II
Smoker			Yes	Beginn unbekannt, 25py, seit 3 Jahren 13 Zigaretten pro Tag

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			
Qualification:	Pharmacist	observestudytype:	Other studies
Literature:			

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20091006	0.83 umol/*s	NA	
Alanine aminotransferase	20091026	49.8 U/l	NA	
Alanine aminotransferase	20091022	46.5 U/l	NA	
Alanine aminotransferase	20091014	41.6 U/l	NA	
Alanine aminotransferase	20091006	45.4 U/l	NA	
Alanine aminotransferase	20091001	0.93 umol/*s	NA	
Alanine aminotransferase	20091005	0.77 umol/*s	NA	
Alanine aminotransferase	20091004	0.75 umol/*s	NA	
Alanine aminotransferase	20091003	0.77 umol/*s	NA	
Alanine aminotransferase	20091002	0.79 umol/*s	NA	
Alkaline phosphatase	20091022	160 U/l	NA	
Alkaline phosphatase	20091014	202 U/l	NA	
Alkaline phosphatase	20091006	205 U/l	NA	
Alkaline phosphatase	20091026	143 U/l	NA	
Alkaline phosphatase	20091003	NA	NA	
Alkaline phosphatase	20091002	NA	NA	
Alkaline phosphatase	20091001	NA	NA	
Alkaline phosphatase	20091001	3.50 SI-Einheit	NA	
Alkaline phosphatase	20091002	3.03 SI-Einheit	NA	
Alkaline phosphatase	20091003	NA	NA	
Alkaline phosphatase	20091005	3.61 SI-Einheit	NA	
Alkaline phosphatase	20091006	4.03 SI-Einheit	NA	
Alkaline phosphatase	20091004	3.29 SI-Einheit	NA	

Ammonia	20091001	66 umol/l	NA
Aspartate aminotransferase	20091005	2.69 umol/*s	NA
Aspartate aminotransferase	20091006	204 U/l	NA
Aspartate aminotransferase	20091014	145 U/l	NA
Aspartate aminotransferase	20091022	211 U/l	NA
Aspartate aminotransferase	20091026	231 U/l	NA
Aspartate aminotransferase	20091006	3.26 umol/*s	NA
Aspartate aminotransferase	20091004	2.53 umol/*s	NA
Aspartate aminotransferase	20091003	2.12 umol/*s	NA
Aspartate aminotransferase	20091002	1.98 umol/*s	NA
Aspartate aminotransferase	20091001	2.29 umol/*s	NA
Bilirubin direct	20091003	155.7 umol/l	NA
Bilirubin direct	20091001	182.1 umol/l	NA
Bilirubin direct	20091002	146.1 umol/l	NA
Bilirubin direct	20091006	281.2 umol/l	NA
Bilirubin direct	20091005	214.9 umol/l	NA
Bilirubin direct	20091004	151.9 umol/l	NA
Bilirubin total	20091007	NA	NA
Bilirubin total	20091013	NA	NA
Bilirubin total	20091006	298.6 ?mol/l	NA
Bilirubin total	20091020	198 ?mol/l	NA
Bilirubin total	20091022	164 ?mol/l	NA
Bilirubin total	20091026	119 ?mol/l	NA
Bilirubin total	20091014	329 ?mol/l	NA
Bilirubin total	20091005	230.4 ?mol/l	NA
Bilirubin total	20091004	193.6 ?mol/l	NA
Bilirubin total	20091003	161.9 ?mol/l	NA
Bilirubin total	20091002	161.8 ?mol/l	NA
Bilirubin total	20091001	193.1 ?mol/l	NA
Biopsy liver	20091013	NA	NA
Blood bilirubin indirect	20091006	126 umol/l	NA
Chest X-ray	20091001	NA	NA
C-reactive protein	20091026	22.2 mg/L	NA
C-reactive protein	20091022	28.2 mg/L	NA
C-reactive protein	20091020	33.9 mg/L	NA
C-reactive protein	20091001	2.3 mg/L	NA
C-reactive protein	20091016	56.7 mg/L	NA
CT scan	20091022	NA	NA
ECG	20091001	NA	NA
Gamma GT	20091022	268 U/l	NA
Gamma GT	20091004	NA	NA
Gamma GT	20091001	11.66 umol/l	NA
Gamma GT	20091026	272 U/l	NA
Gamma GT	20091003	NA	NA

Gamma GT	20091014	243 U/l	NA
Gamma GT	20091006	363 U/l	NA
Gamma GT	20091007	NA	NA
Gamma GT	20091006	6.37 umol/l	NA
Gamma GT	20091005	6.06 umol/l	NA
Gamma GT	20091002	9.73 umol/l	NA
Gastroscopy	20091007	NA	NA
LDH	20091006	282 U/L	NA
Ultrasonography	20091001	NA	NA
Ultrasonography	20091007	NA	NA

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 13.01.2010 DE-PVZIBE-02-10236

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

cholestatiche Hepatitis mit beginnendem zirrhotischen Umbau

KURZZUSAMMENFASSUNG:

alkohol- und medikamententoxische cholestatiche Hepatitis mit beginnendem zirrhotischen Umbau durch Paracetamol- und Ibuprofenabusus, Rasilez und HCT könnten interagierend gewirkt haben

BESCHREIBUNG:

seit 1999 bis 23.09.2009 am Tag alle 4 Stunden 1 Tablette Paracetamol á 500mg wegen Bandscheibenproblemen, danach 40 Tabletten Ibuprofen á 500mg in 7 Tagen;

am 30.09.2009 Gelbfärbung der Haut, die bis 01.10.2009 zugenommen hatte, daraufhin Hausarzt aufgesucht, Klinikeinweisung (01.-06.10.2009 peripheres Krankenhaus) mit reduziertem Allgemeinzustand und deutlichem Haut- und Sklerenikterus

THERAPIE:

zuweisende Klinik: Vit.-B-duo, Vit.-K. intravenös, fettfreie Kost, daraufhin sanken die Leberwerte, seit 04.10.2009 stiegen diese jedoch wieder (daraufhin Verlegung ins tertiäre Krankenhaus);

tertiäres Krankenhaus: Vit.-K, Vit.-B1 und Vit.-B6, Fenistil und Cholestyramin gegen Juckreiz;

Paracetamol abgesetzt am 23.09.09; Ibuprofen abgesetzt am 01.10.09

unter der Therapie sinkende Leber- und Bilirubinwerte

VERLAUF:

01.10.2009 EKG: Sinustachykardie, unauffälliger Erregungsablauf;

01.10.2009 Röntgen-Thorax: unauffällig;

Sonographie Abdomen: massive Hepatomegalie mit Zirrhosezeichen, kein Aszitis, keine Cholestasezeichen;

07.10.2009 Autoimmundiagnostik negativ: ANA=1:320, AMA=1:320, AMA-M2 grenzwertig, 3E(BPO) AK positiv (als Abbaustörung aufgrund der Leberinsuffizienz interpretiert), negativ: Ros2-AK, SP-100-AK, PML-AK, gp210-AK, LKM1-AK, LC1-AK, SLA/LP, PCA-AK; intrinsikfaktor-AK, IgA-AK (ASCA), IgG-AK (ASCA);

07.10.2009 Virologie: HBS-AK und HCV-IgG/M: keine Infektion; HA-IgM/G: entweder vor Jahren durchgemachte Erkrankung oder Antikörper nach Impfung;

07.10.2009 Sonographie Abdomen: Hepatomegalie mit Leberparenchymveränderungen wie bei Fettleberhepatitis, Gallenblase vergrößert mit betonter Gallenblasenwand als Begleitödem bei Hepatitis;

07.10.2009 Gastroskopie: Refluxösophagitis II° bei axialer Hiatushernie, Gastritis, Histologie: geringgradige chronisch aktive Helicobacter-pylori-Gastritis von Antrum und Korpus, kein Anhalt für Malignität;

13.10.2009 Biopsie rechter Leberlappen: Histologie: mäßig aktive (alkoholische) Fettleberhepatitis mit beginnendem zirrhotischem Umbau und Cholestasezeichen, kombinierte toxische Leberschädigung mit zusätzlichen medikamentösen Noxen, kein Anhalt für Malignität und Spezifität

16.10.2009 Röntgen-Thorax: unauffällig;

22.10.2009 CT-Thorax: vergrößerte Leber, ansonsten unauffällig

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 06.-27.10.2009;

Krankenseinweisung aufgrund von Ikterus und reduziertem Allgemeinzustand bei hepatischen Veränderungen mit beginnendem zirrhotischem Umbau;

Differentialdiagnostisch: Autoimmundiagnostik negativ, Gallenblase unauffällig, Virologie unauffällig;

Entlassungsbrief: "Diagnose einer mäßig aktiven alkoholischen Fettleberhepatitis mit beginnendem zirrhotischen Umbau- und Cholestasezeichen, eventuell auch mit zusätzlicher Schädigung durch medikamentöse Noxen, so dass in Zusammenschau aller Befunde, der chronische Alkoholabusus der Patientin, sowie die zuletzt massive Einnahme von Schmerzmedikamenten als Ursachen für die beginnende Leberzirrhose anzusehen sind." ; kein Anhalt für akuten Alkoholexzeß und Atemwegsinfekt;

In der Zusammenschau ist eine chronische Leberparenchymschädigung auf dem Boden eines chronischen Alkoholkonsums (anamnestisch bis 2002) sowie 10jähriger chronischer Paracetamoleinnahme als möglich zu erachten. Beide Noxen stellen auch für sich alleine eine mögliche Ursache dar -

die Toizität von Paracetamol ist bei begleitendem Alkoholkonsum erhöht. Langjährige hepatitische Schäden könnten den beginnenden zirrhotischen Umbau bedingt haben. Akuter Auslöser könnte möglicherweise die erhöhte Ibuprofeneinnahme darstellen.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: rasilez hct

BETREFFEN<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 18.01.2010 DE-SANOFI-AVENTIS-2010SA002246

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10002875).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the WHO score.

Short summary: alcohol- and drug-toxic cholestatic hepatitis with beginning cirrhotic conversion by paracetamol and ibuprofen abuse.

Aliskiren+hydrochlorothiazide could have an interactive effect.

This case involves a 51-year-old female (smoker, body weight 75 kg) with a past medical history of alcohol abuse from 1999 to 2002 (since 2002 only occasionally one glass of wine) and duodenal ulcer; at time of event, the patient was suffering from polydrug abuse (paracetamol and ibuprofen), arterial hypertension, and intervertebral disc disorder. The following drug was reported to have been taken concomitantly: pantoprazole.

The patient started treatment with paracetamol (trade name not ascertainable) 500mg every four hours in 1999 for intervertebral disc disorder. In Sep.2009, she additionally initiated treatment with ibuprofen (trade name not ascertainable) 40 tablets in seven days.

On 30-Sep-2009, the patient observed yellow colouration of her skin that increased on the following day, so that the patient went to her family doctor.

The patient was hospitalised in a peripheral hospital with reduced general condition and pronounced icterus of skin and sclerae. She was treated with vitamin B1+B6, intravenous vitamin K, and fat-free diet, leading to a decrease in liver values. On 04-Oct-2009, the values started to increase again, so that the patient was transferred to a tertiary hospital on 06-Oct-2009. Here she was treated with vitamin K, vitamin B1 and B6, and dimetindene maleate and colestyramine for pruritus. Under treatment, liver and bilirubin values decreased. Paracetamol was discontinued on 23-Sep-2009, ibuprofen was discontinued on 01-Oct-2009. Autoimmune diagnostics were negative, virology was unremarkable, gallbladder was unremarkable.

Outcome was reported as "recovered/resolved with sequel" on 22-Oct-2009. The patient was discharged from hospital on 27-Oct-2009. Discharge letter read: "diagnosis of a moderately active alcoholic fatty liver hepatitis with signs of beginning cirrhotic conversion and cholestasis, maybe also with additional damage by medicamentous noxae, so that, as summary of all findings, the patient's chronic alcohol abuse as well as the at last massive intake of analgesics could be considered as causes of the beginning liver cirrhosis." There was no indication of acute alcoholic excess or respiratory tract infection. As synopsis, a chronic damage of liver parenchyma on the base of chronic alcohol consumption and 10 years of chronic paracetamol intake has to be considered possible. Each noxa by itself can also be regarded as possible cause - toxicity of paracetamol is increased under additional intake of alcohol. Long years of hepatic damage could have led to beginning cirrhotic conversion. Acute catalyst could have possibly been the increased ibuprofen intake.

Causality determined by PV centre:

"possible" for paracetamol, ibuprofen, and aliskiren+hydrochlorothiazide (interacting).

Comments concerning causality: Increased liver enzymes are a described adverse effect of paracetamol. In connection with alcohol, liver damage can occur. The long-term, regular high-dose intake could have caused chronic hepatic damages, that could have led to beginning cirrhotic conversion.

Liver function disorders, liver damage, and liver failure (especially under long-term therapy) and acute hepatitis are described adverse effects of ibuprofen. Drugdex:cases of icterus are described after oral treatment, hepatotoxicity after three days, at the earliest. Icterus is described as adverse effect of aliskiren+hydrochlorothiazide, the chronologic element is remote, there is a possibility of interaction with paracetamol and ibuprofen.

Possible alternative cause: chronic alcohol consumption in past medical history (acute alcohol excess was negated by patient).

Bayer Vital GmbH, 21.01.2010 DE-BAYER-201011670GPV

Sender diagnosis: not reported

Narrative

This solicited report was forwarded to the German Health Authority from a pharmacist in Germany via a pharmacovigilance center, which systematically screens for adverse drug reactions in certain hospitals. The report was identified on the ADR-internet page of the German Health Authority (BfArM) on 20-Nov-2009, referring to a 51 year-old female (height: 168 cm, weight: 75 kg).

Medical and drug history:

Patient's concomitant conditions included smoking, alcohol abuse until 2002, then alcohol use continuing, and arterial hypertension. Patient had a duodenal ulcer until 2000. Concomitant drugs have been given. For details see section "Concomitant Drugs".

Patient received the following drugs, reported as 'suspect drugs':

-oral Paracetamol (trade name unknown) 500 mg was taken every 4 hours at a total daily dose of 3000 mg (6 tablets) a day for intervertebral disc disorder from 1999 to 23-Sep-2009,

-oral Ibuprofen (trade name unknown) 500 mg film-coated tablet was taken thrice daily at a total dose of 40 tablets within 7 days for intervertebral disc disorder from 24-Sep-2009 until 01-Oct-2009,

-oral Rasilez (aliskiren/ hydrochlorothiazide) 40 mg film-coated tablet was taken twice daily for arterial hypertension from unspecified start date until 01-Oct-2009.

Onset, progression and outcome of the event:

On 30-Sep-2009 skin icterus, increasing until 01-Oct-2009. Consultation of family physician; hospitalization (01-Oct-2009 to 06-Oct-2009, and 06-Oct-2009 to 27-Oct-2009) with reduced general condition and significant scleral and skin icterus. Tests showed CHOLESTATIC HEPATITIS WITH BEGINNING CIRRHOTIC ALTERATIONS (see 'Other Tests' and 'Lab Test' sections).

Therapy: Vitamin B duo, Vitamin K intravenously, greaseless nutrition. Laboratory values dropped, but raised again on 04-Oct-2009. Therapy was continued with Vitamin K, B1 and B6, Fenistil/ dimetindenmaleate and cholestyramin against pruritus. Paracetamol was withdrawn on 23-Sep-2009, Ibuprofen on 01-Oct-2009. Undertherapy dropping liver and BILI values. At the time of the report, patient has recovered with sequelae.

Reporter's causality assessment:

Alcohol- and drug induced CHOLESTATIC HEPATITIS WITH BEGINNING CIRRHOTIC ALTERATIONS caused by Paracetamol- and Ibuprofen abuse. Rasilez HCT (aliskiren/ hydrochlorothiazide) could have been interacting.

Comment: Chronic abuse of alcohol and intake of pain-reliever were considered as cause of beginning livercirrhosis. No signs of acute alcohol excess nor respiratory tract infection. In synopsis, chronic hepatosis was possibly caused by alcohol abuse (until 2002) and 10 years of intake of paracetamol. However, both noxa could be causes on their own.

Toxicity of paracetamol is increased with concomitant consumption of alcohol. Long- time damage of liver could have caused cirrhotic alteration. Acute trigger could have been intake of ibuprofen.

HELIOS Klinikum Wuppertal, 22.01.2010 DE-PVZIBE-02-10236

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

cholestatiche Hepatitis mit beginnendem zirrhotischen Umbau

KURZZUSAMMENFASSUNG:

alkohol- und medikamententoxische cholestatiche Hepatitis mit beginnendem zirrhotischen Umbau durch Paracetamol- und Ibuprofenabusus (Aliskiren/HCT; Pantoprazol)

BESCHREIBUNG:

seit 1999 bis 23.09.2009 am Tag alle 4 Stunden 1 Tablette Paracetamol á 500mg wegen Bandscheibenproblemen, danach 40 Tabletten Ibuprofen á 500mg in 7 Tagen; bis 2002 chronischer Alkoholkonsum

am 30.09.2009 Gelbfärbung der Haut, die bis 01.10.2009 zugenommen hatte, daraufhin Hausarzt aufgesucht, Klinikeinweisung (01.-06.10.2009 peripheres Krankenhaus; 06.10. bis 27.10.2009 tertiäres Krankenhaus) mit reduziertem Allgemeinzustand und deutlichem Haut- und Sklerenikterus

THERAPIE:
zuweisende Klinik: Vit.-B-duo, Vit.-K. intravenös, fettfreie Kost, daraufhin sanken die Leberwerte, seit 04.10.2009 stiegen diese jedoch wieder (daraufhin Verlegung ins tertiäre Krankenhaus);

tertiäres Krankenhaus: Vit.-K, Vit.-B1 und Vit.-B6, Fenistil und Cholestyramin gegen Juckreiz;

Paracetamol abgesetzt am 23.09.09; Ibuprofen abgesetzt am 01.10.09

unter der Therapie sinkende Cholestasewerte

VERLAUF:

01.10.2009 EKG: Sinustachykardie, unauffälliger Erregungsablauf;

01.10.2009 Röntgen-Thorax: unauffällig;

Sonographie Abdomen: massive Hepatomegalie mit Zirrhosezeichen, kein Aszitis, keine Cholestasezeichen;

07.10.2009 Autoimmundiagnostik negativ: ANA=1:320, AMA=1:320, AMA-M2 grenzwertig, 3E(BPO) AK positiv (als Abbaustörung aufgrund der Leberinsuffizienz interpretiert), negativ: Ros2-AK, SP-100-AK, PML-AK, gp210-AK, LKM1-AK, LC1-AK, SLA/LP, PCA-AK; intrinsikfaktor-AK, IgA-AK (ASCA), IgG-AK (ASCA);

07.10.2009 Virologie: HBS-AK und HCV-IgG/M: keine Infektion; HA-IgM/G: entweder vor Jahren durchgemachte Erkrankung oder Antikörper nach Impfung;

07.10.2009 Sonographie Abdomen: Hepatomegalie mit Leberparenchymveränderungen wie bei Fettleberhepatitis, Gallenblase vergrößert mit betonter Gallenblasenwand als Begleitödem bei Hepatitis;

07.10.2009 Gastroskopie: Refluxösophagitis II° bei axialer Hiatushernie, Gastritis, Histologie: geringgradige chronisch aktive Helicobacter-pylori-Gastritis von Antrum und Korpus, kein Anhalt für Malignität;

13.10.2009 Biopsie rechter Leberlappen: Histologie: mäßig aktive (alkoholische) Fettleberhepatitis mit beginnendem zirrrogenem Umbau und Cholestasezeichen, kombinierte toxische Leberschädigung mit zusätzlichen medikamentösen Noxen, kein Anhalt für Malignität und Spezifität

16.10.2009 Röntgen-Thorax: unauffällig;

22.10.2009 CT-Thorax: vergrößerte Leber, ansonsten unauffällig

unter der Therapie sinkende Cholestaseparameter, Transaminasen noch unverändert

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 01.-27.10.2009;

Krankenseinweisung aufgrund von Ikterus und reduziertem Allgemeinzustand bei hepatitischen Veränderungen mit beginnendem zirrhotischem Umbau;

Differentialdiagnostisch: Autoimmundiagnostik negativ, Gallenblase unauffällig, Virologie unauffällig;

Entlassungsbrief: "Diagnose einer mäßig aktiven alkoholischen Fettleberhepatitis mit beginnendem zirrrogenen Umbau- und Cholestasezeichen, eventuell auch mit zusätzlicher Schädigung durch medikamentöse Noxen, so dass in Zusammenschau aller Befunde, der chronische Alkoholabusus

der Patientin, sowie die zuletzt massive Einnahme von Schmerzmedikamenten als Ursachen für die beginnende Leberzirrhose anzusehen sind." ; kein Anhalt für akuten Alkoholexzeß und Atemwegsinfekt;

In der Zusammenfassung ist eine chronische Leberparenchymschädigung auf dem Boden eines chronischen Alkoholkonsums (anamnestisch bis 2002) sowie 10jähriger chronischer Paracetamoleinnahme als möglich zu erachten. Beide Noxen stellen auch für sich alleine eine mögliche Ursache dar - die Toxizität von Paracetamol ist bei begleitendem Alkoholkonsum erhöht. Langjährige hepatitische Schäden könnten den beginnenden zirrhotischen Umbau bedingt haben. Akuter Auslöser könnte möglicherweise die erhöhte Ibu<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 25.01.2010 DE-002147023-PHHY2010DE02669

Sender diagnosis: not reported

Narrative

Initial report received from received from a regional pharmacovigilance center from a physician (intensive monitoring project) via HA on 13 Jan 2010: This patient had a history of alcohol abuse, smoker, polydrug abuse (ibuprofen, paracetamol, Tegen), duodenal ulcer and intervertebral disc disorder. The patient started treatment with Rasilez HCT (aliskiren/hydrochlorothiazide), 150/12.5 mg for hypertension arterial on an unspecified date. She was also on treatment with paracetamol, 500 mg for intervertebral disc disorder since 1999 (unknown date). On 23 Sep 2009 treatment with paracetamol was stopped. On 24 Sep 2009 treatment with Ibuprofen, 1000 mg for intervertebral disc disorder was initiated. On 30 Sep 2009 she experienced alcohol and drug toxic cholestatic hepatitis with incipient cirrhotic remodeling due to paracetamol and ibuprofen abuse. On 30 Sep 2009, it was seen yellow coloration of the skin, which had increased up to 01 Oct 2009. On 01 Oct 2009 treatment with Rasilez HCT and Ibuprofen were stopped. She was admitted to hospital, with impaired general condition and with pronounced skin and scleral jaundice on 01 Oct 2009. Following lab abnormalities were seen: On 01 Oct 2009, Abdominal sonography showed massive hepatomegaly with signs of cirrhosis, no ascites, no signs of cholestasis. On 07 Oct 2009, 3E(BPO) antibodies positive (interpreted as metabolic disorder due to the hepatic insufficiency) was seen, abdominal sonography showed hepatomegaly with liver parenchymal changes of the kind associated with fatty liver hepatitis, gallbladder enlarged with accentuated gallbladder wall as accompanying edema in hepatitis. On 13 Oct 2009 biopsy of right lobe of liver; histology: moderately active (alcoholic) fatty liver hepatitis with incipient cirrhotic remodelling and signs of cholestasis, combined toxic liver damage with additional medicinal drug related injurious factors, no evidence of malignancy and specificity. On 22 Oct 2009 Thoracic computed tomography (CT) showed enlarged liver, otherwise no abnormalities. She was treated with vitamin B duo, vitamin K intravenous, fat-free diet, vitamin K, vitamin B1 and vitamin B6, Fenistil and cholestyramine for pruritus. On the treatment, decreasing liver and bilirubin values. On 22 Oct 2009, the patient was resolved/ recovered with sequel. The reporter suspected the relationship between the events and Rasilez HCT, paracetamol and Ibuprofen commenting Rasilez HCT, could have had an interacting effect.

sanofi-aventis, 29.01.2010 DE-SANOFI-AVENTIS-2010SA002246

Sender diagnosis: not reported

Narrative

Initial report: This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10002875).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital. Based on this information the centre establishes diagnoses that are assessed by the centre using the WHO score.

Short summary: alcohol- and drug-toxic cholestatic hepatitis with beginning cirrhotic conversion by paracetamol and ibuprofen abuse.

Aliskiren+hydrochlorothiazide could have an interactive effect.

This case involves a 51-year-old female (smoker, body weight 75 kg) with a past medical history of alcohol abuse from 1999 to 2002 (since 2002 only occasionally one glass of wine) and duodenal ulcer; at time of event, the patient was suffering from polydrug abuse (paracetamol and ibuprofen), arterial hypertension, and intervertebral disc disorder. The following drug was reported to have been taken concomitantly: pantoprazole.

The patient started treatment with paracetamol (trade name not ascertainable) 500mg every four hours in 1999 for intervertebral disc disorder. In Sep.2009, she additionally initiated treatment with ibuprofen (trade name not ascertainable) 40 tablets in seven days.

On 30-Sep-2009, the patient observed yellow colouration of her skin that increased on the following day, so that the patient went to her family doctor. The patient was hospitalised in a peripheral hospital with reduced general condition and pronounced icterus of skin and sclerae. She was treated with vitamin B1+B6, intravenous vitamin K, and fat-free diet, leading to a decrease in liver values. On 04-Oct-2009, the values started to increase again, so that the patient was transferred to a tertiary hospital on 06-Oct-2009. Here she was treated with vitamin K, vitamin B1 and B6, and dimetindene maleate and colestyramine for pruritus. Under treatment, liver and bilirubin values decreased. Paracetamol was discontinued on 23-Sep-2009, ibuprofen was discontinued on 01-Oct-2009. Autoimmune diagnostics were negative, virology was unremarkable, gallbladder was unremarkable. Outcome was reported as "recovered/resolved with sequel" on 22-Oct-2009. The patient was discharged from hospital on 27-Oct-2009. Discharge letter read: "diagnosis of a moderately active alcoholic fatty liver hepatitis with signs of beginning cirrhotic conversion and cholestasis, maybe also with additional damage by medicamentous noxae, so that, as summary of all findings, the patient's chronic alcohol abuse as well as the at last massive intake of analgesics could be considered as causes of the beginning liver cirrhosis." There was no indication of acute alcoholic excess or respiratory tract infection. As synopsis, a chronic damage of liver parenchyma on the base of chronic alcohol consumption and 10 years of chronic paracetamol intake has to be considered possible. Each noxa by itself can also be regarded as possible cause - toxicity of paracetamol is increased under additional intake of alcohol. Long years of hepatic damage could have led to beginning cirrhotic conversion. Acute catalyst could have possibly been the increased ibuprofen intake.

Causality determined by PV centre:

"possible" for paracetamol, ibuprofen, and aliskiren+hydrochlorothiazide (interacting).

Comments concerning causality: Increased liver enzymes are a described adverse effect of paracetamol. In connection with alcohol, liver damage can occur. The long-term, regular high-dose intake could have caused chronic hepatic damages, that could have led to beginning cirrhotic conversion. Liver function disorders, liver damage, and liver failure (especially under long-term therapy) and acute hepatitis are described adverse effects of

ibuprofen. Drugdex: cases of icterus are described after oral treatment, hepatotoxicity after three days, at the earliest. Icterus is described as adverse effect of aliskiren+hydrochlorothiazide, the chronologic element is remote, there is a possibility of interaction with paracetamol and ibuprofen. Possible alternative cause: chronic alcohol consumption in past medical history (acute alcohol excess was negated by patient).

Additional information extracted from the website of the German health authority on 25-Jan-2010. Information by PV centre: the patient was treated with ibuprofen manufactured by Ratiopharm. Pantoprazole was changed to "suspect drug". The patient's cholestasis parameters decreased under treatment, transaminases remained unchanged.

Causality assessment by PV centre (method: WHO score):

"possible" for paracetamol and ibuprofen

"unlikely" for aliskiren+hydrochlorothiazide and pantoprazole

Comment concerning pantoprazole: liver damage and icterus are described under treatment with pantoprazole; cholestasis improved under continuation of treatment with pantoprazole.

NOVARTIS, 03.02.2010 DE-002147023-PHHY2010DE02669

Sender diagnosis: not reported

Narrative

Initial report received from received from a regional pharmacovigilance center from a physician (intensive monitoring project) via HA on 13 Jan 2010: This patient had a history of alcohol abuse, smoker, polydrug abuse (ibuprofen, paracetamol, Tegen), duodenal ulcer and intervertebral disc disorder. The patient started treatment with Rasilez HCT (aliskiren/hydrochlorothiazide), 150/12.5 mg for hypertension arterial on an unspecified date. She was also on treatment with paracetamol, 500 mg for intervertebral disc disorder since 1999 (unknown date). On 23 Sep 2009 treatment with paracetamol was stopped. On 24 Sep 2009 treatment with Ibuprofen, 1000 mg for intervertebral disc disorder was initiated. On 30 Sep 2009 she experienced alcohol and drug toxic cholestatic hepatitis with incipient cirrhotic remodeling due to paracetamol and ibuprofen abuse. On 30 Sep 2009, it was seen yellow coloration of the skin, which had increased up to 01 Oct 2009. On 01 Oct 2009 treatment with Rasilez HCT and Ibuprofen were stopped. She was admitted to hospital, with impaired general condition and with pronounced skin and scleral jaundice on 01 Oct 2009. Following lab abnormalities were seen: On 01 Oct 2009, Abdominal sonography showed massive hepatomegaly with signs of cirrhosis, no ascites, no signs of cholestasis. On 07 Oct 2009, 3E(BPO) antibodies positive (interpreted as metabolic disorder due to the hepatic insufficiency) was seen, abdominal sonography showed hepatomegaly with liver parenchymal changes of the kind associated with fatty liver hepatitis, gallbladder enlarged with accentuated gallbladder wall as accompanying edema in hepatitis. On 13 Oct 2009 biopsy of right lobe of liver; histology: moderately active (alcoholic) fatty liver hepatitis with incipient cirrhotic remodelling and signs of cholestasis, combined toxic liver damage with additional medicinal drug related injurious factors, no evidence of malignancy and specificity. On 22 Oct 2009 Thoracic computed tomography (CT) showed enlarged liver, otherwise no abnormalities. She was treated with vitamin B duo, vitamin K intravenous, fat-free diet, vitamin K, vitamin B1 and vitamin B6, Fenistil and cholestyramine for pruritus. On the treatment, decreasing liver and bilirubin values. On 22 Oct 2009, the patient was resolved/ recovered with sequel. The reporter suspected the relationship between the events and Rasilez HCT, paracetamol and Ibuprofen commenting Rasilez HCT, could have had an interacting effect.

Follow-up information was received from a regional pharmacovigilance center (intensive monitoring project) via HA on 22 Jan 2010: This patient had chronic alcohol abuse until 2002. Alcohol and drug toxic cholestatic hepatitis with incipient cirrhotic remodeling due to paracetamol and ibuprofen abuse (aliskiren/hydrochlorothiazide; pantoprazole). Cholestasis value was decreasing during therapy, transaminases were unchanged. Hospitalisation in a peripheral hospital from 01 Oct until 06 Oct 2009 and in a tertiary hospital from 06 Oct until 27 Oct 2009. To the explanations of causalities Pantozol (pantoprazole) was added regarding the ADR cholestatic hepatitis with the following comment: Hepatocellular damages and icterus are described for pantoprazole. There was an improvement of the cholestasis while pantoprazole was continued. Symptomatically the symptoms were explainable by the chronic intake of paracetamol and high doses of ibuprofen. Alternative cause: chronic ethanol toxic damage. Outcome of the patient's condition was reported as not recovered. The causal relationship for Rasilez HCT was changed from 'possible' to unlikely and the causal relationship to Pantozol was noted as unlikely.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10004588

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 51 Year	weight (kg): 75	BMI_int: 26.57	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis cholestatic	20090930	20091022	23 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	20090924-20091001	Intervertebral disc disorder
ibuprofen		82	8 Day	
2 PANTOZOL	Oral	80 Mg milligram(s)/Day	2000-	Gastric ulcer prophylaxis
pantoprazole sodium			-	
3 PARACETAMOL	Oral	/	1999-20090923	Intervertebral disc disorder
paracetamol		245	-	
4 rasilez hct		/	-20091001	Hypertension arterial
aliskiren fumarate; hydrochlorothiazide		82	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Smoker			Unknown	25 py; 13 cigarettes per day for about 3 years
Alcohol use	2002		Unknown	1 glas of wein occasionally
Intervertebral disc disorder	1999		Unknown	
Duodenal ulcer		2000	No	
Hypertension arterial			Unknown	
Polydrug abuse	1999		Unknown	Paracetamol 500 mg and ibuprofene
Alcohol abuse	1991	2002	No	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 15.01.2010 DE-NOVNCHHQ-GXKR2010DE00650

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 13 Jan 2010 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-10002875), provided by a Regional Pharmacovigilance Center, reported by a pharmacist: This multimorbid patient developed hepatitis cholestatic during treatment with paracetamol (MAH unknown), ibuprofen (MAH unknown) and Rasilez HCT (aliskiren/hydrochlorothiazide). Rasilez was considered as interacting drug. The patient received Pantozol (pantoprazole) concomitantly. The outcome was reported as resolved with sequel. The causality was assessed as possible.

Serious spontaneous report (nosptalisation) of hepatitis cholestatic, already listed in the Core Data Sheet information. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 26.01.2010 DE-NOVNCHHQ-GXKR2010DE00650

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 13 Jan 2010 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-10002875), provided by a Regional Pharmacovigilance Center, reported by a pharmacist: This multimorbid patient developed hepatitis cholestatic during treatment with paracetamol (MAH unknown), ibuprofen (MAH unknown) and Rasilez HCT (aliskiren/hydrochlorothiazide). Rasilez was considered as interacting drug. The patient received Pantozol (pantoprazole) concomitantly. The outcome was reported as resolved with sequel. The causality was assessed as possible.

Follow-up report received on 22 Jan 2010 from the German Health Authority (BfArM-homepage; case-no. DE-BfArM-10002875), provided by a Regional Pharmacovigilance Center, reported by a pharmacist: This multimorbid patient developed hepatitis cholestatic during treatment with Rasilez HCT (aliskiren/hydrochlorothiazide), Pantozol (pantoprazole), paracetamol (MAH unknown) and Ibuprofen Ratiopharm 500 lysinat (ibuprofen lysinate). In this report, Pantozol was no longer considered as concomitant drug and Rasilez was not classified as interacting drug. The outcome was reported as not resolved. For paracetamol and ibuprofen, the causality was assessed as possible. The causality was assessed as unlikely for Rasilez and Pantozol. No more information was expected. Therefore, the case was closed by the Regional Pharmacovigilance Center.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10023080

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19540930	Age: 55 Year	weight (kg): 96	BMI_int: 28.67	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug interaction					Unknown
2 Hepatitis	20100202				not recovered/not resolved
3 Icterus					not recovered/not resolved
4 Pruritus					not recovered/not resolved
5 Weight loss					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibuflam 600 ibuprofen	Oral	1200 Mg milligram(s)/Day	20100120-20100201 13 Day	Pain
2 Pantoprazol-CT 20 mg magensaftresistente Ta pantoprazole	Oral	20 Mg milligram(s)/Day 245	20100120-20100201 13 Day	Gastritis prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Shoulder operation			No	
Hay fever			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase		306	U/l	
Alanine aminotransferase	20100304	38	U/l	
Alkaline phosphatase		323	U/l	
Alkaline phosphatase	20100304	302	U/l	
Aspartate aminotransferase		94	U/l	
Aspartate aminotransferase	20100304	35	U/l	
Bilirubin		8.2	mg/dl	
Bilirubin	20100304	8.2	mg/dl	
Gamma GT		357	U/l	
Gamma GT	20100304	62	U/l	

Narratives (nur bei elektronischem Eingang)

CT-Arzneimittel GmbH, 18.02.2010 DE-CT-2010CT0201080

Sender diagnosis: not reported

Narrative

Case reference number 2010CT0201080 is a spontaneous case report sent by a physician which refers to a male aged 55 years. Case was confirmed by a health professional.

The patient's past medical history included Shoulder operation (Shoulder operation). There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Pantoprazol-CT 20 mg magensaftresistente Tabletten (PANTOPRAZOLE), indication unknown on an unknown date and Ibuprofen (IBUPROFEN), indication unknown on an unknown date. The patient developed Hepatitis (Hepatitis)Icterus (Jaundice)and Drug interaction pantoprazol - ibuprofen (Drug interaction).

The following action was taken: for Pantoprazol-CT 20 mg magensaftresistente Tabletten (PANTOPRAZOLE) - Unknown for Ibuprofen (IBUPROFEN) - Unknown.

The outcome for Hepatitis (Hepatitis) is Recovering / Resolving. The outcome for Icterus (Jaundice) is Recovering / Resolving. The outcome for Drug interaction pantoprazol - ibuprofen (Drug interaction) is Not applicable.

Rechallenge information for Pantoprazol-CT 20 mg magensaftresistente Tabletten is not applicable. Rechallenge information for Ibuprofen is not applicable. Dechallenge information for Pantoprazol-CT 20 mg magensaftresistente Tabletten is not applicable. Dechallenge information for Ibuprofen is not applicable.

The reporter's assessment for drug Pantoprazol-CT 20 mg magensaftresistente Tabletten related with Hepatitis is as follows: Unknown. The reporter's assessment for drug Pantoprazol-CT 20 mg magensaftresistente Tabletten related with Icterus is as follows: Unknown. The reporter's assessment for drug Pantoprazol-CT 20 mg magensaftresistente Tabletten related with Drug interaction pantoprazol - ibuprofen is as follows: Unknown.

CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: Yes, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

CT-Arzneimittel GmbH, 11.03.2010 DE-CT-2010CT0201080

Sender diagnosis: not reported

Narrative

Case reference number 2010CT0201080 is a spontaneous case report sent by a male physician (=patient) aged 55 years.

The patient's past medical history included Shoulder operation (Shoulder operation). There is no information about risk factors. Concomitant diseases include Hay fever (Seasonal allergy). No prior drugs are known to have been taken by the patient.

The patient started taking Pantoprazol-CT 20 mg magensaftresistente Tabletten (PANTOPRAZOLE), indication Stomach prophylaxis on 20-Jan-2010 and Ibuflam 600 (IBUPROFEN), indication Pain due to shoulder operation on 20-Jan-2010. The patient developed Hepatitis (Hepatitis)(start date 02-Feb-2010), Icterus (Jaundice), Drug interaction pantoprazol - ibuprofen (Drug interaction), Pruritus (Pruritus) and Weight loss (10 kg) (Weight decreased).

The following action was taken: for Pantoprazol-CT 20 mg magensaftresistente Tabletten (PANTOPRAZOLE) - Drug withdrawn for Ibuflam 600 (IBUPROFEN) - Drug withdrawn.

The outcome for Hepatitis (Hepatitis) is Not Recovered / Not Resolved. The outcome for Icterus (Jaundice) is Not Recovered / Not Resolved. The outcome for Drug interaction pantoprazol - ibuprofen (Drug interaction) is Not applicable. The outcome for Weight loss (10 kg) (Weight decreased) is Not Recovered / Not Resolved. The outcome for Pruritus (Pruritus) is Not Recovered / Not Resolved.

Rechallenge information for Pantoprazol-CT 20 mg magensaftresistente Tabletten is not applicable. Rechallenge information for Ibuflam 600 is not applicable. Dechallenge information for Pantoprazol-CT 20 mg magensaftresistente Tabletten is not applicable. Dechallenge information for Ibuflam 600 is not applicable.

The reporter's assessment for drug Pantoprazol-CT 20 mg magensaftresistente Tabletten related with Hepatitis is as follows: Certain. The reporter's assessment for drug Pantoprazol-CT 20 mg magensaftresistente Tabletten related with Icterus is as follows: Certain. The reporter's assessment for drug Pantoprazol-CT 20 mg magensaftresistente Tabletten related with Drug interaction pantoprazol - ibuprofen is as follows: Unknown. The reporter's assessment for drug Pantoprazol-CT 20 mg magensaftresistente Tabletten related with Pruritus is as follows: Certain. The reporter's assessment for drug Pantoprazol-CT 20 mg magensaftresistente Tabletten related with Weight loss (10 kg) is as follows: Certain. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: Yes, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10025212

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19480823	Age: 61 Year	weight (kg): 118	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20091122				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Clinda-Saar, 600 mg, Filmtabl., MIP clindamycin	Oral	1800 Mg milligram(s)/Day 82	20091019-20091025 7 Day	Tooth extraction
2 Ibu ratiopharm, 800 mg, Filmtabl ibuprofen	Oral	400 Mg milligram(s)/Day 82	2004-20091020 -	Pain in spine
3 Paracetamol, 500 mg, Tabl, Ratiopharm paracetamol	Oral	2*500 Mg milligram(s)/We 245	2004-20091208 -	Pain in spine

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Adipositas per magna	197412		k.A.	
Hypertensive heart disease			k.A.	
Hypertension arterial	199812		k.A.	arterielle Hypertonie mit rezidivierenden Entgleisungen
Atrial fibrillation	200902		k.A.	absolute Arrhythmie bei Vorhofflimmern
Type 2 diabetes mellitus	200411		k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20091208	599	U/l	
Alanine aminotransferase increased	20091209	1046	U/l	
Alanine aminotransferase increased	20091211	1584	U/l	
Alanine aminotransferase increased	20091222	943	U/l	
Alanine aminotransferase increased	20100104	79	U/l	
Aspartate aminotransferase increased	20091208	525	U/l	
Aspartate aminotransferase increased	20091210	1087	U/l	
Aspartate aminotransferase increased	20091211	940	U/l	
Aspartate aminotransferase increased	20091222	364	U/l	
Aspartate aminotransferase normal	20100104	28	U/l	
Bilirubin normal	20091209	0.99	mg/dl	
Bilirubin increased	20091210	1.8	mg/dl	
Bilirubin normal	20091221	0.96	mg/dl	
Alkaline phosphatase normal	20091208	81	U/l	
Alkaline phosphatase normal	20091214	111	U/l	

Narratives (nur bei elektronischem Eingang)PVZ-FAKOS, Klinische Pharmakologie, 25.02.2010 DE-FAKOS-170303

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initial report (more info follows).

Reporter's comment:PVZ-FAKOS, Klinische Pharmakologie, 25.03.2010 DE-FAKOS-170303

Sender diagnosis: not reported

Narrative

Am 09.12.2009 erfolgte die stationäre Aufnahme des Patienten wegen seit 2 Wochen bestehender Übelkeit mit Diarrhoen sowie wegen der neu aufgetretenen, starken Erhöhung der Transaminasen. Vorausgegangen war eine Einnahme von Clindamycin in der Zeit vom 19.10. - 25.10.2009 nach Zahnextraktion.

Bei Aufnahme waren die Transaminasen stark erhöht (ALAT 1046 U/l (Normbereich: <41 U/l); ASAT 731 U/l (Normbereich: <37 U/l); außerdem: GGT (Gamma-Glutamyltransferase) am 08.12.2009: 85 U/l (Normbereich: 8-61 U/l); AP (Alkalische Phosphatase) am 08.12.2009: 81 U/l (Normbereich: 40-260 U/l), Gesamtbilirubin 0,99 mg/dl (Normbereich: <1,2 mg/dl). Es kam zu einem weiteren Anstieg in den ersten Tagen der stationären Behandlung (Maximum ALAT am 11.12.2009: 1584 U/l, Maximum ASAT am 10.12.2009: 1087 U/l; geringer Anstieg des Bilirubins mit 1,8 mg/dl (10.12.2009). Im Verlauf rasche Rückbildungstendenz der erhöhten Transaminasen (am 04.01.2010 ALAT 79 U/l; ASAT 28 U/l; GGT 85 U/l) bei klinischer Besserung. Eine umfangreiche Erregerdiagnostik (Cytomegalie, EBV (Epstein-Barr-Virus), Hepatitis A, B und C) war unauffällig. Leber, Gallenblase, Gallengänge und Pankreas waren sowohl sonographisch als auch computertomographisch unauffällig. Die Leberpunktion am 11.12.2009 ergab frische Nekrosen und epitheloidzellige Granulome - Bild wie bei einer toxischen Leberschädigung. Auch laborchemisch keine Hinweise auf eine Autoimmunhepatitis (Auto-Antikörper ANA, AMA, LKM, SLA-LP, SMA negativ). Am 21.12.2009 wurde der Patient aus dem Krankenhaus entlassen.

In der Arzneimittelanamnese auffällig ist eine Selbstmedikation mit verschiedenen Analgetika (Paracetamol, ASS und Ibuprofen) seit mehreren Jahren bei chronischen Wirbelsäulenschmerzen. ASS wurde vom Patienten nach Entlassung aus der stationären Behandlung wieder eingenommen und gut vertragen.

Eine medikamenteninduzierte Genese der Hepatitis ist wahrscheinlich.

Als Auslöser der Hepatitis werden folgende Arzneimittel verdächtigt:

Clinda-Saar Filmtabl., Einnahmezeitraum: 19.-25.10.2009,

Paracetamol ratiopharm Tabl., Einnahmezeitraum: 2004-08.12.2009 und

Ibu-ratiopharm Filmtabl., Einnahmezeitraum: 2004-ca. 20.10.2009.

Möglicherweise spielen mehrere Arzneistoffe gleichzeitig eine Rolle in der Entstehung der Hepatitis, da in der Histologie neben Granulomen auch frische Nekrosen gesehen wurden. Die Arzneimittelkausalität von Clindamycin und Paracetamol wird mit "wahrscheinlich", die von Ibuprofen mit "möglich" bewertet. Die frischen Nekrosen könnten im Zusammenhang mit der Paracetamoleinnahme stehen.

Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10064335

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19210113	Age: 86 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20081003	deathcause: Stevens-Johnson syndrome
deathdate: 20081003	deathcause: Multiorgan failure

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Condition aggravated	20080921			630 Day	fatal
2 Exsiccosis					fatal
3 Impaired liver function					fatal
4 Mycoplasma antibody positive	20080917			626 Day	fatal
5 Hypoalbuminemia					fatal
6 PSA increased					fatal
7 Methicillin-resistant staphylococcal aureus sepsis					fatal
8 Bone marrow toxicity					fatal
9 Edema of legs					fatal
10 Edema generalized	20080907			616 Day	fatal
11 Systemic inflammatory response syndrome					fatal
12 Vein disorder					fatal
13 Thrombophlebitis	20080818			596 Day	fatal
14 Stevens Johnson syndrome	20080818	20081003	47 Day	22 Day	fatal
15 Stevens-Johnson syndrome	20080818			23 Day	fatal
16 Bronchopneumonia					fatal
17 Capillary leak syndrome	2008				fatal
18 Condition aggravated	20080921			630 Day	fatal
19 Decompensation cardiac					fatal
20 Edema generalized	20080907			616 Day	fatal
21 Edema of legs					fatal
22 Hypoalbuminemia					fatal
23 Impaired liver function					fatal
24 Lung infiltration	20080919			628 Day	fatal
25 Mucous membrane disorder					fatal
26 Multiorgan failure	2008			642 Day	fatal
27 Pancytopenia	2008				fatal
28 PSA increased					fatal
29 Respiratory insufficiency	20080924			633 Day	fatal
30 Thrombophlebitis	20080818			596 Day	fatal
31 Vein disorder					fatal
32 Systemic inflammatory response syndrome					fatal
33 Bone marrow toxicity					fatal
34 Exsiccosis					fatal
35 Pneumonia fungal					fatal
36 Mycoplasma antibody positive	20080917			626 Day	fatal
37 Renal insufficiency aggravated					fatal
38 Stevens-Johnson syndrome	20080818			23 Day	fatal
39 Multiorgan failure	2008			642 Day	fatal
40 Pancytopenia	2008				fatal

41	Capillary leak syndrome	2008		fatal
42	Renal insufficiency aggravated			fatal
43	Decompensation cardiac			fatal
44	Respiratory insufficiency	20080924	633 Day	fatal
45	Lung infiltration	20080919	628 Day	fatal
46	Bronchopneumonia			fatal
47	Pneumonia fungal			fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Oral	/	20080727- -	Thrombosis prophylaxis
2 ALLOPURINOL allopurinol	Oral	300 Mg milligram(s)/Day	20080724- -	Hyperuricaemia
3 ass acetylsalicylic acid	Oral	100 Mg milligram(s)/	20080727- -	Thrombosis prophylaxis
4 betaisodona povidone-iodine	Topical	/	20080728- -	
5 CYNT moxonidine	Oral	/	20080728- -	Cardiac insufficiency
6 DETRUSITOL tolterodine l-tartrate	Oral	2 mg/kg milligram(s)/kilogr	20080727- -	Incontinence
7 DEXA POLYSPECTRAN N dexamethasone sodium phosphate; neomycin sulfate; polymyxin b sulfate	Ophthalmic	/	20080724- -	Conjunctivitis
8 FERROUS GLUCONATE ferrous gluconate		/	- -	
9 Folsaeure folic acid	Oral	/	20080727- -	Supplementation therapy
10 ibuprofen ibuprofen	Oral	/ 5	20080727- -	Pain
11 NOVODIGAL digoxin	Oral	0.2 Mg milligram(s)/Day	20080727- -	Cardiac insufficiency
12 TORASEMIDE torasemide	Oral	10 Mg milligram(s)/Day	20070101- -	Hypertension arterial

Drugs (concomitant)

13 MONO-EMBOLEX heparin-fraction, sodium salt		/	20080819-20080820 2 Day	Thrombophlebitis
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cardiac pacemaker insertion			Yes	
Cardiac insufficiency			Yes	
Apoplexy			No	
Alcohol abuse			Yes	
Mycoplasma serology positive			Unknown	
Chronic conjunctivitis			Yes	
Nicotine abuse			Yes	
Chronic atrial fibrillation			Yes	
Sulfonamide allergy			No	
Exsiccosis			Yes	
Thrombophlebitis of the leg			Yes	
Sick sinus syndrome			Yes	
Renal insufficiency			Yes	
PSA increased			Yes	
Hypoalbuminaemia			Yes	
Hyperuricaemia			Yes	
Hypertension arterial			Yes	
Drug allergy			No	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20080828	52	IU/L	
Alanine aminotransferase	20080901	101	IU/L	
Alanine aminotransferase	20080903	113	IU/L	
Alanine aminotransferase	20080915	32	IU/L	
Alanine aminotransferase	20080917	32	IU/L	
Aspartate aminotransferase	20080917	19	IU/L	
Aspartate aminotransferase	20080915	19	IU/L	
Aspartate aminotransferase	20080903	47	IU/L	
Aspartate aminotransferase	20080901	69	IU/L	
Aspartate aminotransferase	20080828	75	IU/L	
Blood creatine	20080428	2,26	mg/dL	
Blood creatine	20080917	1,8	mg/dL	
Blood creatinine	20080901	1,80	mg/dL	
Blood creatinine	20080903	1,51	mg/dL	
Blood creatinine	20080915	1,56	mg/dL	
Blood lactate dehydrogenase	20080915	263	IU/L	
Blood lactate dehydrogenase	20080901	433	IU/L	
Blood lactate dehydrogenase	20080903	384	IU/L	
Blood urea	20080828	125	mg/dL	
Blood urea	20080903	140	mg/dL	
Blood urea	20080903	107	mg/dL	

DE-BFARM-10064335			
Blood urea	20080915	98	mg/dL
C-reactive protein increased	20080912	nicht vorhanden	NA Absent
Gamma-glutamyltransferase	20080428	126	IU/L
Gamma-glutamyltransferase	20080903	171	IU/L
Gamma-glutamyltransferase	20080915	183	IU/L
Gamma-glutamyltransferase	20080901	140	IU/L
Lymphocyte count	20080428	7	%
Lymphocyte count	20080901	7	%
Prostatic specific antigen	200611	13	ng/mL
Prostatic specific antigen	2008	17	ng/dL

Narratives (nur bei elektronischem Eingang)

Whitehall Much GmbH, 06.05.2010 DE-WYE-G06048110

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (Hoercomplex Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (Folplus STUCOPHARM) per oral route from 27Jul2008 for Substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma an an unknown date. On 18Aug2008 the patient developed spots an skin and Steven-Johnson-Syndrome was diagnosed an the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and an 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN lotion). On 28Aug2008 the patient was hospitalised and redness of eyes was diagnosed. On 01 Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed . It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered an 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causa) relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was Seen by the project experts.

No follow-up needed. Follow-up provided automatically by dZh.

PFIZERINC, 11.05.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven

Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalised and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexta-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexta-polyspectran eye drops has submitted the same report to the regulatory authorities.

PFIZERINC, 17.05.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735. A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETASODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

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Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development Company includes: License partner company's case assessment.

PFIZERINC, 27.05.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735. A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to

hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

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Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. New information includes: Additional information now medically confirmed received from a other health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Whitehall Much GmbH, 29.06.2010 DE-WYE-G06048110

Sender diagnosis: not reported

Narrative

case reversioned - additional data received

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence. 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension. 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (Hoercomplex Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (Folplus STUCOPHARM) per oral route from 27Jul2008 for Substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN lotion). On 28Aug2008 the patient was hospitalised and redness of eyes was diagnosed. On 01 Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

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Follow-up (28Apr2010): New Information received from license Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407637, licence party for Ibuprofen. New Information Includes: Additional Information now medically confirmed received from another health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Follow-up (17Jun2010):

The contactable physician reported that according to the patient's carer it was not clear whether the patient took detrusitol, allopurinol, torasemide and digoxin (NOVIDIGAL). The carer stated that the patient only took acetylsalicylic acid (ASS 100 mg). The patient had a medical history of unclear known right-sided focal hepatic lesion (sonography of abdomen on 09Sep2008), condition after apoplexy with aphasia, chronic atrial fibrillation with suspicion of coronary heart disease, differential diagnosis hypertensive cardiac disease with arterial hypertension, condition after implantation of cardiac pacemaker due to sick sinus syndrome, chronic conjunctivitis, thrombophlebitis of right lower leg (on 18Aug2008), renal insufficiency, exsiccosis, hypoalbuminemia left heart insufficiency, condition after alcohol abuse and nicotine abuse, hyperuricemia and increased prostatic specific antigen (PSA, diagnosed in Nov2006). Further a past drug history of allergy to benzocaine and sulfonamide was reported. The patient was concomitantly treated with infusions of heparin-fraction, sodium salt (MONO-EMBOLEX) on 19Aug2008 and on 20 Aug2008 due to thrombophlebitis of right lower leg. During hospitalisation the patient underwent several examinations. Laboratory investigation from 28Aug2008 yielded the following results: Serum urea 125 mg/dl (normal range 16-55), serum creatine (Jaffe) 2.26 mg/dl (normal range 0.81-1.44), serum aspartate aminotransferase (AST) 75 IU/l (normal range 0-50), Serum alanine aminotransferase (ALT) 52 IU/l (normal range 0-50), serum gamma-glutamyltransferase (GGT) 126 IU/l (normal range 0-60) and a lymphocyte count of 7 % (normal range 20-50). On 01 Sep2008 laboratory investigations showed the following results: Serum urea 140 mg/dl (normal range 18-55), serum creatinine (Jaffe) 1.60 mg/dl (normal range 0.81-1.44), serum AST 69 IU/l (normal range 0-50), serum ALT 101 IU/l (normal range 0-50), serum LDH 433 IU/l (normal range 0-250), serum GGT 140 IU/l (normal range 0-60), a **thrombocyte count of 250/nl** (normal range 140-440) and a lymphocyte count of 7 % (normal range 20-50). On 03Sep2008 laboratory investigations showed the following results: Serum urea 107 mg/dl (normal range 18-55), Serum creatinine (Jaffe) 1.51 mg/dl (normal range 0.81-1.44), serum AST 47 IU/l (normal range 0-50), serum ALT 113 IU/l (normal range 0-50), serum LDH 384 IU/l (normal range 0-250) and serum GGT 171 IU/l (normal range 0-60). On 15Sep2008 laboratory investigations showed the following results: Serum urea 98 mg/dl (normal range 18-55), serum creatinine (Jaffe) 1.56 mg/dl (normal range 0.81-1.44), serum AST 191 U/l (normal range 0-50), serum ALT 32 IU/l (normal range 0-50), serum LDH 263 IU/l (normal range 0-250), serum GGT 183 IU/l (normal range 0-60) and a **thrombocyte count of 60/nl** (normal range 140-440). On 17Sep2008 laboratory investigations showed the following results: Serum creatinine (Jaffe) 1.8 mg/dl (normal range 0.81-1.44), serum AST 19 IU/l (normal range 0-50) and serum ALT 32 IU/l (normal range 0-50). The results from that date for serum urea, serum LDH, serum GGT and thrombocyte count was illegible in the source docs. In Nov2006 the patient had been diagnosed with prostatic specific antigen (PSA) level of 13 ng/ml. In Aug2008 or Sep2006 during hospital stay a PSA value of 17 ng/ml was detected. During stay in the dermatologic ward (admission was on 28Aug2008 as previously reported) the patient was treated with 80 mg prednisolone per oral route and with 40 mg pantoprazole for gastric protection. The patient's body was topically treated with dexamethasone sodium phosphate (BETNESOL V) and the face was treated with methylprednisolone aceponate (ADVANTAN). The eyes were treated with topically 4 times daily ofloxacin (FLOXAN) ointment and oral hygiene was performed. Increased creatinine of 2.26 mg/dl was successfully treated with intravenous volume substitution. Histology of back (as reported) was performed on 02 Sep2008 showing multiform drug reaction. Direct immunofluorescence (DIF) from 02Sep2008 was negative. On 07Sep2008 the patient received 20 mg intravenous furosemide (LASIX) due to massive edema. Edema of legs reaching up to the scrotum were then (no date provided) treated with 25 mg hydrochlorothiazide (ESIDRIX) per oral route. Hypoalbuminemia was treated with human albumin 10% for 3 days. Increased C-reactive protein was diagnosed on 12Sep2008 and treated from that day onwards for 7 days with intravenous ceftriaxone sodium (ROCEPHIN). The cause for increasing leukocytopenia and thrombocytopenia was unclear (the patient was not treated with heparin at that time). An internist was consulted and the most probable cause for leukocytopenia and thrombocytopenia was seen in the patient's skin or immune reaction. On 17Sep2008 the patient was diagnosed with increased level of Mycoplasma antibodies in serum, and therefore underwent treatment with once daily 300 mg oral roxithromycin (RULID). The reporting physician stated that Mycoplasma is a known trigger of Stevens-Johnson syndrome. On 19Sep2006 the patient was switched to intravenous parenteral nutrition. The patient had mucosal lesions and a bad venous status due to edema. On 19Sep2008 x-ray of chest was performed revealing extensive infiltrations. After repeated consultation of the internist the patient received cilastatin sodium imipenem (ZIENAM), flucanazole (DIFLUCAN) and erythromycin. The patient was isolated due to leukocyte value of 1.7/nl and was treated with thrombocyte concentrate due to a thrombocyte count of 141/nl. On 21Sep2008 the patient was transferred from the dermatologic ward to the intensive care unit due to increased worsening of the patient's general condition and respiratory insufficiency with bronchopneumonia and fungal pneumonia as a consequence of immunosuppression under cortisone (systemic inflammatory response syndrome, SIRS) and a possible cytotoxic bone marrow damage due to drug-induced (furosemide and allopurinol suspected) allergic reaction. The patient had peripheral exsiccosis, signs of capillary leak syndrome and no peripheral edema. Pancytopenia with anemia, neutropenia and thrombocytopenia was treated with red blood cell concentrate, thrombocyte concentrate and stimulation of leukopoiesis with GRANOZYTE (nos). Symptoms of cardiac decompensation were treated with diuretic therapy (furosemide). From 24Sep2008 onwards the patient developed sepsis and respiratory insufficiency so that intubation, artificial ventilation and catecholamines were required. The patient was isolated due to infection with multidrug-resistant *Streptococcus aureus* (MRSA). In the further course the patient developed worsening renal insufficiency, impaired liver function, changes in bronchial system (both sides) with severe inflammatory structural changes and suspicion of confluent fungal infection. Microbiological investigation revealed infection with mold-like fungus so that an antimycotic therapy with VOICONAZOL (nos) was started. At that time increased doses of catecholamines were required and the patient developed complete anuria. On 03Oct2008 at 12:25 hours the patient died due to multiorgan failure. The reporting physician did not provide a causality assessment of the additionally reported events with fatal outcome. An autopsy report was not available to the reporter.

Case Comment J & J Pharmaceutical Research & Development Company Causality Assessment: The causality of Ibuprofen in relationship with event Death was assessed as possible and with event Stevens-Johnson Syndrome was also assessed as possible.

Contribution of tolterodine, ibuprofen, dexta-polyspectran, moxonidine, torasemide, allopurinol, beta-acetyldigoxin, acetylsalicylic acid, povidone-iodine, ferrous gluconate and folic acid to the reported event cannot be excluded.

PFIZERINC, 30.06.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735. A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexta-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexta-polyspectran eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. New information includes: Additional information now medically confirmed received from a other health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Follow-up (17Jun2010):

This is a follow-up report received from the contactable physician. The physician reported that according to the patient's carer it was not clear whether the patient took tolterodine, allopurinol, torasemide and digoxin (NOVODIGAL). The carer stated that the patient only took acetylsalicylic acid (ASS, 100 mg). The patient had a medical history of unclear known right-sided focal hepatic lesion (sonography of abdomen on 09Sep2008), condition after apoplexy with aphasia, chronic atrial fibrillation with suspicion of coronary heart disease, differential diagnosis hypertensive cardiac disease with arterial hypertension, condition after implantation of cardiac pacemaker due to sick sinus syndrome, chronic conjunctivitis, thrombophlebitis of right lower leg (on 18Aug2008), renal insufficiency, exsiccosis, hypoalbuminemia, left heart insufficiency, condition after alcohol abuse and nicotine abuse, hyperuricemia and increased prostatic specific antigen (PSA, diagnosed in Nov2006). Further a past drug history of allergy to benzocaine and sulfonamide was reported. The patient was concomitantly treated with injections of heparin-fraction, sodium salt (MONO-EMBOLEX) on 19Aug2008 and on 20 Aug2008 due to thrombophlebitis of right lower leg. During hospitalisation the patient underwent several examinations. Laboratory investigation results on 28Aug2008 included the following: Serum urea 125 mg/dl (normal range 18-55), serum creatinine (Jaff) 2.26 mg/dl (normal range 0.81-1.44), serum aspartate aminotransferase (AST) 75 IU/l (normal range 0-50), serum alanine aminotransferase (ALT) 52 IU/l (normal range 0-50), serum gamma-glutamyltransferase (GGT) 126 IU/l (normal range 0-60) and a lymphocyte count of 7 % (normal range 20-50). On 01Sep2008 laboratory investigations showed the following results: Serum urea 140 mg/dl, serum creatinine (Jaff) 1.80 mg/dl, serum AST 69 IU/l, serum ALT 101 IU/l, serum LDH 433 IU/l (normal range 0-250), serum GGT 140 IU/l, a thrombocyte count of 250/nl (normal range 140-440) and a lymphocyte count of 7 %. On 03Sep2008 laboratory investigations showed the following results: Serum urea 107 mg/dl, serum creatinine (Jaff) 1.51 mg/dl, serum AST 47 IU/l, serum ALT 113 IU/l, serum LDH 384 IU/l and serum GGT 171 IU/l. On 15Sep2008 laboratory investigations showed the following results:

Serum urea 98 mg/dl, serum creatinine (Jaff) 1.56 mg/dl, serum AST 19 IU/l, serum ALT 32 IU/l, serum LDH 263 IU/l, serum GGT 183 IU/l and a thrombocyte count of 60/nl. On 17Sep2008 laboratory investigations showed the following results: Serum creatinine (Jaff) 1.8 mg/dl, serum AST 19 IU/l and serum ALT 32 IU/l. The results from that date for serum urea, serum LDH, serum GGT and thrombocyte count were illegible in the source docs. In Nov2006 the patient had been diagnosed with prostatic specific antigen (PSA) level of 13 ng/ml. In Aug2008 or Sep2008 during hospital stay a PSA value of 17 ng/ml was detected. During stay in the dermatologic ward (admission was on 28Aug2008 as previously reported) the patient was treated with 80 mg prednisolone per oral route and with 40 mg pantoprazole for gastric protection. The patient's body was topically treated with bexamethasone sodium phosphate (BETNESOL V) and the face was treated with methylprednisolone aceponate (ADVANTAN). The eyes were treated with topically 4 times daily ofloxacin (FLOXAN) ointment and oral hygiene was performed. Increased creatinine of 2.26 mg/dl was successfully treated with intravenous volume substitution. Histology of back (as reported) was performed on 02 Sep2008 showing multiform drug reaction. Direct immunofluorescence (DIF) from 02Sep2008 was negative. On 07Sep2008 the patient received 20 mg intravenous furosemide (LASIX) due to massive edema. Edema of legs reaching up to the scrotum were then (no date provided) treated with 25 mg hydrochlorothiazide (ESIDRIX) per oral route. Hypoalbuminemia was treated with human albumin 10% for 3 days. Increased C-reactive protein was diagnosed on 12Sep2008 and treated from that day onwards for 7 days with intravenous ceftriaxone sodium (ROCEPHIN). The cause for increasing leukocytopenia and thrombocytopenia was unclear (the patient was not treated with heparin at that time). An internist was consulted and the most probable cause for leukocytopenia and thrombocytopenia was seen in the patient's skin or immune reaction. On 17Sep2008 the patient was diagnosed with increased level of Mycoplasma antibodies in serum, and therefore underwent treatment with once daily 300 mg oral roxithromycin (RULID). The reporting physician stated that Mycoplasma is a known trigger of Stevens-Johnson syndrome. On 19Sep2008 the patient was switched to intravenous parenteral nutrition. The patient had mucosal lesions and a bad venous status due to edema. On 19Sep2008 x-ray of chest was performed revealing extensive infiltrations. After repeated consultation of the internist the patient received cilastatin sodium imipenem (ZIENAM), fluconazole (DIFLUCAN) and erythromycin. The patient was isolated due to leukocyte value of 1.7/nl and was treated with thrombocyte concentrate due to a thrombocyte count of 14/nl. On 21Sep2008 the patient was transferred from the dermatologic ward to the intensive care unit due to increased worsening of the patient's general condition and respiratory insufficiency with bronchopneumonia and fungal pneumonia as a consequence of immunosuppression under cortisone (systemic inflammatory response syndrome, SIRS) and a possible cytotoxic bone marrow damage due to drug-induced (furosemide and allopurinol suspected) allergic reaction. The patient had peripheral exsiccosis, signs of capillary leak syndrome and no peripheral edema. Pancytopenia with anemia, neutropenia and thrombocytopenia was treated with red blood cell concentrate, thrombocyte concentrate and stimulation of leukopoiesis with GRANOZYTE. Symptoms of cardiac decompensation were treated with diuretic therapy (etacrynic acid). From 24Sep2008 onwards the patient developed sepsis and respiratory insufficiency so that intubation, artificial ventilation and catecholamines were required. The patient was isolated due to infection with multiresistant Streptococcus "aureus". In the further course the patient developed worsening renal insufficiency, impaired liver function, changes in bronchial system (both sides) with severe inflammatory structural changes and suspicion of confluent fungal infection. Microbiological investigation revealed infection with mold-like fungus so that an antimycotic therapy with VOICONAZOL was started. At that time increased doses of catecholamines were required and the patient developed complete anuria. On 03Oct2008 at 12:25 hours the patient died due to multiorgan failure. The reporting physician did not provide a causality assessment of the additionally reported events pancytopenia and capillary leak syndrome. An autopsy report was not available to the reporter.

PFIZERINC, 02.07.2010 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735. A contactable physician reported about an 87 year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexta-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexta-polyspectran eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. New information includes: Additional information now medically confirmed received from other health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Follow-up (17Jun2010):

This is a follow-up report received from the contactable physician. The physician reported that according to the patient's carer it was not clear whether the patient took tolterodine, allopurinol, torasemide and digoxin (NOVIDIGAL). The carer stated that the patient only took acetylsalicylic acid (ASS, 100 mg). The patient had a medical history of unclear known right-sided focal hepatic lesion (sonography of abdomen on 09Sep2008), condition after apoplexy with aphasia, chronic atrial fibrillation with suspicion of coronary heart disease, differential diagnosis hypertensive cardiac disease with arterial hypertension, condition after implantation of cardiac pacemaker due to sick sinus syndrome, chronic conjunctivitis, thrombophlebitis of right lower leg (on 18Aug2008), renal insufficiency, exsiccosis, hypoalbuminemia, left heart insufficiency, condition after alcohol abuse and nicotine abuse, hyperuricemia and increased prostatic specific antigen (PSA, diagnosed in Nov2006). Further a past drug history of allergy to benzocaine and sulfonamide was reported. The patient was concomitantly treated with injections of heparin-fraction, sodium salt (MONO-EMBOLEX) on 19Aug2008 and on 20 Aug2008 due to thrombophlebitis of right lower leg. During hospitalisation the patient underwent several examinations. Laboratory investigation results on 28Aug2008 included the following: Serum urea 125 mg/dl (normal range 18-55), serum creatinine (Jaff) 2.26 mg/dl (normal range 0.81-1.44), serum aspartate aminotransferase (AST) 75 IU/l (normal range 0-50), serum alanine aminotransferase (ALT) 52 IU/l (normal range 0-50), serum gamma-glutamyltransferase (GGT) 126 IU/l (normal range 0-60) and a lymphocyte count of 7 % (normal range 20-50). On 01Sep2008 laboratory investigations showed the following results: Serum urea 140 mg/dl, serum creatinine (Jaff) 1.80 mg/dl, serum AST 69 IU/l, serum ALT 101 IU/l, serum LDH 433 IU/l (normal range 0-250), serum GGT 140 IU/l, a thrombocyte count of 250/nl (normal range 140-440) and a lymphocyte count of 7 %. On 03Sep2008 laboratory investigations showed the following results: Serum urea 107 mg/dl, serum creatinine (Jaff) 1.51 mg/dl, serum AST 47 IU/l, serum ALT 113 IU/l, serum LDH 384 IU/l and serum GGT 171 IU/l. On 15Sep2008 laboratory investigations showed the following results: Serum urea 98 mg/dl, serum creatinine (Jaff) 1.56 mg/dl, serum AST 19 IU/l, serum ALT 32 IU/l, serum LDH 263 IU/l, serum GGT 183 IU/l and a thrombocyte count of 60/nl. On 17Sep2008 laboratory investigations showed the following results: Serum creatinine (Jaff) 1.8 mg/dl, serum AST 19 IU/l and serum ALT 32 IU/l. The results from that date for serum urea, serum LDH, serum GGT and thrombocyte count were illegible in the source docs. In Nov2006 the patient had been diagnosed with prostatic specific antigen (PSA) level of 13 ng/ml. In Aug2008 or Sep2008 during hospital stay a PSA value of 17 ng/ml was detected. During stay in the dermatologic ward (admission was on 28Aug2008 as previously reported) the patient was treated with 80 mg prednisolone per oral route and with 40 mg pantoprazole for gastric protection. The patient's body was topically treated with bexamethasone sodium phosphate (BETNESOL V) and the face was treated with methylprednisolone aceponate (ADVANTAN). The eyes were treated with topically 4 times daily ofloxacin (FLOXAN) ointment and oral hygiene was performed. Increased creatinine of 2.26 mg/dl was successfully treated with intravenous volume substitution. Histology of back (as reported) was performed on 02 Sep2008 showing multiform drug reaction. Direct immunofluorescence (DIF) from 02Sep2008 was negative. On 07Sep2008 the patient received 20 mg intravenous furosemide (LASIX) due to massive edema. Edema of legs reaching up to the scrotum were then (no date provided) treated with 25 mg hydrochlorothiazide (ESIDRIX) per oral route. Hypoalbuminemia was treated with human albumin 10% for 3 days. Increased C-reactive protein was diagnosed on 12Sep2008 and treated from that day onwards for 7 days with intravenous ceftriaxone sodium (ROCEPHIN). The cause for increasing leukocytopenia and thrombocytopenia was unclear (the patient was not treated with heparin at that time). An internist was consulted and the most probable cause for leukocytopenia and thrombocytopenia was seen in the patient's skin or immune reaction. On 17Sep2008 the patient was diagnosed with increased level of Mycoplasma antibodies in serum, and therefore underwent treatment with once daily 300 mg oral roxithromycin (RULID). The reporting physician stated that Mycoplasma is a known trigger of Stevens-Johnson syndrome. On 19Sep2008 the patient was switched to intravenous parenteral nutrition. The patient had mucosal lesions and a bad venous status due to edema. On 19Sep2008 x-ray of chest was performed revealing extensive infiltrations. After repeated consultation of the internist the patient received cilastatin sodium imipenem (ZIENAM), fluconazole (DIFLUCAN) and erythromycin. The patient was isolated due to leukocyte value of 1.7/nl and was treated with thrombocyte concentrate due to a thrombocyte count of 14/nl. On 21Sep2008 the patient was transferred from the dermatologic ward to the intensive care unit due to increased worsening of the patient's general condition and respiratory insufficiency with bronchopneumonia and fungal pneumonia as a consequence of immunosuppression under cortisone (systemic inflammatory response syndrome, SIRS) and a possible cytotoxic bone marrow damage due to drug-induced (furosemide and allopurinol suspected) allergic reaction. The patient had peripheral exsiccosis, signs of capillary leak syndrome and no peripheral edema. Pancytopenia with anemia, neutropenia and thrombocytopenia was treated with red blood cell concentrate, thrombocyte concentrate and stimulation of leukopoiesis with GRANOZYTE. Symptoms of cardiac decompensation were treated with diuretic therapy (ethacrynic acid). From 24Sep2008 onwards the patient developed sepsis and respiratory insufficiency so that intubation, artificial ventilation and catecholamines were required. The patient was isolated due to infection with multiresistant Streptococcus "aureus". In the further course the patient developed worsening renal insufficiency, impaired liver function, changes in bronchial system (both sides) with severe inflammatory structural changes and suspicion of confluent fungal infection. Microbiological investigation revealed infection with mold-like fungus so that an antimycotic therapy with VOICONAZOL was started. At that time increased doses of catecholamines were required and the patient developed complete anuria. On 03Oct2008 at 12:25 hours the patient died due to multiorgan failure. The reporting physician did not provide a causality assessment of the additionally reported events pancytopenia and capillary leak syndrome. An autopsy report was not available to the reporter.

Follow-up (17Jun2010): This follow-up report is being submitted to amend previously reported information to correct the isolated bacteria from Streptococcus aureus to Staphylococcus aureus.

Follow-up (25Jun2010): This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. The causality with ibuprofen was assessed as possible for the events of

Stevens-Johnson syndrome, multi-organ failure and pancytopenia by Licensed Partner. The causality for the remaining suspect drugs was stated as unknown.

PFIZERINC, 14.01.2011 DE-PFIZER INC-2010052756

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170735.

A contactable physician reported about an 87-year-old male patient who was treated with the following suspect drugs: 2 mg/day (1-0-0) tolterodine tartrate (DETRUSITOL) per oral route from 27Jul2008 due to incontinence, 600 mg ibuprofen per oral route from 27Jul2008 due to pain (dosage irregular or unknown), 0.3 mg moxonidine (CYNT) per oral route from 28Jul2008 due to cardiac insufficiency, 10 mg/day (1-0-0) torasemide per oral route from 01Jan2007 due to arterial hypertension, 300 mg/day (0-0-1) allopurinol (ALLOPURINOL HEXAL) per oral route from 24Jul2008 due to hyperuricemia, 0.2 mg/day (1-0-0) beta-acetyldigoxin (NOVODIGAL) per oral route from 27Jul2008 due to cardiac insufficiency, 100 mg acetylsalicylic acid (ASS) per oral route from 27Jul2008 for thrombosis prophylaxis, povidone-iodine (BETAISODONA, ointment) topically from 28Jul2008 for an unknown indication (dosage irregular or unknown), 0.5 mg unknown ingredients (HOERCOMPLEX, Batch CJU) per oral route from 28Jul2008 for an unknown indication (dosage irregular or unknown), ferrous gluconate, folic acid (FOLPLUX, STUCOPHARM) per oral route from 27Jul2008 for substitution (dosage irregular or unknown) and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate (DEXA POLYSPECTRAN N) eye drops per ophthalmic route from 24Jul2008 due to conjunctivitis (dosage irregular or unknown). Concomitant medication was not reported. Medical history included a positive serology for Mycoplasma on an unknown date. On 18Aug2008 the patient developed spots on skin and Steven-Johnson-Syndrome was diagnosed on the same day. It was unknown whether any of the suspect drugs was discontinued. On 21Aug2008 and on 25Aug2008 progression of Steven-Johnson-Syndrome was diagnosed. On 25Aug2008 treatment of Steven-Johnson-Syndrome (indication reported as: skin lesions) was started with topical hydrocortisone (HYDROGALEN, lotion). On 28Aug2008 the patient was hospitalized and redness of eyes was diagnosed. On 01Sep2008 erosions of lips and oral mucosa occurred. On 15Sep2008 blistering of skin and conjunctivitis were diagnosed. It was not reported whether this was a worsening of pre-existing conjunctivitis. Outcome was reported as recovered on 03Oct2008. It was unknown whether the patient was still in hospital. On an unspecified date the patient passed away. No death date, no cause of death and no autopsy report were provided. In Mar2009 the project experts assessed the causal relationship as "very probable" for allopurinol (highly suspicious, not taken in the past), as "possible" for tolterodine tartrate, ibuprofen, moxonidine, beta-acetyldigoxin and acetylsalicylic acid, and as "dubious" for torasemide, povidone-iodine, Hoercomplex Batch CJU, ferrous gluconate, folic acid and dexamethasone sodium phosphate, neomycin sulfate, polymyxin b sulfate. Positive serology for Mycoplasma as alternative cause was documented as "possible" by the project experts. No other possible cause was seen by the project experts.

This case contains a Wyeth co-suspect product. The Wyeth case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

Pfizer is Marketing Authorization Holder of dexta-polyspectran eye drops in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of Dexta-polyspectran eye drops has submitted the same report to the regulatory authorities.

Follow-up (28Apr2010): New information received from License Partner, J & J Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen, includes: License partner company's case assessment.

Follow-up (13May2010):

This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. New information includes: Additional information now medically confirmed received from other health professional on 13May2010. The patient died from Stevens-Johnson Syndrome on an unknown date.

Follow-up (17Jun2010):

This is a follow-up report received from the contactable physician. The physician reported that according to the patient's carer it was not clear whether the patient took tolterodine, allopurinol, torasemide and digoxin (NOVIDIGAL). The carer stated that the patient only took acetylsalicylic acid (ASS, 100 mg). The patient had a medical history of unclear known right-sided focal hepatic lesion (sonography of abdomen on 09Sep2008), condition after apoplexy with aphasia, chronic atrial fibrillation with suspicion of coronary heart disease, differential diagnosis hypertensive cardiac disease with arterial hypertension, condition after implantation of cardiac pacemaker due to sick sinus syndrome, chronic conjunctivitis, thrombophlebitis of right lower leg (on 18Aug2008), renal insufficiency, exsiccosis, hypoalbuminemia, left heart insufficiency, condition after alcohol abuse and nicotine abuse, hyperuricemia and increased prostatic specific antigen (PSA, diagnosed in Nov2006). Further a past drug history of allergy to benzocaine and sulfonamide was reported. The patient was concomitantly treated with injections of heparin-fraction, sodium salt (MONO-EMBOLEX) on 19Aug2008 and on 20 Aug2008 due to thrombophlebitis of right lower leg. During hospitalisation the patient underwent several examinations. Laboratory investigation results on 28Aug2008 included the following: Serum urea 125 mg/dl (normal range 18-55), serum creatinine (Jaff) 2.26 mg/dl (normal range 0.81-1.44), serum aspartate aminotransferase (AST) 75 IU/l (normal range 0-50), serum alanine aminotransferase (ALT) 52 IU/l (normal range 0-50), serum gamma-glutamyltransferase (GGT) 126 IU/l (normal range 0-60) and a lymphocyte count of 7 % (normal range 20-50). On 01Sep2008 laboratory investigations showed the following results: Serum urea 140 mg/dl, serum creatinine (Jaff) 1.80 mg/dl, serum AST 69 IU/l, serum ALT 101 IU/l, serum LDH 433 IU/l (normal range 0-250), serum GGT 140 IU/l, a thrombocyte count of 250/nl (normal range 140-440) and a lymphocyte count of 7 %. On 03Sep2008 laboratory investigations showed the following results: Serum urea 107 mg/dl, serum creatinine (Jaff) 1.51 mg/dl, serum AST 47 IU/l, serum ALT 113 IU/l, serum LDH 384 IU/l and serum GGT 171 IU/l. On 15Sep2008 laboratory investigations showed the following results: Serum urea 98 mg/dl, serum creatinine (Jaff) 1.56 mg/dl, serum AST 19 IU/l, serum ALT 32 IU/l, serum LDH 263 IU/l, serum GGT 183 IU/l and a thrombocyte count of 60/nl. On 17Sep2008 laboratory investigations showed the following results: Serum creatinine (Jaff) 1.8 mg/dl, serum AST 19

IU/l and serum ALT 32 IU/l. The results from that date for serum urea, serum LDH, serum GGT and thrombocyte count were illegible in the source docs. In Nov2006 the patient had been diagnosed with prostatic specific antigen (PSA) level of 13 ng/ml. In Aug2008 or Sep2008 during hospital stay a PSA value of 17 ng/ml was detected. During stay in the dermatologic ward (admission was on 28Aug2008 as previously reported) the patient was treated with 80 mg prednisolone per oral route and with 40 mg pantoprazole for gastric protection. The patient's body was topically treated with bexamethasone sodium phosphate (BETNESOL V) and the face was treated with methylprednisolone aceponate (ADVANTAN). The eyes were treated with topically 4 times daily ofloxacin (FLOXAN) ointment and oral hygiene was performed. Increased creatinine of 2.26 mg/dl was successfully treated with intravenous volume substitution. Histology of back (as reported) was performed on 02 Sep2008 showingmultiform drug reaction. Direct immunofluorescence (DIF) from 02Sep2008 was negative. On 07Sep2008 the patient received 20 mg intravenous furosemide (LASIX) due to massive edema. Edema of legs reaching up to the scrotum were then (no date provided) treated with 25 mg hydrochlorothiazide (ESIDRIX) per oral route. Hypoalbuminemia was treated with human albumin 10% for 3 days. Increased C-reactive protein was diagnosed on 12Sep2008 and treated from that day onwards for 7 days with intravenous ceftriaxone sodium (ROCEPHIN). The cause for increasing leukocytopenia and thrombocytopenia was unclear (the patient was not treated with heparin at that time). An internist was consulted and the most probable cause for leukocytopenia and thrombocytopenia was seen in the patient's skin or immune reaction. On 17Sep2008 the patient was diagnosed with increased level of Mycoplasma antibodies in serum, and therefore underwent treatment with once daily 300 mg oral roxithromycin (RULID). The reporting physician stated that Mycoplasma is a known trigger of Stevens-Johnson syndrome. On 19Sep2008 the patient was switched to intravenous parenteral nutrition. The patient had mucosal lesions and a bad venous status due to edema. On 19Sep2008 x-ray of chest was performed revealing extensive infiltrations. After repeated consultation of the internist the patient received cilastatin sodium imipenem (ZIENAM), fluconazole (DIFLUCAN) and erythromycin. The patient was isolated due to leukocyte value of 1.7/nl and was treated with thrombocyte concentrate due to a thrombocyte count of 14/nl. On 21Sep2008 the patient was transferred from the dermatologic ward to the intensive care unit due to increased worsening of the patient's general condition and respiratory insufficiency with bronchopneumonia and fungal pneumonia as a consequence of immunosuppression under cortisone (systemic inflammatory response syndrome, SIRS) and a possible cytotoxic bone marrow damage due to drug-induced (furosemide and allopurinol suspected) allergic reaction. The patient had peripheral exsiccosis, signs of capillary leak syndrome and no peripheral edema. Pancytopenia with anemia, neutropenia and thrombocytopenia was treated with red blood cell concentrate, thrombocyte concentrate and stimulation of leukopoiesis with GRANOZYTE. Symptoms of cardiac decompensation were treated with diuretic therapy (ethacrynic acid). From 24Sep2008 onwards the patient developed sepsis and respiratory insufficiency so that intubation, artificial ventilation and catecholamines were required. The patient was isolated due to infection with multiresistant Streptococcus "aureus". In the further course the patient developed worsening renal insufficiency, impaired liver function, changes in bronchial system (both sides) with severe inflammatory structural changes and suspicion of confluent fungal infection. Microbiological investigation revealed infection with mold-like fungus so that an antimycotic therapy with VOICONAZOL was started. At that time increased doses of catecholamines were required and the patient developed complete anuria. On 03Oct2008 at 12:25 hours the patient died due to multiorgan failure. The reporting physician did not provide a causality assessment of the additionally reported events pancytopenia and capillary leak syndrome. An autopsy report was not available to the reporter.

Follow-up (17Jun2010): This follow-up report is being submitted to amend previously reported information to correct the isolated bacteria from Streptococcus aureus to Staphylococcus aureus.

Follow-up (25Jun2010): This is follow-up report received from Johnson & Johnson Pharmaceutical Research & Development LLC manufacturer control number DE-JNJFOC-20100407837, license party for Ibuprofen. The causality with ibuprofen was assessed as possible for the events of Stevens-Johnson syndrome, multi-organ failure and pancytopenia by Licensed Partner. The causality for the remaining suspect drugs was stated as unknown.

Follow-up (04Jan2011): Case 2010052756 and case DE-WYE-G06048110 are found to be duplicates resulting from case sharing between Pfizer and Wyeth after the merger in Oct2009. This is a follow-up report combining information from duplicate reports 2010052756 and DE-WYE-G06048110. The current and all subsequent follow-up information will be reported under 2010052756.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10069407

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		

Patient

Initials:	Sex: Female	Birth: 19481028	Age: 61 Year	weight (kg): 91	BMI_int: 30.41	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20100412				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibu 600 1 A Pharma, Filmtabl, 1 A Pharma ibuprofen	Oral	600 Mg milligram(s)/ 82	20100215-20100412 57 Day	
2 Katadolon 100mg Kapseln, AWD Pharma flupirtine	Oral	400 Mg milligram(s)/Day 5	20100217-20100416 59 Day	
3 Valoron N retard bzw. Tilidin N Sandoz, 100/8 r naloxone hydrochloride; tilidine hydrochloride	Oral	2 Df dosage form/Day 212	20100217-20100416 59 Day	

Drugs (concomitant)

4 Decortin Tbl 20 mg, Merck prednisone	Oral	20 Mg milligram(s)/Day 245	20091123-20091130 8 Day	
5 Delix Tabl, 2.5 mg, Sanofi Aventis ramipril	Oral	2.5 Mg milligram(s)/Day 245	2005- -	
6 Januvia Tabl, 100mg, MSD sitagliptin	Oral	100 Mg milligram(s)/Day 245	2009- -	
7 Singulair Filmtabletten 10 mg, MSD Dieckmann montelukast	Oral	10 Mg milligram(s)/Day 82	200911- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Asthma bronchial			Unknown	
Coronary artery disease			Unknown	
Type 2 diabetes mellitus			Unknown	
Hypertension arterial	2005		Unknown	
Hypercholesterolaemia			Unknown	
Heart attack			Unknown	

Primary source

Qualification:	Physician	observestudytype:
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Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20100412	790	U/l	
Alanine aminotransferase increased	20100419	465	U/l	
Alanine aminotransferase increased	20100507	47	U/l	
Alanine aminotransferase increased	20100217	131	U/l	
Alkaline phosphatase normal	20100412	56	U/l	
Alkaline phosphatase normal	20100419	57	U/l	
Aspartate aminotransferase increased	20100412	279	U/l	

Aspartate aminotransferase increased	20100419	147	U/l
Aspartate aminotransferase increased	20100217	76	U/l
Aspartate aminotransferase normal	20100507	17	U/l
Bilirubin normal	20100419	0.7	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 17.05.2010 DE-FAKOS-170313

Sender diagnosis: not reported

Narrative**Sender's comment:**

Initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 19.07.2010 DE-FAKOS-170313

Sender diagnosis: not reported

Narrative

Im Februar 2010 erlitt die Patientin einen Bandscheibenprolaps L4/L5, seitdem erfolgte die Behandlung mit Katadolon, Valoron N retard 100 und Ibu 1A Pharma 600 mg Tbl. Am 17.02.2010 Transaminasen gering erhöht (ALAT 131 U/l, ASAT 76 U/l) (Normbereich: ALAT <35 U/l, ASAT <32 U/l), welche am 06.07.2009 im Normbereich lagen. Am 12.04.2010 wurde eine starke Erhöhung der Transaminasen festgestellt. Katadolon, Valoron N retard 100 und Ibu 1A Pharma 600 wurden abgesetzt und die Patientin am 19.04.2010 zur Abklärung der Transaminasenerhöhung in die stationäre Behandlung aufgenommen.

Bereits bei Aufnahme war eine Besserungstendenz der Transaminasen eingetreten (Leberwerte bei Aufnahme am 19.04.2010: ALAT 465 U/l, ASAT 147 U/l, GGT 48 U/l, Alkalische Phosphatase (AP) 57 U/l, Bilirubin gesamt 0,7 mg/dl), welche sich im Verlauf weiter besserten. Am 07.05.2010 lag ALAT bei 47 U/l, ASAT bei 17 U/l und GGT betrug 40 U/l (Normbereich GGT <40 U/l).

Sonographisch stellte sich die Leber verplumpt und höckerig wie bei einem chronischen Leberparenchymschaden dar, intrahepatische Gallenwege waren nicht dilatiert, Z. n. Cholezystektomie. Hepatitisserologie (A, B, und C) sowie Immunserologie bis auf eine diskrete unspezifische Erhöhung von ANA (1:160) unauffällig, klinisch keine Hinweise für eine Infektion oder Autoimmunerkrankung. Leberbiopsie erfolgte nicht.

Der Verlauf der Transaminasen mit rascher Besserung nach Absetzen der oben genannten Medikamente unterstützt den Verdacht einer medikamenten-bedingten Genese der akuten Leberschädigung bei vorbestehendem Leberparenchymschaden. Im Nebenwirkungsspektrum von Katadolon und Ibu 600 1A-Pharma ist eine Hepatitis gut bekannt. Katadolon und Valoron N retard wurden zur gleichen Zeit verabreicht. Ibu 600 begann früher und wurde zeitweise pausiert. Daher ist eine genauere Zuordnung zu einem bestimmten Medikament nicht möglich.

Katadolon 100 mg Kaps (AWD Pharma), Einnahmezeitraum: 20.02. - 16.04.2010

Valoron N retard 100 mg Tbl. (Pfizer), Einnahmezeitraum: 20.02. - 16.04.2010

Ibu 600 Tbl. 1A Pharma, Einnahmezeitraum: 15.-17.02.2010, 03.03.-12.04.2010

Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

PVZ-FAKOS, Klinische Pharmakologie, 30.07.2010 DE-FAKOS-170313

Sender diagnosis: not reported

Narrative

Im Februar 2010 erlitt die Patientin einen Bandscheibenprolaps L4/L5, seitdem erfolgte die Behandlung mit Katadolon, Valoron N retard 100 und Ibu 1A Pharma 600 mg Tbl. Am 17.02.2010 Transaminasen gering erhöht (ALAT 131 U/l, ASAT 76 U/l) (Normbereich: ALAT <35 U/l, ASAT <32 U/l), welche am 06.07.2009 im Normbereich lagen. Am 12.04.2010 wurde eine starke Erhöhung der Transaminasen festgestellt. Katadolon, Valoron N retard 100 und Ibu 1A Pharma 600 wurden abgesetzt und die Patientin am 19.04.2010 zur Abklärung der Transaminasenerhöhung in die stationäre Behandlung aufgenommen.

Bereits bei Aufnahme war eine Besserungstendenz der Transaminasen eingetreten (Leberwerte bei Aufnahme am 19.04.2010: ALAT 465 U/l, ASAT 147 U/l, GGT 48 U/l, Alkalische Phosphatase (AP) 57 U/l, Bilirubin gesamt 0,7 mg/dl), welche sich im Verlauf weiter besserten. Am 07.05.2010 lag ALAT bei 47 U/l, ASAT bei 17 U/l und GGT betrug 40 U/l (Normbereich GGT <40 U/l).

Sonographisch stellte sich die Leber verplumpt und höckerig wie bei einem chronischen Leberparenchymschaden dar, intrahepatische Gallenwege

waren nicht dilatiert, Z. n. Cholezystektomie. Hepatitisserologie (A, B, und C) sowie Immunserologie bis auf eine diskrete unspezifische Erhöhung von ANA (1:160) unauffällig, klinisch keine Hinweise für eine Infektion oder Autoimmunerkrankung. Leberbiopsie erfolgte nicht.

Der Verlauf der Transaminasen mit rascher Besserung nach Absetzen der oben genannten Medikamente unterstützt den Verdacht einer medikamenten-bedingten Genese der akuten Leberschädigung bei vorbestehendem Leberparenchymschaden. Im Nebenwirkungsspektrum von Katadolon und Ibu 600 1A-Pharma ist eine Hepatitis gut bekannt. Katadolon und Valoron N retard wurden zur gleichen Zeit verabreicht. Ibu 600 begann früher und wurde zeitweise pausiert. Daher ist eine genauere Zuordnung zu einem bestimmten Medikament nicht möglich.

Katadolon 100 mg Kaps (AWD Pharma), Einnahmezeitraum: 17.02. - 16.04.2010

Valoron N retard bzw. Tilidin N Sandoz 100/8 mg Retardtbl. (Pfizer bzw. Sandoz), Einnahmezeitraum: 17.02. - 16.04.2010

Ibu 600 Filmtabl. (1A Pharma), Einnahmezeitraum: 15.-17.02.2010, 03.03.-12.04.2010

Die UAW hat sich ambulant entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10071135

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials: Sex: Male Birth: 19800902 Age: 29 Year weight (kg): 83 BMI_int: 26.79 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Scleral icterus					Unknown
2 Vomiting					Unknown
3 Upper abdominal pain					Unknown
4 Tenderness epigastric					Unknown
5 Acute pancreatitis	20091227	20100115	20 Day	26 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	800 Mg milligram(s)/ 245	20091223-20100104 34 Day	Drug use for unknown indication
2 ibuprofen 800mg	Oral	3*800 Mg milligram(s)/ Da 245	20091202-20100104 34 Day	Pain

Drugs (concomitant)

3 clexane 40mg	Subcutaneous	40 Mg milligram(s)/ Day 120	20091202-20100107 37 Day	Thrombosis prophylaxis
4 distraneurin-kapseln 192mg	Oral	3*384 Mg milligram(s)/ Da 7	20091202-20091207 6 Day	Prophylaxis against alcoholic withdrawal syndrome
5 pantozol 40 mg	Oral	40 Mg milligram(s)/ Day 94	20091202- -	Gastric ulcer prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Malleolar fracture	200912	200912	No	OP am 02.12.2009 Zugschraube und Plattenosteosynthese
Alcohol abuse	2007		Yes	seit ca. 3 Jahren 5 Bier/Tag
Smoker			Yes	Beginn und Menge unbekannt
Pain	200912		Yes	nach Fraktur des oberen Sprunggelenkes und operativer Versorgung

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Creatinine	20100108	63	µmol/l	
Creatinine	20100112	76	µmol/l	
Creatinine	20100115	76	µmol/l	
Aspartate aminotransferase	20100108	0.87	µkat/l	
Aspartate aminotransferase	20100110	0.68	µkat/l	
Aspartate aminotransferase	20100112	0.79	µkat/l	
Aspartate aminotransferase	20100115	0.87	µkat/l	
Alanine aminotransferase	20100108	0.88	µkat/l	

Alanine aminotransferase	20100110	0.65	µkat/l
Alanine aminotransferase	20100112	0.75	µkat/l
Alanine aminotransferase	20100115	0.98	µkat/l
Bilirubin total	20100108	16.8	µmol/l
Bilirubin total	20100110	17.4	µmol/l
Bilirubin total	20100112	22.4	µmol/l
Alkaline phosphatase	20100108	1.59	µkat/l
Alkaline phosphatase	20100110	1.49	µkat/l
Gamma GT	20100108	4.62	µkat/l
Gamma GT	20100110	3.92	µkat/l
Gamma GT	20100112	3.87	µkat/l
Gamma GT	20100115	3.14	µkat/l
Thrombocyte count	20100108	368	Gpt/l
Haemoglobin	20100108	10.3	mmol/l
Haematocrit	20100108	0.48	SI-Einheit
Total white blood cell count	20100108	6.2	10E9/l
C-reactive protein	20100108	<3	mg/l
C-reactive protein	20100110	63.9	mg/l
C-reactive protein	20100112	18.5	mg/l
C-reactive protein	20100115	<3	mg/l
Amylase	20100115	2.43	µkat/l
Lipase	20100108	8.1	µkat/l
Lipase	20100110	2.3	µkat/l
Lipase	20100112	2.9	µkat/l
Lipase	20100115	3.6	µkat/l
Blood potassium	20100108	3.8	mmol/l
Blood potassium	20100115	4.0	mmol/l
Sodium	20100108	142	mmol/l
Sodium	20100115	137	mmol/l
Blood calcium	20100110	2.6	mmol/l
Blood calcium	20100112	2.7	mmol/l
Triglycerides	20100115	1.2	mmol/l
Occult blood	20100115	-	positiv/neg:

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 19.05.2010 DE-PVZIBE-02-10275

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
akute Pankreatitis

KURZZUSAMMENFASSUNG:
akute Pankreatitis unter Ibuprofen-Einnahme bei Alkoholabhängigkeit

BESCHREIBUNG:
seit ca. 2 Wochen täglich morgendliches Erbrechen (gallig), kein Fieber oder Teerstuhl, seit 1 Tag heftigste Oberbauchschmerzen;
anamnestisch: chronischer Nikotin- und Alkoholabusus (5 Bier/Tag), zur Zeit keine Alkohol-Exzesse;
bei Aufnahme: Sklerenikterus, epigastrischer Druckschmerz
THERAPIE:

08.-12.01.2010 Flüssigkeitsinfusionen, Gabe von Pantozol, Berlosin, MCP und Dipidolor;

ab 08.01.2010 Nahrungskarenz (Tee), ab 12.01. wieder Kostaufbau;

Entlassungsmedikation: Pantozol, Fragmin

VERLAUF:

08.01.2010 Sonographie Abdomen: Steatosis hepatis, keine fokalen Herde, Gallenblase und Gallenwege regelrecht, Pankreas nicht gut überschaubar, im Kopfbereich vergrößert und echogemindert, Verdacht auf akute Pankreatitis;

stationär keine Entzugssymptomatik;

ab 10.01.2010 Besserung der Übelkeit, kein Erbrechen mehr, weniger Schmerzen;

11.01.2010 psychologisches Konsil: Alkoholabhängigkeit, gegenwärtig abstinenter in geschützter Umgebung, Erläuterungen zum Entzug;

16.01.2010 Entlassung bei subjektivem Wohlbefinden

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 08.-16.01.2010;

Der Patient wurde mit heftigen Oberbauchschmerzen bei Verdacht auf akute Pankreatitis stationär eingewiesen.

Seit ca. 14 Tagen zunehmende Symptomatik bei seit 5 Wochen bestehender Ibuprofen-Einnahme wegen Sprunggelenkfraktur mit operativer Versorgung und seit 3 Jahren bestehendem regelmäßigem Alkoholkonsum von 5 Bier pro Tag. Unter der Therapie (Nahrungs-, Alkohol-, Ibuprofen-Karenz) deutliche Besserung der Symptomatik und Sinken der Lipase-Werte. Es trat erstmalig bei seit 3 Jahre bestehendem regelmäßigem Alkoholkonsum bei kurzfristiger Ibuprofen-Einnahme (nach 3 Wochen Symptombeginn) eine akute Pankreatitis auf. Nach Absetzen aller Noxen Besserung des subjektiven Befindens und sinkende Laborparameter. Es besteht der Verdacht einer multifaktoriellen Pankreasschädigung. Es besteht kein Anhalt für Fett-induzierte, infektiös-bedingte oder durch Gallensteine-bedingte Genese.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: ibuprofen 800mg

BETREFFENDE UAW: akute Pankreatitis

ERLÄUTERUNG: Nach 3 wöchiger Einnahme von Ibuprofen begann die Symptomatik, die nach Absetzen wieder abklang. Symptomatologisch ist eine zusätzliche Schädigung durch Alkoholkonsum (kein akuter Exzess) nicht auszuschließen. Akute Pankreatitis ist als seltenes Ereignis in der Fachinfo für Ibuprofen beschrieben. Es ergaben sich keine Hinweise auf eine biliäre Genese oder ein Trauma.

Insgesamt sind derzeit zwei mögliche Noxen zu betrachten: Ibuprofen und Alkohol. Ersteres wurde kurzzeitig vor dem Ereignis in hoher Dosierung neu angesetzt - letzteres seit Jahren chronisch konsumiert. Beide potentiellen Noxen wurden stationär abgesetzt, worunter es zu einer Besserung der Symptome kam.

HELIOS Klinikum Wuppertal, 01.06.2010 DE-PVZIBE-02-10275

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

akute Pankreatitis

KURZZUSAMMENFASSUNG:

akute Pankreatitis unter Ibuprofen-Einnahme bei Alkoholabhängigkeit

BESCHREIBUNG:

seit ca. 2 Wochen täglich morgendliches Erbrechen (gallig), kein Fieber oder Teerstuhl, seit 1 Tag heftigste Oberbauchschmerzen;

anamnestisch: chronischer Nikotin- und Alkoholabusus (5 Bier/Tag), zur Zeit keine Alkohol-Exzesse;

bei Aufnahme: Sklerenikterus, epigastrischer Druckschmerz

THERAPIE:

08.-12.01.2010 Flüssigkeitsinfusionen, Gabe von Pantozol, Berlosin, MCP und Dipidolor;

ab 08.01.2010 Nahrungskarenz (Tee), ab 12.01. wieder Kostaufbau;

Entlassungsmedikation: Pantozol, Fragmin

VERLAUF:

08.01.2010 Sonographie Abdomen: Steatosis hepatis, keine fokalen Herde, Gallenblase und Gallenwege regelrecht, Pankreas nicht gut überschaubar, im Kopfbereich vergrößert und echogemindert, Verdacht auf akute Pankreatitis;

stationär keine Entzugssymptomatik;

ab 10.01.2010 Besserung der Übelkeit, kein Erbrechen mehr, weniger Schmerzen;

11.01.2010 psychologisches Konsil: Alkoholabhängigkeit, gegenwärtig abstinenter in geschützter Umgebung, Erläuterungen zum Entzug;

16.01.2010 Entlassung bei subjektivem Wohlbefinden

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 08.-16.01.2010;

Der Patient wurde mit heftigen Oberbauchschmerzen bei Verdacht auf akute Pankreatitis stationär eingewiesen.

Seit ca. 14 Tagen zunehmende Symptomatik bei seit 5 Wochen bestehender Ibuprofen-Einnahme wegen Sprunggelenkfraktur mit operativer Versorgung und seit 3 Jahren bestehendem regelmäßigem Alkoholkonsum von 5 Bier pro Tag. Unter der Therapie (Nahrungs-, Alkohol-, Ibuprofen-Karenz) deutliche Besserung der Symptomatik und Sinken der Lipase-Werte. Es trat erstmalig bei seit 3 Jahre bestehendem regelmäßigem Alkoholkonsum bei kurzfristiger Ibuprofen-Einnahme (nach 3 Wochen Symptombeginn) eine akute Pankreatitis auf. Nach Absetzen aller Noxen Besserung des subjektiven Befindens und sinkende Laborparameter. Es besteht der Verdacht einer multifaktoriellen Pankreasschädigung. Es besteht kein Anhalt für Fett-induzierte, infektiös-bedingte oder durch Gallensteine-bedingte Genese.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: ibuprofen 800mg

BETREFFENDE UAW: akute Pankreatitis

ERLÄUTERUNG: Nach 3 wöchiger Einnahme von Ibuprofen begann die Symptomatik, die nach Absetzen wieder abklang. Symptomatologisch ist eine zusätzliche Schädigung durch chronischen Alkoholkonsum (kein akuter Exzess) nicht auszuschließen. Akute Pankreatitis ist als seltenes Ereignis in der Fachinfo für Ibuprofen beschrieben. Es ergaben sich keine Hinweise auf eine biliäre Genese oder ein Trauma.

Insgesamt sind derzeit zwei mögliche Noxen zu betrachten: Ibuprofen und Alkohol. Ersteres wurde kurzzeitig vor dem Ereignis in hoher Dosierung neu angesetzt - letzteres seit Jahren chronisch konsumiert. Im Zusammenhang mit der operativen Versorgung der malleolarfraktur Anfang Dezember 2009 war dem Patienten prophylaktisch Distraneurin zur Prophylaxe eines Alkoholentzuges verabreicht worden, ohne dass im Vorfeld eine Entzugssymptomatik beobachtet wurde. Bei dem jetzigen stationären Aufenthalt trat keine Entzugssymptomatik (ohne begleitende Distraneuringabe) auf. Beide potentiellen Noxen wurden stationär abgesetzt, worunter es zu einer Besserung der Symptome kam.<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 02.06.2010 DE-SANOFI-AVENTIS-2010SA029242

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10071135).

The pharmacovigilance (PV) centre is conducting a hospitalisation registry. Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case involves a 29-year-old male smoking patient with a malleolar fracture in December 2009 wherefore he underwent surgery with traction screw and plate osteosynthesis on 02-Dec-2009. Due to condition after fracture of upper ankle and operative maintenance the patient suffered from pain since December 2009. Since circa 2007 the patient had drunk five beers daily. The following drugs were reported to have been taken concomitantly: pantoprazole for gastric ulcer, clomethiazole edisilate as prophylaxis against alcoholic withdrawal syndrome and enoxaparin sodium as thrombosis prophylaxis. He received ibuprofen (no trade name provided) 2400 mg daily from 02-Dec-2009 to 23-Dec-2009 due to pain; from 23-Dec-2009 onwards ibuprofen 800 mg was taken if necessary. At unspecified date in December 2009 the patient complained about liverish vomiting in the morning for two weeks; no fever; no black stool. Furthermore he developed severe upper abdominal pain; diagnosis: acute pancreatitis since 27-Dec-2009. The patient was hospitalized on 08-Jan-2010 due to suspicion of acute pancreatitis. Within admission the patient suffered from scleral icterus and epigastric tenderness. An abdomen sonography on 08-Jan-2010 revealed following results: steatosis hepatitis, no focal focus, gallbladder and bile ducts inconspicuous, pancreas not well clear, in head area augmented and echo decreased; suspicion of acute pancreatitis. During hospitalization the patient showed no withdrawal signs. On 10-Jan-2010 the patient noticed improvement of vomiting (no vomiting furthermore) as well as less pain. An alcohol abuse (actually abstinent) was mentioned within a psychological council on 11-Jan-2010. Corrective treatment included fluid retention, pantoprazole sodium, metamizole sodium as well as metoclopramide hydrochloride from 08-Jan-2010 to 12-Jan-2010; fasting from 08-Jan-2010 onwards (only tea), from 12-Jan-2010 onwards slowly food construction. During corrective treatment signs and symptoms resolved as well as decreasing of lipase values. The suspicion of multifactorial pancreas damage was mentioned. Signs and symptoms resolved on 15-Jan-2010 and the patient was discharged from the hospital on 16-Jan-2010. Medication at discharge from the hospital: pantoprazole sodium; dalteparin sodium.

Reporter causality (method: WHO Score): possible.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10083649

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 53 Year	weight (kg): 90	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt	20100601	20100601		1 Day	not recovered/not resolved
2 Alcohol abuse	20100601			1 Day	not recovered/not resolved
3 Alanine aminotransferase increased	20100601			1 Day	recovering/resolving
4 Aspartate aminotransferase increased	20100601			1 Day	recovering/resolving
5 Blood creatine phosphokinase increased	20100601			1 Day	recovering/resolving
6 Blood ethanol increased					recovering/resolving
7 Multiple drug overdose intentional	20100601			1 Day	Unknown
8 Elevated liver enzyme levels	20100601			1 Day	recovering/resolving
9 Gamma-glutamyltransferase increased	20100601			1 Day	recovering/resolving
10 Orientation disturbed					Unknown
11 Prothrombin time increased	20100601			1 Day	recovering/resolving
12 Suicide attempt	20100601			1 Day	Unknown
13 Alcohol use	20100601			1 Day	Unknown
14 Blood lactic acid increased	20100601			1 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Betablocker	Oral	/	20100601-20100601 1 Day	Suicide attempt
2 ibuprofen	Oral	50 Df dosage form/Day	20100601-20100601 1 Day	Suicide attempt
3 TRAMADOL	Oral	/	20100601-20100601 1 Day	Suicide attempt
4 VALDOXAN 25 mg	Oral	2450 Mg milligram(s)/Day 82	20100601-20100601 1 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Suicide attempt			Unknown	
Depression			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20100601	89	IU/L	
Aspartate aminotransferase	20100602	49	IU/L	
Alanine aminotransferase	20100601	70	IU/L	
Alanine aminotransferase	20100602	53	IU/L	
GGT	20100601	178	IU/L	
GGT	20100602	139	IU/L	

Lactic acid	20100601	299	IU/L
Creatine kinase	20100601	282	IU/L
Ethanol	20100601	300	mg/dl
Quick's test	20100601	60	%
Quick's test	20100602	58	%

Narratives (nur bei elektronischem Eingang)SERVIER, 09.06.2010 DE-SERVIER-S10002269

Sender diagnosis: not reported

Narrative

This case was received from a Physician working in the hospital at the emergency ward and via a Poison emergency centre in GERMANY. This is a 53-year-old female patient, with a weight of 90 kg, with a medical history of depression since many years, with previous suicide attempt. It is unknown if she had alcohol or drug addiction. The onset date of this episode of depression was not reported and it is not known whether the patient was treated with VALDOXAN before the events. Concomitant treatments were not reported. In the evening (7 pm) of 01-JUN-2010 she took multiple drugs in overdose: unknown amount of Tramadol, 50 tablets of Ibuprofen, unknown amount of betablocker and 98 tablets of VALDOXAN with one litre of wine, with suicidal intention, leading to hospitalisation. The patient was awake, had no circulatory disturbances, but was not fully orientated. At admission lab values showed elevated liver enzymes: ASAT 89 IU/L, ALAT 70 IU/L, LDH 299 IU/L, GGT 178 IU/L, Blood creatine phosphokinase 282, Quick 60% and ethanol level of 300 mg/dL. On 02-JUN-2010 the enzymes decreased: ASAT was 49 IU/L, ALAT 53 IU/L, GGT 139 IU/L and Quick was 58%. As these values were decreasing, on 03-JUN-2010 no blood sample was performed and on the same day the patient was transferred to a psychiatric hospital in stable condition. Unknown not recovered at the time of reporting.

SERVIER, 15.06.2010 DE-SERVIER-S10002269

Sender diagnosis: not reported

Narrative

This case was received from a Physician working in the hospital at the emergency ward and via a Poison emergency centre in GERMANY. This is a 53-year-old female patient, with a weight of 90 kg, with a medical history of depression since many years, with previous suicide attempt. It is unknown if she had alcohol or drug addiction. The onset date of this episode of depression was not reported and it is not known whether the patient was treated with VALDOXAN before the events. Concomitant treatments were not reported. In the evening (7 pm) of 01-JUN-2010 she took multiple drugs in overdose: unknown amount of Tramadol, 50 tablets of Ibuprofen, unknown amount of betablocker and 98 tablets of VALDOXAN with one litre of wine, with suicidal intention, leading to hospitalisation. The patient was awake, had no circulatory disturbances, but was not fully orientated. At admission lab values showed elevated liver enzymes: ASAT 89 IU/L, ALAT 70 IU/L, LDH 299 IU/L, GGT 178 IU/L, Blood creatine phosphokinase 282, Quick 60% and ethanol level of 300 mg/dL. On 02-JUN-2010 the enzymes decreased: ASAT was 49 IU/L, ALAT 53 IU/L, GGT 139 IU/L and Quick was 58%. As these values were decreasing, on 03-JUN-2010 no blood sample was performed and on the same day the patient was transferred to a psychiatric hospital in stable condition. Unknown not recovered at the time of reporting.

NOVARTIS, 16.06.2010 DE-002147023-PHHY2010DE37662

Sender diagnosis: not reported

Narrative

Initial report Physician working in the hospital at the emergency ward and via a Poison emergency centre on 02 Jun 2010 and forwarded by Servier (S10002269) on 10 Jun 2010: This patient had a concurrent condition as depression since many years, with previous suicide attempt. It was unknown if she had alcohol or drug addiction. The onset date of this episode of depression was not reported and it is not known whether the patient was treated with Valdoxan (agomelatine) before the events. It was reported that, in the evening at 7 pm of 01 Jun 2010 she took multiple drugs in overdose: unknown amount of Tramadol, 50 tablets of Ibuprofen, unknown amount of beta-blocker and 98 tablets of Valdoxan with one liter of wine, with suicidal intention, leading to hospitalisation. The patient was awake, had no circulatory disturbances, but was not fully orientated. At admission lab values showed elevated liver enzymes: ASAT 89 IU/L, ALAT 70 IU/L, LDH 299 IU/L, GGT 178 IU/L, Blood creatine phosphokinase 282, Quick 60% and ethanol level of 300 mg/dL. On 02 Jun 2010 the enzymes decreased: ASAT was 49 IU/L, ALAT 53 IU/L, GGT 139 IU/L and quick was 58%. As these

values were decreasing, on 03-JUN-2010 no blood sample was performed and on the same day the patient was transferred to a psychiatric hospital in stable condition. The outcome for the other events was not reported.

All remaining reported leading events were assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10099350

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth: 19690115	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicidal intention					recovered/resolved
2 Elevated liver enzymes					recovered/resolved
3 Drug intoxication					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Alcohol		/	-	
ethanol			-	
2 DICLOFENAC		/	-	
diclofenac diethylamine			-	
3 IBUPROFEN		/	-	
ibuprofen			-	
4 LOPERAMIDE		/	-	
loperamide hydrochloride			-	
5 pantozol 40 mg		1200 Mg milligram(s)/	-	
pantoprazole			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Nycomed, 06.07.2010 DE-Nycomed-0212173

Sender diagnosis: not reported

Narrative

The case was reported by Nycomed Germany with the ref.no. AZ2010009529.

It concerned a patient born on 15-JAN-1969 (gender not reported) who took 1.2 g at once (30 tablets of 40 mg) of Pantozol (pantoprazole) associated with loperamide, ibuprofen, diclofenac and a big amount of alcohol (mixed drug intoxication with suicidal intention) on an unspecified date. Neither medical history nor concomitant treatment was reported. The patient was admitted in stationary treatment and was monitored with ECG and respiratory monitoring. About 11 hours after the intake, the patient was awake, orientated and in good general condition. Lightly increased liver values (GOT, GPT and Gamma-GT) were detected through laboratory values (no values provided). It was specified that an alcohol abuse could not be ruled out.

Outcome of the adverse reactions was not reported.

are involved and are usually not considered to have any informative value in terms of the risk profiles of the drugs taken. Drug intoxication or combined drug intoxication is due to the simultaneous use of multiple drugs. Signs of drug intoxication include heart failure (arrhythmias and shock), respiratory failure, lack of consciousness, confusion, convulsions.

The causal relationship between pantoprazole and the adverse reaction of suicidal intention is considered not related and rather related to one of the above mentioned causes.

Gamma-glutamyl transferase levels rise in hepatobiliary disease, especially cholestasis, and correlates loosely with levels of alkaline phosphatase and 5'-nucleotidase. Levels also increase with induction of microsomal enzymes (e.g. by ingesting certain drugs, such as anticonvulsants, and particularly in alcoholics). An increase in serum GGT also predicts onset of metabolic syndrome or congestive heart failure. Aminotransferases (alanine aminotransferase and aspartate aminotransferase) increase when there is damage to the liver cell membrane resulting in increased permeability and therefore increase of these enzymes in blood. Acute hepatocellular necrosis or injury (e.g. hepatitis) typically results in high values. Modest elevations persist in chronic liver diseases. ASAT (GOT) may be elevated because of rhabdomyolysis or damage to heart or brain tissue. In most acute hepatocellular disorders, the ALAT (GPT) is higher than or e

Reporter's comment:Nycomed, 09.07.2010 DE-Nycomed-0212173

Sender diagnosis: not reported

Narrative

The case was reported by Nycomed Germany with the ref.no. AZ2010009529.

It concerned a patient born on 15-JAN-1969 (gender not reported) who took 1.2 g at once (30 tablets of 40 mg) of Pantozol (pantoprazole) associated with loperamide, ibuprofen, diclofenac and a big amount of alcohol (mixed drug intoxication with suicidal intention) on an unspecified date. Neither medical history nor concomitant treatment was reported. The patient was admitted in stationary treatment and was monitored with ECG and respiratory monitoring. About 11 hours after the intake, the patient was awake, orientated and in good general condition. Lightly increased liver values (GOT, GPT and Gamma-GT) were detected through laboratory values (no values provided). It was specified that an alcohol abuse could not be ruled out.

Outcome of the adverse reactions was not reported.

Follow-up information received on 07-JUL-2010:

The patient was discharged from hospital after 24 hours of observation without having any complications.

above mentioned causes.

Gamma-glutamyl transferase levels rise in hepatobiliary disease, especially cholestasis, and correlates loosely with levels of alkaline phosphatase and 5'-nucleotidase. Levels also increase with induction of microsomal enzymes (e.g. by ingesting certain drugs, such as anticonvulsants, and particularly in alcoholics). An increase in serum GGT also predicts onset of metabolic syndrome or congestive heart failure. Aminotransferases (alanine aminotransferase and aspartate aminotransferase) increase when there is damage to the liver cell membrane resulting in increased permeability and therefore increase of these enzymes in blood. Acute hepatocellular necrosis or injury (e.g. hepatitis) typically results in high values. Modest elevations persist in chronic liver diseases. ASAT (GOT) may be elevated because of rhabdomyolysis or damage to heart or brain tissue. In most acute hepatocellular disorders, the ALAT (GPT) is higher than or e

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10102021

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes

Patient

Initials: Sex: Male Birth: 20081007 Age: 16 Month weight (kg): 8 BMI_int: 13.49 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Marbled skin	20100208		1 Day	1 Day	recovered/resolved
2 Temperature elevation	20100208		1 Day	1 Day	recovered/resolved
3 Tachycardia	20100208		1 Day	1 Day	recovered/resolved
4 Vomiting	20100208		1 Day	1 Day	recovered/resolved
5 Incorrect drug dosage form administered	20100208		1 Day	1 Day	recovered/resolved
6 Trembling	20100208		1 Day	1 Day	recovered/resolved
7 Aspartate aminotransferase increased	20100208		1 Day	1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBU-ratiopharm 2 % Fiebersaft für Kinder ibuprofen	Oral	/ 170	20100208- -	Fever

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Otitis media			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20100209	0.92	µmol/l*s	
Aspartate aminotransferase	20100211	0.92	µmol/l*s	
CRP increased	20100211	3.51	mg/l	
MCH	20100208	1.51	fmol	
MCH	20100211	1.49	fmol	
MCV	20100208	73.0	fl	
MCV	20100211	73.6	fl	
Lymphocytes	20100211	66	%	
CRP increased	20100208	31.2	mg/l	
PCO2 abnormal	20100208	5.60	KPa	
PO2 abnormal	20100208	2.40	KPa	
O2 saturation	20100208	26	%	

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 09.07.2010 DE-RATIOPH-2010R0604622

Sender diagnosis: not reported

Narrative

Case reference number 2010R0604622 is a spontaneous case report sent by a physician which refers to a male aged 16 months.

The patient was hospitalised because of Otitis media (Otitis media). At night on 08-Feb-2010, the child should receive as usual, indication Fever, IBU-ratiopharm 2 % Fiebersaft für Kinder with the applicator enclosed in the package. Because the child was asleep, the mother insisted on giving the drug when it is awake. The mother inadvertently administered intravenously the liquid through the inserted venous cannula.

The patient developed Tachycardia (Tachycardia)(start date 08-Feb-2010), Temperature elevation to 40 degree (Body temperature increased)(start date 08-Feb-2010), Marbled skin (Livedo reticularis)(start date 08-Feb-2010), Vomiting (Vomiting)(start date 08-Feb-2010), ASAT value increased (Aspartate aminotransferase increased)(start date 08-Feb-2010) and Trembling (Tremor)(start date 08-Feb-2010).

The following action was taken: for IBU-ratiopharm 2 % Fiebersaft für Kinder (IBUPROFEN) - Unknown

The outcome for Temperature elevation to 40 degree (Body temperature increased), Marbled skin (Livedo reticularis), ASAT value increased (Aspartate aminotransferase increased), Tachycardia (Tachycardia), Vomiting (Vomiting) and Trembling (Tremor) is Recovered / Resolved.

Rechallenge information for IBU-ratiopharm 2 % Fiebersaft für Kinder is not applicable. Dechallenge information for IBU-ratiopharm 2 % Fiebersaft für Kinder is not applicable.

The reporter's assessment for drug IBU-ratiopharm 2 % Fiebersaft für Kinder related with Marbled skin, ASAT value increased, Temperature elevation to 40 degree, Tachycardia, Vomiting is as follows: Probable. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: Yes, Pregnancy / Lactation tracking: No

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-10109552

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicidal intention					recovered/resolved
2 Drug intoxication					recovered/resolved
3 Hepatic enzymes increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Alkohol	Unknown	/	-	Drug use for unknown indication
ethanol			-	indication
2 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac			-	indication
3 ibuprofen	Unknown	/	-	Drug use for unknown indication
ibuprofen			-	indication
4 Loperamide	Unknown	/	-	Drug use for unknown indication
loperamide			-	indication
5 pantozol 40 mg	Unknown	/	-	Drug use for unknown indication
pantoprazole			-	indication

Primary source

Qualification: Other Health Professional observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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GPT

Aspartate aminotransferase

Gamma GT

Narratives (nur bei elektronischem Eingang)

RANBAXY, 20.07.2010 DE-RANBAXY-2010US-35591

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2010US-35591 from GERMANY refers to a UNK, 41 years-old who reportedly had suicidal intention, drug toxicity and hepatic enzymes increased while on treatment with Ibuprofen unk, unk, loperamide unk, unk and Diclofenac unk, unk for unknown indication. The event required patient hospitalization.

This is a business partner case forwarded By Nycomed.
Per the report, the case was reported by Nycomed, Germany.

It concerned a patient born on 15-Jan-1969 (gender not reported) who took 1.2 gram at once (30 tablets of 40 mg) of Pantozol(Pantoprazole) associated with Loperamide, Ibuprofen, Diclofenac and a big amount of alcohol (Mixed drug intoxication with suicidal intention) on an unspecified date. Neither medical history nor concomitant treatment was reported. The patient was admitted in stationary treatment and was monitored with ECG and respiratory monitoring. About 11 hours after the intake, the patient was awake, oriented and in good general condition. Lightly increased liver values (GOT, GPT and Gamma-GT) were detected through laboratory values (no values provided). It was specified that an alcohol abuse could not be ruled out.

Outcome of the adverse reaction were reported as resolved.

Company (Nycomed) Remark:

Mixed drug intoxication and suicidal intention are unlisted in the current reference safety information of Pantoprazole.

Increased liver values are listed in the current reference safety information of Pantoprazole.

The causal relationship between Pantoprazole and the adverse reaction of suicidal intension is considered not related and rather related to one of the above mentioned causes.

This single case report is not considered to change the current knowledge of the safety profile of Pantoprazole.

Case Outcome: Resolved

F/U # 1: 12-Jul-2010 : (Significant):

Follow-up information was received on 12-Jul-2010.

Per the report, patient was discharged from hospital after 24 hours of observation without having any complications.

The event outcome was reported as resolved at the time of this report.

The case is deemed serious. The events are unexpected as per USPI and are possibly related to the suspect drugs as per WHO UMC system for standardized causality assessment.

Case Outcome: Resolved

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10123729

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes Yes Yes

Patient

Initials: Sex: Female Birth: 19211219 Age: 88 Year weight (kg): 46 BMI_int: 17.97 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20100711			4 Month	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibu akut, 400 mg, Filmtabl, 1A-Pharma ibuprofen	Oral	/ 82	20100104-20100710 6 Month	
2 Salbutamol, Dosieraerosol, Sandoz salbutamol	Respiratory (inhalation)	/ 109	20100414-201007 -	COPD
3 Torasemid, 10 mg, Tabl, 1A-Pharma torasemide	Oral	15 Mg milligram(s)/Day 245	20100301-20100712 4 Month	

Drugs (concomitant)

4 Folsäure, 5 mg, Tabl, NN folic acid	Oral	5 Mg milligram(s)/Week 245	200801- -
5 Pantoprazol, 20 mg, Tabl (mr), Acis pantoprazole	Oral	20 Mg milligram(s)/Day 94	200912- -
6 Predni H Tablinen, 5 mg, Tabl, Winthrop prednisolone	Oral	5 Mg milligram(s)/Day 245	200801- -
7 Ramilich, 2,5 mg, Tabl, Winthrop ramipril	Oral	2.5 Mg milligram(s)/Day 245	20100215- -
8 Spirobeta, 50 mg, Tabl, Betapharm spironolactone	Oral	25 Mg milligram(s)/Day 245	20100615- -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Decompensated heart failure			Yes	
Arthritis rheumatoid			Yes	
COPD			Yes	
Hypertension arterial			Yes	
Chronic atrial fibrillation			Yes	

Primary source

Qualification: Physician observestudytype:
 Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase normal	20090714	32	U/l	
Alanine aminotransferase increased	20100712	155	U/l	
Alanine aminotransferase increased	20100722	78.6	U/l	
Aspartate aminotransferase increased	20090714	37	U/l	
Aspartate aminotransferase increased	20100712	139	U/l	
Aspartate aminotransferase increased	20100722	47.4	U/l	
Alkaline phosphatase increased	20100712	545	U/l	

Alkaline phosphatase increased	20100722	469	U/l
Bilirubin increased	20100712	16.6	mg/dl
Bilirubin increased	20100716	24.4	mg/dl
Bilirubin increased	20100722	14.7	mg/dl
Alanine aminotransferase increased	20100711	107	U/l
Alanine aminotransferase normal	20101008	26	U/l
Aspartate aminotransferase increased	20100711	93.6	U/l
Aspartate aminotransferase increased	20101008	42	U/l
Bilirubin increased	20100711	11.1	mg/dl
Bilirubin normal	20101006	0.9	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 16.08.2010 DE-FAKOS-170341

Sender diagnosis: not reported

Narrative

Sender's comment:

Initial report (more info follows).

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 19.10.2010 DE-FAKOS-170341

Sender diagnosis: not reported

Narrative

Am 11.07.2010 erfolgte die stationäre Aufnahme der Patientin wegen Verschlechterung des Allgemeinzustandes. Es bestand ein Ikterus der Skleren und der Haut bei Aufnahme. Die Laboruntersuchung ergab folgende Werte für Aminotransferasen und Bilirubin (ALAT: 107 U/l (Normbereich: <32 U/l), ASAT: 93,6 U/l (Normbereich: <33 U/l), Bilirubin gesamt: 11,1 mg/dl (Normbereich: <1 mg/dl). ALAT und ASAT waren im Jahr davor, am 14.07.2009, noch im Normbereich gewesen (ALAT: 32 U/l, ASAT: 37 U/l).

Am 10.07.2010 Ende der Therapie mit Ibuprofen (Einnahme seit 04.01.2010). Außerdem wurde im Juli 2010 auch letztmalig Salbutamol eingenommen (Beginn 14.04.2010), am 12.07.2010 auch Ende der Therapie mit Torasemid (Beginn 01.03.2010). Die Maximalwerte der Transaminasen und des Bilirubins wurden erreicht: ALAT: 155 U/l, ASAT: 139 U/l (am 12.07.2010) und Bilirubin gesamt: 24,4 mg/dl (am 16.07.2010). Im Verlauf kam es zu einer langsamen aber deutlichen Besserungstendenz der Aminotransferasen und der Cholestaseparameter bei noch erhöhter Gamma-GT.

Die Leberbiopsie vom 15.07.2010 ergab folgenden Befund: Bild wie bei einer medikamentös-toxischen Leberschädigung mit ausgeprägter Cholestase und teilweise fibrotischen Veränderungen (etwas länger bestehender Prozess?). Kein Hinweis für eine extrahepatische Gallenabflussstörung. Die Hepatitis-Serologie (A, B, C) war negativ.

Am 02.08.2010 wurde die Patientin nach Besserung des Allgemeinzustandes aus dem Krankenhaus entlassen. Laborwerte vom 08.10.2010: ALAT: 26 U/l, ASAT: 42 U/l, Bilirubin am 06.10.2010: 0,9 mg/dl.

Eine medikamenteninduzierte Genese der Hepatitis ist wahrscheinlich.

Die UAW hat sich ambulant entwickelt.

Als Auslöser werden folgende Arzneimittel verdächtigt:

Torasemid 10 mg Tabl., 1A-Pharma, Einnahmezeitraum: 01.03.-12.07.2010,

Ibu 400 akut Filmtabl., 1A-Pharma, Einnahmezeitraum: 04.01.-10.07.2010 und

Salbutamol Dosieraerosol, Sandoz, Einnahmezeitraum: 14.04.-07/2010.

Die Arzneimittelkausalität wird jeweils mit möglich bewertet.<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10130917

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 19600219	Age: 41 Year	weight (kg): 147	BMI_int: 42.95	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blood pressure increased					Unknown
2 Diabetes mellitus					not recovered/not resolved
3 Forgetfulness					Unknown
4 Hepatic disorder NOS	2002				not recovered/not resolved
5 Increased intraocular pressure					Unknown
6 Joint pain	2002				Unknown
7 Chondrolysis					Unknown
8 Metabolic disorder NOS					not recovered/not resolved
9 Spleen disorder NOS	2002				not recovered/not resolved
10 Unevaluable reaction					Unknown
11 Visual impairment					Unknown
12 Performance status decreased					Unknown
13 Libido decreased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac			-	indication
2 ibuprofen	Unknown	600 Mg milligram(s)/	-	Drug use for unknown indication
ibuprofen			-	indication
3 TELFAST	Oral	180 Mg milligram(s)/	-	Allergy NOS
fexofenadine hydrochloride		82	7 Month	

Drugs (concomitant)

4 Antihypertensiva	/	-	Drug use for unknown indication
		-	indication
5 CHOLESTEROL- AND TRIGLYCERIDE REDU	/	-	Drug use for unknown indication
		-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Type II diabetes mellitus			Yes	
Adrenal disorder NOS			Yes	
Impaired liver function			Yes	
Liver enlargement			Yes	
Spleen enlarged			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Blood pressure

UP TO 200 MMHG

Narratives (nur bei elektronischem Eingang)sanofi-aventis, 26.08.2010 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Reporter's comment:sanofi-aventis, 20.09.2010 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicite causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

sanofi-aventis, 09.12.2010 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

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Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicit causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

sanofi-aventis, 16.12.2010 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

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Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

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Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

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The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

Additional letter received from the patient on 08-Dec-2010:

The patient additionally wrote that due to the impaired adrenal function he wasn't no longer able to climb stairs. Additionally he became a diabetic and was unable to do physical work lasting longer than 1.5 hours. He stated that the companys drug had caused severe damages and he likely had no much time left to live. The patient specified the amount of compensation he claimed from the company.

sanofi-aventis, 04.01.2011 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicite causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

Additional letter received from the patient on 08-Dec-2010:

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Two additional letters received from the patient on 22-Dec-2010:

Besides already reported problems the patient provided some new statements: "The liver had torn off partially and therefore he often has to change his sitting position to avoid flow problems of ductus". Furthermore he mentioned "a severe metabolic disorder NOS which would become life-threatening. The liver was massive allergic to alcohol and would permanently be painful. In short, he was currently in an increasing progression. The damages without any doubt resulted from Telfast because he did not take any other drugs except ibuprofen and diclofenac sodium already mentioned previously." He again claimed for compensation and raised the amount of damage. In his view "Telfast was prescribed without complete information and without clarification of contraindication. The drug was prescribed in accordance with the state of the art of the time which was imperfect.

Additionally your drug was included in a list of particularly harmful drugs."

sanofi-aventis, 11.01.2011 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the

drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today.

Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicit causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

Additional letter received from the patient on 08-Dec-2010:

The patient additionally wrote that due to the impaired adrenal function he wasn't no longer able to climb stairs. Additionally he became a diabetic and was unable to do physical work lasting longer than 1.5 hours. He stated that the company's drug had caused severe damages and he likely had no much time left to live. The patient specified the amount of compensation he claimed from the company.

Two additional letters received from the patient on 22-Dec-2010:

Besides already reported problems the patient provided some new statements: "The liver had torn off partially and therefore he often has to change his sitting position to avoid flow problems of ductus". Furthermore he mentioned "a severe metabolic disorder NOS" which would become life-threatening. The liver was massive allergic to alcohol and would permanently be painful. In short, he was currently in an increasing progression. The damages without any doubt resulted from Telfast because he did not take any other drugs except ibuprofen and diclofenac sodium already mentioned previously." He again claimed for compensation and raised the amount of damage. In his view "Telfast was prescribed without complete information and without clarification of contraindication. The drug was prescribed in accordance with the state of the art of the time which was imperfect. Additionally your drug was included in a list of particularly harmful drugs."

Additional information received from the patient on 04-Jan-2011:

In a further letter the patient wrote: "Since 2010 both liver and spleen has been allergic at alcohol consumption. Libido is decreasing severely. Since 2008 I have been a diabetic and have to inject insulin permanently since 2009. I am disabled for the rest of my life and I am very forgetful. I suffer and suffered from increase in blood pressure up to 200 and from increase in intraocular pressure (20) while seated, even after hours with declining vision. Furthermore severe performance inefficiency is present." He again claimed for compensation.

sanofi-aventis, 28.09.2011 DE-SANOFI-AVENTIS-2010SA050121

Sender diagnosis: not reported

Narrative

Initial report from 23-Aug-2010:

This spontaneous case report from the consumer himself involves a male patient with unreported medical history and concomitant medication.

In a letter sent by the patient the case was described as follows:

For his allergy and because no other drugs had been effective his doctor prescribed fexofenadine (Telfast) 120 mg - 180 mg - 240 mg.

The consequences of treatment with the drug were severely damaging as the drug clogged liver and spleen leading to severe swelling of both organs under heavy pain - the liver approx. to the threefold of normal size and the spleen approx. up to two and a half of normal size. At each passage of the drug it destroyed ("hosed") and dilated the capillaries of the organs concerned.

Since that time his blood values were a catastrophe and to avoid a yellow - orange - brown discoloring of his face he had to drink coffee, which he did until today.

Because he wanted to avoid any action for damages as well as any activity of an expert and considering his poor health he would like to reach an agreement with the company.

This is all information received so far.

Additional information received on 16-Sep-2010:

An allergologist provided further information:

Concerning the patient's medical history he mentioned liver and spleen enlargement, impaired liver as well as impaired adrenal function and diabetes type II.

Concomitant medication included a blood pressure lowering agent NOS and a cholesterol lowering agent NOS.

Fexofenadine 180 mg (exact dosing regimen not mentioned) was given from 11-Oct-2004 to 01-Jun-2005 and after that never more. The event was reported as "spleen and liver damage" (as reported to him by the company), starting in 2004. He stated that the patient had problems till today. Reportedly, fexofenadine was withdrawn due to the occurrence of the event but the event worsened after this.

The reporter did not provide an explicit causality assessment but stated that "because the drug had no longer been given for five years this suspicion of causal relationship had to be stated with the utmost caution".

Additional information received on 11-Nov-2010:

The reporting consumer fixed a deadline for fulfilling his claim for compensation.

Additional information received on 01-Dec-2010:

The consumer now reported that the events "spleen and liver damage" started in 2005, not in 2004, in connection with the treatment with fexofenadine hydrochloride interacting with ibuprofen and diclofenac sodium. In further course at an unreported date, he additionally experienced "chondrolysis from the hip downwards and all other glands were damaged (reported as "shooting") via the capillary vessels". Outcome of the events was not reported. No further information was received so far.

Additional letter received from the patient on 08-Dec-2010:

The patient additionally wrote that due to the impaired adrenal function he wasn't no longer able to climb stairs. Additionally he became a diabetic and was unable to do physical work lasting longer than 1.5 hours. He stated that the company's drug had caused severe damages and he likely had no much time left to live. The patient specified the amount of compensation he claimed from the company.

Two additional letters received from the patient on 22-Dec-2010:

Besides already reported problems the patient provided some new statements: "The liver had torn off partially and therefore he often has to change his sitting position to avoid flow problems of ductus". Furthermore he mentioned "a severe metabolic disorder NOS" which would become life-threatening. The liver was massive allergic to alcohol and would permanently be painful. In short, he was currently in an increasing progression. The damages without any doubt resulted from Telfast because he did not take any other drugs except ibuprofen and diclofenac sodium already mentioned previously." He again claimed for compensation and raised the amount of damage. In his view "Telfast was prescribed without complete information and without clarification of contraindication. The drug was prescribed in accordance with the state of the art of the time which was imperfect. Additionally your drug was included in a list of particularly harmful drugs."

Additional information received from the patient on 04-Jan-2011:

In a further letter the patient wrote: "Since 2010 both liver and spleen has been allergic at alcohol consumption. Libido is decreasing severely. Since 2008 I have been a diabetic and have to inject insulin permanently since 2009. I am disabled for the rest of my life and I am very forgetful. I suffer and suffered from increase in blood pressure up to 200 and from increase in intraocular pressure (20) while seated, even after hours with declining vision. Furthermore severe performance inefficiency is present." He again claimed for compensation.

Additional information received from the patient on 25-Jan-2011:

The consumer informed the company that two further physicians he released from their obligation to secrecy had only prescribed ibuprofen and diclofenac but not fexofenadine.

Additional information received on 01-Aug-2011:

No new medical information was provided. He again claimed for compensation but claim for damages was lowered.

Additional information received on 13-Sep-2011:

The Drug commission of the German Association (DE-DCGMA-10154583) reported about duplicate information in the case 2011SA 18379 (received via webpage of health authority: DE-BfArM-11049925). Therefore this second case will be deleted and the case information was adjusted into the current case as follows: diclofenac (trade name not provided) and ibuprofen (CT) 600 mg were upgraded from concomitant to suspect. Contrary to the initial reports, the case onset date was specified with 2002. Initially the consumer reported about the first intake of fexofenadine hydrochloride on 11-Oct-2004 (until 01-Jun-2005). In this follow up the therapy dates were not provided. Furthermore the event joint pain was additionally entered.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-10137498

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 25 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Reversible posterior leukoencephalopathy syndrome					recovering/resolving
2 Right arm paresis					recovering/resolving
3 Inappropriate ADH secretion					recovering/resolving
4 Inappropriate antidiuretic hormone secretion					Unknown
5 Transaminases increased					Unknown
6 Demyelinating polyneuropathy					recovered/resolved
7 Disease aggravation					recovering/resolving
8 Encephalopathy					Unknown
9 Porphyria					not recovered/not resolved
10 Colonic pseudo-obstruction					recovering/resolving
11 Hypertension					Unknown
12 Porphyria aggravated					recovering/resolving
13 Acute intermittent porphyria					recovering/resolving
14 Constipation					recovering/resolving
15 Seizure					recovered/resolved
16 Polyneuropathy					recovering/resolving
17 Megacolon					recovering/resolving
18 Pain					recovering/resolving
19 Hyponatremia					recovering/resolving
20 Hallucination					recovering/resolving
21 Abdominal distension					recovering/resolving
22 Porphyria aggravated					recovering/resolving
23 Anaemia					Unknown
24 Constipation					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cannabis		/	-	Drug use for unknown indication
cannabis			-	indication
2 METOCLOPRAMIDE	Unknown	/	-	Obstipation
metoclopramide			-	
3 nsaid's	Unknown	/	-	Drug use for unknown indication
ibuprofen			-	indication
4 TRAMADOL	Oral	/	-	Analgesic therapy
tramadol			-	

Drugs (concomitant)

5 levetiracetam	/	-	Convulsive seizure
levetiracetam		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Constipation			Yes	on admission
Hypertension			Unknown	
Coprostasis			k.A.	
Paresis in arms			k.A.	
Polyneuropathy			k.A.	
Megacolon			k.A.	
Drug abuse			k.A.	Cannabis abuse, episodic use
Stress at work			Unknown	irregular eating habits due to stress at work
Cannabis abuse, unspecified use			Unknown	
Acute intermittent porphyria			Yes	
General nutrition disorder			Unknown	
Hyponatraemia			Unknown	
Pain			Yes	on admission
Colonic pseudo-obstruction			Unknown	
Porphyria acute			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Lutz P, Maring D, Tschampa HJ, Sauerbruch T. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. Medizinische Klinik. 2010;105:267-272	
Qualification:	Physician	observestudytype:
Literature:	Lutz P. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. Medizinische Klinik. 2010 APR 01;105:267-272.	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aminolevulinic acid urine		21.7	mg/l	
Bilirubin total		0.46	mg/dL	
BP		170/100	mm Hg	
C-reactive protein		20.9	mg/L	
Colonoscopy		megacolone	UNK	
Creatinine		2.28	mg/dL	
CT scan		unremarkable	UNK	
EEG		unremarkable	UNK	
GGT		28	U/L	
GPT		59	U/L	
Haemoglobin		12.1	g/dL	
Lead		(urine, blood) normal	normal	
MRI		posterior reversible	stroke	encephalopathy
Porphyrinuria		10 000	mcg/L	
Potassium		4.17	mmol/L	
Serum sodium		(nadir; day 4) 115	mmol/L	
Serum sodium		132	mmol/L	
Serum urea		2.1	mg/dL	
Thrombocyte count		184 G	/L	
Urine osmolarity		407	mOsm/kg	

	DE-BFARM-10137498	
WBC count	7.34 G	/L
X-ray	marked coprostasis	UNK
Lipase	64	U/L
Ultrasound examination	marked coprostasis	UNK
CSF test	unremarkable	UNK
Blood beta human chorionic gonadotropin	< 2	ng/mL
Porphobilinogen urine increased	18	mg/l
Uroporphyrin	10000	µg/l
Blood test	Hyponatraemia	a.a.
Urine analysis	Markedly elevated	excretion of porphyrins

Narratives (nur bei elektronischem Eingang)

MEDA Pharma GmbH & Co. KG, 07.09.2010 DE-MEDA-M-EU-2010090059

Sender diagnosis: not reported

Narrative

A literature report was received from YES Services (DE-VIGILIT-567949) - literature search project VIGILIT on 01-Sep-2010.

A 25-year-old female patient was admitted with constipation and marked diffuse pain for ten days. Various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day four. An MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

The authors state that extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Literature source:

Lutz P, Maring D, Tschampa HJ, Sauerbruch T. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. Med Klin (Munich). 2010; 105: 267-2.

Intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

NOVARTIS, 08.09.2010 DE-SZ09-GXKR2010DE09714

Sender diagnosis: not reported

Narrative

Initial literature case report received on 26 Aug 2010. The patient was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful. Initial findings included megacolon, abdominal distension and motor demyelinating polyneuropathy with proximal paresis of the right arm, and she had a seizure due to hyponatraemia and syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4. An MRI revealed posterior reversible encephalopathy syndrome. With levetiracetam, the generalised seizures resolved, however, the patient had hallucinations. Laboratory findings were diagnostic for porphyria. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, the patient had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

The authors commented: "Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects".

Outcome: reaction improving

sanofi-aventis, 09.09.2010 DE-SANOFI-AVENTIS-2010SA052042

Sender diagnosis: not reported

Narrative

The literature case was identified on 31-AUG-2010; abstract: Lutz P, Maring D, Tschampa HJ, Sauerbruch T. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. Medizinische Klinik, 2010; 105: 267-272.

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days, various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful (dosages and durations not stated; NSAIDs not specified). Initial findings included megacolon and motor demyelinating polyneuropathy, and she had a seizure due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4. Laboratory findings were diagnostic for porphyria (times to reaction onset not stated). Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminases levels and elevated porphyrin excretion.

Author Comment: Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Reporter's comment:RANBAXY, 09.09.2010 DE-RANBAXY-2010RR-37911

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2010RR-37911 from GERMANY refers to a 25 years old female who developed porphyria aggravated following treatment with metoclopramide, tramadol and different NSAIDs which was medically significant.

This case was reported in the literature.

Patient's medical history:

Constipation: on admission.

Diffuse pain: on admission.

Patient's current condition:

Colonic pseudo-obstruction.

Hyponatraemia.

Hypertension.

The subject received the following suspect products:

Metoclopramide.

Tramadol.

Different NSAIDs.

The patient was treated with:

Glucose.

Haem-arginate.

Morphine.

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she had a seizure due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

The outcome of the event was reported as improved at the time of this report.

The reporter considered the case serious because the ADR was medically significant.

Author Comment: Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake

of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Company Comment:

The case is rated as serious, unexpected and possible due to temporal association with the suspect products.

According to the authors, extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Krewel Meuselbach GmbH, 10.09.2010 DE-KREWEL-002477

Sender diagnosis: not reported

Narrative

A 25-year-old female patient developed acute intermittent porphyria while being treated with metoclopramide, tramadol and non-steroidal antiphlogistic drugs (not specified). The patient presented to the emergency department with constipation and diffuse pain which had been increasing for 10 days. She had consulted several doctors before but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatraemia. A megacolon and polyneuropathy were found. Shortly after admission, she developed generalised seizures while hyponatraemia increased compatible with SIADH (syndrome of inadequate ADH secretion). Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed. The patient was treated with 40% glucose infusion and haem arginate which decreased the excretion of porphyrine. The patient's pain needed initially morphine therapy which could be tapered in the further clinical course. The patient was discharged with mild anaemia, mild hyponatraemia, slightly elevated transaminases and an elevated excretion of porphyrine. The reporters state that the most possible causative trigger for this attack of acute intermittent porphyria was an irregular food intake due to extensive occupational demands. They assume that the intake of different medications such as metoclopramide, tramadol and several non-steroidal antiphlogistic drugs as well as the occasional consumption of cannabis may have aggravated the disease. They point out that this case shows how definite diagnosis of this illness is often delayed because of its rarity and the variety of its possible symptoms and signs. This delay leads to a high risk of aggravating the disease by prescribing porphyrinogenic drugs.

Generics [UK] Ltd, 10.09.2010 DE-MYLANLABS-2010S1016016

Sender diagnosis: not reported

Narrative

Lutz P, Maring D, Tschampa HJ, Sauerbruch T. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. Med-Klin 2010

25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4; an MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Temmler Pharma GmbH & Co. KG, 10.09.2010 DE-TEMMLER06-004133

Sender diagnosis: not reported

Narrative

This literature case was received by Temmler Pharma on 01 September 2010 from vigilit and adis reactions:

A 25-year-old hypertensive patient presented to the Emergency Department with constipation and diffuse pain which had been increasing for 10 days. She had consulted several doctors before, but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatraemia. A megacolon and polyneuropathy were found. Shortly after admission, she developed generalized seizures while hyponatraemia

increased compatible with SIADH (syndrome of inadequate ADH secretion). Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed. This case shows how definite diagnosis of this illness is often delayed because of its rarity and the variety of its possible symptoms and signs. This delay leads to a high risk of aggravating the disease by prescribing porphyrinogenic drugs.

Temmler ID: TRA-2010-016 / MET-2010-015<dbtext>To_Char_NARRATIVE_2</dbtext>

Reporter's comment:

Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

MEDA Pharma GmbH & Co. KG, 10.09.2010 DE-MEDA-M-EU-2010090059

Sender diagnosis: not reported

Narrative

A literature report was received from YES Services (DE-VIGILIT-567949) - literature search project VIGILIT on 01-Sep-2010.

A 25-year-old female patient was admitted with constipation and marked diffuse pain for ten days. Various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day four. An MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

The authors state that extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Literature source:

Lutz P, Maring D, Tschampa HJ, Sauerbruch T. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. Med Klin (Munich). 2010; 105: 267-2.

A follow-up report was received by YES Services - literature search project VIGILIT - on 06-Sep-2010.

The patient developed acute intermittent porphyria while being treated with tramadol, metoclopramide and non-steroidal antiphlogistic drugs (not specified). Besides constipation the patient suffered from diffuse pain which had been increasing for 10 days. She had consulted several doctors before but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatraemia. Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed. The patient's treatment included 40% glucose infusion.

The reporters point out that this case shows how definite diagnosis of this illness is often delayed because of its rarity and the variety of its possible symptoms and signs. This delay leads to a high risk of aggravating the disease by prescribing porphyrinogenic drugs.

JUTA, 13.09.2010 DE-JUTA-2033-10

Sender diagnosis: not reported

Narrative

A 25-year-old female patient developed acute intermittent porphyria while being treated with tramadol, metoclopramide and non-steroidal antiphlogistic drugs (not specified). The patient presented to the emergency department with constipation and diffuse pain which had been increasing for 10 days. She had consulted several doctors before but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatraemia. A megacolon and polyneuropathy were found. Shortly after admission, she developed generalised seizures while hyponatraemia increased compatible with SIADH (syndrome of inadequate ADH secretion). Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed. The patient was treated with 40% glucose infusion and haem arginate which decreased the excretion of porphyrine. The patient's pain needed initially morphine therapy which could be tapered in the further clinical course. The patient was discharged with mild anaemia, mild hyponatraemia, slightly elevated

transaminases and an elevated excretion of porphyrine. The reporters state that the most possible causative trigger for this attack of acute intermittent porphyria was an irregular food intake due to extensive occupational demands. They assume that the intake of different medications such as metoclopramide, tramadol and several non-steroidal antiphlogistic drugs as well as the occasional consumption of cannabis may have aggravated the disease. They point out that this case shows how definite diagnosis of this illness is often delayed because of its rarity and the variety of its possible symptoms and signs. This delay leads to a high risk of aggravating the disease by prescribing porphyrinogenic drugs.

CT-Arzneimittel GmbH, 13.09.2010 DE-CT-2010CT0906635

Sender diagnosis: not reported

Narrative

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4; an MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

[Author comment]: Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No
warning are not necessary.

Reporter's comment:

ratiopharm GmbH, 13.09.2010 DE-RATIOPH-2010R0906569

Sender diagnosis: not reported

Narrative

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4; an MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

[Author comment]: Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No
warning are not necessary.

Reporter's comment:

AbZ Pharma GmbH, 13.09.2010 DE-ABZ-2010A0906634

Sender diagnosis: not reported

Narrative

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including

tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4; an MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

[Author comment]: Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

excluded, but seems less likely. According to NAPOS, the drug database for acute porphyria, metoclopramide and tramadol belong to the group PNP (probably not porphyrinogenic). So far, the MAH's database contains 2 further case reports of porphyria associated with metoclopramide use (MAH case-ID 2002R091312, 2008R128943) and 1 further case report of porphyria associated with tramadol use (MAH case-ID 2010R0705140). However, plausible alternative explanations for aggravation of porphyria were present in all these cases. Based on current data, actions concerning the RSI wording are not necessary.

Reporter's comment:

STADA Arzneimittel AG, 13.09.2010 DE-STADA-028287

Sender diagnosis: not reported

Narrative

The following information was published in the scientific literature.

Reactions Weekly 1317 p. 32; 4 September 2010

Metoclopramide/NSAIDs/tramadol: Porphyria aggravation: case report

Lutz P, Maring D, Tschampa HJ, Sauerbruch T. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. Medizinische Klinik 105: 267-272, Apr 2010. Available from: URL: <http://dx.doi.org/10.1007/s00063-010-1047-1> - Germany

Summary:

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4; an MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

Author Comment: 'Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.'

Heumann Pharma GmbH & Co. Generika KG, 13.09.2010 DE-HEUMANN1-000189

Sender diagnosis: not reported

Narrative

This case has been identified in the literature.

A 25-year-old female patient developed acute intermittent porphyria while being treated with tramadol, metoclopramide, and non-steroidal antiphlogistic drugs (not specified). The patient presented to the emergency department with constipation and diffuse pain which had been increasing for 10 days. She had consulted several doctors before but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatraemia. A megacolon and polyneuropathy were found. Shortly after admission, she developed generalised seizures while hyponatraemia increased compatible with SIADH (syndrome of inadequate ADH secretion). Urine examination revealed a markedly elevated excretion of porphyrins.

Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed. The patient was treated with 40% glucose infusion and haem arginate which decreased the excretion of porphyrine. The patient's pain needed initially morphine therapy which could be tapered in the further clinical course. The patient was discharged with mild anaemia, mild hyponatraemia, slightly elevated transaminases and an elevated excretion of porphyrine. To_Char_NARRATIVE_2

suspected drugs are not mentioned. For all that, the causality was assessed as possible for tramadol and metoclopramide. Neither the database of Heumann, nor the literature shows comparable case reports. However, three case reports were found, in which metoclopramide alone was associated with the aggravation of acute intermittent porphyria. Therefore, this risk should be closely monitored.

Reporter's comment:

The reporters state that the most possible causative trigger for this attack of acute intermittent porphyria was an irregular food intake due to extensive occupational demands. They assume that the intake of different medications such as metoclopramide, tramadol and several non-steroidal antiphlogistic drugs as well as the occasional consumption of cannabis may have aggravated the disease.

TEVA, 14.09.2010 DE-TEVA-247032ISR

Sender diagnosis: not reported

Narrative

30-Aug-2010, Spontaneous Literature Serious report.

Case of a Female patient 25 Years of age who experienced COLONIC PSEUDO-OBSTRUCTION, HYPERTENSION, SIADH, POSTERIOR REVERSIBLE ENCEPHALOPATHY SYNDROME, PORPHYRIA DRUG INDUCED while on METOCLOPRAMIDE, TRAMADOL (Teva's product cannot be excluded).

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4; an MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

Author Comment

Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva's comment: un assessable due to insufficient information

This is a spontaneous case and therefore considered and handled as a related case.

drreddys, 14.09.2010 DE-drreddys-GER/GER/10/0015758

Sender diagnosis: not reported

Narrative

This is a literature report. A 25-year-old female patient developed acute intermittent porphyria while being treated with metoclopramide, tramadol and non-steroidal antiphlogistic drugs (not specified). The patient presented to the emergency department with constipation and diffuse pain which had been increasing for 10 days. She had consulted several doctors before but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatraemia. A megacolon and polyneuropathy were found. Shortly after admission, she developed generalised seizures while hyponatraemia increased compatible with SIADH (syndrome of inadequate ADH secretion). Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed. The patient was treated with 40% glucose infusion and haem arginate which decreased the excretion of porphyrine. The patient's pain needed initially morphine therapy which could be tapered in the further clinical course. The patient was discharged with mild anaemia, mild hyponatraemia, slightly elevated transaminases and an elevated excretion of porphyrine.

The reporters state that the most possible causative trigger for this attack of acute intermittent porphyria was an irregular food intake due to extensive occupational demands. They assume that the intake of different medications such as metoclopramide, tramadol and several non-steroidal antiphlogistic drugs as well as the occasional consumption of cannabis may have aggravated the disease. They point out that this case shows how definite diagnosis of this illness is often delayed because of its rarity and the variety of its possible symptoms and signs. This delay leads to a high risk of aggravating the disease by prescribing porphyrinogenic drugs.

Literature source:

Lutz P, Maring D, Tschampa HJ, Sauerbruch T. [A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain]. Med Klin (Munich). 2010; 105: 267-72

Reporter's comment:biomo pharma GmbH, 14.09.2010 DE-BIOMO-003975

Sender diagnosis: not reported

Narrative

A 25-year-old female patient developed acute intermittent porphyria while being treated with tramadol, metoclopramide, and non-steroidal antiphlogistic drugs (not specified). The patient presented to the emergency department with constipation and diffuse pain which had been increasing for 10 days. She had consulted several doctors before but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatraemia. A megacolon and polyneuropathy were found. Shortly after admission, she developed generalised seizures while hyponatraemia increased compatible with SIADH (syndrome of inadequate ADH secretion). Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed. The patient was treated with 40% glucose infusion and haem arginate which decreased the excretion of porphyrine. The patient's pain needed initially morphine therapy which could be tapered in the further clinical course. The patient was discharged with mild anaemia, mild hyponatraemia, slightly elevated transaminases and an elevated excretion of porphyrine. The reporters state that the most possible causative trigger for this attack of acute intermittent porphyria was an irregular food intake due to extensive occupational demands. They assume that the intake of different medications such as metoclopramide, tramadol and several non-steroidal antiphlogistic drugs as well as the occasional consumption of cannabis may have aggravated the disease. They point out that this case shows how definite diagnosis of this illness is often delayed because of its rarity and the variety of its possible symptoms and signs. This delay leads to a high risk of aggravating the disease by prescribing porphyrinogenic drugs. risk for developing porphyric attacks when treated with tramadol, compared to other analgesics (Lambrecht et al. 1999). The causal relationship between tramadol and aggravation of acute intermittent porphyria is assessed as possible. Other risk factors are irregular food intake, the co-suspect drugs, and cannabis abuse. Literature should be surveyed for similar cases. At the moment, an amendment of the current tramadol-biomo SmPC is not considered necessary.

Reporter's comment:

The reporters state that the most possible causative trigger for this attack of acute intermittent porphyria was an irregular food intake due to extensive occupational demands. They assume that the intake of different medications such as metoclopramide, tramadol and several non-steroidal antiphlogistic drugs as well as the occasional consumption of cannabis may have aggravated the disease.

Dolorgiet GmbH, 17.09.2010 DE-DOLORGIET-2010metotramadol02614

Sender diagnosis: not reported

Narrative

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4; an MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

Reporter's comment:

Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

GRUNENTHAL, 17.09.2010 DE-GRUNENTHAL-2010-15034

Sender diagnosis: not reported

Narrative

Manufacturer Control Number: 2010-15034

This spontaneous literature case from Germany was reported by a health professional and was initially received on 06-Sep-2010. It concerns a 25-year-old female patient (height and weight unknown) who experienced acute intermittent porphyria (coded twice as disease aggravation and porphyria acute) under tramadol hydrochloride treatment.

Other suspected medication included metoclopramide (Metoclopramide, unknown route of administration) for unknown indication, cannabis sativa (Cannabis, unknown route of administration) for unknown indication and unspecified NSAID's (unknown route of administration) for unknown indication.

No relevant concomitant medication was reported. The patient's concurrent conditions included pain and the past conditions included drug abuse. The patient was treated with tramadol hydrochloride (Tramadol hydrochloride) for pain.

The patient presented to the emergency department with constipation and diffuse pain which was increasing for 10 days. She had consulted several doctors before but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatraemia. A megacolon and polyneuropathy were found. Shortly after admission, she developed generalised seizures while hyponatraemia increased compatible with SIADH (syndrome of inadequate ADH secretion). Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed. The patient was treated with 40% glucose infusion and haem arginate which decreased the excretion of porphyrine. The patient's pain needed initially morphine therapy which could be tapered in the further clinical course. The patient was discharged with mild anaemia, mild hyponatraemia, slightly elevated transaminases and an elevated excretion of porphyrine.

The outcome of acute intermittent porphyria was reported as recovering/resolving.

The reporters state that the most possible causative trigger for this attack of acute intermittent porphyria was an irregular food intake due to extensive occupational demands. They assumed that the intake of different medications such as metoclopramide, tramadol and several non-steroidal antiphlogistic drugs as well as the occasional consumption of cannabis may have aggravated the disease. They pointed out that this case shows how definite diagnosis of this illness is often delayed because of its rarity and the variety of its possible symptoms and signs. This delay leads to a high risk of aggravating the disease by prescribing porphyrinogenic drugs.

Third party case numbers

E2B Report Duplicate: DE-VIGILIT-567949

Literature citation:

Lutz P, Maring D, Tschampa HJ, Sauerbruch T. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. Med Klin. 2010; 105 (4): 267-272

NAPPMUNDI, 22.09.2010 DE-NAPPMUNDI-DEU-2010-0006576

Sender diagnosis: not reported

Narrative

Case report reference number DEU-2010-0006576 was received on 15SEP2010 in form of case abstract from Reactions [original article published by Lutz P, et al. in Medizinische Klinik 105: 267-272, 2010. Eine 25-jährige Patientin mit Pseudoobstruktion des Kolons, Hyponatriämie, hypertensiver Entgleisung und diffusem Schmerzsyndrom. [A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain]] and in form of a case abstract from the original article retrieved in the internet.

Case narrative as reported in Reactions:

"Metoclopramide/NSAIDs/tramadol:

Porphyria aggravation: case report

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; various symptomatic treatments with analgesics (including tramadol, metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]). Initial findings included megacolon and motor demyelinating polyneuropathy, and she developed seizures due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4; an MRI was consistent with posterior reversible encephalopathy syndrome. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but "morphine" [morphine not mentioned in the case abstract of the original article, the reporter probably meant tramadol] could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

Author comment: Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Case narrative as reported in the abstract of the original article:

"Case Report:

A 25-year-old hypertensive patient presented to the Emergency Department with constipation and diffuse pain which had been increasing for 10 days. She had consulted several doctors before, but neither various analgesics nor metoclopramide had been beneficial. Blood analysis showed hyponatremia. A megacolon and polyneuropathy were found. Shortly after admission, she developed generalized seizures while hyponatremia increased compatible with SIADH (syndrome of inadequate ADH secretion). Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased, diagnosis of acute intermittent porphyria could be confirmed.

Conclusion:

This case shows how definite diagnosis of this illness is often delayed because of its rarity and the variety of its possible symptoms and signs. This

delay leads to a high risk of aggravating the disease by prescribing porphyrinogenic drugs."

This case was rated to be serious as the adverse event had led to hospitalisation.

Additional information in form of the original article has been requested. The case will be updated accordingly should more information become available.

sanofi-aventis, 24.09.2010 DE-SANOFI-AVENTIS-2010SA052042

Sender diagnosis: not reported

Narrative

The literature case was identified on 31-AUG-2010; abstract: Lutz P, Maring D, Tschampa HJ, Sauerbruch T. A 25-year-old patient with colonic pseudo-obstruction, hyponatremia, hypertension, and diffuse pain. *Medizinische Klinik*, 2010; 105: 267-272.

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days, various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful (dosages and durations not stated; NSAIDs not specified). Initial findings included megacolon and motor demyelinating polyneuropathy, and she had a seizure due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4. Laboratory findings were diagnostic for porphyria (times to reaction onset not stated). Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminases levels and elevated porphyrin excretion.

Author Comment: Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Additional information (complete article) received on 14-Sep-2010:

Case description: A 25-year-old patient presented in the authors' emergency department with constipation and pronounced diffuse pain, especially in her back, spreading into both legs and her neck and also occurring as respiratory dependent chest pain.

As these problems had been persisting for about 10 days with progressive intensity, the patient had been to several other physicians and had unsuccessfully received symptomatic treatment with different analgesics and metoclopramide. The stool retention had been interpreted as functional or drug induced.

The authors saw an alert and afebrile patient; physical examination revealed a pronouncedly distended abdomen and a blood pressure of 170/100 mmHg, other findings were unremarkable.

Laboratory showed an increased creatinine value, glutamic pyruvic transaminase (GPT), and C-reactive protein (CRP), and decreased serum sodium, haemoglobin, and thrombocytes. Previous laboratory (not in authors' hospital) had already revealed creatinine values in the upper range of normal, an increased GPT and decreased serum sodium (no values provided).

Pronounced coprostasis was found in sonography and X-ray. On the following day, colonoscopy and radiologic imaging revealed no obstruction, but a megacolon.

Additionally, the patient developed a proximal paresis of her right arm. Polyneuropathy disclosed the picture of motor demyelinating polyneuropathy, the severe pain could only be controlled by opiates.

Four days after admission, the patient suffered a generalised seizure in the course of progressing hyponatraemia down to 115 mmol/l, leading to the patient's transfer to the recovery ward.

Cranial computerized tomography as well as cerebrospinal fluid puncture and electroencephalography were not helpful. Magnetic resonance tomogram was consistent with a posterior reversible encephalopathy syndrome.

Based on increased urine osmolality of 407 mOsm/kg and exclusion of Addison's disease and hypothyroidism or dehydration, and based on a decreased serum uric acid, the authors assumed a syndrome of inadequate ADH secretion (SIADH).

In the further course, the patient suffered from generalised epileptic fits that resolved under treatment with levetiracetam; however, the patient hallucinated recurrently.

Results of urine tests confirmed an acute hepatic porphyria. Suspicion of acute intermittent porphyria was also backed up by massively increased excretion of uroporphyrin; it was confirmed by pronouncedly decreased porphobilinogen deaminase activity of 30%. These findings led to exclusion of hereditary coproporphyria and porphyria variegata. A delta-aminolevulinic acid dehydratase deficiency had already been excluded based on the increased porphobilinogen. Excretion of porphyrins in stool could not be detected, however, this investigation took place after start of treatment, when increased urine porphyrins had already decreased considerably. Concentrations of lead in serum and urine were normal. Mutation analysis had not been performed.

The patient's physical condition could be stabilised step-by-step.

Additionally to infusions of 40% glucose solution, a haemarginate therapy was carried out, leading to a pronounced reduction of porphyrin excretion. The severe pain could hardly be controlled, but morphine treatment could finally be tapered.

At time of discharge, laboratory values had normalised with the exception of mild anaemia, slightly decreased serum sodium, slightly increased transaminases, and increased porphyrin excretion.

Thorough inquiry of possible triggers resulted in a most likely cause of irregular food intake due to occupational stress. The intake of several drugs, especially metoclopramide, tramadol and different non-steroidal anti-inflammatories, probably had boosted the beginning flare-up.

The occasional consumption of cannabis might also have played an aggravating role.

The acute intermittent porphyria is the most frequent of hereditary porphyrias with an average prevalence of 1-2/100 000; especially high is the prevalence among psychiatric patients (0.21%).

Conclusion: In summary, the case involves a patient with the classical triad of neurologic (severe pain, signs of polyneuropathy and SIADH with seizure), gastrointestinal (megacolon with stool retention), and cardiovascular (hypertension) symptoms. Analysis of urine and serum confirmed the clinical suspicion of acute intermittent porphyria. Under intensive symptomatic therapy and treatment with glucose and haemarginate infusions, the patient improved considerably.

After internal review on 20-Sep-2010, metoclopramide was changed to suspect company drug.

RANBAXY, 22.11.2010 DE-RANBAXY-2010RR-37911

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2010RR-37911 from GERMANY refers to a 25 year old female who developed porphyria aggravated following treatment with metoclopramide, tramadol and different NSAIDs which was medically significant.

This case was reported in the literature.

Patient's medical history:

Constipation: on admission.

Diffuse pain: on admission.

Patient's current condition:

Colonic pseudo-obstruction.

Hyponatraemia.

Hypertension.

Coprostasis.

Proximal right hand paresis.

Polyneuropathy.

Megacolon.

The subject received the following suspect products:

Metoclopramide.

Tramadol.

Different NSAIDs.

She also received the following concomitant product:

Levetiracetam.

The patient was treated with:

Glucose 40% solution.

Haem-arginate.

Morphine.

A 25-year-old woman was admitted with constipation and marked diffuse pain for 10 days; she accused back pain that was irradiating to both legs and neck and were obstructing her respiration, various symptomatic treatments with analgesics (including tramadol), metoclopramide and different NSAIDs had been unsuccessful [dosages and durations not stated; NSAIDs not specified]. Initial findings included megacolon and motor demyelinating polyneuropathy, and she had a seizure due to syndrome of inadequate antidiuretic hormone secretion (SIADH) on day 4. Laboratory findings were diagnostic for porphyria [times to reaction onset not stated]. Treatment comprised glucose and haem-arginate, with marked reduction of porphyrin excretion. Severe pain persisted, but morphine could gradually be tapered. At discharge, she had only mild anaemia, a slightly decreased serum sodium level, mildly elevated transaminase levels and elevated porphyrin excretion.

Patient's generalised convulsive seizures were treated with levetiracetam.

Sonography and radiology examination revealed markedly coprostasis. Colonoscopy showed a megacolon. The patient developed a proximal right hand paresis. Neurography images revealed polyneuropathy.

The outcome of the event was reported as improved at the time of this report.

The reporter considered the case serious because the ADR was medically significant.

Author Comment: Extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent cannabis consumption could have also had aggravating effects.

Follow up report received on 08-NOV-2010 containing further information regarding:

Patient current condition.

Lab data.

Relevant test.

Concomitant and treatment products: details.

Company Comment:

The case is rated as serious, unexpected and possible due to temporal association with the suspect products.

According to the authors, extensive investigations identified irregular eating habits due to stress at work as the most likely cause for the episode. The intake of various drugs, especially metoclopramide, but also tramadol and different NSAIDs, may have exacerbated the initial episode. Intermittent

cannabis consumption could have also had aggravating effects.

AXCOUNT, 27.04.2011 DE-AXCOUNT-2011-008

Sender diagnosis: not reported

Narrative

A 25-year-old hypertensive woman presented to the emergency department with constipation and diffuse pain; her symptoms had persisted for 10 days despite therapy with metoclopramide and various analgesics (including NSAIDs and tramadol). Blood analysis showed hyponatraemia and, in addition, megacolon and polyneuropathy were found. Shortly after admission, the patient developed generalised seizures while hyponatraemia increased compatible with syndrome of inadequate antidiuretic hormone secretion. Urine examination revealed a markedly elevated excretion of porphyrins. Since porphobilinogen deaminase activity was clearly decreased and according to the neurological (seizure, polyneuropathy, pain, syndrome of inadequate antidiuretic hormone secretion), gastrointestinal (megacolon, constipation), and cardiovascular (hypertension) symptoms, the diagnosis of acute intermittent porphyria was confirmed.

literature indicated that, among other drugs, tramadol can increase porphyrin accumulation. The suggested role of tramadol as a trigger of porphyria aggravation will be closely monitored in the future.

Reporter's comment:

The authors stated that various factors including stress and an unbalanced diet as well as the use of porphyrinogenic drugs (including metoclopramide, tramadol, NSAIDs and cannabis) triggered the acute attack.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10144736

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes			Yes		

Patient

Initials:	Sex: Female	Birth: 19530612	Age: 56 Year	weight (kg): 64	BMI_int: 23.8	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vomiting					recovering/resolving
2 Upper abdominal pain					recovering/resolving
3 SGOT increased					recovering/resolving
4 Nausea					recovering/resolving
5 ALT increased					recovering/resolving
6 Hepatitis acute		20100326			recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibu-ratiopharm, 800 mg, Filmtabl ibuprofen	Oral	800 Mg milligram(s)/Day 82	2008-20100325 -	Lumbar syndrome
2 Novalgin, Trpf, Sanofi-Aventis metamizole	Oral	/Day 158	20100325-20100325 -	
3 Tramal, Trpf, Grünenthal tramadol	Oral	/Day 158	20100325-20100325 -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lumbar syndrome	2007		Yes	chronisches LWS-Syndrom
Hip discomfort	2007		Yes	Hüftschmerzen links
Cholecystectomy			No	
Gastritis			No	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase normal	20100211	11	U/l	
Alanine aminotransferase increased	20100326	85	U/l	
Alanine aminotransferase increased	20100327	214	U/l	
Alanine aminotransferase increased	20100329	96	U/l	
Aspartate aminotransferase normal	20100211	12	U/l	
Aspartate aminotransferase increased	20100326	204	U/l	
Aspartate aminotransferase increased	20100327	131	U/l	
Aspartate aminotransferase normal	20100329	19	U/l	
Alkaline phosphatase normal	20100326	92	U/l	
Bilirubin normal	20100326	0.73	mg/dl	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 22.09.2010 DE-FAKOS-170362

Sender diagnosis: not reported

Narrative

Bei der Patientin sind chronische LWS (Lendenwirbelsäulen)-und Hüftschmerzen bekannt. Seit 2008 wurde deshalb Ibuprofen 800 mg eingenommen. Am 26.03.2010 wurde eine stationäre Aufnahme erforderlich. Es bestanden Oberbauchschmerzen (seit 3 Wochen) sowie Übelkeit und Erbrechen. Bei Aufnahme wurde eine neu aufgetretene Erhöhung der Transaminasen nachgewiesen, welche am 11.02.2010 noch im Normbereich lagen (Laborwerte vom 11.02.2010: ALAT 11 U/l (Normbereich: <35 U/l), ASAT 12 U/l (Normbereich: <35 U/l), Laborwerte vom 26.03.2010: ALAT 85 U/l, ASAT 204 U/l). Am 25.03.2010 nahm die Patientin einmalig Novalgin- und Tramal-Tropfen ein. Ibuprofen wurde am 25.03.2010 letztmalig eingenommen. Es kam zu einer raschen Besserung der Transaminasen ab dem 29.03.2010 (ALAT 96 U/l, ASAT 19 U/l).

Die Leber war sonographisch unauffällig. Es erfolgten keine Hepatitis-Serologie und keine Leberbiopsie. Am 29.03.2010 wurde die Patientin aus der stationären Behandlung entlassen.

In Zusammenschau der Klinik und aller Befunde handelte es sich wahrscheinlich um eine akute Hepatitis (Patientin hatte dazu passende Symptome).

Diese wurde wahrscheinlich medikamentös verursacht.

Die UAW ist ambulant entstanden.

Als Auslöser werden folgende Arzneimittel verdächtigt:

Ibu-ratiopharm 800 mg Filmtabl., Einnahmezeitraum: 2008-25.03.2010,

Tramal Trpf., Grünenthal, Einnahmezeitraum: am 25.03.2010 und

Novalgin Trpf., Sanofi-Aventis, Einnahmezeitraum: am 25.03.2010.

Ibu-ratiopharm Filmtabl. werden hier hauptsächlich verdächtigt. Die Arzneimittelkausalität für Tramal und Novalgin wird mit "möglich" bewertet.

<dbtext>To_Char_NARRATIVE_2</dbtext>

sanofi-aventis, 05.10.2010 DE-SANOFI-AVENTIS-2010SA057347

Sender diagnosis: not reported

Narrative

Initial report: This case had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10144736) on 24-Sep-2010. The pharmacovigilance (PV) centre is conducting a hospitalisation registry (FAKOS).

The case involves a 56-year-old female patient with a past medical history of cholecystectomy and gastritis.

At time of event, she was suffering from chronic lumbar syndrome and pain in her left hip.

Since 2008, she had been treated with ibuprofen 800 mg/day for lumbar syndrome.

On 25-Mar-2010, the patient took metamizole sodium (Novalgin) drops and tramadol drops (doses and indications not reported).

On 26-Mar-2010, the patient was hospitalised due to upper abdominal pain for the previous three weeks as well as nausea and vomiting.

Laboratory on 11-Feb-2010 showed normal transaminases.

Laboratory at time of admission revealed increase in transaminases. Hepatitis serology and liver biopsy were not performed.

Metamizole, tramadol and ibuprofen were withdrawn immediately (last intake of drugs on 25-Mar-2010). Transaminases improved rapidly.

On 29-Mar-2010, the patient was discharged from hospital.

The hospital physician diagnosed acute, probably drug-induced hepatitis (based on findings and patient's clinical symptoms). Outcome was reported as "recovering/resolving".

Causality assessment by PV centre: "probable/likely" for ibuprofen, "possible" for metamizole and tramadol.

concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-10154644

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 10 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Arterial hypotension					recovered/resolved
2 Acute abdomen					recovered/resolved
3 Diarrhoea					recovered/resolved
4 Vomiting					recovered/resolved
5 Pleural effusion					recovered/resolved
6 Streptococcal toxic shock syndrome					recovered/resolved
7 Pericardial effusion					recovered/resolved
8 Ascites					recovered/resolved
9 Fever					recovered/resolved
10 Hematuria					recovered/resolved
11 Multi organ failure					recovered/resolved
12 Abdominal pain					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		/	-	
ibuprofen			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Laparoscopy			No	

Primary source

Qualification: Other Health Professional observestudytype:

Literature: Meedt-B, Gharavi-B, Imoehl-M, Becker-J-C. Multi-organ failure in a previously health 10-year old boy: Streptococcal Toxic Syndrome (STSS) a rare differential diagnosis. 2010, vol222: pg310-311.

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
CRP increased		25.6 mg/dl	NA	
DIC		640mg/dl	NA	
Elevated liver enzymes		406 IU/l, GPT 280 IU/l	NA	
Laparoscopy		#	NA	
Platelet count		#	NA	
X-ray		#	NA	

Narratives (nur bei elektronischem Eingang)

RBHC, 07.10.2010 DE-RB-014153-10

Sender diagnosis: not reported

Narrative

Report No 1 received from Literature Alert, Date 27-SEP-2010.

Case reference number DE-RB-014153-10 is a other case report sent by a other health professional which refers to a male aged 10 years.

Literature: Meedt-B, Gharavi-B, Imoehl-M, Becker-J-C. Multi-organ failure in a previously health 10-year old boy: Streptococcal Toxic Syndrome (STSS) a rare differential diagnosis. 2010, vol222: pg310-311.

Streptococcal Toxic Shock Syndrome (STSS) is a severe toxin-mediated disease with acute onset. A previously healthy 10 year old boy presented with abdominal pain, vomiting, diarrhoea and fever for 3 days. He was admitted with signs of an acute abdomen. A treatment with Ibuprofen had been started before admission (no further information provided). The child developed a multi-organ failure with persistent hematuria, persistent fever, ascites, pericardial and pleural effusions. Intensive microbial and viral analysis did not result in any relevant findings. Detection by PCR of DNA of the streptococcal super-antigens speM and speL supported the diagnosis of STSS. After an intensive-care treatment of 4 weeks according to international critical-care guidelines the child could be discharged without residuals.

The reaction was deemed serious by the reporter because it resulted in hospitalization. The patient was hospitalized.

The reporter's causality for the case is Possible.

The reporting other health professional's clinical assessment for drug IBUPROFEN[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case is Possible and unlisted.

Outcome: Recovered / resolved.

Boehringer Ingelheim GmbH, Germany, 20.10.2010 DE-Boehringer Ingelheim GmbH, Germany-2010-DE-05659GD

Sender diagnosis: not reported

Narrative

P10-11765

A previously healthy 10-year-old boy was admitted with signs of an acute abdomen. A treatment with ibuprofen had been started before admission. The child developed a multi-organ failure with septic shock, persistent hematuria, persistent fever, ascites, pericardial and pleural effusions. The diagnosis of Streptococcal Toxic Shock Syndrome (STSS) was made. After an intensive-care treatment of 4 weeks the child could be discharged without residuals.

The boy presented with abdominal pain, vomiting, diarrhoea and fever for three days. Before admission a treatment with ibuprofen had been started. The child showed signs of an acute abdomen and arterial hypotension. The laboratory findings showed elevated inflammatory markers (CRP 25,6 mg/dl, LBP 65,9 mcg/ml, IL6 43100 pg/ml) and liver enzymes (GOT 406 IU/l, GPT 280 IU/l), a pre-renal acute renal failure (creatinine 2.2 mg/dl, oliguria, normal urinalysis) and a disseminated intravascular coagulation (DIC) (fibrinogen 640 mg/dl, D-dimer 14148 ng/ml, platelet count 107/nl). The abdominal X-ray showed numerous air-fluid levels.

An acute appendicitis was suspected and an explorative laparoscopy was performed showing no pathologies. After surgery the child was transferred to the pediatric intensive-care unit. The boy received an intensive-care treatment according to international critical-care guidelines including fluid therapy, administration of vasoactive drugs due to right ventricular failure, tight blood glucose control, stress ulcer prophylaxis and anticoagulation therapy. Mechanical ventilation with daily-awakening procedure was necessary for 4 days. An initial empirical anti-infective therapy was started after obtaining cultures. Intensive antimicrobial and viral T analysis did not result in any relevant finding.

The patient developed multiple effusions (pleural, pericardial, ascites and persistent fever which resolved under anti-inflammatory therapy with ibuprofen. The laboratory inflammatory parameters normalized. Repeated blood cultures remained negative.

Persistent hematuria and ascites led to diagnostic evaluation of antibodies against streptococci showing elevated titers (anti-streptolysine O titer 8427 IU/ml, anti-DNase B 1910 IU/ml, anti-hyaluronidase 1600 IU/ml). The boy developed a nephritic syndrome (hematuria, proteinuria 938 mg/d) with arterial hypertension. In suspicion of a group A streptococcal infection the antibiotic therapy was switched to penicillin G for 14 days and an antihypertensive treatment with nifedipin was started. Under this treatment the blood pressure normalized and hematuria terminated. After an intensive-care treatment of four weeks the child could be discharged without residuals. DNA of the streptococcal super-antigens speM and speL could be detected in serum obtained on the day of admission and the diagnosis of STSS could be substantiated.

Authors' statement: 'STSS is a severe acute illness with high mortality ... Risk factors are underlying chronic lung and heart disease, cancer, skin infections like varicella, alcoholism, intravenous drug use and use of nonsteroidal anti-inflammatory drugs ... This case shows the dilemma to confirm the diagnosis of STSS in a patient with multi-organ failure, septic shock and negative cultures ... In this patient the diagnosis could only be substantiated by the detection of DNA of streptococcal superantigens ... Since our patient did not have any underlying medical conditions, we suspect that the high-dose administration of ibuprofen before admission could have supported the development of a STSS.'

Streptococcal Toxic Shock Syndrome (STSS) is a severe toxin-mediated disease with acute onset.

RBHC, 22.11.2010 DE-RB-015155-10

Sender diagnosis: not reported

Narrative

Report No 1 received from a literature paper, Klin Padiatr 2010 on 08-Nov-2010.

Case reference number RB-15155-2010 is a other case report sent by a other health professional which refers to a male aged 10 years.

Product used: Ibuprofen.

Dosage: Unknown.

A previously healthy 10 year old boy presented with abdominal pain, vomiting, diarrhoea and fever for 3 days. He was admitted with sign of an acute abdomen. A treatment with Ibuprofen had been started before admission. The child developed a multi-organ failure with persistent hematuria,

persistent fever, ascites, pericardial and pleural effusions. Intensive microbial and viral analysis did not result in any relevant finding. Detection by PCR of DNA of the streptococcal super-antigens speM and speL supported the diagnosis of STSS.

It was also reported that the child developed multiple effusions (pleural, pericardial ascites) and persistent fever which resolved under anti-inflammatory therapy with ibuprofen.

The laboratory findings showed elevated inflammatory markets (CRP 25.6mg/dl, LBP 65.9 ug/ml, IL 6 43 100pg/ml) and liver enzymes (GOT 406IU/l, GPT 280 IU/l), a pre-renal acute renal failure (creatinine 2.2mg/dl, oliguria, normal urinalysis) and a disseminated intravascular coagulation (DIC) (fibrinogen 640mg/dl, D-dimer 14148 ng/ml, platelet count 107/nl). The abdominal x-ray showed numerous air-fluid levels. An acute appendicitis was suspected and an explorative laparoscopy was performed showing no pathologies.

In suspicion of a group A streptococcal infection the antibiotic therapy was switched to penicillin G for 14 days and an antihypertensive treatment with nifedipin was started. Under this treatment blood pressure normalized and hematuria terminated.

After an intensive care treatment of 4 weeks the child could be discharged without residuals.

Medical history: Laparoscopy

The reaction was deemed serious by the reporter as it resulted in hospitalization and was classed as medically significant.

The reporter's causality for the case is Possible. The reporting other health professional's clinical assessment for drug IBUPROFEN[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case is Unlikely and Unlisted.

Outcome: Recovered / Resolved.

RBHC, 21.12.2010 DE-RB-015155-10

Sender diagnosis: not reported

Narrative

Report No 1 received from a literature paper, Klin Padiatr 2010 on 08-Nov-2010.

Case reference number RB-15155-2010 is a other case report sent by a other health professional which refers to a male aged 10 years.

Product used: Ibuprofen.

Dosage: Unknown.

A previously healthy 10 year old boy presented with abdominal pain, vomiting, diarrhoea and fever for 3 days. He was admitted with sign of an acute abdomen and arterial hypotension. A treatment with Ibuprofen had been started before admission. The child developed a multi-organ failure with persistent hematuria, persistent fever, ascites, pericardial and pleural effusions. Intensive microbial and viral analysis did not result in any relevant finding. Detection by PCR of DNA of the streptococcal super-antigens speM and speL supported the diagnosis of STSS.

The laboratory findings showed elevated inflammatory markets (CRP 25.6mg/dl, LBP 65.9 ug/ml, IL 6 43 100pg/ml) and liver enzymes (GOT 406IU/l, GPT 280 IU/l), a pre-renal acute renal failure (creatinine 2.2mg/dl, oliguria, normal urinalysis) and a disseminated intravascular coagulation (DIC) (fibrinogen 640mg/dl, D-dimer 14148 ng/ml, platelet count 107/nl). The abdominal x-ray showed numerous air-fluid levels.

An acute appendicitis was suspected and an explorative laparoscopy was performed showing no pathologies.

It was also reported that the child developed multiple effusions (pleural, pericardial ascites) and persistent fever which resolved under anti-inflammatory therapy with ibuprofen.

Persistent haematuria and ascites led to diagnostic evaluation of antibodies against streptococci showing elevated titers (anti-streptolysine O titer 8427 IU/ml, anti-DNase B 1910 IU/ml, anti hyaluronidase 1600IU/ml). The boy developed nephritic syndrome (hematuria, proteinuria 938mg/d) with arterial hypertension.

In suspicion of a group A streptococcal infection the antibiotic therapy was switched to penicillin G for 14 days and an antihypertensive treatment with nifedipin was started. Under this treatment blood pressure normalized and hematuria terminated.

After an intensive care treatment of 4 weeks the child could be discharged without residuals.

Medical history: Laparoscopy

The reaction was deemed serious by the reporter as it resulted in hospitalization and was classed as medically significant.

The reporter's causality for the case is Possible. The reporting other health professional's clinical assessment for drug IBUPROFEN[Ibuprofen] is as

follows: serious; with a relatedness of Possible.

The company's assessment for the case is Unlikely and Unlisted.

Outcome: Recovered / Resolved.

RBHC, 21.12.2010 DE-RB-014153-10

Sender diagnosis: not reported

Narrative

Report No 1 received from Literature Alert, Date 27-SEP-2010.

Case reference number DE-RB-014153-10 is a other case report sent by a other health professional which refers to a male aged 10 years.

Literature: Meedt-B, Gharavi-B, Imoehl-M, Becker-J-C. Multi-organ failure in a previously health 10-year old boy: Streptococcal Toxic Syndrome (STSS) a rare differential diagnosis. 2010, vol222: pg310-311.

Streptococcal Toxic Shock Syndrome (STSS) is a severe toxin-mediated disease with acute onset. A previously healthy 10 year old boy presented with abdominal pain, vomiting, diarrhoea and fever for 3 days. He was admitted with signs of an acute abdomen. A treatment with Ibuprofen had been started before admission (no further information provided). The child developed a multi-organ failure with persistent hematuria, persistent fever, ascites, pericardial and pleural effusions. Intensive microbial and viral analysis did not result in any relevant findings. Detection by PCR of DNA of the streptococcal super-antigens speM and speL supported the diagnosis of STSS. After an intensive-care treatment of 4 weeks according to international critical-care guidelines the child could be discharged without residuals.

The reaction was deemed serious by the reporter because it resulted in hospitalization. The patient was hospitalized.

The reporter's causality for the case is Possible.

The reporting other health professional's clinical assessment for drug IBUPROFEN[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case is Possible and unlisted.

Outcome: Recovered / Resolved.

Report No 2, Date 21-Dec-2010:

Adverse events added: Arterial hypertension, arterial hypotension and acute abdomen.

Medical history added: Laparoscopy.

Lab data added: Liver enzyme, x-ray, platelet count, DIC, laparoscopy, CRP.

A previously healthy 10 year old boy presented with abdominal pain, vomiting, diarrhoea and fever for 3 days. He was admitted with sign of an acute abdomen and arterial hypotension. A treatment with Ibuprofen had been started before admission. The child developed a multi-organ failure with persistent hematuria, persistent fever, ascites, pericardial and pleural effusions. Intensive microbial and viral analysis did not result in any relevant finding. Detection by PCR of DNA of the streptococcal super-antigens speM and speL supported the diagnosis of STSS.

The laboratory findings showed elevated inflammatory markets (CRP 25.6mg/dl, LBP 65.9 ug/ml, IL 6 43 100pg/ml) and liver enzymes (GOT 406IU/l, GPT 280 IU/l), a pre-renal acute renal failure (creatinine 2.2mg/dl, oliguria, normal urinalysis) and a disseminated intravascular coagulation (DIC) (fibrinogen 640mg/dl, D-dimer 14148 ng/ml, platelet count 107/nl). The abdominal x-ray showed numerous air-fluid levels.

An acute apendicitis was suspected and an explorative laparoscopy was performed showing no pathologies.

It was also reported that the child developed multiple effusions (pleural, pericardial ascites) and persistent fever which resolved under anti-inflammatory therapy with ibuprofen.

Persistent haematuria and ascites led to diagnostic evaluation of antibodies against streptococci showing elevated titers (anti-streptolysine O titer 8427 IU/ml, anti-DNase B 1910 IU/ml, anti hyaluronidase 1600IU/ml). The boy developed nephritic syndrome (hematuria, proteinuria 938mg/d) with arterial hypertension.

In suspicion of a group A streptococcal infection the antibiotic therapy was switched to penicillin G for 14 days and an antihypertensive treatment with nifedipin was started. Under this treatment blood pressure normalized and hematuria terminated.

After an intensive care treatment of 4 weeks the child could be discharged without residuals.

Medical history: Laparoscopy

The reaction was deemed serious by the reporter because it resulted in hospitalization.

The reporter's causality for the case is Possible.

The reporting other health professional's clinical assessment for drug IBUPROFEN[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case is Possible and unlisted.

Outcome: Recovered / Resolved.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10157275

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19390116	Age: 71 Year	weight (kg): 65	BMI_int: 25.08	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GOT increased					Unknown
2 Quick's value decreased					Unknown
3 Dysesthesia					Unknown
4 Difficulty in walking					Unknown
5 Vertigo					Unknown
6 Hepatic steatosis					Unknown
7 Hypokalemia					Unknown
8 Hepatopathy					Unknown
9 Hepatic enzymes increased					recovering/resolving
10 Hepatic failure					recovering/resolving
11 Decreased INR					Unknown
12 GPT increased					Unknown
13 Hepatitis		20100712		1 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin cardio acetylsalicylic acid	Unknown	/	-	Chronic back pain
2 avalox moxifloxacin hydrochloride	Oral	400 Mg milligram(s)/Day 82	20100712-20100722 11 Day	Ankle ulcer
3 AVALOX moxifloxacin		/	-	Septic joint
4 brufen ibuprofen	Oral	3000 Mg milligram(s)/Day	-	Chronic back pain
5 carbasalate calcium carbasalate calcium	Unknown	1660 Mg milligram(s)/Day	-	Chronic back pain
6 NOVALGIN metamizole	Unknown	/	-	Chronic back pain
7 PARACETAMOL paracetamol	Unknown	/	-	Chronic back pain
8 PRAVALOTIN pravastatin sodium	Oral	2*20 Mg milligram(s)/Wee	-	Ill-defined disorder
9 RISPERDAL risperidone	Oral	1 Mg milligram(s)/Day	-	Ill-defined disorder

Drugs (concomitant)

10 Beloc Zok mite metoprolol succinate		1 Df dosage form/Day	-	
11 Marcumar phenprocoumon		/	20100701-20100722 22 Day	
12 simvastatin simvastatin		10 Mg milligram(s)/Day	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Venous insufficiency			Unknown	venous insufficiency
Coronary heart disease			Unknown	CHD

Primary source

Qualification: Physician observestudytype:
Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Activated partial thromboplastin time		7	%	
Alkaline phosphatase		133	U/L	
Blood amylase		110	U/L	
Blood bilirubin direct		106	mg/dL	
Blood bilirubin total	20100712	1.72	mmol/L	
GGT	20101123	53	U/L	
GGT	20100819	173	U/L	
GGT		168	U/L	
SGOT	20100913	125	U/L	
SGOT	20101123	102	U/L	
SGOT	20100817	462	U/L	
SGOT	20100712	462	U/L	
SGOT	20080812	38	U/L	
SGPT	20101123	67	U/L	
SGPT	20100913	77	U/L	
SGPT	20100816	401	U/L	
SGPT	20100712	401	U/L	
SGPT	20081216	18	U/L	
Lipase		92	U/L	

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 14.10.2010 DE-BAYER-201041848GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 08-Oct-2010 from a general practitioner in Germany via sales representative, referring to a female patient of unknown age.

Patient's medical and drug history were not reported.

On an unspecified date, patient was treated with Avalox (moxifloxacin hydrochloride) for foot ulcer.

On an unspecified date, during foot ulcer treatment with oral Avalox, the patient developed MASSIVE HEPATITIS.
At the time of report, the hepatitis was abating.

The reporter did not assess seriousness of the event nor did he assess drug-event relationship.

Bayer Vital GmbH, 21.10.2010 DE-BAYER-201041848GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 08-Oct-2010 from a general practitioner in Germany via sales representative, referring to a female patient of unknown age.

Patient's medical and drug history were not reported.

On an unspecified date, patient was treated with Avalox (moxifloxacin hydrochloride) for foot ulcer.

On an unspecified date, during foot ulcer treatment with oral Avalox, the patient developed MASSIVE HEPATITIS.

At the time of report, the hepatitis was abating.

The reporter did not assess seriousness of the event nor did he assess drug-event relationship.

** Follow-up on 4-Oct-2010: Completed report form provided by physician.

Patient concerned a 71 year-old female (initials, birth date, height and weight provided)

Patient had concomitant conditions of CHD (coronary heart disease) and venous insufficiency. Patient suffered from ulcer left lateral. Lateral malleolus showed intermalleolar sepsis. Concomitant drugs were Marcumar (phenprocoumon), Beloc Zok mite (metoprolol succinate) and simvastatin were given.

Patient received:

- Avalox (moxifloxacin) from 12-Jul-2010 for 10 days (after smear test and resistance determination).

During antibiotics, the patient developed drop in INR (no data were provided). Lab control testing revealed massive increase in transaminases (GPT 401, GOT 462, BILI 1.72). Onset of the event was reported as 12-Jul-2010. No remedial treatment was given. Patient has improved at the time of the report.

Reporter did not assess seriousness of the event nor did he assess drug-event relationship.

Janssen-Cilag, Germany Local Safety Officers, 16.11.2010 CH-JNJFOC-20101101051

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority (Swiss Agency for therapeutic products (Switzerland) - 2010-03148), and concerns a 77 year old female patient from Switzerland: NW-11.10-331-S.

The patient's weight was 88 kilograms and height was not reported. The patient's medical history and concurrent conditions included: chronic back pain and motion independent pain in right lateral upper leg. The patient had applied high doses of different analgesic drugs due to motion independent pain in right lateral upper leg and chronic back pain which included ibuprofen 600 mg 2 to 6 times, acetyl salicylate upto 1100 mg, metamizole sodium several times daily and paracetamol (unspecified, unknown) initiated on an unspecified date. The patient was treated with risperidone (unspecified, oral) 1 mg once a day, initiated on an unspecified date for unknown indication. Non-company suspect drugs also included: Alcacyl* (carbasalate calcium) for motion independent pain in right lateral upper leg and chronic back pain and pravastatin sodium for unknown indication. All drugs were applied for several weeks and it was suspected that the patient applied them for several months. Concomitant medications were not reported. The patient was admitted to a hospital due to a significant increase of liver values and hypokalemia on an unspecified date. The patient showed up at general practitioner due to vertigo, aggravation of walking disorder and dysesthesia of the legs. The right hypogastric zone was pressure sensitive and there was a labor-chemical finding of decreased spontaneous quick of 32 %, a remarkable increase of transaminases (Serum glutamate oxaloacetate transaminase: GOT 11869 U/L and serum glutamate pyruvate transaminase: GPT 2424 U/L), a clear increase of gamma glutamyl transpeptidase (gamma-GT) and creatine kinase (CK), a slight increase of inflammatory parameters and a hypokalemia of 2.5 mmol/L. The abdominal sonography showed an enlarged hepatopathy without intra and extrahepatic cholestasis. The abdominal computed tomography (CT) showed a remarkable liver steatosis. These findings were interpreted in the hospital as acute/subacute liver failure with a probable medical-toxic etiology. Treatment with risperidone, paracetamol, ibuprofen, carbasalate calcium and pravastatin sodium was withdrawn and action taken with metamizole sodium, acetylsalicylic acid and metamizole was not reported. The patient was recovering from hepatic failure and hepatic enzymes increased and outcome was not reported for hepatopathy, hypokalemia, liver steatosis, vertigo, aggravation of walking disorder and dysesthesia.

This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Bayer Vital GmbH, 24.11.2010 DE-BAYER-201041848GPV

Sender diagnosis: not reported

Narrative

This spontaneous case report was received on 08-Oct-2010 from a general practitioner in Germany via sales representative, referring to a female patient of unknown age.

Patient's medical and drug history were not reported.

On an unspecified date, patient was treated with Avalox (moxifloxacin hydrochloride) for foot ulcer.

On an unspecified date, during foot ulcer treatment with oral Avalox, the patient experienced MASSIVE HEPATITIS.

At the time of report, the hepatitis was abating.

The reporter did not assess seriousness of the event nor did he assess drug-event relationship.

**** Follow-up on 4-Oct-2010:** Completed report form provided by physician.

Patient concerned a 71 year-old female (initials, birth date, height and weight provided)

Patient had concomitant conditions of CHD (coronary heart disease) and venous insufficiency. Patient suffered from ulcer left lateral. Lateral malleolus showed intermalleolar sepsis. Concomitant drugs were Marcumar (phenprocoumon), Beloc Zok mite (metoprolol succinate) and simvastatin were given.

Patient received:

- Avalox (moxifloxacin) from 12-Jul-2010 for 10 days (after smear test and resistance determination).

During antibiotics, the patient developed drop in INR (no data were provided). Lab control testing revealed massive increase in transaminases (GPT 401, GOT 462, BILI 1.72). Onset of the event was reported as 12-Jul-2010. No remedial treatment was given. Patient has improved at the time of the report.

Reporter did not assess seriousness of the event nor did he assess drug-event relationship.

**** FU 19-Nov-2010:**

Comments on the moxifloxacin questionnaire:

Drugs were administered from 12-Jul-2010 to 22-Jul-2010, once daily. Laboratory testing revealed infected crural ulcer. Investigations (ultrasound): no abnormal findings.

Hepatitis marker (A, B, C, D, E) were negative. The patient had no liver specific symptoms.

No specific concomitant drugs associated with liver events or other drugs were given. Medical history and concomitant diseases - no findings.

Quick's value dropped from 26% to 7% under Avalox.

Smear of foot ulcer revealed the presence of pseudomonas aeruginosa, staphylococcus aureus, peptostreptococcus anaerobius.

Further lab values were provided.

*****Bayer Schering Global comment (part one):**

Initial 08-Oct-2010: MASSIVE HEPATITIS (considered serious due to medical relevance by the company) is considered listed according to the CCDS of Avalox (moxifloxacin). Based on the limited provided information, a plausible causality assessment cannot be made. However, due to the fact that the event was reported to have occurred during treatment, a temporal relationship was implied and therefore, due to the lack of another explanation, a role of Avalox cannot be excluded. Further information has been requested (i.e. completion of Liver questionnaire).

****FU 14-Oct-2010:** Physician provided further details including increased ALT and AST values, and reported as onset date of the event, the day that Avalox treatment was started for an lateral malleolus ulcer with intermalleolar sepsis. An improvement was reported (no lab values provided) but it is unclear whether this was during continuing treatment or after completion of the Avalox therapy. Therefore the company assessment remains unchanged for the massive hepatitis. Additionally an INR DROP was reported to have occurred during antibiotics (retrieved from unstructured information by company). No INR lab values were provided. The patient was using Marcumar (phenprocoumon) concomitantly. Purely due to the statement that the event occurred during antibiotics, a temporal relationship is given and a role of Avalox cannot be excluded. However, antibiotics normally lead to an increase in INR, therefore a causal role of Avalox seems less likely. The most likely explanation of this patient's decreased INR might be an unreliable intake/dose change of his Marcumar or the underlying infection. Further follow-up request still pending (i.e. completion of questionnaire) and clarification of INR drop requested.

(cont.)

Bayer Vital GmbH, 19.01.2011 DE-BAYER-201041848GPV

Sender diagnosis: not reported

Narrative

serious, related, listed

This spontaneous case report was received on 08-Oct-2010 from a general practitioner in Germany via sales representative, referring to a female patient of unknown age.

Patient's medical and drug history were not reported.

On an unspecified date, patient was treated with Avalox (moxifloxacin hydrochloride) for foot ulcer.

On an unspecified date, during foot ulcer treatment with oral Avalox, the patient experienced MASSIVE HEPATITIS. At the time of report, the hepatitis was abating.

The reporter did not assess seriousness of the event nor did he assess drug-event relationship.

**** Follow-up on 4-Oct-2010:** Completed report form provided by physician.

Patient concerned a 71 year-old female (initials, birth date, height and weight provided)

Patient had concomitant conditions of CHD (coronary heart disease) and venous insufficiency. Patient suffered from ulcer left lateral. Lateral malleolus showed intermalleolar sepsis. Concomitant drugs were Marcumar (phenprocoumon), Beloc Zok mite (metoprolol succinate) and simvastatin were given.

Patient received:

- Avalox (moxifloxacin) from 12-Jul-2010 for 10 days (after smear test and resistance determination).

During antibiosis, the patient developed drop in INR (no data were provided). Lab control testing revealed massive increase in transaminases (GPT 401, GOT 462, BILI 1.72). Onset of the event was reported as 12-Jul-2010. No remedial treatment was given. Patient has improved at the time of the report.

Reporter did not assess seriousness of the event nor did he assess drug-event relationship.

**** FU 19-Nov-2010:**

Comments on the moxifloxacin questionnaire:

Drugs were administered from 12-Jul-2010 to 22-Jul-2010, once daily. Laboratory testing revealed infected crural ulcer. Investigations (ultrasound): no abnormal findings.

Hepatitis marker (A, B, C, D, E) were negative. The patient had no liver specific symptoms.

No specific concomitant drugs associated with liver events or other drugs were given. Medical history and concomitant diseases - no findings.

Quick's value dropped from 26% to 7% under Avalox.

Smear of foot ulcer revealed the presence of pseudomonas aeruginosa, staphylococcus aureus, peptostreptococcus anaerobius.

Further lab values were provided.

[Addendum <28-NOV-2010>: This case was converted from the Clintrace database into Argus. The medical content of the case was not changed.]

****Follow-up information was provided by the attending physician on 12-Jan-2011:**

Regarding the course of the adverse reaction, the physician reported about declining but still elevated transaminases. The female patient had not developed a sepsis, white blood count revealed 3.8, CRP normal findings. No statement was provided regarding the erythrocyte sedimentation rate. Marcumar / phenprocoumon treatment between 1 and 22-Jul-2010 followed the regimen 1/4, 1/4, 1/2, 1/4, 1/2, 1/2, 1/4.

The reporter considered the elevated liver values and the declining Quick's value as associated to the intake of Avalox/ moxifloxacin.

For further case information refer to section LAB DATA

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10190869

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19860217	Age: 24 Year	weight (kg): 100	BMI_int: 24.51	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic lesion	20101123			2 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen 800 von ct ibuprofen	Oral	3200 Mg milligram(s)/Day 245	20101122-20101122 -	Intercostal neuralgia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Intercostal neuralgia	20101122		Unknown	war Verdachtsdiagnose
Pneumonia	20101122		Yes	klinisch bis 27.11.10
Bronchitis	20101122		Yes	klinisch bis 27.11.10

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Haemoglobin	20101123	9	mmol/l	
Haematocrit	20101123	0.43		
Mean corpuscular haemoglobin	20101123	84.2	fl	
Total white blood cell count	20101123	9.58	Gpt/l	
Thrombocyte count	20101123	360	Gpt/l	
Quick's test	20101123	107	Prozent	
INR	20101123	0.95		
Activated partial thromboplastin time	20101123	35.7	sec	
Fibrinogen	20101123	7	g/l	
Sodium	20101123	138	mmol/l	
Potassium	20101123	4.1	mmol/l	
Calcium	20101123	2.41	mmol/l	
Creatinine	20101123	71.4	µmol/l	
Urea	20101123	4.9	mmol/l	
Aspartate aminotransferase	20101123	1.0	µmol/s.l	
Aspartate aminotransferase	20101124	0.74	µmol/s.l	
Aspartate aminotransferase	20101126	1.11	µmol/s.l	
Aspartate aminotransferase	20101129	1.99	µmol/s.l	
Aspartate aminotransferase	20101201	2.89	µmol/s.l	
Alanine aminotransferase	20101123	1.81	µmol/s.l	

Alanine aminotransferase	20101124	1.63	µmol/s.l
Alanine aminotransferase	20101126	2.23	µmol/s.l
Alanine aminotransferase	20101129	4.22	µmol/s.l
Alanine aminotransferase	20101201	6.41	µmol/s.l
Alkaline phosphatase	20101123	2.68	µmol/l
Alkaline phosphatase	20101124	2.67	µmol/l
Alkaline phosphatase	20101129	4.28	µmol/l
Alkaline phosphatase	20101201	5.35	µmol/l
Gamma GT	20101123	1.81	µmol/l.s
Gamma GT	20101124	1.8	µmol/l.s
Gamma GT	20101201	5.12	µmol/l.s
C-reactive protein	20101123	159.7	mg/l
C-reactive protein	20101124	214	mg/l
C-reactive protein	20101126	153	mg/l
C-reactive protein	20101129	28	mg/l
C-reactive protein	20101201	9.7	mg/l

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 08.12.2010 DE-PVZIBE-07-10422

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
Leberschädigung

KURZZUSAMMENFASSUNG:

Leberschädigung bei Ibuprofen-Überdosierung.

BESCHREIBUNG:

Am 23.11.10 Einweisung wegen Allgemeinzustandsverschlechterung mit Schwächegefühl und Kräfteverlust. Am 22.11.10 verspürte der Patient im rechten Thoraxbereich Schmerzen und bekam wegen V.a. Intercostalneuralgie Ibuprofen 800 verordnet, was der Patient gemäß Packungsbeilage einnehmen sollte. Da nach der Einnahme von drei Tabletten (a 800 mg) kein Wirkungseintritt vorhanden war, nahm der Patient am 22.11.10 eine vierte Tablette Ibuprofen 800 mg ein. Am 23.11.10 kam es neben dem Thoraxschmerz zu angegebenen Schwächegefühl und Kräfteverlust und zur Vorstellung in der Rettungsstelle.

THERAPIE:

- Ibuprofen wurde bei Aufnahme (23.11.10) abgesetzt, da am Vortag lt. Anamnese Überdosierung stattfand.

VERLAUF:

Röntgen-Thorax: Diskrete pneumonische Infiltration im rechten Unterfeld, sonst unauffälliger Befund.

Der Patient erhielt stationär eine Antibiose aus Ceftriaxon bzw. dann Wechsel auf Cefixim wegen der pneumonischen Infiltration. Auffällig war jedoch nach anfänglicher Regredienz der leicht erhöhten Leberwerte am 24.11.10 eine ab 26.11.10 beginnende Progredienz der Leberwerte bei deutlich sinkendem CrP und Besserung des Befindens ab 28.11.10. Bei der poststationären Kontrolluntersuchung am 01.12.10 waren die Leberwerte erneut angestiegen. Eine Umfelddiagnostik (Sono-Abdomen, CT, Hepatitisserologie etc.) fand nicht statt. Der Pat. will dies beim Hausarzt durchführen lassen.

ERLÄUTERUNGEN ZUR UAW:

Lt. Entlassungsbrief besteht #V.a. medikamentöse Transaminasenerhöhung (DD infektiobedingte Transaminasenerhöhung#. Alternativursache der Krankenhauseinweisung ist prinzipiell die Bronchitis-/Pleuropneumonie, die mit starkem Schwächegefühl und Kräfteverlust symptomatisch war. Außerdem bestanden Schmerzen am rechten Thorax. Die Bronchitis-/Pleuropneumonie wurde im Röntgen-Bild bestätigt. Der Patient nahm am 22.11.10 4x800 mg Ibuprofen ein, wegen Schmerzen im rechten Thorax-Bereich (zunächst V.a. Interkostalneuralgie). Am 23.11.10 fühlte sich der Patient extrem schwach und kraftlos und stellte sich in der Rettungsstelle vor. Diese Symptomatik ab 23.11.10 könnte insgesamt auch Ausdruck des überdosierten Ibuprofens gewesen sein (auch i.Sinne eines akuten hepatozellulären Schädigungsbegins). Aufnahmebedingt war der schlechte Allgemeinzustand des Patienten (Schwächegefühl, Kräfteverlust), der zum einen durch die Bronchitis/Pneumonie zu erklären ist (siehe Labor + Röntgenbefund, CrP deutlich erhöht), andererseits ist aber, wie von den Klinikern lt. Arztbrief beschrieben, die Wirkung des überdosierten Ibuprofens mit beginnender Leberschädigung in den erwähnten Symptomkomplex (Schwächegefühl, Kräfteverlust) mit einzubeziehen und zu diskutieren. Die Leberschädigung mit der möglichen Konsequenz der zur Aufnahme führenden Allgemeinzustandsverschlechterung könnte DD aber auch infektiobedingt i.R. der Bronchitis/Pneumonie zu sehen sein. Komplex wird der Fall durch den Fakt, dass die Leberwerte am 24.11.10 zunächst leicht regredient waren und dann unter Antibiose (siehe Verlauf) progredient wurden. Möglich wäre hier eine verzögerte Leberschädigung infolge der Ibuprofenintoxikation bzw. eine Schädigungsverstärkung unter Antibiose/Infektion.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: ibuprofen 800 von ct

BETREFFENDE UAW: Leberschädigung

ERLÄUTERUNG: C1: Challenge kompatibel, Dechallenge inconclusive, da Follow-up zu kurz nach Absetzen. S2: Symptom plausibel, Alternativen vorhanden. Die Einstufung als UAW unter Ibuprofen beruht auf dem Fakt, dass es durch den Patienten zu einer Überdosierung des Medikaments kam, weshalb der Fokus der Verdächtigung im Wesentlichen auf Ibuprofen lag. Der noch poststationär nach dem Absetzen des Präparates auffällige Leberwerteanstieg wurde als verzögerter Anstieg nach Ibuprofeneinnahme gewertet, evtl. auch im Zusammenhang mit der Antibiose (statistischer Zusammenhang nicht nachweisbar).

HELIOS Klinikum Wuppertal, 17.01.2011 DE-PVZIBE-07-10422

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

Leberschädigung

KURZZUSAMMENFASSUNG:

Leberschädigung bei Ibuprofen-Überdosierung.

BESCHREIBUNG:

Am 23.11.10 Einweisung wegen Allgemeinzustandsverschlechterung mit Schwächegefühl und Kräfteverlust. Am 22.11.10 verspürte der Patient im rechten Thoraxbereich Schmerzen und bekam wegen V.a. Interkostalneuralgie Ibuprofen 800 verordnet, was der Patient gemäß Packungsbeilage einnehmen sollte. Da nach der Einnahme von drei Tabletten (à 800 mg) kein Wirkungseintritt vorhanden war, nahm der Patient am 22.11.10 eine vierte Tablette Ibuprofen 800 mg ein. Am 23.11.10 kam es neben dem Thoraxschmerz zum angegebenen Schwächegefühl und Kräfteverlust und zur Vorstellung in der Rettungsstelle.

THERAPIE:

- Ibuprofen wurde bei Aufnahme (23.11.10) abgesetzt, da am Vortag lt. Anamnese Überdosierung stattfand.

VERLAUF:

Röntgen-Thorax: Diskrete pneumonische Infiltration im rechten Unterfeld, sonst unauffälliger Befund.

Der Patient erhielt stationär eine Antibiose aus Ceftriaxon bzw. dann Wechsel auf Cefixim wegen der pneumonischen Infiltration. Auffällig war jedoch nach anfänglicher Regredienz der leicht erhöhten Leberwerte am 24.11.10 eine ab 26.11.10 beginnende Progredienz der Leberwerte bei deutlich sinkendem CrP und Besserung des Befindens ab 28.11.10. Bei der poststationären Kontrolluntersuchung am 01.12.10 waren die Leberwerte erneut angestiegen. Eine Umfelddiagnostik (Sono-Abdomen, CT, Hepatitisserologie etc.) fand nicht statt. Der Pat. will dies beim Hausarzt durchführen lassen.

ERLÄUTERUNGEN ZUR UAW:

Lt. Entlassungsbrief besteht V.a. medikamentöse Transaminasenerhöhung (DD infektiogene Transaminasenerhöhung). Alternativursache der Krankenhauseinweisung ist prinzipiell die Bronchitis/Pleuropneumonie, die mit starkem Schwächegefühl und Kräfteverlust symptomatisch war. Außerdem bestanden Schmerzen am rechten Thorax. Die Bronchitis/Pleuropneumonie wurde im Röntgen-Bild bestätigt. Der Patient nahm am 22.11.10 4x800 mg Ibuprofen ein, wegen Schmerzen im rechten Thorax-Bereich (zunächst V.a. Interkostalneuralgie). Am 23.11.10 fühlte sich der Patient extrem schwach und kraftlos und stellte sich in der Rettungsstelle vor. Diese Symptomatik ab 23.11.10 könnte insgesamt auch Ausdruck des überdosierten Ibuprofens gewesen sein (auch i.Sinne eines akuten hepatozellulären Schädigungsbeginnes). Aufnahmebedingend war der schlechte Allgemeinzustand des Patienten (Schwächegefühl, Kräfteverlust), der zum einen (und wohl auch mit größerer Wahrscheinlichkeit) durch die Bronchitis/Pneumonie zu erklären ist (siehe Labor + Röntgenbefund, CrP deutlich erhöht), andererseits ist aber, wie von den Klinikern lt. Arztbrief beschrieben, die Wirkung des überdosierten Ibuprofens mit beginnender Leberschädigung in den erwähnten Symptomkomplex (Schwächegefühl, Kräfteverlust) zumindest minimal miteinzubeziehen und zu diskutieren. Die Leberschädigung mit der möglichen Konsequenz der zur Aufnahme führenden Allgemeinzustandsverschlechterung könnte DD aber auch infektiogen i.R. der Bronchitis/Pneumonie zu sehen sein, was plausibler erscheint. Komplex wird der Fall durch den Fakt, dass die Leberwerte am 24.11.10 zunächst leicht regredient waren und dann unter Antibiose (siehe Verlauf) progredient wurden. Möglich wäre hier auch eine verzögerte Leberschädigung infolge der Ibuprofenintoxikation und/bzw. eine Schädigungsverstärkung unter Antibiose/Infektion.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: ibuprofen 800 von ct

BETREFFENDE UAW: Leberschädigung

ERLÄUTERUNG: C1: Challenge kompatibel, Dechallenge inconclusive, da Follow-up zu kurz nach Absetzen. S2: Symptom plausibel, Alternativen vorhanden. Die Einstufung als UAW unter Ibuprofen beruht auf dem Fakt, dass es durch den Patienten zu einer Überdosierung des Medikaments kam, weshalb der Fokus der Verdächtigung im Wesentlichen auf Ibuprofen lag. Der noch poststationär nach dem Absetzen des Präparates auffällige Leberwerteanstieg wurde als verzögerter Anstieg nach Ibuprofeneinnahme gewertet, evtl. auch im Zusammenhang mit der Antibiose (statistischer Zusammenhang nicht nachweisbar).

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11008417

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19691114	Age: 40 Year	weight (kg): 78	BMI_int: 24.34	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug intolerance	201011				Unknown
2 Subacute hepatic failure	20101226				recovering/resolving
3 Drug-induced hepatitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	-	Migraine prophylaxis
ibuprofen			-	
2 TOPAMAX	Oral	200 Mg milligram(s)/Day	201009-	Migraine prophylaxis
topiramate			-	

Drugs (concomitant)

3 levothyroxine	Unknown	/	-	Ill-defined disorder
levothyroxine sodium			-	
4 selen	Unknown	/	-	Ill-defined disorder
selenium			-	
5 SUMATRIPTAN	Unknown	/	-	Migraine prophylaxis
sumatriptan			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Migraine			k.A.	
Acquired coagulation factor deficiency			k.A.	acquired deficit of blood coagulation factors
Caesarean section			k.A.	three times
Hashimoto's thyroiditis			k.A.	underlying disease, manifested after a pregnancy
Hyperthyroidism			k.A.	factitia
Upper respiratory tract infection			k.A.	
Surgery			k.A.	sterilization surgery, tubal ligation

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
SGPT		1516		
Bilirubin		23		
Quick's test		26		
SGOT		1780		
Creatinine	20110202	1.2	mg/dL (mill	
Gamma GT	20101226	283	IU/L (intern	
Gamma GT	20110105	254	IU/L (intern	
Gamma GT	20110131	113	IU/L (intern	

Gamma GT	20110202	129	IU/L (intern
Gamma GT	20101231	275	IU/L (intern
GFR	20110103	<416	
GFR	20101226	147.9	
GFR	20101231	<416	
HBsAg	20101226	negative	
HDL	20101226	16	mg/dL (mill
Hematocrit	20110131	0.333	
Hematocrit	20101226	37.6	% (percent
Hematocrit	20110105	35.0	% (percent
Hematocrit	20101231	33.4	% (percent
Hemoglobin	20101231	12.3	g/dL (gram
Hemoglobin	20110131	11.3	g/dL (gram
Hemoglobin	20101226	13.3	g/dL (gram
Lactate	20101226	36	mg/dL (mill
LDH	20101226	369	IU/L (intern
LDL	20101226	65	mg/dL (mill
MCH	20101226	23.7	pg (picogra
MCH	20110105	24.4	pg (picogra
MCH	20101231	24.1	pg (picogra
MCHC	20101231	36.8	g/dL (gram
MCHC	20101226	35.4	g/dL (gram
MCV	20110202	78	fL (femtolit
MCV	20110131	77.8	fL (femtolit
MCV	20101231	65.4	fL (femtolit
MCV	20101226	67.0	fL (femtolit
MCV	20110105	65.7	fL (femtolit
Partial prothrombin time	20101231	58.3	sec (secon
pH urine	20110201	7 stick test	
Platelet count	20101226	157	giga/L (10*
Platelet count	20101231	178	giga/L (10*
Potassium	20110105	2.9	mmol/L (mi
Potassium	20101226	4.0	mmol/L (mi
RBC urine	20110201	3-10	
Red blood cell count	20101231	5.11	tera/L (10*
Red blood cell count	20110105	5.33	tera/L (10*
Red blood cell count	20101226	5.61	tera/L (10*
Red blood cell count	20110131	4.28	tera/L (10*
SGOT	20110131	319	IU/L (intern
SGOT	20101231	1353	IU/L (intern
SGOT	20110105	1009	IU/L (intern
SGOT	20110202	316	IU/L (intern
SGOT	20101226	1780	IU/L (intern
SGPT	20110202	148	IU/L (intern

SGPT	20101231	1287	IU/L (intern
SGPT	20110105	1125	IU/L (intern
SGPT	20110131	153	IU/L (intern
SGPT	20101226	1516	IU/L (intern
Sodium	20101226	135	mmol/L (mi
TSH	20101226	0.01 mU/L	
Urea	20110103	13	mg/dL (mill
Leukocyte count NOS	20101231	9.19	giga/L (10*
Leukocyte count NOS	20110105	11.70	giga/L (10*
Leukocyte count NOS	20101226	6.5	giga/L (10*
Leukocyte count NOS	20110131	7.0	giga/L (10*
Ammonia	20110202	54	mcmol/L (n
Protein total	20110105	5.9	mg/dL (mill
Protein total	20101226	7.0	mg/dL (mill
Lipase	20101226	62	IU/L (intern
White blood cells urine	20110201	ca 500 stick test	mcL (micro
White blood cells urine	20110201	20-50	
Red blood cells urine	20110201	>250 stick test	mcL (micro
Quick's test	20110131	46	% (percent
Quick's test	20101231	29	% (percent
Quick's test	20101226	32	% (percent
Quick's test	20110105	26	% (percent
Anti-HBc antibody	20101226	negative	
Epstein-Barr virus antibody	20101226	EIA 700 U/mL	
Epstein-Barr virus antibody	20101226	agglutination (Paul-B) negative	
Bacteria urine	20110201	sediment mass	
International normalized ratio	20101226	2.2	
International normalized ratio	20110131	1.6	
International normalized ratio	20101231	2.5	
International normalized ratio	20110105	2.7	
Influenza A virus IgG	20101226	21 U/mL	
Influenza A virus IgA	20101226	12.3 (index)	
Influenza B virus IgG	20101226	21.4 (index)	
Influenza B virus IgA	20101226	4.3 (index)	
Epstein-Barr virus IgM antibody	20101226	EIA<10 (NR: Index <0.7)	
Uric acid	20101226	2.2	mg/dL (mill
Activated partial thromboplastin time	20110131	34	sec (secon
Albumin	20110202	2.8	g/dL (gram
Alkaline phosphatase	20101231	147	IU/L (intern
Alkaline phosphatase	20110105	190	IU/L (intern
Alkaline phosphatase	20110131	203	IU/L (intern
Alkaline phosphatase	20110201	205	IU/L (intern
Alkaline phosphatase	20101226	161	IU/L (intern
Amylase	20101226	46	IU/L (intern

Bilirubin	20101231	18.86	mg/dL (mill
Bilirubin	20110105	32.83	mg/dL (mill
Bilirubin	20110103	23.31	mg/dL (mill
Bilirubin	20101226	10.93	mg/dL (mill
Bilirubin direct	20110103	18.25	mg/dL (mill
Bilirubin direct	20101231	14.15	mg/dL (mill
Bilirubin total	20110131	16.4	mg/dL (mill
Bilirubin total	20110202	15.4	mg/dL (mill
Blood bilirubin indirect	20101231	4.71	mg/dL (mill
Blood bilirubin indirect	20110105	5.06	mg/dL (mill
Blood triglycerides	20101226	56	mg/dL (mill
C-reactive protein	20110105	9.4	mg/L (millic
C-reactive protein	20101231	6.9	mg/L (millic
C-reactive protein	20101226	6.5	mg/L (millic
C-reactive protein	20020202	1.4 (high sen.)	mg/dL (mill
Calcium	20101226	2.45	mmol/L (mi
Cholesterol	20101226	92	mg/dL (mill
Cholinesterase	20110131	1.94	IU/L (intern
Cholinesterase	20101230	2.8 kU/l	
Cholinesterase	20110202	2.13	IU/L (intern
CK-MB	20101226	27	IU/L (intern
CK-MB	20101226	32.1 quotient	% (percent
Coagulation factor V level	20110202	60	% (percent
Creatine kinase	20101226	84	IU/L (intern
Creatinine	20110103	<0.2	mg/dL (mill
Creatinine	20101226	0.49	mg/dL (mill
Creatinine	20110131	1.1	mg/dL (mill

Narratives (nur bei elektronischem Eingang)

Janssen-Cilag, Germany Local Safety Officers, 13.01.2011 DE-JNJFOC-20110100685

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician and concerns a 41-year-old female patient from Germany. The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: migraine. The patient was treated with topiramate (unspecified, oral) initiated in SEP-2010 for migraine prophylaxis. Concomitant medications included selenium and levothyroxine for unspecified indication and sumatriptan for migraine prophylaxis. On an unspecified date, the patient experienced acute liver failure. Laboratory data included: bilirubin- 23 (units unspecified), quick's test- 26 (units unspecified), serum glutamic oxaloacetic transaminase (SGOT)- 1780 (units unspecified) and serum glutamic pyruvic transaminase (SGPT)- 1516 (units unspecified). The patient was treated with Konakion* (phytomenadion) and steroids. Treatment with topiramate was withdrawn. The patient's outcome was not reported for acute liver failure. This report was serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>
no measures according to AS 29 (1) AMG have to be taken.

Reporter's comment:

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician and concerns a 41-year-old female patient from Germany. The patient's height and weight were not reported. The patient's medical history and concurrent conditions included: migraine. The patient was treated with topiramate (unspecified, oral) initiated in SEP-2010 for migraine prophylaxis. Concomitant medications included selenium and levothyroxine for unspecified indication and sumatriptan for migraine prophylaxis. On an unspecified date, the patient experienced acute liver failure. Laboratory data included: bilirubin- 23 (units unspecified), quick's test- 26 (units unspecified), serum glutamic oxaloacetic transaminase (SGOT)- 1780 (units unspecified) and serum glutamic pyruvic transaminase (SGPT)- 1516 (units unspecified). The patient was treated with Konakion* (phytomenadion) and steroids. Treatment with topiramate was withdrawn. The patient's outcome was not reported for acute liver failure. This report was serious (medically significant).

Additional information was received from a physician on 18-FEB-2011.

Patient's demographics were updated (patient's date of birth). The patient's height was 179 centimeters and weight was 78 kilograms. The patient's medical history and concurrent conditions included acquired deficit of blood coagulation factors, caesarean section (three times), Hashimoto's thyroiditis (underlying disease, manifested after a pregnancy), hyperthyroidism factitia, migraine (underlying disease), sterilization surgery (tubal ligation) and upper respiratory tract infection. The patient had previously experienced drug allergy when taking penicillin. On 02-FEB-2002, c-reactive protein (NR: 0 - 0.5 mg/dl) 1.4 (high sen.) mg/dL. The patient was treated with topiramate (unspecified, oral) 200 mg a day, initiated in SEP-2010 and 400 mg a day, initiated on an unspecified date. Non-company suspect drugs included: NSAR (non steroidal antiinflammatory drugs) for migraine prophylaxis. In NOV-2010, the patient experienced drug intolerance. In hospital discharge letter it was described that the patient discontinued topiramate in the beginning of NOV-2010 due to drug intolerance. On 24-DEC-2010 the patient had fever. On 26-DEC-2010, the patient experienced subacute liver failure. The patient was hospitalized on 26-DEC-2010 due to epigastric complaints and progressive icterus. The patient had bright yellow mushy stool, dark coloured urine, she vomited several times and shivering was questionable. Laboratory examination showed increased activities of amino transferases. In serology there was no sign for viral hepatitis. The influenza, parainfluenza-serology showed pathological findings. Anamnestic there was a former infection of upper respiratory tract. There was no confirmation of an autoimmune hepatitis. Within known Hashimoto's thyroiditis the TSH (Thyroid stimulating hormone) value was decreased (hyperthyroidism factitia) due to this the L-thyroxine medication was reduced. In course the activities of GOT (Aspartate aminotransferase) and GPT (Alanine aminotransferase) decreased, but the liver synthesis disorder (Quick 26%; INR (International normalized ratio) 2.7) continued. The start of steroid therapy was suggested. In total there was suspected drug-induced hepatitis with topiramate was soonest probable. There was no sign for hepatic encephalopathy and the patient felt clinical better. On 26-DEC-2010, Alkaline phosphatase (NR: 35 - 105 IU/L) 161 IU/L, Amylase (NR: 0 - 110 IU/L) 46 IU/L, anti-hbc antibody showed negative, sediment mass bilirubin (NR: 0 - 1.2 mg/dL) 10.93 mg/dL, blood triglycerides (NR: 0 - 150 mg/dL) 56 mg/dL, calcium (NR: 2.15 - 2.58 mmol/L) 2.45 mmol/L, cholesterol (NR: 112 - 224 mg/dL) 92 mg/dL, cholinesterase (NR: 4.3 - 11.3) 30-DEC-2010 2.8 kU/L, ck-mb (Blood-creatinine phosphokinase) (NR: 0 - 6 %) 32.1, quotient % ck-mb (NR: 0 - 25 IU/L) 27 IU/L, c-reactive protein (NR: 0 - 5.0 mg/L) 6.5 mg/L, Creatine kinase (NR: 0 - 170 IU/L) 84 IU/L, c<dbtext> To_Char_NARRATIVE_2</dbtext>GPT 1287 IU/L. On 03-JAN-2011, bilirubin 23.31 mg/dL, bilirubin direct (NR: 0 - 0.25 mg/dL) 18.25 mg/dL, grf <416 and urea (NR: 15 - 40 mg/dl) 13 mg/dL. On 05-JAN-2011 the patient was transferred to another hospital. The diagnose was sub acute liver failure, with differential diagnose of drug induced genesis was possible (with differential diagnose topiramate; with differential diagnose NSAR (non steroidal anti-rheumatic)). After discussion with colleagues of transplantation team a acute high urgency listing within stable laboratory parameter was not necessary. Within symptomatic therapy and discontinuation of potential hepatotoxic noxa the serum liver values were in course stable and at last distinct decreasing. The factor V was at discharge 60%. A hepatic encephalopathy did not occur within Bifiteral* (lactulose) therapy. Within distinct edema at legs with minor ascites a diuretic therapy was started. On 05-JAN-2011, Alkaline phosphatase 190 IU/L, bilirubin 32.83, bilirubin indirect 5.06 mg/dL, c-reactive protein 9.4 mg/L, gamma gt 254 IU/L, hematocrit 35.0 %, international normalized ratio 2.7, leucocytes 11.70 giga/L, MCH 24.4 pg, MCV 65.7 fl, quick's test 26 %, potassium (NR: 3.5 - 5.1 mmol/L) 2.9 mmol/L, SGOT 1009 IU/L, SGPT 1125 IU/L, and total protein 5.9 mg/dL. Abdominal sonography; colour duplex sonography of arteria and veins in abdomen/retroperitoneum on 05-JAN-2011 showed difficult condition due to overlapping air; liver veins open; portal vein open with hepatopetal fluidity of 23.1 cm / sec; A. hepatica open with RI 0.8; liver with homogeneous interior pattern as far as assessable; a little bit increased echo in comparison to kidney. Magnetic resonance tomography on 07-JAN-2011 showed diffuse edema in liver and perfusion inhomogeneity; consistent with acute/subacute liver failure: missing storage of contrast medium in liver total and missing excretion of contrast medium, in the sense of severe hepatocytes function disorder; no confirmation of suspected tumor liver lesion. Abdominal sonography; colour duplex sonography of arteria and veins in abdomen/retroperitoneum on 19-JAN-2011 showed homogeneous interior reflex pattern with increased echo with Steatosis hepatis; singulary liver cysts; starting ascites; otherwise liver without pathological findings. On 10-JAN-2011: Anti-HEV (EIA) negative and Anti-HEV IgM (EIA) negative. Serum examination on 10-JAN-2011 result: Anti-HEV (EIA) negative; Anti-HEV IgM (EIA) negative; quantitative HCV-RNA not confirmable; assessment: the missing of Anti-HEV excluded currently Hepatitis-E-infection. Abdominal sonography; colour duplex sonography of arteria and veins in abdomen/retroperitoneum on 27-JAN-2011 showed distinct limited sonography condition due to overlapping bowel gas; liver only transcostal visible; no confirmation of steatosis hepatis; liver perfusion without pathological findings; suspected two liver cysts (7 and 6 mm); no confirmation of significant cholestasis; spleen borderline big; ubiquiteous multiple ascites; suspected enlargement of uterus. On 31-JAN-2011, activated partial thromboplastin time (NR: 23 - 35 sec) 34 sec, Alkaline phosphatase 203 IU/L, bilirubin total 16.4 mg/dL, cholinesterase 1.94 IU/L, creatinine 1.1 mg/dL, erythrocytes 4.28 tera/L, gamma gt 113 IU/L, hematocrit 0.333 hemoglobin 11.3 g/dL, international normalized ratio 1.6, leucocytes 7.0 giga/L, mcv 77.8 fl, quick's test 46 %, sgot 319 IU/L, and SGPT 153 IU/L. Other diagnostics included 31-JAN-2011: aPTT-ratio (NR: 1.7-2.7) 1.2; thyroperoxidase-antibody (NR: 0-34 U/ml) 469.00. On 01-FEB-2011 an acute urinary tract infection was confirmed and the patient received oral antibiotics with ciprofloxacin. On 01-FEB-2011, Alkaline phosphatase 205 IU/L, bacteria, urine showed sediment mass, ph urine (NR: 5 - 6) 7 stick test, urine sediment 3-10 red blood cells urine >250 stick test mcl, wbc, urine sediment 20-50, white blood cells urine, ca 500 stick test mcl. On 02-FEB-2011, albumin (NR: 3.5 - 5.0 g/dL) 2.8 g/dL, ammonia (NR: 11 - 47 mcmol/L) 54 mcmol/L, bilirubin total 15.4 mg/dL, cholinesterase 2.13 IU/L, coagulation factor v level (NR: 70 - 130 %) 60 %, creatinine 1.2 mg/dL, gamma gt 129 IU/L, MCV 78 fl, RBC 316 IU/L and SGPT 148 IU/L. The patient was discharged on her own wish on 02-FEB-2011. Treatment with NSAID was withdrawn. The patient was recovering from subacute liver failure and outcome for drug intolerance and suspected drug-induced hepatitis was not reported. The reporter assessed the causality between topiramate and subacute liver failure as probable.

The case was serious (hospitalization, life threatening, medically significant).<dbtext>To_char_NARRATIVE_5</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11009933

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19850421	Age: 25 Year	weight (kg): 50	BMI_int: 18.82	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	20101126			9 Day	recovering/resolving
2 Flu-like symptoms	20101124	20101124	1 Day	9 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 extavia interferon beta-1b	Subcutaneous	.2 ml millilitre(s)/ 231	20101118-20101124 8 Day	Multiple sclerosis
2 ibuprofen ibuprofen	Oral	150 Mg milligram(s)/ 245	20101124-20101124 1 Day	
3 Yasminelle drospirenone; ethinylestradiol	Oral	1 Df dosage form/Day 245	20101104-20101126 23 Day	Contraception

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Liver failure	201001		Unknown	Liver failure (genesis unknown) since Jan 2010

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[NOVARTIS, 14.01.2011 DE-002147023-PHHY2011DE02147](#)

Sender diagnosis: not reported

Narrative

Initial report was received from the physician via the Extracare-Service program on 07 Jan 2011: This polymedicated patient was treated with Extavia (interferon beta-1b) 250 ug/ml at a dose of 125 ug every other day from 18 Nov 2010 to 25 Nov 2010, for multiple sclerosis. On 26 Nov 2010, the patient developed recurrent liver failure and was hospitalized. The outcome and causality were not explicitly mentioned.

Sender's comment:

The available information was considered inadequate to fully assess the case.

Serious spontaneous report,

Hepatic failure [hospitalisation], assessed as unlisted according to the Core Data Sheet.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Insufficient information to provide a proper medical assessment.

Reporter's comment:

Sender diagnosis: not reported

Narrative

Initial report was received from the physician via the Extracare-Service program on 07 Jan 2011: This polymedicated patient was treated with Extavia (interferon beta-1b) 250 ug/ml at a dose of 125 ug every other day from 18 Nov 2010 to 25 Nov 2010, for multiple sclerosis. On 26 Nov 2010, the patient developed recurrent liver failure and was hospitalized. The outcome and causality were not explicitly mentioned.

Initial report was received from the physician via the Extracare-Service program on 07 Jan 2011: This polymedicated patient was treated with Extavia (interferon beta-1b) 250 ug/ml at a dose of 125 ug every other day from 18 Nov 2010 to 25 Nov 2010, for multiple sclerosis. On 26 Nov 2010, the patient developed recurrent liver failure and was hospitalized. The outcome and causality were not explicitly mentioned.

Follow up physician report in writing was received on 03 Feb 2011: This patient had a medical history of liver failure (genesis unknown) in Jan 2010. The treating neurologist asked a university hospital for advice before starting treatment with Extavia in this patient due to the patient's history of liver failure. The consulted physicians had no concerns to start Extavia. The patient was treated with Extavia due to multiple sclerosis in the period from 18 Nov 2010 to 24 Nov 2010 at a dose of 0.2 ml every other day (corresponding to 50 mcg). On 24 Nov 2010, the patient took one-time 150 mg ibuprofen due to flu-like symptoms due to Extavia treatment. She further took Yasminelle (drospirenon, ethinylestradiol) taken for oral contraception from 04 Nov 2011 to 26 Nov 2011. On 26 Nov 2010, the patient developed acute liver failure and was hospitalized. The final outcome with respect to liver failure was assessed as improved but not yet recovered. The physician suspected no causal relationship to Extavia for the liver failure but suspected a causal relationship to ibuprofen and Yasminelle and other unknown reasons. On 24 Nov 2010, patient recovered completely from the event flu-like symptoms. No further information available at the time of this report.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11011791

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 46 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Redness of legs	2010				recovered/resolved
2 Gamma-GT increased	201005				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Antidepressiva		/	-	Multiple sclerosis
2 ibuprofen		/	-	Pain
ibuprofen			-	
3 PARACETAMOL		/	-	Pain
paracetamol			-	
4 SIRDALUD		/	-	Multiple sclerosis
tizanidine hydrochloride			-	
5 Targin 20/10 mg Retardtabletten	Oral	20 Mg milligram(s)/	200911-201005	Pain
naloxone hydrochloride; oxycodone hydrochloride		212	6 Month	

Drugs (concomitant)

6 LAXATIVES		/	-
7 OMEPRAZOL	/00661201/	/	-
omeprazole			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			Yes	
Multiple sclerosis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Gamma-glutamyltransferase	200911	142	unk	
Gamma-glutamyltransferase	201005	1055	unk	
Gamma-glutamyltransferase	201007	422	unk	
Gamma-glutamyltransferase	201101	See notes	unk	

Narratives (nur bei elektronischem Eingang)

NAPPMUNDI, 24.01.2011 DE-NAPPMUNDI-DEU-2011-0006884

Sender diagnosis: not reported

Narrative

Case report reference number DEU-2011-0006884 was received on 13JAN2011 from a physician in Germany via a company representative and additional information on 17JAN2011 from the initially reporting physician via a company representative. The new information received on 17JAN2011 made this case serious. This spontaneous report relates to a 46 year-old female patient.

The patient's current conditions included pain due to multiple sclerosis. Her concomitant medication included laxatives, paracetamol, omeprazole, and several other unspecified medication. The medical history was not reported. Her historical medication included Durogesic (fentanyl). In NOV2009 the patient was switched from Durogesic to Targin 20/10mg (oxycodone/naloxone, 20mg twice daily) due to increased constipation under Durogesic therapy. On an unspecified date she experienced red spots on her legs and her gamma-glutamyltransferase was at 142 [unit not reported] in NOV2009. In MAY2010 another blood analyses showed gamma-glutamyltransferase at 1055 [unit not reported]. The treatment with Targin, paracetamol, and omeprazole was discontinued. In JUL2010 her gamma-glutamyltransferase was at 422 [unit not reported] and the red spots were disappearing. At the time of this report the gamma-glutamyltransferase was normal and she had recovered from her red spots. The patient had been switched back to Durogesic.

The reporting physician found the increased gamma-glutamyltransferase to be possibly related to the treatment with Targin. A detailed causality assessment between the red spots and the treatment with Targin was not given.

This case was rated to be serious because of the medical significance of the adverse event "Gamma GT increased up to 1055 [unit not reported]". Additional information has been requested. The case will be updated accordingly should more information become available.

NAPPMUNDI, 27.05.2011 DE-NAPPMUNDI-DEU-2011-0006884

Sender diagnosis: not reported

Narrative

Case report reference number DEU-2011-0006884 was received on 13JAN2011 from a physician in Germany via a company representative and additional information on 17JAN2011 from the initially reporting physician via a company representative. The new information received on 17JAN2011 made this case serious. This spontaneous report relates to a 46 year-old female patient.

The patient's current conditions included pain due to multiple sclerosis. Her concomitant medication included laxatives, paracetamol, omeprazole, and several other unspecified medication. The medical history was not reported. Her historical medication included Durogesic (fentanyl). In NOV2009 the patient was switched from Durogesic to Targin 20/10mg (oxycodone/naloxone, 20mg twice daily) due to increased constipation under Durogesic therapy. On an unspecified date she experienced red spots on her legs and her gamma-glutamyltransferase was at 142 [unit not reported] in NOV2009. In MAY2010 another blood analyses showed gamma-glutamyltransferase at 1055 [unit not reported]. The treatment with Targin, paracetamol, and omeprazole was discontinued. In JUL2010 her gamma-glutamyltransferase was at 422 [unit not reported] and the red spots were disappearing. At the time of this report the gamma-glutamyltransferase was normal and she had recovered from her red spots. The patient had been switched back to Durogesic.

The reporting physician found the increased gamma-glutamyltransferase to be possibly related to the treatment with Targin. A detailed causality assessment between the red spots and the treatment with Targin was not given.

This case was rated to be serious because of the medical significance of the adverse event "Gamma GT increased up to 1055 [unit not reported]".

***Additional information was received on 17MAY2011 from the initially reporting physician.

Accordingly the patient had an isolated gamma-glutamyltransferase increase which had normalised again. The patient was on a variety of concomitant medications also including Sirdalud (tizanidine), ibuprofen and antidepressants for her multiple sclerosis. All medications including Targin had been discontinued permanently.

The reporter found that the cause of the isolated gamma-glutamyltransferase increase was most likely due to the number of medications used by the patient.

No additional information is expected.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11031726

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19850421	Age: 25 Year	weight (kg): 50	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	20101124			21 Day	recovering/resolving
2 Flu-like symptoms	20101124	20101124	1 Day	21 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 extavia interferon beta-1b	Subcutaneous	.2 ml millilitre(s)/	20101118-20101124 7 Day	Multiple sclerosis
2 ibuprofen ibuprofen		150 Mg milligram(s)/	20101124-20101124 1 Day	Unevaluable reaction
3 Yasminelle drospirenone; ethinylestradiol	Oral	1 Df dosage form/Day 82	20101104-20101128 25 Day	Contraception

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Liver failure	201001		Yes	Liver Failure
Multiple sclerosis			Unknown	Multiple Sclerosis

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 23.02.2011 DE-BAYER-2011-015802

Sender diagnosis: not reported

Narrative

Serious, Related, Unlisted

This case is a spontaneous case report received from an other pharmaceutical company (classified as spontaneous by this company) from GERMANY on 16-FEB-2011 which refers to a 25 year old female patient who was prescribed DROSPIRENONE + ETHINYLESTRADIOL (Yasminelle), IBUPROFEN and INTERFERON BETA-1B (EXTAVIA) and experienced RECURRENT LIVER FAILURE and FLU-LIKE SYMPTOMS.

The patient's concurrent condition included Liver Failure (Genesis unknown) and Multiple Sclerosis.

On 04-NOV-2010 the patient started Yasminelle at 1 DF, QD oral(1) for contraception. It was unknown whether Yasminelle was used previously. The patient received co-suspect medication.

On 24-NOV-2010 the patient started IBUPROFEN (IBUPROFEN) at 150 mg for unknown. It was unknown whether IBUPROFEN was used previously. The patient received co-suspect medication.

On 18-NOV-2010 the patient started INTERFERON BETA-1B (EXTAVIA) at 150 mg at 0.2 ml, QOD subcutaneous for Multiple sclerosis. It was unknown whether EXTAVIA was used previously.

On 24-NOV-10, approximately 21 days after starting Yasminelle, approximately 1 day after starting IBUPROFEN, approximately 7 days after starting EXTAVIA the patient experienced RECURRENT LIVER FAILURE and therefore the patient was hospitalized.

On 24-NOV-10, approximately 21 days after starting Yasminelle, approximately 1 days after starting IBUPROFEN, approximately 7 days after starting EXTAVIA the patient experienced FLU-LIKE SYMPTOMS considered non-serious.

Yasminelle was discontinued on 28-NOV-10.

IBUPROFEN was discontinued on 24-NOV-10.

EXTAVIA was discontinued on 24-NOV-10.

The outcome of the event RECURRENT LIVER FAILURE was recovering / resolving and the outcome of the event FLU-LIKE SYMPTOMS was recovered / resolved.

The reporting other pharmaceutical company considered RECURRENT LIVER FAILURE and FLU-LIKE SYMPTOMS as related to treatment with DROSPIRENONE + ETHINYLESTRADIOL and IBUPROFEN treatment.

Additionally, the physician reporting the other pharmaceutical company supposed a relationship to other unknown reasons.

No further information was available at the time of the report from an other pharmaceutical company.

Possible duplicate in central database since it also refers to extavia.

For further details please refer to attached english CIOMS I report.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11043926

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19580921	Age: 52 Year	weight (kg): 65	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Raised liver function tests					Unknown
2 Pancytopenia	20110214			22 Day	not recovered/not resolved
3 Renal function test abnormal					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal	Oral	2400 Mg milligram(s)/Day	20110124-20110126	Lumbar syndrome
ibuprofen		82	1 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sudden deafness	201010		Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Hemoglobin		8.8	NA	
Thrombocyte count		30,000	NA	
Renal function test		nicht vorhanden	NA	absent
Liver function test		nicht vorhanden	NA	absent
Leukocyte count NOS		3,000	NA	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 24.02.2011 DE-SZ09-CHPA2011DE02773

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 15 Feb 2011 from a pharmacist: This female patient with an unknown medical history developed bone marrow aplastic at an unknown date 3 weeks after treatment with IBUHEXAL 800 FILMTABLETTEN(ibuprofen) at a dosage of 800 mg three times in one day for unspecified indication. This event involved or prolonged hospitalization (dates not specified). No information on concomitant treatment was given. The outcome of the adverse event was not reported. The reporter provided no causality assessment. Additional information was requested.

Serious spontaneous report, Bone marrow aplastic [Hospitalization], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

NOVARTIS, 10.03.2011 DE-SZ09-CHPA2011DE02773

Sender diagnosis: not reported

Narrative

Initial spontaneous report received on 15 Feb 2011 from a pharmacist: This female patient with an unknown medical history developed bone marrow aplastic at an unknown date 3 weeks after treatment with IBUHEXAL 800 FILMTABLETTEN(ibuprofen) at a dosage of 800 mg three times in one day for unspecified indication. This event involved or prolonged hospitalization (dates not specified). No information on concomitant treatment was given. The outcome of the adverse event was not reported. The reporter provided no causality assessment. Additional information was requested.

Follow-up information received on 03 Mar 2011 from a physician. The patient, with a medical history of sudden deafness in Oct 2010, developed severe pancytopenia on 14 Feb 2011. Treatment with IBUHEXAL lasted from about 24 Jan 2011 until 26 Jan 2011 at a daily dosage of 1200 mg for lumbar syndrome. Laboratory results revealed: hemoglobin 8.8; platelet count 30,000; leukocyte count 3,000; renal and liver values increased. Treatment with IBUHEXAL was discontinued. Condition remained unchanged. Causality was assessed as possible by the physician. No additional information was available.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11039110

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19700603	Age: 40 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Flatulence	2011				recovered/resolved
2 Leucopenia	20110201	20110330	58 Day	6 Day	recovered/resolved
3 Neutropenia	2011	2011			recovered/resolved
4 Weight gain	2011				not recovered/not resolved
5 Abdominal pain	2011	2011			recovered/resolved
6 Fatty liver	20110301			34 Day	not recovered/not resolved
7 Hair growth increased	20110301			34 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	Abdominal pain
diclofenac			-	
2 ibuprofen		/	-	Abdominal pain
ibuprofen			-	
3 NAVELBINE	Intravenous (not otherwis	25 Mg milligram(s)/	20110202-20110202	
vinorelbine tartrate		231	1 Day	
4 NAVELBINE	Intravenous (not otherwis	25 mg/m2 milligram(s)/sq.	20110126-20110126	Breast cancer
vinorelbine tartrate		231	1 Day	
5 NAVELBINE	Intravenous (not otherwis	25 Mg milligram(s)/	20110323-20110323	
vinorelbine tartrate		231	1 Day	
6 NAVELBINE	Intravenous (not otherwis	25 Mg milligram(s)/21 Day	20110330-	
vinorelbine tartrate		231	-	
7 Xeloda	Oral	2000 Mg milligram(s)/Day	201101-	Breast cancer
capecitabine			14 Day	

Drugs (concomitant)

8 neurobion	/	-	Drug use for unknown
cyanocobalamin; pyridoxine hydrochloride; thiamine hydrochloride		-	indication
9 Pantozol	40 Mg milligram(s)/12 Ho	201101-	Prophylaxis
pantoprazole sodium		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hand and foot syndrome	2010	2010	No	
Microscopic hematuria			Unknown	
Kidney contracted			Unknown	contracted kidney on one side
Hypermenorrhea			Unknown	
Abdominal pain upper			Unknown	chronic abdominal pain upper

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
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Alanine aminotransferase	20110217	69	U/l
Alanine aminotransferase	20110228	116	U/l
Alanine aminotransferase	20110316	58	U/l
Alanine aminotransferase	20110125	33	U/l
Aspartate aminotransferase	20110228	76	U/l
Aspartate aminotransferase	20110316	41	U/l
Aspartate aminotransferase	20110217	42	U/l
Aspartate aminotransferase	20110125	24	U/l
Gamma GT	20110217	48	U/l
Gamma GT	20110228	61	U/l
Gamma GT	20110316	37	U/l
WBC	20110201	2600	Giga/l
WBC	20110217	2300	Giga/l
WBC	20110221	2620	Giga/l
WBC	20110330	3000	Giga/l

Narratives (nur bei elektronischem Eingang)

PFM, 09.03.2011 DE-PFM-DE-2011-0585

Sender diagnosis: not reported

Narrative

Case number: DE-2011-0585

DFR: 03-March-2011

INTRODUCTION:

This case was reported by a gynecologist via phone call and referred to a female patient who received Navelbine (vinorelbine) IV and developed weight gain, hairiness of face and fore arm, flatulence, moderate fatty liver, and severe abdominal pain.

EVENT:

A female patient received Navelbine IV 25 mg/m² on day first and 8th of therapy and Xeloda (capecitabine) oral 2000 mg for 14 days in Jan-2011 for the indication of breast cancer.

On an not specified date after first cycle of therapy she developed severe abdominal pain (that worsened after second cycle), moderate fatty liver, hairiness of sides of face and forearms, weight gain from 55 to 65 kg during one week, and flatulence.

She was hospitalized because of severe abdominal pain on 27-Feb-2011.

TREATMENT and ACTION TAKEN: not specified

OUTCOME: Severe abdominal pain subsided (exact date not specified) and the patient was discharged from hospital on 01-March-2011. The other mentioned events still not abated.

Reporter considered abdominal pain as serious and provided no assessment for the other mentioned events.

AFFILIATE ASSESSMENT:

Causality: according to the WHO algorithm, causality is possible for mentioned events, because of positive time correlation, and Xeloda can be another explanation.

Seriousness: hospitalization for abdominal pain and medically significant for fatty liver.

Expectedness: According to the SPC of Navelbine IV, the mentioned events are unexpected, except pain that is expected.

Further information has been requested.

PIERRE FABRE MEDICAMENT HEADQUARTER'S PRODUCT SAFETY DEPT. MEDICAL ASSESSMENT.

Company evaluation : According to the French method, the assessment of the causality is C1S1 I1, dubiously related to Navelbine for all events except abdominal pain which was assessed as C1S2I1, dubiously related to Navelbine. Xeloda is also considered as suspect.

Seriousness criteria : hospitalization for abdominal pain and medically significant for fatty liver.

Other ADRs: Non serious

Labeling : Referring to Pierre Fabre Medicament Core Data Sheet of Navelbine injectable solution, all ADRs were assessed as unlisted except abdominal pain assessed as listed.

PFM, 08.04.2011 DE-PFM-DE-2011-0585

Sender diagnosis: not reported

Narrative

Case number: DE-2011-0585

DFR: 03-March-2011

INTRODUCTION:

This case was reported by a gynecologist via phone call and referred to a female patient who received Navelbine (vinorelbine) IV and developed fatty liver, moderate hepatomegalia, considerable liver parenchymal damage, severe aggravated abdominal pain, afebrile neutropenia NCI/ CTC grade 3, leucopenia NCI/CTC NCI/ CTC grade 2 , flatulence, weight gain, and hairiness of face and fore arm.

PAST DRUG HISTORY and PAST MEDICAL HISTORY:

The patient received Taxotere and Cyclophosphamide for breast cancer since 07-Sep-2010 , then this therapy stopped before Navelbine therapy (date not specified) . She developed hand and foot syndrome during Taxotere and Cyclophosphamide therapy, that was treated with Lyrica (Pregabalin).

CONCOMITANT DISEASE:

- progression of disease under therapy with Taxotere and Cyclophosphamide
- shrank kidney on one side, chronic upper abdominal pain , microhematuria, hypermenorrhea
- no history of allergies
- no previous abnormal hairiness
- abdominal pain

CONCOMITANT MEDICATION:

Pantozol (Pantoprazol) for prophylaxis (stomach protection), Neurobion (Cyanocobalamin, Pyridoxine hydrochloride, Thiamine hydrochloride) for unknown indication. The patient received Ibuprofen (date not specified) and Diclofenac since Jan-2011 for abdominal pain.

EVENT:

A 40 year-old female patient received Taxotere and Cyclophosphamide for the indication of breast cancer since 07-Sep-2010. Because of progression of disease under this therapy stopped (date not specified) and the chemotherapy switched to Navelbine IV 25 mg/m2 from 26-Jan-2011 cyclically as well as Xeloda (capecitabine) oral 2000 mg for a duration of 14 days.

She received Navelbine IV on 26-Jan-2011 and on 02-Feb-2011.

She experienced leucopenia NCI/CTC grade 2 on 01-Feb-2011.

After first cycle of therapy she developed severe abdominal pain, flatulence, neutropenia NCI/CTC grade 3 and weight gain from 55 to 65 kg during one week (exact onset dates could not be specified).

Abdominal pain worsened after second administration of therapy and she was hospitalized because of severe aggravated abdominal pain on 27-Feb-2011.

On 01-Mar-2011 she experienced moderate fatty liver, considerable liver parenchymal damage, moderate hepatomegalia and hairiness of sides of face and forearms.

Hepatitis infection has been excluded.

TREATMENT:

Diclofenac since Jan 2011 and Ibuprofen (date not specified) continued for abdominal pain and on an not specified date were discontinued because the physician considered them as suspected drugs for events. Granulocyte colony stimulating factor for leucopenia and neutropenia (date not specified). No treatment for the other events.

RELEVANT LABORATORY VALUES:

WBC: 01.02.2011: 2600/microliter, 17.02.2011: 2300//microliter, 21.02.2011 2620/microliter, 30.03.2011 3000/microliter.

Neutrophils were below 1000/microliter during the period of leucopenia (exact dates not provided).

Reportedly the liver laboratory values on 23-Dec-2010 (about one month before Navebine therapy) were normal.

GOT: 25.01.2011: 24 U/l, 17.2.2011: 42 U/l, 28.02.2011: 76 U/l, 16.03.2011: 41 U/l

GPT: 25.01.2011: 33 U/l, 17.2.2011: 69 U/l, 28.02.2011: 116 U/l, 16.03.2011: 58 U/l

GGT: 17.2.2011: 48 U/l, 28.02.2011: 61 U/l, 16.03.2011: 37 U/l

ACTION TAKEN :

Navelbine therapy has been postponed because of abdominal pain, and neutropenia and she received again Navelbine IV on 23-Mar-2011 and on 30-Mar-2011. According to the physician suspected drugs (Ibuprofen, Diclofenac) has been discontinued.

OUTCOME:

Severe abdominal pain subsided (exact date not specified) and the patient was discharged from hospital on 01-March-2011.

Flatulence has been resolved on a not specified date.

Leucopenia NCI/ CTC grade 2 resolved on 30-Mar-2011 and afebrile neutropenia grade 3 resolved in the same period of time (exact date not specified).

The other mentioned events still not abated. However the patient's situation is clinically stable.

Reporter considered the events as serious and as possibly related to Navelbine IV. He does not know if there is a link between abdominal pain and the hepatic disorders or any other causes.

AFFILIATE ASSESSMENT:

Causality: according to the WHO algorithm, causality is possible for mentioned events, because of positive time correlation. Xeloda and other concomitant medication can be another explanation. Causality for abdominal pain is unlikely related as the patient has prior history.

Seriousness: hospitalization for abdominal pain. Medically significant for fatty liver, moderate hepatomegalia, neutropenia grade 3, considerable liver parenchymal damage. Other events are non serious.

Expectedness: According to the SPC of Navelbine IV, the mentioned events are unexpected, except pain, afebrile neutropenia and leucopenia that are expected.

PIERRE FABRE MEDICAMENT HEADQUARTER'S PRODUCT SAFETY DEPT. MEDICAL ASSESSMENT.

Company evaluation : According to the French method, the assessment of the causality is C2S2I2, possibly related to Navelbine for leucopenia and neutropenia, C1S1 I1, dubiously related to Navelbine for flatulence, fatty liver, weight gain and hairiness and C1S2I1, dubiously related to Navelbine for abdominal pain. Xeloda is also considered as suspect. Diclofenac and ibuprofen were also considered as suspect by the reporter.

Seriousness criteria : hospitalization for abdominal pain and medically significant for fatty liver and neutropenia.

Other ADRs: Non serious

Labeling : Referring to Pierre Fabre Medicament Core Data Sheet of Navelbine injectable solution, all ADRs were assessed as unlisted except abdominal pain, neutropenia and leukopenia, assessed as listed.

FU received on 01-Apr-2011 from physician:

- patients initial, birth date
- NVB dosage
- concomitant disease and medication
- past drug and medical history
- action taken outcome
- onset dates
- his assessment

Narrative and affiliate assessment modified.

Further information has been requested.

FU received on 07-Apr-2011, the physician added

- neutropenia and leucopenia as event
- laboratory values
- outcome
- concomitant medication, treatment, past medical history and past drug history
- his assessment

Narrative and affiliate assessment modified.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11049060

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19600603	Age: 50 Year	weight (kg): 93	BMI_int: 37.25	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ANA positive					Unknown
2 Skin discoloration	201101				recovering/resolving
3 Nausea	201102				recovering/resolving
4 Liver necrosis					recovering/resolving
5 Jaundice	201102	15.02.2011			recovering/resolving
6 Hepatitis acute					recovering/resolving
7 Fatigue	201102				recovering/resolving
8 Discoloured stools	201102				recovering/resolving
9 Cholestasis					recovering/resolving
10 Hepatopathy	201101			12 Year	recovering/resolving
11 Hepatitis	201101			10 Year	recovering/resolving
12 Tiredness					recovering/resolving
13 Stool discolored					recovering/resolving
14 Skin discolouration					recovering/resolving
15 Icterus					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 carbamazepin carbamazepine	Oral	100 Mg milligram(s)/Day 212	201101-20110228 1 Month	Restless leg syndrome
2 CARBAMAZEPIN carbamazepine	Oral	100 Mg milligram(s)/Day 212	201012-201101 32 Day	Restless leg syndrome
3 FEMOSTON dydrogesterone; estradiol	Oral	1 Df dosage form/Day 212	1999-20110304 12 Year	Menopause
4 IBUPROFEN ibuprofen	Oral	2*800 Mg milligram(s)/We 82	2001-20110304 10 Year	Headache NOS
5 LISINAPRIL lisinopril	Oral	5 Mg milligram(s)/Day 245	2006-20110304 5 Year	Hypertension
6 T-LONG tramadol hydrochloride	Oral	100 Mg milligram(s)/Day 212	2006-20110304 5 Year	Back pain

Drugs (concomitant)

7 beloc zok metoprolol succinate	Oral	95 Mg milligram(s)/Day 212	2001- -	Hypertension
8 BELOC ZOK metoprolol succinate	Oral	95 Mg milligram(s)/ Day 212	2001- -	Hypertension

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Menopause			Yes	
Renal insufficiency			Yes	Beginn unbekannt; Stad. III
Chronic back pain			Yes	Beginn unbekannt; BWS/LWS
Headache tension			Yes	Beginn unbekannt
Restless leg syndrome			Yes	Beginn unbekannt
Hypertension arterial			Yes	Beginn unbekannt

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit weitere Erläuterung
Aspartate aminotransferase	20110305	14.6	µmol/l*s
Aspartate aminotransferase	20110306	13.6	µmol/l*s
Aspartate aminotransferase	20110307	12.3	µmol/l*s
Aspartate aminotransferase	20110310	10.6	µmol/l*s
Alanine aminotransferase	20110304	28.4	µmol/l*s
Alanine aminotransferase	20110305	21.6	µmol/l*s
Alanine aminotransferase	20110306	18.2	µmol/l*s
Alanine aminotransferase	20110307	17.1	µmol/l*s
Alanine aminotransferase	20110310	15.2	µmol/l*s
Bilirubin total	20110304	575	µmol/l
Bilirubin total	20110305	541	µmol/l
Bilirubin total	20110306	515	µmol/l
Bilirubin total	20110307	432	µmol/l
Bilirubin total	20110310	248	µmol/l
Alkaline phosphatase	20110304	2.02	µmol/l*s
Alkaline phosphatase	20110305	1.81	µmol/l*s
Alkaline phosphatase	20110306	1.93	µmol/l*s
Alkaline phosphatase	20110307	1.79	µmol/l*s
Alkaline phosphatase	20110310	1.9	µmol/l*s
Ammonia	20110305	44	µmol/l
Ammonia	20110306	52	µmol/l
Ammonia	20110307	62	µmol/l
Ammonia	20110310	43	µmol/l
Quick's test	20110304	67	Prozent
Quick's test	20110305	66	Prozent
INR	20110304	1.3	NA
C-reactive protein	20110304	6.6	mg/l
C-reactive protein	20110306	9.1	mg/l
C-reactive protein	20110310	5.2	mg/l
Lipase	20110304	0.28	µmol/l*s
Anti-HCV positive	20110304	-	positiv/neg:
Anti-HBs antibody	20110304	-	IU/l
Liver biopsy	20110307	Abnormal	NA
Gamma GT	20110304	4.53	NA
Gamma GT	20110305	4.11	NA
Gamma GT	20110306	4.45	NA
Gamma GT	20110307	4.33	NA
Gamma GT	20110310	3.82	NA

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 24.03.2011 DE-PVZIBE-04-10236

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
akute Hepatopathie

KURZZUSAMMENFASSUNG:

Leberschädigung nach Carbamazepin-Einnahme

BESCHREIBUNG:

Stat. Aufnahme zur # Abklärung eines schmerzlosen Ikterus sowie akuter Hepatitis #. Seit ca. Mitte Januar leichte Hautverfärbung, ab Mitte Februar dtl. Ikterus mit Übelkeit, Müdigkeit und entfärbtem Stuhl. Pat. nimmt seit Mitte Dez. Carbamazepin 200 mg 1xd aufgrund eines Restless-legs-Syndroms, hat Dosis nach Beschwerdebeginn halbiert. Alkohol- und Drogeneinnahme werden verneint, kein Auslandsaufenthalt.

THERAPIE:

alle Medikamente abgesetzt, dann weiter mit Beloc zok 95 1-0-0, Pantozol 40 1-0-0, Vit B duo 1-0-1.

VERLAUF:

Abdomensonographie 4.3.11: eingeschränkte Beurteilbarkeit, kein Nachweis freier Flüssigkeit. Leberbiopsie 7.3.11: aus Leberstanzylinder: schwergradige Entzündungsreaktion, dtl. Eosinophilie; Leberzellnekrosen mit Blutung. Anteil nekrotischer Hepatozyten: 50%; mäßiggradige Cholestase; keine Fettleber; Beurteilung: Bild eines medikamentös-tox. Leberschadens, Regenerationsfähigkeit wird als gut eingeschätzt; Schädigung könnte länger zurückliegen; Antikörper-Screening: ANA-Hep:1:5120 --> pos. Leberwerte besserten sich im Verlauf, so daß die Patientin mit Wiedervorstellungstermin entlassen wurde. Fermoston und Ibuprofen sollen in Zukunft vermieden werden.

ERLÄUTERUNGEN ZUR UAW:

Stat. Aufnahme aufgrund einer symptomatischen Hepatopathie mit starker Fermenterhöhung nach Carbamazepin-Einnahme. Einen Anhalt für virale Genese gab es nicht.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: carbamazepin

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: In der Fachinfo werden unter selten bis sehr selten Ikterus sowie versch. Hepatitis-Formen beschrieben, die häufig in den ersten Therapiemonaten auftreten. Da vor der Carbamazepintherapie die Leberwerte anamnestisch im Normbereich lagen (regelmäßige Kontrollen fanden statt) und nach Absetzen wieder rückläufig sind --> C3.

MEDIKAMENT: femoston

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: FI: selten Veränderungen der Leberfkt. evtl. verbunden mit Unwohlsein, Gelbsucht, abdominalen Schmerzen.

Abbott Arzneimittel GmbH, 01.04.2011 DE-SOLVAY-00311002550

Sender diagnosis: not reported

Narrative

This case was provided by the German Health Authority (Bundesanstalt fuer Arzneimittel und Medizinprodukte, BFARM).

A physician report concerning a 50-year-old adult female patient who experienced acute hepatopathy while being treated with two suspect drugs: FEMOSTON and Carbamazepin.

FEMOSTON (unknown daily dose) was started on an unknown date in 1999 for menopause. Carbamazepin (unknown daily dose) was started on an unknown date in mid DEC 2010 for restless leg syndrome.

On an unknown date in JAN 2011, the patient experienced acute hepatopathy. The patient was admitted for investigation of painful jaundice and acute hepatitis. Since the symptoms started the Carbamazepin the patient had halved the dose. Since about mid-JAN 2011 there had been a slight discolouration of the skin, since mid-FEB 2011 definite jaundice with nausea, tiredness and discoloured stools. The patient had taken no alcohol, no drugs and had not stayed abroad. On 28 FEB 2011, the patient discontinued Carbamazepin. On 04 MAR 2011, the patient discontinued FEMOSTON, T-Long and Ibuprofen. All medications were stopped and then with Beloc Zok was restarted with Pantozol and Vitamin B duo. On 04 MAR 2011, abdominal ultrasound showed limited assessment possible and no evidence of free fluid. On 07 MAR 2011, biopsy of punch cylinder from the liver showed severe inflammation reaction, definite eosinophilia; liver cell necroses with bleeding. Proportion of necrotic hepatocytes was 50%; moderate cholestasis; no fatty liver. The assessment was appearance of liver damage due to medication, regeneration capability was assessed as good; the damage could date back longer; antibody screening: ANA-HEp: 1:5120 - positive. Liver values improved later so the patient was discharged with a date for a further appointment. The reporter informed that FEMOSTON and Ibuprofen should be avoided in future. The reporter informed that the admission was owing to symptomatic hepatopathy with severe increase in fermenters after taking Carbamazepine. No evidence of a viral origin. At the

time of reporting, the patient discontinued FEMOSTON and Carbamazepin and the adverse event was abating.

Outcome: Abating.

The reporter assessed the causal relationship between FEMOSTON and the adverse event as 'unlikely' and with Carbamazepin as 'probable'. Abbott Products (formerly Solvay Pharmaceuticals) judged the case as 'suspect'.

The explanation regarding the causality between FEMOSTON and the adverse event as in the specialist information rarely changes in the liver function, possibly associated with nausea, jaundice, abdominal pain. The explanation regarding the causality between Carbamazepine and the adverse event as in the specialist information jaundice and various forms of hepatitis are described under rare to very rare, mostly occurring in the first months of treatment. Since, according to the history, the liver values were within the normal range before the Carbamazepine therapy (regular checks were carried out) and after stopping they again receded.

The reporter judged the case as 'serious' due to 'prolonged inpatient hospitalization'.

BFARM's comment: Additional information is expected.

Generics [UK] Ltd, 06.04.2011 DE-MYLANLABS-2011S1006187

Sender diagnosis: not reported

Narrative

Regulatory authority report received 25 Mar 2011/

This case, received from the German health authority, involved a 50 year-old female patient who reportedly experienced hepatopathy (liver disorder) while receiving Carbamazepine for the treatment of restless leg syndrome.

Medical history included hypertension arterial, headache tension, chronic back pain, renal insufficiency and restless leg syndrome. The patient was menopausal.

Concomitant medications were ibuprofen, beloc zok (metoprolol succinate), t-long (tramadol hydrochloride) and lisinopril.

Non-company suspect drug included femoston (estradiol + dydrogesterone) initiated 1999 for treatment of menopause.

1999: The patient initiated femoton therapy (oral, 1 DF, Q24) for menopause

Dec 2010: The patient initiated Carbamazepine (oral, 200 mg, Q24) because of restless legs syndrome

Jan 2011: The patient experienced hepatopathy - the patient was hospitalised due to a painless jaundice and acute hepatitis, around the middle of the month.

Unknown date: The patient reduced the daily carbamazepine dosage to 100 mg, following the start of the event. Alcohol and Drug use was ruled out, no foreign travel

Feb 2011: The patient had developed light skin discolouration. Jaundice with nausea, fatigue and pale stools.

28 Feb 2011: The patient discontinued carbamazepine therapy.

04 Mar 2011: The patient discontinued femoston therapy. Abdominal ultrasound: limited evaluability, no evidence of free fluid. Liver biopsy

Unknown date: The patient was treated with Beloc zok 95, Pantozol 40, Vit B duo.

07 Mar 2011: Liver punch biopsy: Severe inflammatory response, clear. eosinophilia, liver cell necrosis with hemorrhage. Proportion of necrotic hepatocytes: 50%; moderate cholestasis, no fatty liver; assessment; picture of a drug-toxicity. Liver damage; regenerative capacity is considered good; injury might be older.

Antibody screening: ANA-Hep: 1:5120 -> pos. Liver function tests improved in the course so that the patient is discharged with an upcoming consultation. Femoston and ibuprofen should be avoided in future.

Outcome for event was provided as recovering.

No further information was provided.

NOVARTIS, 08.04.2011 DE-002147023-PHHY2011DE25418

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE25418 is a PMS report from a regional pharmacovigilance center (intensive monitoring project) via German Health Authority (DE-BfArM-11049060) received on 24 Mar 2011: This report refers to a 50-year-old female patient. This patient had medical condition of hypertension arterial, headache tension, chronic back pain and renal insufficiency. This polymedicated patient was treated with carbamazepine (unknown manufacturer) for restless leg syndrome at a daily dosage of 200 mg since Dec 2010 and Femoston (estradiol plus dydrogesterone) for menopause at a daily dosage of 1x10 mg since 1999. It was reported that on an unspecified date, the patient was admitted for diagnostic evaluation of painless jaundice and acute hepatitis. The patient experienced slight skin discoloration since about mid Jan, from mid Feb onwards pronounced jaundice with nausea, fatigue and discoloured stools. When in mid of Jan 2011 the first symptoms occurred (skin discolouration), 50 % reduction of the current dose of carbamazepine was undertaken. Blood test done on 04 Mar 2011 showing increased alanine aminotransferase 28.4 umol/l's (normal value 0 to 0.56), increased aspartate aminotransferase 14.6 umol/l's (normal value 0 to 0.54), increased ammonia 44 umol/l's (normal value 11.5 to 38), increased bilirubin total 575 umol/l's (normal value 0 to 21) and increased gamma GT 4.53 umol/l's (normal value 0 to 0.63). Abdominal

sonography done on 4 Mar 2011 restricted examination conditions, no evidence of free fluid. Liver biopsy on 7 Mar 2011: from liver punch biopsy cylinder, severe inflammatory reaction, marked eosinophilia, liver cell necroses with bleeding. Proportion of necrotic hepatocytes: 50 %; moderate cholestasis, no fatty liver, assessment: pattern of drug-toxic liver damage. Regenerability was estimated as good, damage could be long-standing, antibody screening: antinuclear antibodies (ANA) Hep: 1:5120 -> positive. Liver parameter values improved during the course, allowing the patient to be discharged with an appointment for re-attendance. Admission to hospital with symptomatic hepatopathy and a marked increase in enzymes following carbamazepine medication. No evidence of a viral pathogenesis. The patient was treated with pantozol 40mg daily and vitamin B duo (BID). The outcome of the adverse events was reported as recovering. It was stated that hepatopathy might have happened long time before. Causality was assessed as probable for (carbamazepine) and unlikely for (femoston) by the reporter.

Abbott Arzneimittel GmbH, 21.04.2011 DE-SOLVAY-00311002550

Sender diagnosis: not reported

Narrative

This case was provided by the German Health Authority (Bundesanstalt fuer Arzneimittel und Medizinprodukte, BFARM).

A physician report concerning a 50-year-old adult female patient who experienced acute hepatopathy while being treated with two suspect drugs: FEMOSTON and Carbamazepin.

FEMOSTON (unknown daily dose) was started on an unknown date in 1999 for menopause. Carbamazepin (unknown daily dose) was started on an unknown date in mid DEC 2010 for restless leg syndrome.

On an unknown date in JAN 2011, the patient experienced acute hepatopathy. The patient was admitted for investigation of painful jaundice and acute hepatitis. Since the symptoms started the Carbamazepin the patient had halved the dose. Since about mid-JAN 2011 there had been a slight discolouration of the skin, since mid-FEB 2011 definite jaundice with nausea, tiredness and discoloured stools. The patient had taken no alcohol, no drugs and had not stayed abroad. On 28 FEB 2011, the patient discontinued Carbamazepin. On 04 MAR 2011, the patient discontinued FEMOSTON, T-Long and Ibuprofen. All medications were stopped and then with Beloc Zok was restarted with Pantozol and Vitamin B duo. On 04 MAR 2011, abdominal ultrasound showed limited assessment possible and no evidence of free fluid. On 07 MAR 2011, biopsy of punch cylinder from the liver showed severe inflammation reaction, definite eosinophilia; liver cell necroses with bleeding. Proportion of necrotic hepatocytes was 50%; moderate cholestasis; no fatty liver. The assessment was appearance of liver damage due to medication, regeneration capability was assessed as good; the damage could date back longer; antibody screening: ANA-Hep: 1:5120 - positive. Liver values improved later so the patient was discharged with a date for a further appointment. The reporter informed that FEMOSTON and Ibuprofen should be avoided in future. The reporter informed that the admission was owing to symptomatic hepatopathy with severe increase in fermenters after taking Carbamazepine. No evidence of a viral origin. At the time of reporting, the patient discontinued FEMOSTON and Carbamazepin and the adverse event was abating.

Outcome: Abating.

The reporter assessed the causal relationship between FEMOSTON and the adverse event as 'unlikely' and with Carbamazepin as 'probable'. Abbott Products (formerly Solvay Pharmaceuticals) judged the case as 'suspect'.

The explanation regarding the causality between FEMOSTON and the adverse event as in the specialist information rarely changes in the liver function, possibly associated with nausea, jaundice, abdominal pain. The explanation regarding the causality between Carbamazepine and the adverse event as in the specialist information jaundice and various forms of hepatitis are described under rare to very rare, mostly occurring in the first months of treatment. Since, according to the history, the liver values were within the normal range before the Carbamazepine therapy (regular checks were carried out) and after stopping they again receded.

The reporter judged the case as 'serious' due to 'prolonged inpatient hospitalization'.

BFARM's comment: Additional information is expected.

***Additional information received via email on 14 APR 2011:

The reporter informed that he was not able to send anonymized discharge letter etc.

HELIOS Klinikum Wuppertal, 04.05.2011 DE-PVZIBE-04-10236

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
akute Hepatopathie

KURZZUSAMMENFASSUNG:
Leberschädigung nach Carbamazepin-Einnahme
BESCHREIBUNG:

Stat. Aufnahme zur # Abklärung eines schmerzlosen Ikterus sowie akuter Hepatitis #. Seit ca. Mitte Januar leichte Hautverfärbung, ab Mitte Februar dtl. Ikterus mit Übelkeit, Müdigkeit und entfärbtem Stuhl. Pat. nimmt seit Mitte Dez. Carbamazepin 200 mg 1xd aufgrund eines Restless-legs

Syndroms, hat Dosis nach Beschwerdebeginn halbiert. Alkohol- und Drogeneinnahme werden verneint, kein Auslandsaufenthalt.

THERAPIE:

alle Medikamente abgesetzt, dann weiter mit Beloc zok 95 1-0-0, Pantozol 40 1-0-0, Vit B duo 1-0-1.

VERLAUF:

Abdomensonographie 4.3.11: eingeschränkte Beurteilbarkeit, kein Nachweis freier Flüssigkeit. Leberbiopsie 7.3.11: aus Leberstanzzyylinder: schwergradige Entzündungsreaktion, dtl. Eosinophilie; Leberzellnekrosen mit Blutung. Anteil nekrotischer Hepatozyten: 50%; mäßiggradige Cholestase; keine Fettleber; Beurteilung; Bild eines medikamentös-tox. Leberschadens, Regenerationsfähigkeit wird als gut eingeschätzt; Schädigung könnte länger zurückliegen; Antikörper-Screening: ANA-Hep:1:5120 --> pos., AAK geg. MPO 1.2 U/ml (Normbereich: 0-5 U/ml), HBs-Antigen neg., Anti-HCV neg., IgG: 12,0, IgA: 4,28, IgM: 2,64; Leberwerte besserten sich im Verlauf, so daß die Patientin mit Wiedervorstellungstermin entlassen wurde. Femoston und Ibuprofen sollten in Zukunft vermieden werden, Lisinopril könnte nach Besserung der Leberwerte wieder angesetzt werden

ERLÄUTERUNGEN ZUR UAW:

Stat. Aufnahme aufgrund einer symptomatischen Hepatopathie mit starker Fermenterhöhung nach Carbamazepin-Einnahme. Einen Anhalt für virale Genese gab es nicht.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: ibuprofen

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: FI: sehr selten Leberfunktionsstörungen, Leberschäden, Leberversagen, akute Hepatitis

MEDIKAMENT: t-long

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: FI: sehr selten: erhöhte Transaminasen

MEDIKAMENT: carbamazepin

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: In der Fachinfo werden unter selten bis sehr selten Ikterus sowie versch. Hepatitis-Formen beschrieben, die häufig in den ersten Therapiemonaten auftreten. Da vor der Carbamazepintherapie die Leberwerte anamnestisch im Normbereich lagen (regelmäßige Kontrollen fanden statt) und nach Absetzen wieder rückläufig sind --> C3.

MEDIKAMENT: lisinopril

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: FI: sehr selten: cholestatischer Ikterus oder Hepatitis, Leberinsuffizienz

MEDIKAMENT: femoston

BETREFFENDE UAW: Hepatopathie

ERLÄUTERUNG: FI: selten Veränderungen der Leberfkt. evtl. verbunden mit Unwohlsein, Gelbsucht, abdominalen Schmerzen.

Bayer Vital GmbH, 10.05.2011 DE-BAYER-2011-038783

Sender diagnosis: not reported

Narrative

Serious/unlisted/related

Initial case report submitted by a health care professional, identified on the internet page of the German authority, was received by the Bayer on 05-May-2011, which refers to a 50 year old female patient.

The patient's concurrent condition included arterial hypertension, headache tension, chronic back pain, renal insufficiency stage III, restless leg syndrome.

The patient received the following concomitant medication: METOPROLOL SUCCINATE, PANTOPRAZOLE SODIUM and VITAMIN B DUO.

The patient received the following suspected drugs:

- from 2011 to 04-Mar-2011, IBUPROFEN at 800 mg, BIW, as needed oral for headache NOS.
- from 2006 to 04-Mar-2011, LISINAPRIL at 5 mg, QD oral for hypertension.
- from 1999 to 04-Mar-2011, ZUMESTON (FEMOSTON) at 1 DF, QD oral for menopause.
- from 2006 to 04-Mar-2011, t-long (TRAMADOL) at 100 mg, QD oral for back pain.
- from Dec-2010 to 28-Feb-2011, CARBAMAZEPINE at 100 mg, QD oral for restless leg syndrome.

In Jan-2011 after Carbamazepin-intake the patient experienced acute hepatopathy and therefore the patient was hospitalized for acute Hepatopathy.

DESCRIPTION:

Admission for clarification of painless icterus and acute hepatitis. Since the middle of Jan-2011 slight skin discoloration, since middle of Feb-2011 marked icterus with nausea, tiredness and discolored stool. Pat. administered Carbamazepin in a dosage of 200 mg 1xd due to restless-leg-Syndrom since middle of DEC-2010. Dosage has been halved after commencement of complaints. Alcohol and drug consumption has been negated. No stay in a foreign country.

THERAPY:

all drugs withdrawn.

Afterwards therapy continued with Beloc zok 95 1-0-0, Pantozol 40 1-0-0, Vit B duo 1-0-1.

COURSE:

Abdomen sonography on 04-MAR-2011: partially assessable, no detection of free fluid.

Liver biopsy on 07-MAR-2011: hepatic samples of biopsy showed inflammatory reaction of high degree, marked eosinophilia and liver cell necroses with bleeding. Part of necrotic hepatocytes: 50%; moderate degree of cholestasis; no fatty liver;

Assessment: Picture of drug induced toxic liver damage. Ability to Regeneration described as good. Damage could be dated back a longer time.

Antibody screening:

ANA-Hep:1:5120 --> pos., AAK neg. MPO 1.2 U/ml (Normal range: 0-5 U/ml), HBs-Antigen neg., Anti-HCV neg., IgG: 12,0, IgA: 4,28, IgM: 2,64; Hepatic parameters improved in the following course, so that the patient could be discharged on date of scheduled revisit.

Femoston and Ibuprofen should be avoided in the future, Lisinopril could be resumed after improvement of hepatic parameters.

EXPLANATION FOR UAW:

Admission due to symptomatic hepatopathy with severe increase in enzyme level after Carbamazepin intake. Hint to viral origin did not exist.

Further Reporter Causality Comment: EXPLANATION DUE TO CAUSALITIES:

DRUG: ibuprofen, possible

DRUG: t-long, unlikely

DRUG: carbamazepin, probable

DRUG: lisinopril, possible

DRUG: femoston, unlikely

sanofi-aventis, 11.05.2011 DE-SANOFI-AVENTIS-2011SA027922

Sender diagnosis: not reported

Narrative

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-11049060) on 06-May-2011.

The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

It's involved a 50-year-old female patient, whose present medical history included renal insufficiency stage III. For headache tension ibuprofen 800 mg twice weekly was taken as needed since 2001. Lisinopril 5 mg/day, used for arterial hypertension, was started in 2006. Tramadol hydrochloride 100 mg/day for chronic back pain (thoracic and lumbal spine), was given since 2006. For restless leg syndrome the treatment with carbamazepine 100 mg/day was initiated in Dec-2010. Dydrogesterone/estradiol for menopause was started in 1999. Concomitantly, the arterial hypertension was also treated with metoprolol succinate 95 mg/day. In the middle of January 2011 the patient began to present mild skin discoloration, which developed to icterus with nausea, tiredness and decolorized stool by one month later. Already on 28-Feb-2011 the dosage of carbamazepine was halved by the patient. On 04-Mar-2011 the patient was hospitalised for investigation of hepatitis and all suspected products were discontinued on the same day. The performed abdominal sonography (04-Mar-2011) with limited evaluability did not reveal free fluids. The liver biopsy (07-Mar-2011) disclosed severe inflammation reaction, clear eosinophilia, liver cell necroses with bleeding (50% necrotic hepatocytes), moderate cholestasis, and no fatty liver.

Medical assessment: drug-induced toxic liver damage with positive regeneration ability (coded as acute hepatopathy). At date of report the patient was recovering, could be discharged and will be further monitored.

Alternative explanations such as alcohol use, drug consumption and a stay abroad were denied. A viral cause of hepatitis could be excluded.

Reporter causality: possible for ibuprofen and lisinopril, unlikely for tramadol hydrochloride and dydrogesterone/estradiol, probable for carbamazepine

Abbott Arzneimittel GmbH, 13.05.2011 DE-SOLVAY-00311002550

Sender diagnosis: not reported

Narrative

This case was provided by the German Health Authority (Bundesanstalt fuer Arzneimittel und Medizinprodukte, BFARM).

A physician report concerning a 50-year-old adult female patient who experienced acute hepatopathy while being treated with two suspect drugs: FEMOSTON and Carbamazepin.

FEMOSTON (unknown daily dose) was started on an unknown date in 1999 for menopause. Carbamazepin (unknown daily dose) was started on an unknown date in mid DEC 2010 for restless leg syndrome.

On an unknown date in JAN 2011, the patient experienced acute hepatopathy. The patient was admitted for investigation of painful jaundice and acute hepatitis. Since the symptoms started the Carbamazepin the patient had halved the dose. Since about mid-JAN 2011 there had been a slight discolouration of the skin, since mid-FEB 2011 definite jaundice with nausea, tiredness and discoloured stools. The patient had taken no alcohol, no drugs and had not stayed abroad. On 28 FEB 2011, the patient discontinued Carbamazepin. On 04 MAR 2011, the patient discontinued FEMOSTON, T-Long and Ibuprofen. All medications were stopped and then with Beloc Zok was restarted with Pantozol and Vitamin B duo. On 04 MAR 2011, abdominal ultrasound showed limited assessment possible and no evidence of free fluid. On 07 MAR 2011, biopsy of punch cylinder from the liver showed severe inflammation reaction, definite eosinophilia; liver cell necroses with bleeding. Proportion of necrotic hepatocytes was 50%; moderate cholestasis; no fatty liver. The assessment was appearance of liver damage due to medication, regeneration capability was assessed as good; the damage could date back longer; antibody screening: ANA-HEp: 1:5120 - positive. Liver values improved later so the patient was discharged with a date for a further appointment. The reporter informed that FEMOSTON and Ibuprofen should be avoided in future. The reporter informed that the admission was owing to symptomatic hepatopathy with severe increase in fermenters after taking Carbamazepine. No evidence of a viral origin. At the time of reporting, the patient discontinued FEMOSTON and Carbamazepin and the adverse event was abating.

Outcome: Abating.

The reporter assessed the causal relationship between FEMOSTON and the adverse event as 'unlikely' and with Carbamazepin as 'probable'. Abbott Products (formerly Solvay Pharmaceuticals) judged the case as 'suspect'.

The explanation regarding the causality between FEMOSTON and the adverse event as in the specialist information rarely changes in the liver function, possibly associated with nausea, jaundice, abdominal pain. The explanation regarding the causality between Carbamazepine and the adverse event as in the specialist information jaundice and various forms of hepatitis are described under rare to very rare, mostly occurring in the first months of treatment. Since, according to the history, the liver values were within the normal range before the Carbamazepine therapy (regular checks were carried out) and after stopping they again receded.

The reporter judged the case as 'serious' due to 'prolonged inpatient hospitalization'.

BFARM's comment: Additional information is expected.

***Additional information received via email on 14 APR 2011:

The reporter informed that he was not able to sent anonymized discharge letter etc.

***Additional information received on 05 MAY 2011:

Changed concomitant medications Ibuprofen, T-Long and Lisinopril to suspect, further relevant tests and explanation regarding the causalities were provided.

Previously reported concomitant medications Ibuprofen, T-Long and Lisinopril were changed to suspect.

Ibuprofen (unknown daily dose) was started on an unknown date in 2001 for headache NOS. T-Long (unknown daily dose) was started on an unknown date in 2006 for back pain. Lisinopril (unknown daily dose) was started on an unknown date in 2006 for hypertension.

On 04 MAR 2011, the patient discontinued Ibuprofen, T-Long and Lisinopril. The reporter stated that however Lisinopril could be re-started once liver values have improved.

The patient auto antibodies against MPO was 1.2 U/ml (reference range: 0-5 U/ml), HBs antigen negative, Anti HCV negative, IgG 12.0, IgA 4.28 and IgM: 2.64.

The reporter assessed the causal relationship between the suspect drugs 'Ibuprofen and Lisinopril' and the adverse event as 'possible' and with T-Long as 'unlikely'. Abbott Products (formerly Solvay Pharmaceuticals) judged the case as 'suspect'.

The explanation regarding the causality between Ibuprofen and the adverse event as very rare liver function disorders, liver damage, liver failure and acute hepatitis. The explanation regarding the causality between T-Long and the adverse event as very rare increased transaminases. The explanation regarding the causality between T-Long and the adverse event as very rare cholestatic jaundice or hepatitis, liver failure.

BFARM's comment: Case closed. No further additional information expected.

Generics [UK] Ltd, 18.05.2011 DE-MYLANLABS-2011S1006187

Sender diagnosis: not reported

Narrative

Regulatory authority report received 25 Mar 2011

This case, received from the German health authority, involved a 50 year-old female patient who reportedly experienced hepatopathy (liver disorder)

while receiving Carbamazepine for the treatment of restless leg syndrome.

Medical history included hypertension arterial, headache tension, chronic back pain, renal insufficiency and restless leg syndrome. The patient was menopausal.

Concomitant medications were ibuprofen, beloc zok (metoprolol succinate), t-long (tramadol hydrochloride) and lisinopril.

Non-company suspect drug included femoston (estradiol + dydrogesterone) initiated 1999 for treatment of menopause.

1999: The patient initiated femoton therapy (oral, 1 DF, Q24) for menopause

Dec 2010: The patient initiated Carbamazepine (oral, 200 mg, Q24) because of restless legs syndrome

Jan 2011: The patient experienced hepatopathy - the patient was hospitalised due to a painless jaundice and acute hepatitis, around the middle of the month.

Unknown date: The patient reduced the daily carbamazepine dosage to 100 mg, following the start of the event. Alcohol and Drug use was ruled out, no foreign travel

Feb 2011: The patient had developed light skin discolouration. Jaundice with nausea, fatigue and pale stools.

28 Feb 2011: The patient discontinued carbamazepine therapy.

04 Mar 2011: The patient discontinued femoston therapy. Abdominal ultrasound: limited evaluability, no evidence of free fluid. Liver biopsy

Unknown date: The patient was treated with Beloc zok 95, Pantozol 40, Vit B duo.

07 Mar 2011: Liver punch biopsy: Severe inflammatory response, clear. eosinophilia, liver cell necrosis with hemorrhage. Proportion of necrotic hepatocytes: 50%; moderate cholestasis, no fatty liver; assessment; picture of a drug-toxicity. Liver damage; regenerative capacity is considered good; injury might be older.

Antibody screening: ANA-Hep: 1:5120 -> pos. Liver function tests improved in the course so that the patient is discharged with an upcoming consultation. Femoston and ibuprofen should be avoided in future.

Outcome for event was provided as recovering.

No further information was provided.

Follow up information received on 06 May 2011 from BFARM

Additional information received on antibody screening: positive Ab against MPO 1.2 U/ml (Normal range: 0-5 U/ml), HBs-Antigen neg., Anti-HCV neg., IgG: 12,0, IgA: 4,28, IgM: 2,64.

Lisinopril can be reintroduced after improvement in liver enzymes.

Ibuprofen, lisinopril and t-long reclassified from concomitant to co-suspect.

Case is closed.

Generics [UK] Ltd, 18.05.2011 DE-MYLANLABS-2011S1006187

Sender diagnosis: not reported

Narrative

Regulatory authority report received 25 Mar 2011

This case, received from the German health authority, involved a 50 year-old female patient who reportedly experienced hepatopathy (liver disorder) while receiving Carbamazepine for the treatment of restless leg syndrome.

Medical history included hypertension arterial, headache tension, chronic back pain, renal insufficiency and restless leg syndrome. The patient was menopausal.

Concomitant medications were ibuprofen, beloc zok (metoprolol succinate), t-long (tramadol hydrochloride) and lisinopril.

Non-company suspect drug included femoston (estradiol + dydrogesterone) initiated 1999 for treatment of menopause.

1999: The patient initiated femoton therapy (oral, 1 DF, Q24) for menopause

Dec 2010: The patient initiated Carbamazepine (oral, 200 mg, Q24) because of restless legs syndrome

Jan 2011: The patient experienced hepatopathy - the patient was hospitalised due to a painless jaundice and acute hepatitis, around the middle of the month.

Unknown date: The patient reduced the daily carbamazepine dosage to 100 mg, following the start of the event. Alcohol and Drug use was ruled out, no foreign travel

Feb 2011: The patient had developed light skin discolouration. Jaundice with nausea, fatigue and pale stools.

28 Feb 2011: The patient discontinued carbamazepine therapy.

04 Mar 2011: The patient discontinued femoston therapy. Abdominal ultrasound: limited evaluability, no evidence of free fluid. Liver biopsy

Unknown date: The patient was treated with Beloc zok 95, Pantozol 40, Vit B duo.

07 Mar 2011: Liver punch biopsy: Severe inflammatory response, clear. eosinophilia, liver cell necrosis with hemorrhage. Proportion of necrotic hepatocytes: 50%; moderate cholestasis, no fatty liver; assessment; picture of a drug-toxicity. Liver damage; regenerative capacity is considered good; injury might be older.

Antibody screening: ANA-Hep: 1:5120 -> pos. Liver function tests improved in the course so that the patient is discharged with an upcoming consultation. Femoston and ibuprofen should be avoided in future.

Outcome for event was provided as recovering.

No further information was provided.

Follow up information received on 06 May 2011 from BFARM

Additional information received on antibody screening: positive Ab against MPO 1.2 U/ml (Normal range: 0-5 U/ml), HBs-Antigen neg., Anti-HCV neg., IgG: 12,0, IgA: 4,28, IgM: 2,64.

Lisinopril can be reintroduced after improvement in liver enzymes.

Ibuprofen, lisinopril and t-long reclassified from concomitant to co-suspect.

Case is closed.

NOVARTIS, 19.05.2011 DE-002147023-PHHY2011DE25418

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE25418 is a PMS report from a regional pharmacovigilance center (intensive monitoring project) via German Health Authority (DE-BfArM-11049060) received on 24 Mar 2011 with follow-up PMS report from a regional pharmacovigilance center (intensive monitoring project) via German Health Authority (DE-BfArM-11049060) on 05 May 2011. This report refers to a 50-year-old female patient. This patient had medical condition of hypertension arterial, headache tension, chronic back pain and renal insufficiency. This polymedicated patient was treated with carbamazepine (unknown manufacturer) for restless leg syndrome at a daily dosage of 200 mg since Dec 2010 and Femoston (estradiol plus dydrogesterone) for menopause at a daily dosage of 1x10 mg since 1999. The patient also received Ibuprofen (ibuprofen) for headache since 2001, T-long (tramadol hydrochloride) 100 mg for back pain since 2006 and Lisinopril (lisinopril) 5 mg for hypertension since 2006. It was reported that on an unspecified date, the patient was admitted for diagnostic evaluation of painless jaundice and acute hepatitis. The patient experienced slight skin discoloration since about mid Jan, from mid Feb onwards pronounced jaundice with nausea, fatigue and discoloured stools. When in mid of Jan 2011 the first symptoms occurred (skin discoloration), 50 % reduction of the current dose of carbamazepine was undertaken. Blood test done on 04 Mar 2011 showing increased alanine aminotransferase 28.4 umol/l's (normal value 0 to 0.56), increased aspartate aminotransferase 14.6 umol/l's (normal value 0 to 0.54), increased ammonia 44 umol/l's (normal value 11.5 to 38), increased bilirubin total 575 umol/l's (normal value 0 to 21) and increased gamma GT 4.53 umol/l's (normal value 0 to 0.63). Abdominal sonography done on 4 Mar 2011 restricted examination conditions, no evidence of free fluid. On 04 Mar 2011 treatment with Ibuprofen, T-long and lisinopril were discontinued. Liver biopsy on 7 Mar 2011: from liver punch biopsy cylinder, severe inflammatory reaction, marked eosinophilia, liver cell necroses with bleeding. Proportion of necrotic hepatocytes: 50 %; moderate cholestasis, no fatty liver, assessment: pattern of drug-toxic liver damage. Regenerability was estimated as good, damage could be long-standing, antibody screening: antinuclear antibodies (ANA) Hep: 1:5120 - positive. Antibodies against MPO 1.2 U/ml (normal range 0-5 /ml), hepatitis B surface antigen negative, Anti HCV negative, IgG 12.0, IgA 4.28, IgM 2.64. Liver parameter values improved during the course, allowing the patient to be discharged with an appointment for re-attendance. Admission to hospital with symptomatic hepatopathy and a marked increase in enzymes following carbamazepine medication. No evidence of a viral pathogenesis. The patient was treated with pantozol 40 mg daily and vitamin B duo (BID). The outcome of adverse events was reported as recovering. It was stated that hepatopathy might had happened long time before. Causality was assessed as probable for (carbamazepine), unlikely for (femoston), possible for Ibuprofen, unlikely for T-Long, and possible for Lisinopril by the reporter.

Follow-up PMS report received on 05 May 2011. Lab data, co-suspect drugs information and causality with the co-suspect drugs was updated.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11049925

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	Yes	No	No		Yes

Patient

Initials:	Sex: Male	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Joint pain	2002				Unknown
2 Disorder liver	2002				Unknown
3 Spleen disorder	2002				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac			-	indication
2 ibuprofen ct 600		/	2001-	Gout
ibuprofen			-	
3 telfast	Unknown	/	-	Drug use for unknown indication
fexofenadine hydrochloride			-	indication

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 31.03.2011 DE-SANOFI-AVENTIS-2011SA018379

Sender diagnosis: not reported

Narrative

Initial report:

On 28-Mar-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11049925); documenting a case, in which a consumer of unknown age had reported about himself to the Drug Committee of the German Physicians Association. Relevant medical history, concomitant disease(s) and drug(s) were not provided.

For unspecified indication the patient took diclofenac and fexofenadine hydrochloride (Telfast) during an unspecified time period. Ibuprofen (Company: CT) 600 mg for gout was received in 2001. Twelve months later (in 2002) joint pain, liver and spleen disorder were detected. No further information was received.

The seriousness criterion was specified with disability.

A causal assessment was not provided.

performed: 31-MAR-2011, Sanofi-Aventis Germany

This case report includes clinical diagnoses, which are considered locally unlabelled for one (or more) suspected product(s). The medical evaluation and the facts presented here however are not very suggestive of a causal relationship (please refer to the international company comment), and thus do not imply a change in the state of scientific knowledge for the suspected product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

NOVARTIS, 08.04.2011 DE-002147023-PHHY2011DE25017

Sender diagnosis: not reported

Narrative

18.03.2013

BfArM - Pharmacovigilance

372 / 511

Case number PHHY2011DE25017, is an initial spontaneous report from a consumer through the Drug Commission of the German Medical Association of Physicians via Health Authority (BfArM- Ref no DE-BFARM-11049925) on 25 Mar 2011: This report refers to a male patient (age not specified). This patient received diclofenac (manufacturer unknown), Telfast for an unspecified indication from an unknown dates and Ibuprofen 600 for gout from 2011. In 2002, patient experienced joint pain. On an unknown date patient experienced liver disorder and spleen disorder. The outcome and the causality were not reported.

Serious spontaneous report,

- arthralgia, spleen disorder [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

- liver disorder [medically significant], assessed as listed in the core data sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11058012

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19451030	Age: 64 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Transaminases increased	20100721	201008		139 Day	recovered/resolved
2 Nausea	20100721	201008		139 Day	recovered/resolved
3 Bilirubin increased	20100721	201008		139 Day	recovered/resolved
4 Jaundice	20100721	201008		139 Day	recovered/resolved
5 Hepatosis					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUFLAM /00109201/ ibuprofen	Oral	1600 Mg milligram(s)/	20100617-20100725	Back pain
2 KATADOLON S flupirtine maleate	Oral	1 Df dosage form/	20100305-20100725	Back pain
3 NOVAMINSULFON metamizole sodium		40 Gtt drop(s)/	20100719-	Back pain
4 TARGIN Retardtabletten naloxone hydrochloride; oxycodone hydrochloride	Oral	10 Mg milligram(s)/ 212	20100621-20100725	Back pain
5 TARGIN Retardtabletten naloxone hydrochloride; oxycodone hydrochloride	Oral	5 Mg milligram(s)/ 212	20100614-20100620	
			7 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain			Yes	
Herniated nucleus pulposus	201003		Yes	
Osteochondrosis of spine			Yes	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase		27	unk	
Protein		58	unk	
MRI	20100804	See notes	Other	
Hepatitis A		>1:1500	unk	
Heart rate	20100725	87	bpm	
Gamma GT		6.7	unk	
Gamma GT		11.2	unk	
C-reactive protein		13	unk	
Coagulation test		70.9	unk	
Coagulation test		66.4	unk	
Blood pressure	20100725	165/90	mmHg	

	DE-BFARM-11058012	
LDH	6.52	unk
LDH	8.3	unk
Cholesterol	5.3	unk
Bilirubin total	122	unk
Bilirubin total	155	unk
Bilirubin direct	78	unk
Bilirubin direct	101	unk
Aspartate aminotransferase	5.58	unk
Aspartate aminotransferase	18.1	unk
Aspartate aminotransferase	15.9	unk
Alanine aminotransferase	26.1	unk
Alanine aminotransferase	11.23	unk

Narratives (nur bei elektronischem Eingang)

NAPPMUNDI, 25.03.2011 DE-NAPPMUNDI-DEU-2010-0006823

Sender diagnosis: not reported

Narrative

Report reference number DEU-2010-0006823 was received on 09DEZ2010 from a physician in Germany via company representative. This spontaneous report relates to a 65 year-old female patient.

The patient current conditions included pain. The patient received treatment with Targin 10/5 mg (oxycodone/naloxone, twice daily) for her pain. Additional medication included Katadolon (flupirtine maleate) and ibuprofen. The patient experienced "toxic hepatitis" (no details on laboratory or other findings provided). Before the patient started the pain treatment she had normal laboratory values. Measures concerning the adverse events and its outcome were not reported.

The reporting physician gave no detailed causality assessment concerning the adverse events and treatment with Targin.

This case was rated to be non-serious.

*** Follow-up information was received on 22MAR2011 from the initially reporting physician and additional information in form of a hospital discharge summary.

The patient's current conditions included lumbo-radicular pain syndrome, osteochondrosis of L5/S1, and nucleus pulposus prolapse of L4/5. Beyond the four suspect products the patient had not received any concomitant medication. The reporter confirmed that the patient received treatment with Targin started at 5/2.5 mg twice daily on 14JUN2010 and increased to 10/5mg twice daily on 21JUN2010. In addition the patient received treatment with Katadolon S long (flupirtine maleate, started on 05MAR2010 at one dosage form daily), Ibufam (ibuprofen, started on 17JUN2010 at 1600mg daily), and Novaminsulfon (metamizole sodium, started on 19JUL2010, up to 40 drops three times daily).

On 21JUL2010 she experienced nausea, skin and scleral jaundice, increased transaminases, and increased bilirubin.

The patient was hospitalised on 25JUL2010. In the hospital discharge summary report a 'toxic hepatitis most probably drug-induced through Katadolon, Ibuprofen and Targin' was provided as diagnosis. Treatment with Targin, Katadolon S long, Ibufam, and Novaminsulfon was discontinued on the same day and the patient was switched to a Durogesic 25 patch product (fentanyl).

The patient's wellbeing already improved whilst she was still hospitalised and the transaminases gradually decreased. An MRI [magnetic resonance imaging] was performed on 04AUG2010 and revealed suspicion of liver cirrhosis due to an apparently reduced liver size with a humpy liver surface and inhomogenous contrast media colouring as well as tiny cysts in the liver. No suspicion of any neoplastic lesions. Ductus hepatocholedochus appeared quite marked (up to 11 mm) without proof of an obstacle hindering bile flow - no contrast agent in the deducing bile tract, potentially due to disturbed excretory liver function. The patient's gall bladder, bile duct, pancreas, spleen, kidneys, and adrenal glands showed no pathological findings, retroperitoneal lymph nodes were not swollen. Hepatitis serology was Hepatitis A positive (over 1:1500).

Saturation of transferrin at rest together with markedly increased ferritin levels were interpreted as a marked inflammatory reaction. Also the tumor marker AFP - reportedly a specific liver marker - was markedly increased (66mcg/l).

The liver cirrhosis suspected based on the MRI findings could not be shown in the liver sonogramm. Potentially the severe hepatitis appears as beginning cirrhotic change. For a final determination a liver puncture would be needed. Because of the marked decrease of the transaminases no liver puncture was conducted.

The patient was treated with Bifiteral (lactulose). Following a further decrease of the transaminases a nucleotomy to treat her orthopedic disease is intended. The patient was discharged from hospital on 07AUG2010 and recovered from her adverse events approximately on 19AUG2010; six months later her liver enzyme values were normal again.

The reporter found the causality between the adverse events and the treatment with Targin to be not assessable as the adverse events could possibly have been caused by combination therapy.

This case was assessed as serious because of the involved hospitalisation and the life-threatening nature of the adverse event.

Additional information has been requested. The case will be updated accordingly should more information become available.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11056649

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No				Yes		

Patient

Initials:	Sex: Male	Birth: 20081007	Age: 16 Month	weight (kg): 8.2	BMI_int: 13.83	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Chills	20100209		2 Hour	1 Day	recovered/resolved
2 Fever	20100209		2 Hour	1 Day	recovered/resolved
3 Tachycardia	20100209		2 Hour	1 Day	recovered/resolved
4 Vomiting	20100209		2 Hour	1 Day	recovered/resolved
5 Glutamic-oxaloacetic transaminase increased	20100209		2 Hour	1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibu-ratiopharm 2% Fiebersaft ibuprofen	Intravenous (not otherwis	2.5 ml millilitre(s)/	20100209-	Fever
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			Unknown	
Acute non-suppurative otitis media			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Glutamic-oxaloacetic transferase		0.92	µmol/l	

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-11082290

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 51 Year	weight (kg): 85	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abnormal liver function tests					recovered/resolved
2 AST increased					recovered/resolved
3 ALT increased					recovered/resolved
4 Bilirubin total increased					recovered/resolved
5 Shoulder operation					recovered/resolved
6 Pain					recovered/resolved
7 Jaundice					recovered/resolved
8 Acute liver failure	2011	20110421			recovered/resolved
9 Liver injury					recovered/resolved
10 Nausea					recovered/resolved
11 Vomiting					recovered/resolved
12 Discolored stools					recovered/resolved
13 Brown urine					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUPROFEN		1200 Mg milligram(s)/Day	201101-201103	Shoulder pain
ibuprofen			60 Day	
2 TRACLEER	Oral	250 Mg milligram(s)/Day	20100121-20110325	Skin ulcer
bosentan		245	14 Month	
3 TRACLEER	Oral	125 Mg milligram(s)/Day	20091217-20100120	
bosentan		245	35 Day	

Drugs (concomitant)

4 CYCLOPHOSPHAMID	/	-
cyclophosphamide		-
5 cymbalta	/	-
duloxetine hydrochloride		-
6 DOMPERIDON	/	-
domperidone		-
7 OMEPRAZOL	/	-
omeprazole		-
8 PREDNISOLON	/	-
prednisolone		-
9 VERAPAMIL	/	-
verapamil		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Systemic sclerosis			Unknown	Limited
Skin ulcer			Unknown	Digital ulcer associated with limited systemic sclerosis
CREST syndrome			Unknown	
Lung fibrosis			k.A.	

Primary source

Qualification: Physician

observestudytype: Other studies

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alk phos	20101118	73	U/L	
Alk phos	20110325	223	U/L	
Alk phos	20110421	94	U/L	
Alk phos	20110530	107	U/L	
Asp transferase	20101118	30	U/L	
Asp transferase	20110421	78	U/L	
Asp transferase	20110530	35	U/L	
Bilirubin total	20110530	1.0	mg/dL	
Bilirubin total	20101118	0.5	mg/dL	
Bilirubin total	20110421	3.6	mg/dL	
Glutamic-pyruvate transaminase	20110530	29	U/L	
INR	20101118			
INR	20110325			
INR	20110530			
Pro-thrombin time	20101118	30	s	
Pro-thrombin time	20110325	45	s	
Pro-thrombin time	20110530	28	s	

Narratives (nur bei elektronischem Eingang)ACTELION, 09.05.2011 DE-ACTELION-A-CH2011-48079

Sender diagnosis: not reported

Narrative

Case initial reception date: 27-APR-2011

This solicited Health Care Professional report received from Tracleer (bosentan) post-marketing surveillance program (DUO Registry) concerns a 51-year-old female patient (# 0114-02198) with digital ulcer associated with limited systemic sclerosis who was hospitalized due to acute liver failure. The patient had a medical history of CREST syndrome. Concomitant medication included prednisolone, verapamil, omeprazole, domperidone, duloxetine, cyclophosphamide, and ibuprofen which was also considered as co-suspect medication. Tracleer was started at 62.5 mg b.i.d. on 17Dec09, was uptitrated to 125 mg b.i.d. on 21Jan10, and was discontinued on 25Mar11.

In 2011, the patient experienced acute liver failure. On 25Mar11, she was admitted to hospital and LFTs showed its highest values ALT 35.7xULN (1070 U/L, ULN 30 assumed), AST 38.2 U/L, ULN 1147 U/L, ULN 30 assumed), total bilirubin 14.9xULN (14.9 mg/dL, ULN 1.0 assumed) and direct bilirubin 35.3xULN (10.6 mg/dL, ULN 0.3 assumed). AP was not provided. All medications including Tracleer were temporarily interrupted due to event. Prior to event, LFTs were normal (18Nov10). Further, the patient underwent shoulder operation on 07Jan11 and received ibuprofen 400 mg b.i.d. from Jan11 to mid Mar11 against pain. On 21Apr11, the event resolved without sequelae. As of 27Apr11 (date of reporting), Tracleer restart date was not provided. The reporter assessed the event as not related to Tracleer and assumed that ibuprofen was causing acute liver failure.

Note: The causality assessment of the event acute liver failure was upgraded to related by Actelion.

ACTELION, 15.09.2011 DE-ACTELION-A-CH2011-48079

Sender diagnosis: not reported

Narrative

Case initial reception date: 27-APR-2011

This solicited Health Care Professional report received from Tracleer (bosentan) post-marketing surveillance program (DUO Registry) concerns a 51-year-old female patient (# 0114-02198) with digital ulcer associated with limited systemic sclerosis who was hospitalized due to acute liver failure. The patient had a medical history of CREST syndrome. Concomitant medication included prednisolone, verapamil, omeprazole, domperidone, duloxetine, cyclophosphamide, and ibuprofen which was also considered as co-suspect medication. Tracleer was started at 62.5 mg b.i.d. on 17Dec09, was uptitrated to 125 mg b.i.d. on 21Jan10, and was discontinued on 25Mar11.

The patient underwent shoulder operation on 07Jan11 and received ibuprofen 400 mg b.i.d. from Jan11 to mid Mar11 against pain. On 25Mar11, she

was admitted to hospital and LFTs showed its highest values ALT 35.7xULN (1070 U/L, ULN 30 assumed), AST 38.2 xULN (1147 U/L, ULN 30 assumed), total bilirubin 14.9xULN (14.9 mg/dL, ULN 1.0 assumed) and direct bilirubin 35.3xULN (10.6 mg/dL, ULN 0.3 assumed). AP was not provided. All medications including Tracleer and ibuprofen were temporarily interrupted on 25Mar11 due to event. Prior to event, LFTs were normal (18Nov10). On 21Apr11, the event resolved without sequelae. As of 27Apr11 (date of reporting), Tracleer restart date was not provided. The reporter assessed the event as not related to Tracleer and assumed that ibuprofen led to acute liver failure.

Note: The causality assessment of the event acute liver failure was upgraded to related by Actelion.

Follow-up information received on 30Aug11 and on 06Sep11:

During the event, the patient experienced clinical signs of liver injury with jaundice, discoloured stool, brown urine, nausea and vomiting. On hospital admission, she experienced sickness and fatigue. On 18Nov10, prior to event, laboratory values showed AP and total bilirubin within normal range, Prothrombin time was 30s (reference range 11.1-13.1 assumed) and INR 1.0 (reference range 0.85-1.15 assumed). On 25Mar11, at the time of event, AP was 1.8xULN (223U/L, ULN 120 assumed), prothrombin time was 45 sec and INR 1.6. On 21Apr11, AP was within normal range. On 30May11, last laboratory values provided showed AST 1.2xULN (35U/L, ULN 30 assumed), ALT, AP, total bilirubin and INR were within normal range. Prothrombin time was 28 sec.

Note: This case became re-expedible upon receipt of significant follow-up information received on 06Sep11.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11094624

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital Yes	Other Yes	Pregnancy YES	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: 20090829	Age:	weight (kg): 1.995	BMI_int: 10.79	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Premature baby	20090829				recovered/resolved
2 Intrauterine growth retardation					recovered/resolved
3 Drug exposure during pregnancy					Unknown
4 Ductus arteriosus stenosis fetal					recovered/resolved
5 Drug exposure in utero	20090829				recovered/resolved
6 Hyperbilirubinaemia neonatal	20090829				recovered/resolved
7 Small for gestational age	20090829				recovered/resolved
8 Testicle undescended	20090829				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 decortin-h prednisolone	Transplacental	/	- 246 Day	Arthritis rheumatoid
2 DECORTIN-H prednisolone	Transplacental	30 Mg milligram(s)/Day	-	Rheumatoid arthritis
3 DECORTIN-H prednisolone	Transplacental	5 Mg milligram(s)/Day	-	
4 DECORTIN-H prednisolone	Transplacental	10 Mg milligram(s)/Day	-	
5 DECORTIN-H prednisolone	Transplacental	15 Mg milligram(s)/Day	-	
6 DECORTIN-H prednisolone	Transplacental	20 Mg milligram(s)/Day	-	
7 DECORTIN-H prednisolone	Transplacental	15 Mg milligram(s)/Day	-	
8 ibuprofen ibuprofen	Transplacental	/	- 43 Day	Arthritis rheumatoid
9 voltaren resinat diclofenac resinate	Transplacental	/	- 190 Day	Arthritis rheumatoid

Drugs (concomitant)

10 FOLSÄURE folinic acid	Transplacental	/	20081115-20090815 9 Month	Prophylaxis of neural tube defect
11 humira adalimumab	Transplacental	/	-	Arthritis rheumatoid
12 paracetamol paracetamol	Transplacental	/	- 57 Day	Arthritis rheumatoid

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

BBGes, 24.05.2011 DE-EMBRYOTOX-20094140-B

Sender diagnosis: not reported

Narrative

The preterm baby boy was born in gw 35+1 and was small for gestational age. He needed 24 hours of phototherapy because of hyperbilirubinemia. Otherwise he was healthy, only the testicles were not descended. If they were descended in the later controls is not noticed.

Sender's comment:

Premature birth and growth retardation are described with cortisone intake. They could also be due to the underlying disease of the mother. Hyperbilirubinemia is a common condition with preterm babies, a causality with prednisolon can not be ruled out. Non descensus of testicles is typical in preterm children, a causality to ibuprofen and diclofenac is not very likely but can not be ruled out. Newer studies with causality to paracetamol only saw correlation when paracetamol was taken in the first or second trimester. the patient took paracetamol only in the third trimester therefore causality is unlikely.

Reporter's comment:

sanofi-aventis, 31.05.2011 DE-SANOFI-AVENTIS-2011SA033122

Sender diagnosis: not reported

Narrative

Initial report received on 26-May-2011:

The case was extracted from the webpage of the German health authority (authority no: DE-BFARM-11094624); initially reported from a German physician, provided via Embryotox (id. no: DE-EMBRYOTOX-20094140-B).

The case involves a male neonate whose mother was treated with ibuprofen (no trade name provided) 600 mg daily for 43 days from the 16th to 22nd week of gestation for rheumatoid arthritis. Additional therapy for rheumatoid arthritis included diclofenac resinate 140 mg twice daily from the beginning of the pregnancy until 27th week as well as prednisolone as following: 5 mg daily from the beginning of the pregnancy until 14th week of gestation, 10 mg daily from 14th week of gestation until 16th week of gestation, 15 mg daily from 16th week of gestation until 20th week of gestation, 20 mg daily from 20th week of gestation until 22nd week of gestation and 15 mg daily from 27th week of gestation until 35th week of gestation. In the 22nd week of gestation oligohydramnios was diagnosed (see associated case id: 2011SA032848), this recovered and was normally in 34th week of gestation. In the 27th week of gestation constriction of ductus arteriosus was diagnosed, which improved in the 29th + 4 week of gestation and was recovered in the 34th +1 week of gestation. The child was controlled and intrauterine growth restriction was noticed. The preterm baby boy was born in the 35th + 1 week of gestation, after premature rupture of membranes. He was small for gestational age. He needed 24 hours of phototherapy because of hyperbilirubinemia. Otherwise he was healthy, only the testicles were not descended. If they were descended in later controls was not noticed. Concomitantly she was treated with paracetamol and folic acid. Adalimumab was taken 3 month prior to conception. Further information including significant past and present medical history was not provided.

Causality Assessment according Drug Embryotox (method: Global Introspection (WHO GI)): possible for testicle undescended; unlikely for premature baby 33 to 36 weeks, small for gestational age and hyperbilirubinaemia neonatal.

NOVARTIS, 03.06.2011 DE-002147023-CHNY2011DE00451

Sender diagnosis: not reported

Narrative

Initial spontaneous case report received on 25 May 2011 from BBGes (Regional Pharmacovigilance Center, Department of Embryonaltoxikologie) via BfArM (German Health Authority, ref number DE-BfArM-11094624). This retrospective pregnancy case described the occurrence of ductus arteriosus stenosis fetal, cryptorchism, intrauterine growth retardation, hyperbilirubinaemia neonatal and premature baby in a neonate exposed to different drugs in utero. The preterm baby boy was born in gestational week 35+1. He needed 24 hours of phototherapy because of hyperbilirubinaemia. Otherwise, he was healthy, only the testicles were not descended. Suspected drugs (taken by patient's mother during pregnancy) included: Decortin-H (prednisolone, different dosages, from 0 to 35 gestational week, total 246 days), Voltaren resinate (diclofenac resinate, from 0 to 27 gestational week, total 190 days), and ibuprofen (unknown manufacturer, taken from 16 to 22 gestational week). Concomitant medication included: paracetamol (from 27 to 35 gestational week, total 57 days), Folsaure (folic acid, from 0 to 33.5 gestational week, for prophylaxis of neural tube defect). Decortin-H, Voltaren resinate, ibuprofen, and paracetamol were taken for arthritis rheumatoid. The mother's patient took Humira (adalimumab), 40 mg (frequency unknown) in preconceptional period, for arthritis rheumatoid. The outcome of the events was reported as follows: recovery after removal of diclofenac for ductus arteriosus stenosis fetal; unknown for cryptorchism; recovered for foetal growth restriction; resolved for premature baby and recovered for hyperbilirubinaemia neonatal. The reporter commented: Premature birth and growth retardation are described with cortisone intake. They could also be due to the underlying disease of the mother (arthritis rheumatoid). Hyperbilirubinaemia is a common condition with preterm babies, a causality with

prednisolone can not be ruled out. Non descensus of testicles is typical in preterm children, a causality to ibuprofen and diclofenac is not very likely but can not be ruled out. Newer studies with causality to paracetamol only saw correlation when paracetamol was taken in the first or second trimester. The mother's patient took paracetamol only in the third trimester therefore causality is unlikely. Linked mother case CHNY2011DE00450.

Reporter's comment:

MENARINIP, 08.06.2011 DE-MEN-22182

Sender diagnosis: not reported

Narrative

This case, initially reported by a regional pharmacovigilance center (ID: DE-EMBRYOTOX-20094140-B) and then published by BfArM as an ICH-E2B-compliant ICSR on its website, involved a boy who was born in 35th+1 week of pregnancy as a premature baby with neonatal hyperbilirubinaemia and undescended testicle. Further, the newborn was small for gestational age.

The mother had a medical history of rheumatoid arthritis for which she had been on treatment with 600 mg/day ibuprofen from 16th to 22th gestation week (gw), 140 mg/twice daily diclofenac resinate (Voltaren resinat) from 0th to 27th week of pregnancy and prednisolone (Decortin H), which was taken during the entire pregnancy (gw 0-14: 5 mg/day, gw 14-16: 10 mg/day, gw 16-20: 15 mg/day, gw 20-22: 20 mg/day, gw 27-35: 15 mg/day).

The mother's concomitant medication had included paracetamol between 1500 mg/die and 3000 mg/die from 27th to 35th gw for rheumatoid arthritis and folic acid for prophylaxis of neural tube defect between 15-Nov-2008 and 15-Aug-2009. Preconceptional she had received 40 mg/day adalimumab (Humira) for rheumatoid arthritis.

The boy who was born in 35th+1 week of pregnancy as a premature baby, small for gestational age, with neonatal hyperbilirubinaemia and undescended testicle. The boy needed 24 hours of phototherapy because of hyperbilirubinaemia. Outcome of undescended testicle was unknown, otherwise the baby was healthy.

The reporter assessed the events to be possibly related to prednisolone. Only the event undescended testicle was considered as possibly related to ibuprofen and diclofenac resinate (Voltaren resinat), the other events were assessed to be unlikely related to these drugs.

The case of the mother was described in case report DE-MEN-39647.

NOVARTIS, 09.06.2011 DE-002147023-PHHY2011DE45641

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE45641 is an initial PMS report received from a regional pharmacovigilance center (intensive monitoring project) via Health Authority (DE-EMBRYOTOX-20094140-B) on 25 May 2011. This report refers to a male (baby) patient. Baby's mother had current condition of rheumatoid arthritis (See the case: PHHY2011DE45626). This poly-medicated baby's mother received Voltaren resinat (diclofenac resinate) at a dose of 280 mg per day for rheumatoid arthritis. She also received Decortin-h (prednisolone) at a dose of 30 mg per day, gestation weeks (gw): 0-14, 5 mg per day, gw: 14-16, 10 mg per day, gw: 16-20, 15 mg per day, gw: 20-22, 20 mg per day, gw: 27-35 and 15 mg per day for rheumatoid arthritis and Ibuprofen at a dose of 600 mg per day for rheumatoid arthritis. She took prednisolone in rising dosages in the entire pregnancy. The patient took from gestation weeks (gw) 16-22 ibuprofen, from gw 0-27 diclofenac and from gw 27-35 paracetamol. In gw 27 constriction of ductus arteriosus was diagnosed, improvement in gw 29+4, normal in gw 34+1 after therapy with diclofenac was stopped in gw 27. The child was controlled and intrauterine growth restriction was noticed. The preterm baby was born in gw 35+1 and was small for gestational age. He needed 24 hours of phototherapy because of hyperbilirubinaemia. He was healthy only the testicles were not descended. The outcome was reported as recovered for the events premature baby, small for gestational age, hyperbilirubinaemia neonatal, foetal growth retardation, ductus arteriosus stenosis foetal and the outcome was not reported for testicle undescended. The causality was reported as not suspected for the events premature baby, small for gestational age, hyperbilirubinaemia neonatal and the causality was reported as suspected for testicle undescended, constriction of ductus arteriosus and foetal growth retardation.

Reporter's comment:

MENARINIP, 09.06.2011 DE-MEN-22182

Sender diagnosis: not reported

Narrative

This case, initially reported by a regional pharmacovigilance center (ID: DE-EMBRYOTOX-20094140-B) and then published by BfArM as an ICH-E2B-compliant ICSR on its website, involved a boy who was born in 35th+1 week of pregnancy as a premature baby with neonatal hyperbilirubinaemia and undescended testicle. Further, the newborn was small for gestational age.

The mother had a medical history of rheumatoid arthritis for which she had been on treatment with 600 mg/day ibuprofen from 16th to 22th gestation week (gw), 140 mg/twice daily diclofenac resinate (Voltaren resinat) from 0th to 27th week of pregnancy and prednisolone (Decortin H), which was taken during the entire pregnancy (gw 0-14: 5 mg/day, gw 14-16: 10 mg/day, gw 16-20: 15 mg/day, gw 20-22: 20 mg/day, gw 27-35: 15 mg/day).

The mother's concomitant medication had included paracetamol between 1500 mg/die and 3000 mg/die from 27th to 35th gw for rheumatoid arthritis and folic acid for prophylaxis of neural tube defect between 15-Nov-2008 and 15-Aug-2009. Preconceptional she had received 40 mg/day adalimumab (Humira) for rheumatoid arthritis.

The boy who was born in 35th+1 week of pregnancy as a premature baby, small for gestational age, with neonatal hyperbilirubinaemia and undescended testicle. The boy needed 24 hours of phototherapy because of hyperbilirubinaemia. Outcome of undescended testicle was unknown, otherwise the baby was healthy.

The reporter assessed the events to be possibly related to prednisolone. Only the event undescended testicle was considered as possibly related to ibuprofen and diclofenac resinate (Voltaren resinat), the other events were assessed to be unlikely related to these drugs.

The case of the mother was described in case report DE-MEN-22181.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11096431

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19620902	Age: 48 Year	weight (kg): 90	BMI_int: 27.78	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Appetite disorder	20101108			6 Day	not recovered/not resolved
2 Nausea	20101108			6 Day	not recovered/not resolved
3 Toxic acute liver disease	20101108			6 Day	not recovered/not resolved
4 Discoloration urine	20101105			3 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 azithromycin	Oral	500 Mg milligram(s)/Day	20101102-20101104	Acute upper respiratory tract infection
azithromycin			3 Day	
2 azithromycin	Oral	500 Mg milligram(s)/Day	20101102-20101104	Otitis
azithromycin			3 Day	
3 ibuprofen	Oral	2400 Mg milligram(s)/Day	20101103-20101103	Drug use for unknown indication
ibuprofen			5 Day	
4 ibuprofen	Oral	600 Mg milligram(s)/Day	20101101-20101101	Drug use for unknown indication
ibuprofen			5 Day	
5 ibuprofen	Oral	1800 Mg milligram(s)/Day	20101102-20101102	Drug use for unknown indication
ibuprofen			5 Day	
6 ibuprofen	Oral	1800 Mg milligram(s)/Day	20101104-20101104	Drug use for unknown indication
ibuprofen			5 Day	
7 ibuprofen	Oral	600 Mg milligram(s)/Day	20101105-20101105	Drug use for unknown indication
ibuprofen			5 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fever			Unknown	
Acute upper respiratory tract infection			Unknown	
Hypertension			Yes	
Mitral valve prolapse			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin	20101111	69.1	UNK	
Bilirubin	20101115	50.6	UNK	
Bilirubin	20101122	19.7	UNK	
Bilirubin	20101109	59	UNK	
Gamma GT	20101108	2.96	UNK	
Gamma GT	20101115	2.20	UNK	
Gamma GT	20101111	2.94	UNK	
Gamma GT	20101109	2.84	UNK	

Gamma GT	20101122	1.06	UNK
SGPT	20101122	2.15	UNK
SGPT	20101115	5.82	UNK
SGPT	20101109	3.71	UNK
SGPT	20101108	3.33	UNK
SGPT	20101111	3.33	UNK

Narratives (nur bei elektronischem Eingang)sanofi-aventis, 31.05.2011 DE-SANOFI-AVENTIS-2011SA033835

Sender diagnosis: not reported

Narrative**Initial Report:**

This case was extracted from the website of the German Health Authority (authority number DE-BFARM-11096431) on 30-May-2011, initially reported by an anonymized physician via the Drug Commission of the German Medical Association (case no.: DE-DCGMA-10154651) to the BfArM.

The case involves a 48-year-old female patient suffering from hypertension. The patient's medical history included mitral valve prolapse, acute upper respiratory tract infection and fever. On 02-Nov-2010, a therapy with azithromycin (no trade name provided) at a daily dose of 500 mg was started for otitis. Also, a therapy with ibuprofen at a dose of 600 mg for an unspecified indication was initiated on 01-Nov-2010. The report form indicates 600 mg ibuprofen on 1 Nov 2011, 1800 mg on 2 Nov 2010 and 2400 mg on 3 Nov 2010. Therapy with azithromycin was discontinued on 4 Nov 2010, ibuprofen was discontinued on 5 Nov 2010. On 05-Nov-2010, the patient noticed dark urine. On 8 Nov 2010, the patient experienced acute toxic liver disease, nausea and appetite disorder. The events were considered life threatening. On the date of the report, the patient had not recovered. The Drug Commission assessed the causal relationship between the events and both suspect drugs according to Global Introspection (WHO GI) as possible.

No further information has been provided so far on corrective treatment.

sanofi-aventis, 17.08.2011 DE-SANOFI-AVENTIS-2011SA033835

Sender diagnosis: not reported

Narrative**Initial Report:**

This case was extracted from the website of the German Health Authority (authority number DE-BFARM-11096431) on 30-May-2011, initially reported by an anonymized physician via the Drug Commission of the German Medical Association (case no.: DE-DCGMA-10154651) to the German health authority.

The case involves a 48-year-old female patient suffering from hypertension. The patient's medical history included mitral valve prolapse. On 02-Nov-2010, the therapy with azithromycin (no trade name provided) 500 mg/day was started for otitis and acute upper respiratory tract infection (with high fever). One day before, ibuprofen 600 mg (no trade name provided) was commenced, and taken on the consecutive days as follows: one tablet, three tablets, four tablets, three tablets and one tablet, indication was not explicitly specified.

On 05-Nov-2010, the patient noticed dark urine and suffered from nausea and appetite disorder; azithromycin was not taken any longer. Laboratory values on 08-NOV-2011 revealed: bilirubin 69.1 (max.), GPT 3.33 (max. 5.82 on 15-Nov-2011) and gamma GT 2.96 (max.), no units were provided. An acute toxic liver disease was diagnosed (onset date documented as 08-NOV-2010) and considered to have been life-threatening. No corrective treatment was provided. Although the laboratory values on 22-NOV-2010 were improving (bilirubin 19.7, GPT 2.15 and gamma GT 1.06) the event outcome was described as "not recovered".

The Drug Commission assessed the causal relationship between the events and both suspect drugs according to Global Introspection (WHO GI) as possible.

Additional information received on 11-Aug-2011:

The original document for the initial data collection was sent by the Drug Commission.

Patient's birthday and initials, laboratory values were amended. The concomitant disease (acute upper respiratory tract infection) was changed to indication for the first suspected product. The events like dark urine, nausea and appetite disorder were reclassified to symptoms of toxic acute liver disease.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11105672

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes Yes Yes Yes

Patient

Initials: Sex: Male Birth: 19651204 Age: 45 Year weight (kg): 105 BMI_int: 31.7 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20110225			5 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuhexal	Oral	1200 Mg milligram(s)/Day	20110221-20110228	
ibuprofen		82	8 Day	
2 Pantozol, 40 mg, Tabl (mr), Nycomed	Oral	40 Mg milligram(s)/Day	20110221-20110303	
pantoprazole		94	11 Day	
3 Zyprexa, 5/10 mg, Tabl, Lilly	Oral	/Day	20110209-20110303	
olanzapine		9	23 Day	

Drugs (concomitant)

4 acc akut	Oral	1 Df dosage form/Day	20110219-20110221	
acetylcysteine		59	3 Day	
5 Orfiril long/Valproat, 300 mg, Retardkps/Retard	Oral	/Day	20110124-20110303	
valproic acid			39 Day	
6 Seroquel, 25/100 mg, Filmtabl, Astra Zeneca	Oral	/Day	20110117-20110304	
quetiapine		82	47 Day	
7 Tavor Expidet, 1 mg, Täfelchen, Pfizer	Oral	/Day	20110124-20110223	
lorazepam		285	31 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Smoker	1985		Yes	15 cigarettes daily.
Bipolar affective disorder	1995		Yes	
Alcohol use			Yes	
Depressive episode			k.A.	

Primary source

Qualification: Physician observestudytype:
 Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase normal	20110218	41	U/l	
Alanine aminotransferase increased	20110225	614	U/l	
Alanine aminotransferase increased	20110304	1158	U/l	
Alanine aminotransferase increased	20110318	190	U/l	
Aspartate aminotransferase normal	20110218	40	U/l	
Aspartate aminotransferase increased	20110225	379	U/l	
Aspartate aminotransferase increased	20110304	765	U/l	
Aspartate aminotransferase increased	20110318	69	U/l	
Alkaline phosphatase normal	20110228	91	U/l	

		DE-BFARM-11105672	
Alkaline phosphatase increased	20110308	176	U/l
Alkaline phosphatase normal	20110318	110	U/l
Ultrasonography		#	NA

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 07.06.2011 DE-FAKOS-170396

Sender diagnosis: not reported

Narrative

Sender's comment:

Initial report, more info follows.

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 14.07.2011 DE-FAKOS-170396

Sender diagnosis: not reported

Narrative

Am 29.11.2010 erfolgte die stationäre Aufnahme des Patienten in eine psychiatrische Klinik zur Behandlung einer depressiven Episode. Am 25.02.2011 traten erstmalig erhöhte Transaminasen auf (ALAT 614 U/l, Normbereich: <45 U/l; ASAT 379 U/l, Normbereich: <50 U/l), die nach weiterem Ansteigen (am 04.03.2011 ALAT 1158 U/l, ASAT 765 U/l) sich ab dem 05.03.2011 wieder langsam besserten. Am 18.03.2011, drei Tage vor Entlassung des Patienten am 21.03.2011, betrug ASAT 190 U/l und ALAT 69 U/l.
In engerem zeitlichem Zusammenhang steht die Einnahme von Pantozol Tabl. (Einnahmezeitraum: 21.02.-03.03.2011), Ibuhexal Filmtabl. (Einnahmezeitraum: 21.-28.02.2011) sowie Zyprexa Tabl. (Einnahmezeitraum: 09.02.-03.03.2011) (medikamenteninduzierte Genese wahrscheinlich). Nach Absetzen dieser Arzneimittel trat die Besserung der Transaminasen ein. In der Sonographie wurde eine nur geringe diffuse Steatosis hepatis nachgewiesen, die die akute Transaminasenerhöhung nicht erklären würde. Die Kausalitäten für die genannten Arzneimittel werden mit "möglich" bewertet, da sie zur gleichen Zeit eingenommen wurden. Die UAW hat sich im Krankenhaus entwickelt.<dbtext>To_Char_NARRATIVE_2</dbtext>

NOVARTIS, 22.07.2011 DE-002147023-CHPA2011DE09392

Sender diagnosis: not reported

Narrative

Initial physician report received via BFARM (DE-BFARM-11105672) on 14 Jul 2011. A 45 year old male patient on Pantozol (pantoprazole), Ibuhexal (ibuprofen) and Zyprexa (olanzapine) developed acute hepatitis. On 29 Nov 2010, the patient was hospitalized in a psychiatric clinic for the treatment of a depressive episode. On 25 Feb 2011, increased transaminases appeared for the first time (ALAT 614 U/l, normal range: <45 U/l; ASAT 379 U/l, normal range: <50 U/l) that - after further increase (on 04 Mar 2011: ALAT 1158 U/l, ASAT 765 U/l) declined slowly from 05 Mar 2011. On 18 Mar 2011, three days before discharge, ASAT was 190 U/l and ALAT 69 U/l. After discontinuation of the treatment with suspect drugs transaminases decreased. Sonography showed only minor diffuse steatosis hepatic. The event developed while the patient had been hospitalized.

PANTOZOL:

Serious spontaneous report, acute hepatitis [hospitalization], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

IBUHEXAL:

The event, Hepatitis acute, was assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

NOVARTIS, 31.07.2012 DE-002147023-CHPA2011DE09392

Sender diagnosis: not reported

Narrative

Initial physician report received via BFARM (DE-BFARM-11105672) on 14 Jul 2011. A 45 year old male patient on Pantozol (pantoprazole), Ibuhexal (ibuprofen) and Zyprexa (olanzapine) developed acute hepatitis. On 29 Nov 2010, the patient was hospitalized in a psychiatric clinic for the treatment of a depressive episode. On 25 Feb 2011, increased transaminases appeared for the first time (ALAT 614 U/l, normal range: <45 U/l; ASAT 379 U/l, normal range: <50 U/l) that - after further increase (on 04 Mar 2011: ALAT 1158 U/l, ASAT 765 U/l) declined slowly from 05 Mar 2011. On 18 Mar 2011, three days before discharge, ASAT was 190 U/l and ALAT 69 U/l. After discontinuation of the treatment with suspect drugs transaminases decreased. Sonography showed only minor diffuse steatosis hepatic. The event developed while the patient had been hospitalized.

Following internal review on 30 Jul 2012 of information received on 14 Jul 2011, the labeling of Ibuhexal for the event, acute hepatitis, was corrected to Listed. The Company Comment was updated.

Individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

IBUHEXAL:

The adverse event, Hepatitis acute, is listed in the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11130494

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19290720	Age: 81 Year	weight (kg): 62	BMI_int: 22.23	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stool light coloured					recovered/resolved
2 Skin colouring yellow					recovered/resolved
3 Icterus	20110323	20110330	8 Day		recovered/resolved
4 Bilirubin increased					recovered/resolved
5 Transaminases increased	20110323	20110330	8 Day		recovered/resolved
6 Hepatitis acute	20110323	20110330	8 Day		recovering/resolving
7 Cholecystolithiasis	20110323	20110330	8 Day		recovered/resolved
8 Steatosis hepatic		20110330			recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	600 Mg milligram(s)/245	20100909-20110531	Drug use for unknown indication
ibuprofen			8 Month	
2 melperone	Oral	/	20110318-20110321	Drug use for unknown indication
melperone		82	4 Day	
3 Risperidon, 1 mg, Tabl, 1A-Pharma	Oral	1 Mg milligram(s)/Day	20110528-20110608	
risperidone		245	12 Day	

Drugs (concomitant)

4 artelac	Ophthalmic	2 Gtt drop(s)/Day	200804-	
hypromellose		71	-	
5 dulcolax	Oral	/	201010-20110322	
bisacodyl		9	-	
6 Nachtruhe Einschlafkapseln	Oral	1 Df dosage form/Day	201010-	
valeriana officinalis root		5	-	
7 simvastatin	Oral	20 Mg milligram(s)/Day	201001-	Drug use for unknown indication
simvastatin		82	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hyperlipoproteinemia			Yes	
Apoplexy			Unknown	Zustand nach Apoplex mit Aphasie
Amnestic syndrome			Unknown	organisches amnestisches Syndrom, nicht durch Alkohol oder andere psychotrope Substanzen bedingt.
Aphasia			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase	20110325	520	U/l	
GPT	20110323	457	U/l	
Gamma GT	20110325	113	U/l	

Bilirubin direct	20110323	4.65	mg/dl
Blood bilirubin indirect	20110325	1.72	mg/dl
Quick's test	20110328	52	%
Albumin	20110330	2900	NA
Alanine aminotransferase increased	20110411	188	U/l
Alanine aminotransferase increased	20110609	475	U/l
Alanine aminotransferase increased	20110622	306	U/l
Alanine aminotransferase increased	20110323	457	U/l
Alkaline phosphatase increased	20110323	209	U/l
Alkaline phosphatase increased	20110622	145	U/l
Alkaline phosphatase increased	20110609	125	U/l
Aspartate aminotransferase increased	20110609	595	U/l
Aspartate aminotransferase increased	20110622	410	U/l
Aspartate aminotransferase increased	20110411	228	U/l
Aspartate aminotransferase increased	20110323	479	U/l
Bilirubin increased	20110323	6.5	mg/dl
Bilirubin increased	20110609	12.5	mg/dl
Bilirubin increased	20110622	7.3	mg/dl
Aspartate aminotransferase	20110325	520	IU/L (intern
Aspartate aminotransferase	20110411	228	IU/L (intern
Aspartate aminotransferase	20110622	410	IU/L (intern
Aspartate aminotransferase	20110609	595	IU/L (intern
Aspartate aminotransferase	20110323	479	IU/L (intern
Bilirubin conjugated	20110323	4.65	mcg/dL (mi
Blood alkaline phosphatase	20110323	209	IU/L (intern
Blood alkaline phosphatase	20110622	145	IU/L (intern
Blood alkaline phosphatase	20110609	125	IU/L (intern
Blood bilirubin	20110323	6.5	mg/dL (mill
Blood bilirubin	20110609	12.5	mg/dL (mill
Blood bilirubin	20110622	7.3	mg/dL (mill
Gamma-glutamyltransferase	20110325	113	IU/L (intern
Prothrombin time	20110328	52	% (percent
Quick's test	20110328	52	% (percent
Quick's test	201106	45 (normally not above 70%)	
Sonogram	20110325	cholecystolithiasis	
Sonogram		of abdomen and magnetic resonanace tomography (no	

Narratives (nur bei elektronischem Eingang)

[sanofi-aventis, 13.07.2011 DE-SANOFI-AVENTIS-2011SA043691](#)

Sender diagnosis: not reported

Narrative

Initial report:

On 11-Jul-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11130494); documenting a case, in which a physician reported about an 82-year old female patient.

The medical history included apoplexy with following aphasia, amnesic syndrome (organic; not caused by alcohol or any psychotropic substances) and hyperlipoproteinemia. Concomitantly, the patient took simvastatin 20 mg for an unspecified indication. For a longer time the patient used ibuprofen

(trade name was not provided) 600 mg. Melperone was started on 16-Mar-2011 (20 or 50 mg). The appropriate indications were not provided. On 23-Mar-2011 the patient was hospitalised because of presentation of yellowish skin colour and a lighter stool than usual. Ibuprofen and melperone were stopped on the same day.

On the same day the increased transaminases and bilirubin were measured as follows: GPT 457 U/L, blood bilirubin total 6.48 mg/dl and bilirubin direct 4.65 mg/dl.

The CRP with 0.85 mg/dl (reference range until 1.0 mg/dl) was normal. Alternative explanations were reported, however assessed as less likely from the physician: cholecystolithiasis, questionable cholecystitis and mild steatosis hepatitis revealed during abdominal sonography on 23-Mar-2011. On 25-Mar-2011 an unclear structure on the head of pancreas was seen two days later.

Further lab values were measured as follows: aspartate aminotransferase 520 U/L, blood bilirubin indirect 1.72 mg/dl and gamma GT 113 U/L on 25-Mar-2011. Quick's test 52 % on 28-Mar-2011. Anti-HBc antibody was positive, alpha 1 fetoprotein 2.09 ng/ml and alkaline phosphatase 209 U/L (reference ranges between 106 and 265) on 29-Mar-2011 as well as albumin 2900 on 30-Mar-2011.

The abdominal CT from 29-Mar-2011 did not reveal an evidence for posthepatic bile ducts occlusion or tumor in the area of the pancreas, the bile ducts or in the upper abdomen.

Corrective treatment was not provided; the patient recovered until 30-Mar-2011.

Causality by AMSP: possible for ibuprofen and melperone

PVZ-FAKOS, Klinische Pharmakologie. 02.08.2011 DE-FAKOS-170409

Sender diagnosis: not reported

Narrative

Die Patientin wurde am 23.03.2011 wegen eines neu aufgetretenen Ikterus stationär aufgenommen. Bei Aufnahme waren die Transaminasen (ALAT 457 U/l, Normbereich: <34 U/l; ASAT 479 U/l, Normbereich: <31 U/l) und auch das Bilirubin (6,5 mg/dl, Normbereich: <1 mg/dl) stark erhöht. Wenige Tage vor dem Indexdatum wurden zur Behandlung von Angstzuständen und Halluzinationen Melperon AL 25 mg Filmtabl. eingenommen (18.-21.03.2011), welche bei Aufnahme pausiert wurden. Im Verlauf normalisierten sich die Transaminasen rasch (laut Arztbrief). Die Patientin wurde am 01.04.2011 aus der stationären Behandlung entlassen. Am 11.04.2011 wurden ambulant erneut erhöhte Transaminasen festgestellt (ALAT 188 U/l, ASAT 228 U/l), die im weiteren Verlauf ambulant nicht mehr kontrolliert wurden. Am 28.05.2011 wurde mit der Behandlung mit Risperidon 1A-Pharma 1 mg Tabl. begonnen. Am 08.06.2011 wurde die Patientin erneut stationär aufgenommen mit schmerzlosem Ikterus (Bilirubin am 09.06.2011: 12,5 mg/dl, ALAT: 475 U/l, ASAT: 595 U/l). Risperidon 1A-Pharma wurde bei Aufnahme abgesetzt (letzte Einnahme am 08.06.2011). Die Lebersyntheseleistung war jetzt deutlich reduziert mit einem verringerten Quick-Wert von etwa 45% (Normbereich: >70 %). Im Verlauf war eine tendenzielle Besserung der Transaminasen nachzuweisen, allerdings waren diese am 22.06.2011 noch deutlich erhöht (ALAT: 306 U/l, ASAT: 410 U/l), auch der Bilirubinwert war abfallend mit 7,3 mg/dl.

Abdomensonographie, MRT (Magnetresonanztomographie): Cholecystolithiasis, kein Nachweis einer intra- oder extrahepatischen Cholestase.

Hepatitisserologie: A, B und C (anti-HCV) negativ.

Am 24.06.2011 wurde die Patientin aus dem Krankenhaus entlassen. Die Leberpunktion vom 23.06.2011 ergab den Befund einer mäßiggradigen portalen Entzündungszellinfiltration mit Grenzzonen-Hepatitis, teils mit beginnendem zirrhotischem Umbau. Eine medikamentenbedingte Genese der Hepatitis ist wahrscheinlich. Aufgrund der Histologie mit teils zirrhotischem Umbau kann vermutet werden, dass das Arzneimittel eher schon länger eingenommen wurde. Hier kommen die seit 09.09.2010 eingenommenen Ibuprofen-CT 600 mg Filmtabletten am wahrscheinlichsten infrage. Ende der Einnahme am 31.05.2011. Ein Zusammenhang mit Melperon AL und Risperidon 1A-Pharma kann allerdings nicht ausgeschlossen werden, weswegen die Arzneimittelkausalität mit "möglich" bewertet wird. Die UAW's haben sich ambulant entwickelt. <dbtext>To_Char_NARRATIVE_2</dbtext>

Janssen-Cilag, Germany Local Safety Officers, 16.08.2011 DE-JNJFOC-20110707105

Sender diagnosis: not reported

Narrative

This was a spontaneous report from a physician, via a business partner (Pfizer Inc. - 2011155475) downloaded from the web-portal of the German Health Authority (BfArM) concerning an 82 year old female patient from Germany:DE-BFARM-11130494.

The patient's weight and height were not reported. The patient's medical history and concurrent conditions included: organic amnesic syndrome (not due to alcohol or other psychotrope substances), apoplexy with aphasia, and hyperlipoproteinaemia.

The patient was treated with ibuprofen (unspecified, oral) 600 mg daily initiated on an unspecified date "long ago" for an unspecified indication. Non-company suspect drug included melperone (unspecified, oral) 20-50 mg daily initiated on 16-MAR-2011 for an unspecified indication. Concomitant medication included simvastatin.

On an unspecified date, the patient experienced cholecystolithiasis and hepatic steatosis. She was hospitalized due to yellow skin color. She had no pain, no nausea, no vomiting, and no diarrhea. The patient's transaminases were increased as laboratory workout on 23-MAR-2011 showed alanine aminotransferase level of 457 IU/l and aspartate bilirubin conjugated level of 4.65 mcg/dl. On 25-MAR-2011, aminotransferase level was 520 IU/l and gamma-glutamyltransferase level was 113 IU/l. On 25-MAR-2011, sonography revealed cholecystolithiasis. On 28-MAR-2011, prothrombin time was 52 %, and Quick's test was 52 %. All laboratory values were reported without reference ranges. On 29-MAR-2011, computed tomography showed no indication of post-hepatic bile drainage obstruction and no indication for tumor growth in pancreas or bile flow tract. Treatment with ibuprofen and melperone was permanently withdrawn on 23-MAR-2011. The patient recovered from cholecystolithiasis, steatosis hepatic and transaminases increased on 30-MAR-2011. The reporting physician assessed the adverse events to have a possible causal relationship with both suspect drugs based on the Global Introspection (WHO GI) method for causality assessment. Further risk factor was previous damage of the affected organ NOS. This report was serious (hospitalization, medically significant).

Additional information received on 04-AUG-2011 from the German Health Authority (BfArM), via a business partner (Pfizer Inc. - 2011155475) . New information included: patients age, dosage, product start date, suspect medications, concomitant medications, medical history, hospitalization dates and diagnosis, laboratory reference ranges, laboratory results, ibuprofen stop date, tests and results, outcome and causality assessment.

This report concerns an 81 year old female (previously reported as 82 years old). The patient's weight was 62 kilograms. The patient was treated with ibuprofen (unspecified, oral) 600 mg once a week, (daily), initiated on 09-SEP-2010 to 31-MAR-2011 for an unspecified indication and risperidone 1mg daily initiated on 28-MAY-2011 to 08-JUN-2011 for an unspecified indication. Non-company suspect drugs included: melperone for anxiety disorder. Concomitant medications included simvastatin, bisacodyl, and hypromellose, and hop strobole. On 23-MAR-2011, the patient was hospitalized due to jaundice and she was diagnosed with acute hepatitis. On 23-MAR-2011, the following laboratory values were recorded: Blood Alkaline phosphatase 209 IU/l (NR: 30 - 120 IU/l), Alanine aminotransferase (GPT) 457 U/l (normal high: 34), bilirubin conjugated 4.65 mg/dL, Aspartate aminotransferase 479 U/l (normal high: 31). Bilirubin was increased with 6.5 mg/dl (normal high: 1). In the further course transaminase values normalized (as per physician's letter). On 25-MAR-2011: Aspartate aminotransferase 520 IU/L, gamma GT 113 U/L. On 28-MAR-2011, the following laboratory values were recorded: Quick's test and prothrombin time 52 %. The patient recovered 30-MAR-2011 from the event. On 01-APR-2011 the patient was discharged from hospital. On 11-APR-2011 the patient was again diagnosed to have

Janssen-Cilag, Germany Local Safety Officers, 18.08.2011 DE-JNJFOC-20110707105

Sender diagnosis: not reported

Narrative

This was a spontaneous report from a physician, via a business partner (Pfizer Inc. - 2011155475) downloaded from the web-portal of the German Health Authority (BfArM) concerning an 82 year old female patient from Germany: DE-BFARM-11130494. The patient's weight and height were not reported. The patient's medical history and concurrent conditions included: organic amnesic syndrome (not due to alcohol or other psychotrope substances), apoplexy with aphasia, and hyperlipoproteinaemia. The patient was treated with ibuprofen (unspecified, oral) 600 mg daily initiated on an unspecified date "long ago" for an unspecified indication. Non-company suspect drug included melperone (unspecified, oral) 20-50 mg daily initiated on 16-MAR-2011 for an unspecified indication. Concomitant medication included simvastatin. On an unspecified date, the patient experienced cholecystolithiasis and hepatic steatosis. She was hospitalized due to yellow skin color. She had no pain, no nausea, no vomiting, and no diarrhea. The patient's transaminases were increased as laboratory work on 23-MAR-2011 showed alanine aminotransferase level of 457 IU/l and aspartate bilirubin conjugated level of 4.65 mcg/dl. On 25-MAR-2011, aminotransferase level was 520 IU/l and gamma-glutamyltransferase level was 113 IU/l. On 25-MAR-2011, sonography revealed cholecystolithiasis. On 28-MAR-2011, prothrombin time was 52 %, and Quick's test was 52 %. All laboratory values were reported without reference ranges. On 29-MAR-2011, computed tomography showed no indication of post-hepatic bile drainage obstruction and no indication for tumor growth in pancreas or bile flow tract. Treatment with ibuprofen and melperone was permanently withdrawn on 23-MAR-2011. The patient recovered from cholecystolithiasis, steatosis hepatic and transaminases increased on 30-MAR-2011. The reporting physician assessed the adverse events to have a possible causal relationship with both suspect drugs based on the Global Introspection (WHO GI) method for causality assessment. Further risk factor was previous damage of the affected organ NOS. This report was serious (hospitalization, medically significant).

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FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11136262

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: 19620206	Age: 49 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GPT increased	20110701				recovering/resolving
2 GOT increased	20110701				recovering/resolving
3 Gamma GT increased	20110701				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Arcoxia 60 mg Filmtabletten etoricoxib	Oral	120 Mg milligram(s)/Day	- 7 Day	Lumbar syndrome
2 ibuprofen ibuprofen	Oral	1200 Mg milligram(s)/Day	- 7 Day	Lumbar syndrome
3 Katadolon S Long flupirtine	Oral	1 Df dosage form/Day 245	- 7 Day	Lumbar syndrome

Drugs (concomitant)

4 femigoa ethinylestradiol; levonorgestrel	/	- -	Contraception
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Smoker			Yes	5 cigarettes daily
Alcohol use			Yes	casually

Primary source

Qualification: Physician	observestudytype:
Literature:	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT	20110701	2545	IU/L	
Glutamic-oxaloacetic transferase	20110701	789	IU/L	
Gamma GT	20110701	145	IU/L	
Glutamic-oxaloacetic transferase	20110702	647	IU/L	
GPT	20110702	2221	IU/L	
Gamma GT	20110702	136	IU/L	
GPT	20110704	2214	IU/L	
Glutamic-oxaloacetic transferase	20110704	592	IU/L	
Gamma GT	20110704	123	IU/L	
GPT	20110706	1996	IU/L	
Gamma GT	20110706	119	IU/L	
Glutamic-oxaloacetic transferase	20110706	579	IU/L	

Narratives (nur bei elektronischem Eingang)

TEVA, 14.07.2011 DE-TEVA-288999GER

Sender diagnosis: not reported

Narrative

05-Jul-2011, Spontaneous Serious Report

A physician reported the case of a female patient 49 years of age who experienced MASSIVE GPT INCREASE, MASSIVE GOT INCREASE, GAMMA GT INCREASED while on Katadolon S Long (FLUPIRTINE, Teva drug), IBUPROFEN (Teva drug cannot be excluded) and Arcoxia 60 mg Filmtabletten (non-Teva drug).

Patient was hospitalized due to massive increase of transaminases. Arcoxia was started 14 days prior to the reported event together with Katadolon S long for treatment of lumbar spine syndrome. Both drugs were stopped after 7 days and treatment was continued with Ibuprofen 400mg 3 times daily for another 7 days. Patient is occasionally drinking a glass wine in the evening. Lab data is provided. After discontinuation of drugs, transaminases were gradually decreasing, but still extremely high.

Concomitant medication: Femigoa (ethinylestradiol/levonorgestrel).

At the time of the report the outcome was: not recovered/not resolved.

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Due to data available as well Katadolon S Long as Ibuprofen may have contributed to all symptoms reported: possibly related. Nevertheless, contribution of other drugs and circumstances cannot be excluded. Retrospectively it is hardly possible to assess, which drug contributed most to symptoms reported.

This is a spontaneous case and therefore considered and handled as a related case.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11139605

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19440917	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20110125			8 Day	not recovered/not resolved
2 Jaundice	20110125			8 Day	not recovered/not resolved
3 Scleral icterus	20110125				not recovered/not resolved
4 Liver injury	20110125			8 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	50 Mg milligram(s)/ 245	-	Knee pain
diclofenac			-	
2 ibuprofen 400 mg	Oral	400 Mg milligram(s)/Day 245	20110118-20110118	Knee pain
ibuprofen			1 Day	

Drugs (concomitant)

3 ENALAPRIL	/	-
enalapril		-
4 Hydrochlorothiazid	/	-
hydrochlorothiazide		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Unknown	hypertension arterial

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Aspartate aminotransferase		15.69	µmol/s*l	
Alanine aminotransferase		25.53	µmol/s*l	
Bilirubin		117	µmol/l	
Alkaline phosphatase		3.29	NA	
Gamma GT		6.16	NA	

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 25.07.2011 DE-BAYER-2011-063717

Sender diagnosis: not reported

Narrative

Serious, Unlisted, Related

This initial spontaneous case was reported by a health professional in Germany via the Drug Commission of the German Medical Association and was identified on the internet page of the health authority (BFARM), was received on 20-JUL-2011 and refers to a 66 year old female patient who received IBUPROFEN and experienced jaundice and hepatic enzymes increased.

The patient's concurrent condition included hypertension arterial.

It is unknown whether the patient received any concomitant medication.

On 18-Jan-2011 the patient took IBUPROFEN at 400 mg, QD for knee pain.

On 25-JAN-2011 the patient experienced jaundice, hepatic enzymes increased and therefore the patient was hospitalized. IBUPROFEN was withdrawn. The outcome of jaundice and hepatic enzymes increased were not resolved.

Reporter causality:

relationship between jaundice and IBUPROFEN related

relationship between hepatic enzymes increased and IBUPROFEN related.

sanofi-aventis, 27.07.2011 DE-SANOFI-AVENTIS-2011SA046657

Sender diagnosis: not reported

Narrative

Initial report:

On 21-Jul-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11139605); documenting a case, in which a physician had reported to the Drug Committee of the German Physicians Association about a 66-year-old female patient.

Concomitantly, the patient suffered from arterial hypertension.

On 18-Jan-2011 the patient took one tablet ibuprofen 400 mg (trade name not provided) for knee pain. On 25-Jan-2011 jaundice and increased hepatic enzymes were detected leading to hospitalization. Following lab values (without reference ranges) were revealed at unspecified time point: aspartate aminotransferase 15.69 mcmol/s*l, alanine aminotransferase 25.53 mcmol/s*l and bilirubin 117 mcmol/l as well as alkaline phosphatase 3.29 and gamma GT 6.16 (units were not reported). Corrective therapy was not mentioned; the outcome was classified with not recovered.

Causality by Drug Commission: possible

Reporter's comment:

sanofi-aventis, 26.09.2011 DE-SANOFI-AVENTIS-2011SA046657

Sender diagnosis: not reported

Narrative

Initial report:

On 21-Jul-2011 this case was extracted from the website of the German health authority (authority number DE-BFARM-11139605); documenting a case, in which a physician had reported to the Drug Committee of the German Physicians Association about a 66-year-old female patient.

Concomitantly, the patient suffered from arterial hypertension.

The initial report concerned intake of ibuprofen 400 mg for knee pain on 18-Jan-2011. The originally document included only diclofenac (trade name not provided) 50 mg, which was taken for knee pain at four unspecified time points. Ibuprofen was not mentioned.

On 25-Jan-2011 the jaundice of skin, scleral icterus and increased hepatic enzymes were diagnosed. Following lab values (without reference ranges) were revealed at unspecified time point: aspartate aminotransferase 15.69 mcmol/s*l, alanine aminotransferase 25.53 mcmol/s*l and bilirubin 117.3 mcmol/l as well as alkaline phosphatase 3.29 and gamma GT 6.16 (units were not reported).

Subsequently the patient was hospitalised and corrective treatment with enalapril and hydrochlorothiazide was started. At date of report the patient had not yet recovered.

Causality by Drug Commission: possible

Additional information received on 20-Sep-2011:

The originally document for the initial data collection was provided by the Drug Commission of the German Medical Association.

Patients birth date and initials were amended. The suspected product was changed and the events were adjusted. Corrective therapy was amended.

Bayer Vital GmbH, 27.09.2011 DE-BAYER-2011-063717

Sender diagnosis: not reported

Narrative

Serious, Unlisted, Related

This initial spontaneous case was reported by a health professional in Germany via the Drug Commission of the German Medical Association and was identified on the internet page of the health authority (BFARM), was received on 20-JUL-2011 and refers to a 66 year old female patient who received IBUPROFEN and experienced jaundice and hepatic enzymes increased.

The patient's concurrent condition included hypertension arterial.
It is unknown whether the patient received any concomitant medication.

On 18-Jan-2011 the patient took IBUPROFEN at 400 mg, QD for knee pain.
On 25-JAN-2011 the patient experienced jaundice, hepatic enzymes increased and therefore the patient was hospitalized.
IBUPROFEN was withdrawn. The outcome of jaundice and hepatic enzymes increased were not resolved.

Reporter causality:
relationship between jaundice and IBUPROFEN related
relationship between hepatic enzymes increased and IBUPROFEN related.

Follow-up information was received on 16-sep-2011:
Patient information was updated.
Concomitant drugs were added
Lab data at time of admission updated: bilirubin -d 108, 9 (units not recorded).

Suspected drug was amended from Ibuprofen to Diclofenac.
The reporter has not provided assessment.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11145170

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzymes					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fraxiparin	Unknown	/	-	
nadroparin calcium		120	-	
2 ibuprofen	Unknown	/	-	
ibuprofen			-	

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)GlaxoSmithKline, 26.07.2011 DE-GlaxoSmithKline-D0072134A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of increase in liver values in a female patient who received Nadroparine calcium (Fraxiparin) injection for an unknown drug indication.

On an unknown date the patient started Nadroparine calcium (unknown) at an unknown dosing.

At an unknown time after starting Nadroparine calcium, the patient experienced increase in liver values. This case was assessed as medically serious by GSK.

At the time of reporting, the outcome of the event was unknown.

Reporter's comment:

GlaxoSmithKline, 12.08.2011 DE-GlaxoSmithKline-D0072134A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of increase in liver values in a female patient who received Nadroparine calcium (Fraxiparin) injection for an unknown drug indication.

On an unknown date the patient started Nadroparine calcium (unknown) at an unknown dosing.

At an unknown time after starting Nadroparine calcium, the patient experienced increase in liver values.

This case was assessed as medically serious by GSK.

Oral follow-up information was received from the physician on 10 August 2011.

Co-suspect medication included Ibuprofen.

On unknown dates, the patient started Nadroparine calcium (unknown), unknown dosing, and Ibuprofen (unknown), unknown dosing.

Ibuprofen was the reason for the increase in liver values. Ibuprofen was discontinued and the event resolved.

The reporting physician considered the event was unrelated to treatment with Nadroparine calcium.

No further information will be available.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-11156782

Serious Yes	Death No	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth:	Age: 81 Year	weight (kg): 62	BMI_int: 22.23	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20110323			5 Month	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dulcolax bisacodyl	Oral	/ 9	201010-20110322	-
2 ibuprofen 600mg ibuprofen	Oral	600 Mg milligram(s)/ 245	20100909-20110531	8 Month
3 MELPERON AL melperone hydrochloride	Oral	25 Mg milligram(s)/ 245	20110318-20110321	4 Day
4 RISPERIDON risperidone	Oral	1 Mg milligram(s)/ 245	20110528-20110608	12 Day

Drugs (concomitant)

5 ABTEI NACHTRUHE EINSCHLAFKAPSELN humulus lupulus extract; valerian extract	Oral	/ 5	201010-	-
6 ARTELAC ADVANCED EDO DR. MANN hypromellose	Intraocular	/ 158	200804-	-
7 SIMVASTATIN BLUEFISH simvastatin	Oral	20 Mg milligram(s)/ 245	201001-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hyperlipoproteinemia			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20110323	457	U/l	
Alanine aminotransferase	20110411	188	U/l	
Alanine aminotransferase	20110609	475	U/l	
Alanine aminotransferase	20110622	306	U/l	
Aspartate aminotransferase	20110323	479	U/l	
Aspartate aminotransferase	20110411	228	U/l	
Aspartate aminotransferase	20110609	595	U/l	
Aspartate aminotransferase	20110622	410	U/l	
Alkaline phosphatase increased	20110323	209	U/l	
Alkaline phosphatase increased	20110609	125	U/l	
Alkaline phosphatase increased	20110609	125	U/l	
Alkaline phosphatase increased	20110622	145	U/l	

Bilirubin increased	20110323	6.5	mg/dl
Bilirubin increased	20110609	12.5	mg/dl
Bilirubin increased	20110622	7.3	mg/dl

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 11.08.2011 DE-Boehringer Ingelheim GmbH, Germany-2011-DE-04449DE

Sender diagnosis: not reported

Narrative

Initial information was received from the German Health Authority concerning a 81-year-old female patient, who was on Dulcolax 5mg (bisacodyl), 1x1 coated tablet oral at 5mg 2-3y weekly, between Oct2010 and 22Mar2011, for the treatment of an unknown indication; Ibuprofen 600mg, at least 1 film-coated tablet weekly per os, between 09Sep2010 and 31May2011, for the treatment of an unknown indication; Melperon AL, altogether nearly 3 film-coated tablets, between 18Mar2011 and 21Mar2011, for the treatment of an unknown indication and Risperidon, 1 tablet daily orally, between 28May2011 and 08Jun2011, for the treatment of an unknown indication.

On 23Mar2011, the patient experienced acute hepatitis. The event was considered to be serious due to hospitalisation. The patient fully recovered.

Concomitant medication(s) included Simvastatin Bluefish, 1 film-coated tablet daily orally, since Jan2010; Abtei Nachtruhe Einschlafkapseln, 1 capsule daily orally, since Oct2010 and Artelac Advanced Edo Dr. Mann (Hyaluronic acid), 1 drop intraocular daily, since Apr2008. Further concomitant diseases included hyperlipoproteinemia.

The causal relationship for Dulcolax was not reported.

The reporter rated the causal relationship with Ibuprofen as probable/likely.

The reporter rated the causal relationship with Melperon AL as possible.

The reporter rated the causal relationship with Risperidon as possible.

Reporter Remarks:

Case narrative including clinical course, therapeutic measures, outcome and additional relevant information:

The patient was admitted to hospital on 23Mar2011 because a new onset jaundice.

At the admission were the transaminases (ALAT 457 U/l, normal range: <34 U/l; ASAT 479 U/l, normal range <31 U/l) and the bilirubin (6,5 mg/dl, normal range: <1 mg/dl) greatly increased.

A few days before the index date were melperone AL 25mg Tabl. used to treat anxiety and hallucinations (18 to 21Mar2011), which were paused at admission. In the course of the transaminases normalized rapidly (according to doctor's letter). The patient was discharged from inpatient treatment on 01Apr2011. On the 11Apr2011 were ambulatory found increased transaminases again (ALAT 188U/l, ASAT 228 U/l), were no longer ambulatory monitored. On the 28May2011 the treatment with Risperidon 1A-Pharma Tabl. started. On the 08Jun2011 the patient was hospitalized again with painful jaundice (bilirubin on the 09Jun2011: 12,5 mg/dl, ALAT: 475 U/l, ASAT: 595 U/l). Risperidon 1A-Pharma was discontinued in the receptive (last intake on the 08Jun2011). The liver synthesis capacity was now significantly reduced with a Quick value decreased by about 45% (normal range: <70%). In the course of a trend towards improvement in transaminases was detected, although these were still significantly increased at 22Jun2011 (ALAT: 306 U/l, ASAT: 410U/l), also the bilirubin value was sloping with 7,3mg/dl. Abdominal sonography, MRT (Magnetic Resonance Imaging): cholecystolithiasis, no evidence of intra- or extrahepatic cholestasis. Hepatitis serology: A, B and V (anti-HCV) negative.

On the 24Jun2011 the patient was discharged from inpatient treatment. The liver biopsy from 23.06.2011 revealed the findings a moderate portal inflammatory cell infiltration with border zone hepatitis, cirrhotic some with incipient conversion. The genesis is a drug-induced hepatitis is probably. Because the histology with some cirrhotic remodeling, it can be assumed that the drug was taken rather a long time. Here come the occupied Ibuprofen-CT 600mg tablets since 09Sep2010 are most likely eligible. End of the intake was 31May2011. A connection with Melperone AL-1A Pharma and Risperidone can be given, however, why the drug is evaluated with possible causality. The ADRs have developed on an outpatient basis.

Tracking of Changes:

04Aug2011 - initial information received

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11178455

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 19421121	Age: 68 Year	weight (kg): 62	BMI_int: 21.97	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20110822	20110920	30 Day	36 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Amiada 250 mg Tabletten terbinafine hydrochloride	Oral	250 Mg milligram(s)/ Day 245	20110720-20110821 33 Day	Onychomycosis
2 Ibu 400 akut - 1 A-Pharma ibuprofen	Oral	1200 Mg milligram(s)/Day 82	20110815-20110820 6 Day	Fever

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Onychomycosis			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20110824	342	U/l	
Alanine aminotransferase increased	20110826	336	U/l	
Aspartate aminotransferase increased	20110824	256	U/l	
Aspartate aminotransferase increased	20110826	159	U/l	
Alkaline phosphatase increased	20110824	638	U/l	
Alkaline phosphatase increased	20110826	585	U/l	
Bilirubin normal	20110823	0.39	mg/dl	
Bilirubin normal	20110826	0.5	mg/dl	
Alanine aminotransferase increased	20110822	278	U/l	
Alanine aminotransferase increased	20110902	80	U/l	
Alanine aminotransferase normal	20110920	27	U/l	
Aspartate aminotransferase increased	20110822	179	U/l	
Aspartate aminotransferase normal	20110902	32	U/l	

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 09.09.2011 DE-FAKOS-170417

Sender diagnosis: not reported

Narrative

Sender's comment:

Initial report, more info follows.

Reporter's comment:

PVZ-FAKOS, Klinische Pharmakologie, 10.11.2011 DE-FAKOS-170417

Sender diagnosis: not reported

Narrative

Der Patient wurde am 23.08.2011 mit stark erhöhten Transaminasen (ALAT 278 U/l, Normbereich: <41 U/l; ASAT 179 U/l, Normbereich: <37 U/l) und Cholestaseparametern (Gamma-GT 970 U/l, Normbereich: 8-61 U/l; AP (Alkalische Phosphatase) 638 U/l, Normbereich: 40-129 U/l) sowie erhöhten Temperaturen stationär aufgenommen (Laborwerte vom 22.08.2011, außer AP vom 24.08.2011). Bereits am 12.08.2011 traten starke Gliederschmerzen und Fieber auf. Die pathologisch erhöhten Leberwerte waren innerhalb von wenigen Tagen schnell rückläufig (am 02.09.2011: ALAT 80 U/l, ASAT 32 U/l, Gamma-GT 520 U/l). Die Serologie auf Hepatitis A, B und C war negativ. Leber sonographisch unauffällig, keine intrahepatisch erweiterten Gallenwege.

Am 26.08.2011 wurde der Patient in subjektivem Wohlbefinden und stabilem Allgemeinzustand aus dem Krankenhaus entlassen.

Differenzialdiagnostisch kommt neben einem Infekt mit Begleithepatitis auch eine medikamentöse Ursache in Frage. Verdächtigt werden:

Amiada 250 mg Tabl., Intendis, Einnahmezeitraum: 20.07.-21.08.2011 (mit Pause) und

Ibu akut 1A-Pharma 400 mg Filmtabl., Einnahmezeitraum: 15.-20.08.2011.

Die Einnahme dieser Arzneimittel steht in engem zeitlichem Zusammenhang mit dem Auftreten der UAW. Die Arzneimittelkausalität wird mit "möglich" bewertet, da keine genaue Zuordnung zu einem Arzneimittel möglich ist. Die UAW ist ambulant entstanden.<dbtext>

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FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11179526

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes			Yes		

Patient

Initials:	Sex: Female	Birth: 19650227	Age: 46 Year	weight (kg): 60	BMI_int: 21.26	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20110416				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cerazette	Oral	75 µg microgram(s)/Day	20110107-20110417	The pill
desogestrel		82	3 Month	
2 Cipro-1 A Pharma 500 mg	Oral	1000 Mg milligram(s)/Day	20110415-20110417	
ciprofloxacin hydrochloride		82	3 Day	
3 falithrom	Oral	/	20101202-20110417	Thrombosis of leg deep venous
phenprocoumon		82	4 Month	
4 ibuhexal 600	Oral	600 Mg milligram(s)/3 We	2005-20110414	
ibuprofen		82	-	

Drugs (concomitant)

5 eferox	Oral	37.5 µg microgram(s)/Day	2001-	Hypothyroidism
levothyroxine sodium		245	-	
6 Votum plus 20 mg/25 mg	Oral	1 Df dosage form/Day	2010-	
hydrochlorothiazide; olmesartan medoxomil		82	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Thrombosis of leg deep venous	201011		Unknown	
Hypothyroidism	2010		Yes	
Hypertension arterial			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase increased	20110417	1660	U/l	
Alanine aminotransferase increased	20110421	1652	U/l	
Alanine aminotransferase increased	20110426	1326	U/l	
Alanine aminotransferase increased	20110517	164	U/l	
Alanine aminotransferase increased	20110812	36	U/l	
Aspartate aminotransferase increased	20110417	1204	U/l	
Aspartate aminotransferase increased	20110421	1518	U/l	
Aspartate aminotransferase increased	20110426	1106	U/l	
Aspartate aminotransferase increased	20110517	102	U/l	
Aspartate aminotransferase increased	20110812	43	U/l	
Alkaline phosphatase increased	20110417	141	U/l	
Alkaline phosphatase increased	20110426	182	U/l	

Alkaline phosphatase increased	20110517	169	U/l
Alkaline phosphatase normal	20110812	70	U/l
Bilirubin increased	20110417	2.77	mg/dl
Bilirubin increased	20110426	13.6	mg/dl
Bilirubin increased	20110517	4.6	mg/dl
Bilirubin normal	20110812	0.6	mg/dl

Narratives (nur bei elektronischem Eingang)

PVZ-FAKOS, Klinische Pharmakologie, 14.09.2011 DE-FAKOS-170397

Sender diagnosis: not reported

Narrative

Aufgrund von seit einer Woche bestehenden Schmerzen im linken Nierenlager war die Patientin vor der stationären Aufnahme (17.04.2011) unter dem Verdacht auf einen Harnwegsinfekt in der Zeit vom 15.-17.04.2011 mit Cipro 1A-Pharma 500 mg Filmtabl. behandelt worden. Am 16.04.2011 trat eine Dunkelfärbung des Urins sowie eine Entfärbung des Stuhls auf, außerdem Schmerzen im linken Nierenlager. Am Tag der Aufnahme bestand ein leichter Sklerenikterus und die Transaminasen (ALAT 1660 U/l (Normbereich: <31 U/l), ASAT 1204 U/l (Normbereich: <31 U/l)) und Cholestaseparameter (Gamma-GT 298 U/l (Normbereich: 5-36 U/l), AP (Alkalische Phosphatase) 141 U/l (Normbereich: 35-104 U/l), Bili gesamt 2,77 mg/dl (Normbereich: <1,2 mg/dl)) waren stark erhöht. Bei deutlich erniedrigtem Quick-Wert, der nach Vitamin-K-Gabe nur auf 40% anstieg, bestand der Verdacht auf eine Synthesestörung der Leber.

Die Patientin nahm seit 02.12.2010 Falithrom Filmtabl. ein (Einnahme bis 17.04.2011) bei Z.n. Phlebothrombose der V. poplitea links. Diese Medikation und die weitere ambulante Medikation mit Ibuhexal 600 mg Filmtabl. bei Bedarf zur Migränebehandlung (Einnahme 2005-14.04.2011), Votum plus 20/25 mg Filmtabl. (Anfang 2010-22.04.2011) sowie Cerazette Filmtabl. (07.01.-17.04.2011) wurde abgesetzt. Die Patientin wurde mit weiterhin stark erhöhten Transaminasen (21.04.2011: ALAT 1652 U/l, ASAT 1518 U/l) und Cholestaseparametern am 22.04.2011 erst einmal zur weiteren ambulanten Beobachtung aus der stationären Behandlung entlassen.

Am 26.04.2011 erfolgte die erneute stationäre Aufnahme wegen Verschlechterung des Ikterus (Bilirubin jetzt 13,6 mg/dl, ALAT 1326 U/l, ASAT 1106 U/l, Gamma-GT 489 U/l, AP 182 U/l). Am 29.04.2011 wurde mit einer Steroidtherapie (Solu-Decortin H) begonnen, intravenöse Vitamin K-Gaben zur Erhöhung des Quicks erbrachten einen unzureichenden Effekt, sodass eine fortbestehende Syntheseleistungsstörung der Leber angenommen wurde. Am 26.04.2011 traten auch Zeichen der hepatischen Enzephalopathie auf (Müdigkeit und Schwäche), sodass keine durchgreifende Besserung zu verzeichnen war und insgesamt eine kritische Situation vorlag. Die Patientin wurde am 29.04.2011 mit Verdacht auf Leberversagen in ein Krankenhaus der Maximalversorgung verlegt. Dort wurde unter dem fortbestehenden Verdacht auf medikamentös-toxisches Leberversagen die Steroidtherapie fortgesetzt (Ende 27.07.2011). Ab dem 27.04.2011 trat eine langsame Besserung der Transaminasen und der Cholestaseparameter ein (am 17.05.2011: ALAT 164 U/l, ASAT 102 U/l, Gamma-GT 630 U/l, AP 169 U/l, Bilirubin 4,6 mg/dl), die Blutgerinnung normalisierte sich. Die Patientin wurde am 09.05.2011 unter weiter bestehender Steroidtherapie in gebessertem und stabilem Allgemeinzustand aus der stationären Behandlung entlassen.

Eine Leberbiopsie wurde nicht durchgeführt. Sonographisch stellte sich die Leber normal groß dar, keine intra- oder extrahepatische Cholestase, kein Gallengangskonkrement, keine Raumforderung oder anderer Focus.

Hepatitisserologie: A und C negativ, Z.n. durchgemachter Hepatitis B.

Eine medikamenten-bedingte Ursache der Hepatitis ist wahrscheinlich. Infrage kommen Cipro 1A-Pharma Filmtabl., Falithrom Tabl., Cerazette Filmtabl. und Ibuhexal Filmtabl. Die Arzneimittelkausalitäten werden jeweils mit "möglich" bewertet, da eine genauere Zuordnung zu einem bestimmten Arzneimittel nicht möglich ist. Die UAW hat sich ambulant entwickelt.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11187511

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: 19530504	Age: 58 Year	weight (kg): 55	BMI_int: 19.72	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GPT increased					Unknown
2 Acute hepatic failure	20110701		4 Week	58 Day	not recovered/not resolved
3 GOT increased					Unknown
4 General physical condition abnormal	201107				not recovered/not resolved
5 Hepatic necrosis	201107				not recovered/not resolved
6 Hepatitis toxic drug-induced	201107				not recovered/not resolved
7 Bilirubin increased					Unknown
8 Scleral icterus					Unknown
9 Icterus	201107				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	2 Df dosage form/ 245	20110430-20110713	Spinal stenosis NOS
2 katadolon kapseln	Oral	3 Df dosage form/ 5	20110505-20110713	Lumbar pain
flupirtine maleate			75 Day	
			70 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ex-alcoholic			Unknown	Entwöhnungstherapie Alkohol 1994 für 6 Monate, kurzer Rückfall 01 + 02, seither glaubhaft trocken
Skin rash			Unknown	vor Jahren Hautausschlag nach Behandlung mit Penizillin
Alcohol abuse			Unknown	1994 sechsmonatige Alkohol-Entwöhnungstherapie, kurzfristiger Rückfall in 2001 und 2002
Depressive episode	2005		Unknown	
Hepatic lesion	201103		Unknown	große echofreie fokale Läsion /7,8 x 6,4 cm) im rechten Leberlappen
Lumbar pain			Unknown	
Intervertebral foraminal encroachment			Unknown	
Liver injury			Unknown	Abdomen Sono: 18.03.11 glatt begrenzte große echofreie fokale Läsion 7,8 + 6,4 cm im re. Leberlappen

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		9.2	mg/dL	
GPT		354	U/l	
GPT		1773	U/l	
Glutamic-oxaloacetic transferase		232	U/l	

Glutamic-oxaloacetic transferase

1475

U/l

Narratives (nur bei elektronischem Eingang)TEVA, 27.09.2011 DE-TEVA-302003GER

Sender diagnosis: not reported

Narrative

16-Sep-2011, Serious Literature Report

Case of a Female patient 58 Years of age who experienced SCLERAL ICTERUS, ICTERUS OF SKIN, GPT INCREASED, GOT INCREASED, BILIRUBIN INCREASED while on FLUPIRTINE and IBUPROFEN (Teva's products cannot be excluded).

The patient received Ibuprofen (Ibuflam Lichtenstein and others) against PAINFUL LUMBAR SPINE SYNDROME, PAINFUL INTERVERTEBRAL FORAMINAL ENCROACHMENT. Five days later she additionally received FLUPIRTINE (Katadolon, generics). After two months on both drugs she was hospitalized due to SCLERAL ICTERUS and ICTERUS OF SKIN and both drugs were discontinued. GPT was 1773 U/l, GOT 1475 U/l and bilirubin 9,2 mg/dl. Serological examination did not provide any hints for a hepatitis (viral or autoimmune) or the involvement of inheritable factors. After approx. two weeks GPT and GOT levels decreased to 354 U/l and 232 U/l, respectively. Liver biopsy resulted in the suspicion that it might be a drug-induced toxic hepatitis. An ALCOHOL ABUSE (contraindication for FLUPIRTINE) is past for more than eight years.

At the time of the report the outcome of the AEs were:

SCLERAL ICTERUS : Unknown, ICTERUS OF SKIN : Unknown, GPT INCREASED : Unknown, GOT INCREASED : Unknown, BILIRUBIN INCREASED : Unknown.

Medical History:

PAINFUL LUMBAR SPINE SYNDROME, PAINFUL INTERVERTEBRAL FORAMINAL ENCROACHMENT, ALCOHOL ABUSE

This case was considered serious based on the following criteria: (Hospitalization Required, Other Serious (Important Medical Events))

Teva Comment: Both drugs are known to cause increase of liver enzymes and liver disorder. Additionally, due to temporal relationship symptoms reported are possibly related to Flupirtine and Ibuprofen. Retrospectively it is not assessable which drug contributed most to symptoms reported.

This is a spontaneous case and therefore considered and handled as a related case.

McNeil Germany, Local Safety Officers, 30.09.2011 DE-JNJFOC-20110910037

Sender diagnosis: not reported

Narrative

Report received from literature, via a business partner (Pfizer Inc. - 2011222330): Netzwerk aktuell, Leberschaeden unter Flupirtin. Arzneitelegamm 16-Sep-2011;42:79.

This report concerns a 58 year old female from Germany. Her weight and height were not reported. The patient's medical history and concurrent conditions included: alcohol abuse (longer than 8 yrs ago), lumbar pain, and spinal column stenosis.

The patient was treated with ibuprofen (dose, route and dates of use unspecified) for pain. Non-company suspect drugs included: flupirtine for pain. No concomitant medications reported.

On an unspecified date the patient was hospitalized for icterus of the eyes and skin and experienced drug induced toxic hepatitis. Laboratory findings include: alanine aminotransferase: 1773 ul, aspartate aminotransferase: 1475 ul and blood bilirubin: 9.2 mg/dl (milligram/deciliter) (dates and reference range not reported). There were no hints serological for a virus-order autoimmune hepatitis (NOS). After a liver biopsy was done (date unspecified), suspicion of drug induced toxic hepatitis was raised. Treatment with ibuprofen and flupirtine was withdrawn on an unspecified date. Within two weeks the laboratory findings included: alanine aminotransferase 354 ul and aspartate aminotransferase 232 ul. The patient was recovering from drug induced toxic hepatitis.

This report is serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-11186565

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis toxic					Unknown
2 Icterus					Unknown
3 Hepatitis toxic drug-induced					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 flupirtine		/	-	Pain
flupirtine			-	
2 ibuprofen		/	-	Pain
ibuprofen		5	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol abuse			Unknown	
Spinal column stenosis			Unknown	
Lumbar pain			Unknown	
Lumbar syndrome			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	UNKNOWN. Netzwerk aktuell, Leberschaeden unter Flupirtin. Arzneitelegamm, 42. Jahrgang, 16September2011. 2011;42:79	
Qualification:	Other Health Professional	observestudytype:
Literature:	anonymous. Leberschaeden unter Flupirtin (Katadolon, Generika). Arzneitelegamm 2011 Sep;42:9:79.	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GPT		1773	ul	
GPT		354	ul	
Aspartate transferase		1475	ul	
Aspartate transferase		232	ul	
Bilirubin		9.2	mg/dl	

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 26.09.2011 DE-PFIZER INC-2011222330

Sender diagnosis: not reported

Narrative

This is a literature report from Arzneimittellegramm, Title: Netzwerk aktuell, Leberschaeden unter Flupirtin, Number 9/2011, Volume 42, page 79. The English translation for the article has not been requested. A 58-year-old female patient was treated with ibuprofen (IBUPROFEN), dose and route of administration not reported, due to pain caused by lumbar spinal syndrome and spinal stenosis. Five days later (NOS) flupirtine (KATALADON) was added to the pain therapy. After two month of therapy with both drugs she was hospitalized because of icterus of eyes and skin. Both pain drugs were discontinued. Lab data were GPT 1773 U/l, GOT 1475 U/l and bilirubine 9.2 mg/dl (date and reference range not provided). Within two weeks the transaminases were reduced to 354 U/l and 232 U/l. There were no hints serological for a virus- oder autoimmune hepatitis (NOS). After a liver biopsy was done, suspicion of drug-induced toxic hepatitis was raised. Alcohol abuse was longer than 8 years ago. Within two weeks the transaminases were reduced to 354 U/l and 232 U/l.

No follow-up attempts needed/possible. No further information expected.
Nutzen-/Risikoverhältnisse.

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 03.10.2011 DE-Boehringer Ingelheim GmbH, Germany-2011-DE-05345GD

Sender diagnosis: not reported

Narrative

P11-11200

Drug-induced toxic hepatitis occurred in a patient treated with flupirtine and ibuprofen.

A 58-year-old woman was taking ibuprofen (IBUFLAM LICHTENSTEIN plus others) for pain associated with lumbar spine syndrome and stenosis of the spinal canal and, five days later, also began taking flupirtine (KATADOLON, generics). After the best part of two months' treatment with both analgesics, she was admitted as an in-patient with jaundice of the skin and sclera. The painkillers were discontinued. With ALT of 1,773 U/l, AST of 1,475 U/l and bilirubin of 9.2 mg/dL, there were no serological indications of viral or autoimmune hepatitis or of any hereditary causes. Within two weeks, the transaminase values fell to 354 U/l and 232 U/l, respectively.

Following a liver biopsy, drug-induced toxic hepatitis was suspected. Until eight years previously, this patient had a history of alcohol abuse in which flupirtine is contraindicated

Authors' statement: 'Non-steroidal anti-rheumatics can sometimes lead to liver damage. We also received two further reports of suspected cases involving flupirtine. The Federal Institute for Drugs and Medical Devices (BfArM) has received 375 reports of suspected cases of flupirtine-induced hepatic damage from Germany, with 148 involving hepatitis and 37 involving liver failure. Nine cases were fatal. Within the last four years, the number of reports involving flupirtine has more than doubled.'

The case is presented in an article (network latest) describing liver damage under treatment with flupirtine.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11223290

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth:	Age: 46 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver failure	20110801				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dolormin ibuprofen lysinate		/	2011-	
2 dolormin ibuprofen lysinate		/	20100822-20100825	Influenza like illness
3 ibuprofen ibuprofen	Unknown	800 Mg milligram(s)/	201008-201108	Influenza like illness
4 peginterferon peginterferon alfa-2a		/	20100805-20110630	Hepatitis C
5 ribavirin ribavirin	Unknown	1000 Mg milligram(s)/	20100805-	Hepatitis C

Drugs (concomitant)

6 Acetylsalizylsaeure, Paracetamol acetylsalicylic acid; paracetamol	/	20100805-
7 PASPERTIN metoclopramide hydrochloride	/	20100812-
8 xylocaine lidocaine hydrochloride	/	20101013-
9 YASMIN drospirenone; ethinylestradiol	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Influenza like illness			Unknown	
Caesarean section			Unknown	
Haemorrhoids			Unknown	
Hepatitis C			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin direct	20110512	0.1	mg/dl	
Bilirubin direct	20110808	10.6	mg/dl	
Bilirubin direct	20110926	0.9	mg/dl	
Bilirubin direct	20100805	0.1	mg/dl	
Albumin	20110512	3.4	g/dl	
Albumin	20110808	2.4	g/dl	

Albumin	20110926	2.9	g/dl
Albumin	20100805	3.8	g/dl
Bilirubin total	20100805	0.4	mg/dl
Bilirubin total	20110512	0.4	mg/dl
Bilirubin total	20110808	15.3	mg/dl
Bilirubin total	20110926	1.5	mg/dl

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 09.11.2011 DE-PFIZER INC-2011268208

Sender diagnosis: not reported

Narrative

This is a spontaneous report based on information received by Pfizer from Johnson and Johnson (DE-JNJFOC-20111011942), license partner for ibuprofen.

This was a spontaneous report from a health professional concerning a 46 year old female from Germany: 11445737A.

The patient's weight and height were not reported. The patient's medical history and concurrent conditions included: caesarean section, hemorrhoids, hepatitis C, and influenza like illness.

The patient was treated with ibuprofen (unspecified) 800 mg initiated on an unspecified date in AUG-2010 for flu like symptoms and ibuprofen lysinate (film coated tablet, unspecified) initiated on 22-AUG-2010 for flu like symptoms. Non-company suspect drugs included: ribavirin (unspecified) 1000 mg initiated on 05-AUG-2010 for hepatitis C, investigational drug (PEG-r-IL-20, unspecified) or peginterferon alfa-2a (unspecified) initiated on 05-AUG-2010 for hepatitis C. Concomitant medications included drospirenone w/ethinylestradiol, metoclopramide hydrochloride, paracetamol, and lidocaine.

On an unspecified date in 2011, the patient experienced abdominal pain and on 27-JUL-2011, she had jaundice. On 01-AUG-2011, the patient was hospitalized for liver failure with high transaminases, elevated international normalized ratio, hyperbilirubinemia and fatigue. The patient did not have encephalopathy. Baseline liver biopsy on 21-APR-2010 revealed liver fibrosis (Metavir score 1). On an unspecified date, a repeat of liver biopsy revealed signs of toxic necrosis due to ibuprofen and ibuprofen lysinate. Hepatitis C virus RNA was negative. On 02-AUG-2011, a liver biopsy and abdominal ultrasound abdomen were performed (results not provided). Treatment was not required for the event of liver failure.

Laboratory data was provided as follows: **On 05-AUG-2010:** alanine aminotransferase: 17 u/l (NR: 6 - 34), albumin 3.8 g/dl (NR: 3.3 - 4.9 g/dl), alkaline phosphatase 49 u/l (NR: 31 - 106), aspartate aminotransferase 19 u/l (NR: 9 - 34), bilirubin direct 0.1 mg/dl (NR: 0.0-0.4), bilirubin total 0.4 mg/dl (NR: 0.2 - 1.2 mg/dl) , international normalized ratio (INR) 0.9 (NR: 0.8 - 1.2), blood lactate dehydrogenase (LDH) 154 u/l (NR: 53 - 234). **On 12-MAY-2011:** alanine aminotransferase 31 u/l, albumin 3.4 g/dl, alkaline phosphatase 75 u/l, aspartate aminotransferase 47 u/l, bilirubin direct 0.1 mg/dl, bilirubin total 0.4 mg/dl, INR 0.9, LDH 142 u/l.

On 07-JUL-2011: alanine aminotransferase 39 u/l, albumin normal, alkaline phosphatase normal, aspartate aminotransferase 56 u/l, bilirubin direct , normal , bilirubin total normal, INR normal, LDH normal. **On 01-AUG-2011:** INR high. **On 08-AUG-2011:** alanine aminotransferase 361 u/l, albumin 2.4 g/dl, alkaline phosphatase 135 u/l, aspartate aminotransferase 295 u/l, bilirubin direct 10.6 mg/dl, bilirubin total 15.3 mg/dl, INR 1.1, LDH 241 u/l.

On 26-SEP-2011: alanine aminotransferase 237 u/l, albumin 2.9 g/dl, alkaline phosphatase 51 u/l, aspartate aminotransferase 179 u/l, bilirubin direct , 0.9 mg/dl, bilirubin total 1.5 mg/dl, INR 0.9, LDH 192. And on unspecified dates: alanine aminotransferase 10 u/l and aspartate aminotransferase 18 u/l.

The patient was discharged from the hospital on 04-AUG-2011 and was monitored on an outpatient basis. The reporter stated that the liver failure was not related to reinfection or study drugs ribavirin and investigational drug (PEG-r-IL-20) or peginterferon alfa-2a.

Treatment with ibuprofen was discontinued on an unspecified date in AUG-2011, treatment with ribavirin was discontinued on 06-JUL-2011, and treatment with investigational drug (PEG-r-IL-20) or peginterferon alfa-2a was discontinued on 30-JUN-2011. Action taken with ibuprofen lysinate was not reported. The patient had not yet recovered from the liver failure.

This report was serious (hospitalization).

IBUPROFEN (IBUPROFEN) Unspecified

Causality

1. HEPATIC FAILURE [10019663]

Change in dose Drug Withdrawn/Discontinued

Outcome after Change in dose Not Applicable

Outcome after Reintro. of dose Not Applicable

Causality as per reporter Possible

Causality as per Mfr. Possible

Seq. No. 2

Drug DOLORMIN (IBUPROFEN LYSINATE) Film Coated Tablet

Daily Dose 1)

2)

J&J Pharmaceutical Research & Development, LLC

100 Tournament Drive

Horsham, PA 19044

UNITED STATES

01-NOV-2011

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Continuation Sheet for CIOMS report Mfr. Control No.: DE-JNJFOC-20111011942

Indication 1) INFLUENZA LIKE ILLNESS

Therapy Dates/Duration 1) 22-AUG-2010 - 25-AUG-2010

2) ??-??-2011 - ??-??-???

Causality

1. HEPATIC FAILURE [10019663]

Change in dose Unknown

Outcome after Change in dose Unknown

Outcome after Reintro. of dose Unknown

Causality as per reporter Possible

Causality as per Mfr. Possible

Seq. No. 3

Drug RIBAVIRIN (RIBAVIRIN) Unspecified

Daily Dose 1) 1000 mg

Indication 1) HEPATITIS C

Route of Admin. 1) Unknown

Therapy Dates/Duration 1) 05-AUG-2010 - Stopped

Causality

1. HEPATIC FAILURE [10019663]

Change in dose Drug Withdrawn/Discontinued

Outcome after Change in dose

Outcome after Reintro. of dose

Causality as per reporter

Causality as per Mfr.

Seq. No. 4

Drug INVESTIGATIONAL DRUG (INVESTIGATIONAL DRUG)

Daily Dose 1)

Indication 1) HEPATITIS C

Therapy Dates/Duration 1) 05-AUG-2010 - Stopped

Causality

1. HEPATIC FAILURE [10019663]

Change in dose Drug Withdrawn/Discontinued

Outcome after Change in dose

Outcome after Reintro. of dose

Causality as per reporter

Causality as per Mfr.

Seq. No. 5

Drug PEGINTERFERON ALFA-2A (PEGINTERFERON ALFA-2A)

Daily Dose 1)

Indication 1) HEPATITIS C

Therapy Dates/Duration 1) 05-AUG-2010 - Stopped

Causality

1. HEPATIC FAILURE [10019663]

Change in dose Drug Withdrawn/Discontinued

Outcome after Change in dose

J&J Pharmaceutical Research & Development, LLC

100 Tournament Drive

Horsham, PA 19044

UNITED STATES

01-NOV-2011

Page 6 of 6

Continuation Sheet for CIOMS report Mfr. Control No.: DE-JNJFOC-20111011942

Causality

Outcome after Reintro. of dose

Causality as per reporter

Causality as per Mfr.

Concomitant Drugs (Cont...)

Seq. No. 2

Drug PASPERTIN (METOCLOPRAMIDE HYDROCHLORIDE)

Daily Dose 1)

Therapy Dates/Duration 1) 12-AUG-2010 - ??-??-????

Seq. No. 3

Drug PARACETAMOL (PARACETAMOL)

Daily Dose 1)

Therapy Dates/Duration 1) 05-AUG-2010 - ??-??-????

Seq. No. 4

Drug XYLOCAINE (LIDOCAINE)

Daily Dose 1)

Therapy Dates/Duration 1) 13-OCT-2010 - ??-??-????

Other relevant history (Cont...)

Medical History: HAEMORRHOIDS ??-??-???? - ??-??-????

Medical History: HEPATITIS C ??-??-???? - Ongoing

Medical History: INFLUENZA LIKE ILLNESS ??-??-???? - ??-??-????

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11228365

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19570515	Age: 54 Year	weight (kg): 53	BMI_int: 20.7	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatitis acute	20110521				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 COTRIM FORTE-RATIOPHARM sulfamethoxazole; trimethoprim	Oral	960 Mg milligram(s)/Day	20110513-20110515 3 Day	Urinary tract infection
2 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/	-	Pain
3 ibuprofen ibuprofen	Oral	600 Mg milligram(s)/	-	Arthrosis

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20110519	1273	UNK	
Alanine aminotransferase	20110521	1454	UNK	
Alanine aminotransferase	20110522	1527	UNK	
Alanine aminotransferase	20110523	1540	UNK	
Alkaline phosphatase	20110523	311	UNK	
Alkaline phosphatase	20110521	324	UNK	
Alkaline phosphatase	20110519	377	UNK	
Alkaline phosphatase	20110522	314	UNK	
Aspartate aminotransferase	20110521	895	UNK	
Aspartate aminotransferase	20110523	971	UNK	
Aspartate aminotransferase	20110519	625	UNK	
Aspartate aminotransferase	20110522	1130	UNK	
Gamma-glutamyltransferase	20110519	1573	UNK	
Gamma-glutamyltransferase	20110521	1369	UNK	
Gamma-glutamyltransferase	20110522	1252	UNK	
Gamma-glutamyltransferase	20110523	1258	UNK	
LDH	20110521	355	UNK	
LDH	20110522	350	UNK	
LDH	20110519	367	UNK	
LDH	20110523	267	UNK	
Glutamate dehydrogenase	20110522	54	UNK	
Glutamate dehydrogenase	20110521	44	UNK	

Glutamate dehydrogenase	20110519	51.8	UNK
Glutamate dehydrogenase	20110523	54.6	UNK

Narratives (nur bei elektronischem Eingang)sanofi-aventis, 05.12.2011 DE-SANOFI-AVENTIS-2011SA077596

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR was extracted from the website of the German health authority (authority number: DE-BFARM-11228365) on 25-Nov-2011, initially reported by a physician to the Drug Commission of the German Medical Association (case id: DE-DCGMA-11155861).

The case involves a 54-year-old female patient whose medical history was not reported.

The patient was treated with ibuprofen (trade name not provided) in the dose of 600 mg as needed for arthrosis and unclear diffuse pain and with sulfamethoxazole+trimethoprim 960 mg daily for urinary tract infection from 13-May-2011 to 15-May-2011. Start date of ibuprofen and concomitant medication were not reported.

On 21-May-2011 the patient developed acute hepatitis (drug-toxic, histologically confirmed) which required/prolonged hospitalization. Corrective treatments were not provided.

The ibuprofen therapy was discontinued on an unspecified date.

Outcome of the event was reported as "not recovered/not resolved".

Causality determined by the Drug Commission (method: WHO Global Introspection):

"possible" for ibuprofen and sulfamethoxazole+trimethoprim.

sanofi-aventis, 09.03.2012 DE-SANOFI-AVENTIS-2011SA077596

Sender diagnosis: not reported

Narrative

Initial report: This ICSR was extracted from the website of the German health authority (authority number: DE-BFARM-11228365) on 25-Nov-2011, initially reported by a physician to the Drug Commission of the German Medical Association (case id: DE-DCGMA-11155861) and concerns a 54-year-old female patient who experienced hepatitis acute (drug-toxic, histologically confirmed) whilst receiving Ibuprofen (trade name not provided) for arthrosis and unclear diffuse pain and sulfamethoxazole/trimethoprim (Cotrim Forte) for urinary tract infection.

On an unknown date, the patient commenced therapy with oral ibuprofen at a dose of 600mg as needed (strength, form, batch no and expiry date unspecified) and from 13-May-2011 to 15-May-2011, with oral sulfamethoxazole/trimethoprim at a dose of 960 mg daily (form batch no and expiry date unspecified). On 19 May 2011, her aspartate aminotransferase (ASAT) was 625 (normal range and units unspecified), alanine aminotransferase was 1273 (normal range and units unspecified), glutamate dehydrogenase (GLDH) was 51.8 (normal range and units unspecified), gamma-glutamyltransferase (GGT) was 1573 (normal range and units unspecified), alkaline phosphatase (AP) was 377 (normal range and units unspecified) and lactate dehydrogenase (LDH) was 367 (normal range and units unspecified). On 21-May-2011, the patient developed acute hepatitis (drug-toxic, histologically confirmed) which required prolonged hospitalization. On the same day, her ASAT was 895 (normal range and units unspecified), alanine aminotransferase was 1454 (normal range and units unspecified), GLDH was 44 (normal range and units unspecified), GGT was 1369 (normal range and units unspecified), AP was 324 (normal range and units unspecified) and LDH was 355 (normal range and units unspecified). As a corrective conduct prednisolone was taken. On 22 May 2011, her ASAT was 1130 (normal range and units unspecified), alanine aminotransferase was 1527 (normal range and units unspecified), GLDH was 54 (normal range and units unspecified), GGT was 1252 (normal range and units unspecified), AP was 314 (normal range and units unspecified) and LDH was 350 (normal range and units unspecified). On 23 May 2011, her ASAT was 971 (normal range and units unspecified), alanine aminotransferase was 1540 (normal range and units unspecified), GLDH was 54.6 (normal range and units unspecified), GGT was 1258 (normal range and units unspecified), AP was 311 (normal range and units unspecified) and LDH was 267 (normal range and units unspecified). On an unspecified date, therapy with ibuprofen was discontinued and the event of hepatitis acute (drug-toxic, histologically confirmed) was persisting.

No medical history, concomitant or past medication was reported.

Causality determined by the Drug Commission (method: WHO Global Introspection): "possible" for ibuprofen and sulfamethoxazole+trimethoprim.

Additional information was received on 28 Feb 2012:

Patient details, laboratory data and corrective treatment was provided and narrative updated.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11242475

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19420714	Age: 68 Year	weight (kg): 62	BMI_int: 22.77	Age fetus:
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Patientdeath

deathdate: 20110721	deathcause: Hepatic failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Icterus					fatal
2 Hepatic failure	201106				fatal
3 Overdose					Unknown
4 Drug use for unapproved indication					Unknown
5 Protein-losing gastroenteropathy	201106				fatal
6 Hepatic coma	201106				fatal
7 Cholestasis	201106				fatal
8 Hepatic encephalopathy	201106				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	1800 Mg milligram(s)/Day	-20110629	Metastatic bone pain
ibuprofen			-	

Drugs (concomitant)

2 novalgine	Oral	60 Gtt drop(s)/Day	-20110701	Metastatic bone pain
metamizole sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Breast operation			No	
Breast cancer metastatic	201105		Yes	
Bone metastases			Yes	
Radiation therapy	201106		Yes	

Primary source

Qualification: Physician	observestudytype:
Literature:	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin	20110629	40	MG/DL	
Transaminases	20110629	INCREASED	NA	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 20.12.2011 DE-SZ09-CHPA2011DE018207

Sender diagnosis: not reported

Narrative

Initial report received from a physician via BfArM (reference number DE-BFARM-11242475) on 15 Dec 2011. Physician reports that a 68-year-old female patient with a history of metastatic breast cancer, experienced intra-parenchymal cholestasis, hepatic coma, hepatic encephalopathy, hepatic failure, and severe protein-deficiency oedema, after treatment with Ibuprofen (ibuprofen), for an unknown duration, daily dose of "3 x 600mg". The outcome of the events was reported as death on an unknown date. It was unknown if the product was received. It was unknown if the product was used under medical advice for metastatic bone pain. The product was discontinued on 29 Jun 2011. Additional information is not available.

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (death) confirmed by a health care professional. The adverse events, Cholestasis and all associated symptoms, and Protein-losing gastroenteropathy, were assessed as unlisted according to the Reference Safety Information. It is Novartis policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as possible outcome of the listed risks. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

sanofi-aventis, 22.12.2011 DE-SANOFI-AVENTIS-2011SA082453

Sender diagnosis: not reported

Narrative

Initial report:

This ICSR was extracted from the website of the German health authority (authority number: DE-BFARM-11242475) on 16-Dec-2011, initially reported by a physician to the Drug Commission of the German Medical Association (case id: DE-DCGMA-11156293).

The case involves a 68-year-old female patient with breast cancer metastasizing to bone (first diagnosed in May 2011). She underwent operation and received radiation since June 2011.

The patient was treated with ibuprofen (trade name not provided) 1800 mg daily and metamizole sodium 60 drops daily for pain due to bone metastases. Start dates of the medications were not provided.

In June 2011 the patient developed hepatic failure, intraparenchymal cholestasis, hepatic coma, hepatic encephalopathy and severe protein deficiency edema (coded as protein-losing gastroenteropathy by BfArM).

The ibuprofen therapy was discontinued on 29-Jun-2011 and metamizole sodium was withdrawn on 01-Jul-2011. Corrective treatments were not reported.

The patient died on 21-Jul-2011 (outcome of all the events: fatal). Autopsy was not performed. The reporter suspected ibuprofen as cause of the events.

Causality determined by the Drug Commission concerning hepatic failure, intraparenchymal cholestasis, hepatic coma, hepatic encephalopathy and severe protein deficiency edema (method: WHO Global Introspection): "possible" for ibuprofen.

sanofi-aventis, 29.02.2012 DE-SANOFI-AVENTIS-2011SA082453

Sender diagnosis: not reported

Narrative

initial report:

This ICSR was extracted from the website of the German health authority (authority number: DE-BFARM-11242475) on 16-Dec-2011, initially reported by a physician to the Drug Commission of the German Medical Association (case id: DE-DCGMA-11156293).

The case involves a 68-year-old female patient with breast cancer metastasizing to bone (first diagnosed in May 2011). She underwent operation and received radiation since June 2011. The patient was treated with ibuprofen (trade name not provided) 600 mg at three times a day and metamizole sodium 20 drops daily at three times a day for pain due to bone metastases. Start dates of the medication were not provided.

On 29-Jun-2011 the patient was hospitalised because of increasing icterus for two weeks. She presented severe increased transaminases, intraparenchymal cholestasis (bilirubin 40 mg/dl), hepatic coma, hepatic encephalopathy and severe protein deficiency edema (coded as protein-losing gastroenteropathy by BfArM) albumin substituted. In the further course the patient was treated symptomatically (wash out of edemas caused by protein insufficiency) and received potassium substitution. The hepatic encephalopathy was treated with ornithin. A viral hepatitis could be excluded. The ibuprofen therapy was discontinued on the same day, and metamizole sodium was stopped on 01-Jul-2011. During the hospitalization, urinary tract infection (escherichia coli) was detected and treated with cefuroxime.

On 21-Jul-2011 the patient died due to subacute liver failure. Autopsy was not performed. The reporter suspected toxicity caused by ibuprofen as cause of the events.

Causality determined by the Drug Commission concerning hepatic failure, intraparenchymal cholestasis, hepatic coma, hepatic encephalopathy and severe protein deficiency edema (method: WHO Global Introspection): "possible" for ibuprofen.

Additional information received on 27-Feb-2012:

The original document for the initial data collection and the hospital report with the course until patient's death were provided by the Drug Commission of the German Medical Association. Therefore patients birth date and initials, further concomitant disease, further symptom of main event, lab values and corrective therapy could be amended.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12006392

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: 19431105	Age: 67 Year	weight (kg): 86	BMI_int: 29.41	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Albuminuria					recovered/resolved
2 Anemia	201109				recovered/resolved
3 Immune haemolytic anaemia drug-induced	201109			0 Month	recovered/resolved
4 Urinary test abnormal					recovered/resolved
5 Sweating increased					recovered/resolved
6 Sleepiness					recovered/resolved
7 SGOT increased					recovered/resolved
8 Loss of weight					recovered/resolved
9 Leukocytosis					recovered/resolved
10 LDH increased					recovered/resolved
11 Lab test abnormal					recovered/resolved
12 Immune system disorder					recovered/resolved
13 Icterus					recovered/resolved
14 Haemoglobin decreased					recovered/resolved
15 Anorexia					recovered/resolved
16 Bilirubinaemia					recovered/resolved
17 Burping					recovered/resolved
18 Constipation					recovered/resolved
19 Creatinine increased					recovered/resolved
20 Diarrhoea					recovered/resolved
21 Fever					recovered/resolved
22 Haematuria					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen ibuprofen	Oral	800 Mg milligram(s)/	201109- -	Lumboischialgia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Yes	
Lumboischialgia			Unknown	Lumboischialgie re.

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin		5.3	MG/DL	
C-reactive protein		105	MG/DL	
Creatinine		2.16	MG/DL	
Hemoglobin		10.5	G/DL	
Hemoglobin		11.8	G/DL	

		DE-BFARM-12006392	
Hemoglobin		4.2	G/DL
Hemoglobin		9.4	G/DL
Hemoglobin	201109	7.1	G/DL
LDH		348	U/L
LDH		1.602	U/L
SGOT		124	U/L
Urinalysis		POSITIVE	NA
Urinalysis		POSITIVE	NA
White blood cell analysis		200000	NA
Red blood cell analysis		3.4	NA

Narratives (nur bei elektronischem Eingang)

sanofi-aventis, 25.01.2012 DE-SANOFI-AVENTIS-2012SA004207

Sender diagnosis: not reported

Narrative

Initial report:

On 20-Jan-2012 this case was extracted from the website of the German health authority (authority number DE-BFARM-12006392); documenting a case in which a physician had reported about a 67-year-old male patient who suffered concomitantly from arterial hypertension. Co-medication was not reported.

For lumboischialgia right the patient took ibuprofen (trade name not provided) 800 mg starting in Sep-2011.

In the same month the patient was hospitalised due to following symptoms: anorexia, loss of weight (5 kg within two weeks), burping, urinary test abnormal, icterus, sweating increased, fever, change between constipation and diarrhoea, sleepiness, bilirubinaemia, LDH increased, SGOT increased, haemoglobin decreased, anemia, leukocytosis, lab test abnormal, creatinine increased, haematuria, albuminuria and immune system disorder.

The complete medication was stopped and the patient received correctively cortisone and infusions. Subsequently the patient was recovering; however due to recurrent lumboischialgia the therapy with ibuprofen was restarted. The patient developed the same symptoms again. After permanent discontinuation of ibuprofen, treatment with infusion NOS as well as pain therapy with tilidine and naloxone the patient recovered at unspecified time point.

The life-threatening autoimmune hemolytic anemia induced by ibuprofen was considered as diagnosis.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12012030

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 20070304	Age: 4 Year	weight (kg): 18.75	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hypercholesterolaemia	20111225			5 Day	Unknown
2 Eczema	20111225			5 Day	Unknown
3 Ascites	20111225			5 Day	Unknown
4 Edema	20111225			5 Day	Unknown
5 Nephrotic syndrome					Unknown
6 Minimal change glomerulonephritis					Unknown
7 Glomerulosclerosis					Unknown
8 Proteinuria	20111225			5 Day	Unknown
9 Nephrotic syndrome	20111225			5 Day	not recovered/not resolved
10 Hypoproteinaemia	20111225			5 Day	Unknown
11 Hypoalbuminaemia	20111225			5 Day	Unknown
12 Hypertriglyceridaemia	20111225			5 Day	Unknown
13 Swelling of eyelid	20111225			5 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 nurofen	Oral	/	20111221-20120103	Influenza
ibuprofen		170	14 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Proteinuria		(>1.0 g/m2 x day)	NA	
Investigation		normal range	NA	
Investigation		normal range	NA	
Albumin	20120118	20.6	g/dl	
Albumin	20120107	21.7	g/dl	
Blood glucose	20120107	Normal	NA	
C-reactive protein	20120107	Normal	NA	
Cholesterol	20120107	354	mg/dl	
Cholesterol	20120118	511	mg/dl	
Electrolytes NOS abnormal	20120107	Normal	NA	
HDL	20120107	Normal	NA	
HDL	20120118	Normal	NA	
Hypercholesterolaemia	20120118	-	NA	
Hypertriglyceridemia	20120118	-	NA	
Hypoalbuminaemia	20120118	Normal	NA	

	DE-BFARM-12012030		
Hypoproteinaemia	20120118	-	NA
LDL	20120118	346	mg/dl
LDL	20120107	240	mg/dl
Proteinuria		(>1.0 g/m ² x d) NA	
Serum creatinine	20120118	0.24	mg/dl
Thrombocytes abnormal (NOS)	20120118	462	G/l
Triglycerides	20120107	195	mg/dl
Triglycerides	20120118	338	mg/dl
Urea	20120118	Normal	NA
Urea	20120107	Normal	NA
Urine analysis	20120107	protein 500mg/dl	NA
Urine analysis	20120118	protein 500mg/dl	NA
Protein total	20120107	3.8	g/dl
Protein total	20120118	4.2	g/dl
Erythrocytes decreased	20120107	250/μl	NA
Complement factor C3	20120118	Normal	NA
Complement factor C4	20120118	Normal	NA
Sonogram	20120118	Normal	NA
Sonogram	20120118	ascites	NA
Sonogram	20120107	Normal	NA
Investigation	20120118	Normal	NA
Investigation	20120118	250/μl	NA
Investigation	20120118	17.6	g/l
Investigation		normal range	NA
Investigation		normal range	NA
Investigation	20120118	85%	NA
Uric acid	20120107	Normal	NA

Narratives (nur bei elektronischem Eingang)

RBHC, 26.01.2012 DE-RB-033407-12

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 12-Jan-2012, Reference No: RBD-36416770.

Case reference number RB-33407-2012 is a spontaneous case report sent by a physician which refers to a male aged 4 years.

Suspect Drug: Nurofen Suspension.

Dosing: Unknown

This ICSR refers to a request of a paediatrician made by a telephone conversation with Reckitt Benckiser Germany. Additional case information was obtained by a telephone conversation with the attending hospital doctor on 16-JAN-2012. For this case a medical confirmation does exist. Please note the suspected drug "Nurofen Suspension" (ibuprofen suspension, NOS) is referred to as "Nurofen suspension" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Nephrotic syndrome
2. Edemata affecting the whole body
3. Swelling of eyelids
4. Proteinuria

NARRATIVE

A 4-year-old boy with an uneventful medical history was administered Nurofen suspension from 21-DEC-2011 to 03-JAN-2012 due to influenzal infect (common cold). On 25-DEC-2011 the parents observed for the first time swelling of the eyelids. On 7-JAN-2012 the boy was hospitalized due to the occurrence of edemata. The attending paediatrician reported that the boy suffered from edemata affecting the whole body and that he showed proteinuria. The hospital made the working diagnosis of a nephrotic syndrome and started pharmacotherapy with prednisolone. The father of the boy read in the SPC of Nurofen suspension about the risk of the development of a nephrotic syndrome and told this the attending physician. The reporting physician suspected initially a causal relationship between the adverse events and the administration of ibuprofen and wanted to get some more information about the frequency of occurrence of those adverse events. On 16-JAN-2012 a further attending physician stated that the edemata did not yet regress in spite of ongoing corticoid therapy and that the edemata appear in various clinical manifestations. However the time of treatment was up to the date of this report too short for a final assessment of the therapeutic success. She reported furthermore that in the meantime after discussion with nephrologists the attending hospital physicians doubt the suspected causal relationship with the administration of Nurofen suspension.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Unlikely. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Unlikely.

The company's assessment for the case was Possible and listed.

Case Outcome: Unknown

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00).

As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was medically confirmed; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Expected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

- 1.Nephrotic syndrome: possible
- 2.Edemata affecting the whole body: possible
- 3.Swelling of eyelids: possible
- 4.Proteinuria: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

- 1.Nephrotic syndrome: expected
- 2.Edemata affecting the whole body: expected
- 3.Swelling of eyelids: verbatim unexpected, however this is a typical symptom of a nephrotic syndrome, which is labelled; this labelling is assessed to cover the reported reaction sufficiently.
- 4.Proteinuria: verbatim unexpected, however this is a typical laboratory finding of nephrotic syndrome, which is labelled; this labelling is assessed to cover the reported reaction sufficiently.

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension

zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

The reported body edemata and the proteinuria observed in this case are typical clinical and laboratory findings of a nephrotic syndrome.

RBHC, 05.03.2012 DE-RB-033407-12

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 12-Jan-2012, Reference No: RBD-36416770.

Case reference number RB-33407-2012 is a spontaneous case report sent by a physician which refers to a male aged 4 years.

Suspect Drug: Nurofen Suspension.

Dosing: Unknown

This ICSR refers to a request of a paediatrician made by a telephone conversation with Reckitt Benckiser Germany. Additional case information was obtained by a telephone conversation with the attending hospital doctor on 16-JAN-2012. For this case a medical confirmation does exist. Please note the suspected drug "Nurofen Suspension" (ibuprofen suspension, NOS) is referred to as "Nurofen suspension" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Nephrotic syndrome
2. Edemata affecting the whole body
3. Swelling of eyelids
4. Proteinuria

NARRATIVE

A 4-year-old boy with an uneventful medical history was administered Nurofen suspension from 21-DEC-2011 to 03-JAN-2012 due to influenzal infect (common cold). On 25-DEC-2011 the parents observed for the first time swelling of the eyelids. On 7-JAN-2012 the boy was hospitalized due to the occurrence of edemata. The attending paediatrician reported that the boy suffered from edemata affecting the whole body and that he showed proteinuria. The hospital made the working diagnosis of a nephrotic syndrome and started pharmacotherapy with prednisolone. The father of the boy read in the SPC of Nurofen suspension about the risk of the development of a nephrotic syndrome and told this the attending physician. The reporting physician suspected initially a causal relationship between the adverse events and the administration of ibuprofen and wanted to get some more information about the frequency of occurrence of those adverse events. On 16-JAN-2012 a further attending physician stated that the edemata did not yet regress in spite of ongoing corticoid therapy and that the edemata appear in various clinical manifestations. However the time of treatment was up to the date of this report too short for a final assessment of the therapeutic success. She reported furthermore that in the meantime after discussion with nephrologists the attending hospital physicians doubt the suspected causal relationship with the administration of Nurofen suspension.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Unlikely. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Unlikely.

The company's assessment for the case was Possible and listed.

Case Outcome: Unknown

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00).

As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was medically confirmed; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Expected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

- 1.Nephrotic syndrome: possible
- 2.Edemata affecting the whole body: possible
- 3.Swelling of eyelids: possible
- 4.Proteinuria: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

- 1.Nephrotic syndrome: expected
- 2.Edemata affecting the whole body: expected
- 3.Swelling of eyelids: verbatim unexpected, however this is a typical symptom of a nephrotic syndrome, which is labelled; this labelling is assessed to cover the reported reaction sufficiently.
- 4.Proteinuria: verbatim unexpected, however this is a typical laboratory finding of nephrotic syndrome, which is labelled; this labelling is assessed to cover the reported reaction sufficiently.

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

The reported body edemata and the proteinuria observed in this case are typical clinical and laboratory findings of a nephrotic syndrome.

Report No 2 received from RB Germany, Date 23-Feb-2012, Reference No: RBD-36416770.

Case reference number RB-33407-2012 is a spontaneous case report sent by a physician which refers to a male aged 4 years.

Information added/updated:

Personal data: born on 04-MAR-2007, weight 18 kg

Reaction onset: 25-DEC-2011(oedema of the eyelid, then (no date) development of an nephrotic syndrome)

Diagnose confirmed: Nephrotic syndrome

Laboratory findings confirmed proteinuria (>1.0 g/m2 x day); renal parameters and blood picture were within normal range.

Course and therapy: transfer to another hospital, due to unsuccessful steroid therapy.

Causality assessment reporter: changed from unlikely to possible

The following drugs are known to have been taken by the patient prior to the event: NUROFEN SUSPENSION (NOS)(indication: DRUG USE FOR UNKNOWN INDICATION).

The patient has been exposed to the Nurofen Suspension [Ibuprofen] for influenzal infection according PIL. The patient was experienced with proteinuria, nephrotic syndrome, and edemata affecting the whole body and swelling of eyelids which began on 25-Dec-2011. The patient had a drug history of Nurofen Suspension NOS. In the ADR-report of 23-FEB-2012 the attending physician assessed the causal relationship between the administration of Nurofen suspension and the developed nephrotic syndrome with the related symptoms as possible. Additionally he stated that former administration of Nurofen suspension was well tolerated. Laboratory findings confirmed proteinuria (>1.0 g/m2 x day); renal parameters and blood picture were within normal range. The outcome was reported as unknown, because the boy had been transferred to another paediatric hospital (date unknown) due to unsuccessful steroid therapy. No further information was available.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Unknown

The company's causality for the case was Possible. Company evaluation:

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00). As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was medically confirmed; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system) Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL Expected

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge. RISK-BENEFIT-RATIO The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case. Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS The reported body edemata and the proteinuria observed in this case are typical clinical and laboratory findings of a nephrotic syndrome.

RBHC, 20.04.2012 DE-RB-033407-12

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 12-Jan-2012, Reference No: RBD-36416770.

Case reference number RB-33407-2012 is a spontaneous case report sent by a physician which refers to a male aged 4 years.

Suspect Drug: Nurofen Suspension.

Dosing: Unknown

This ICSR refers to a request of a paediatrician made by a telephone conversation with Reckitt Benckiser Germany. Additional case information was obtained by a telephone conversation with the attending hospital doctor on 16-JAN-2012. For this case a medical confirmation does exist. Please note the suspected drug "Nurofen Suspension" (ibuprofen suspension, NOS) is referred to as "Nurofen suspension" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Nephrotic syndrome
2. Edemata affecting the whole body
3. Swelling of eyelids
4. Proteinuria

NARRATIVE

A 4-year-old boy with an uneventful medical history was administered Nurofen suspension from 21-DEC-2011 to 03-JAN-2012 due to influenzal infect (common cold). On 25-DEC-2011 the parents observed for the first time swelling of the eyelids. On 7-JAN-2012 the boy was hospitalized due to the occurrence of edemata. The attending paediatrician reported that the boy suffered from edemata affecting the whole body and that he showed proteinuria. The hospital made the working diagnosis of a nephrotic syndrome and started pharmacotherapy with prednisolone. The father of the boy

read in the SPC of Nurofen suspension about the risk of the development of a nephrotic syndrome and told this the attending physician. The reporting physician suspected initially a causal relationship between the adverse events and the administration of ibuprofen and wanted to get some more information about the frequency of occurrence of those adverse events. On 16-JAN-2012 a further attending physician stated that the edemata did not yet regress in spite of ongoing corticoid therapy and that the edemata appear in various clinical manifestations. However the time of treatment was up to the date of this report too short for a final assessment of the therapeutic success. She reported furthermore that in the meantime after discussion with nephrologists the attending hospital physicians doubt the suspected causal relationship with the administration of Nurofen suspension.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Unlikely. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Unlikely.

The company's assessment for the case was Possible and listed.

Case Outcome: Unknown

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00).

As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was medically confirmed; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Expected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

- 1.Nephrotic syndrome: possible
- 2.Edemata affecting the whole body: possible
- 3.Swelling of eyelids: possible
- 4.Proteinuria: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

- 1.Nephrotic syndrome: expected
- 2.Edemata affecting the whole body: expected
- 3.Swelling of eyelids: verbatim unexpected, however this is a typical symptom of a nephrotic syndrome, which is labelled; this labelling is assessed to cover the reported reaction sufficiently.
- 4.Proteinuria: verbatim unexpected, however this is a typical laboratory finding of nephrotic syndrome, which is labelled; this labelling is assessed to cover the reported reaction sufficiently.

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

The reported body edemata and the proteinuria observed in this case are typical clinical and laboratory findings of a nephrotic syndrome.

Report No 2 received from RB Germany, Date 23-Feb-2012, Reference No: RBD-36416770.

Case reference number RB-33407-2012 is a spontaneous case report sent by a physician which refers to a male aged 4 years.

Information added/updated:

Personal data: born on 04-MAR-2007, weight 18 kg

Reaction onset: 25-DEC-2011(oedema of the eyelid, then (no date) development of an nephrotic syndrome)

Diagnose confirmed: Nephrotic syndrome

Laboratory findings confirmed proteinuria (>1.0 g/m2 x day); renal parameters and blood picture were within normal range.

Course and therapy: transfer to another hospital, due to unsuccessful steroid therapy.

Causality assessment reporter: changed from unlikely to possible

The following drugs are known to have been taken by the patient prior to the event: NUROFEN SUSPENSION (NOS)(indication: DRUG USE FOR UNKNOWN INDICATION).

The patient has been exposed to the Nurofen Suspension [Ibuprofen] for influenzal infection according PIL. The patient was experienced with proteinuria, nephrotic syndrome, and edemata affecting the whole body and swelling of eyelids which began on 25-Dec-2011. The patient had a drug history of Nurofen Suspension NOS. In the ADR-report of 23-FEB-2012 the attending physician assessed the causal relationship between the administration of Nurofen suspension and the developed nephrotic syndrome with the related symptoms as possible. Additionally he stated that former administration of Nurofen suspension was well tolerated. Laboratory findings confirmed proteinuria (>1.0 g/m2 x day); renal parameters and blood picture were within normal range. The outcome was reported as unknown, because the boy had been transferred to another paediatric hospital (date unknown) due to unsuccessful steroid therapy. No further information was available.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Unknown

The company's causality for the case was Possible. Company evaluation:

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00). As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was medically confirmed; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system) Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL Expected

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge. RISK-BENEFIT-RATIO The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case. Hence no safety relevant measure with regard to the status of the

marketing authorization of this medicinal product is currently deemed necessary.

REMARKS The reported body edemata and the proteinuria observed in this case are typical clinical and laboratory findings of a nephrotic syndrome.

Report No 3 received from RB Germany, Date 11-Apr-2012, Reference No: RBD-36416770.

Information Added/updated:

Dechallenge: No

Company Diagnosis updated: Eczema on the cheeks.

Adverse event Added: Severe and unselective Proteinuria, Hypercholesterolaemia, Hypertriglyceridaemia, Hypoproteinaemia, Hypoalbuminaemia, Ascites, Eczema on the cheeks.

Reporter's causality changed from Possible to Unlikely.

Patient's weight updated: 18.75 kg

Lab Data Added: Laboratory and examination findings on 07-JAN-2012

Blood picture, HDL-cholesterol, blood glucose, uric acid, urea, electrolytes and CRP within normal range

Cholesterol 354 mg/dl; triglycerides 195 mg/dl; LDL-cholesterol 240 mg/dl; total protein 3.8 g/dl; albumin 21.7 g/dl

Urine analysis: protein 500mg/dl, erythrocytes ca. 250/µl; others uneventful

24-h urine collection: protein 4076 mg/l ? 147 mg/m² body surface/h (08-JAN-2012)

Sonogram kidneys: normal

Laboratory and examination findings on 18-JAN-2012

Leucocytes 17.6 g/l; thrombocytes 462 G/l; neutrocytes 85 %; remaining blood picture normal

HDL-cholesterol; urea; electrolytes and BSR within normal range

Cholesterol 511 mg/dl; triglycerides 338 mg/dl; LDL-cholesterol 346 mg/dl; total protein 4.2 g/dl; serum creatinine

0.24 mg/dl, albumin 20.6 g/dl

ASL, C3-C-complement and C4-complement within normal range

Urine analysis: protein 500mg/dl, erythrocytes ca. 250/µl; others uneventful

24-h urine collection: protein 2267 mg/l ? 86 mg/m² body surface/h (19-JAN-2012)

Hypercholesterolaemia; hypertriglyceridaemia; hypoproteinaemia and hypoalbuminaemia

Sonogram kidneys: normal

Sonogram abdomen: ascites

On 10-APR-2012 the practice of the family doctor confirmed that the boy had tolerated former administration of Nurofen Suspension and other ibuprofen containing suspensions in DEC 2008, MAY 2009 and March 2011 very well. In the hospital discharge report of 20-JAN-2012 the attending physician gave the following information: on admission (07-JAN-2012) the 4-year-old boy, weight: 18.75 kg, showed eczema on the cheeks. For therapy of the edemata and the ascites, which occurred during the inpatient treatment, furosemide ("Lasix") was used.

On 20-JAN-2012 the boy was transferred to a university hospital with the diagnose of a nephrotic syndrome, due to unsuccessful prednisolone therapy ("Prednisolut") from 60 mg per day in the beginning on 09-JAN-2012, down to 40 mg per day at referral date on 20-Jan-2012) of the severe and unselective proteinuria.

On 11-APR-2012 the attending physician of the university hospital confirmed during a telephone conversation with the practice of the family doctor that a causal relationship between the nephrotic syndrome with all the related findings and the ibuprofen therapy was excluded. The pathogenesis of the nephrotic syndrome was considered as most likely caused by a genetic factor, favored by a corresponding genetic familial disposition. Additionally the permission for any further administration of ibuprofen, as well as of paracetamol was given. The nephrotic syndrome is ongoing; the boy is under immunosuppressive therapy with ciclosporin. Up to the date of this report the discharge record of the university hospital where the boy was referred to on 20-JAN-2012 was not available. No further information was available.

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00).

As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was investigated by several physicians; the obtained case information is assessed as sufficient for a conclusive medical case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Unlikely

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Unexpected (This expectedness assessment only refers to the eczema on the cheeks, which is not at all in the focus of the clinical picture of this patient, suffering from a nephrotic syndrome.)

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

1.Nephrotic syndrome: unlikely

2.Edemata affecting the whole body: unlikely

- 3.Swelling of eyelids: unlikely
- 4.Severe and unselective proteinuria: unlikely
- 5.Hypercholesterolaemia: unlikely
- 6.Hypertriglyceridaemia : unlikely
- 7.Hypoproteinaemia: unlikely
- 8.Hypoalbuminaemia: unlikely
- 9.Ascites: unlikely
- 10.Eczema on the cheeks: unlikely

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

- 1.Nephrotic syndrome: expected
- 2.Edemata affecting the whole body: expected (in the context of a nephrotic syndrome as a typical symptom)
- 3.Swelling of eyelids: expected (in the context of a nephrotic syndrome as a typical symptom)
- 4.Severe and unselective proteinuria: expected (in the context of a nephrotic syndrome as a typical laboratory finding)
- 5.Hypercholesterolaemia: expected (in the context of a nephrotic syndrome as a typical laboratory finding)
- 6.Hypertriglyceridaemia : expected (in the context of a nephrotic syndrome as a typical laboratory finding)
- 7.Hypoproteinaemia: expected (in the context of a nephrotic syndrome as a typical laboratory finding)
- 8.Hypoalbuminaemia: expected (in the context of a nephrotic syndrome as a typical laboratory finding)
- 9.Ascites: expected (as a finding in the context of a nephrotic syndrome)
- 10.Eczema on the cheeks: unexpected

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

A causal relationship between the nephrotic syndrome with all the related findings and the ibuprofen therapy was excluded. The pathogenesis of the nephrotic syndrome was considered as most likely caused by a genetic factor, favoured by a corresponding genetic familial disposition.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

NUROFEN SUSPENSION[Ibuprofen] was dechallenged but NEPHROTIC SYNDROME[Nephrotic syndrome] persisted.

The reporter's causality for the case is Unlikely. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Unlikely.

The company's assessment for the case was Unlikely and Unlisted.

Case Outcome: Unknown

RBHC, 11.09.2012 DE-RB-033407-12

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 12-Jan-2012, Reference No: RBD-36416770.

Case reference number RB-33407-2012 is a spontaneous case report sent by a physician which refers to a male aged 4 years.

Suspect Drug: Nurofen Suspension.

Dosing: Unknown

This ICSR refers to a request of a paediatrician made by a telephone conversation with Reckitt Benckiser Germany. Additional case information was obtained by a telephone conversation with the attending hospital doctor on 16-JAN-2012. For this case a medical confirmation does exist. Please note the suspected drug "Nurofen Suspension" (ibuprofen suspension, NOS) is referred to as "Nurofen suspension" in this ICSR.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

- 1. Nephrotic syndrome

2. Edemata affecting the whole body
3. Swelling of eyelids
4. Proteinuria

NARRATIVE

A 4-year-old boy with an uneventful medical history was administered Nurofen suspension from 21-DEC-2011 to 03-JAN-2012 due to influenzal infect (common cold). On 25-DEC-2011 the parents observed for the first time swelling of the eyelids. On 7-JAN-2012 the boy was hospitalized due to the occurrence of edemata. The attending paediatrician reported that the boy suffered from edemata affecting the whole body and that he showed proteinuria. The hospital made the working diagnosis of a nephrotic syndrome and started pharmacotherapy with prednisolone. The father of the boy read in the SPC of Nurofen suspension about the risk of the development of a nephrotic syndrome and told this the attending physician. The reporting physician suspected initially a causal relationship between the adverse events and the administration of ibuprofen and wanted to get some more information about the frequency of occurrence of those adverse events. On 16-JAN-2012 a further attending physician stated that the edemata did not yet regress in spite of ongoing corticoid therapy and that the edemata appear in various clinical manifestations. However the time of treatment was up to the date of this report too short for a final assessment of the therapeutic success. She reported furthermore that in the meantime after discussion with nephrologists the attending hospital physicians doubt the suspected causal relationship with the administration of Nurofen suspension.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case is Unlikely. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Unlikely.

The company's assessment for the case was Possible and listed.

Case Outcome: Unknown

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00).

As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was medically confirmed; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Expected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

- 1.Nephrotic syndrome: possible
- 2.Edemata affecting the whole body: possible
- 3.Swelling of eyelids: possible
- 4.Proteinuria: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

- 1.Nephrotic syndrome: expected
- 2.Edemata affecting the whole body: expected
- 3.Swelling of eyelids: verbatim unexpected, however this is a typical symptom of a nephrotic syndrome, which is labelled; this labelling is assessed to cover the reported reaction sufficiently.
- 4.Proteinuria: verbatim unexpected, however this is a typical laboratory finding of nephrotic syndrome, which is labelled; this labelling is assessed to cover the reported reaction sufficiently.

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

The reported body edemata and the proteinuria observed in this case are typical clinical and laboratory findings of a nephrotic syndrome.

Report No 2 received from RB Germany, Date 23-Feb-2012, Reference No: RBD-36416770.

Case reference number RB-33407-2012 is a spontaneous case report sent by a physician which refers to a male aged 4 years.

Information added/updated:

Personal data: born on 04-MAR-2007, weight 18 kg

Reaction onset: 25-DEC-2011(oedema of the eyelid, then (no date) development of an nephrotic syndrome)

Diagnose confirmed: Nephrotic syndrome

Laboratory findings confirmed proteinuria (>1.0 g/m2 x day); renal parameters and blood picture were within normal range.

Course and therapy: transfer to another hospital, due to unsuccessful steroid therapy.

Causality assessment reporter: changed from unlikely to possible

The following drugs are known to have been taken by the patient prior to the event: NUROFEN SUSPENSION (NOS)(indication: DRUG USE FOR UNKNOWN INDICATION).

The patient has been exposed to the Nurofen Suspension [Ibuprofen] for influenzal infection according PIL. The patient was experienced with proteinuria, nephrotic syndrome, and edemata affecting the whole body and swelling of eyelids which began on 25-Dec-2011. The patient had a drug history of Nurofen Suspension NOS. In the ADR-report of 23-FEB-2012 the attending physician assessed the causal relationship between the administration of Nurofen suspension and the developed nephrotic syndrome with the related symptoms as possible. Additionally he stated that former administration of Nurofen suspension was well tolerated. Laboratory findings confirmed proteinuria (>1.0 g/m2 x day); renal parameters and blood picture were within normal range. The outcome was reported as unknown, because the boy had been transferred to another paediatric hospital (date unknown) due to unsuccessful steroid therapy. No further information was available.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Unknown

The company's causality for the case was Possible. Company evaluation:

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00). As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was medically confirmed; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system) Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL Expected

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge. RISK-BENEFIT-RATIO The assessment

of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case. Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS The reported body edemata and the proteinuria observed in this case are typical clinical and laboratory findings of a nephrotic syndrome.

Report No 3 received from RB Germany, Date 11-Apr-2012, Reference No: RBD-36416770.

Information Added/updated:

Dechallenge: No

Company Diagnosis updated: Eczema on the cheeks.

Adverse event Added: Severe and unselective Proteinuria, Hypercholesterolaemia, Hypertriglyceridaemia, Hypoproteinaemia, Hypoalbuminaemia, Ascites, Eczema on the cheeks.

Reporter's causality changed from Possible to Unlikely.

Patient's weight updated: 18.75 kg

Lab Data Added: Laboratory and examination findings on 07-JAN-2012

Blood picture, HDL-cholesterol, blood glucose, uric acid, urea, electrolytes and CRP within normal range

Cholesterol 354 mg/dl; triglycerides 195 mg/dl; LDL-cholesterol 240 mg/dl; total protein 3.8 g/dl; albumin 21.7 g/dl

Urine analysis: protein 500mg/dl, erythrocytes ca. 250/μl; others uneventful

24-h urine collection: protein 4076 mg/l ? 147 mg/m² body surface/h (08-JAN-2012)

Sonogram kidneys: normal

Laboratory and examination findings on 18-JAN-2012

Leucocytes 17.6 g/l; thrombocytes 462 G/l; neutrocytes 85 %; remaining blood picture normal

HDL-cholesterol; urea; electrolytes and BSR within normal range

Cholesterol 511 mg/dl; triglycerides 338 mg/dl; LDL-cholesterol 346 mg/dl; total protein 4.2 g/dl; serum creatinine

0.24 mg/dl, albumin 20.6 g/dl

ASL, C3-C-complement and C4-complement within normal range

Urine analysis: protein 500mg/dl, erythrocytes ca. 250/μl; others uneventful

24-h urine collection: protein 2267 mg/l ? 86 mg/m² body surface/h (19-JAN-2012)

Hypercholesterolaemia; hypertriglyceridaemia; hypoproteinaemia and hypoalbuminaemia

Sonogram kidneys: normal

Sonogram abdomen: ascites

On 10-APR-2012 the practice of the family doctor confirmed that the boy had tolerated former administration of Nurofen Suspension and other ibuprofen containing suspensions in DEC 2008, MAY 2009 and March 2011 very well. In the hospital discharge report of 20-JAN-2012 the attending physician gave the following information: on admission (07-JAN-2012) the 4-year-old boy, weight: 18.75 kg, showed eczema on the cheeks. For therapy of the edemata and the ascites, which occurred during the inpatient treatment, furosemide ("Lasix") was used.

On 20-JAN-2012 the boy was transferred to a university hospital with the diagnose of a nephrotic syndrome, due to unsuccessful prednisolone therapy ("Prednisolut") from 60 mg per day in the beginning on 09-JAN-2012, down to 40 mg per day at referral date on 20-Jan-2012) of the severe and unselective proteinuria.

On 11-APR-2012 the attending physician of the university hospital confirmed during a telephone conversation with the practice of the family doctor that a causal relationship between the nephrotic syndrome with all the related findings and the ibuprofen therapy was excluded. The pathogenesis of the nephrotic syndrome was considered as most likely caused by a genetic factor, favored by a corresponding genetic familial disposition. Additionally the permission for any further administration of ibuprofen, as well as of paracetamol was given. The nephrotic syndrome is ongoing; the boy is under immunosuppressive therapy with ciclosporin. Up to the date of this report the discharge record of the university hospital where the boy was referred to on 20-JAN-2012 was not available. No further information was available.

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00).

As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was investigated by several physicians; the obtained case information is assessed as sufficient for a conclusive medical case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Unlikely

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Unexpected (This expectedness assessment only refers to the eczema on the cheeks, which is not at all in the focus of the clinical picture of this patient, suffering from a nephrotic syndrome.)

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

1.Nephrotic syndrome: unlikely

2.Edemata affecting the whole body: unlikely

3.Swelling of eyelids: unlikely

4.Severe and unselective proteinuria: unlikely

5.Hypercholesterolaemia: unlikely

6.Hypertriglyceridaemia : unlikely

7.Hypoproteinaemia: unlikely

8.Hypoalbuminaemia: unlikely

9.Ascites: unlikely

10.Eczema on the cheeks: unlikely

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

1.Nephrotic syndrome: expected

2.Edemata affecting the whole body: expected (in the context of a nephrotic syndrome as a typical symptom)

3.Swelling of eyelids: expected (in the context of a nephrotic syndrome as a typical symptom)

4.Severe and unselective proteinuria: expected (in the context of a nephrotic syndrome as a typical laboratory finding)

5.Hypercholesterolaemia: expected (in the context of a nephrotic syndrome as a typical laboratory finding)

6.Hypertriglyceridaemia : expected (in the context of a nephrotic syndrome as a typical laboratory finding)

7.Hypoproteinaemia: expected (in the context of a nephrotic syndrome as a typical laboratory finding)

8.Hypoalbuminaemia: expected (in the context of a nephrotic syndrome as a typical laboratory finding)

9.Ascites: expected (as a finding in the context of a nephrotic syndrome)

10.Eczema on the cheeks: unexpected

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

A causal relationship between the nephrotic syndrome with all the related findings and the ibuprofen therapy was excluded. The pathogenesis of the nephrotic syndrome was considered as most likely caused by a genetic factor, favoured by a corresponding genetic familial disposition.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

NUROFEN SUSPENSION[Ibuprofen] was dechallenged but NEPHROTIC SYNDROME[Nephrotic syndrome] persisted.

The reporter's causality for the case is Unlikely. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Unlikely.

The company's assessment for the case was Unlikely and Unlisted.

Case Outcome: Unknown

Report No 4 received from RB Germany, Date 29-Aug-2012, Reference No: RBD-36416770.

Adverse Event Added: Steroid resistant nephrotic syndrome, Minimal change glomerulopathy (MCNS, Non-excludable transition to focal segmental glomerulosclerosis.

Based on several hospital records the attending paediatrician confirmed the diagnosis of steroid resistant nephrotic syndrome at minimal change glomerulopathy (MCNS; histological finding of the biopsy of the kidney performed on 30-JAN-2012); transition to focal segmental glomerulosclerosis (FSGS; histological finding of the biopsy of the kidney performed on 30-JAN-2012) is non-excludable. Relapses should be expected. Suspicion of a genetic disposition as cause for the nephrotic syndrome could not be confirmed. Current health status of the boy: good condition, clinically in remission (no proteinuria) under treatment with "Ciclosporin A" (ciclosporin); treatment with steroids not longer necessary; Causal involvement of ibuprofen:

„very unlikely.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

NUROFEN SUSPENSION[Ibuprofen] was dechallenged but NEPHROTIC SYNDROME[Nephrotic syndrome] persisted.

The reporter's causality for the case is Unlikely. The reporting physician's clinical assessment for drug NUROFEN SUSPENSION[Ibuprofen] is as follows: serious; with a relatedness of Unlikely.

The company's assessment for the case was Unlikely and Unlisted.

Case Outcome: Unknown

INDIVIDUAL CASE ASSESSMENT

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Unlikely

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Expected

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

1. Steroid resistant nephrotic syndrome (diagnosis): expected
2. Minimal change glomerulopathy (MCNS): expected (histological finding in the context of a nephrotic syndrome)
3. Non-excludable transition to focal segmental glomerulosclerosis (FSGS): expected (histological finding in the context of a nephrotic syndrome)
4. Edemata affecting the whole body: expected (in the context of a nephrotic syndrome)
5. Swelling of eyelids: expected (in the context of a nephrotic syndrome)
6. Severe and unselective proteinuria: expected (in the context of a nephrotic syndrome)
7. Hypercholesterolaemia: expected (in the context of a nephrotic syndrome)
8. Hypertriglyceridaemia: expected (in the context of a nephrotic syndrome)
9. Hypoproteinaemia: expected (in the context of a nephrotic syndrome)
10. Hypoalbuminaemia: expected (in the context of a nephrotic syndrome)
11. Ascites: expected (in the context of a nephrotic syndrome)
12. Eczema on the cheeks: expected (in the context of a nephrotic syndrome)

CAUSALITY ASSESSMENT (Reporter)

„very unlikely“

REMARKS

None

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12020246

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth:	Age: 54 Year	weight (kg): 87	BMI_int: 31.2	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzyme increased	20111206				recovering/resolving
2 Cholecystolithiasis					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	/	201108-	Headache
ibuprofen			-	
2 LYRICA	Oral	300 Mg milligram(s)/Day	-	Anxiety disorder
pregabalin			-	
3 LYRICA	Oral	150 Mg milligram(s)/Day	20111128-	Anxiety disorder
pregabalin			-	
4 TRIMIPRAMINE	Oral	250 Mg milligram(s)/	201110-	Depression
trimipramine			-	
5 TRIMIPRAMINE	Oral	250 Mg milligram(s)/	-	Depression
trimipramine			-	

Drugs (concomitant)

6 PANTOPRAZOLE	40 Mg milligram(s)/	-	Drug use for unknown indication
pantoprazole		-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol use			Unknown	occasional
Hospitalization NOS	20110830	20111202	No	or psychiatric treatment
Major depression			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Gamma-glutamyltransferase	20110831	0.73	nmol/L (nai	
Alanine aminotransferase	20110831	0.27	nmol/L (nai	
Aspartate aminotransferase	20110831	033	nmol/L (nai	
Aspartate aminotransferase	20110929	0.33	nmol/L (nai	
Alanine aminotransferase	20110929	0.27	nmol/L (nai	
Gamma-glutamyltransferase	20110929	0.56	nmol/L (nai	
Gamma-glutamyltransferase	20111018	0.31	nmol/L (nai	
Aspartate aminotransferase	20111018	0.31	nmol/L (nai	
Alanine aminotransferase	20111018	0.20	nmol/L (nai	
Alanine aminotransferase	20111117	0.21	nmol/L (nai	
Aspartate aminotransferase	20111117	0.26	nmol/L (nai	

Gamma-glutamyltransferase	20111117	0.43	nmol/L (naï
Gamma-glutamyltransferase	20111206	4.76	nmol/L (naï
Aspartate aminotransferase	20111206	0.87	nmol/L (naï
Alanine aminotransferase	20111206	1.67	nmol/L (naï
Gamma-glutamyltransferase	20111213	5.66	nmol/L (naï
Glutamate dehydrogenase	20111213	48.5	IU/L (intern
Aspartate aminotransferase	20111213	0.99	nmol/L (naï
Alanine aminotransferase	20111213	2.32	nmol/L (naï
Glutamate dehydrogenase	20111221	8.70	IU/L (intern
Alanine aminotransferase	20111221	1.12	nmol/L (naï
Aspartate aminotransferase	20111221	0.57	nmol/L (naï
Gamma-glutamyltransferase	20111221	4.29	nmol/L (naï
Gamma-glutamyltransferase	20111229	4.11	nmol/L (naï
Alanine aminotransferase	20111229	0.97	nmol/L (naï
Aspartate aminotransferase	20111229	0.57	nmol/L (naï
Gamma-glutamyltransferase	20120110	3.13	nmol/L (naï
Alanine aminotransferase	20120110	0.75	nmol/L (naï
Aspartate aminotransferase	20120110	0.53	nmol/L (naï
Sonogram		see below	NA

Narratives (nur bei elektronischem Eingang)

McNeil Germany, Local Safety Officers, 07.02.2012 DE-JNJFOC-20120200868

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician via license partner Pfizer (2011312289) and concerns a 54 year old female from Germany.

The patient's weight was 87 kilograms and height 167 centimeters. The patient's medical history and concurrent conditions included: alcohol use (occasional), hospitalization for psychiatric treatment between 30-AUG-2011 and 02-DEC-2011, and major depression. The patient had a drug history of Cassadan (alprazolam), citalopram, lorazepam, venlafaxine, and Zyprexa (olanzapine). The patient had used ibuprofen in the past, it was unknown if it was well tolerated.

The patient was treated with ibuprofen (unspecified, oral) 400-600mg (has taken up to 1200mg a day as needed) initiated in AUG-2011 for headache. Non-company suspect drugs included: pregabalin for anxiety disorder and trimipramine for depression. Concomitant medications included pantoprazole for drug use for unspecified indication.

On an unspecified date, under therapy from trimipramine and pregabalin, the patient experienced liver parameters increased which was not further specified. Labs on 31-AUG-2011 included: aspartate aminotransferase (AST/SGOT) 0.33umol/L; alanine aminotransferase (ALT/SGPT) 0.27 umol/L; GGT (gamma-glutamyltransferase) 0.73 umol/L. In hospital treatment was switched to citalopram and pipamperone. In mid Oct-2011, citalopram was discontinued and treatment switched to trimipramine. Intermittently, the patient had received lorazepam. In Nov-2011, the patient had used up to 1200mg of ibuprofen as needed for strong headache several times per week. In the past, the patient had used 400-600mg ibuprofen. On 02-DEC-2011, the patient was discharged from the hospital. On 05-DEC-2011, the patient was admitted to a psychiatric day clinic. Routine lab tests on 06-DEC-2012 showed increased GGT (gamma-glutamyltransferase) 4.76 umol/L; SGPT 1.67 umol/L; and SGOT 0.87 umol/L. The patient had no other symptoms. The patient was told to stop using ibuprofen and use novaminsulfon instead if needed. Treatment with trimipramine, pantoprazole and pregabalin was continued. Abdominal sonography (15-DEC-2011): no findings except cholecystolithiasis currently without signs of inflammation or intrahepatic cholestasis. According to internal medicine specialist, the increased liver enzymes might be related to this finding. After stop of ibuprofen treatment, transaminases were decreasing. Outcome of the event was assessed as partially recovered. The reporter assessed the event as possibly related to ibuprofen but the causality as not assessable. Treatment with ibuprofen was withdrawn and action taken with pregabalin and trimipramine was not provided.

The patient was recovering from hepatic enzyme increased and cholecystolithiasis.

This report was serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12022536

Serious No	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other No	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: 20090610	Age: 30 Month	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Liver function tests raised	20111212	20120116	36 Day	6 Month	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBU-ratiopharm 4% Fiebersaft für Kinder ibuprofen	Oral	390 Mg milligram(s)/Day	20110612-20111212 6 Month	

Drugs (concomitant)

2 naproxen naproxen	Oral	250 Mg milligram(s)/Day	20111212-20120116 36 Day
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Still's disease			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12035138

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: 20090610	Age: 30 Month	weight (kg): 13.5	BMI_int: 15.61	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GOT increased	20111212	20120116	36 Day	7 Day	recovered/resolved
2 GPT increased	20111212	20120116	36 Day	7 Day	recovered/resolved
3 Gamma GT increased	20111212	20120116	36 Day	7 Day	recovered/resolved
4 LDH increased	20111212	20120116	36 Day	7 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibu-ratiopharm 4% Fiebersaft fuer Kinder ibuprofen	Oral	390 Mg milligram(s)/Day	20111206-20111212	Arthritis
			7 Day	

Drugs (concomitant)

2 naloxone naloxone	Oral	2*125 Mg milligram(s)/ 245	20111212-20120116	Arthritis
			36 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arthritis			Yes	
Juvenile idiopathic arthritis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Glutamic-oxaloacetic transferase	20111212	500	U/l	
GPT	20111212	676	U/l	
Gamma GT	20111212	39	U/l	
Glutamic-oxaloacetic transferase	20111213	299	U/l	
GPT	20111213	624	U/l	
Gamma GT	20111213	42	U/l	
LDH	20111213	402	U/l	
Glutamic-oxaloacetic transferase	20111215	243	U/l	
LDH	20111215	403	U/l	
Gamma GT	20111215	42	U/l	
GPT	20111215	394	U/l	
Gamma GT	20120116	15	U/l	
GPT	20120116	304	U/l	
Glutamic-oxaloacetic transferase	20120116	404	U/l	
LDH	20120116	256	U/l	

Narratives (nur bei elektronischem Eingang)

TEVA, 17.02.2012 DE-TEVA-321643GER

Sender diagnosis: not reported

Narrative

08-Feb-2012, Spontaneous Serious Report.

(DRUG COMMISSION OF GERMAN PHARMACISTS; 167202)

A Physician reported the case of a Male patient 2 Years of age who experienced GOT INCREASED, GPT INCREASED, GAMMA GT INCREASED, LDH INCREASED while on Ibu-ratiopharm 4% Fiebersaft fuer Kinder (IBUPROFEN) (Teva's drug).

At the time of the report the outcome of the AEs were: GOT INCREASED : recovered/resolved, GPT INCREASED : recovered/resolved, GAMMA GT INCREASED : recovered/resolved, LDH INCREASED : recovered/resolved.

Concomitant Medication: NALOXONE

Medical History: OLIGOARTHRITIS(Concomitant) ,SUSPECTED MORBUS STILL(Concomitant)

Action Taken with suspect Drugs: Ibu-ratiopharm 4% Fiebersaft fuer Kinder - Drug discontinued. Patient's antiphlogistic therapy was changed from Ibuprofen to Naloxone.

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events)).

Lab data available.

Teva Comment: Contribution of Ibuprofen to symptoms reported cannot be excluded, especially as symptoms obviously improved after discontinuation and change of therapy respectively. Nevertheless, contribution of other reasons and circumstances, per example the underlying disease itself, cannot be excluded as there are no lab data prior to ibuprofen application are available and no information available about all symptoms of M.Still present in this child.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12035238

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 32 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ALT increased					Unknown
2 Drug-induced liver disease					Unknown
3 AST increased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 FLUPIRTINE		600 Mg milligram(s)/Day	-	Pain
flupirtine			3 Day	
2 ibuprofen		/	-	Pain
ibuprofen			-	
3 METAMIZOL		/	-	Pain
metamizole magnesium			-	
4 PREGABALIN		/	-	
pregabalin			-	

Drugs (concomitant)

5 ENOXAPARIN	/	-
enoxaparin		-
6 INSULIN	/	-
insulin		-
7 POTASSIUM CITRATE	/	-
potassium citrate		-
8 promethazine	/	-
promethazine		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Guillain Barre syndrome			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Anderson N,Borlak J. Correlation versus causation? Pharmacovigilance of the analgesic flupirtine exemplifies the need for refined spontaneous ADR reporting. Plos One 2011 Oct;6:10:E25221.	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 21.02.2012 DE-Boehringer Ingelheim GmbH, Germany-2012-EU-00903GD

Sender diagnosis: not reported

Narrative

P12-01684

A 32-year-old man who was treated with metamizole, ibuprofen and flupirtine for pain, experienced drug-induced liver injury (DILI).

Due to the medical circumstances this case was rated serious: seriousness criterion 'other' (medically significant).

While hospitalized, a 32-year old male patient (case no. 1) diagnosed with Guillain Barre Syndrome had received flupirtine, ibuprofen, and metamizole as pain medication. Reported comedications were: pregabalin, insulin, promethazine, enoxaparin and potassium citrate. After 3 days of flupirtine administration (600 mg/day) he displayed increased ALT (4xULN) and AST levels (3xULN). Treatment with flupirtine and pregabalin was discontinued. With a latency of 6 days he was re-exposed to pregabalin and responded with massively increases in ALT (28xULN) and AST (24xULN) levels. Drug-induced liver injury - induced by pregabalin was rated certain.

This case was part of a report on the case of flupirtine, a centrally acting non-opioid analgesic. The plausibility and causality of 226 unselected, spontaneously reported hepatobiliary adverse drug reactions according to the adapted Bradford-Hill criteria, CIOMS score and WHO-UMC scales

were reevaluated. Only about 20% of the reported cases were probable or likely for flupirtine treatment, suggesting an incidence of flupirtine-related liver injury of 1: 100,000 when estimated prescription data are considered, or 0.8 in 10,000 on the basis of all 226 reported adverse drug reactions. Neither daily or cumulative dose nor duration of treatment correlated with markers of liver injury. In the majority of cases (151/226), an average of 3 co-medications with drugs known for their liver liability was observed that may well be causative for adverse drug reactions, but were reported under a suspected flupirtine ADR. This study highlights the need to improve the quality and standards of ADR reporting. This should be done with utmost care taking into account contributing factors such as concomitant medications including over-the-counter drugs, the medical history and current health conditions, in order to avoid unjustified flagging and drug warnings that may erroneously cause uncertainty among healthcare professionals and patients, and may eventually lead to unjustified safety signals of useful drugs with a reasonable risk to benefit ratio.

Author's opinion:

'The 226 cases also included cases, where comedications rather than concomitant diseases (hepatitis) were more likely to be responsible for the hepatobiliary ADR. Specifically, in case no.1 re-challenge with pregabalin led to a massive increase of serum transaminases. This case can therefore be rated as unlikely for flupirtine exposure to cause DILI.'

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12055826

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 62 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Acute liver failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure			56 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		/	-	Drug use for unknown indication
ibuprofen			-	indication

Primary source

Qualification: Physician observestudytype:

Literature: Hadem J, Tacke F, Bruns T, Langgartner J, Strnad P, Denk GU, et al. Etiologies and Outcomes of Acute Liver Failure in Germany. Clin Gastroenterol Hepatol. 2012; doi: 10.1016/j.cgh.2012.02.016

Qualification: Physician observestudytype:

Literature: Hadem J, Tacke F, Bruns T, Langgartner J, Strnad P, Denk GU, et al. Etiologies and Outcomes of Acute Liver Failure in Germany. Clin Gastroenterol Hepatol. 2012;doi: 10.1016/j.cgh.2012.02.016

Qualification: Other Health Professional observestudytype:

Literature: Hadem J, Tacke F, Bruns T, Langgartner J, Strnad P, Denk G, et al. Etiologies and Outcomes of Acute Liver Failure in Germany Clin Gastroenterol Hepatol. 2012;.

Narratives (nur bei elektronischem Eingang)

drreddys, 16.03.2012 DE-drreddys-GER/GER/12/0023496

Sender diagnosis: not reported

Narrative

This is a report from a literature describing a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF). This case pertains to a 62-year-old female patient (case 21 in article) who died due to acute liver failure (ALF) after ibuprofen administration for an unknown indication.

The patient's medical history and concomitant medications were not reported.

On an unspecified date, the patient started treatment with non-paracetamol drug-induced ibuprofen (at an unknown dose, route and frequency). No co-morbidities were reported. On an unspecified date, the patient developed acute liver failure. Although the patient underwent emergency liver transplantation (ELT) on day six of hospitalisation, she died. The patient underwent emergency liver transplantation (ELT) on day five of hospitalisation days between emergency liver transplantation and death were 51. Days between referral and death were 56. The patient was one of 20 patients who died after ALF on an unknown date.

It was reported that eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

The authors stated that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate 824%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

The reporter assessed the case as serious due to fatal outcome.

Literature source: Hadem J, Tacke F, Bruns T, Langgartner J, Strnad P, Denk GU, et al. Etiologies and Outcomes of Acute Liver Failure in Germany.

Please also refer other cases (GER/GER/12/0023494, GER/GER/12/0023493 and GER/GER/12/0023495)

Merz Pharmaceuticals GmbH, 20.03.2012 DE-Merz Pharmaceuticals GmbH-12-0779

Sender diagnosis: not reported

Narrative

This literature report concerns a 62-year-old female patient (case 21 in article) who developed acute liver failure (ALF) after ibuprofen administration. She was identified in a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF). Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

Case 21 in article: in a 62-year old female patient, the suspected ALF aetiology was non-paracetamol drug-induced (ibuprofen). No co-morbidities were reported. The patient underwent emergency liver transplantation (ELT) on day five of hospitalisation. Days between emergency liver transplantation and death were 51. Days between referral and death were 56. The patient was one of 10 patients who underwent ELT, but died after surgery.

The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate (24%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

however, as the event was fatal, the case is assessed as unlisted. As the event is known for ibuprofen and the authors report ibuprofen as ALF aetiology and alternative explanations are missing, causality is assessed as possibly.

Reporter's comment:

Dr. R. Pflieger Chemische Fabrik GmbH, 21.03.2012 DE-PFLEGER-001677

Sender diagnosis: not reported

Narrative

In a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF), a 62-year-old female patient (case 21 in article) was identified who developed acute liver failure (ALF) after ibuprofen administration. Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

Case 21 in article: in a 62-year old female patient, the suspected ALF aetiology was non-paracetamol drug-induced (ibuprofen). No co-morbidities were reported. Although the patient underwent emergency liver transplantation (ELT) on day six of hospitalisation, she died. The patient underwent emergency liver transplantation (ELT) on day five of hospitalisation Days between emergency liver transplantation and death were 51. Days between referral and death were 56. The patient was one of 20 patients who died after ALF.<dbtext>To_Char_NARRATIVE_2</dbtext>

Reporter's comment:

The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate (24%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

MENARINIP, 23.03.2012 DE-MEN-25854

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, was presented within a publication concerning a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF). Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF. Thirty cases had a fatal outcome. Two of these patients had received ibuprofen and paracetamol, respectively.

This case involved patient number 21, a 62-year old female patient, with suspected ALF aetiology of ibuprofen ('non-paracetamol drug-induced'). The patient had no significant co-morbidities. Although she underwent emergency liver transplantation (ELT) on day five of hospitalisation, the woman died. Days between emergency liver transplantation and death were 51. Days between referral and death were 56.

The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate (24%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

This case was derived from the same publication as DE-MEN-25855 (paracetamol).

MERCKLE, 23.03.2012 DE-RECORDATI PHARMA GMBH-2012RC000949

Sender diagnosis: not reported

Narrative

In a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF), a 62-year-old female patient (case 21 in article) was identified who developed acute liver failure (ALF) after ibuprofen administration. Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

Case 21 in article: in a 62-year old female patient, the suspected ALF aetiology was non-paracetamol drug-induced (ibuprofen). No co-morbidities were reported. Although the patient underwent emergency liver transplantation (ELT) on day five of hospitalisation, she died. Days between emergency liver transplantation and death were 51. Days between referral and death were 56. The patient was one of 20 patients who died after ALF. The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate 24%, and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

Evaluation and Comment by Recordati Pharma GmbH:

Acute liver failure (ACUTE HEPATIC FAILURE):

Labelling reference document: listed

Company causality assessment: possible

Temporal relationship: plausible

Alternative explanation: unknown

Plausibility of adverse drug event: yes

Dechallenge information: n.a.

Rechallenge information: n.a.

Comment: Hepatic failure is a known and listed adverse event for ibuprofen. It should be noted that this case report lacks of crucial information for causality assessment, as there were no therapy dates or onset date of the adverse event provided as well as no information on the patient's underlying disease or concomitant medication. Nevertheless, while there were no data provided on a temporal relationship between ingestion of ibuprofen and the occurrence of acute liver failure, Recordati Pharma GmbH assessed company causality as possibly related in a conservative approach, since an association of the reported event to ibuprofen cannot be entirely excluded under the assumption of a plausible temporal relationship. No corrective action is considered necessary.

Overall documentation quality: poor

Risk / Benefit assessment: unchanged

Action to be taken by MAH: not necessary

Heumann Pharma GmbH & Co. Generika KG, 30.03.2012 DE-HEUMANN1-001790

Sender diagnosis: not reported

Narrative

This case has been identified in the literature.

In a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF), a 62-year-old female patient (case 21 in article) was identified who developed acute liver failure (ALF) after ibuprofen administration. Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

Case 21 in article: in a 62-year-old female patient, the suspected ALF aetiology was non-paracetamol drug-induced (ibuprofen). No co-morbidities were reported. Although the patient underwent emergency liver transplantation (ELT) on day six of hospitalisation, she died. The patient underwent emergency liver transplantation (ELT) on day five of hospitalisation Days between emergency liver transplantation and death were 51. Days between referral and death were 56. The patient was one of 20 patients who died after ALF.<dbtext>To_Char_NARRATIVE_2</dbtext>

Reporter's comment:

The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate 824%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.

Hennig Arzneimittel GmbH & Co KG, 27.04.2012 DE-HENNIG-000297

Sender diagnosis: not reported

Narrative

In a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF), a 62-year-old female patient (case 21 in article) was identified who developed acute liver failure (ALF) after ibuprofen administration. Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (SALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with SALI and 109 patients with ALF.

Case 21 in article: in a 62-year old female patient, the suspected ALF aetiology was non-paracetamol drug-induced (ibuprofen). No co-morbidities were reported. Although the patient underwent emergency liver transplantation (ELT) on day six of hospitalisation, she died. The patient underwent emergency liver transplantation (ELT) on day five of hospitalisation Days between emergency liver transplantation and death were 51. Days between referral and death were 56. The patient was one of 20 patients who died after ALF.

The authors state that the most frequent aetiologies of ALF were non-paracetamol drug-induced (32%), indeterminate 824%), and viral (21%); paracetamol ingestion was the cause of ALF in only 9% of patients. Patients with ALF are at great risk of progressing to multiple organ failure, but 80% of patients that receive emergency liver transplantation survive until discharge from the hospital.<dbtext>To_Char_NARRATIVE_2</dbtext>

Boehringer Ingelheim GmbH, Germany, 22.06.2012 DE-Boehringer Ingelheim GmbH, Germany-2012-EU-03163GD

Sender diagnosis: not reported

Narrative

P12-07087

A 62-year-old female patient (case 21, suppl. table 1) is reported who developed drug-induced acute liver failure (ALF) with hepatic encephalopathy due to use of ibuprofen. The patient underwent emergency liver transplantation on day 5 after referral and diagnosis of ALF and died 51 days after liver transplantation (day 56 after referral).

In 2010, all university clinics participating in the Acute Liver Failure Study Group Germany (including all major liver transplant centers) were asked to retrospectively identify patients with imminent and overt primary ALF treated at their clinics during a 2-year period (2008-2009).

Eleven university medical centers in Germany were asked to report patients with (primary) severe acute liver injury (SALI) (international normalized ratio (INR) >1.5 but no hepatic encephalopathy) and primary ALF (INR > 1.5 with overt hepatic encephalopathy) treated from 2008 to 2009. Data were analyzed from 46 patients with SALI and 109 patients with ALF.

Imminent ALF or severe acute liver injury (sALI) was defined as acute (< 12 weeks) liver dysfunction leading to an INR >1.5, but without documented level of hepatic encephalopathy (HE). Overt ALF was defined as acute (<12 weeks) liver dysfunction leading to an INR > 1.5 plus any documented level of HE.

Two hundred seventy-two patients were reported by the centers and assumed to have ALF. The remaining 46 patients with primary SALI and 109 patients with primary ALF were included in the analysis.

2 additional cases are available:

2012-EU-03166GD

2012-EU-03167GD

AXCOUNT, 04.07.2012 DE-AXCOUNT-2012-107

Sender diagnosis: not reported

Narrative

This case has been identified in the literature.

In a retrospective study, performed to investigate causes and outcomes of acute liver failure (ALF), a 62-year-old female patient (case 21 in article) was identified who developed acute liver failure (ALF) after ibuprofen administration. Eleven university medical centers in Germany were asked to report patients with primary severe acute liver injury (sALI) and primary ALF treated from 2008 to 2009. Data were analysed from 46 patients with sALI and 109 patients with ALF.

Case 21 in article: In a 62-year-old female patient, the suspected ALF aetiology was non-paracetamol drug-induced (ibuprofen). No co-morbidities were reported. Although the patient underwent emergency liver transplantation on day five of hospitalisation, she died. Days between emergency liver transplantation and death were 51. Days between referral and death were 56. The patient was one of 20 patients who died after ALF. Further data are not provided.

While no corresponding cases are contained in the AxCOUNT Safety database, one case report of fatal outcome of liver failure after ingestion of among other drugs ibuprofen (see below) has been retrieved from a timely unrestricted literature search in Medline on 20.06.2012. However this reaction was mainly attributed to colchicine in association with the co-ingested atorvastatin and propofol, and not with ibuprofen. Thus, since liver failure is listed no action is needed based on the relatively poorly reported patient from the retrospective study published by Hadem et al. (2012). The benefit/risk ratio of Ibuprofen axcount 200 mg/400 mg akut, Ibuprofen axcount 600 mg or Ibuprofen axcount 800 mg retard, respectively, is not affected by this case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12069738

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Female	Birth:	Age: 63 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		/	-	
ibuprofen			-	

Primary source

Qualification: Physician observestudytype:

Literature: Hadem J, Tacke F, Bruns T, Langgartner J, Stinad P, Denk GU, et al. Etiologies and Outcomes of Acute Liver Failure in Germany. Clin Gastroenterol Hepatol. 2012

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12065826

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19520829	Age: 55 Year	weight (kg): 99	BMI_int: 38.67	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oesophageal varices with bleeding	20080215	20080305	20 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clexane 40 enoxaparin	Subcutaneous	40 Mg milligram(s)/ Day 399	20080213-20080217 5 Day	Knee operation
2 ibuprofen ibuprofen	Oral	2*400 Mg milligram(s)/ Da 82	20080213-20080217 5 Day	Knee operation

Drugs (concomitant)

3 atenifbeta atenolol; nifedipine	Oral	Df dosage form/ Day 212	-20080219 -	Hypertension arterial
4 metformin metformin	Oral	850 Mg milligram(s)/ Day 82	1991-20080217 -	Diabetes mellitus without mention of complication
5 triamteren comp hydrochlorothiazide; triamterene	Oral	Df dosage form/ Day 82	-20080218 -	Hypertension arterial
6 venostasin horse chestnut extract; thiamine hydrochloride	Oral	50 Mg milligram(s)/ Day 212	-20080217 -	Venous insufficiency

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 30.03.2012 DE-PVZIBE-2-3643

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
OESOPHAGUSVARIZENBLUTUNG

KURZZUSAMMENFASSUNG:

Ösophagusvarizenblutung unter Heparin und Ibuprofen

BESCHREIBUNG:

seit 15.02.08: Oberbauchschmerzen, Übelkeit, Erbrechen und Teerstuhl;

seit 16.02.08 Hämatemesis;

Notarzt: RR 130/80 mmHg, Hf 110/min

Art des medikamentenbedingten Problems: UAW

THERAPIE:

Clexane, Ibuprofen am 17.02.08 abgesetzt;

17.02.08: Unacid 3x1.5 g i.v. über 3 Tage, dann Oralisierung, 2x40 mg Pantozol i.v. über 3 Tage, dann Oralisierung, 3x1 Eßl. Eugalac;

18.02.08: Transfusion von 2 Erythrozytenkonzentraten, langsamer Kostaufbau und zuckerfreie Kost;

VERLAUF:

Umstellung der antihypertensiven Therapie auf 2x25 mg Aldactone, 1x10 mg Xipamid, 2x12.5 Obsidan;

17.02.08 Gastroskopie: akute Ösophagusvarizenblutung, Ösophagusvarizen II-III°, Applikation von 5 Gummibändern mit Fibrinkleber, Blutung sistierte, Schleimhaut von Fundus, Corpus, Antrum unauffällig, Bulbus und tieferes Duodenum unauffällig;

19.02.08 Sonographie: toxische bzw. med.-toxische grobknotige Leberzirrhose;

26.02.08 Mikrobiologie Stuhl: kein Nachweis pathogener Keime/kein Nachweis von Clostridium difficile;

04.03.08 Leberbiopsie: subchron. Leberparenchymschaden;

kein Teerstuhl während des stationären Aufenthaltes, Hb-Anstieg von 6.9 auf 7.8 mmol/l;

Entlassung ohne Blutungszeichen

ERLÄUTERUNGEN ZUR UAW:

Clexane und Ibuprofen können Blutungen im GIT-Trakt verursachen oder verstärken. Beide Medikamente wurden seit 4 Tagen eingenommen. Ursache für die Blutung waren die Ösophagusvarizen bei Leberzirrhose (Erstdiagnose). Die PTT war verlängert, der Quick erniedrigt (siehe Labor), was mit den beschriebenen Leberschaden (Synthesedefekt) vereinbar ist. Die Ösophagusvarizenblutung hätte insofern auch ohne zu verdächtigende Arzneimittel zur Symptomatik und Krankenhausaufnahme führen können.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12065919

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19520829	Age: 55 Year	weight (kg): 99	BMI_int: 38.67	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oesophageal varices with bleeding	20080215	20080305	20 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clexane 40 enoxaparin	Subcutaneous	40 Mg milligram(s)/ Day 399	20080213-20080217 5 Day	Knee operation
2 ibuprofen ibuprofen	Oral	2*400 Mg milligram(s)/ Da 82	20080213-20080217 5 Day	Knee operation

Drugs (concomitant)

3 atenifbeta	Oral	Df dosage form/ Day 212	-20080219 -	Hypertension arterial
4 metformin metformin	Oral	850 Mg milligram(s)/ Day 82	1991-20080217 -	Diabetes mellitus without mention of complication
5 triamteren comp hydrochlorothiazide; triamterene	Oral	Df dosage form/ Day 82	-20080218 -	Hypertension arterial
6 venostasin horse chestnut extract; thiamine hydrochloride	Oral	50 Mg milligram(s)/ Day 212	-20080217 -	Venous insufficiency

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 30.03.2012 DE-PVZIBE-2-3643

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
OESOPHAGUSVARIZENBLUTUNG

KURZZUSAMMENFASSUNG:

Ösophagusvarizenblutung unter Heparin und Ibuprofen

BESCHREIBUNG:

seit 15.02.08: Oberbauchschmerzen, Übelkeit, Erbrechen und Teerstuhl;

seit 16.02.08 Hämatemesis;

Notarzt: RR 130/80 mmHg, Hf 110/min

Art des medikamentenbedingten Problems: UAW

THERAPIE:

Clexane, Ibuprofen am 17.02.08 abgesetzt;

17.02.08: Unacid 3x1.5 g i.v. über 3 Tage, dann Oralisierung, 2x40 mg Pantozol i.v. über 3 Tage, dann Oralisierung, 3x1 Eßl. Eugalac;

18.02.08: Transfusion von 2 Erythrozytenkonzentraten, langsamer Kostaufbau und zuckerfreie Kost;

VERLAUF:

Umstellung der antihypertensiven Therapie auf 2x25 mg Aldactone, 1x10 mg Xipamid, 2x12.5 Obsidan;

17.02.08 Gastroskopie: akute Ösophagusvarizenblutung, Ösophagusvarizen II-III°, Applikation von 5 Gummibändern mit Fibrinkleber, Blutung sistierte, Schleimhaut von Fundus, Corpus, Antrum unauffällig, Bulbus und tieferes Duodenum unauffällig;

19.02.08 Sonographie: toxische bzw. med.-toxische grobknotige Leberzirrhose;

26.02.08 Mikrobiologie Stuhl: kein Nachweis pathogener Keime/kein Nachweis von Clostridium difficile;

04.03.08 Leberbiopsie: subchron. Leberparenchymschaden;

kein Teerstuhl während des stationären Aufenthaltes, Hb-Anstieg von 6.9 auf 7.8 mmol/l;

Entlassung ohne Blutungszeichen

ERLÄUTERUNGEN ZUR UAW:

Clexane und Ibuprofen können Blutungen im GIT-Trakt verursachen oder verstärken. Beide Medikamente wurden seit 4 Tagen eingenommen. Ursache für die Blutung waren die Ösophagusvarizen bei Leberzirrhose (Erstdiagnose). Die PTT war verlängert, der Quick erniedrigt (siehe Labor), was mit den beschriebenen Leberschaden (Synthesedefekt) vereinbar ist. Die Ösophagusvarizenblutung hätte insofern auch ohne zu verdächtigende Arzneimittel zur Symptomatik und Krankenhausaufnahme führen können.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12067362

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: 19521025	Age: 55 Year	weight (kg): 69.6	BMI_int: 27.19	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20080428				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cymbalta duloxetine hydrochloride	Oral	60 Mg milligram(s)/Day 5	-20080523 -	Depression
2 ibuprofen ibuprofen	Oral	800 Mg milligram(s)/Day 82	2001-20080523 -	Back pain
3 katadolon flupirtine maleate	Oral	100 Mg milligram(s)/Day 5	2001-20080523 -	Back pain

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 30.03.2012 DE-PVZIBE-4-1536

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
LEBERWERTE ERHOEHT

KURZZUSAMMENFASSUNG:

Leberwerterhöhung unter Ibuprofen, Flupirtin, Duloxetin

BESCHREIBUNG:

Einweisung vom HA bei V.a. Cholelithiasis. Seit Ende April erhöhte Leberwerte. Erbrechen seit 4 Monaten, initial 4xtäglich, aktuell nur abends bis zum 18.05.2008. Ständig Übelkeit. Patientin fühlt sich schlapp und müde.

Ein Ikterus besteht nicht.

Art des medikamentenbedingten Problems: UAW

THERAPIE:

23.05.2008 Pantozol 40 1xd

ab 25.05.2008 3x20 gtt Novalgin bis 29.05.2008

ab 28.05.2008 Tramal long ret. 1xd zur Entlassung pausiert

ab 23.05.2008 Cymbalta abgesetzt

VERLAUF:

klin. Untersuchung Druckschmerzhaftigkeit re Oberbauch, keine Abwehrspannung, kein Loslaßschmerz, kein Ikterus
ambulant ergaben sich paraklinisch keine Hinweise auf Autoimmungenese, virale Genese, Hämochromatose oder Morbus Wilson

26.05.2008 ERCP: kein Nachweis einer Cholelithiasis, etwas erweiterter Gallengang nach CCE; regelrecht;

Ballondurchzug durch die Papille ergab keine Entleerung von Material

29.05.2008 psychiatr. Konsil: V.a. Somatisierungsstörung mittlerer Stärke, Schmerzmittelabhängigkeit (v.a.

Ibuprofen, Katadolon)

Angststörung mittlerer Stärke: stationäre Behandlung angeraten, Patientin wünscht das nicht.

Abklärung paraklinisch auffälliger Thrombozytopenie: Thrombozyten 75 (23.05.08)

30.05.2008 Beckenkampunktion. Histo: D: normozelluläres hämatopoetisches Mark. Vorliegender Befund lässt

an einen vermehrten peripheren Abbau der Thrombozyten bei klin. angegebener Thrombozytopenie

denken. Stationär kein Erbrechen.

E-Brief: Leberwerterhöhung, am ehesten medikament-toxisch induziert. Leberbiopsie soll ambulant vorgenommen werden, wenn Rückläufigkeit der Parameter stagniert.

ERLÄUTERUNGEN ZUR UAW:

Die Patientin wurde wegen unklarer Leberwerterhöhung zur Abklärung stationär eingewiesen. Nach Absetzen der Medikamente waren die Leberwerte langsam rückläufig, erreichten bis zum Zeitpunkt der Entlassung jedoch noch nicht den Normbereich. Aus klinischer Sicht wurde eine med.-tox. Ursache favorisiert, da sich für virale, metabolische und autoimmune Genese kein Anhalt bot. Aus klin.-pharmakologischer Sicht ist ein Zusammenhang möglich, jedoch weder der zeitliche Zusammenhang noch der symptomatologische (Alternativursachen: Infekt, andere Noxen) stichfest. Bei fragiler psychischer Situation der Patientin wären auch andere Noxen, die sie eventuell nicht erwähnt hat, denkbar.

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12076191

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: 20040127	Age: 5 Year	weight (kg): 23	BMI_int: 17.09	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaphylaxis	20100121				recovered/resolved
2 Quincke's oedema	20100121				recovered/resolved
3 Urticaria	20100121				recovered/resolved
4 Airways obstruction	20100121				recovered/resolved
5 Exanthema	20100121				recovered/resolved
6 Facial swelling	20100121				recovered/resolved
7 Icteric conjunctivae	20100121				recovered/resolved
8 Periorbital swelling	20100121				recovered/resolved
9 Pruritus	20100121				recovered/resolved
10 Stridor	20100121				recovered/resolved
11 Swelling of tongue	20100121				recovered/resolved
12 Tachycardia	20100121				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Nurofen Junior Fiebersaft Erdbeer 4% Suspension ibuprofen	Oral	/ 170	201001-20100120 -	Pain

Drugs (concomitant)

2 Flammazine creme sulfadiazine silver	Topical	/	201001- -	Burn
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Burn			Unknown	some days before 21-JAN-2010
Asthma			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

RBHC, 04.04.2012 DE-RB-037054-12

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 20-Mar-2012, Reference No: RBD-37706910.

Case reference number RB-37054-2012 is a spontaneous case report sent by a physician which refers to a patient of an unknown aged and sex.

The patient has been exposed to the Nurofen Fiebersaft Erdbeer administration of Nurofen suspension. The pediatrician told about a case of an intense allergic reaction (NOS) in connection with the administration of Nurofen suspension. The reporter further mentioned that the concerned child had been admitted to hospital (NOS). Additionally he stated that the mother of the child told him that her child did not show any reaction after another (later) administration of Nurofen suspension with orange flavor. No further information was available.

NOTE

Please note a summarizing evaluation of non-serious cases will be made in the PSUR of the concerned active substance. As reference product information for the following case assessment the currently approved German SPC of "Nurofen Junior Fieber- und Schmerzsaft Erdbeer 40 mg/ml Suspension zum Einnehmen" of SEP 2010 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

The present case information is assessed as insufficient for case evaluation, due to very poor and imprecise data.

CAUSALITY ASSESSMENT OF REPORTER

Not reported

SERIOUSNESS ON CASE-LEVEL

Non-serious

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Expected

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

Intense allergic reaction (NOS): expected

REMARKS

None

The outcome for INTENSE ALLERGIC REACTION (NOS)[Hypersensitivity] was Unknown.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN FIEBERSAFT ERDBEER [Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Unknown

Report No 2 received from RB Germany, Date 29-Mar-2012, Reference No: RBD-37706910.

Case reference number RB-37054-2012 is a spontaneous case report sent by a physician which refers to a male aged 5 years.

Information Added/Updated:

Seriousness Serious due to hospitalization for 3 days

Patient Boy 5 years; born: 27-JAN-2004; height: 116 cm, weight: 23 kg

Medical history- Asthma

Suspect drug- "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured); oral

Indication- Pain (due to burn of the left hand)

Concomitant drug: "Flammazine Creme" (silver sulfadiazine cream); topical

Indication: Burn of the left hand

Adverse Reactions

1- Anaphylaxis.

2- Quincke's Oedema

3- Urticaria

Reaction Onset: 21-Jan-2010.

Reaction Duration: 3 Days.

INTRODUCING COMMENT

This followup (1) ICSR refers to information obtained during a telephone conversation with the reporting paediatric practice on 29-MAR-2012.

Please note the suspected drug "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured) is referred to as "Nurofen suspension" in this ICSR. The followup information is listed below the narrative.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Anaphylaxis

2. Quincke's Oedema

3. Urticaria

On 21-JAN-2010 a 5-year-old boy (height: 116 cm; weight: 23 kg; born 27-JAN-2004) was hospitalized with an anaphylaxis, Quincke's Oedema and urticaria after the administration (duration and dose unknown) of Nurofen suspension due to pain, caused by a burn of his left hand, which he had experienced some days before (date unknown). The burn had been treated for several days (duration unknown) with silver sulfadiazine cream ("Flammazine Creme"). In the hospital the boy was treated with prednisolone and on 23-JAN-2010 he was released in a recovered condition. From his history asthma was known.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN JUNIOR FIEBERSAFT ERDBEER 4% SUSPENSION ZUM EINNEHMEN[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's causality for the case was Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Recovered / Resolved.

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00). As reference product information the currently approved German SPC of SEP 2010 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was confirmed by a physician; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT (Reporter)

Not reported

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Unexpected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

1. Anaphylaxis: possible
2. Quincke's Oedema: possible
3. Urticaria: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

1. Anaphylaxis: unexpected
2. Quincke's Oedema: unexpected
3. Urticaria: unexpected

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

Nurofen suspension with orange flavour was at later date well tolerated. Anaphylaxis and urticaria are known as ADRs of silver sulfadiazine. Therefore silver sulfadiazine should be considered as suspect drug too.

RBHC, 06.04.2012 DE-RB-037054-12

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 20-Mar-2012, Reference No: RBD-37706910.

Case reference number RB-37054-2012 is a spontaneous case report sent by a physician which refers to a patient of an unknown aged and sex.

The patient has been exposed to the Nurofen Fiebersaft Erdbeer administration of Nurofen suspension. The pediatrician told about a case of an intense allergic reaction (NOS) in connection with the administration of Nurofen suspension. The reporter further mentioned that the concerned child had been admitted to hospital (NOS). Additionally he stated that the mother of the child told him that her child did not show any reaction after another (later) administration of Nurofen suspension with orange flavor. No further information was available.

NOTE

Please note a summarizing evaluation of non-serious cases will be made in the PSUR of the concerned active substance. As reference product information for the following case assessment the currently approved German SPC of "Nurofen Junior Fieber- und Schmerzsaft Erdbeer 40 mg/ml Suspension zum Einnehmen" of SEP 2010 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

The present case information is assessed as insufficient for case evaluation, due to very poor and imprecise data.

CAUSALITY ASSESSMENT OF REPORTER

Not reported

SERIOUSNESS ON CASE-LEVEL

Non-serious

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Expected

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

Intense allergic reaction (NOS): expected

REMARKS

None

The outcome for INTENSE ALLERGIC REACTION (NOS)[Hypersensitivity] was Unknown.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN FIEBERSAFT ERDBEER [Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Unknown

Report No 2 received from RB Germany, Date 29-Mar-2012, Reference No: RBD-37706910.

Case reference number RB-37054-2012 is a spontaneous case report sent by a physician which refers to a male aged 5 years.

Information Added/Updated:

Seriousness Serious due to hospitalization for 3 days

Patient Boy 5 years; born: 27-JAN-2004; height: 116 cm, weight: 23 kg

Medical history- Asthma

Suspect drug- "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured); oral

Indication- Pain (due to burn of the left hand)

Concomitant drug: "Flammazine Creme" (silver sulfadiazine cream); topical

Indication: Burn of the left hand

Adverse Reactions

1- Anaphylaxis.

2- Quincke's Oedema

3- Urticaria

Reaction Onset: 21-Jan-2010.

Reaction Duration: 3 Days.

INTRODUCING COMMENT

This followup (1) ICSR refers to information obtained during a telephone conversation with the reporting paediatric practice on 29-MAR-2012.

Please note the suspected drug "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured) is referred to as "Nurofen suspension" in this ICSR. The followup information is listed below the narrative.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Anaphylaxis

2. Quincke's Oedema

3. Urticaria

On 21-JAN-2010 a 5-year-old boy (height: 116 cm; weight: 23 kg; born 27-JAN-2004) was hospitalized with an anaphylaxis, Quincke's Oedema and urticaria after the administration (duration and dose unknown) of Nurofen suspension due to pain, caused by a burn of his left hand, which he had

experienced some days before (date unknown). The burn had been treated for several days (duration unknown) with silver sulfadiazine cream ("Flammazine Creme"). In the hospital the boy was treated with prednisolone and on 23-JAN-2010 he was released in a recovered condition. From his history asthma was known.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN JUNIOR FIEBERSAFT ERDBEER 4% SUSPENSION ZUM EINNEHMEN[ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's causality for the case was Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Recovered / Resolved.

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00). As reference product information the currently approved German SPC of SEP 2010 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was confirmed by a physician; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT (Reporter)

Not reported

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Unexpected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

1. Anaphylaxis: possible
2. Quincke's Oedema: possible
3. Urticaria: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

1. Anaphylaxis: unexpected
2. Quincke's Oedema: unexpected
3. Urticaria: unexpected

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

Nurofen suspension with orange flavour was at later date well tolerated. Anaphylaxis and urticaria are known as ADRs of silver sulfadiazine. Therefore

silver sulfadiazine should be considered as suspect drug too.

Report No 3 received from RB Germany, Date 30-Mar-2012, Reference No: RBD-37706910.

Information Added/Updated:

New adverse reactions: Facial swelling, Periorbital swelling, Sensation of a swollen tongue, Tachycardia, Inspiratoric stridor, Prolonged exspirium, Exanthema in the neck, Pruritus, and Conjunctivae reddened.

Co-suspect drug: Silver sulfadiazine cream (Flammazine Creme) added.

Treatment: Cetirizine (NOS) Adrenaline inhalations (NOS) Prednisolone IV (NOS) Dimetindene IV (Fenistil) 1 l sodium chloride solution (0.9 % NaCl)

INTRODUCING COMMENT

This followup (2) ICSR refers to the discharge record of the hospital, obtained on 30-MAR-2012. Please note the suspected drug "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured) is referred to as "Nurofen suspension" in this ICSR.

On 21-JAN-2010 a 5-year-old boy (height: 116 cm; weight: 23 kg; born 27-JAN-2004) was hospitalized with an anaphylaxis showing tachycardia, inspiratoric stridor, prolonged exspirium, Quincke's Oedema and urticaria after the last administration (duration and dose unknown) of Nurofen suspension in the evening of 20-JAN-2010 due to pain, caused by a burn of his left palm, which he had experienced some days before (date unknown). The burn had been treated for several days (duration unknown) with silver sulfadiazine cream ("Flammazine Creme"). About 1 hour before the presentation in the emergency unit of the hospital the boy experienced exanthema on the neck, pruritus, reddened conjunctivae, facial and periorbital swelling and a sensation of a swollen tongue, which was treated at home with cetirizine (NOS). In the hospital the boy was treated with adrenaline inhalations (NOS), prednisolone IV (NOS), dimetindene IV ("Fenistil") and 1 l sodium chloride solution (0.9 % NaCl). Former administration of orange flavoured Nurofen suspension (NOS) had been well tolerated, therefore, in his discharge record, the attending physician of the hospital assumed the red colouring agent azorubine of the Nurofen suspension with strawberry flavour as the "most probable" cause of the anaphylactic reaction and ibuprofen as well as silver sulfadiazine as the "less probable" cause.

CAUSALITY ASSESSMENT (according to the hospital discharge report)

"Reaction most probably" caused by the colouring agent azorubine of the Nurofen suspension, "less probable" caused by ibuprofen or silver sulfadiazine.

VII.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN JUNIOR FIEBERSAFT ERDBEER 4% SUSPENSION ZUM EINNEHMEN [ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Recovered / Resolved.

INDIVIDUAL CASE ASSESSMENT

REFERENCE MEDICINAL PRODUCT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00). As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was confirmed by a physician; the obtained case information is assessed as sufficient for medical case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

CAUSALITY ASSESSMENT (Reporter)

Possible (for azorubine)

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Unexpected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

1. Anaphylaxis: possible
2. Quincke's Oedema: possible
3. Facial swelling: possible
4. Periorbital swelling: possible

5. Sensation of a swollen tongue: possible
6. Tachycardia: possible
7. Inspiratory stridor: possible
8. Prolonged exhalation: possible
9. Urticaria: possible
10. Exanthema on the neck: possible
11. Pruritus: possible
12. Conjunctivae reddened: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

1. Anaphylaxis: expected
2. Quincke's Oedema: expected (verbatim unexpected; however facial swelling, swelling of tongue and larynx, including bronchoconstriction are labelled; this labelling is assessed to cover the reported reaction sufficiently.)
3. Facial swelling: expected
4. Periorbital swelling: expected (verbatim unexpected; however facial swelling is labelled; this labelling is assessed to cover the reported reaction sufficiently.)
5. Sensation of a swollen tongue: expected (verbatim unexpected; however swelling of tongue is labelled; this labelling is assessed to cover the reported reaction sufficiently.)
6. Tachycardia: expected
7. Inspiratory stridor: expected (verbatim unexpected; however constriction of airways, dyspnoea and asthma attacks are expected; this labelling is assessed to cover the reported reaction sufficiently.)
8. Prolonged exhalation: expected (verbatim unexpected; however constriction of airways, dyspnoea and asthma attacks are expected; this labelling is assessed to cover the reported reaction sufficiently.)
9. Urticaria: unexpected
10. Exanthema on the neck: expected
11. Pruritus: expected
12. Conjunctivae reddened: unexpected

THE FOLLOWING STATEMENTS REFERS TO THE REFERENCE MEDICINAL PRODUCT
"Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen"

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

Former administration of orange flavoured Nurofen suspension (NOS) had been well tolerated, therefore the attending physician of the hospital assumed the red colouring agent azorubine of the Nurofen suspension with strawberry flavour as the most probable cause of the anaphylactic reaction whereas ibuprofen and silver sulfadiazine were assessed as less probable cause.

RBHC, 06.04.2012 DE-RB-037054-12

Sender diagnosis: not reported

Narrative

Report No 1 received from RB Germany, Date 20-Mar-2012, Reference No: RBD-37706910.

Case reference number RB-37054-2012 is a spontaneous case report sent by a physician which refers to a patient of an unknown aged and sex.

The patient has been exposed to the Nurofen Fiebersaft Erdbeer administration of Nurofen suspension. The pediatrician told about a case of an intense allergic reaction (NOS) in connection with the administration of Nurofen suspension. The reporter further mentioned that the concerned child had been admitted to hospital (NOS). Additionally he stated that the mother of the child told him that her child did not show any reaction after another (later) administration of Nurofen suspension with orange flavor. No further information was available.

NOTE

Please note a summarizing evaluation of non-serious cases will be made in the PSUR of the concerned active substance. As reference product information for the following case assessment the currently approved German SPC of "Nurofen Junior Fieber- und Schmerzsaft Erdbeer 40 mg/ml Suspension zum Einnehmen" of SEP 2010 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

The present case information is assessed as insufficient for case evaluation, due to very poor and imprecise data.

CAUSALITY ASSESSMENT OF REPORTER

Not reported

SERIOUSNESS ON CASE-LEVEL

Non-serious

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Expected

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

Intense allergic reaction (NOS): expected

REMARKS

None

The outcome for INTENSE ALLERGIC REACTION (NOS)[Hypersensitivity] was Unknown.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN FIEBERSAFT ERDBEER [Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Unknown

Report No 2 received from RB Germany, Date 29-Mar-2012, Reference No: RBD-37706910.

Case reference number RB-37054-2012 is a spontaneous case report sent by a physician which refers to a male aged 5 years.

Information Added/Updated:

Seriousness Serious due to hospitalization for 3 days

Patient Boy 5 years; born: 27-JAN-2004; height: 116 cm, weight: 23 kg

Medical history- Asthma

Suspect drug- "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured); oral

Indication- Pain (due to burn of the left hand)

Concomitant drug: "Flammazine Creme" (silver sulfadiazine cream); topical

Indication: Burn of the left hand

Adverse Reactions

1- Anaphylaxis.

2- Quincke's Oedema

3- Urticaria

Reaction Onset: 21-Jan-2010.

Reaction Duration: 3 Days.

INTRODUCING COMMENT

This followup (1) ICSR refers to information obtained during a telephone conversation with the reporting paediatric practice on 29-MAR-2012.

Please note the suspected drug "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum

Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured) is referred to as "Nurofen suspension" in this ICSR. The followup information is listed below the narrative.

REPORT TERMS AND DESCRIBE REACTIONS (including relevant test/lab data)

1. Anaphylaxis

2. Quincke's Oedema

3. Urticaria

On 21-JAN-2010 a 5-year-old boy (height: 116 cm; weight: 23 kg; born 27-JAN-2004) was hospitalized with an anaphylaxis, Quincke's Oedema and urticaria after the administration (duration and dose unknown) of Nurofen suspension due to pain, caused by a burn of his left hand, which he had experienced some days before (date unknown). The burn had been treated for several days (duration unknown) with silver sulfadiazine cream ("Flammazine Creme"). In the hospital the boy was treated with prednisolone and on 23-JAN-2010 he was released in a recovered condition. From his history asthma was known.

The reaction was deemed serious by the reporter because it resulted in prolonged hospitalization, required hospitalization. The patient was hospitalized.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN JUNIOR FIEBERSAFT ERDBEER 4% SUSPENSION ZUM EINNEHMEN[Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's causality for the case was Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Recovered / Resolved.

BASIS OF CASE ASSESSMENT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00). As reference product information the currently approved German SPC of SEP 2010 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was confirmed by a physician; the obtained case information is assessed as sufficient for case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT (Reporter)

Not reported

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Unexpected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

1. Anaphylaxis: possible
2. Quincke's Oedema: possible
3. Urticaria: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

1. Anaphylaxis: unexpected
2. Quincke's Oedema: unexpected
3. Urticaria: unexpected

THE FOLLOWING STATEMENTS REFER TO THE SUSPECTED MEDICINAL PRODUCT AND TO THE STATUS OF ITS MARKETING AUTHORIZATION.

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

Nurofen suspension with orange flavour was at later date well tolerated. Anaphylaxis and urticaria are known as ADRs of silver sulfadiazine. Therefore silver sulfadiazine should be considered as suspect drug too.

Report No 3 received from RB Germany, Date 30-Mar-2012, Reference No: RBD-37706910.

Information Added/Updated:

New adverse reactions: Facial swelling, Periorbital swelling, Sensation of a swollen tongue, Tachycardia, Inspiratoric stridor, Prolonged expirium,

Exanthema in the neck, Pruritus, and Conjunctivae reddened.

Co-suspect drug: Silver sulfadiazine cream (Flammazine Creme) added.

Treatment: Cetirizine (NOS) Adrenaline inhalations (NOS) Prednisolone IV (NOS) Dimetindene IV (Fenistil) 1 l sodium chloride solution (0.9 % NaCl)

INTRODUCING COMMENT

This followup (2) ICSR refers to the discharge record of the hospital, obtained on 30-MAR-2012. Please note the suspected drug "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured) is referred to as "Nurofen suspension" in this ICSR.

On 21-JAN-2010 a 5-year-old boy (height: 116 cm; weight: 23 kg; born 27-JAN-2004) was hospitalized with an anaphylaxis showing tachycardia, inspiratory stridor, prolonged expirium, Quincke's Oedema and urticaria after the last administration (duration and dose unknown) of Nurofen suspension in the evening of 20-JAN-2010 due to pain, caused by a burn of his left palm, which he had experienced some days before (date unknown). The burn had been treated for several days (duration unknown) with silver sulfadiazine cream ("Flammazine Creme"). About 1 hour before the presentation in the emergency unit of the hospital the boy experienced exanthema on the neck, pruritus, reddened conjunctivae, facial and periorbital swelling and a sensation of a swollen tongue, which was treated at home with cetirizine (NOS). In the hospital the boy was treated with adrenaline inhalations (NOS), prednisolone IV (NOS), dimetindene IV ("Fenistil") and 1 l sodium chloride solution (0.9 % NaCl). Former administration of orange flavoured Nurofen suspension (NOS) had been well tolerated, therefore, in his discharge record, the attending physician of the hospital assumed the red colouring agent azorubine of the Nurofen suspension with strawberry flavour as the "most probable" cause of the anaphylactic reaction and ibuprofen as well as silver sulfadiazine as the "less probable" cause.

CAUSALITY ASSESSMENT (according to the hospital discharge report)

"Reaction most probably" caused by the colouring agent azorubine of the Nurofen suspension, "less probable" caused by ibuprofen or silver sulfadiazine.

VII.

The reporter's causality for the case was Possible. The reporting physician's clinical assessment for drug NUROFEN JUNIOR FIEBERSAFT ERDBEER 4% SUSPENSION ZUM EINNEHMEN [Ibuprofen] is as follows: serious; with a relatedness of Possible.

The company's assessment for the case was Possible and Unlisted.

Case Outcome: Recovered / Resolved.

INDIVIDUAL CASE ASSESSMENT

REFERENCE MEDICINAL PRODUCT

The following case evaluation refers to the medicinal product "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" (oral suspension containing 40 mg ibuprofen per ml, strawberry flavoured, MA-No.: 57417.00.00). As reference product information the currently approved German SPC of MAR 2008 was used.

EXTEND AND QUALITY OF REPORTED CASE INFORMATION

This case was confirmed by a physician; the obtained case information is assessed as sufficient for medical case evaluation.

SERIOUSNESS ON CASE-LEVEL

Serious

CAUSALITY ASSESSMENT ON CASE-LEVEL (according to the WHO-UMC system)

Possible

CAUSALITY ASSESSMENT (Reporter)

Possible (for azorubine)

EXPECTEDNESS ASSESSMENT ON CASE-LEVEL

Unexpected

CAUSALITY ASSESSMENT ON EVENT-LEVEL (according to the WHO-UMC system)

1. Anaphylaxis: possible
2. Quincke's Oedema: possible
3. Facial swelling: possible
4. Periorbital swelling: possible
5. Sensation of a swollen tongue: possible
6. Tachycardia: possible
7. Inspiratory stridor: possible
8. Prolonged expirium: possible
9. Urticaria: possible
10. Exanthema on the neck: possible
11. Pruritus: possible

12. Conjunctivae reddened: possible

EXPECTEDNESS ASSESSMENT ON EVENT-LEVEL

1. Anaphylaxis: expected
2. Quincke's Oedema: expected (verbatim unexpected; however facial swelling, swelling of tongue and larynx, including bronchoconstriction are labelled; this labelling is assessed to cover the reported reaction sufficiently.)
3. Facial swelling: expected
4. Periorbital swelling: expected (verbatim unexpected; however facial swelling is labelled; this labelling is assessed to cover the reported reaction sufficiently.)
5. Sensation of a swollen tongue: expected (verbatim unexpected; however swelling of tongue is labelled; this labelling is assessed to cover the reported reaction sufficiently.)
6. Tachycardia: expected
7. Inspiratory stridor: expected (verbatim unexpected; however constriction of airways, dyspnoea and asthma attacks are expected; this labelling is assessed to cover the reported reaction sufficiently.)
8. Prolonged expiration: expected (verbatim unexpected; however constriction of airways, dyspnoea and asthma attacks are expected; this labelling is assessed to cover the reported reaction sufficiently.)
9. Urticaria: unexpected
10. Exanthema on the neck: expected
11. Pruritus: expected
12. Conjunctivae reddened: unexpected

THE FOLLOWING STATEMENTS REFERS TO THE REFERENCE MEDICINAL PRODUCT

"Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen"

CURRENT STATE OF SCIENTIFIC KNOWLEDGE

The assessment of this individual case reveals no evidence for a change of the current scientific knowledge.

RISK-BENEFIT-RATIO

The assessment of this individual case reveals no evidence for a change of the current risk-benefit ratio.

RISK PREVENTION MEASURES

The assessment of this individual case reveals no evidence for a need to initiate risk prevention measures with regard to the status of the marketing authorization.

CONCLUSION

A change of the current state of scientific knowledge and/or the risk-benefit ratio with regard to "Nurofen Junior Fiebersaft Erdbeer 4% Suspension zum Einnehmen" cannot be deduced from the assessment of this individual case.

Hence no safety relevant measure with regard to the status of the marketing authorization of this medicinal product is currently deemed necessary.

REMARKS

Former administration of orange flavoured Nurofen suspension (NOS) had been well tolerated, therefore the attending physician of the hospital assumed the red colouring agent azorubine of the Nurofen suspension with strawberry flavour as the most probable cause of the anaphylactic reaction whereas ibuprofen and silver sulfadiazine were assessed as less probable cause.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12085514

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 38 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	20120320		4 Week		recovered/resolved
2 Renal function test abnormal	20120320		4 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ENBREL etanercept	Unknown	50 Mg milligram(s)/Week	20111205-	Psoriatic arthritis
2 ibuprofen ibuprofen	Unknown	/	-	Product used for unknown indication

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

[McNeil Germany, Local Safety Officers, 25.04.2012 DE-JNJFOC-20120410391](#)

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a consumer and concerns a 38-year-old male patient of unspecified ethnicity via a business partner (PFIZER INC-2012094761) from Germany.

The patient's height, weight and medical history were not reported. The patient was treated with ibuprofen (unspecified, unspecified route of administration) initiated on an unspecified date for an unknown indication. Non-company suspect drugs included: etanercept (unspecified route of administration) from 05-Dec-2011 to an unspecified date at 50 mg, weekly for psoriatic arthritis. Concomitant medications were not reported. After taking the products, on 20-MAR-2012 the patient experienced hepatic enzymes increased and increased kidney values. The patient assessed the event was unrelated to etanercept treatment. The patient stated that the his treating physician assessed that the event was related to ibuprofen treatment. Action taken with ibuprofen was not reported and treatment with etanercept was withdrawn on unspecified date. The patient had recovered from hepatic enzymes increased and increased kidney values on an unspecified date.

This report was serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12111578

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: 19590208	Age: 52 Year	weight (kg): 55	BMI_int: 21.76	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Abdominal pain	201110				not recovered/not resolved
2 Jaundice	20111021		2 Week		recovered/resolved
3 Bilirubin increased					not recovered/not resolved
4 Cholestatic liver disease	201110			3 Month	not recovered/not resolved
5 Faeces discoloured	20111021		2 Week		recovered/resolved
6 Icterus	201110				not recovered/not resolved
7 Nausea	20111021		2 Week		recovered/resolved
8 SGOT increased	201110				not recovered/not resolved
9 SGPT increased	201110				not recovered/not resolved
10 GGT increased					not recovered/not resolved
11 Bilirubinaemia	201110				not recovered/not resolved
12 Faeces discoloured	201110				not recovered/not resolved
13 Gamma GT increased	201110				not recovered/not resolved
14 Laboratory test abnormal	201110				not recovered/not resolved
15 Blood alkaline phosphatase increased	201110				not recovered/not resolved
16 Drug toxicity	201110			112 Day	not recovered/not resolved
17 Abdominal discomfort	20111021		2 Week		recovered/resolved
18 Cholestasis	20111021		2 Week		recovered/resolved
19 Hepatic cirrhosis	20111021		2 Week		recovered/resolved
20 Hepatic necrosis	20111021		2 Week		recovered/resolved
21 Impaired liver function	20111021		2 Week		recovered/resolved
22 Al-P increased					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	1800 Df dosage form/Day	201107-20111020	Back pain
ibuprofen		245	-	
2 katadolon s long retard	Oral	1 Df dosage form/Day	201107-20111020	Back pain
flupirtine maleate		212	-	

Drugs (concomitant)

3 calcimagon d3	Oral	/Day	-	Osteoporosis prophylaxis
calcium carbonate; colecalciferol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain in extremity			Unknown	
Cervicobrachial syndrome			Unknown	
Panic disorder			Unknown	
Depressive disorder			Unknown	F33.1
Chronic pain			Unknown	
Panniculitis			Unknown	M54.0
Spondylolisthesis			Unknown	
Lumboischialgia			Unknown	

Primary source

Qualification: Physician

observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Bilirubin	2011	10.5	mg/dl	
Blood alkaline phosphatase	2011	140	U/l	
C-reactive protein	2011	6.2	mg/l	
Gamma GT	2011	319	U/l	
SGOT	2011	807	U/l	
SGPT	2011	1392	U/l	
Alanine aminotransferase		1392	U/L	
Aspartate aminotransferase		807	U/L	

Narratives (nur bei elektronischem Eingang)sanofi-aventis, 05.06.2012 DE-SANOFI-AVENTIS-2012SA039047

Sender diagnosis: not reported

Narrative

Initial Report received on 31-May-2012:

This serious case (hospitalization or prolongation) was extracted from the website of the German Health Authority (authority number DE-BFARM-12111578), initially reported by an anonymous physician via the magazine 'Arznei-Telegramm' (case no.: 16.021).

The case involves a 52-year-old female patient. The patient started therapy with ibuprofen 1800 DF (brand unknown) orally taken as needed for back pain in July 2011. Additionally, she started therapy with flupirtine maleate (Katadolon S long retard) orally 1 DF/day for back pain in July 2011. In October 2011, the patient experienced cholestatic liver disease with faeces discoloration, abdominal pain, nausea, increase in SGOT up to 807 U/L, increase in SGPT up to 1392 U/L, increase in GGT up to 319 U/L, increase in AP up to 140 U/L, increase in bilirubin up to 10.5 mg/dL and icterus. Reference ranges were not provided. Histological examination of liver puncture revealed acute liver dystrophy with necrosis and incomplete cirrhosis and high-graded cholestases. As corrective treatment acetylcysteine (Acetylstein) and glucocorticoids were given. Suspicion of toxic event according to drug anamnesis was mentioned. Therapy with ibuprofen as well as flupirtine maleate was discontinued on 20-Oct-2011. Outcome of the reaction was reported as not recovered.

Concomitant diseases chronic pain, depression and panic disorder were provided.

No details were provided on relevant history and concurrent drugs.

No causality assessment was provided for ibuprofen. Flupirtine maleate was suspected, too.

MENARINIP, 11.06.2012 DE-MEN-26787

Sender diagnosis: not reported

Narrative

This case, initially reported by a German physician via Arznei-Telegramm (16.021) and then published by BfArM as an ICH-E2B-compliant ICSR on its website, involved a 52-year-old female patient, who experienced drug toxicity resulting in cholestatic liver disease (discoloured faeces, abdominal pain, nausea, elevated liver function tests and icterus) in Oct-2011 and was therefore hospitalised.

The woman started therapy with 1800 mg/day of oral ibuprofen and an unspecified dose of flupirtine maleate (Katadolon® S long Retardtbl.) for back pain in the middle of Jul-2011. She took no concomitant medication. Her medical history included chronic pain disease with physical and mental consequences. The woman suffered from pain in the area of pelvis spreading to thigh and has spondylolisthesis in the lumbar region as well as cervicobrachial syndrome and sciatica. The mental disorder consisted of recurrent depressive disturbance in the context of complex traumatic stress- and panic disorder. Transaminases were within normal limits on 20-May-2012. No history of alcohol- or nicotine abuse. Serological examination indicated former Hepatitis A infection or status post Hepatitis A vaccination and former Hepatitis B without evidence of active infection.

Stool colour changed shortly after initiation of therapy with flupirtine maleate (Katadolon® S long Retardtbl.) and sporadic intake of ibuprofen. For three weeks the woman suffered from abdominal pain and nausea. Three months after therapy start with flupirtine maleate (Katadolon® S long Retardtbl.) and ibuprofen, while the woman was staying in a psychosomatic clinic, elevated liver function tests were discovered (GOT/AST = 807 U/L; GPT/ALT = 1.392 U/L; Gamma-GT = 319 U/L, blood alkaline phosphatase = 140 mg/dl and C-reactive protein = 6.2 mg/dl).

Thereupon, flupirtine maleate (Katadolon® S long Retardtbl.) and ibuprofen were withdrawn. Five days later there was no significant change in transaminase level, bilirubin level was 10.5 mg/dl and progressive icterus was noticed. For further diagnosis and therapy the patient was transferred to another hospital. Histological examination of liver biopsy revealed acute liver dystrophy with extensive necrosis as well as evidence of incomplete cirrhosis and high-grade cholestasis.

After corrective treatment with acetylcysteine as an antidote and glucocorticoid, liver values markedly declined.

The reporter suspected a toxic event according to medication anamnesis.

Bayer Vital GmbH, 11.06.2012 DE-BAYER-2012-053714

Sender diagnosis: not reported

Narrative

This case is a spontaneous case report received from a physician via regulatory authority from GERMANY (case # DE-BFARM-12111578) on 30-MAY-2012 and refers to a 52 year old female patient who received IBUPROFEN and FLUPIRTINE MALEATE and experienced CHOLESTATIC LIVER DISEASE, DISCOLOURED FAECES, ABDOMINAL PAIN, NAUSEA, SGOT INCREASED, SGPT INCREASED, GAMMA GT INCREASED, SERUM ALKALINE PHOSPHATASE INCREASED, ABNORMAL LAB TEST, BILIRUBINAEMIA and ICTERUS.

The patient's concurrent condition included chronic pain, depression and panic disorder.

In Jul-2011 the patient started IBUPROFEN at 1800 dose forms oral as needed for back pain. In Jul-2011 the patient started Katadolon s long retard (FLUPIRTINE MALEATE) at 1 dose form oral once daily for back pain.

Soon after treatment start with Flupirtin and intake of Ibuprofen as needed, discoloured faeces were observed; abdominal pain and nausea lasting for three weeks; approx. three months after treatment start, at a hospital for psychosomatic conditions, the following laboratory findings: GOT 807 U/l, GPT 1.393 U/l, Gamma GT 319 U/l, alkaline phosphatase 140 U/l, c-reactive protein 6.2 mg/dl. Flupirtin and Ibuprofen were withdrawn. Five days later, the transaminases had not improved substantially. Bilirubin was at 10.5 mg/dl, increasing icterus. The patient was transferred to another clinic for further diagnostics and treatment. Histological examination of a liver puncture; acute liver dystrophy with necrotic areas, and evidence of an incomplete cirrhosis and high-grade cholestasis; upon treatment with acetylcystein as antidote and glucocorticoids clearly improving liver values; a toxic event in the presence of corresponding drug history was assumed.

In OCT-2011 the patient experienced CHOLESTATIC LIVER DISEASE, DISCOLOURED FAECES, ABDOMINAL PAIN, NAUSEA, SGOT INCREASED, GAMMA GT INCREASED, SERUM ALKALINE PHOSPHATASE INCREASED, ABNORMAL LAB TEST, BILIRUBINAEMIA, ICTERUS, and SGPT INCREASED and therefore was hospitalized.

IBUPROFEN and FLUPIRTINE were discontinued on 20-OCT-2011. The events were ongoing at time of report.

No causality assessment was provided.

Senders comment:

Pain disease with physical and emotional consequences. Pain in the pelvis area down to the thighs. Spondylolisthesis in the lumbar region, cervicobrachial syndrome, lumboschialgia, relapsing depressive disorder in the presence of complex post-traumatic disturbance, panic disorder, on examination on 20-May-2011, the liver transaminases were still within the normal range, no nicotine or alcohol abuse. Treatment with Flupirtin and Ibuprofen from mid-July 2011 onward. No administration of other drugs, serological evidence of a former hepatitis A or status post hepatitis A vaccination, and of a former hepatitis B without evidence of an active infection.

MENARINIP, 19.06.2012 DE-MEN-26787

Sender diagnosis: not reported

Narrative

This case, initially reported by a German physician via Arznei-Telegramm (16.021) and then published by BfArM as an ICH-E2B-compliant ICSR on its website, involved a 52-year-old female patient, who experienced drug toxicity resulting in cholestatic liver disease (discoloured faeces, abdominal pain, nausea, elevated liver function tests and icterus) in Oct-2011 and was therefore hospitalised.

The woman started therapy with 1800 mg/day of oral ibuprofen and an unspecified dose of flupirtine maleate (Katadolon® S long Retardtbl.) for back pain in the middle of Jul-2011. She took no concomitant medication. Her medical history included chronic pain disease with physical and mental consequences. The woman suffered from pain in the area of pelvis spreading to thigh and has spondylolisthesis in the lumbar region as well as cervicobrachial syndrome and sciatica. The mental disorder consisted of recurrent depressive disturbance in the context of complex traumatic stress- and panic disorder. Transaminases were within normal limits on 20-May-2012. No history of alcohol- or nicotine abuse. Serological examination indicated former Hepatitis A infection or status post Hepatitis A vaccination and former Hepatitis B without evidence of active infection. Stool colour changed shortly after initiation of therapy with flupirtine maleate (Katadolon® S long Retardtbl.) and sporadic intake of ibuprofen. For three weeks the woman suffered from abdominal pain and nausea. Three months after therapy start with flupirtine maleate (Katadolon® S long Retardtbl.) and ibuprofen, while the woman was staying in a psychosomatic clinic, elevated liver function tests were discovered (GOT/AST = 807 U/L; GPT/ALT = 1.392 U/L; Gamma-GT = 319 U/L, blood alkaline phosphatase = 140 mg/dl and C-reactive protein = 6.2 mg/dl).

Thereupon, flupirtine maleate (Katadolon® S long Retardtbl.) and ibuprofen were withdrawn. Five days later there was no significant change in transaminase level, bilirubin level was 10.5 mg/dl and progressive icterus was noticed. For further diagnosis and therapy the patient was transferred to another hospital. Histological examination of liver biopsy revealed acute liver dystrophy with extensive necrosis as well as evidence of incomplete cirrhosis and high-grade cholestasis.

After corrective treatment with acetylcysteine as an antidote and glucocorticoid, liver values markedly declined.

The reporter suspected a toxic event according to medication anamnesis.

McNeil Germany, Local Safety Officers, 21.08.2012 DE-JNJFOC-20120600236

Sender diagnosis: not reported

Narrative

This spontaneous report from a physician, via a regulatory authority (BfArM (Germany) - DE-BFARM-12111578) and concerns a 52-year-old female

patient from Germany: 00755/12.

The patient's height was 159 centimeters and weight was 55 kilograms. The patient's medical history and concurrent conditions included: chronic pain, depression, and panic disorder. The patient was treated with ibuprofen (unspecified formulation, oral, batch unspecified) 1800 dosage form in one day, initiated in JUL-2011 to 20-OCT-2011 for back pain. Non-company suspect drugs included: katadolon s long retard (flupirtine maleate, unspecified formulation, oral) 400mg once a day initiated in JUL-2011 to 20-OCT-2011 for back pain. Concomitant medications were not reported. On an unspecified date in OCT-2011, the patient experienced cholestatic liver disease, faeces discoloured, abdominal pain, nausea, bilirubinaemia, SGOT (serum glutamic oxaloacetic transaminase) increased, SGPT (serum glutamic pyruvic transaminase) increased, icterus, gamma GT (glutamyltransferase) increased, alkaline phosphatase serum increased and laboratory test abnormal. Faeces discoloration had occurred soon after therapy was started with flupirtinmaleat and ibuprofen. The patient had experienced abdominal disorders and nausea for 3 weeks; approximately 3 months after therapy started in a psychosomatic clinic the reported diagnostic data were determined. Treatment with ibuprofen and flupirtine maleate was withdrawn, 5 days later there was no significant change of the transaminases values; bilirubin was 10.5mg/dl, icterus was increasing. The Patient was transferred to another hospital for diagnosis and therapy. Histologic examination after a liver puncture showed acute hepatic dystrophy with extensive necrosis and proof of an incomplete cirrhosis and high grade cholestasis; after treatment with acetylcystein as antidote and a glucocorticoide the liver values had decreased significantly; the toxic events were suspected with corresponding use of medicines. Laboratory data on an unspecified date in 2011 included: bilirubin: 10.5 mg/dl (Normal range: unspecified), blood alkaline phosphatase: 140 iu/l (Normal range: unspecified), C-reactive protein: 6.2 mg/l (Normal range: unspecified), gamma GT: 319 iu/l (Normal range: unspecified), SGOT: 807 iu/l (Normal range: unspecified), SGPT: 1392 iu/l (Normal range: unspecified). Treatment with ibuprofen and flupirtine maleate was withdrawn. The patient had not recovered from cholestatic liver disease, faeces discoloured, abdominal pain, nausea, bilirubinaemia, SGOT increased, SGPT increased, icterus, gamma GT increased, alkaline phosphatase serum increased and lab test abnormal.

Reporters comment: Pain disease with physical and psychological consequences. Pain in pelvis and thighs. Spondylolisthesis in lumbal area, cervico brachial syndrome, lumbal sciatica, recurrent depression with complex trauma sequelae, panic disorder, liver transaminase values were still normal during a control on 20-May-2011; no smoking, no alcoholism. Treatment with flupirtine maleate and ibuprofen as of mid of Jul-2011. No concomitant drugs, serologic indications to a bettered hepatitis A or a hepatitis A vaccination and a bettered hepatitis B without indications to acute infections This report was serious (hospitalization).

Additional information was received on 09-AUG-2012.

It was determined that Manufacturer Report Number 20120604341 was a duplicate of this case. All relevant information regarding this case will be submitted under Manufacturer Report Number 20120600236.

This case was received from a physician via a web portal of German health Authority (Bundesinstitut für Arzneimittel und Medizinprodukte BfArM (Germany) - 12111578), via business partner Pfizer (DE-PFIZER INC-2012130673). The patient's medical history and concurrent conditions included: abstains from alcohol, cervicobrachial syndrome, pain (pain from pelvic area to thighs), hepatitis A (serologi<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12133434

Serious Yes	Death No	Lifethr. No	Hospital. No	Disabling No	Congenital No	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: 19950411	Age: 17 Year	weight (kg): 55	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bradycardia	20120618			2.5 Week	recovered/resolved
2 Inflammation NOS	20120618			2.5 Week	recovered/resolved
3 Renal colic	20120618			2.5 Week	recovered/resolved
4 Multiple drug overdose intentional	201206				recovered/resolved
5 Suicide attempt	201206				recovered/resolved
6 Hepatic enzyme increased	20120618			2.5 Week	recovered/resolved
7 Renal insufficiency	20120618			2.5 Week	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Grippostad C Hartkapseln caffeine; chlorphenamine maleate; paracetamol	Oral	/ 6	201206-201206 -	Product used for unknown indication
2 ibuprofen ibuprofen	Oral	30 Df dosage form/Day 245	201206-201206 -	Product used for unknown indication
3 Omeprazol omeprazole	Oral	/ 245	201206-201206 -	Product used for unknown indication
4 Paracetamol 500 paracetamol	Oral	/ 245	201206-201206 -	Product used for unknown indication

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
C-reactive protein	20120618	100	-	
Bilirubin	20120618	1.7	-	
Aspartate aminotransferase	20120618	normal	-	
GPT	20120618	normal	-	
Creatinine	20120618	3.3	-	

Narratives (nur bei elektronischem Eingang)

STADA Arzneimittel AG, 25.06.2012 DE-STADA-048489

Sender diagnosis: not reported

Narrative

This report was received from a physician via the 'Giftinformationszentrum' (Poison Control Center) of Rheinland-Pfalz and Hessen (GIZ No. AZ 2012014025) on 2012-Jun-19.

A 17-year-old female patient took 2-3 capsules Grippostad C Hartkapseln (INN: ascorbic acid, caffeine, chlorphenamine, paracetamol), 30 tablets ibuprofen, 10-15 tablets Paracetamol 500 (INN: paracetamol) and 10-15 tablets Omeprazol (INN: omeprazole) two and a half week prior to this report in a suicide attempt.

On 2012-Jun-18 she suffered from renal colic on the right side, renal insufficiency, increased hepatic values, inflammation nos and bradycardia 45-50.

Performed tests showed following values: CRP 100 (norm <5), bilirubin 1.7, GOT and GPT normal, creatinine 3.3.

Hospitalisation was recommended.

No further information was provided. Follow-up requested.<dbtext>To_Char_NARRATIVE_2</dbtext>

GRIPPOSTAD C, PARACETAMOL

Renal failure and hepatic enzymes increased are listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Informations (RSIs).

IBUPROFEN

Renal failure is listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Information (RSI). Impaired liver functions and liver damage are listed, hepatic enzymes increased is not explicitly stated.

OMEPRAZOLE

Hepatic enzymes increased is listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Information (RSI).

This case does not change the overall benefit-risk balance of the medicinal products.

Reporter's comment:

STADA Arzneimittel AG, 18.07.2012 DE-STADA-048489

Sender diagnosis: not reported

Narrative

This report was received from a physician via the 'Giftinformationszentrum' (Poison Control Center) of Rheinland-Pfalz and Hessen (GIZ No. AZ 2012014025) on 2012-Jun-19.

A 17-year-old female patient took 2-3 capsules Grippostad C Hartkapseln (INN: ascorbic acid, caffeine, chlorphenamine, paracetamol), 30 tablets ibuprofen, 10-15 tablets Paracetamol 500 (INN: paracetamol) and 10-15 tablets Omeprazol (INN: omeprazole) two and a half week prior to this report in a suicide attempt.

On 2012-Jun-18 she suffered from renal colic on the right side, renal insufficiency, increased hepatic values, inflammation nos and bradycardia 45-50.

Performed tests showed following values: CRP 100 (norm <5), bilirubin 1.7, GOT and GPT normal, creatinine 3.3.

Hospitalisation was recommended.

No further information was provided. Follow-up requested.

Follow-up #1 report was received from a physician via the 'Giftinformationszentrum' (Poison Control Center) of Rheinland-Pfalz and Hessen (GIZ No. AZ 2012014025) on 2012-Jul-18.

The outcome was reported to be completely recovered.<dbtext>To_Char_NARRATIVE_2</dbtext>

IBUPROFEN

Renal failure is listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Information (RSI). Impaired liver functions and liver damage are listed, hepatic enzymes increased is not explicitly stated.

OMEPRAZOLE

Hepatic enzymes increased is listed, the other suspected adverse reactions are unlisted according to the current Reference Safety Information (RSI).

This case does not change the overall benefit-risk balance of the medicinal products.

Follow-up information #1 received on 2012-Jul-18:

The company's assessment of the case remains unchanged.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12164268

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Alcoholic liver disease					Unknown
2 Hepatotoxicity	20080219	20080228	10 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Ibuprofen generic	Unknown	/	-	Drug use for unknown indication
ibuprofen			-	
2 umckaloabo	Oral	/	2008-	Viral infection
pelargonium sidoides root			10 Day	

Drugs (concomitant)

3 sinupret	Unknown	/	-	Drug use for unknown indication
ascorbic acid; gentiana; primula flowers; rumicis aetosa; sambucus flowers; verben herb			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Viral infection NOS			Unknown	
Alcoholic liver disease NOS			Yes	concurrent condition
Viral infection			Unknown	
Alcohol use			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology. 2012; 63(1): 1-9.	
Qualification:	Other Health Professional	observestudytype:
Literature:	Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regul-Toxicol-Pharmacol 2012; 63(1) 1-9	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase	20080228	normal	NA	
Aspartate aminotransferase	20080228	normal	NA	
Gamma glutamyl transpeptidase	20080219	73	U/L	
Gamma glutamyl transpeptidase	20080228	normal	NA	
Glutamate dehydrogenase	20080219	13	U/L	

Narratives (nur bei elektronischem Eingang)

Generics [UK] Ltd, 30.07.2012 DE-MYLANLABS-2012S1014894

Sender diagnosis: not reported

Narrative

Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regul-Toxicol-Pharmacol 2012; 63(1) 1-9

A 44-year-old man was identified from a study after he developed hepatotoxicity due to treatment with ibuprofen. The man, who had a virus infection, had been receiving Pelargonium, as well as ibuprofen [dosage, route and duration of treatment not stated; indication not clearly stated]. On 19 February 2008, elevated levels of GGT and glutamate dehydrogenase activity were noted. His GGT, ALT and AST levels were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity. The authors excluded causality with respect to Pelargonium according to Council for International Organisations of Medical Sciences (CIOMS) criteria.

Reporter's comment:

KRKAPR, 30.07.2012 DE-KRKA-DE2012K2482LIT

Sender diagnosis: not reported

Narrative

Various drugs

Hepatotoxicity: 7 case reports

In a study of patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity, causality was assessed as "possible" for three patients [routes not stated]. In a further four patients, Pelargonium causality was either "unlikely" or "excluded"; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, olmesartan medoxomil and/or Rhodiola rosea [Rosenwurz]. The outcomes were favourable for all patients.

A 44-year-old man, with a history of viral infection, presented with elevated levels of GGT and glutamate dehydrogenase activity on 19 February 2008. His medications included ibuprofen [dosage, route and duration of treatment not stated; indication not clearly stated]. His liver enzymes were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity.

Author Comment "In all cases causality for [Pelargonium] was assessed using the updated [Council for International Organisations of Medical Sciences] scale . . . In the study patients, causality for [Pelargonium] was found possible (n = 3), unlikely (n = 8), and excluded (n = 4)."

CITATION DETAILS

Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology 63: 1-9, No. 1, Jun 2012. Available from: URL: <http://dx.doi.org/10.1016/j.yrtph.2012.02.009> (English)

Krka's comment: S, UL (for alcoholic liver disease), possibly related for hepatotoxicity
Hepatotoxicity could also be explained by disease (alcoholic liver disease).

Dr. R. Pflieger Chemische Fabrik GmbH, 31.07.2012 DE-PFLEGER-001891

Sender diagnosis: not reported

Narrative

In a study of patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity, causality was assessed as possible for three patients [routes not stated]. In a further four patients, Pelargonium causality was either unlikely or excluded; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, olmesartan medoxomil and/or Rhodiola rosea [Rosenwurz]. The outcomes were favourable for all patients.

There is one patient relevant for Dr. R. Pflieger medicinal products:

A 44-year-old mal, with a history of viral infection, presented with elevated levels of GGT and glutamate dehydrogenase activity on 19 February 2008. His medications included ibuprofen [dosage, route and duration of treatment not stated; indication not clearly stated]. His liver enzymes were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity.<dbtext>To_Char_NARRATIVE_2</dbtext>
No change to the RSI is necessary.

Reporter's comment:

Temmler Pharma GmbH & Co. KG, 02.08.2012 DE-TEMMLER06-005199

Sender diagnosis: not reported

Narrative

This case was received by Temmler Pharma on 24 July 2012 from Adis Reactions via literature research:

A 44-year-old man was identified from a study after he developed hepatotoxicity due to treatment with ibuprofen. The man, who had a virus infection, had been receiving Pelargonium, as well as ibuprofen [dosage, route and duration of treatment not stated; indication not clearly stated]. On 19 February 2008, elevated levels of GGT and glutamate dehydrogenase activity were noted. His GGT, ALT and AST levels were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity. The authors excluded causality with respect to Pelargonium according to Council for International Organisations of Medical Sciences (CIOMS) criteria.

Temmler ID: IBU-2012-057<dbtext>To_Char_NARRATIVE_2</dbtext>
adverse reaction could not be excluded.

Of note, the BfArM has initiated in June 2012 a "Stufenplan" concerning liver injury due to pelargonium-containing drugs.

This case is medically confirmed and the overall benefit-risk-ratio is not changed.

Reporter's comment:

RANBAXY, 02.08.2012 DE-RANBAXY-2012RR-58510

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-58510 from GERMANY refers to a 44 year-old male patient (Patient 4) who experienced hepatotoxicity which was medically significant following treatment with ibuprofen.

The case was reported in the literature.

Patient's medical history:
Viral infection.

Patient's current condition:
Alcohol use.
Alcoholic liver disease.

The patient received the following suspect product:
Ibuprofen: dose, route and indication unknown.

On an unspecified date, the subject developed hepatotoxicity while receiving ibuprofen.

Reaction text: "In a study of patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity, causality was assessed as "possible" for three patients [routes not stated]. In a further four patients, Pelargonium causality was either "unlikely" or "excluded"; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, olmesartan medoxomil and/or Rhodiola rosea [Rosenwurz]. The outcomes were favourable for all patients.

A 52-year-old man received Pelargonium 3 x 30 drops daily from 8-17 February 2009 for a virus infection. On 17 February, he had elevated levels of ALT, AST and ALP. Fourteen days later, normalisation was reported. He was diagnosed with hepatitis due to ongoing virus infection, with possible causality with respect to Pelargonium.

A 46-year-old man received Pelargonium 3 x 20 drops daily for an infection in March and April 2004. On 3 May, tests showed elevated liver enzymes and inflammatory markers. He was admitted mid-May for jaundice and increasing ALT levels. He was diagnosed with a prolonged bacterial infection with bacterial hepatitis, with possible causality related to Pelargonium.

A 4-year-old girl, who had an infection with cough and fever, received Pelargonium 2 x 10 drops daily during 14 and 19 December 2006, and 1 x 10 drops on 20 December. On 21 December, increased ALT levels were noted. She was diagnosed with hepatitis associated with infectious disease, with possible causality with respect to Pelargonium.

A 44-year-old man, with a history of viral infection, presented with elevated levels of GGT and glutamate dehydrogenase activity on 19 February 2008. His medications included ibuprofen [dosage, route and duration of treatment not stated; indication not clearly stated]. His liver enzymes were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity.

A 54-year-old man received olmesartan medoxomil in November 2010 for arterial hypertension and Rhodiola rosea from May to November 2010 for impaired concentration [dosages and routes not stated]. He subsequently developed hepatotoxicity [time to onset not clearly stated]. He was diagnosed with pre-existing liver disease, nonalcoholic steatohepatitis, olmesartan hepatotoxicity and liver injury by "Rosenwurz".

A 46-year-old man had been regularly receiving ibuprofen twice a week for migraines for years [dose and route not stated]. After developing a viral infection, his bilirubin, ALT and AST levels increased [duration of treatment to reaction onset not stated]. Liver histology showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy revealed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen-induced liver injury was deemed "less probable".

An 82-year-old woman received amoxicillin [dosage and route not stated] for sinusitis from 13-20 August 2010. On 6 September, increased liver values were recorded. She was diagnosed with amoxicillin hepatotoxicity."

The outcome of the event was reported as improved at the time of this report.

The case was reported as serious because the ADR was medically significant.

Author Comment "In all cases causality for [Pelargonium] was assessed using the updated [Council for International Organisations of Medical Sciences] scale . . . In the study patients, causality for [Pelargonium] was found possible (n = 3), unlikely (n = 8), and excluded (n = 4)."

DEXPHARM, 06.08.2012 DE-DEXPHARM-20120950

Sender diagnosis: not reported

Narrative

Teschke, R.; Frenzel, C.; Schulze, J.; Eickhoff, A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology 2012, V63, P1-9.

The study described patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity causality was assessed as "possible" for three patients. In a further four patients, Pelargonium causality was either "unlikely" or "excluded"; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, etc?. The outcomes were favourable for all patients.

A 44-year-old man, with a history of viral infection, presented with elevated levels of GGT and glutamate dehydrogenase activity on 19 February 2008. His medications included ibuprofen [dosage, route, duration and indication unknown]. His liver enzymes were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity

RANBAXY, 08.08.2012 DE-RANBAXY-2012RR-58510

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-58510 from GERMANY refers to a 44 year-old male patient (Patient 4) who experienced hepatotoxicity which was medically significant following treatment with ibuprofen.

The case was reported in the literature.

Patient's medical history:

Viral infection.

Patient's current condition:

Alcohol use.

Alcoholic liver disease.

The patient received the following suspect product:

Ibuprofen: dose, route and indication unknown.

On an unspecified date, the subject developed hepatotoxicity while receiving ibuprofen.

Reaction text: "In a study of patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity, causality was assessed as "possible" for three patients [routes not stated]. In a further four patients, Pelargonium causality was either "unlikely" or "excluded"; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, olmesartan medoxomil and/or Rhodiola rosea [Rosenwurz]. The outcomes were favourable for all patients.

A 52-year-old man received Pelargonium 3 x 30 drops daily from 8-17 February 2009 for a virus infection. On 17 February, he had elevated levels of ALT, AST and ALP. Fourteen days later, normalisation was reported. He was diagnosed with hepatitis due to ongoing virus infection, with possible causality with respect to Pelargonium.

A 46-year-old man received Pelargonium 3 x 20 drops daily for an infection in March and April 2004. On 3 May, tests showed elevated liver enzymes

and inflammatory markers. He was admitted mid-May for jaundice and increasing ALT levels. He was diagnosed with a prolonged bacterial infection with bacterial hepatitis, with possible causality related to Pelargonium.

A 4-year-old girl, who had an infection with cough and fever, received Pelargonium 2 x 10 drops daily during 14 and 19 December 2006, and 1 x 10 drops on 20 December. On 21 December, increased ALT levels were noted. She was diagnosed with hepatitis associated with infectious disease, with possible causality with respect to Pelargonium.

A 44-year-old man, with a history of viral infection, presented with elevated levels of GGT and glutamate dehydrogenase activity on 19 February 2008. His medications included ibuprofen [dosage, route and duration of treatment not stated; indication not clearly stated]. His liver enzymes were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity.

A 54-year-old man received olmesartan medoxomil in November 2010 for arterial hypertension and Rhodiola rosea from May to November 2010 for impaired concentration [dosages and routes not stated]. He subsequently developed hepatotoxicity [time to onset not clearly stated]. He was diagnosed with pre-existing liver disease, nonalcoholic steatohepatitis, olmesartan hepatotoxicity and liver injury by "Rosenwurz".

A 46-year-old man had been regularly receiving ibuprofen twice a week for migraines for years [dose and route not stated]. After developing a viral infection, his bilirubin, ALT and AST levels increased [duration of treatment to reaction onset not stated]. Liver histology showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy revealed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen-induced liver injury was deemed "less probable".

An 82-year-old woman received amoxicillin [dosage and route not stated] for sinusitis from 13-20 August 2010. On 6 September, increased liver values were recorded. She was diagnosed with amoxicillin hepatotoxicity."

The outcome of the event was reported as improved at the time of this report.

The case was reported as serious because the ADR was medically significant.

Author Comment "In all cases causality for [Pelargonium] was assessed using the updated [Council for International Organisations of Medical Sciences] scale . . . In the study patients, causality for [Pelargonium] was found possible (n = 3), unlikely (n = 8), and excluded (n = 4)."

Follow up report received on 27-JUL-2012 containing further information regarding:

Reporter details.

New lab tests were added.

New suspect drug: Sinupret.

New event: Alcoholic liver disease.

Reaction text: "Details: Report from a general practitioner. PS (Pelargonium sidoides) used to treat a virus infection with unknown daily dose 10 days before February 19, 2008 when increased levels of GGT of 73 U/L (normal until 55 U/L) and a glutamate dehydrogenase activity of 13 U/L (normal range non communicated) was noticed. No values of ALT and AST reported, possibly in the normal range. On February 28, 2008 normal values of ALT, AST, and GGT reported. Comedication with ibuprofen and Sinupret, a drug consisting of five different herbs. No other data available.

Final diagnoses: Alcoholic liver disease; ibuprofen hepatotoxicity.

Causality: Excluded causality with respect to PS according to CIOMS criteria.

Comment: Poorly documented, basically unassessable case. Aminotransferases not reported, differential diagnoses not considered."

Duplicate number: DE-DCGMA-08146828.

Boehringer Ingelheim GmbH, Germany, 23.08.2012 DE-Boehringer Ingelheim GmbH, Germany-2012-EU-04169GD

Sender diagnosis: not reported

Narrative

P12-09615

A 44-year-old male patient (patient 3, tab. 1) is reported who developed ibuprofen-induced hepatotoxicity and was using Pelargonium sidoides for viral infection. The patient had been reported to the Drug Commission of the German Medical Association.

The case had been reported from a general practitioner. Pelargonium sidoides (PS) was used to treat a virus infection with unknown daily dose 10 days before February 19, 2008, when increased levels of gamma-GT of 73 U/L (normal until 55 U/L) and a glutamate dehydrogenase activity of 13 U/L (normal range non communicated) was noticed. No values of ALT and AST were reported, they were possibly in the normal range. On February 28, 2008 normal values of ALT, AST, and gamma-GT were reported.

Comedication included ibuprofen and Sinupret, a drug consisting of five different herbs. No other data were available.

Final diagnoses included alcoholic liver disease; ibuprofen hepatotoxicity.

Causality: Excluded causality with respect to Pelargonium sidoides according to CIOMS criteria.

Comment: Poorly documented, basically unassessable case. Aminotransferases not reported, differential diagnoses not considered.

Due to the medical circumstances the case was rated serious: seriousness criterion 'other' (medically significant).

The authors evaluated the reports of spontaneous cases of purported Pelargonium sidoides (Umckaloabo) hepatotoxicity to assess data quality and causality as originally presented since 2004 (Drug Commission of the German Medical Association, DCGMA, 2011).

The study group consisted finally of 15 patients originating from Germany and included cases of spontaneous reports with liver disease in primarily assumed temporal and causal association with the treatment by PS, as referenced before by the DCGMA (DCGMA, 2011). PS is considered an herbal medicinal product with well established use in the pharmaceutical form of film-coated tablet and oral liquid.

The data of the 15 cases as presented by DCGMA appeared more or less scattered and included assessments of causality levels only for the eight DCGMA cases but not for the seven BfArM cases. Therefore, additional data for the 15 cases and results of the causality levels of all patients including the seven BfArM cases were requested from BfArM. Obtaining these data proved difficult but the Dr. Schwabe pharmaceutical company in Karlsruhe (Germany), the manufacturer of the PS drug (Umckaloabo), then provided additional information of these 15 cases.

The data of all 15 cases were submitted to a causality algorithm that consisted of four steps. In the first step, clear information was required for timing of the start and end of PS use as well as of the appearance of symptoms and/or increased liver values. For the second step, values of alanine aminotransferase (ALT) and alkaline phosphatase (ALP) are required as clear criteria for liver injury and for the differentiation between the hepatocellular, cholestatic or mixed hepatocellular-cholestatic pattern. The third step of the diagnostic algorithm requires the application of the liver specific, structured, quantitative and updated CIOMS scale, and for purposes of comparison, the original CIOMS scale was used as well. In the fourth step, all case data were analyzed regarding various other differential diagnoses not yet evaluated with the updated CIOMS scale.

2 additional cases are available:

2011-DE-04315GD

2012-EU-04173GD

Tracking of changes:

17Aug2012, initial article received.

McNeil Germany, Local Safety Officers, 30.08.2012 DE-JNJFOC-20120812939

Sender diagnosis: not reported

Narrative

Report received from literature. Teschke R. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained? Regulatory Toxicology and Pharmacology 2012; 63(1):1-9 via a regulatory authority (BfArM (Germany) - 08146828), via business partner (Pfizer Inc 2012203354).

This case refers to 44-year-old male patient. The patient's height and weight were not reported. The patient was treated with ibuprofen (unspecified formulation and route of administration) initiated on an unspecified date for an unknown indication. Non-company suspect drugs included: sinupret (ascorbic acid, gentian, primula flowers, rumicis asetosia, sambucus flowers, verbena herb, unspecified formulation and route of administration) and pelargonium sidoides (unspecified formulation and route of administration) initiated on an unspecified date for viral infection. Concomitant medications were not reported. Pelargonium sidoides (PS) were used to treat a virus infection with unknown daily dose 10 days before 19-FEB-2008 when increased levels of c-GT of 73 IU/L (normal until 55 IU/L) and a glutamate dehydrogenase activity of 13 IU/L (normal range unspecified) was noticed. No values of ALT and AST were reported, possibly were in the normal range. On 28-FEB-2008 normal values of ALT, AST, and c-GT were reported. On an unspecified date, the patient was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity. Action taken with ibuprofen, pelargonium sidoides and sinupret was not reported. The patient's outcome was not reported for the alcoholic liver disease and ibuprofen hepatotoxicity. Aminotransferases were not reported and the differential diagnoses not considered. The causality for PS by Drug Commission of the German Medical Association was possible.

This report was serious (medically significant).

This case, from the same reporter is linked to 20120812317.<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 07.09.2012 DE-JNJFOC-20120812939

Sender diagnosis: not reported

Narrative

Report received from literature. Teschke R. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained? Regulatory Toxicology and Pharmacology 2012; 63(1):1-9 via a regulatory authority (BfArM (Germany) - 08146828), via business partner (Pfizer Inc 2012203354).

This case refers to 44-year-old male patient. The patient's height and weight were not reported. The patient was treated with ibuprofen (unspecified formulation and route of administration) initiated on an unspecified date for an unknown indication. Non-company suspect drugs included: sinupret (ascorbic acid, gentian, primula flowers, rumicis asetosia, sambucus flowers, verbena herb, unspecified formulation and route of administration) and pelargonium sidoides (unspecified formulation and route of administration) initiated on an unspecified date for viral infection. Concomitant medications were not reported. Pelargonium sidoides (PS) were used to treat a virus infection with unknown daily dose 10 days before 19-FEB-2008 when increased levels of c-GT of 73 IU/L (normal until 55 IU/L) and a glutamate dehydrogenase activity of 13 IU/L (normal range unspecified) was noticed. No values of ALT and AST were reported, possibly were in the normal range. On 28-FEB-2008 normal values of ALT, AST, and c-GT were reported. On an unspecified date, the patient was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity. Action taken with ibuprofen, pelargonium sidoides and sinupret was not reported. The patient's outcome was not reported for the alcoholic liver disease and ibuprofen hepatotoxicity. Aminotransferases were not reported and the differential diagnoses not considered. The causality for PS by Drug Commission of the German Medical Association was possible.

This report was serious (medically significant).

This case, from the same reporter is linked to 20120812317.

Additional information was received from an other health professional via a business partner (Pfizer Inc 2012203354) on 29-AUG-2012.

Additional authors included Frenzel C, Schulze J, Eickhoff A. The author's details were updated in the literature and reporter screens.<dbtext>

To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12164269

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth:	Age: 46 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic fibrosis					Unknown
2 Shoulder pain					Unknown
3 Drug-induced liver injury	20060112				Unknown
4 Hepatotoxicity					Unknown
5 Viral hepatitis					Unknown
6 Fatigue					Unknown
7 Liver injury					recovering/resolving
8 Hepatitis					recovering/resolving
9 Back pain					Unknown
10 Cholestasis					Unknown
11 Drug-induced liver injury					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUPROFEN	Unknown	2* /Week	-	Migraine
ibuprofen			-	
2 UMCKALOABO	Oral	/	200512-	Viral infection
pelargonium sidoides root			3 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Viral infection			Unknown	concurrent condition, with associated hepatitis
Migraine			Yes	
Hepatitis			Yes	associated with concurrent viral infection
Hepatitis viral			Unknown	
Migraine			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology. 2012; 63(1): 1-9.	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alanine aminotransferase		1694	N/A	
Alanine aminotransferase		1716	N/A	
Aspartate aminotransferase		911	N/A	
Aspartate aminotransferase		1064	N/A	
Bilirubin		increased	N/A	
Histology		Seven weeks later, repeat biopsy revealed periport		
Histology		Liver histology with eosinophilic and aptotic hepa		
Lab test		Exclusion of various forms of virus hepatitis desc		
Alanine aminotransferase	20060112	1694	U/L	

Alanine aminotransferase	20060116	1716	U/L
Aspartate aminotransferase	20060112	911	U/L
Aspartate aminotransferase	20060116	1064	U/L

Narratives (nur bei elektronischem Eingang)Generics [UK] Ltd, 30.07.2012 DE-MYLANLABS-2012S1014895

Sender diagnosis: not reported

Narrative

Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regul-Toxicol-Pharmacol 2012; 63(1) 1-9

A 46-year-old man was identified from a study after developing hepatotoxicity following treatment with ibuprofen. The man had been regularly receiving ibuprofen twice a week for migraines for years [dose and route not stated; duration of treatment not clearly stated]. After developing a viral infection, he received Pelargonium for 3 days. Subsequently, his ALT and AST levels increased to 1716 U/L and 1064 U/L, respectively. He also had increased bilirubin levels and was admitted. Liver biopsy showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy showed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen-induced liver injury was deemed "less probable". His outcome was favourable. The authors excluded causality with respect to Pelargonium according to Council for International Organisations of Medical Sciences (CIOMS) criteria.

KRKAPR, 30.07.2012 DE-KRKA-DE2012K2487LIT

Sender diagnosis: not reported

Narrative

Various drugs

Hepatotoxicity: 7 case reports

In a study of patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity, causality was assessed as "possible" for three patients [routes not stated]. In a further four patients, Pelargonium causality was either "unlikely" or "excluded"; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, olmesartan medoxomil and/or Rhodiola rosea [Rosenwurz]. The outcomes were favourable for all patients.

A 46-year-old man had been regularly receiving ibuprofen twice a week for migraines for years [dose and route not stated]. After developing a viral infection, his bilirubin, ALT and AST levels increased [duration of treatment to reaction onset not stated]. Liver histology showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy revealed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen-induced liver injury was deemed "less probable".

Author Comment "In all cases causality for [Pelargonium] was assessed using the updated [Council for International Organisations of Medical Sciences] scale . . . In the study patients, causality for [Pelargonium] was found possible (n = 3), unlikely (n = 8), and excluded (n = 4)."

CITATION DETAILS

Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology 63: 1-9, No. 1, Jun 2012. Available from: URL: <http://dx.doi.org/10.1016/j.yrtph.2012.02.009> (English)

Krka's comment: S, UL (for viral hepatitis), possibly related for hepatotoxicity

Hepatotoxicity could likely be explained by viral infection. Ibuprofen-induced liver injury was appeared to be less probable.

TEVA, 31.07.2012 DE-TEVA-350234GER

Sender diagnosis: not reported

Narrative

23-Jul-2012, Serious Literature Report received from Reaction (803074278). (SPRINGER HEALTHCARE; NZ-ADIS-3074278-27279)

Case of a Male patient 46 Years of age who experienced IBUPROFEN INDUCED LIVER INJURY while on Ibuprofen (IBUPROFEN) (Teva's product cannot be excluded).

A 46-year old man was identified from a study after developing hepatotoxicity following treatment with ibuprofen. The man had been regularly receiving ibuprofen twice a week for migraines for years dose and route not stated; duration of treatment not clearly stated. After developing a viral infection, he received Pelargonium for 3 days. Subsequently, his ALT and AST levels increased to 1716 U/L and 1064 U/L, respectively. He also had

increased bilirubin levels and was admitted. Liver biopsy showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy showed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen induced liver injury was deemed "less probable". His outcome was favourable.

The authors excluded causality with respect to Pelargonium according to Council for International Organisations of Medical Sciences (CIOMS) criteria.

At the time of the report the outcome of the AEs were: IBUPROFEN INDUCED LIVER INJURY : recovering/resolving.

Concomitant Medication:

Umckaloabo(pelargonium)

Medical History:

MIGRAINE(Concomitant) ,VIRAL INFECTION NOS(Concomitant) ,HEPATITIS(Concomitant)

Action Taken with suspect Drugs: Ibuprofen - Unknown

Literature Reference:

Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained. Regul Toxicol Pharmacol 2012; 63(1) 1-9

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Due to data available contribution of Ibuprofen to liver injury cannot be completely excluded: possibly related.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case.

Temmler Pharma GmbH & Co. KG, 02.08.2012 DE-TEMMLER06-005198

Sender diagnosis: not reported

Narrative

This literature case was received by Temmler Pharma on 24 July 2012 from Adis Reactions via literature research:

A 46-year-old man was identified from a study after developing hepatotoxicity following treatment with ibuprofen. The man had been regularly receiving ibuprofen twice a week for migraines for years [dose and route not stated; duration of treatment not clearly stated]. After developing a viral infection, he received Pelargonium for 3 days. Subsequently, his ALT and AST levels increased to 1716 U/L and 1064 U/L, respectively. He also had increased bilirubin levels and was admitted. Liver biopsy showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy showed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen-induced liver injury was deemed 'less probable'. His outcome was favourable. The authors excluded causality with respect to Pelargonium according to Council for International Organisations of Medical Sciences (CIOMS) criteria.

Temmler ID: IBU-2012-056<dbtext>To_Char_NARRATIVE_2</dbtext>

Of note, the BfArM has initiated in June 2012 a "Stufenplan" concerning liver injury due to pelargonium-containing drugs. In their letter to the MAHs a 46 year old male patient with adverse reactions as described, is mentioned. So, this case is probably already reported to the authority.

This case is medically confirmed and the overall benefit-risk-ratio is not changed.

.

Reporter's comment:

RANBAXY, 02.08.2012 DE-RANBAXY-2012RR-58511

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-58511 from GERMANY refers to a 46 year-old male patient (Patient 6) who experienced liver injury which was medically significant following treatment with ibuprofen.

The case was reported in the literature.

Patient's medical history:

Viral infection with associated hepatitis.

Patient's current condition:
Migraines.

The patient received the following suspect product:
Ibuprofen: twice per week, route unknown, for migraines.

On an unspecified date, the subject developed liver injury while receiving ibuprofen.

Reaction text: "In a study of patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity, causality was assessed as "possible" for three patients [routes not stated]. In a further four patients, Pelargonium causality was either "unlikely" or "excluded"; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, olmesartan medoxomil and/or Rhodiola rosea [Rosenwurz]. The outcomes were favourable for all patients.

A 52-year-old man received Pelargonium 3 x 30 drops daily from 8-17 February 2009 for a virus infection. On 17 February, he had elevated levels of ALT, AST and ALP. Fourteen days later, normalisation was reported. He was diagnosed with hepatitis due to ongoing virus infection, with possible causality with respect to Pelargonium.

A 46-year-old man received Pelargonium 3 x 20 drops daily for an infection in March and April 2004. On 3 May, tests showed elevated liver enzymes and inflammatory markers. He was admitted mid-May for jaundice and increasing ALT levels. He was diagnosed with a prolonged bacterial infection with bacterial hepatitis, with possible causality related to Pelargonium.

A 4-year-old girl, who had an infection with cough and fever, received Pelargonium 2 x 10 drops daily during 14 and 19 December 2006, and 1 x 10 drops on 20 December. On 21 December, increased ALT levels were noted. She was diagnosed with hepatitis associated with infectious disease, with possible causality with respect to Pelargonium.

A 44-year-old man, with a history of viral infection, presented with elevated levels of GGT and glutamate dehydrogenase activity on 19 February 2008. His medications included ibuprofen [dosage, route and duration of treatment not stated; indication not clearly stated]. His liver enzymes were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity.

A 54-year-old man received olmesartan medoxomil in November 2010 for arterial hypertension and Rhodiola rosea from May to November 2010 for impaired concentration [dosages and routes not stated]. He subsequently developed hepatotoxicity [time to onset not clearly stated]. He was diagnosed with pre-existing liver disease, nonalcoholic steatohepatitis, olmesartan hepatotoxicity and liver injury by "Rosenwurz".

A 46-year-old man had been regularly receiving ibuprofen twice a week for migraines for years [dose and route not stated]. After developing a viral infection, his bilirubin, ALT and AST levels increased [duration of treatment to reaction onset not stated]. Liver histology showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy revealed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen-induced liver injury was deemed "less probable".

An 82-year-old woman received amoxicillin [dosage and route not stated] for sinusitis from 13-20 August 2010. On 6 September, increased liver values were recorded. She was diagnosed with amoxicillin hepatotoxicity."

The outcome of the event was reported as improved at the time of this report.

The case was reported as serious because the ADR was medically significant.

Author Comment "In all cases causality for [Pelargonium] was assessed using the updated [Council for International Organisations of Medical Sciences] scale . . . In the study patients, causality for [Pelargonium] was found possible (n = 3), unlikely (n = 8), and excluded (n = 4)."

DEXPHARM, 06.08.2012 DE-DEXPHARM-20120951

Sender diagnosis: not reported

Narrative

Teschke, R.; Frenzel, C.; Schulze, J.; Eickhoff, A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology 2012, V63, P1-9.

The study described patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity causality was assessed as "possible" for three patients. In a further four patients, Pelargonium causality was either "unlikely" or "excluded"; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, etc?. The outcomes were favourable for all patients.

A 46-year-old man had been regularly receiving ibuprofen twice a week for migraines for years [dose and route not stated]. After developing a viral infection, his bilirubin, ALT and AST levels increased [duration of treatment to reaction onset not stated]. Liver histology showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy revealed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen-induced liver injury was deemed "less probable".

Company assessment: Case 20120951 relates to a 46-year-old male with chronic exposure to ibuprofen over years who developed a viral infection and was noted to have increased levels of bilirubin, ALT and AST. The results of biopsy suggested an acute process that over a few weeks settled into a healing phase. Therefore the main causative factor was considered to be the viral infection. However as ibuprofen is associated with hepatotoxicity in some cases, the possibility cannot be excluded that ibuprofen played a contributory role in damage to a liver made more susceptible by viral infection.

RANBAXY, 08.08.2012 DE-RANBAXY-2012RR-58511

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-58511 from GERMANY refers to a 46 year-old male patient (Patient 6) who experienced liver injury which was medically significant following treatment with ibuprofen.

The case was reported in the literature.

Patient's medical history:

Viral infection with associated hepatitis.

Patient's current condition:

Migraines.

The patient received the following suspect product:

Ibuprofen: twice per week, route unknown, for migraines.

On an unspecified date, the subject developed liver injury while receiving ibuprofen.

Reaction text: "In a study of patients with primarily suspected Pelargonium [Umckaloabo] hepatotoxicity, causality was assessed as "possible" for three patients [routes not stated]. In a further four patients, Pelargonium causality was either "unlikely" or "excluded"; these patients were diagnosed with hepatotoxicity associated with amoxicillin, ibuprofen, olmesartan medoxomil and/or Rhodiola rosea [Rosenwurz]. The outcomes were favourable for all patients.

A 52-year-old man received Pelargonium 3 x 30 drops daily from 8-17 February 2009 for a virus infection. On 17 February, he had elevated levels of ALT, AST and ALP. Fourteen days later, normalisation was reported. He was diagnosed with hepatitis due to ongoing virus infection, with possible causality with respect to Pelargonium.

A 46-year-old man received Pelargonium 3 x 20 drops daily for an infection in March and April 2004. On 3 May, tests showed elevated liver enzymes and inflammatory markers. He was admitted mid-May for jaundice and increasing ALT levels. He was diagnosed with a prolonged bacterial infection with bacterial hepatitis, with possible causality related to Pelargonium.

A 4-year-old girl, who had an infection with cough and fever, received Pelargonium 2 x 10 drops daily during 14 and 19 December 2006, and 1 x 10 drops on 20 December. On 21 December, increased ALT levels were noted. She was diagnosed with hepatitis associated with infectious disease, with possible causality with respect to Pelargonium.

A 44-year-old man, with a history of viral infection, presented with elevated levels of GGT and glutamate dehydrogenase activity on 19 February 2008. His medications included ibuprofen [dosage, route and duration of treatment not stated; indication not clearly stated]. His liver enzymes were normal on 28 February. He was diagnosed with alcoholic liver disease and ibuprofen hepatotoxicity.

A 54-year-old man received olmesartan medoxomil in November 2010 for arterial hypertension and Rhodiola rosea from May to November 2010 for impaired concentration [dosages and routes not stated]. He subsequently developed hepatotoxicity [time to onset not clearly stated]. He was diagnosed with pre-existing liver disease, nonalcoholic steatohepatitis, olmesartan hepatotoxicity and liver injury by "Rosenwurz".

A 46-year-old man had been regularly receiving ibuprofen twice a week for migraines for years [dose and route not stated]. After developing a viral infection, his bilirubin, ALT and AST levels increased [duration of treatment to reaction onset not stated]. Liver histology showed eosinophilic and apoptotic hepatocytes, and cholestasis. Seven weeks later, repeat biopsy revealed periportal fibrosis. He was diagnosed with a viral infection with associated hepatitis; ibuprofen-induced liver injury was deemed "less probable".

An 82-year-old woman received amoxicillin [dosage and route not stated] for sinusitis from 13-20 August 2010. On 6 September, increased liver values were recorded. She was diagnosed with amoxicillin hepatotoxicity."

The outcome of the event was reported as improved at the time of this report.

The case was reported as serious because the ADR was medically significant.

Author Comment "In all cases causality for [Pelargonium] was assessed using the updated [Council for International Organisations of Medical Sciences] scale . . . In the study patients, causality for [Pelargonium] was found possible (n = 3), unlikely (n = 8), and excluded (n = 4)."

Follow up report received on 27-JUL-2012 containing further information regarding:

Reporter details.

New lab tests were added.

Reaction text was updated with : "Details: Report by BfArM and arznei-telegramm. PS treatment of unknown daily dose for virus infection over a period of 3 days, but the type of the initial, preexisting viral infection was not subsequently evaluated. Later on, but time interval not described, fatigue with pains in the shoulders and back. Subsequently and without reported time frame, increased values of bilirubin and liver values and hospitalization. ALT 1694 U/L with rise to 1716 U/L within the next 4 days; AST with 911 U/L and rise to 1064 U/L within 4 days. Exclusion of various forms of virus hepatitis described, but details not reported; for causality assessment by CIOMS, it was assumed that hepatitis A-C has adequately been excluded in the hospital setting. No exclusion of hepatic autoimmune or genetic diseases and no imaging data reported. Liver histology with eosinophilic and apoptotic hepatocytes, minimal intracellular cholestasis. Seven weeks after the initial liver biopsy, another biopsy due to poor general condition and still increased liver values, now with the finding of an incipient periportal fibrosis. Since years regular use of ibuprofen for migraine twice a week.

Final diagnoses: Preexisting viral infection with associated hepatitis, not further specified due to poor data; ibuprofen induced liver injury less probable. Causality: Excluded causality with respect to PS according to CIOMS criteria.

Comment: Poorly documented case with unclear ALT course in the months following PS cessation. Medical report of the hospital has not been made

available for critical analysis. The type of the preexisting, initial virus infection has not been assessed, and it remained unclear whether and to what extent other causes for the observed liver disease has been excluded. The clinical symptoms of pains in the shoulders and the back are compatible with liver disease in association with a viral infection but not with a drug reaction. Even with regard to the temporal association open questions remain. PS use was described for 3 days somewhere in 12/2005, and symptoms and increased liver values have been reported on January 12, 2006. Therefore, the latency period remains unknown, raising doubts on a temporal association. In addition, Liver values rose further although PS had been withdrawn for at least 2 weeks."

Boehringer Ingelheim GmbH, Germany, 23.08.2012 DE-Boehringer Ingelheim GmbH, Germany-2012-EU-04173GD

Sender diagnosis: not reported

Narrative

P12-09615

A 46-year-old male patient is reported (patient 10, tab. 1) who developed hepatotoxicity and was using ibuprofen for migraine and Pelargonium sidoides for virus infection. The patient had been reported to the Bundesinstitut für Arzneimittel und Medizinprodukte (BfArM, Germany) and from the 'Arznei-telegramm'. The patient presented with minimal intracellular cholestasis, incipient periportal fibrosis, pains in the shoulder and back, and fatigue.

Pelargonium sidoides (PS) treatment of unknown daily dose was used for virus infection over a period of 3 days in Dec-2005, but the type of the initial, preexisting viral infection was not subsequently evaluated. Later on, but time interval not described, fatigue with pains in the shoulders and back occurred. Subsequently and without reported time frame, increased values of bilirubin and liver values were found leading to hospitalization. ALT values on 12-Jan-2006 were 1694 U/L with rise to 1716 U/L within the next 4 days; AST values on 12-Jan-2006 were 911 U/L and rise to 1064 U/L within 4 days. Exclusion of various forms of virus hepatitis was described, but details not reported; for causality assessment by CIOMS, it was assumed that hepatitis A-C has adequately been excluded in the hospital setting. No exclusion of hepatic autoimmune or genetic diseases and no imaging data were reported. Liver histology included eosinophilic and apoptotic hepatocytes, minimal intracellular cholestasis. Seven weeks after the initial liver biopsy, another biopsy was performed due to poor general condition and still increased liver values were found, now with the finding of an incipient periportal fibrosis. Since years the patient had a regular use of ibuprofen for migraine twice a week.

Final diagnoses included preexisting viral infection with associated hepatitis, not further specified due to poor data; ibuprofen induced liver injury was assessed as less probable.

Causality: Excluded causality with respect to Pelargonium sidoides according to CIOMS criteria.

Authors' comment: Poorly documented case with unclear ALT course in the months following PS cessation. Medical report of the hospital has not been made available for critical analysis. The type of the preexisting, initial virus infection has not been assessed, and it remained unclear whether and to what extent other causes for the observed liver disease has been excluded. The clinical symptoms of pains in the shoulders and the back are compatible with liver disease in association with a viral infection but not with a drug reaction. Even with regard to the temporal association open questions remain. PS use was described for 3 days somewhere in 12/2005, and symptoms and increased liver values have been reported on January 12, 2006. Therefore, the latency period remains unknown, raising doubts on a temporal association. In addition, Liver values rose further although PS had been withdrawn for at least 2 weeks.

The authors evaluated the reports of spontaneous cases of purported Pelargonium sidoides(Umckaloabo) hepatotoxicity to assess data quality and causality as originally presented since 2004 (Drug Commission of the German Medical Association, DCGMA, 2011).

The study group consisted finally of 15 patients originating from Germany and included cases of spontaneous reports with liver disease in primarily assumed temporal and causal association with the treatment by PS, as referenced before by the DCGMA (DCGMA, 2011). PS is considered an herbal medicinal product with well established use in the pharmaceutical form of film-coated tablet and oral liquid.

The data of the 15 cases as presented by DCGMA appeared more or less scattered and included assessments of causality levels only for the eight DCGMA cases but not for the seven BfArM cases. Therefore, additional data for the 15 cases and results of the causality levels of all patients including the seven BfArM cases were requested from BfArM. Obtaining these data proved difficult but the Dr. Schwabe pharmaceutical company in Karlsruhe (Germany), the manufacturer of the PS drug (Umckaloabo), then provided additional information of these 15 cases.

The data of all 15 cases were submitted to a causality algorithm that consisted of four steps. In the first step, clear information was required for timing of the start and end of PS use as well as of the appearance of symptoms and/or increased liver values. For the second step, values of alanine aminotransferase (ALT) and alkaline phosphatase (ALP) are required as clear criteria for liver injury and for the differentiation between the hepatocellular, cholestatic or mixed hepatocellular- cholestatic pattern. The third step of the diagnostic algorithm requires the application of the liver specific, structured, quantitative and updated CIOMS scale, and for purposes of comparison, the original CIOMS scale was used as well. In the fourth step, all case data were analyzed regarding various other differential diagnoses not yet evaluated with the updated CIOMS scale.

2 additional cases are available:

2011-DE-04315GD

2012-EU-04169GD

Tracking of changes;

17Aug2012, initial article received.

PFIZERINC, 23.08.2012 DE-PFIZER INC-2012203378

Sender diagnosis: not reported

Narrative

This is a literature study report from the Regulatory Toxicology and Pharmacology, 2012, 63(1); 1-9 entitled Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained? This is the third of four reports. A 46-year-old male patient of an unspecified ethnicity started to receive ibuprofen (manufacturer unspecified) via an unspecified route of administration from an

unspecified date at dosage regimen of twice a week for migraine. Medical history included viral infection from an unknown date and unknown if ongoing. Concomitant medication included pelargonium sidoides (PELARGONIUM SIDOIDES) (PS). The virus infection was treated with pelargonium sidoides at an unknown daily dose for over three days. The pre-existing viral infection was not subsequently evaluated. Later, the patient experienced fatigue with pains in the shoulder and back. Subsequently, the patient experienced increased bilirubin values and liver values and the patient was hospitalized as a result. On 12Jan2006, the patient experienced ibuprofen induced liver injury. The action taken in response to the event for ibuprofen was unknown. Relevant laboratory data included: Alanine Aminotransferase (ALT) (date unknown): 1694 U/L with rise to 1716 U/L within the next 4 days; Aspartate Aminotransferase (AST) (date unknown): 911 U/L and rise to 1064 U/L within 4 days; Liver histology (date unknown): eosinophilic and aptotic hepatocytes, minimal intracellular cholestasis. A liver biopsy was performed seven weeks later, due to poor general condition and still increased liver values. The results of liver biopsy showed incipient periportal fibrosis. Exclusion of various forms of virus hepatitis described, but details not reported; for causality assessment by CIOMS, it was assumed that hepatitis A-C has adequately been excluded in the hospital setting. No exclusion of hepatic autoimmune or genetic diseases and no imaging data reported. The final diagnosis was pre-existing viral infection with associated hepatitis, not further specified due to poor data; ibuprofen induced liver injury less probable. Causality with respect to PS was excluded according to Council for International Organizations of Medical Sciences (CIOMS) criteria. The author concluded this was a poorly documented case with unclear ALT course in the months following PS cessation. Medical report of the hospital was not made available for critical analysis. The type of pre-existing initial virus infection has not been assessed and it remained unclear whether and to what extent other causes for the observed liver disease has been excluded. The clinical symptoms of pains in the shoulders and the back are compatible with liver disease in association with a viral infection but not with a drug reaction. Even with regard to the temporal association open questions remain. PS use was described for 3 days somewhere in Dec2005, and symptoms and increased liver values have been reported on January 12, 2006. Therefore, the latency period remains unknown, raising doubts on a temporal association. In addition, liver values rose further although PS had been withdrawn for at least 2 weeks. At the time of this report, it was unknown if the patient had recovered.

Pfizer is the Marketing Authorization Holder of Ibuprofen in Germany. This may be a duplicate report in situations where another Marketing Authorization Holder of Ibuprofen has submitted the same report to the regulatory authorities.

McNeil Germany, Local Safety Officers, 28.08.2012 DE-JNJFOC-20120812317

Sender diagnosis: not reported

Narrative

Report received from literature: Teschke R. Spontaneous reports of primarily suspected herbal hepatotoxicity by pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology 2012;63 (1):1-9.

This literature report was received via business partner Pfizer (2012203378) and concerns a 46-year-old male patient of unspecified height, weight and ethnicity. The patient's medical history included viral infection since an unspecified date and unspecified if it was ongoing. The patient was treated with ibuprofen (unspecified formulation, route of administration and dosage) (The brand name was not reported and it is assumed to be J&J brand for reporting purposes) initiated on an unspecified date at a dosage regimen of twice a week for migraine. Concomitant medication included pelargonium sidoides (PS) initiated on an unspecified date in DEC-2005 at an unspecified daily dose for over 3 days for viral infection. The pre-existing viral infection was not subsequently evaluated. On an unspecified date, the patient experienced fatigue with pains in the shoulder and back. Subsequently, the patient experienced increased bilirubin values and liver values due to which the patient was hospitalized. On 12-JAN-2006, the patient experienced ibuprofen induced liver injury. The action taken in response to the event for ibuprofen was not reported. Relevant laboratory data included: alanine aminotransferase (ALT) (on an unspecified date): 1694 U/L with rise to 1716 U/L within the next 4 days. Aspartate Aminotransferase (AST) (on an unspecified date): 911 U/L and rise to 1064 U/L within 4 days. Liver histology (on an unspecified date): eosinophilic and aptotic hepatocytes, minimal intracellular cholestasis. Liver biopsy (on an unspecified date), performed 7 weeks later due to poor general condition and persistent increased liver values, showed incipient periportal fibrosis. Exclusion of various forms of virus hepatitis were described, but details were not reported, it was assumed that hepatitis A-C has adequately been excluded in the hospital setting. No exclusion of hepatic autoimmune or genetic diseases and no imaging data were reported. The final diagnosis was pre existing viral infection with associated hepatitis, not further specified due to poor data, ibuprofen induced liver injury less probable. Causality with respect to PS was excluded according to CIOMS (Council for International Organizations of Medical Sciences) criteria. The author concluded that it was a poor documented case with unclear ALT course in the months following PS cessation. Medical report of the hospital was not made available for critical analysis. The type of pre existing initial virus infection was not assessed and it remained unclear whether and to what extent other cause for observed liver disease was excluded. The clinical symptoms of pains in the shoulders and back were compatible with liver disease in association with a viral infection but not with a drug reaction. Even with regard to the temporal association open questions remain. The PS use was described for 3 days in DEC-2005 and symptoms and increased liver values have been reported on 12-JAN-2006 and hence latency period remained unknown, raising doubts on a temporal association. In addition, liver values rose further although PS had been withdrawn for at least 2 weeks. At the time of report, it was not reported if the patient had recovered. The sender commented that, a possible contributory role of the product ibuprofen to the reported event ibuprofen induced liver injury cannot be excluded based on temporal association.

This report was serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 06.09.2012 DE-JNJFOC-20120812317

Sender diagnosis: not reported

Narrative

Report received from literature: Teschke R. Spontaneous reports of primarily suspected herbal hepatotoxicity by pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology 2012;63 (1):1-9.

This literature report was received via business partner Pfizer (2012203378) and concerns a 46-year-old male patient of unspecified height, weight and ethnicity. The patient's medical history included viral infection since an unspecified date and unspecified if it was ongoing. The patient was treated with ibuprofen (unspecified formulation, route of administration and dosage) (The brand name was not reported and it is assumed to be J&J brand for reporting purposes) initiated on an unspecified date at a dosage regimen of twice a week for migraine. Concomitant medication included pelargonium

sidoides (PS) initiated on an unspecified date in DEC-2005 at an unspecified daily dose for over 3 days for viral infection. The pre-existing viral infection was not subsequently evaluated. On an unspecified date, the patient experienced fatigue with pains in the shoulder and back. Subsequently, the patient experienced increased bilirubin values and liver values due to which the patient was hospitalized. On 12-JAN-2006, the patient experienced ibuprofen induced liver injury. The action taken in response to the event for ibuprofen was not reported. Relevant laboratory data included: alanine aminotransferase (ALT) (on an unspecified date): 1694 U/L with rise to 1716 U/L within the next 4 days. Aspartate Aminotransferase (AST) (on an unspecified date): 911 U/L and rise to 1064 U/L within 4 days. Liver histology (on an unspecified date): eosinophilic and aptotic hepatocytes, minimal intracellular cholestasis. Liver biopsy (on an unspecified date), performed 7 weeks later due to poor general condition and persistent increased liver values, showed incipient periportal fibrosis. Exclusion of various forms of virus hepatitis were described, but details were not reported, it was assumed that hepatitis A-C has adequately been excluded in the hospital setting. No exclusion of hepatic autoimmune or genetic diseases and no imaging data were reported. The final diagnosis was pre existing viral infection with associated hepatitis, not further specified due to poor data, ibuprofen induced liver injury less probable. Causality with respect to PS was excluded according to CIOMS (Council for International Organizations of Medical Sciences) criteria. The author concluded that it was a poor documented case with unclear ALT course in the months following PS cessation. Medical report of the hospital was not made available for critical analysis. The type of pre existing initial virus infection was not assessed and it remained unclear whether and to what extent other cause for observed liver disease was excluded. The clinical symptoms of pains in the shoulders and back were compatible with liver disease in association with a viral infection but not with a drug reaction. Even with regard to the temporal association open questions remain. The PS use was described for 3 days in DEC-2005 and symptoms and increased liver values have been reported on 12-JAN-2006 and hence latency period remained unknown, raising doubts on a temporal association. In addition, liver values rose further although PS had been withdrawn for at least 2 weeks. At the time of report, it was not reported if the patient had recovered. The sender commented that, a possible contributory role of the product ibuprofen to the reported event ibuprofen induced liver injury cannot be excluded based on temporal association.

This report was serious (medically significant).

Additional information was received from an other health professional via business partner Pfizer on 29-AUG-2012.

The patient's concurrent conditions included migraine. The patient had used ibuprofen regularly twice a week since years. After the fatigue with pains in the shoulder and back, the patient had inc<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12168562

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth:	Age: 30 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ALT increased					recovered/resolved
2 ALP increased					recovered/resolved
3 AST increased					recovered/resolved
4 ALT increased					recovered/resolved
5 AST increased					recovered/resolved
6 ALP increased	27.02.2011				Unknown
7 Drug-induced liver injury					recovered/resolved
8 Drug-induced liver injury					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Enalapril enalapril	Unknown	/	-	Hypertension
2 ibuprofen ibuprofen		/	20110221-20110223 3 Day	
3 METHYLDOPA methyldopa	Unknown	/	-	Hypertension NOS
4 PANTOZOL pantoprazole sodium		/	20110221-20110223 3 Day	
5 paracetamol with codeine codeine phosphate; paracetamol	Unknown	/	-	Product used for unknown indication
6 PELARGONIUM SIDOIDES pelargonium sidoides		90 Gtt drop(s)/Day	20110223-20110225 3 Day	Nasopharyngitis
7 PELARGONIUM SIDOIDES pelargonium sidoides		15 Gtt drop(s)/	20110226-20110226 1 Day	Nasopharyngitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Unknown	arterial hypertension
Hepatic disease			Unknown	preexisting liver disease

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Teschke R, Frenzel C, Schulze J, Eickhoff A. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology. 2012;63 (1):1-9	

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20110227	168	U/L	
SGOT	20110227	1520	U/L	
SGOT	201103	31	U/L	
SGPT	20110227	1419	U/L	

Narratives (nur bei elektronischem Eingang)

Bayer Vital GmbH, 03.08.2012 DE-BAYER-2012-078293

Sender diagnosis: not reported

Narrative

serious; listed, related

This case report from Germany was derived from medical literature on 01-AUG-2012 and refers to a 30 year old female patient who was prescribed IBUPROFEN, PANTOZOLE and PELARGONIUM SIDOIDES and experienced liver injury with increased levels of ALT 1419 U/L and AST 1520 U/L and ALP 168 U/L. This case is report 11106963 from the Bundesinstitut für Arzneimittel und Medizinprodukte (BfArM) (Federal Institute for Drugs and Medical Devices) and was initially reported by a physician.

Trade name for ibuprofen was not reported

The 30 year old female patient (patient number 15) received Pelargonium sidonides (PS) for nasopharyngitis with 3 x 30 drops daily from 23 Feb 2011 to 25 Feb 2011 with 15 drops on 26 Feb 2011. Comedication for arterial hypertension included enalapril until 25 Jan 2011 than switch to methyldopa

She received ibuprofen and pantozole from 21 to 23 Feb 2011.

On On 27 Feb 2011, the following lab values were found: ALT 1419 U/L, AST 1520 U/L, and ALP 168 U/L. One month later, ALT was 40 U/L and AST was 31 U/L.

Exclusion of hepatitis A-C reported, but details not presented. Lack of exclusion of other virus infections such as cytomegalovirus (CMV), Epstein Barr virus (EBV), herpes simplex virus (HSV), varizella zooster virus (VZV), and hepatitis E. Hepatobiliary sonography was without abnormalities.

Final diagnoses by the authors were not further assessed preexisting liver disease and liver injury by comedicated drug(s).

Comment by the authors:

Unlikely causality with respect to PS according to CIOMS criteria

Short-term use of PS on 31=3 days, hardly compatible with ALT 1419 U/L on day 4 after PS start. Most probably some preexisting infection with liver involvement or preexisting primary liver disease. Insufficient exclusion of differential diagnoses. Basically poorly documented case of a patient with multiple comorbidities and comedications.

Abstract

Spontaneous reports of primarily assumed hepatotoxicity in connection with the use of Pelargonium sidoides (PS) have been interpreted by the Drug Commission of the German Medical Association (DCGMA) as showing some hepatotoxic potential of PS used to treat common cold and other respiratory tract infections.

Causality for PS was assessed using the liver specific, structured, quantitative, and updated scale of the Council for International Organizations of Medical Sciences (CIOMS). In none of the 15 cases was there a highly probable or probable causality for PS. Analysis revealed confounding factors such as numerous final diagnoses unrelated to PS and poor data quality in virtually all cases. In only a minority of the cases were data provided to consider even common other diseases of the liver. For instance, biliary tract imaging data were available in only 3 patients; data to exclude virus infections by hepatitis AC were provided in 4 cases and by CMV and EBV in 1 case, whereas HSV and VZV virus infections remained unconsidered. Thus, convincing evidence is lacking that PS was a potential hepatotoxin in the analyzed cases.

Cases derived from this article include 2012-078283 and 2012-078293.

JUTA, 15.08.2012 DE-JUTA-2012-14115

Sender diagnosis: not reported

Narrative

Date of initial report: 01-AUG-2012 An article from Regulatory Toxicology and Pharmacology, entitled "Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?," describes a 30 year old female patient (Table 1, Patient 15) that experienced drug induced liver injury attributed to ibuprofen, enalapril, methyldopa, pantozole taken for various medical reasons.

Details: Report from a physician. PS used for nasopharyngitis with 3 _ 30 drops daily from February 23 to 25, 2011 and with 15 drops on February 26, 2011. On February 27, 2011 ALT 1419 U/L, AST 1520 U/L, and ALP 168 U/L. One month later, ALT 40 U/L and AST 31 U/L. Comedication for arterial hypertension with enalapril until January 25, 2011, than switch to methyldopa. Ibuprofen and pantozole on February 21 to 23, 2011. Exclusion of hepatitis A-C reported, but details not presented. Lack of exclusion of other virus infections such as CMV, EBV, HSV, VZV, and hepatitis E. Hepatobiliary sonography without abnormalities.

Final diagnoses: Not further assessed preexisting liver disease, liver injury by comedicated drug(s).

Causality: Unlikely causality with respect to PS according to CIOMS criteria.

Comment: Short-term use of PS on 31=3 days, hardly compatible with ALT 1419 U/L on day 4 after PS start. Most probably some preexisting infection with liver involvement or preexisting primary liver disease. Insufficient exclusion of differential diagnoses. Basically poorly

documented case of a patient with multiple comorbidities and comedications.

See article for further information. Related cases: 2012-14112, 2012-14113, 2012-14114, 2012-14115

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12190939

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth:	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxicity					Unknown
2 Alcoholic liver disease	20080219				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen		/	-	
ibuprofen			-	
2 ibuprofen		/	-	
ibuprofen			-	
3 pelargonium		/	200802-	Viral infection
pelargonium sidoides			-	
4 SINUPRET		/	-	Infection viral
ascorbic acid; gentiana; primula flowers; rumicis asetosa; sambucus flowers; verben herb			-	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Teschke, R.. Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?. Regulatory Toxicology and Pharmacology. 2012;63(1):1-9	

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Gamma-glutamyltransferase	20080219	73	IU/l	
Glutamate dehydrogenase	20080219	13	IU/l	

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 23.08.2012 DE-PFIZER INC-2012203354

Sender diagnosis: not reported

Narrative

This is a Literature study report from Regulatory Toxicology and Pharmacology, 2012, 63(1), 1-9, "Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?" This is the second of 4 reports. This case refers to 44 year old male patient who is started taking Ibuprofen (Unknown Manufacturer) and ascorbic acid, gentian, primula flowers, rumicis asetosa, sambucus flowers, verben herb (SINUPRET). Details from BfArM Drug Commission of the German Medical Association- 08146828 included: Report from a general practitioner. Pelargonium sidoides (PS) were used to treat a virus infection with unknown daily dose 10 days before February 19, 2008 when increased levels of c-GT of 73 U/L (normal until 55 U/L) and a glutamate dehydrogenase activity of 13 U/L (normal range non communicated) was noticed. No values of ALT and AST reported, possibly in the normal range. On February 28, 2008 normal values of ALT, AST, and c-GT reported. Comedication with ibuprofen and Sinupret, a drug consisting of five different herbs. No other data available. Final diagnoses: Alcoholic liver disease; ibuprofen hepatotoxicity. Causality: Excluded causality with respect to PS according to CIOMS criteria. Comment: Poorly documented, basically unassessable case. Aminotransferases not reported, differential diagnoses not considered. TCase: Causality for PS by Drug Commission of the German Medical Association: Possible. In conclusion, the present analysis of spontaneous reports of 15 patients reveals little if any evidence for a hepatotoxicity of PS. Problems included poor data quality, lack of efforts to exclude event common liver diseased, and numerous PS unrelated diagnoses. Moreover, assessments of these cases do not fulfill essential criteria to propose the existence of PS hepatotoxicity.

Pfizer is the Marketing Authorization Holder of Ibuprofen in Germany. This may be a duplicate report in situations where another Marketing Authorization Holder of Ibuprofen has submitted the same report to the regulatory authorities.

Sender diagnosis: not reported

Narrative

This is a Literature study report from Regulatory Toxicology and Pharmacology, 2012, 63(1), 1-9, "Spontaneous reports of primarily suspected herbal hepatotoxicity by Pelargonium sidoides: Was causality adequately ascertained?" This is the second of 4 reports. This case refers to 44 year old male patient who started to receive Ibuprofen (Unknown Manufacturer) and ascorbic acid, gentian, primula flowers, rumicis asetosa, sambucus flowers, verbenas herb (SINUPRET). Details from BfArM Drug Commission of the German Medical Association- 08146828 included: Report from a general practitioner: Pelargonium sidoides (PS) was used to treat a virus infection with unknown daily dose 10 days before February 19, 2008 when increased levels of c-GT of 73 U/L (normal until 55 U/L) and a glutamate dehydrogenase activity of 13 U/L (normal range non communicated) was noticed. No values of ALT and AST reported, possibly in the normal range. On February 28, 2008 normal values of ALT, AST, and c-GT reported. Comedication included ibuprofen and Sinupret, a drug consisting of five different herbs. No other data was available. Final diagnoses: Alcoholic liver disease; ibuprofen hepatotoxicity. Causality: Excluded causality with respect to PS according to CIOMS criteria. Comment: Poorly documented, basically unassessable case. Aminotransferases not reported, differential diagnoses not considered. TCASE: Causality for PS by Drug Commission of the German Medical Association: Possible. In conclusion, the present analysis of spontaneous reports of 15 patients reveals little if any evidence for a hepatotoxicity of PS. Problems included poor data quality, lack of efforts to exclude event common liver diseases, and numerous PS unrelated diagnoses. Moreover, assessments of these cases do not fulfill essential criteria to propose the existence of PS hepatotoxicity.

Pfizer is the Marketing Authorization Holder of Ibuprofen in Germany. This may be a duplicate report in situations where another Marketing Authorization Holder of Ibuprofen has submitted the same report to the regulatory authorities.

Follow up (30Aug2012): New information received from Johnson and Johnson, license partner for ibuprofen, included manufacturer control number 20120812939.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12248083

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex:	Birth:	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxicity					Unknown
2 Overdose					Unknown
3 Sedation					Unknown
4 Addiction					Unknown
5 Cardiovascular disorder					Unknown
6 Liver injury					Unknown
7 Toxicity renal					Unknown
8 Hemarthrosis					Unknown
9 Gastro intestinal bleed					Unknown
10 Peptic ulcer					Unknown
11 Constipation					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 celecoxib		/	-	Pain
celecoxib			-	
2 codeine		/	-	Pain
codeine			-	
3 diclofenac		/	-	Pain
diclofenac			-	
4 ETORICOXIB		/	-	Pain
etoricoxib			-	
5 FENTANYL	Transdermal	/	-	Pain
fentanyl			-	
6 ibuprofen		/	-	Pain
ibuprofen			-	
7 MELOXICAM		/	-	Pain
meloxicam			-	
8 METAMIZOL		/	-	Pain
metamizole magnesium			-	
9 MORPHINE		/	-	Pain
morphine			-	
10 naproxen		/	-	Pain
naproxen			-	
11 PARACETAMOL		/	-	Pain
paracetamol			-	
12 piroxicam		/	-	Pain
piroxicam			-	
13 TRAMADOL	Unknown	/	-	Pain
tramadol hydrochloride			-	
14 TRAMADOL HYDROCHLORIDE		/	-	Pain
tramadol hydrochloride			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Haemophilia			Yes	
Pain			Yes	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia Sep-2012;18 (5):743-752.	

Narratives (nur bei elektronischem Eingang)

Janssen-Cilag, Germany Local Safety Officers, 27.09.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported.

This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 27.09.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of

administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported. This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

GRUNENTHAL, 01.10.2012 DE-GRUNENTHAL-2012-15367

Sender diagnosis: not reported

Narrative

Manufacturer Control Number: 2012-15367

This spontaneous literature case from Germany was reported by a physician via license partner and was initially received on 18-Sep-2012. It was extracted from the literature article 'Pain management in patients with haemophilia: a European survey' published by Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. in Haemophilia Sep-2012;18 (5):743-752

concerns a patient of unknown age (height and weight unknown) who experienced cardiovascular disorder, liver injury, nephropathy toxic, haemarthrosis, gastrointestinal haemorrhage, peptic ulcer, constipation, dependence, sedation and overdose under tramadol hydrochloride treatment.

Other suspected medication included fentanyl (Fentanyl, transdermal) for pain, paracetamol (Paracetamol, unknown route of administration) for pain, naproxen (Naproxen, unknown route of administration) for pain, ibuprofen (Ibuprofen, unknown route of administration) for pain, codeine (Codeine, unknown route of administration) for pain, metamizole magnesium (Metamizol, unknown route of administration) for pain, diclofenac (Diclofenac, unknown route of administration) for pain, piroxicam (Piroxicam, unknown route of administration) for pain, meloxicam (Meloxicam, unknown route of administration) for pain, morphine (Morphine, unknown route of administration) for pain, celecoxib (Celecoxib, unknown route of administration) for pain and etoricoxib (Etoricoxib, unknown route of administration) for pain.

No relevant concomitant medication was reported.

The patient's concurrent conditions included haemophilia and pain.

The patient was treated with tramadol hydrochloride (Tramadol hydrochloride,) for pain.

Reporter's notes: Report received from literature.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74

% of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported.

This report was serious (medically significant).

No treatment of the AEs was reported.

Third party case numbers

e2b company number: de-jnjfoc-20120909195

grt litdb accno: 803382

Literature citation:

holstein k, klamroth r, richards m, carvalho m, perez-garrido r, gringeri a. pain management in patients with haemophilia: a european survey. haemophilia. 2012; 18(5): 743-752

Janssen-Cilag, Germany Local Safety Officers, 18.10.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. Haemophilia SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported.

This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 22.10.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. *Haemophilia* SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported. This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

Janssen-Cilag, Germany Local Safety Officers, 07.11.2012 DE-JNJFOC-20120909195

Sender diagnosis: not reported

Narrative

Report received from literature.

Holstein K, Klamroth R, Richards M, Carvalho M, Perez-Garrido R, Gringeri A. Pain management in patients with haemophilia: a European survey. *Haemophilia* SEP-2012;18 (5):743-752.

The European Haemophilia Therapy Standardization Board (EHTSB) conducted a literature review and a survey of pain assessment and management in 22 European Haemophilia Centers using a questionnaire and seven clinical scenarios. This study aimed to investigate existing evidence on pain assessment and management in haemophilia and current clinical practice throughout Europe. The survey investigated approaches to pain management: timing of assessment and modality, existence of specific (national/institutional) guidelines, availability of pain specialists, experience and use of pain medication and knowledge about side effects. This report concerns multiple patients of unspecified age and sex. The patients' height and weight were not reported. The patients' concurrent condition included haemophilia. The patients were treated with tramadol hydrochloride (unspecified formulation, unknown route of administration) unspecified dose and frequency, fentanyl (unspecified patch, transdermal) unspecified dose and frequency, paracetamol (unspecified formulation, unknown route of administration) unspecified dose and frequency, ibuprofen (unspecified formulation, unknown route of administration) unspecified dose and frequency and naproxen (unspecified formulation, unknown route of administration) unspecified dose and frequency all initiated on unspecified date for the treatment of pain due to hemophilia. The brand names were not reported and they were assumed to be J&J brand for reporting purposes. Non-company suspect drugs included meloxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency, morphine (unspecified formulation, unknown route of administration) unspecified dose and frequency, celecoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, etoricoxib (unspecified formulation, unknown route of administration) unspecified dose and frequency, codeine (unspecified formulation, unknown route of administration) unspecified dose and frequency, metamizole (unspecified formulation, unknown route of administration) unspecified dose and frequency, diclofenac (unspecified formulation, unknown route of administration) unspecified dose and frequency and piroxicam (unspecified formulation, unknown route of administration) unspecified dose and frequency all for pain due to hemophilia. Concomitant medications were not reported. The side effects that limited the dose and duration of treatment for strong and weak opioids (weak opioids include tramadol hydrochloride) were addiction (74%), sedation (61%) and constipation (43%). Gastrointestinal bleeding or peptic ulcers were seen in 83 % of patients treated with non-steroidal anti-inflammatory drugs (NSAIDs). Ibuprofen was one of the NSAIDs. Increased haemarthroses was seen among 43% of patients who used naproxen and 48 % of the patients treated with NSAIDs experienced increased haemarthroses. Ibuprofen was one of the NSAIDs. Increased liver toxicity was experienced by 74 % of the patients who took high-dose (overdose) of paracetamol. Renal toxicity was experienced by 39 % of the patients who had prolonged use of NSAIDs (39%) and Cox-2 inhibitors and cardiovascular events for Cox-2 inhibitors (35%). Action taken with tramadol hydrochloride, fentanyl (unspecified patch), paracetamol, naproxen, ibuprofen, codeine, metamizole, diclofenac, piroxicam, meloxicam, morphine, celecoxib and etoricoxib was not reported. The patients' outcome for addiction, sedation, constipation, liver toxicity, overdose of paracetamol, gastro intestinal bleed, peptic ulcers, haemarthroses, renal toxicity and cardiovascular events were not reported. This report was serious (medically significant).

Labeling assessment and seriousness for tra<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12311081

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: 19381211	Age: 73 Year	weight (kg): 95	BMI_int: 35.32	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cholestatic hepatitis	20120710				recovered/resolved
2 Hepatic steatosis	20120710				recovered/resolved
3 Hepatitis NOS	20120710				recovering/resolving
4 Acute cholestatic hepatitis	20120710				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ibuprofen	Oral	2400 Mg milligram(s)/Day	-201207	Intervertebral foraminal encroachment
ibuprofen		245	-	
2 ibuprofen	Oral	/	-	Intervertebral foraminal encroachment
ibuprofen		245	-	
3 IBUPROFEN		/	-	Spinal column stenosis
ibuprofen		245	-	
4 IBUPROFEN		/	-	Pain
ibuprofen		245	-	
5 terbinafine	Oral	/	-	Drug use for unknown indication
terbinafine			-	
6 TERBINAFINE (ORAL)	Oral	250 Mg milligram(s)/Day	-201207	
terbinafine		245	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Adipositas			Unknown	obesity, BMI: 35.3 kg/m2
Fatty liver			Unknown	fatty liver

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Alkaline phosphatase	20120710	105	U/L	
Alkaline phosphatase	20120716	146	U/L	
Blood bilirubin total	20120710	2.7	mg/dL	
Blood bilirubin total	20120716	0.9	mg/dL	
GGT	20120710	780	U/L	
GGT	20120716	540	U/L	
SGOT	20120710	682	U/L	
SGOT	20120716	87	U/L	
SGPT	20120710	671	U/L	
SGPT	20120716	311	U/L	

Narratives (nur bei elektronischem Eingang)

INTENDIS DEUTSCHLAND, 28.11.2012 DE-BAYER-2012-123794

Sender diagnosis: not reported

Narrative

Serious, Related, Unlisted

This is a spontaneous case report received from a physician via Drug Commission of German Medical Association in GERMANY on 26-NOV-2012 which refers to a 73-year-old female patient who received TERBINAFINE (ORAL) (terbinafine) and IBUPROFEN (ibuprofen) and experienced CHOLESTATIC HEPATITIS.

No information given on patient's historical conditions and drug history. The patient's concurrent condition included obesity.

On an unspecified date, the patient started TERBINAFINE (ORAL) (terbinafine) at 250 mg, once daily, orally. The packaged/bottle count was not provided and no additional product information was provided. Terbinafine was withdrawn at the time of reporting. It was unspecified whether TERBINAFINE (ORAL) was used previously.

The patient received co-suspect medication as IBUPROFEN (ibuprofen) at 2400 mg, once daily, orally for intervertebral foraminal encroachment. The packaged/bottle count was not provided and no additional product information was provided. Ibuprofen was withdrawn at the time of reporting. It was unspecified whether IBUPROFEN was used previously.

On 10-JUL-2012 the patient experienced CHOLESTATIC HEPATITIS and therefore the patient was hospitalized. The event was recovering at the time of reporting. Treatment details were not provided.

No reporter causality was provided.

Bayer Vital GmbH, 28.11.2012 DE-BAYER-2012-123794

Sender diagnosis: not reported

Narrative

Serious, Related, Unlisted

This is a spontaneous case report received from a physician via Drug Commission of German Medical Association in GERMANY on 26-NOV-2012 which refers to a 73-year-old female patient who received TERBINAFINE (ORAL) (terbinafine) and IBUPROFEN (ibuprofen) and experienced CHOLESTATIC HEPATITIS.

No information given on patient's historical conditions and drug history. The patient's concurrent condition included obesity.

On an unspecified date, the patient started TERBINAFINE (ORAL) (terbinafine) at 250 mg, once daily, orally. The packaged/bottle count was not provided and no additional product information was provided. Terbinafine was withdrawn at the time of reporting. It was unspecified whether TERBINAFINE (ORAL) was used previously.

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On 10-JUL-2012 the patient experienced CHOLESTATIC HEPATITIS and therefore the patient was hospitalized. The event was recovering at the time of reporting. Treatment details were not provided.

No reporter causality was provided.

sanofi-aventis, 03.12.2012 DE-SANOFI-AVENTIS-2012SA087315

Sender diagnosis: not reported

Narrative

Initial Report received on 27-Nov-2012:

This spontaneous case was extracted from the website of the German Health Authority (authority number DE-BFARM-12311081), initially reported by an anonymous consumer or other non health professional via the Drug Commission of the German Medical Association (case no.: DE-DCGMA-12158657).

The case involves a 73-year-old female patient.

The patient started therapy with ibuprofen (brand unknown) 2400 mg/day orally for intervertebral foraminal encroachment several years ago.

She started therapy with terbinafin (brand unknown) 250 mg/day orally for an unreported indication at an unreported day.

On 10-Jul-2012, she experienced cholestatic hepatitis which required hospitalization. No information on corrective treatment was provided. Therapy with terbinafin as well as ibuprofen was withdrawn. Outcome of the reaction was reported as recovering.

Laboratory results from 10-Jul-2012:

Bilirubin total 2.7 mg/dL, SGOT 682 U/L, SGPT 671 U/L, GGT 780 U/L and alkaline phosphatase 105 U/L.

Laboratory results from 16-Jul-2012:

Bilirubin total 0.9 mg/dL, SGOT 87 U/L, SGPT 311 U/L, GGT 540 U/L and alkaline phosphatase 146 U/L.

Reference ranges were not provided.

No details were provided on relevant history, concomitant diseases and concurrent drugs.

Causality assessment from the Drug Commission of the German Medical Association according to Global Introspection (WHO GI): Possible for terbinafin as well as ibuprofen.

McNeil Germany, Local Safety Officers, 03.12.2012 DE-JNJFOC-20121112904

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority BfArM (Germany) - DE-BFARM-12311081) and concerns a 73 year old female patient from Germany: 02400/12.

The patient's height was 64.57 inches and weight was 209.44 pounds. The patient's medical history was not reported. The patient was treated with ibuprofen (formulation unspecified, oral) 2400 mg a day, initiated on an unspecified date for intervertebral foraminal encroachment. Non-company suspect drugs included: terbinafine hydrochloride (250mg, unspecified dosage form, oral) daily for an unspecified indication. Concomitant medications were not reported. On 10-JUL-2012, the patient developed cholestatic hepatitis. Laboratory data was provided as follows: on 10-JUL-2012: bilirubin total = 2.7 mg/dL, serum glutamic oxaloacetic transaminase (SGOT) 682 IU/L, serum glutamic pyruvic transaminase (SGPT) = 671 IU/L, Gamma-glutamyl transferase (GGT) = 780 IU/L, alkaline phosphatase = 105 IU/L. On 16-JUL-2012: bilirubin total = 0.9 mg/dL, SGOT = 87 IU/L, SGPT = 311 IU/L, GGT = 540 IU/L and alkaline phosphatase 146 IU/L. Treatment with ibuprofen and terbinafine hydrochloride was withdrawn on unspecified date. The patient was recovering from cholestatic hepatitis.

This report was serious (hospitalization).<dbtext>To_Char_NARRATIVE_2</dbtext>

Galenpharma GmbH, 04.12.2012 DE-HRB2237KIP-000178

Sender diagnosis: not reported

Narrative

A 73-year old female (95 kg, 1.64 m) suffered from cholestatic hepatitis after oral treatment with terbinafine (250 mg/day). Co-medication with ibuprofen 2400 mg/day for years due to pain syndrome.

Sender's comment:

GALENpharma assesses the causal relationship between the administration of terbinafine and the adverse event as possible due to the temporal and pharmacological relation. Suspected co-medication ibuprofen, especially because of high dose and very long administration time ('for years'), possible co-factors: elderly and adipositas.

The case is classified as serious, listed according to the current Summary of Product Characteristics (Fachinformation) of TerbiGalen 250 mg Tabletten, tablets.

The quality of documentation is poor (especially indication and treatment duration of terbinafine is missing).

No change of the benefit/risk-ratio derives from this ICSR.

Thus no actions were necessary.

Reporter's comment:

NOVARTIS, 05.12.2012 DE-002147023-GXKR2012DE003665

Sender diagnosis: not reported

Narrative

Case number GXKR2012DE003665, is an initial spontaneous report from a physician received via the Drug Commission of the German Medical Association (DE-DCGMA-12158657) from the German health authority (BfArM homepage, DE-BFARM-12311081) on 26-NOV-2012. This report refers to a 73-year-old female patient with adipositas (BMI 35 kg/m²) and otherwise unknown medical history. No concomitant medication was reported. The patient received TERBINAFINE (MAH unknown) at a daily dose of 250 mg for unknown indication and duration of treatment and IBUPROFEN (MAH unknown) for intervertebral foraminal encroachment at a daily dose of 2400 mg for several years. On 10-JUL-2012 after starting the suspect drug the patient experienced Hepatitis cholestatic. The lab test (normal ranges not provided) of 10Jul2012 showed increased bilirubin total 2.7 mg/dl, increased transaminases with aspartate aminotransferase (SGOT) 682 U/l and alanine aminotransferase (SGPT 671 U/l) and increased gamma glutamyltransferase (GGT) of 780 U/l. This event hepatitis cholestatic was considered serious by the physician. Hepatitis cholestatic resulted in hospitalization of the patient. Treatment with terbinafine and ibuprofen was stopped due to adverse event. Lab data of 16Jul2012 showed declining

liver values with Bilirubin total of 0.9 mg/dl, SGOT 87 U/l, SGPT 311 U/l and GGT 540 U/l. Outcome was provided as condition of Hepatitis cholestatic improved. Hepatitis cholestatic was of suspected causality (possibly-related) to IBUPROFEN as as suspected causality (possibly-related) to TERBINAFINE as reported by the Physician.

001307602, 05.12.2012 DE-ABBOTT-12P-062-1014969-00

Sender diagnosis: not reported

Narrative

Spontaneous report from GERMANY of CHOLESTATIC HEPATITIS with IBUPROFEN TABLETS (IBUPROFEN).

Case received on 26 Nov 2012 from Health Authority; reference number DE-BFARM-12311081.

The patient took IBUPROFEN for years for pain syndrome. On 10 Jul 2012, the patient experienced CHOLESTATIC HEPATITIS. TERBINAFIN (TERBINAFINE HYDROCHLORIDE) was also considered suspect.

Reporter's comment:

NOVARTIS, 05.12.2012 DE-NOVNCHHQ-GXKR2012DE003665

Sender diagnosis: not reported

Narrative

Case number GXKR2012DE003665, is an initial spontaneous report from a physician received via the Drug Commission of the German Medical Association (DE-DCGMA-12158657) from the German health authority (BfArM homepage, DE-BFARM-12311081) on 26-NOV-2012. This report refers to a 73-year-old female patient with adipositas (BMI 35 kg/m²) and otherwise unknown medical history. No concomitant medication was reported. The patient received TERBINAFINE (MAH unknown) at a daily dose of 250 mg for unknown indication and duration of treatment and IBUPROFEN (MAH unknown) for intervertebral foraminal encroachment at a daily dose of 2400 mg for several years. On 10-JUL-2012 after starting the suspect drug the patient experienced Hepatitis cholestatic. The lab test (normal ranges not provided) of 10Jul2012 showed increased bilirubin total 2.7 mg/dl, increased transaminases with aspartate aminotransferase (SGOT) 682 U/l and alanine aminotransferase (SGPT 671 U/l) and increased gamma glutamyltransferase (GGT) of 780 U/l. This event hepatitis cholestatic was considered serious by the physician. Hepatitis cholestatic resulted in hospitalization of the patient. Treatment with terbinafine and ibuprofen was stopped due to adverse event. Lab data of 16Jul2012 showed declining liver values with Bilirubin total of 0.9 mg/dl, SGOT 87 U/l, SGPT 311 U/l and GGT 540 U/l. Outcome was provided as condition of Hepatitis cholestatic improved. Hepatitis cholestatic was of suspected causality (possibly-related) to IBUPROFEN as as suspected causality (possibly-related) to TERBINAFINE as reported by the Physician.

McNeil Germany, Local Safety Officers, 07.12.2012 DE-JNJFOC-20121112904

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority BfArM (Germany) - DE-BFARM-12311081) and concerns a 73 year old female patient from Germany: 02400/12.

The patient's height was 64.57 inches and weight was 209.44 pounds. The patient's medical history was not reported. The patient was treated with ibuprofen (formulation unspecified, oral) 2400 mg a day, initiated on an unspecified date for intervertebral foraminal encroachment. Non-company suspect drugs included: terbinafine hydrochloride (250mg, unspecified dosage form, oral) daily for an unspecified indication. Concomitant medications were not reported. On 10-JUL-2012, the patient developed cholestatic hepatitis. Laboratory data was provided as follows: on 10-JUL-2012: bilirubin total = 2.7 mg/dL, serum glutamic oxaloacetic transaminase (SGOT) 682 IU/L, serum glutamic pyruvic transaminase (SGPT) = 671 IU/L, Gamma-glutamyl transferase (GGT) = 780 IU/L, alkaline phosphatase = 105 IU/L. On 16-JUL-2012: bilirubin total = 0.9 mg/dL, SGOT = 87 IU/L, SGPT = 311 IU/L, GGT = 540 IU/L and alkaline phosphatase 146 IU/L. Treatment with ibuprofen and terbinafine hydrochloride was withdrawn on unspecified date. The patient was recovering from cholestatic hepatitis.

This report was serious (hospitalization).

Additional information was received from a contactable physician via a regulatory authority BfArM (Germany) - DE-BFARM-12311081, via business partner Pfizer (DE-PFIZER-2012296996) on 27-NOV-2012.

This report concerns a female patient of unspecified ethnicity. The patient was treated with ibuprofen for years (unspecified number of years) for pain syndrome. The formulation of terbinafine hydrochloride was updated to tablet previously reported as unspecified. It was reported that the hospitalization was required for the patient due to the event. The laboratory data also included liver biopsy: result was not reported.<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 12.12.2012 DE-JNJFOC-20121112904

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a physician, via a regulatory authority BfArM (Germany) - DE-BFARM-12311081) and concerns a 73 year old female patient from Germany: 02400/12.

The patient's height was 64.57 inches and weight was 209.44 pounds. The patient's medical history was not reported. The patient was treated with ibuprofen (formulation unspecified, oral) 2400 mg a day, initiated on an unspecified date for intervertebral foraminal encroachment. Non-company suspect drugs included: terbinafine hydrochloride (250mg, unspecified dosage form, oral) daily for an unspecified indication. Concomitant medications were not reported. On 10-JUL-2012, the patient developed cholestatic hepatitis. Laboratory data was provided as follows: on 10-JUL-2012: bilirubin total = 2.7 mg/dL, serum glutamic oxaloacetic transaminase (SGOT) 682 IU/L, serum glutamic pyruvic transaminase (SGPT) = 671 IU/L, Gamma-glutamyl transferase (GGT) = 780 IU/L, alkaline phosphatase = 105 IU/L. On 16-JUL-2012: bilirubin total = 0.9 mg/dL, SGOT = 87 IU/L, SGPT = 311 IU/L, GGT = 540 IU/L and alkaline phosphatase 146 IU/L. Treatment with ibuprofen and terbinafine hydrochloride was withdrawn on unspecified date. The patient was recovering from cholestatic hepatitis. This report was serious (hospitalization).

Additional information was received from a contactable physician via a regulatory authority BfArM (Germany) - DE-BFARM-12311081, via business partner Pfizer (DE-PFIZER-2012296996) on 27-NOV-2012.

This report concerns a female patient of unspecified ethnicity. The patient was treated with ibuprofen for years (unspecified number of years) for pain syndrome. The formulation of terbinafine hydrochloride was updated to tablet previously reported as unspecified. It was reported that the hospitalization was required for the patient due to the event. The laboratory data also included liver biopsy: result was not reported.

Additional information was received from a contactable physician via a regulatory authority BfArM (Germany) - DE-BFARM-12311081, via business partner Pfizer (DE-PFIZER-2012296996) on 30-NOV-2012.

The patient's weight was 94.984 kilograms. It was reported that terbinafine hydrochloride (250mg, unspecified dosage form, oral) was confirmed as Non-company suspect drug of Pfizer.<dbtext>To_Char_NARRATIVE_2</dbtext>

Bayer Vital GmbH, 20.02.2013 DE-BAYER-2012-123794

Sender diagnosis: not reported

Narrative

Serious, Related, Unlisted

This is a spontaneous case report received from a physician via Drug Commission of German Medical Association in GERMANY on 26-NOV-2012 which refers to a 73-year-old female patient who received TERBINAFINE (ORAL) (terbinafine) and IBUPROFEN (ibuprofen) and experienced CHOLESTATIC HEPATITIS.

No information given on patient's historical conditions and drug history. The patient's concurrent condition included obesity.

On an unspecified date, the patient started TERBINAFINE (ORAL) (terbinafine) at 250 mg, once daily, orally. The packaged/bottle count was not provided and no additional product information was provided. Terbinafine was withdrawn at the time of reporting. It was unspecified whether TERBINAFINE (ORAL) was used previously.

The patient received co-suspect medication as IBUPROFEN (ibuprofen) at 2400 mg, once daily, orally for intervertebral foraminal encroachment. The packaged/bottle count was not provided and no additional product information was provided. Ibuprofen was withdrawn at the time of reporting. It was unspecified whether IBUPROFEN was used previously.

On 10-JUL-2012 the patient experienced CHOLESTATIC HEPATITIS and therefore the patient was hospitalized. The event was recovering at the time of reporting. Treatment details were not provided.

No reporter causality was provided.

**Follow up information received on 05-FEB-2013.
BfArM (regulatory authority) was added as reporter.
No new information was obtained despite requests.

Addendum opened, based on initial report:
The reporter considered the event CHOLESTATIC HEPATITIS related to both products.

****Follow-up received on 18-FEB-2013, from physician:**

It was reported that patient used Ibuprofen for some years for pain syndrome due to spinal canal stenosis.

On 10-JUL-2012, the patient experienced STEATOHEPATITIS, MODERATELY ACTIVE. The event CHOLESTATIC HEPATITIS was updated to CHOLESTATIC HEPATITIS, MILD, POSSIBLY DRUG-INDUCED.

On 16-Jul-2012, liver biopsy showed moderately active steatohepatitis on the basis of fatty liver, accompanied by mild portal fibrosis with isolated incomplete septal formation. In addition, a mild cholestatic hepatitis was found, possibly drug-related. No signs of malignancy.

The events clearly recovered after discontinuation of both suspect products.

No further information was provided.

INTENDIS DEUTSCHLAND, 20.02.2013 DE-BAYER-2012-123794

Sender diagnosis: not reported

Narrative

Serious, Related, Unlisted

This is a spontaneous case report received from a physician via Drug Commission of German Medical Association in GERMANY on 26-NOV-2012 which refers to a 73-year-old female patient who received TERBINAFINE (ORAL) (terbinafine) and IBUPROFEN (ibuprofen) and experienced CHOLESTATIC HEPATITIS.

No information given on patient's historical conditions and drug history. The patient's concurrent condition included obesity.

On an unspecified date, the patient started TERBINAFINE (ORAL) (terbinafine) at 250 mg, once daily, orally. The packaged/bottle count was not provided and no additional product information was provided. Terbinafine was withdrawn at the time of reporting. It was unspecified whether TERBINAFINE (ORAL) was used previously.

The patient received co-suspect medication as IBUPROFEN (ibuprofen) at 2400 mg, once daily, orally for intervertebral foraminal encroachment. The packaged/bottle count was not provided and no additional product information was provided. Ibuprofen was withdrawn at the time of reporting. It was unspecified whether IBUPROFEN was used previously.

On 10-JUL-2012 the patient experienced CHOLESTATIC HEPATITIS and therefore the patient was hospitalized. The event was recovering at the time of reporting. Treatment details were not provided.

No reporter causality was provided.

****Follow up information received on 05-FEB-2013.**

BfArM (regulatory authority) was added as reporter.

No new information was obtained despite requests.

Addendum opened, based on initial report:

The reporter considered the event CHOLESTATIC HEPATITIS related to both products.

****Follow-up received on 18-FEB-2013, from physician:**

It was reported that patient used Ibuprofen for some years for pain syndrome due to spinal canal stenosis.

On 10-JUL-2012, the patient experienced STEATOHEPATITIS, MODERATELY ACTIVE. The event CHOLESTATIC HEPATITIS was updated to CHOLESTATIC HEPATITIS, MILD, POSSIBLY DRUG-INDUCED.

On 16-Jul-2012, liver biopsy showed moderately active steatohepatitis on the basis of fatty liver, accompanied by mild portal fibrosis with isolated incomplete septal formation. In addition, a mild cholestatic hepatitis was found, possibly drug-related. No signs of malignancy.

The events clearly recovered after discontinuation of both suspect products.

No further information was provided.

ALM-HQ, 28.02.2013 DE-ALM-HQ-DE-2012-1577

Sender diagnosis: not reported

Narrative

Initial information received on 26 Nov 2012:

ICSR of a serious (hospitalization) adverse drug reaction received from a healthcare professional (Physician) via German Regulatory Authority (BfArM, ref. number DE-BFARM-12311081). Duplicate number DE-DCGMA-12158657 from the Drug Commission of the German Medical Association.

A 73-year-old elderly female patient (95 kg, 164 cm) with unknown medical history took TERBINAFIN orally 250 mg per day. She also took as suspect drug ibuprofen (trade name unknown) orally 2400 mg per day, for intervertebral foraminal encroachment with pain syndrome, taken for years and stopped on an unknown date.

On 10 July 2012, the patient experienced cholestatic hepatitis. A laboratory test was performed on 10 Jul 2012 showing the following results: Bilirubin total 2.7 mg/dL, SGOT 682 U/L, SGPT 671 U/L, GGT 780 U/L and Alkaline phosphatase 105 U/L. On 16 Jul 2012, a second test was performed with

following results: Bilirubin total 0.9 mg/dL, SGOT 87 U/L, SGPT 311 U/L, GGT 540 U/L and Alkaline phosphatase 146 U/L. A liver biopsy was also performed on an unknown date with unknown result. Treatment with TERBINAFIN was stopped on an unreported date. The patient is recovering at the time of reporting.

The reporting Health Authority has assessed this reaction as serious and possibly related to TERBINAFIN and to ibuprofen according to Global Introspection (WHO GI) method.

Closing information 03 Jan 2013:

No further information could be obtained from the reporter after two attempts. The case was considered closed.

Follow-up information received on 18 Feb 2013:

On 13 July 2012, another laboratory test was performed showing the following results: Bilirubin total 2.4 mg/dL, SGOT 272 U/L, SGPT 470 U/L, GGT 783 U/L and Alkaline phosphatase 155 U/L. The microscopic histologic result showed a steatohepatitis at hepatic steatosis. There was also a mild cholestatic hepatitis, which could be an indication of a medicamentous toxic concomitant component.

Corporate Drug Safety comment:

This serious case (hospitalization) received from a healthcare professional (physician) via German Regulatory Authority BfArM concerns a 73-year-old female patient with unknown medical history who took TERBINAFINE orally 250 mg per day for an unknown duration for an unreported indication. She also took ibuprofen (trade name unknown) orally 2400 mg per day as suspect drug, for intervertebral foraminal encroachment with pain syndrome, for years.

On 10 July 2012, the patient experienced CHOLESTATIC HEPATITIS. On days 10 July, 13 July and 16 July 2012 laboratory tests were performed (Bilirubin: total 2.7 mg/dL vs 2.4 mg/dL vs 0.9 mg/dL; SGOT: 682 U/L vs 272 U/L vs 87 U/L; SGPT: 671 U/L vs 470 U/L vs 311 U/L; GGT: 780 U/L vs 783 U/L vs 540 U/L; Alkaline phosphatase: 105 U/L vs 155 U/L vs 146 U/L). A liver biopsy specimen was received at the testing laboratory on 16 July 2012 and the histologic result showed a steatohepatitis at hepatic steatosis. There was also a mild cholestatic hepatitis, which could be an indication for a medicamentous toxic concomitant component.

Patient was hospitalized on an unreported date. Treatments with TERBINAFINE and ibuprofen were stopped on unreported dates. According to last received information it is stated that liver values decreased after stop of treatments with TERBINAFINE and ibuprofen. It is unknown whether treatment with any of the two drugs were restarted.

The patient is recovering at the time of reporting.

The reporting Health Authority has assessed this reaction as serious and possibly related to TERBINAFINE and to ibuprofen.

Due to lack of sufficient information on e.g. concomitant diseases and concomitant medication details (e.g. start and duration of TERBINAFINE therapy) the company's causality is given as not assessable.

CHOLESTATIC HEPATITIS is listed in the reference safety information for Almirall's TERBINAFINE product MYCONORMIN as cholestasis and hepatitis are listed. Hepatitis of a hepatic steatosis (= steatohepatitis) is not listed in the reference safety information for Almirall's TERBINAFINE product MYCONORMIN.

Follow-up information will be requested.

001307602, 04.03.2013 DE-ABBOTT-12P-062-1014969-00

Sender diagnosis: not reported

Narrative

Spontaneous report from GERMANY by a physician of a 73 year old female with events of CHOLESTATIC HEPATITIS and STEATOHEPATITIS with IBUPROFEN. There was no reported medical history.

Case received on 21-FEB-2013 from Health Authority; reference number DE-BFARM-12311081 and DE-DCGMA-12158657.

The patient took IBUPROFEN for years for pain syndrome. On an unknown date, the patient experienced STEATOHEPATITIS. On 10 Jul 2012, the patient experienced CHOLESTATIC HEPATITIS. TERBINAFIN (TERBINAFINE HYDROCHLORIDE) was also considered suspect. After discontinuation of the suspect drugs, the liver values clearly declined.

Relevant Laboratory & Other Diagnostic Tests

 10 Jul 2012 Alkaline phosphatase: 105 U/L
 10 Jul 2012 Alkaline phosphatase: 146 U/L
 13 Jul 2012 Alkaline Phosphatase: 155 U/L
 16 Jul 2012 Alkaline Phosphatase: 146 U/L
 13 Jul 2012 ALT: 470 U/L
 13 Jul 2012 AST: 272 U/L
 10 Jul 2012 Bilirubin total: 2.7 MG/DL
 13 Jul 2012 Bilirubin total: 2.4 MG/DL
 16 Jul 2012 Bilirubin total: 0.9 MG/DL
 10 Jul 2012 GGT: 780 U/L
 13 Jul 2012 GGT: 783 U/L

16 Jul 2012 GGT: 540 U/L
16 Jul 2012 Needle biopsy of Liver:
10 Jul 2012 SGOT: 682 U/L
16 Jul 2012 SGOT: 87 U/L
10 Jul 2012 SGPT: 671 U/L
16 Jul 2012 SGPT: 311 U/L

16-Jul-2012: Needle biopsy of the liver: Result: Material: six histoid pieces of 0.1 up to 1.2 cm length. Finding: liver parenchyma with an approx. 75 % fatty liver cells of varying drop like appearance. Infiltration of neurophilic granuloocytes. Lymphoid infiltrate, signs of an intracellular cholestasis. Diffusion of portal fields with particular with formation of septa. Assessment: Moderate active steatohepatitis for fatty liver, small portal fibrosis with particular incomplete formation of septa, low-grade cholestatic hepatitis, which point out to a medicinal-toxic concomitant component. No evidence for a malignity.

Change History

On 21 Feb 2013, received updates to patient demographics, event information, reporter opinion of causality, suspect drug information, laboratory/diagnostic procedures and narrative description. The serious event of "STEATOHEPATITIS" was added.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13006749

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: 19730820	Age: 38 Year	weight (kg): 97	BMI_int: 31.31	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 GPT increased	201202				not recovered/not resolved
2 GGT increased	201202				not recovered/not resolved
3 Alanine aminotransferase increased	201202				not recovered/not resolved
4 Gamma GT increased	201202				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 IBUPROFEN ibuprofen	Unknown	600 Mg milligram(s)/Day	-	Lumbar syndrome

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Migraine			Unknown	
Abstains from alcohol			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
GGT	201202	115	IU/L (intern	
GPT	201202	78	IU/L (intern	
Hepatitis C virus test		negative		
Hepatitis A virus test		negative		

Narratives (nur bei elektronischem Eingang)

Drug Commission of the German Medical Association, 06.02.2013 DE-DCGMA-12159800

Sender diagnosis: not reported

NarrativeSender's comment:Reporter's comment:

Patient trinkt keinen Alkohol!
HAV-, HBs-, HBc-, HCV- negativ

McNeil Germany, Local Safety Officers, 20.02.2013 DE-JNJFOC-20130208507

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a contactable physician, via a regulatory authority (BfArM (Germany) - DE-BFARM-13006749), via a business partner (Pfizer Inc 2013053962) and concerns a 38-year-old male patient of unspecified ethnicity from Germany:

The patient's height was 176 centimeters and weight was 97 kilograms. The patient's medical history and concurrent conditions included: abstains from alcohol and migraine. The patient was treated with ibuprofen (unspecified formulation and route of administration) 600 mg once a day, initiated on an unspecified date for lumbar syndrome. Concomitant medications were not reported. On an unspecified date in FEB-2012, the patient experienced alanine aminotransferase (GPT) increased and gamma glutamyltransferase (GGT) increased. The patient underwent lab tests and procedures which included: alanine aminotransferase: 78 IU/l and gamma-glutamyltransferase: 115 IU/l. HAV-, HBs-, HBc-, HCV- on an unspecified date were negative. Treatment with ibuprofen was withdrawn on an unspecified date. The patient had not recovered from alanine aminotransferase increased and gamma glutamyltransferase increased.

This report was serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

McNeil Germany, Local Safety Officers, 28.02.2013 DE-JNJFOC-20130208507

Sender diagnosis: not reported

Narrative

This spontaneous report was received from a contactable physician, via a regulatory authority (BfArM (Germany) - DE-BFARM-13006749), via a business partner (Pfizer Inc 2013053962) and concerns a 38-year-old male patient of unspecified ethnicity from Germany:

The patient's height was 176 centimeters and weight was 97 kilograms. The patient's medical history and concurrent conditions included: abstains from alcohol and migraine. The patient was treated with ibuprofen (unspecified formulation and route of administration) 600 mg once a day, initiated on an unspecified date for lumbar syndrome. Concomitant medications were not reported. On an unspecified date in FEB-2012, the patient experienced alanine aminotransferase (GPT) increased and gamma glutamyltransferase (GGT) increased. The patient underwent lab tests and procedures which included: alanine aminotransferase: 78 IU/l and gamma-glutamyltransferase: 115 IU/l. HAV-, HBs-, HBc-, HCV- on an unspecified date were negative. Treatment with ibuprofen was withdrawn on an unspecified date. The patient had not recovered from alanine aminotransferase increased and gamma glutamyltransferase increased.

This report was serious (medically significant).

No new information was received from the business partner Pfizer (DE-PFIZER-2013053962) on 20-FEB-2013. No changes were made to the report.

<dbtext>To_Char_NARRATIVE_2</dbtext>

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13007342

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Male	Birth: 19620916	Age: 50 Year	weight (kg): 74	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Multiple drug overdose intentional	20130123				Unknown
2 Quick's value decreased	20130123				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CITALOPRAM citalopram	Oral	200 Mg milligram(s)/ 245	20130123-20130123	Suicide attempt
2 decortin prednisone	Oral	20 Df dosage form/ 245	20130123-20130123	Suicide attempt
3 ethanol ethanol	Oral	/	20130123-20130123	Suicide attempt
4 ibuprofen ibuprofen	Oral	12000 Mg milligram(s)/ 245	20130123-20130123	Suicide attempt
5 MARCUMAR phenprocoumon	Oral	/	20130123-20130123	Suicide attempt
6 meteospasmyl alverine citrate; methionine	Oral	/	20130123-20130123	Suicide attempt
7 SALOFALK mesalazine	Oral	10000 Mg milligram(s)/ 245	20130123-20130123	Suicide attempt
8 simeticon simeticone	Oral	/	20130123-20130123	Suicide attempt

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Self inflicted laceration	20130123		Yes	
Arterial laceration	20130123		Yes	

Primary source

Qualification: Physician observestudytype:

Literature:

Laborwerte/Tests/Diagnostik

Testname/Diagnostik	Datum	Ergebnis	Maßeinheit	weitere Erläuterung
Quick's test	20130123	31	%	
Quick's test	20130124	24	%	
Quick's test	20130125	29	%	

Narratives (nur bei elektronischem Eingang)

MEDA Pharma GmbH & Co. KG, 08.02.2013 DE-MEDA-M-EU-2013010294

Sender diagnosis: not reported

Narrative

A report was received by the GIFTINFORMATIONSZENTRUM (German Drug and Poison Information Call Centre, ref. no. AZ 2013001778) on 23-Jan-2013.

A hospital physician called and asked for advice on 23-Jan-2013. He reported on a 50-year-old male patient (weight 74 kg) who attempted suicide by taking an intentional overdose of an unknown amount of Marcumar (phenprocoumon) orally with following co-suspect medication: 20 tablets of ibuprofen 600 (total dose of 12000 mg), 20 tablets of Decortin (prednison), 10-28 tablets of escitalopram together with an unknown amount of meteospasmyl, Simeticon (simethicone) and other unknown human medical tablets. All drugs were taken orally but intake was not assured. The patient had also cut open his radial artery. He was scheduled for surgery which requires full anesthesia.

The GIFTINFORMATIONSZENTRUM recommended the following diagnostics and therapies: Medical examination and inpatient treatment, observation, monitoring, laboratory test including control of coagulation and renal parameters. Administration of stomach protection and if necessary vitamin K and symptomatic therapy.

At the time of reporting, the patient had not developed any adverse event and the outcome was unknown.

A follow-up report was created by the GIFTINFORMATIONSZENTRUM on 02-Feb-2013.

In this report previously reported escitalopram was modified to citalopram 10. In addition to previously reported drug the patient also took an overdose of 20 tablets citalopram 10 (total dose of 200 mg) and 20 tablets of Salofalk 500 (mesalazine, total dose of 10 000 mg). She also drank an unknown amount of Rum (alcohol). All taken also orally but intake was not assured. The patient had not developed any symptom and the outcome was unknown at the time of this report.

After the first recommendation the following diagnostic and therapeutic measures were performed: intensive care treatment and administration of electrolyte solution, observation, and monitoring, psychiatric examination, laboratory tests, including blood gas analysis, coagulation, renal parameters and ECG (unsuspicious).

Laboratory tests revealed Quick's values of 31 % on 23-Jan-2013, 24 % on 24-Jan-2013 and 29 % on 25-Jan-2013.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13020792

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: 19810508	Age: 31 Year	weight (kg): 66	BMI_int: 21.55	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute liver failure	20130126				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ciprofloxacin ciprofloxacin	Oral	/	20121227-20130102	Postoperative antibiotic cover 7 Day
2 Ibuprofen Abbott 400 mg ibuprofen	Oral	/	20121229-20130109	Postoperative analgesia 12 Day

Drugs (concomitant)

3 fragmin dalteparin sodium	2 Df dosage form/Day	20120122-20130102	Postoperative care 11 Month
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Factor V Leiden mutation			k.A.	
Alcohol use			k.A.	sehr selten
Breast reconstruction	20121227		No	

Primary source

Qualification: Physician

observestudytype:

Literature:

number of reports: 165